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A Pluralist Approach

HOWARD JACOB KARGER | DAVID STOESZ



EIGHTH EDITION

American Social Welfare Policy

A Pluralist Approach

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PREFACE

The years since the publication of the last full edition have been marked by dramatic events on the domestic and international fronts. Although the U.S. economy bounced back from the global financial crisis (GFC) of 2007 to 2008, the post-recession gains were largely realized by the top 1 percent of U.S. wage earners who accounted for 85 percent of total income growth from 2009 to 2013. By 2013, the 1.6 million families in the top 1 percent earned 25 times more than the 161 million families in the bottom 99 percent. It is little wonder that this egregious income inequality led to large numbers of angry people, which in turn, fueled the rise of presidential candidate Donald Trump.

The international front was especially turbulent as the Arab Spring toppled or destabilized governments in Tunisia, Egypt, Yemen, Libya, and Syria. By 2016, at the same time, the war in Iraq and Afghanistan continued to drag on. By mid-2016, an increasingly bloody civil war in Syria claimed more than 400,000 lives and was largely responsible for the 1 million immigrants that entered Europe in 2015.

The instability in the Middle East led to the creation of ISIL (Islamic State), a militant group directly or indirectly responsible for numerous massacres, including the 2015 attack on the Bataclan Theatre near Paris (130 dead and 368 injured); the 2015 Ankara, Turkey, bombing (102 dead and 400 injured); the 2015 San Bernardino attack (14 dead and 24 injured); and the 2016 Orlando nightclub attack (49 dead and 53 injured). Western nations continue the struggle to find a balance between protecting privacy, civil liberties, and public safety.

Despite the domestic and international challenges, the way forward was stymied as the federal government was virtually paralyzed by the Republican Party's control of the Senate and the House of Representatives. With the death of Antonin Scalia, even the Supreme Court was divided between liberals and conservatives. A divided government resulted in a virtual standstill of policy options.

In the midst of this virtual paralysis, several important policy developments emerged in the first term of the Obama administration. Some of these achievements include the Dodd-Frank Wall Street Reform and Consumer Protection Act, one of the

most significant financial reform acts since the Great Depression; the Patient Protection and Affordable Care Act of 2010 (known as Obamacare); repeal of the military's Don't Ask Don't Tell (DADT) rule; the Credit Card Accountability, Responsibility, and Disclosure Act; and the Children's Health Insurance Program Reauthorization Act of 2009 (CHIPS).

The presidential election of 2012 proved to be one of the most acrimonious in recent memory. Extreme Republican Party ideologues drove the party far to the right in areas such as contraception, abortion, health care, voter's rights, and immigration. In the end, President Obama's moderate approach triumphed as he won 303 electoral votes compared to Mitt Romney's 206 votes. The election illustrated the sharp divisions in American society between the more liberal Northeast, West Coast, some Western and Midwestern states, and the more conservative South and rural areas. These patterns reflect differing perceptions of where America should be heading.

The acrimony of the 2012 presidential election was far eclipsed by the 2016 presidential election, as Republican candidates vied to outdo each other in appealing to the white and increasingly conservative base of the party. Extremist candidates like Ted Cruz and Donald Trump handily defeated more moderate candidates such as Jeb Bush and John Kasich.

On the Democratic side, Vermont Senator Bernie Sanders introduced a European-style democratic socialist vision to mainstream American politics. Defying all odds, Sanders won several primaries against favored Democratic nominee Hillary Clinton, including Oregon, North Dakota, Minnesota, New Hampshire, Michigan, Indiana, and Vermont. In the raw primary vote count, Sanders received 12 million votes compared to Clinton's 15.8 million.

In one of the most shocking upsets in recent political history, Donald Trump beat Hillary Clinton for the presidency, despite some polls showed her chances for victory at between 70 and 99 percent. Progressives of all ilk and Democrats were in shock, disbelief, and fear.

Several changes will be required if human service professionals are to reclaim a prominent role in social policy that they had at the turn of the century through luminaries such as Jane Addams, Lillian

Wald, Grace and Edith Abbott, Mary Simkhovitch, and others. Markets have been a primary means of distributing goods and services to the non-poor, and the application of market dynamics to low-income families should be evaluated on merit, not discarded solely on ideological grounds. State and local politics have been important arenas for introducing innovations in social welfare and for providing social workers a first step on the ladder of public service. Such opportunities should be celebrated, not dismissed.

Public policy involves the kind of power that occurs in three basic forms: money, votes, and networks. Although these resources have been the staple of politics, the information age requires players to possess a higher level of sophistication. To be competitive, one must have command of information systems, large data sets, and complex decision menus.

If social work can educate students about these methods and begin to insert itself into the policy environment, the profession will again become an influential force in social policy. On the other hand, if the profession rests on its historic laurels, it will remain tangential in the policy arena. Such an eventuality would essentially waste the substantial assets that social work brings to social affairs: a distinguished legacy, the altruism of the young, and a unique moral imperative.

This edition of *American Social Welfare Policy* attempts to provide the information necessary for understanding social welfare policy nationally and internationally. In addition to discussing the basic concepts, policies, and programs that comprise

American welfare state, the text includes information on the voluntary nonprofit sector, the for-profit corporate sector, and the new strategy in social policy (i.e., tax policy and expenditures). The penultimate chapter examines food policy, and environmental and sustainability issues. The final chapter examines the influence of global capitalism, a development that not only weds the developed nations to the undeveloped nations but also in the process shifts capital and jobs in unprecedented numbers. In recognition of our increasingly interconnected global environment, this edition has put more emphasis on international social welfare policy.

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American Social Welfare Policy, they have coauthored three other books: *The Politics of Child Abuse in America* (with Lela Costin) (Oxford University Press, 1996); *Reconstructing the American Welfare State* (Rowman and Littlefield, 1992); and (with Terry Carrillo), *A Dream Deferred* (Aldine, 2010). Howard's book, *Short-changed: Life and Debt in the Fringe Economy* (Berret-Koehler, 2005) examines the financial practices and products that exploit millions of American families. The book won the 2006 Independent Publishers Award in Finance/Investment/Economics. David's book, *Quixote's Ghost: The Right, the Liberati, and the Future of Social Policy* (Oxford University Press, 2005), explains how conservatives have assumed control of domestic policy and proposes a new framework for social policy. *Quixote's Ghost* won the 2006 Pro-Humanitate Literary Award.

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Social Policy and the American Welfare State



Source: Jeff Greenberg/The Image Works

Social welfare policy is arguably best viewed through the lens of political economy (i.e., the interaction of economic, political, and ideological forces). This chapter provides an overview of the American welfare state through that lens. In particular, it examines various definitions of social welfare policy, the relationship between social policy and social problems, and the values and ideologies that drive social welfare in the United States. In addition, the chapter examines the effects of ideology on the U.S. welfare state, including the important roles played by conservatism and liberalism (and their variations) in shaping welfare policy. An understanding of social welfare policy requires the ability to grasp the economic justifications and consequences that underlie policy decisions. As such, this chapter contains a brief introduction to Keynesianism, free market economics, socialism, and communitarianism, among others.

American social welfare is in transition. Starting with the Social Security Act of 1935, liberals argued that federal social programs were the best way to help the disadvantaged. Now, after 70 years of experimenting with the [welfare state](#), a discernible shift has occurred. The conservatism of U.S. culture—so evident in the Reagan, Bush (both Bushes), and even Clinton and Obama presidencies—has left private institutions to shoulder more of the welfare burden. For proponents of [social justice](#), the suggestion that the private sector should assume more responsibility for welfare represents a retreat from the hard-won governmental, social legislation that provided essential benefits to millions of Americans. Justifiably, social advocates fear the loss of basic goods and services during the transition in social welfare.

The election of Barack Obama as the 44th President of the United States in 2008 not only broke a racial barrier but also promised to sweep away the strident conservatism that had defined the presidency of George W. Bush. The Obama victory, with 52 percent of the vote and increased Democratic majorities in both chambers of Congress, heartened liberals who had anticipated an expansion of government social programs. However, the euphoria among liberals soon gave way to despair as the Democratic Party lost control of the House of Representatives and barely held on to the Senate in the midterm elections of 2010. Although Obama won the presidency for a second term in 2012, the

midterm election of 2014 saw the Democratic Party also losing control of the Senate.

While liberal pundits hailed the resurgence of “a vast new progressive movement,”¹ structural limits and the emergence of a strong reactive element would restrain Obama’s ambitions. Massive deficits left by the Bush administration, compounded by a severe global financial crisis and two unfunded wars, meant that economic issues would trump other priorities. Reduced tax revenues would impede the ability of the government to meet existing obligations, let alone expand social programs. Obama’s centrist inclinations to build bipartisan support for his legislative agenda failed as newly elected extremist Tea Party legislators squashed most of his attempts at compromise. Instead, ideologically driven legislators focused on social issues such as abortion, and even resuscitated previously long-dead issues like contraception. Parts of the nation had not just turned right, but hard right. The fires were further stoked by allegations around Obama’s birth certificate, whether he was a Muslim and a socialist, and so forth.

The 2012 presidential election was marked by the often extreme positions taken by Republican presidential contenders. Long-dormant issues resurfaced as Republican candidates vied for the support of the religious right and Tea Partiers. This political climate led to an anti-science orientation, often reflected in wildly unsubstantiated claims like birth control pills can cause prostate cancer. Former Missouri Republican congressperson Todd Akin stated that doctors had told him it is extremely rare for “legitimate” rape victim to become pregnant: “If it’s a ‘legitimate’ rape, the female body has ways to try to shut that whole thing down.”² Despite the lack of any medical evidence, former Republican presidential candidate Michelle Bachmann warned that mental retardation could occur from the HPV (human papilloma virus) vaccine.

The 2016 presidential race continued the Republican Party’s anti-science orientation. U.S. Senator and presidential candidate Ted Cruz compared himself to Galileo when he stated that “Today, the global warming alarmists are the equivalent of the flat-Earthers. It used to be [that] it is accepted scientific wisdom the Earth is flat, and this heretic named Galileo was branded a denier.” For one, Galileo never argued against flat-Earthers; instead he argued that the Earth revolved around the sun. Second, he never disputed the scientific data

of his time. On the contrary, Galileo imposed scientific data where there was none.³ Presidential candidates like former Texas Governor Rick Perry and Rick Santorum believe that evolution is still an open question. Virtually all 2016 Republican presidential contenders argued that climate change is either a hoax or unrelated to human activity.⁴ The same denial of climate change is also true for Donald Trump who was elected in one of the most contentious political races in modern history.

Nowhere is the power of conservatives more evident than in gun control. Despite the spate of mass shootings—no gun legislation has been passed. The response by the National Rifle Association (NRA) is to have more—not less—guns. The response of many politicians was to pray for the families of the dead.

The harsh rhetoric illustrates the nation's regional fissures. For instance, a 2012 Public Policy poll of registered Republican voters found that 45 percent of Alabamians and 52 percent of Mississippians believed that Obama is a Muslim (the other 40 percent were not sure). Only about 25 percent of those voters believed in evolution.⁵ These fissures were the most apparent in the surprise victory of Donald Trump over Hillary Clinton in the 2016 presidential race.

All told, the 2016 presidential campaigns cost billions, much of that coming from super Political Action committees (PACs). However, the final list of 2016 expenditures might never be known since some of the biggest spending groups were nonprofit organizations that were permitted to hide their spending from public scrutiny.⁶ This spending spree was spurred on by the U.S. Supreme Court decision in *Citizens United v. Federal Election Commission*. The Court ruled that the First Amendment prohibited government from restricting independent political expenditures by corporations and unions.⁷ This decision overturned a century-old precedent allowing the government to regulate such spending.

Structural features of the American welfare state militate against a major expansion of government, per se. A pluralistic mix of private and public services is an overriding feature of U.S. social welfare. As in other realms, such as education, in social welfare, private institutions coexist alongside those of the public sector. U.S. social welfare has a noble tradition of voluntary citizen groups taking the initiative to solve local problems. Today, private voluntary groups provide valuable services to AIDS patients, the homeless, immigrants, victims of domestic violence, and refugees.

Social welfare has become big business. During the last 30 years, the number of human service corporations—for-profit firms providing social welfare through the marketplace—has increased dramatically. Human service corporations are prominent in long-term nursing care, health maintenance, child day care, psychiatric and substance abuse services, and even corrections. For many welfare professionals, the privatizing of social services is troubling, occurring as it does at a time when government has reduced its commitment to social programs. Yet, human service corporations will likely continue to be prominent players in shaping the nation's social welfare policies. As long as U.S. culture is democratic and capitalistic, entrepreneurs will be free to establish social welfare services in the private sector, both as nonprofit agencies and as for-profit corporations.

The **mixed welfare economy** of the United States, in which the voluntary, governmental, and corporate sectors coexist, poses serious questions for social welfare policy. To what extent can voluntary groups be held responsible for public welfare, given their limited fiscal resources? For which groups of people, if any, should government divest itself of responsibility? Can human service corporations care for poor and multiproblem clients while continuing to generate profits? Equally important, how can welfare professionals shape coherent social welfare policies, given the fragmentation inherent in such pluralism? Clearly, the answers to these questions have much to say about how social welfare programs are perceived by human service professionals, their clients, and the taxpayers who continue to subsidize social programs.

The multitude of questions posed by the transition of social welfare is daunting. Temporarily satisfied by the 1996 welfare reform bill, conservatives have shifted their attention to advocating privatization of **social insurance** programs such as Social Security and Medicare. Past advocates of social justice such as Jane Addams, Whitney Young Jr., and Wilbur Cohen, to name a few, interpreted the inadequacy of social welfare provision as an opportunity to further social justice. It remains for another generation of welfare professionals to demonstrate the same imagination, perseverance, and courage to advance social welfare in the years ahead. Those accepting this challenge will need to be familiar with the various meanings of social welfare policy, differing political and economic explanations of social welfare, and the multiple interest groups that have emerged within the U.S. social welfare system.

Definitions of Social Welfare Policy

The English social scientist Richard Titmuss defined **social services** as “a series of collective interventions that contribute to the general welfare by assigning claims from one set of people who are said to produce or earn the national income to another set of people who may merit compassion and charity.”⁸ Welfare policy, whether it is the product of governmental, voluntary, or corporate institutions, is concerned with allocating goods, services, and opportunities to enhance social functioning.

William Epstein defined **social policy** as “social action sanctioned by society.”⁹ Social policy can also be defined as the formal and consistent ordering of human affairs. **Social welfare policy**, a subset of social policy, regulates the provision of benefits to people to meet basic life needs, such as employment, income, food, housing, health care, and relationships.

Social welfare policy is influenced by the context in which benefits are provided. For example, social welfare is often associated with legislatively mandated programs of the **governmental sector**, such as **Temporary Assistance for Needy Families (TANF)**. In the TANF program, social welfare policy consists of the rules by which the federal and state governments apportion cash benefits to an economically disadvantaged population. TANF benefits are derived from general revenue taxes (often paid by citizens who are better-off). But this is a simplification of benefits provided to those deemed needy. Benefits provided through governmental social welfare policy include cash, along with noncash or in-kind benefits, including personal social services.¹⁰ Cash benefits can be further divided into social insurance and public assistance grants (discussed in depth in Chapters 10 and 11).

In-kind benefits (provided as proxies for cash) include benefits such as food stamps; Medicaid; housing vouchers; Women, Infants, and Children (WIC) coupons; and low-income energy assistance. Personal social services are designed to enhance relationships between people as well as institutions, such as individual, family, and mental health treatment; child welfare services; rehabilitation counseling; and so forth. Although complicated, this classification reflects a common theme—the redistribution of resources from the better-off to the more disadvantaged. This redistributive aspect of social welfare policy is generally accepted by those who view social welfare as a legitimate function

of the state. Governmental social welfare policy is often referred to as “public” policy because it is the result of decisions reached through a legislative process intended to represent the entire population.

But social welfare is also provided by nongovernmental entities, in which case social welfare policy is a manifestation of “private” policy. For example, a nonprofit agency with a high demand for its services and limited resources may establish a waiting list as agency policy. As other agencies adopt the same strategy for rationing services, clients begin to pile up on waiting lists, and some are eventually denied services. Or consider the practice of “dumping,” a policy that has been used by some private health care providers to abruptly transfer uninsured patients to public hospitals while they are suffering from traumatic injuries. Rescission refers to terminating an insurance policy due to concealment, misrepresentation, or fraud. In health insurance, it refers to terminating a policy following the diagnosis of an expensive illness, with the insurance company claiming the policyholder withheld relevant information about a **pre-existing medical condition**. Although partially limited by the Patient Protection and Affordable Care Act of 2010, it continues in some form by some insurance companies. Patients sometimes die as a result of private social welfare policy.

Because U.S. social welfare has been shaped by policies of governmental and nonprofit agencies, confusion exists about the role of for-profit social service firms. The distinction between the public and private sectors was traditionally marked by the boundary between governmental and nonprofit agencies. Profit-making firms are “private” nongovernmental entities that differ from the traditional private voluntary agencies because they operate on a for-profit basis. Within private social welfare, it is therefore necessary to distinguish between policies of for-profit and nonprofit organizations. A logical way to redraw the social welfare map is to adopt the following definitions: *Governmental social welfare policy* refers to decisions made by the state, *voluntary social welfare policy* refers to decisions reached by nonprofit agencies, and *corporate social welfare policy* refers to decisions made by for-profit firms.

Social Problems and Social Welfare Policy

Social welfare policy often develops in response to social problems. The relationship between social problems and social welfare policy is not linear, and

not all social problems result in social welfare policies. Or, social welfare policies are funded at such low levels that they are ineffectual. For example, the Child Abuse Prevention and Treatment Act of 1974 was designed to ameliorate the problem of child abuse, yet underbudgeting left Child Protective Service (CPS) workers unable to promptly investigate the increase in child abuse reports, resulting in many children dying or undergoing serious injury.

Social welfare is an expression of social altruism that contributes to the maintenance and survival of society by helping to hold together a society that can fracture along social, political, and economic stress lines. Social welfare policy is also useful in enforcing social control, especially as a proxy for more coercive law-based measures.¹¹ Simply put, the poor are less likely to revolt against the unequal distribution of wealth and privilege when their basic needs are met. Social welfare benefits also subsidize employers by supplementing low and non-livable wages, thereby maintaining a work incentive. Without social welfare benefits like earned income tax credit (EITC), employers would have to raise wages and therefore consumer prices. Social welfare benefits also support key industries, such as agriculture (food stamps), housing (e.g., Section 8), and health care (e.g., Medicaid and Medicare). If welfare benefits were suddenly eliminated, several U.S. businesses would collapse, and prices for many goods and services would rise. Social welfare benefits help stabilize prices and maintain economic growth.

Social welfare policies also relieve the social and economic dislocations caused by the uneven nature of economic development under **capitalism**. For example, one of the main features of capitalism is a constantly changing economy where jobs are created in one sector and lost (or exported) in another, thereby resulting in large islands of unemployed workers. Examples of this include closing Blockbuster, Borders, Radio Shack, Circuit City, and other retail store outlets. The increased use of scanners in supermarkets will result in fewer cashiers. Myriad social welfare programs, such as unemployment insurance and food stamps, help soften the transition. Finally, social welfare policies are a means for rectifying past and present injustices. For example, affirmative action policies were designed to remedy the historical discrimination that denied large numbers of Americans access to economic opportunities and power. Teacher incentive pay and other educational policies are designed to help ameliorate the unequal distribution of resources

between underfunded urban and better-funded suburban school systems.

Social Work and Social Policy

Social work practice is driven by social policies that dictate how the work is done, with whom, for how much, and toward what ends. For example, a social worker in a public mental health center may have a caseload in excess of 200 clients. The size of that caseload makes it unlikely that the worker will be able to engage in any kind of sustained therapeutic intervention beyond **case management**. Or consider the case worker who—in the midst of high unemployment—must find employment for recipient mothers about to lose benefits due to mandatory time limits. In these and other instances, economic and political factors structure the work of agencies and impede the ability of workers to succeed in their job.

An ideological preference among policymakers for private sector social services has resulted in less funding for public agencies. In response to diminishing revenues, public agencies adjust in predictable ways, such as cutting staff (or replacing them with lower paid and less qualified workers) and expecting existing staff to do more with less. In addition, they promote short-term (or drug-based) interventions to more cheaply process clients. Cuts are made by freezing or reducing the salaries and benefits of professional staff. In large part, the accomplishments of social workers depend on available agency resources.

Social workers in private practice that depend on managed care experience similar constraints. For instance, managed care plans dictate how much a social worker will be paid and how many times they will be permitted to see a client. Accordingly, these plans structure the kinds of interventions that can be realistically implemented in the allotted time. Governmental and agency policies structure the day-to-day work of social work.

Values, Ideology, and Social Welfare Policy

Social welfare policies are shaped by a set of social and personal values that reflect the preferences of those in decision-making capacities. According to David Gil, “choices in social welfare policy are

heavily influenced by the dominant beliefs, values, ideologies, customs, and traditions of the cultural and political elites recruited mainly from among the more powerful and privileged strata.”¹² How these values are played out in the realm of social welfare is the domain of the policy analyst. As Chapter 3 illustrates, social welfare policy is rarely based on a rational set of assumptions backed up by valid research.

The Pareto Optimality is a state whereby making one person or group better-off through the allocation of resources is impossible without making another person or group worse off. A Pareto Improvement occurs when a person or group is made better-off through the allocation of resources without making another person or group worse off. In the real world of social policy, the Pareto Optimality is typically the dominant mode.

Social policy is typically a zero-sum game whereby some people are advantaged at the expense of others. Or, at least they perceive themselves as being treated unfairly. For example, the upper 1 percent of Americans bring home nearly a quarter of the U.S. income every year and control 40 percent of the nation’s wealth.¹³ Despite their privilege, many see increased taxes and regulation as an unfair infringement and an attack on the most productive members of society.¹⁴ Although not directly affected, some groups see the recent U.S. Supreme Court ruling legalizing gay marriage as an assault on their religious freedom and principles.

Recent U.S. social welfare policy has been largely shaped by values around self-sufficiency, work, and the omniscience of the marketplace. As policymakers expected disadvantaged people to be more independent, support for government social programs was cut to presumably discourage dependency. Although these cuts saved money in the short run, most of them fell squarely on the shoulders of children. Eventually, cuts in social programs can lead to greater expenditures as the generation of children who have gone without essential services begins to require programs to remedy problems associated with poor maternal and infant health care, poverty, illiteracy, and family disorganization. In 2011, the International Monetary Fund (IMF) ranked the United States 32nd in public spending on family benefits, just above Lithuania, Latvia, Greece, Malta, Mexico, Chile, and Korea.¹⁵

Social values are organized through the lens of ideology. Simply put, an **ideology** is the framework

of commonly held beliefs through which we view the world. It is a set of assumptions about how the world works: what has value, what is worth living and dying for, what is good and true, and what is right. For the most part, these beliefs are rarely examined and are simply assumed to be true. Hence, the ideological tenets around which society is organized exist as a collective social consciousness that defines the world for its members. All societies reproduce themselves partly by reproducing their ideology; in this way, each generation accepts the basic ideological suppositions of the preceding one. When widely held ideological beliefs are questioned, society often reacts with strong sanctions. Ideological trends influence social welfare when adherents of one orientation hold sway in decision-making bodies.

The hold of ideology on social policy is especially strong in times of threat, such as the “War on Terror.” In this instance, social welfare policy fades into the background as the perceived need for national security takes center stage. The social history of the United States has seen periods where oppressed groups assert their rights in the face of mainstream norms. Sometimes social unrest is met with force, such as in the labor strikes of 1877, while at other times, such as the Great Depression, it is met with the expansion of social welfare programs.

The Political Economy of American Social Welfare

The term **political economy** refers to the interaction of political and economic theories in understanding society. The political economy of the United States has been labeled **democratic capitalism**—a representative form of government that coexists with a market economy. Social welfare policy plays an important role in stabilizing society by modifying the play of market forces and softening the social and economic inequalities generated by the marketplace.¹⁶ To that end, two sets of activities are necessary: state provision of social services (benefits of cash, in-kind benefits, and personal social services) and state regulation of private activities to alter (and sometimes improve) the lives of citizens. Social welfare bolsters social stability by helping to mitigate the problems associated with economic dislocation, thereby allowing society to remain in a state of more or less controlled balance.



John Maynard Keynes is best known as the economic architect of the modern welfare state.

Source: Pictorial Press Ltd/Alamy Stock Photo

Ideally, the political economy of the welfare state should be an integrated fabric of politics and economics; but in reality, some schools of thought contain more political than economic content, and vice versa. For example, most economic theories contain sufficient political implications to qualify them as both economic and political. Conversely, most political schools of thought contain significant economic content. It is therefore difficult to separate political from economic schools of thought. For the purposes of this chapter, we will organize the political economy of U.S. welfare into two separate categories: (1) predominantly economic schools of thought and (2) predominantly political schools of thought. Nevertheless, the reader will find a significant overlap among and between these categories.

The U.S. Economic Continuum

In large measure, economics forms the backbone of the political system. For example, the modern welfare state would not exist without the contribution of economist John Maynard Keynes. Conversely, the conservative movement would be

weaker without the contribution of classical or free market economists such as Adam Smith and Milton Friedman. Virtually every political movement is somehow grounded in economic thought. The three major schools that have traditionally dominated American thought are Keynesian economics; classical or free market economics (and its variants); and to a lesser degree, democratic socialism.

Keynesian Economics

Keynesian economics drives liberalism and most welfare state ideologies. John Maynard Keynes' economic theories formed the substructure and foundation of the modern welfare state, and virtually all welfare societies are built along his principles. Sometimes called demand or consumer-side economics, this model emerged from Keynes's 1936 book, *The General Theory of Employment, Interest and Money*.

An Englishman, Keynes took the classical model of economic analysis (self-regulating markets, perfect competition, the laws of supply and demand, etc.) and added the insight that macroeconomic stabilization by government is necessary to keep the economic clock ticking smoothly.¹⁷ He rejected the idea that a perfectly competitive economy tended automatically toward full employment and that the government should not interfere in the process. Keynes argued that instead of being self-correcting and readily able to pull themselves out of recessions, modern economies were recession prone and had difficulty providing full employment.

According to Keynes, periodic and volatile economic situations that cause high unemployment are primarily caused by the instability in investment expenditures. The government can stabilize and correct recessionary or inflationary trends by increasing or decreasing total spending on output. Governments can accomplish this by increasing or decreasing taxes (thereby increasing or decreasing consumption) and by the transfer of public goods or services. For Keynes, a "good" government is an activist government in economic matters, especially when the economy gets out of full employment mode. Keynesians believe that social welfare expenditures are investments in **human capital** that eventually increase the national wealth (e.g., by increasing productivity) and thereby boost everyone's net income.

Keynes's doctrine emerged from his attempt to understand the nature of recessions and depressions.

Specifically, he saw recessions and depressions as emerging from businesses' loss of confidence in investments (e.g., focusing on risk rather than gain), which in turn causes the hoarding of cash. This loss of confidence eventually leads to a shortage of money as everyone tries to hoard cash simultaneously. Keynes's answer to this problem was that government should make it possible for people to satisfy their economic needs without cutting their spending, which prevents the spiral of shrinking incomes and shrinking spending. Simply put, in a depression the government should print more money and get it into circulation.¹⁸

Keynes also understood that this monetary policy alone would not suffice if a recession spiraled out of control, as in the Great Depression of the 1930s. He pointed to a liquidity trap whereby people hoard cash because they expect deflation (a decrease—extreme in a depression—in the price of goods or services), insufficient consumer or industry demand, or some catastrophe such as war. In a depression, businesses and households fail to increase spending regardless of how much cash they have. To help an economy exit this trap, government must do what the private sector will not—namely, spend. This spending can take the form of public works projects (financed by borrowing) or direct governmental subsidization of demand (welfare entitlements). To be fair, Keynes saw public spending only as a last resort to be employed if monetary expansion failed. Moreover, he sought an economic balance: Print money and spend in a recession; stop printing and stop spending once it is over. Keynes understood that too much money in circulation, especially in times of high production and full employment, leads to inflation. Although relatively simple, Keynes's theories represent one of the great insights of twentieth-century economic thought.¹⁹ These ideas also formed the economic basis for the modern welfare state.

Conservative or Free Market Economics

Whereas liberalism is guided by Keynesian economics, the conservative view of social welfare is guided by free market economics.

Adam Smith is known as the father of modern capitalism, and conservative economics was arguably born in *The Wealth of Nations*. Smith believed in the “invisible hand” of the marketplace, or in other words, the view that the economic system was automatic, and when left undisturbed by

government or other forces, it would self-regulate, thereby ensuring maximum economic efficiency. This self-regulation, however, would be threatened by monopolies, preferential tax structures, or other treatment that favors one group over another. To ensure efficiency, markets had to be left alone. Smith believed the main measure of a nation's wealth was in the goods and services it produced and traded (the forerunner of gross domestic product), which would lead to further economic growth. Within Smith's economic paradigm, the proper role of government was defense, the creation and maintenance of public infrastructure, public safety and education. In turn, these activities would be financed by a fair system of taxation.²⁰

Although friends with John Maynard Keynes, Friedrich Hayek was his intellectual adversary. Representing the Austrian economic school, Hayek focused on the business cycle. He believed that markets were organic, and any interference with their spontaneous order would hamper their efficient operation. Hayek argued that the major problem for an economy is how people's actions are coordinated. He observed that free markets effectively and spontaneously (i.e., not part of anyone's plan) coordinated people's actions. Hayek believed that the market evolved as the result of human actions in the context of economic exchanges.²¹

Hayek was also a realist who understood that markets are not necessarily perfect. One problem he observed was based on the increase in the money supply by central banks. In particular, the increased money supply drives down interest rates thereby making credit artificially cheap. This leads to “malinvestments” (i.e., bad business investments) that would not occur without a distorted price signal from the market. For instance, driven by cheap credit, investors may build what turns out to be half-filled shopping malls or new commercial buildings in an already saturated market. The dot .com and housing bubbles are examples of malinvestments. Hayek saw recessions and depressions as part of a necessary readjustment. For him, the best way to avoid busts was to avoid the booms that cause them. In contrast to the economic activism of Keynesianism, Hayek's strategy for the Great Depression was to allow only minimal regulation of market functions since the market is too



Watch this video for an entertaining contrast of Keynes' and Hayek's viewpoints.

www.youtube.com/watch?v=dOnERTFo-Sk

complex to engage in any serious forecasting. Moreover, government interference not only worsens the situation, but leads to further economic chaos.²²

Free market economics is predicated on a belief in the existence of many small buyers and sellers who exchange homogeneous products with perfect information in a setting in which each can freely enter and exit the marketplace at will.²³ As an ideal type, none of these assumptions hold in the real world of economics. For instance, the free market model does not address the dominance of distribution networks by a single retailer like Walmart. There is nothing in the free market model that addresses the lack of equitable distribution of knowledge, experience, opportunity, and access to resources enjoyed by buyers and sellers. The free market model ignores theft, fraud, and deception in cases like Enron, and it ignores the competitive advantages that accrue through lobbying and special interest negotiations like Halliburton's no competition bids for Iraq reconstruction projects. It also ignores the power of large retailers to control the market by instituting late shopping hours or 24/7 businesses that make it impossible for small family-owned businesses to compete. In short, an unregulated market economy becomes monopolistic as more of the market is taken over by fewer enterprises.

The ascendance of the conservative economic (and social) argument accelerated after 1973, when the rise in living standards began to slow for most Americans. Conservatives blamed this economic slowdown on governmental policies—specifically, deficit spending, high taxes, and excessive regulations.²⁴ In a clever sleight of hand, government went from having the responsibility to address economic problems (à la Keynes) to being the cause of them.

Milton Friedman, considered by some to be the father of modern conservative economics, was one of Keynes's more ardent critics. In opposition to Keynes, Friedman argued that using fiscal and monetary policy to smooth out the business cycle is harmful to the economy and worsens economic instability.²⁵ He contended that the Depression did not occur because people were hoarding money; rather, there was a fall in the quantity of money in circulation. Friedman argued that Keynesian economic policies must be replaced by simple monetary rules (hence the term *monetarism*). In effect, he believed that the role of government was to keep

the money supply growing steadily at a rate consistent with stable prices and long-term economic growth.²⁶

Friedman counseled against active efforts to stabilize the economy. Instead of pumping money into the economy, government should simply make sure enough cash is in circulation. He called for a relatively inactive government in economic affairs that did not try to manage or intervene in the business cycle. For Friedman, welfare spending existed only for altruistic rather than economic reasons.²⁷ To the right of Milton Friedman was Robert Lucas, 1994 Nobel Prize winner and developer of the "theory of rational expectations." Lucas argued that Friedman's monetary policy was still too interventionist and would invariably do more harm than good.²⁸

Developing outside of conventional economics, **supply-side economics** enjoyed considerable popularity during the early 1980s. Led by Robert Barth, editorial page head of the *Wall Street Journal*, supply-siders were journalists, policymakers, and maverick economists who argued that demand-side policies and monetary policies were ineffective.²⁹ They maintained that the incentive effects of reduced taxation would be so large that tax cuts would dramatically increase economic activity to the point where tax revenues would rise rather than fall. (Former President George H. W. Bush referred to this as *voodoo economics* in 1980.³⁰) Specifically, supply-siders argued that tax cuts would lead to a large increase in labor supply and investment and therefore to a large expansion in economic output. The budget deficit would evaporate because taxes, increased savings, and higher economic output would offset the deficit. In the early 1980s, supply-siders seized power from the Keynesians and mainstream conservative economists, many of whom believed in the same things but wanted to move more slowly.³¹

Although some supporters preferred to think of supply-side economics as pure economics, the theory contained enough political implications to qualify as a political as well as an economic theory. Popularized by supporters such as Jack Kemp, Arthur Laffer, and Ronald Reagan, supply-side economics provided the rationale for the dramatic cuts in social programs executed under the Reagan administration.

Despite their popularity in the early years of the Reagan administration, the term *supply-side economics* fell out of favor when it became evident that massive tax cuts for the wealthy and corporations

did not result in increased productivity. Instead, the wealthy spent their tax savings on luxury items, and corporations used tax savings to purchase other companies in a merger mania that took Wall Street by surprise. Some corporations took advantage of temporary tax savings to transfer their operations abroad, further reducing the supply of high-paying industrial jobs in the United States. For these and other reasons, the budget deficit grew from about \$50 billion a year in the Carter term to \$352 billion a year in 1992.³²

Although the term *supply-side economics* fell out of favor by the late 1980s, its basic tenets, such as the belief that massive tax cuts for the rich would increase productivity (and the necessity of social welfare spending cuts), were adopted enthusiastically by the G.W. Bush administration in the form of the Economic Growth and Tax Relief Reconciliation Act of 2001 (EGTRRA) and the Jobs and Growth Tax Relief Reconciliation Act of 2003 (JGTRRA). Citizens for Tax Justice estimated that more than \$1 trillion has been lost to the U.S. Treasury as a result of the Bush tax cuts (later continued by the Obama administration).³³ The result of these policies mirrored the effects of the earlier supply-side doctrine: huge federal and state budget shortfalls, corporate hoarding, greater economic inequality, and stagnant wages.³⁴ The federal budget deficit rose to \$1.4 trillion in 2009. It stayed above the \$1 trillion mark until 2012, after which it fell to \$583 billion in 2015.³⁵

Conservative economists argue that large social welfare programs—including unemployment benefits and public service jobs—are detrimental to the society in two ways. First, government social programs erode the work ethic by supporting those not in the labor force. Second, because they are funded by taxes, public sector social welfare programs divert money that could otherwise be invested in the private sector. Conservative economists believe that economic growth helps everyone because overall prosperity creates more jobs, income, and goods, and these eventually filter down to the poor. For conservative economists, investment is the key to prosperity and the engine that drives the economic machine. Accordingly, many conservative economists favor tax breaks for the wealthy based on the premise that such breaks will result in more disposable after-tax income freed up for investment. In turn, high taxes are an impediment to economic progress because they channel money into “public” investments and away from “private” investments.

For conservatives, opportunity is based on one’s relationship to the marketplace and legitimate rewards can only occur only through that participation. In contrast to liberals who emphasize mutual self-interest, interdependence, and social equity, conservative economists argue that the highest form of social good is realized by the maximization of self-interest. In the conservative view (as epitomized by author Ayn Rand³⁶), the best society is one in which everyone actively—and selfishly—pursues their own good. Through a leap of faith, the maximization of self-interest is somehow transformed into a mutual good.

Conservative economists maintain not only that high taxation and government regulation of business serve as disincentives to investment but also that individual claims on social insurance and public welfare grants discourage work. Together these factors lead to a decline in economic growth and an increase in the expectations of beneficiaries of welfare programs. The only way to correct the irrationality of governmental social programs is to eliminate them. Charles Murray has suggested that the entire federal assistance and income support structure for working-aged persons (Medicaid, the former Aid to Families with Dependent Children [AFDC], food stamps, etc.) be scrapped. This would leave working-aged persons no recourse except to actively engage in the job market or turn to family, friends, or privately funded services.³⁷

Some conservative economists argue that economic insecurity is an important part of the entrepreneurial spirit. Unless people are *compelled* to work, they will choose leisure over work. Conversely, providing economic security for large numbers of people through welfare programs leads to diminished ambition and fosters an unhealthy dependence on the state. Social programs thereby harm rather than help the most vulnerable members of society. The belief in economic insecurity formed the basis for the 1996 welfare reform bill that included a maximum time limit on welfare benefits (see Chapter 11).

The **public choice school** gained traction among conservatives as faith ebbed in supply-side theories. This theory was not widely known outside academic circles until James Buchanan was awarded the Nobel Prize for economics in 1986. In effect, the public choice is predicted on the belief that public sector bureaucrats are self-interested utility-maximizers and that strong incentives exist for interest groups to make demands on

government. The resulting concessions flow directly to the interest group as their costs are spread among all taxpayers. Initial concessions lead to demands for further concessions, which are likely to be forthcoming so long as interest groups are vociferous in their demands. Under such an incentive system, different interests are also encouraged to band together to make demands since there is no reason for one interest group to oppose the demands of others. As demands for goods and services increase, revenues tend to decrease. This happens because interest groups resist paying taxes directed specifically toward them and because no interest group has much incentive to support general taxes. The result of this scenario is predictable: Strong demands for government benefits accompanied by declining revenues lead to government borrowing, which in turn, results in large budget deficits.³⁸ Adherents of public choice theory view social welfare as a series of endless concessions to disadvantaged groups that will eventually bankrupt the government. On the other hand, it would be logical also to apply public choice theory to defense industry interest groups who make similar demands on government while not paying a fair share of taxes.

The Global Financial Crisis (GFC)

Alan Greenspan, the former head of the Federal Reserve, admitted that he “made a mistake” in trusting free markets to regulate themselves without governmental oversight. Greenspan further admitted that “I made a mistake in presuming that the self-interests of organizations, specifically banks and others, were . . . capable of protecting their own shareholders and their equity in the firms.”³⁹ This was an amazing series of admissions from the man known as the “oracle” in economic matters. More importantly, he questioned the belief that unregulated free markets inevitably yield superior economic gain.

The initial event triggering the 2008 GFC was the collapse of the U.S. housing market and the realization that domestic and foreign banks, investment houses, and institutions were holding hundreds of billions of dollars of subprime mortgages (i.e., nonviable mortgages held by problematic borrowers) that were little more than toxic debt offering little hope for repayment. However, multiple factors converged to create the crisis, including the largely unregulated derivatives market and the

reliance on various forms of dodgy financial instruments. Derivatives are used by banks and corporations to hedge risk or engage in speculation. They are financial instruments whose value depends on an underlying commodity, bond, equity, or currency. Investors purchase derivatives to bet on the future (or as a hedge against the potential adverse impacts of an investment), to mitigate a risk associated with an underlying security, to protect against interest rate or stock market changes, and so forth.

Derivatives are used in several financial areas. For example, credit derivatives can involve a contract between two parties that allows one of them to transfer their credit risk to the other. The party transferring the risk pays a fee to the party that assumes it. These derivatives are risky investments because they are basically bets made in large amounts, often in the billions. Like all forms of gambling, derivatives only work if the casino has the money to meet their obligation to bettors. If the casino lacks the cash to pay winners (i.e., it has a liquidity problem), the entire system collapses. The 2008 GFC was partly based on the failure of the derivatives market.

Various government bailouts—including \$25 billion to the U.S. auto industry—helped ease the financial crisis. Much of the money to pay for the bailouts came from foreign investors who purchased U.S. Treasury bills.⁴⁰ One unexpected outcome of the GFC and the collapse of Wall Street was that it temporarily chilled the debate on privatizing Social Security (see Chapter 10).

Democratic Socialism

Democratic socialism (as opposed to Soviet-style communism) is based on the belief that radical economic change is necessary and can be achieved within a democratic context. They question the fundamental precepts of capitalism and its ability to meet the needs of people. This view is at odds with both Keynesianism and conservative economics. Specifically, Keynesians basically believe in the market economy but want to make it more responsive to human needs by smoothing out the rough edges. Conservatives maintain that the economy should be left alone except for a few minor tweaks, such as regulating the money supply. Others argue that the market should be left totally alone. On balance, both Keynesians and economic conservatives believe that capitalism is compatible with the public good. Keynesians and economic conservatives have more in common with each other than Keynesians have with socialists.

Proponents of **socialism** argue that the fundamental nature of capitalism is anathema to advancing the public good. They contend that a system predicated on pursuing profit and individual self-interest can only lead to greater inequality. The creation of a just society requires a fundamental transformation of the economic system, and the pursuit of profit and self-interest must be replaced by the collective pursuit of the common good. Not surprisingly, they repudiate Keynes's belief that economic problems can be fixed by technicalities instead of sweeping institutional change. In short, socialists criticize conservatives for the primary importance they place on markets and their belief in subordinating social welfare initiatives to market needs.

Left-wing theorists maintain that the failure of capitalism has led to political movements that have pressured institutions to respond with increased social welfare services. They believe that real social welfare must be structural and can only be accomplished by redistributing resources. In a just society where goods, resources, and opportunities are available to everyone, only the most basic forms of social welfare (health care, rehabilitation, counseling, etc.) would be necessary. In this worldview, poverty is directly linked to structural inequality: People need welfare because they are exploited and denied access to resources. In an unjust society, welfare functions as a substitute, albeit a puny one, for social justice.⁴¹

Some socialists argue that social welfare is an ingenious arrangement to have the public assume the costs associated with the social and economic dislocations inherent in capitalism. According to these theorists, social welfare expenditures "socialize" the costs of capitalist production by making public the costs of private enterprise. Thus, social welfare serves both the needs of people and the needs of capitalism. For other socialists, social welfare programs support an unjust economic system that continues to generate problems requiring yet more programs. These radicals argue that social welfare programs function like junk food for the impoverished: They provide just enough sustenance to discourage revolution but not enough to make a real difference in anyone's life. Social welfare is simply viewed as a form of social control. Frances Fox Piven and the late Richard Cloward summarize the argument:

Relief arrangements are ancillary to economic arrangements. Their chief function is to

regulate labor, and they do that in two general ways. First, when mass unemployment leads to outbreaks of turmoil, relief programs are ordinarily initiated or expanded to absorb or control enough of the unemployed to restore order; then, as turbulence subsides, the relief system contracts, expelling those who are needed to populate the labor markets.⁴²

For radicals, real social welfare can occur only in a socialist economic system.

The U.S. Political Continuum

Differing views on political economy produce differing conceptions of the public good. Competition among ideas about the public good and the welfare state has long been a knotty issue in the political economy of the United States. Since governmental policy is driven largely by an ideologically determined view of the public good, it will vary depending which political party is in power.

The major American ideologies, (neo)liberalism and (neo)conservatism, hold vastly different views of social welfare and the public good. Since conservatives believe that the public good is best served through marketplace participation, they prefer private sector approaches over governmental welfare programs. Conservatives believe that government should have a minimal role (via a safety net) in ensuring the social welfare of citizens. Traditional liberals, on the other hand, view government as the primary institution capable of bringing a measure of social justice to millions of Americans who cannot fully participate because of obstacles such as racism, poverty, and sexism. Traditional liberals view government social welfare programs as a key component in promoting the public good. One of the major differences between these orientations lies in their differing perceptions of how the public good is enhanced or hurt by welfare state programs.

The understanding of "the public good" is lodged in the political and ideological continuum that makes up the U.S. political economy. An appreciation of this requires an understanding of the interaction of schools of political thought and how they evolved. These ideological tenets also shape the platforms of the major political parties and can be divided into two categories: (1) liberalism and left-of-center movements and (2) traditional conservatives and the far right.

Liberalism and Left-of-Center Movements

Liberalism Since Franklin Delano Roosevelt's New Deal, liberal advocates have argued for advancing the public good by promoting an expanding economy coupled with the growth of universal, non-means-tested social welfare and health care programs. Traditional liberals used Keynesianism as the economic justification for expanding the welfare state, and as such, the general direction of policy from the 1930s to the early 1970s was for the federal government to assume greater amounts of responsibility for the public good.

American liberals established the welfare state with the passage of the Social Security Act of 1935. Harry Hopkins—a social worker, the head of the Federal Emergency Relief Administration, a confidant of President Roosevelt, a co-architect of the New Deal, and a consummate political operative—developed the calculus for American liberalism: “tax, tax; spend, spend; elect, elect.”⁴³ This approach was elegant in its simplicity: The government taxes the wealthy, thereby securing the necessary revenues to fund social programs for workers and the poor. This approach dominated social policy for almost 50 years. In fact, it was so successful that by 1980 the social welfare accounted for 57 percent of all federal expenditures.⁴⁴

By the mid-1960s, the welfare state had become a central fixture in America, and politicians sought to expand its benefits to more constituents. Focusing on the expansion of middle-class programs such as Federal Housing Administration (FHA) home mortgages, federally insured student loans, Medicare, and veterans' pensions, liberal policymakers secured the political loyalty of the middle class. Even conservative politicians respected voter support for the welfare state, and not surprisingly, the largest growth in social welfare spending occurred under Republican president, Richard M. Nixon.

Despite such support, the promise of the U.S. welfare state to provide social protection similar to Western Europe never materialized. By the mid-1970s, the hope of traditional liberals to build a welfare state mirroring those of northern Europe had been replaced by an incremental approach that focused narrowly on consolidating and fine-tuning the programs of the Social Security Act. One reason for this failure was the ambivalence of many Americans toward centralized government. “The emphasis consistently has been on the local, the

pluralistic, the voluntary, and the business-like over the national, the universal, the legally entitled, and the governmental,” observed policy analyst Marc Bendick.⁴⁵

Liberalism lost ground for another reason. The Social Security Act of 1935—the hallmark of American liberalism—was primarily a self-financing social insurance program that rewarded working people. Public assistance programs that contained less political capital were therefore a better measure of public compassion, were rigorously means-tested, sparse in their benefits, and operated by the less than generous states. For example, although Social Security benefits were indexed to the cost of living in the mid-1970s, AFDC benefits deteriorated so badly that about half its value was lost between 1975 and 1992. At the same time that Social Security reforms reduced the elderly poverty rate by 50 percent, the plight of poor non-working families worsened.

Neoliberalism By the late 1970s, the liberal belief that the welfare state was the best mechanism to advance the public good was in retreat. What remained of traditional liberalism was replaced by a neoliberalism that was more cautious of government, less antagonistic toward big business, and more skeptical about the value of universal entitlements.

The defeat of Jimmy Carter and the election of a Republican Senate in 1980 forced many liberal Democrats to reevaluate their party's traditional position on domestic policy. This reexamination, which Charles Peters christened “neoliberalism” to differentiate it from old-style liberalism, attracted only a small following in the early 1980s.⁴⁶ By the mid-1990s, however, most leading Democrats could be classified as neoliberal. Randall Rothenberg charted signs of the influence of neoliberalism on the Democratic domestic policy platform as early as 1982, when he observed that the party's midterm convention did not endorse a large-scale federal jobs program, did not endorse a national health insurance plan, and did not submit a plan for a guaranteed annual income.⁴⁷

In the late 1980s, a cadre of prominent mainstream Democrats established the Democratic Leadership Council (DLC). In part, their goal was to wrest control of the Democratic Party from traditional liberals and to create a new Democratic Party that was more attuned to the beliefs of traditional core voters. In 1989, the DLC released *The New Orleans Declaration: A Democratic Agenda for the*

1990s, which promised that Democratic Party politics would shift toward a middle ground combining a corporatist economic analysis with Democratic compassion. Two of the founders of the DLC were Al Gore and Bill Clinton, who chaired the DLC just before announcing his candidacy.⁴⁸

Compared to traditional liberals, neoliberals were more forgiving of the behavior of large corporations and were opposed to economic protectionism. They were also opposed to strong financial regulation, which helps explain why the repeal of the 1933 Glass-Steagall Act (the act curbed speculation in commercial banking) was passed under the neoliberal Clinton administration. Some commentators partly attribute the 2007 global financial crisis to the repeal of Glass-Steagall.⁴⁹ (Clinton later apologized for supporting the repeal of Glass-Steagall.)

Grounded in *realpolitik*, neoliberals viewed the New Deal approach as too expensive and antiquated to address the mood of voters. They distanced themselves from the large-scale governmental welfare programs associated with Democrats since the New Deal. Like their neoconservative counterparts, they called for reliance on personal responsibility, work, and thrift as an alternative to governmental programs. Accordingly, their welfare proposals mirrored conservative proposals by emphasizing labor market participation (workfare), personal responsibility (time-limited welfare benefits), family obligations (child support enforcement), and frugality in governmental spending.

Former Secretary of Labor Robert Reich advocated a postliberal formulation that replaced social welfare entitlements with investments in human capital. Public spending was divided into “good” and “bad” categories. Bad was unproductive expenditures on welfare and price supports; good was investments in human capital, such as education, research, and job training.⁵⁰

Neoliberalism altered the traditional liberal concept of the public good. Instead of viewing the interests of large corporations as antithetical to the best interests of society, they argued for free trade, less regulation, and a laissez-faire approach to social problems. They also viewed labor unions with caution. The new shapers of the public good had systematically excluded key actors of the old liberal coalition.

The neoliberal view of the public good reflects a kind of postmodern perspective. For neoliberals, the public good is elusive, and its form is fluid.



Obama Presidential Campaign 2008

Source: John Beam/Alamy Stock Photo

Definitions of the public good change as new power relationships emerge. Hence, neoliberals view the public good in the context of a postindustrial society composed of new opportunities and new institutional forms.

Neoliberalism is largely a political strategy and pragmatic mode of operation rather than a political philosophy with a firm position on the public good. That is both its strength and weakness. The strength of neoliberalism is its ability to compromise and change to accomplish goals. Its weakness is that when faced with an ideological critique, it is incapable of formulating a cogent ideological response. President Obama fits squarely within the pragmatic neoliberal orientation, which partly explains his refusal to enact strong banking regulations in the aftermath of the global financial crisis and his support for the Trans-Pacific Partnership trade deal.

The Self-Reliance School A perspective gaining influence in economically distressed areas and in developing countries is the **self-reliance school**.⁵¹ This school maintains that industrial economic models are irrelevant to the economic needs of poor communities and are often damaging to the spiritual life of people.⁵² Adherents of self-reliance reject the emphasis of Western economies on economic growth and the belief that the quality of life can be measured by material acquisitions. These economists stress a balanced economy based on the real needs of people, production designed for internal consumption rather than export, productive technologies that are congruent with the culture and background of the population, the use of appropriate

and manageable technologies, and a small-scale and decentralized form of economic organization.⁵³ Simply put, proponents of self-reliance postulate that more is less, and less is more. The objective of self-reliance is the creation of a no-poverty society where economic life is organized around issues of subsistence rather than trade and economic expansion. Accepting a world of finite resources and inherent limitations to economic growth, proponents argue that the true question of social and economic development is not what people think they want or need but what they require for survival. The self-reliance school accepts the need for social welfare programs that ameliorate the dislocations caused by industrialization, but it prefers low-technology and local solutions to social problems. This contradicts the conventional wisdom of the welfare state, which is predicated on a prescribed set of programs on a national scale, administered by large bureaucracies and sophisticated management systems.

Classical Conservatives and the Far Right

Classic Conservatism Former politicians, such as Nelson Rockefeller, Richard Nixon, and Barry Goldwater represented traditional conservatism, a perspective that is now all but extinct. Replaced by cultural conservatives, few traditional conservatives currently occupy important leadership positions in the Republican Party.

On one level, most conservatives agree on important social policy issues. They are anti-union, oppose governmental regulations, demand lower taxes and less social spending, want local control of public education, oppose extending civil rights legislation, are pro-gun and opposed to gun control, and strongly believe in states' rights. Beneath this agreement, fundamental differences exist among various conservative factions.

Traditional conservatives were at odds with cultural conservatives on a range of social issues. First, traditional conservatives were strict constitutionalists and believed strongly in the separation of church and state. They viewed prayer and religion as personal choices in which government has no constitutional right to promote. Second, while traditional and cultural conservatives want a weaker federal government, cultural conservatives demand that government promote a religious-based agenda in areas such as abortion, contraception, and gay

marriage. U.S. presidential candidate Mike Huckabee said that as president he would use federal troops to stop abortions. Presidential candidate Rick Santorum decries the separation of church and state. "I don't believe in an America where the separation of church and state is absolute." In fact, he stated that it makes him want to throw up.⁵⁴

Third, classical conservatives were more socially liberal than their cultural counterparts. For example, the late Barry Goldwater, a conservative icon and former U.S. Senator, stated that, "I have been, and am still, a traditional conservative, focusing on three general freedoms—economic, social, and political. . . . The conservative movement is founded on the simple tenet that people have the right to live life as they please, as long as they don't hurt anyone else in the process."⁵⁵ Following that line of reason, Goldwater's outspoken support of gays in the military was directly opposed to the tenets of cultural conservatives. Regarding reproductive freedom, classical conservatives might challenge cultural conservatives on various measures that limit or ban abortions.

From the late 1970s onward, old-style conservatives such as Nelson Rockefeller, Barry Goldwater, and William Cohen—who were more concerned with foreign policy than with domestic issues—were replaced by a new breed of cultural conservatives. These cultural conservatives were committed to reversing 50 years of liberal influence in social policy. How the cultural conservatives came to shape social policy warrants elaboration, although it is first important to examine neoconservatives, the forerunners of cultural conservatism.

Neoconservatism Before the 1970s, conservatives were content to merely snipe at welfare programs, reserving their attention for areas more consistent with their traditional concerns such as the economy, defense spending, and foreign affairs. However, by the mid-1970s, conservative intellectuals recognized that their former stance toward social welfare was myopic as welfare was too important to be lightly dismissed. Consequently, **neoconservatives** sought to arrest the growth in governmental welfare programs while simultaneously transferring as much welfare responsibility as possible from government to the private sector.⁵⁶ They faulted government programs for a breakdown in the mutual obligation between groups; the lack of attention to how programs were operated and benefits awarded; the dependency of recipients; and the growth of the

welfare industry and its special interest groups, particularly professional associations.⁵⁷ To counter the liberal goals of full employment, national health care, and a guaranteed annual income, neoconservatives maintained that high unemployment was good for the economy, that health care should remain in the private marketplace, and that competitive income structures were critical to productivity. They argued that income inequality was socially desirable because social policies that promote equality encourage coercion, limit individual freedom, and damage the economy.⁵⁸ By the late 1970s, the neoconservative position began to be usurped by the emerging cultural conservatives.

Cultural and Social Conservatism The neoconservative assault on liberal social policy was soon taken over by a coalition of cultural and social conservatives who raged against governmental intrusion in the marketplace while simultaneously attempting to use the authority of government to advance their objectives in the areas of sexual abstinence, school prayer, abortion, birth control, evolution (i.e., creationism), gun rights, and antigay rights proposals. These conservatives cleverly promoted a dual attitude toward the role of government. Mimicking traditional conservatives, they demanded a *laissez-faire* approach to economics but steadfastly refused to apply that orientation to social affairs. Instead, they argued for social conformity and a level of governmental intrusion into private affairs that most traditional conservatives would have found appalling. In contrast to the traditional conservative position on the separation of church and state, social conservatives opportunistically embraced the rising tide of fundamentalist religion, even to the point of rewriting history by arguing that the Founding Fathers were opposed to a secular state and were guided by Christian principles.⁵⁹

By the late 1980s, this coalition of economic conservatives, right-wing Christians, and opportunistic politicians had virtually decimated what remained of Republican liberalism, whose adherents had become an endangered species like liberal Democrats.

For liberals, the state represents the best vehicle for promoting the public good. Cultural conservatives view the state as the cause rather than the solution to social problems. With the exception of protecting people (police and defense) and property, cultural conservatives argue that the very existence of the state is antithetical to the public good

since government interferes with the maximization of individual self-interest. Their posture toward government is adversarial, except when the state is used to further their social agenda. In tandem with this agenda, conservative presidents, such as Reagan and the two Bushes, prohibited the future growth of the welfare state by using tax policy and federal budget deficits to thwart increased public spending.

After hammering away at social programs, conservatives had accomplished relatively little regarding social insurance and health programs—costs for entitlement programs such as Social Security and Medicare continued to soar. Conservatives underestimated three key factors: (1) the resiliency of the welfare state, (2) the continued support (however tenuous) of the middle class, and (3) the difficulty of translating rhetoric into viable reform proposals. Nevertheless, conservatives learned from past mistakes, and instead of toying with incremental policies, they proposed bold new social initiatives that were incorporated into the Contract with America (designed to alter most of the safety net programs within a two-year period), a document signed by more than 300 House Republicans in 1994.⁶⁰ The crowning victory occurred with the passage of the Personal Responsibility and Work Opportunity Reconciliation Act (PRWORA) in 1996.

Social and cultural conservatism flourished in the 2014 Congressional midterm elections and in the 2016 presidential run. By 2015, the conservative coalition pushed most Republican presidential candidates into adopting hard-right positions on gay rights, abortion, health care, and even long-resolved issues like contraception. Heated rhetoric followed on the pressure to solidify a conservative “street cred.” In 2015, 2016 presidential contender Donald Trump claimed that Mexican immigrants were criminals and rapists; Senator Tom Cotton (R-Ark) likened Obama Secretary of State John Kerry to Pontius Pilate; Senator Ted Cruz’s (R-TX) oft-repeated line was that the president was “the leading state sponsor of terrorism”; and former New York City Mayor Rudy Giuliani’s comment about Obama “I do not believe . . . that the president loves America . . . He doesn’t love you. And he doesn’t love me. He wasn’t brought up the way you were brought up and I was brought up through love of this country.”⁶¹

Libertarianism **Libertarians** reflect another perspective. Specifically, this school of thought believes

in little or no government regulation. Libertarians basically want the government to stay out of people's pocketbooks and their bedrooms.

We, the members of the Libertarian Party, challenge the cult of the omnipotent state and defend the rights of the individual. We hold that all individuals . . . have the right to live in whatever manner they choose, so long as they do not forcibly interfere with the equal right of others to live in whatever manner they choose. We . . . hold that governments . . . must not violate the rights of any individual: namely, (1) the right to life—accordingly we support the prohibition of the initiation of physical force against others; (2) the right to liberty of speech and action—accordingly we oppose all attempts . . . [at] . . . government censorship in any form; and (3) . . . we oppose all government interference with private property. . . .⁶²

Libertarians argue that governmental growth occurs at the expense of individual freedom. They also believe that the proper role for a government is to provide a police force and a military that possesses only defensive weapons. Libertarians are highly critical of taxation because it fuels governmental growth. Apart from advocating minimal taxation earmarked for defense and police activities, they oppose the income tax. Because libertarians emphasize individual freedom and personal responsibility, they advocate the decriminalization of narcotics and believe that government should intercede in social affairs only when an individual's behavior threatens the safety of another. In 2015, Senator Rand Paul (R-KY) was a Republican contender for president running under a libertarian orientation. True libertarians had a problem with Paul who was against abortion rights, gay marriage, open borders, and for strong defense spending.

Cultural conservatives, populists, and libertarians of various ilk banded together in 2009 to form the loosely-knit Tea Party. This political movement advocated a rigid interpretation of the U.S. Constitution, especially on issues like gun control. The movement also focused on reducing government spending, eliminating the national debt, cutting social programs, and dramatically reducing taxes. Although not initially a religiously inspired movement, it soon allied itself with social and religious conservatives such as former Alaska governor Sarah Palin and former U.S. Congressperson Michele Bachman.

According to a 2010 Pew study, in addition to strong economic view, a 2010 Pew Research Center study found that Tea Party supporters also have conservative opinions on abortion and same-sex marriage. With considerable representation by white evangelical Protestants, they are more likely than average voters to say that their religion is the most important variable in shaping their opinion on social issues.⁶³

The Welfare Philosophers and the Neoconservative Think Tanks

Many early welfare thinkers envisioned a U.S. welfare state based on a European model.⁶⁴ This vision was shared by virtually every social welfare scholar writing in the late 1960s and early 1970s.⁶⁵ In turn, most social workers supported a liberal welfare philosophy grounded in a system of national social programs that would be deployed as more citizens demanded greater services and benefits. This framework was informed by European welfare states, especially the Scandinavian variant that spread health care, housing, income benefits, and employment opportunities equitably across the population.⁶⁶ It also led Richard Titmuss to hope that the welfare state, as an instrument of government, would eventually lead to a “welfare world.”⁶⁷

Despite the widespread acceptance of this liberal vision, an alternative vision arose that questioned the fundamental nature of welfare and social services. Throughout the 1970s and 1980s, conservatives (especially right-wing **think tanks**, or conservative policy institutes) busily made proposals for welfare reform. In fact, no conservative policy institute could prove its mettle until it produced a plan to clean up “the welfare mess.” The Hoover Institution at Stanford University helped shape the early conservative position on welfare. “There is no inherent reason that Americans should look to government for those goods and services that can be individually acquired,” argued Hoover’s Alvin Rabushka.⁶⁸ Martin Anderson, a Hoover senior fellow and domestic policy adviser to the Reagan administration, elaborated the conservative position on welfare in terms of the need to (1) reaffirm the need-only philosophical approach to welfare and state it as explicit national policy; (2) increase efforts to eliminate fraud; (3) establish and enforce a fair, clear work requirement; (4) remove inappropriate beneficiaries from the welfare rolls;

(5) enforce support of dependents by those who have the responsibility and are shirking it; (6) improve the efficiency and effectiveness of welfare administration; and (7) shift more responsibility from the federal government to state and local governments and private institutions.⁶⁹ These recommendations formed the backbone of the 1996 PRWORA.

In turn, the Heritage Foundation featured *Out of the Poverty Trap: A Conservative Strategy for Welfare Reform* by Stuart Butler and Anna Kondratas.⁷⁰ Following along the same lines, the Free Congress Research and Education Foundation proposed reforming welfare through “cultural conservatism”—that is, by reinforcing “traditional values such as delayed gratification, work and saving, commitment to family and to the next generation, education and training, self-improvement, and rejection of crime, drugs, and casual sex.”⁷¹

A handful of other works also served as beachheads for the conservative assault on the liberal welfare state. George Gilder’s *Wealth and Poverty* argued that beneficent welfare programs represented a “moral hazard” that insulated people against risks essential to capitalism and thus contributed to dependency.⁷² Martin Anderson concluded that income calculations should include the cash equivalent of in-kind benefits, such as food stamps, Medicaid, and housing vouchers, thus effectively lowering the poverty rate by 40 percent.⁷³ Taken together, these ideas and recommendations provided a potent critique of welfare programs.

Perhaps the most enduring change engineered by the conservative movement is what Jacob Hacker calls the “Great Risk Shift.”⁷⁴ Private ownership of property and the acceptance of personal responsibility have long been core American values, which partly explains why opposition to former President Bush’s “ownership society” had not materialized. In *The Great Risk Shift*, Hacker examines Bush’s ownership society and the Republican Party’s emphasis on personal responsibility as the code for shifting economic risk away from government and corporations and onto the back of the American family.

Hacker argues that private and public support mechanisms have fallen behind the pace of change in contemporary society. Almost half of marriages end in divorce. Over a third of employed Americans are frequently worried about losing their jobs. Structural changes in the nature of employment, primarily seen in a shift away from manufacturing

to the lower-paying service sector, have left many without the skills needed for new jobs or the resources to retrain. The likelihood of family income dropping 50 percent has almost tripled since the 1970s; personal bankruptcies and home foreclosures have increased by a factor of five.⁷⁵ Hacker maintains that during a 30-year period in which middle-class incomes have remained stagnant, the need for economic security has been neglected by public and private institutions.⁷⁶

The risk shift is occurring in almost all sectors. Corporate retirement programs are transitioning from defined benefit plans (i.e., retirees are guaranteed a set retirement income) to defined contribution plans whereby retirement income depends upon the savvy of the employees’ investment managers. Whether these changes will help or hurt the individual depends on many factors, but it is clear that it is a shift in risk to the individual worker.

Despite Obamacare, the absence of real universal health care has underscored the importance of employer-provided health insurance. However, the increasing instability of employment often means that job transitions are accompanied by the failure to acquire health coverage. Conservatives have proposed Health Savings Accounts as a means of activating market forces to control health costs, but they reflect another risk shift from the corporation to the individual worker. The former Bush administration suggested the elimination of employer-provided health insurance in favor of tax deductions for health insurance premiums, yet another risk shift from corporations to the individual or family.⁷⁷ An important implication of Hacker’s argument is that good social welfare policy analysis can no longer be restricted to a focus on income; it must also attend to the shifting dynamics of risk. As such, progressive social welfare policies must work to mitigate the degree of risk the individual family must bear.

Conclusion

John Judis and Michael Lind argue that, “Ultimately American economic policy must meet a single test: Does it tend to raise or depress the incomes of most Americans? A policy that impoverishes the ordinary American is a failure, regardless of its alleged benefits for U.S. corporations or for humanity as a whole.”⁷⁸ We would add: “What are the effects of an economic policy on the social health of the nation?” Researchers at Fordham

University's Institute for Innovation in Social Policy have argued that the nation's quality of life has become unhinged from its economic growth. "We really have to begin to reassess this notion that the gross domestic product—the overall growth of the society—necessarily is going to produce improvements in the quality of life."⁷⁹ Constructing an Index for Social Health that encompassed governmental data from 1970 to 1993, researchers found that in six categories—children in poverty, child abuse, health insurance coverage, average weekly earnings adjusted for inflation, out-of-pocket health costs for senior citizens, and the gap between rich and poor—"social health" hit its lowest point in 1993. As current poverty data suggest, these indicators have worsened since 1993.

A corollary question is, "What's the economy for, anyway?" In other words, do we exist to serve the economy or should the economy serve us? Economists often discuss the gross national product (GNP) or gross domestic product (GDP), productivity, and overall economic growth as if they were true indications of an economic quality of life. This approach avoids examining the beneficiaries of economic growth in the midst of growing income inequality. While discussions typically revolve around how to best grow the economy, too little of the economic discourse involves environmental sustainability or quality of life issues. John de Graaf has addressed these issues in *Affluenza* (the film and the book) as have other authors in various forms (see Spotlight Box 1.1).

Spotlight 1.1

WHAT'S THE ECONOMY FOR, ANYWAY?

by John de Graaf

In the global economy, it seems *everyone* is dissatisfied and looking for different models. One by one, Latin American countries are moving from Right to Left. On the other hand, in Europe, the parties of social democracy have been losing ground to the Center (Europe's "right-wing" parties would be Centerist or Left in the United States), one after another.

All of this frenetic searching begs the fundamental question: What's the Economy for, Anyway? How much stock can we take in the Dow Jones? Is the Gross Domestic Product the measure (the grosser the better), and stuff the *stuff*, of happiness? Is the good life the *goods* life?

If so, then there's little doubt that the freer-market regimes win big. U.S. per capita GDP is still 30 percent higher than the average in Western Europe, just as it was a generation ago. We've got bigger homes, bigger cars, and more high-definition televisions. On the other hand, if we measure success by the happiness, health, fairness, and sustainability of economies, the picture looks very different.

I've been doing a little number-crunching lately, comparing data from sources such as the

2007 OECD [Organization for Economic Cooperation and Development] Fact Book, the World Health Organization, and the UN [United Nations] Human Development Index, trying to see how countries are doing in real, empirical terms when it comes to health, quality of life, justice, and sustainability. The results, I'm afraid, would come as a shock to those who look to the United States as the model of economic success.

Let me do a few of the numbers: Compared, for example, to the western European nations, the United States ranks *worst* or next-to-worst when it comes to child welfare, health care, poverty, income equality, pollution, CO₂ emissions, ecological footprint, personal savings, income and pension security, balance of payments, municipal waste, development assistance, longevity, infant mortality, child abuse, depression, anxiety, obesity, murder, incarceration, motor vehicle fatalities, and leisure time. We do slightly better in education. Our unemployment rate looks pretty low, unless you count those 2.3 million people we've got behind bars, an incarceration rate 7 to 10 times as high as Europe's.

Since 1970, Europeans have traded a portion of their productivity gains for free time instead of

stuff, a trade that pays off in many ways. New studies show that long working hours, the norm in the United States, contribute to poor health, weakened family and community bonds, and environmental damage. Americans, who are far less healthy than Europeans, spend twice as much for health care per person. In fact, we spend nearly half the world's *total* health care budget, an amount that will reach 20 percent of our GDP by 2010—with the worst outcomes. Yet, all of that spending counts as a *plus* when it comes to GDP. The leisure that Europeans enjoy, the long meals and café conversations, and the long walks and bike rides count only as wasted time, adding not a single point to GDP. *La dolce vita*, by that measure, is for losers.

But which countries come out on *top* in measures of quality of life? It's the northern European nations, those that combine a strong social safety net with shorter working hours, high but progressive tax rates, and strong environmental regulations. *The pattern is as clear as can be.*

I have found no one who refutes these figures. They simply explain them away by saying that the United States can't be like Europe. Why not?

One argument for why the United States can't even have things such as paid maternity leave—a reality in every country on the globe except the United States, Swaziland, Lesotho, Liberia, and Papua New Guinea—is that we're so affected by *globalization*. But with its massive domestic market, the United States is just about the *least* affected by globalization of all industrial countries.

American conservatives argue that Europeans can't continue to compete in the global economy. But according to the World Economic Forum, over the past few years, four of the six most globally competitive countries have been in Europe. Even American businesses invest five times as much each year in Germany as they do in China and more in Belgium than in India. And they make money doing it.

When all else fails, there's the final appeal: The United States may not be very healthy, fair, or sustainable, but it's "the land of opportunity," where

anyone can make it big if they're willing to work hard enough. Yet a recent study finds that Americans actually have only about one-half to one-third as much chance as Europeans of escaping low-income lives and rising to the top.

The steady drone from some European business leaders about the American economic miracle masks what should be obvious—they'd like to join our CEOs in making 400 times as much as their average workers, instead of the miserable 30 to 40 times as much they now make. Their voices speak louder than those of the average European citizen, who enjoys his or her six weeks of vacation, restful meals, family leave, health care, sick pay, free college education, and secure pension plan.

Since Ronald Reagan declared that "government cannot be the solution because government is the problem," indices of American quality of life, fairness, economic security, and environmental sustainability have all fallen sharply in comparison with those in Europe. The conservative economic revolution has produced a *gush-up* instead of a "trickle-down." For most of us, the "ownership society," emphasizing privatization, deregulation and massive tax cuts for the wealthy, is really a "*you're on your own* ship" society.

To make America better, our President tells us, we must do even *more* of these things, making tax cuts for the wealthy permanent, for example. But the working definition of insanity is to keep doing the same things hoping for a different result.

If we want to build societies that really work for people, we need to ask, "What's the Economy for, anyway?" And then we need to separate the real results from the myths, shed a little of our American hubris and start looking at how other countries are actually edging us out by providing policies that succeed. That way lies a happier, healthier, more just, and sustainable world.

John De Graaf is a documentary filmmaker and co-author (with David Wann, Thomas Naylor, and Vicki Robin) of *Affluenza: The All-Consuming Epidemic* (San Francisco, CA: Berrett-Koehler, 2005). Reprinted with permission from John De Graaf.

As this chapter has demonstrated, social welfare in the United States is characterized by a high degree of diversity rather than a monolithic, highly centralized, or well-coordinated system of programs. Rather, a great variety of organizations

provide a wide range of benefits and services to different client populations. The vast array of social welfare organizations contributes to what is commonly called "the welfare mess." Consequently, different programs serving different groups through

different procedures have created an impenetrable tangle of institutional red tape that is problematic for administrators, human service professionals, and clients.

The complexity of U.S. social welfare policy can be attributed to several influences, some of which are peculiar to the American experience. For instance, the U.S. Constitution outlines a federal system whereby states vest certain functions in the national government. Although the states have assumed primary responsibility for social welfare through much of U.S. history, this changed with Franklin Delano Roosevelt's New Deal that ushered in a raft of federal programs. Over subsequent decades, federal social welfare initiatives played a dominant role in the nation's welfare effort. Still, states continued to manage important social welfare programs, such as mental health, corrections, and social services. Over time, the relationship between the federal government and the states changed. From the New Deal of the 1930s through the Great Society of the 1960s, federal welfare programs expanded, forming the American version of the "welfare state." Beginning in the 1980s, the Reagan administration sought to return more of the responsibility for welfare to the states, a process called devolution.⁸⁰ This process was furthered by the Clinton administration with the signing of the PRWORA.

A second factor is attributed to the relatively open character of U.S. society. Often referred to as a melting pot, the national culture is a protean brew of immigrant groups that become an established part of national life.⁸¹ A staggering influx of Europeans in the late nineteenth century gave way to waves of Hispanics and Asians a century later.⁸² Historically, social welfare programs have played a prominent role in the acculturation of these groups. At the same time, many ethnic groups brought with them their own fraternal and community associations, which not only provide welfare benefits to members of the community but also serve to maintain its norms. Other groups that have exerted important influences on U.S. social welfare are African Americans, the aged, women, and Native Americans. The very pluralism of U.S. society—a diverse collection of peoples, each with somewhat different needs—contributes to the complexity of social welfare.

The economic system exacerbates the complexity of social welfare. With important exceptions, the U.S. economy is predominantly capitalist, with most goods and services being owned, produced,

and distributed through the marketplace. In a capitalist economy, people are expected to meet their basic needs in the marketplace through labor force participation. When people are unable to participate fully in the labor market, like the aged or the handicapped, social programs are deployed to support these groups. These programs take various forms. Many are governmental programs. Private sector programs often complement those of the public sector. Within the private sector, two organizational forms are common—nonprofit organizations and for-profit corporations. Often these private sector organizations coexist, proximate to one another.⁸³ For instance, in many communities, family planning services are provided by the public health departments; Planned Parenthoods (a private nonprofit); and by for-profit health maintenance organizations.

Finally, various religious or faith-based organizations strongly influence American social welfare. This is seen most clearly in the range of faith-based agencies that offer social services, such as Jewish Family Services, Lutheran Social Services, Catholic Charities, and the Salvation Army. In many cases, religious-based agencies provide services to groups that would not otherwise receive them. Today, many faith-based agencies receive federal funds for various services they provide to the public. It is likely that this trend will grow.

The pluralism of national culture is of increasing interest to social welfare policy analysts as the influence of the federal government in social policy diminishes. In light of reductions in many federal social programs and calls for the private sector to assume more responsibility for welfare, the prospect of molding the diverse entities involved in American social welfare into one unified whole under the auspices of a central federal authority seems remote. This vision of a unified social welfare system is implicit in the proposals of advocates for nationalized programs that ensure basic goods and services such as food, housing, education, health, and income as a right of citizenship. Although programs of this nature have been integral to the welfare states of northern Europe for decades, there is a serious question as to their plausibility for the United States given the complexity built into its social welfare system.⁸⁴ Given these developments, welfare professionals face a formidable challenge: How can basic goods and services be brought to vulnerable populations within a context of such complexity and uncertainty?

DISCUSSION QUESTIONS

1. According to the authors, American social welfare is undergoing a transition. Which ideologies, schools of political economy, and interest groups within social welfare stand to gain most from this transition?
2. Ideology tends to parallel schools of political economy. How would classical conservatives and liberals address current social welfare issues such as health care, long-term care for the aged, and substance abuse? How would neoconservatives and neoliberals diverge from traditional conservatives and liberals on these issues?
3. Which schools of political, social, and economic thought discussed in this chapter would come closest to being classified as moderate? Why?
4. The chapter argues that in large measure social policy dictates social work practice. Do you agree with that premise? Explain your position. Can you think of any instances (historic or otherwise) in which social work practice has led to changes in social welfare policy?
5. In your opinion, which schools of economic and political thought are the most compatible with social work practice? What are the incompatibilities in the various schools of thought with macro- and micro-level social work and practice?
6. Chapter 1 describes the U.S. political continuum ranging from Liberalism to Libertarianism. Access the website of the National Association of Social Workers (www.naswdc.org) and read through the Advocacy and Legislative Issues links. Where would you place the NASW along the U.S. political continuum? Why?
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A Brief History of the American Social Welfare State



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Understanding the American social welfare state requires knowing the historical foundations upon which it was built. This chapter examines the historical antecedents of the American social welfare state, taking the reader from its distant roots to the more recent developments that have shaped and defined it.

Early Antecedents of Welfare Statism

Virtually all societies have developed some form of welfare-related activities. These governmental, societal, familial, or cultural institutions may address several human needs: (1) they help ameliorate the individual suffering of those judged worthy of help, (2) they promote the common good by distributing economic and other resources from a higher social group or class to a lower one, (3) they extend the rights of citizenship to groups outside of the societal mainstream, and (4) they promote social stability by managing lower social or economic class grievances. Different developed economies highlight different parts of these welfare functions.

Judeo-Christian Doctrine and Social Welfare

Social welfare functions are found in early history, and Walter Trattner notes that Hammurabi, the Babylonian ruler who died in 1750 BC, made the protection of women and children part of his famous code. The Greeks and Romans also developed welfare-related policies that included daily allowances for the handicapped, the public distribution of grain for the needy, and institutions for the custodial care of various vulnerable groups.¹

From its early beginnings, Judaism contained strict laws for dealing with the poor and for giving *tzedakah* (Hebrew for charity). These laws helped in forming the basis of the modern welfare state in many ways. For instance, the Jewish law required farmers to leave a minimum of one-sixtieth of their field crops unharvested, which was left for the poor. To discourage dependence, the poor were encouraged to be competitive in gathering the *peah* (the standing allotment). Each was allowed to take as much as he or she could gather.

Jewish Rabbinic law also established one of the earliest means tests. Anyone who had 200 *Zuz* or

above (established by the Rabbis as sufficient to purchase food for one year) was considered above the poverty line and ineligible for assistance. Like modern means tests, complex exemptions were employed in calculating the poverty level. For example, the residence, household utensils, religious articles, and certain of the wife's property were exempt. The poor were not forced to give up the essential elements of their lifestyle to be eligible for assistance.

The process of Jewish charity was highly organized in biblical times, and it was common for each community to have two types of charity organizations: one for poor travelers and the other for poor residents. The organization of the charity was administered by a tax committee, and each family was assessed a tax, which was then distributed to the resident poor on Fridays. In addition, plates of prepared food were collected for the traveling poor. The law also prescribed what was to be given. If a person was hungry, he or she must be given food; if a person is without adequate clothing, he or she must be supplied with clothing.

Jewish law was complicated in terms of charity, mirroring the ambivalence and contradictions of the modern welfare state. On the one hand, the poor who were entitled to alms but who worked hard and were self-supporting instead of burdening the public were worthy of high praise. Instead of needing others for help, people came to them for assistance. On the other hand, those who were old, sick, or otherwise incapable of eking out the barest of existence yet refused help were depriving themselves of essentials and therefore endangering their lives. These people were viewed as unworthy of pity. False pride was scorned upon.

It was forbidden in Jewish law to turn away a poor person empty-handed. Of the degrees of Jewish charity, the highest forms were assisting the poor with a gift or a loan, accepting them into a business partnership, or helping them find employment. In other words, the highest form of charity was helping the poor become self-sufficient. Conversely, the lowest kind of charity was giving it reluctantly, as in involuntary tithing. Charity was expected to be given with an open heart: "Whoever gives charity to a poor man ill-manneredly and with downcast looks has lost all the merit of his action even though he should give him a thousand gold pieces."² The next level of charity was when the giver did not know who received the gift and the recipient did not know who gave it. The provision

of anonymous charity to strangers also forms the backbone of the modern welfare state. Taxpayers pay taxes that provides assistance to strangers who cannot reciprocate, nor even recognize or acknowledge the individual whose tax money went to them.

Since most of the early Christians including Jesus, Paul, Peter, and other early founders of the Christian church were Jews, it is not surprising that Jewish charity customs were embedded in Christian doctrine. Beginning in the sixth century, monasteries became important sources of relief for the poor, and some religious orders specifically focused on aiding the needy. The funds to run charitable endeavors came from rents received on church lands, donations, endowments, collections, and tithing. Some of this money was used to help the needy by providing food, provisions, and medical services.

The English Poor Laws

In some cases, the feudal system that dominated Europe throughout the Middle Ages helped the poor mitigate the hardship and distress of the period. Many, if not most, people in the medieval period were serfs who lived on a manor consisting of a castle, church, small village, and the surrounding farm land. Serfs worked as tenant farmers for the feudal lords. If the feudal lord was generous, the basic needs of the serfs such as employment, food, clothing, and aid in sickness were met. Conversely, life on the manor could be miserable if the lord of the manor was cruel and vindictive. Serfs were generally forbidden to leave the feudal estate without the permission of their lord.

Others in the medieval period lived in towns, which began as religious centers and later became commercial centers. People living in towns were “free men” compared to serfs. Social welfare needs were met by service, craft, or merchant guilds. Although the primary responsibility of guilds was to protect its members and meet the social and economic needs, it sometimes helped others, which might include feeding the needy or providing free lodging to travelers. In addition to the guilds, medieval hospitals (usually connected to monasteries) provided an important service by offering free medical care for weary travelers, the old, orphans, and the destitute. Many of the early hospitals, especially those in the burgeoning industrial cities, were later taken over by municipal authorities. The growth of

industrial cities combined with the increased power of the state and the collapse of feudalism led to the creation of the English Poor Laws, the first modern codification of laws and responsibilities relating to the poor.

The English Poor Laws functioned as an early model for much of American social welfare. Early social welfare relief in England was considered a private and church matter. Individual benefactors took responsibility for building almshouses, hospitals, and even bridges and roads. Despite this, the major responsibility for meeting the needs of the poor rested with the church. Most European governments, including the English government, assumed little responsibility for the care of the poor. This situation would change with industrialization and its attendant social and economic problems.

Rapid industrialization and the growth of manufacturing led to the conversion of farmland into more profitable sheep pastures, a transformation that was necessary to feed England’s growing woolen mills. As newly released serfs followed economic opportunities, the urban migration of displaced and impoverished peasants led to untoward social consequences, including begging and vagrancy.

The Black Death (bubonic plague) of 1349 drastically reduced England’s population, leading King Edward III to create a Statute of Labourers that fixed maximum wages, placed travel restrictions on the unemployed, forced the jobless to work for any employer willing to hire them, and outlawed alms for the able-bodied.³ In 1531, the English Parliament outlawed begging for the able-bodied. Although repressive, the act also instructed local officials to seek out the worthy poor and assign them areas where they could beg.⁴

The passage of the Act for the Punishment of Sturdy Vagabonds and Beggars in 1536 (one of the series of Poor Laws) further mandated the English government to assume limited responsibility for the poor. Although this act increased the punishment for begging, it also ordered officials to obtain resources through voluntary church donations to care for the poor, the sick, the handicapped, and the aged. The statute required local officials to find work for the able-bodied and to arrange for the apprenticeship of poor children aged 5 to 14. In 1572, the English Parliament enacted yet another poor law, this time requiring local officials to implement a mandatory tax for the provision of economic relief to the poor.⁵

In 1601, the English government established the Elizabethan Poor Laws. These laws were meant to control the poor who were unable to obtain employment in the new industrial sector and who, because of that, might become disruptive. Taxes were levied to finance a law that by modern standards was harsh. The law continued the separation of the “deserving” from the “undeserving” poor. The worthy poor were the lame, the blind, orphaned children, and those unemployed through no fault of their own; the unworthy poor were vagrants, drunkards, and the slothful. The core of the Elizabethan Poor Laws would stand for 250 years.

The Elizabethan Poor Laws contained both positive and repressive features. For example, parents with means were legally responsible for supporting their children and grandchildren, and children were responsible for supporting their parents and grandparents. On the repressive side, the unworthy poor were sent to workhouses and forced to do menial work for the minimum necessities of life. Poor people who refused to work could be sent to jail, or in some cases, even executed. The English Poor Laws established the principle of “less eligibility,” the idea that any form of public assistance should be less than the lowest prevailing wage.

These laws institutionalized the responsibility of the English government to provide relief to the needy and provided the needy with a legal right to receive that assistance. To clarify the boundaries of government help, the law recognized three classes of dependents and proposed remedial measures: Needy children were given apprenticeships, the able-bodied were given work, and the worthy poor were provided either indoor (institutional) or outdoor (home) relief. The law also required local governments to assume responsibility for the needy.⁶ These Poor Laws formed the basis for statutes enacted in both colonial and postcolonial America.⁷

The Poor in Colonial America

Key aspects of the Elizabethan welfare system were adopted by the American colonies. Like its English corollary, the parish or colonial town was responsible for its residents. Until the 1700s when almshouses began to appear, cases of pauperism were handled on an individual basis in town meetings.

When the number of poverty cases increased due to indentured servants and abandoned children, the English system of overseers—an official who administered poor relief—was introduced.

Most settlers in colonial America were poor, but unlike their European ancestors, they were not destitute and so abject pauperism was not widespread.⁸ According to Robert Morris, less than 1 percent of American colonists received help from outside sources.⁹

In small towns unable to support an almshouse, it was not uncommon for the town council to auction the poor off to neighboring farmers, apprentice out children, place the poor in private homes at public expense, or send them to privately operated almshouses. Settlers believed that children should be part of a family unit and the practice of indentured child servitude was widespread. By the end of the colonial period, the locus of responsibility for the poor began to shift from the town to the state or colony.¹⁰

While colonial settlers were often compassionate to indigent townspeople, they showed less compassion for strangers who were destitute. Some communities enacted laws of settlement as the numbers of poor increased. Residency requirements were enforced through policies of “warning out” or “passing on.” “Warning out” meant that newcomers were strongly encouraged to move on if they appeared indigent. “Passing on” was returning the transient poor to their former counties of residence. Some colonies established residency requirements to determine eligibility for public assistance. The able-bodied poor in colonial America were treated harshly and idleness was regarded as a vice whereby the able-bodied loafer was indentured, expelled from town, whipped, or jailed. By the late eighteenth century, the able-bodied unemployed were placed in workhouses or almshouses.¹¹

The process for helping the poor changed radically by the early 1800s. The quasi-benevolence of the town council was replaced by a reliance on workhouses. In some areas, the use of outdoor relief was virtually abandoned in favor of institutional care. It was only in the mid-1800s that the federal government conceded even limited responsibility for the poor. Local government activities were based on a belief that poverty was a consequence of moral weakness, a theory linked to the Puritan values more prevalent in New England.¹²

Social Welfare in the Civil War Era

The federal government's responsibility to provide relief has historically been a contentious issue. Dorothea Dix, a name synonymous with the humane care of the mentally ill, brought this issue to a head. While volunteering as a Sunday school teacher for an insane asylum in 1841, Dix underwent a spiritual and emotional awakening. Appalled by the conditions at the asylum, she committed herself to fighting for the humane care of the mentally ill.

The majority of mentally ill people in the 1840s were placed in public mental institutions, jails, or almshouses. Their treatment was often brutal and consisted of beatings, being chained, or being forced into cages or pens. Dix concluded that neither private philanthropy nor local action could remedy the problem—the solution laid in state and federal intervention.

After successfully lobbying for state action, Dix realized that the large expenditures necessary to reform mental health services demanded federal intervention. With the support of well-known clergy, prominent citizens, newspaper people, and public and private organizations, Congress passed a bill in 1854 that provided federal support for the mentally ill. Unfortunately, President Franklin Pierce vetoed the bill, claiming that, "If Congress has the power to make provisions for the indigent insane . . . it has the same power for the indigent who are not insane. . . . I cannot find any authority in the Constitution for making the Federal Government the great almoner of public charity throughout the United States."¹³ Pierce's veto was largely based on his belief in states' rights. For the next 75 years, this veto would provide the rationale for the federal government's refusal to provide social welfare services.¹⁴

The Civil War ushered in an important period of relief activities. Families who lost a breadwinner or whose breadwinner returned from the war permanently disabled could not be blamed for their misfortunes. In response, localities enacted laws that raised funds for the sick and needy, and in some instances, for the founding of homes for disabled soldiers.

Other health issues that emerged during the Civil War included the rampant disease and filth in army camps and hospitals and the shortage of trained medical personnel. To remedy this situation, a group of citizens (composed largely of women)

organized the U.S. Sanitary Commission in 1861, the first important national public health group. Functioning as a quasi-governmental body, the commission was financed and directed by the private voluntary sector. Initially working in preventive health education, the commission eventually became involved in a variety of direct and indirect services for soldiers.¹⁵

Another institution that emerged from the Civil War was the Freedmen's Bureau. By the close of the war, political leaders realized that the immediate emancipation of millions of slaves would create severe social problems. Former slaves with no occupational training, land, or jobs would require various kinds of assistance, which prompted Congress to establish the Bureau of Refugees, Freedmen, and Abandoned Lands in 1865. The Freedmen's Bureau was responsible for directing a program of temporary relief for the duration of the war and one year afterward. After a bitter struggle, Congress extended the Freedmen's Bureau for an additional six years.

Under General Oliver Howard, the bureau offered services designed to help African Americans make the transition from slavery to freedom. For example, it staffed an emergency relief center that distributed 22 million rations to needy Southerners. The bureau also functioned as an employment agency, a settlement agency, a health center that employed doctors and operated hospitals, an educational agency that promoted African American colleges and provided financial aid, and as a legal agency that maintained courts where civil and criminal cases involving African Americans were heard. The Freedmen's Bureau was an important precedent for federal involvement in a variety of human services. In 1872, it was dissolved by Congress.¹⁶

Industrialization and the Voluntary Sector

Voluntary efforts to enhance community welfare has been a prominent feature of social welfare throughout U.S. history. During his travels through early nineteenth-century America, Alexis de Tocqueville commented on the tendency of Americans to band together in voluntary organizations to solve community problems:

Americans of all ages, all conditions, and all dispositions, constantly form associations.

They have not only commercial and manufacturing companies, in which all take part, but associations of a thousand other kinds religious, moral, serious, futile, extensive or restrictive, enormous and diminutive. The Americans make associations to give entertainments, to found establishments for education, to build inns, to construct churches, to diffuse books, to send missionaries to the antipodes; and in this manner they found hospitals, prisons, and schools. If it be proposed to advance some truth, or to foster some feeling of encouragement of a great example, they form a society. Wherever, at the head of some new undertaking, you see the Government of France, or a man of rank in England, in the United States you will be sure to find an association.¹⁷

Reliance on voluntary associations to solve social problems reflected the nature of early American communities. Prior to industrialization, most people lived in communities with institutions that afforded them a high degree of self-sufficiency. Survival depended on interdependence. This kind of cohesiveness can still be found among certain religious groups, such as the Amish, who manage human needs through the voluntary impulses of its members and see good deeds as a normal extension of their daily activities.

With the advent of industrialization, the traditional method of managing social welfare needs proved inadequate for most groups, and special institutions and services were needed.¹⁸ It is not surprising that specialized social welfare institutions would emerge in the wake of industrialization and the subsequent migration of millions of families searching for economic opportunities. From 1890 to 1920, 22 million immigrants came to the United States with 75 percent of them settling in cities. During the 1930s, 6 million people went from farms to cities.¹⁹

Life in late nineteenth-century urban America was hard. The dream of milk and honey that motivated immigrants to leave their homeland became a nightmare for many. Instead of being paved with gold, the streets of American cities were overcrowded, rampant with disease and crime, and economically destitute. Many tenement houses contained no windows or indoor plumbing. Tuberculosis was widespread and infant mortality among some immigrant groups ran as high as 50 percent. Scant medical care existed for the poor. There was

no public education and mental illness and prostitution rates were high among immigrants.²⁰ The industrial and economic prospects were equally bleak. Factory conditions were often abominable with workers expected to work six or seven days a week, and often on Sunday. Eighteen-hour days were common, especially in summer.²¹ Factories were poorly lit, unsanitary, and easily turned into fire traps. Industrial jobs offered almost no security. Moreover, homework (taking piecework home for assembly by whole families in one- or two-room tenements) was common. Women were forced to work night shifts and then take care of their homes and children by day.²² No protective legislation existed for women until the early 1900s and child labor was legal. According to Richard Hofstadter, industrial accidents affected one out of every 10 to 12 workers, and employees had neither worker's compensation nor disability insurance.²³ Every 15 to 20 years there was another economic depression. Taken together, it is obvious that the lot of the immigrant and many working-class Americans was hard. The extent of this suffering is illustrated in a description of New York City during the late nineteenth century:

With the shift of population from the grange to the tenement house came a degree of over expansion and under management that brought large cities to the crisis point: sanitation and health were plainly inadequate; there was a constant fear of rioting and crime; the police used their night sticks against the people they were sworn to protect. Fed by immigrants streaming through Castle Garden and, after 1891, Ellis Island, New York's East Side was the end point of all cities. With over 500 people for each acre, nearly five times the average for the rest of Manhattan, the Tenth Ward was the most densely settled area in the world, and its ghetto was larger than Warsaw's. It was "the suicide ward" and "the typhus ward," and the breeding place of the "white plague" of tuberculosis, epidemics of all kinds, crime, pauperism, alcoholism, sweated labor, hopelessness, and a frightful mortality rate.²⁴

A concerned middle class struggled with the mounting social crisis in American cities. There was an urgency to this task. By 1900, medical science had identified the cause of many contagious diseases but had yet to develop preventive vaccines or cures. It was no accident that Charles Loring Brace,

a child welfare pioneer, titled his book *The Dangerous Classes of New York*. These conditions were rife for the growth of graft and corruption as immigrants struggled to attain jobs, housing, and other resources. The political machines that emerged converted city governments into fiefdoms of patronage. In an exposé of graft in New York City, Jacob Riis, a muckraking journalist, alleged that the political machine of Boss Tweed's Tammany Hall was nothing more than "a band of political cutthroats." Not particularly surprising, Riis noted that Tweed was the product of a Fourth Ward tenement.²⁵ Even when muckrakers uncovered abuses and posted them in banner headlines, political bosses were so confident of their "machine" that they responded with defiance. For example, when hearing of an exposé by muckraker Lincoln Steffens, George Washington Plunkett, a Tammany Hall crony, quipped: "Steffens means well but, like all reformers, he don't know how to make distinctions. He can't see no difference between honest graft and dishonest graft and, consequently, he gets things all mixed up."²⁶

Social Darwinism

Prominent scholars tried to draw lessons from the natural sciences that could be used in social policy and social engineering. Borrowing from Darwin, some American proponents of the new science of sociology tried to apply the principles of natural selection to social affairs.

Social Darwinism was a bastard outgrowth of Charles Darwin's theory of evolution described in his 1859 classic, *The Origin of the Species*.²⁷ Social theorists, such as Britain's Herbert Spencer and America's William Graham Sumner, reasoned that if Darwin's laws of evolution determined the development of species, they could also be applied to the laws of society.²⁸ Applying Darwin's observations to society along with laissez-faire economic principles led to a problematic set of assumptions. First, if the *survival of the fittest* (a term coined by Spencer) was a law governing the lower species, then it must also govern the higher species. Subsidizing the poor and allowing them to survive and reproduce circumvented the law of natural selection, and because the poor reproduced more rapidly than the middle class, society was funding its own demise. Social Darwinists believed that the poor would eventually overrun society and lower the general level of civilization.

If competition for resources is a law of nature, then the poor are poor because they cannot compete against other classes. Conversely, the economic elite are entitled to their spoils because of their fitness and competitive abilities. By aiding the poor and allowing them to reproduce, society is altering the laws of nature and weakening the human gene pool. Social Darwinists believed that the poor must pay the price demanded by nature and be allowed to die out. According to the Social Darwinists, social welfare thwarts evolutionary progress toward higher forms of social life. Speaking for some intellectuals, the Herbert Spencer drew this conclusion:

It seems hard that widows and orphans should be left to struggle for life or death. Nevertheless, when regarded not separately but in connexion with the interests of universal humanity, these harsh fatalities are seen to be full of beneficence—the same beneficence which brings to early graves the children of diseased parents, and singles out the intemperate and the debilitated as the victims of an epidemic.²⁹

While some intellectuals supported the harsh strictures of Social Darwinism, progressive Christianity provided another approach.

Religion and Social Welfare

Social welfare and religion were inextricably linked in nineteenth-century America. Almost all forms of relief emanated from church groups, and all major denominations had some mechanism for providing social welfare services.³⁰ As early as 1880, there were 500 private, church-related social welfare organizations in New York City alone, with the largest network of social services operated by Protestant churches.

Poverty was seen as a moral failing in orthodox Protestant theology. Martin Luther viewed work as a responsibility to God that conferred dignity. In Luther's view, a person served God by heeding his "calling" and doing the work of his vocation. Conversely, those who are able-bodied and unemployed were sinners. John Calvin took Luther's argument one step further by claiming that work carried out the will of God, and as such, would ultimately help to create God's kingdom on earth. According to both Luther and Calvin, God-fearing people must work regardless of the wage or type of employment.³¹

If the command to work is God-given, then economic success is a sign of God's favor. In modern

conservative theology, this idea is expressed in prosperity theology, also called the prosperity gospel, the health and wealth gospel, or the gospel of success. Simply put, this theology teaches that God rewards faith—and generous tithing—with financial blessings, good physical health, and happiness.³² Conversely, poverty is a sign of God’s disappointment with a person’s lack of faith. This Protestant work ethic promoted a work-oriented society and provided the religious foundation for neglecting the plight of the poor. Adding a religious dimension to poverty also helped conservatives better distinguish the worthy from the unworthy poor.³³

Nineteenth-century conservative theologians employed a religious framework to connect poverty and improvidence: People were poor because of drinking, slothfulness, and licentious behavior and gambling. Some critics have argued that despite the social services provided by churches, conservative Protestant theology was essentially opposed to social welfare.³⁴

Early social work was largely made up of untrained volunteer workers, most of whom were middle or upper-middle-class women connected to a church. This group fostered the view that to reclaim providence, the poor must be taught to live a moral and disciplined life. While some early religious social workers clung to teaching the moral life, many also understood the need for material assistance.³⁵ Overall, the major emphasis of early social workers was often more on spiritual guidance than material aid. When material aid was given, it was often tied to demands, such as abstinence from alcohol and other vices.

Church-based assistance was often linked to harsh criteria. For example, it was not uncommon for social workers to appraise the worth of the family’s possessions and then instruct them to sell off everything to qualify for relief. Nor was it uncommon for social workers to deny relief because they felt that the poor family was intemperate and insufficiently contrite. If the family refused moral guidance, they could be declared ineligible for relief. Even though volunteer social workers dispensed relief, many were opposed to the concept of it. They believed that distributing relief was counterproductive since a reliance on charity weakened the moral fabric of the poor and provided a disincentive to work.

Conservative Protestant theology was not without its opposition. The Social Gospel movement emerged in the late nineteenth and early twentieth

century. Composed of theologians concerned with the abuses inherent in industrialization and the excesses of capitalism, Social Gospelists such as Josiah Strong, Graham Taylor, and others believed that the church must recapture the militant spirit of Christ by taking on social justice and poverty issues.³⁶

The Social Gospelists argued that many mainstream churches wrongfully stressed spirituality over morality.³⁷ They condemned classical economics, business ethics, and the lawlessness of the plutocracy. Social Gospelists believed that social reform could not occur without a regeneration of character.³⁸ Although the movement reflected a wide range of thought, all members were moved by a sense of social crisis, and all believed in the necessity of a Christian solution.³⁹ The legacy of the Social Gospel movement is evident in Liberation Theology, a grass-roots movement of largely Catholic theologians who minister in many Latin American and African nations. The ideologies underlying Social Darwinism, Christian charity, and the Social Gospel were reflected in industrial era social welfare organizations.



Watch this video that highlights Obama’s eloquent speech connecting the killings at Emanuel African Methodist Episcopal Church in 2015 with social justice and religion.

www.youtube.com/watch?v=RK7tYOVD0Hs

Charity Organization Societies

Begun in 1870, by 1900 the Charity Organization Societies (COSs) had offices in most American cities.⁴⁰ With the exception of limited state indoor and outdoor relief, the COSs were a major provider of care to the destitute. Although COSs varied in their structures and methods, they generally coordinated relief by operating community-wide registration bureaus, provided direct relief, and educated the upper and lower classes as to their mutual obligations.

The work of the COSs was carried out by a committee of volunteers and agency representatives who examined cases of needy applicants and decided on a course of action. The task of the COS “friendly visitor” was to conduct an investigation into the circumstances surrounding the case and instruct the poor in ways to better manage their lives. Drawn from the middle and upper classes, friendly visitors often held morally superior attitudes toward their clientele. Some observers

interpreted these intervention as a kind of social control as much as a means of providing assistance.⁴¹ Moreover, the charity provided by these organizations was often less than generous since COS leaders believed that beneficent charity was counterproductive and contributed to sloth and dependency. Josephine Shaw Lowell, president of the New York COS, believed that charity should be dispensed “only when starvation was imminent.”⁴²

It was difficult for some friendly visitors to maintain their sense of Christian duty when faced with immoral behavior. When souls were beyond instruction and charity, more radical measures were necessary. Charles Loring Brace, head of the New York Children’s Aid Society, described his approach to dealing with a German mother who worked as a swill-gatherer (collector of refuse for use as pig feed):

On the eastern side of the city, in the neighborhood of Fortieth Street, is a village of squatters, which enjoys the title of “Dutch Hill.” The inhabitants are not, however, “Dutch,” but mainly poor Irish, who have taken temporary possession of unused sites on a hill, and have erected shanties which serve at once for pig-pens, hen-coops, bed-rooms, and living-rooms. They enjoy the privilege of squatters in having no rent to pay; but they are exposed to the penalty of being at any moment turned out from their dens, and losing land and house at once. . . . The village is filled with snarling dogs, which aid in drawing the swill or coal carts, for the children are mainly employed in collecting swill and picking coals through the streets.

[An] old rag-picker I remember whose shanty was a sight to behold; all the odds and ends of a great city seemed piled up in it bones, broken dishes, rags, bits of furniture, cinders, old tin, useless lamps, decaying vegetables, ribbons, cloths, legless chairs, and carrion, all mixed together, and heaped up nearly to the ceiling, leaving hardly room for a bed on the floor where the woman and her two children slept. Yet all these [children] were marvels of health and vigor, far surpassing most children I know in the comfortable classes. The woman was German, and after years of effort could never be induced to do anything for the education of her children, until finally I put the police on their track as vagrants, and they were safely housed in the “Juvenile Asylum.”⁴³

What happened to the children placed in the “Juvenile Asylum”? Brace’s solution was simple: The Children’s Aid Society transported between 50,000 and 100,000 “orphans” westward by train where they were placed with farm families.⁴⁴ The advertisement in Figure 2.1, posted in McPherson, Kansas, provides some detail on the procedures devised by Brace and his associates.

Settlement Houses

Begun in the 1880s, the settlement house movement—a progressive response to the harsh urban conditions of the times—emerged in most big U.S. cities over the next two decades. Settlement houses were started in immigrant and low-income neighborhoods by wealthy people and staffed by college students, single women, teachers, doctors, lawyers, and others who moved into settlement houses as temporary residents. Rather than engaging in friendly visiting, settlement residents tried to bridge class differences and develop a less patronizing form of assistance. Instead of coordinating charities like COSs, settlements sought to help neighborhood residents organize themselves. Because they lived in the same neighborhoods as immigrants, settlement workers had insight and knowledge about the social and economic conditions of American cities.

Jane Addams started Hull House in 1889. Initially, she approached the Chicago ethnic community with a brand of Christian Socialism that was derived from a “strenuous moral purgation.”⁴⁵ For Addams, the mission of Hull House was residence, research, and reform. Settlements would work with neighborhood residents to study the causes of poverty, communicate this to the public, and work for political and legislative reform.⁴⁶ The co-founder of Hull House, Ellen Gates Starr, described the values of the settlement house worker.

After we had been here long enough and people see that we don’t catch diseases and that vicious people do not destroy us or our property . . . we have well founded reason to believe that there are at least half a dozen girls in the city who will be glad to come and stay a while and learn to know the people and understand them and their ways of life; to give out of their culture and leisure and overindulgence and to receive the culture that comes from self-denial and poverty and failure which these people have always known.⁴⁷

Homes For Children

WANTED

A Company of Homeless Children from the East Will
Arrive at

McPherson, Friday, September 15.

These children are of various ages and of both sexes, having been thrown friendless upon the world. They come under the auspices of the Children's Aid Society, of New York. They are well disciplined, having come from various orphanages. The citizens of this community are asked to assist the agent in finding good homes for them. Persons taking these children must be recommended by the local committee. They must treat the children in every way as members of the family, sending them to school, church, Sabbath school and properly clothe them until they are 18 years old. Protestant children placed in Protestant homes and Catholic children in Catholic homes. The following well known citizens have agreed to act as a local committee to aid the agents in securing homes:

Dr. Heaston

H. A. Rowland

C. W. Bachelor

F. A. Vaniman

W. J. Krehbiel

K. Sorensen

Applications must be made to and endorsed by the local committee.

An address will be given by the agents. Come and see the children and hear the address. Distribution will take place at

Opera House, Friday, September 15

at 10:00 a. m. and 2:00 p. m.

Miss A. L. HILL and MISS C. B. COMSTOCK, Agents, 105 East 22nd Street, New York City. W. W. BUGBEE, Elderado, Kansas, State Agent.

FIGURE 2.1 Handbill Promoting Orphan Train, McPherson, KS, 1911

Source: "Wanted Homes for Children" Advertisement in Troy, Missouri, Free Press, February, 1910.



One of the best-known settlement houses “Hull House” was established by Jane Addams in 1889.

Source: AP Images

There were more than 300 settlement houses by 1915, and most larger American cities could claim at least one or more settlements.⁴⁸

Although settlement houses provided individual services, such as adult education and literacy classes, the larger ones were heavily reform-oriented. These reforms were achieved by organizing neighborhood residents, by using interest groups containing elite citizens, and by forming and joining national alliances. Settlement-pioneered reforms included tuberculosis prevention, well-baby clinics, the implementation of housing codes, the construction of outdoor playgrounds, child labor laws and industrial safety legislation, and some of the first studies of urban African Americans, such as W. E. B. Du Bois’s *The Philadelphia Negro*. Many New Deal leaders had worked in settlements, including Edith and Grace Abbott (helped draft the Social Security Act); Julia Lathrop, former director of the U.S. Children’s Bureau; and Frances Perkins, Secretary of Labor under Franklin Roosevelt and the first woman appointed to a cabinet post.⁴⁹

African American Associations

If conditions faced by immigrants were difficult, those faced by African Americans were even more so. In the absence of government programs, African Americans had to rely on welfare from the few voluntary agencies that would serve them. In response, a number of fraternal associations began in the African American community, such as the Knights of Tabor, the Knights of Pythias, the Ancient Sons of Israel, and the Grand United Order of True Reformers.⁵⁰ An example of mutual aid is reflected in the constitution of one health and burial society:

- Sec. 1. This society shall be known as the Sons and Daughters of Zion.
- Sec. 2. The object of this society shall be to care for its sick and bury its dead members and all moneys paid therein shall be expended for same. . . .
- Sec. 17. The Society shall not employ more than three doctors, who shall be elected annually, and shall purchase all medicine from one drug store.
- Sec. 18. The burial expenses shall in no case exceed \$25.00. . . .
- Rule 19. The chairman of the sick and burial committee shall summon as many male members as are necessary to dig a grave. . . . Members who fail to assist in digging a grave . . . shall pay a fine of 75 cents or be suspended for six months.

Important achievements were connected with voluntary African American associations. Talladega College, the first liberal arts college serving rural African Americans, had its origin in a carpenter shop where David White, Sr., a freedman, and Leonard Johnson, “a black man who had in some way acquired the rudiments of learning,” began a school.⁵¹ Morehouse College, the alma mater of Martin Luther King, Jr., began in 1867 as a night school in Georgia’s Springfield Baptist Church.⁵² When it appeared that Booker T. Washington’s industrial education approach would not lead to full African American rights, W.E.B. Du Bois galvanized a group of reformers, among them social workers such as Ida B. Wells, Mary White Covington, and Jane Addams into the Niagara Movement. The Niagara Movement became the National Association for the Advancement of Colored People (NAACP). Meanwhile, a concern for economic justice led George Edmund Haynes, a

Columbia University graduate student, to write *The Negro at Work in New York City*. The attention attracted by this study contributed to the creation of the National Urban League on Urban Conditions (Urban League) in 1911. Using philanthropy from private foundations, the League started a program for social work training that “made possible the education of many of America’s most distinguished social work leaders in the next generation.”⁵³

The Social Casework Agency

The COSs focus on individuals and families led to their transformation into social casework agencies. Like settlement houses, COSs and social casework agencies relied on a variety of private donations, the Community Chest (forerunner of the United Way), and foundations.⁵⁴ COS workers were initially female volunteers, but as their investigative techniques grew more refined, they began to focus on “social diagnosis” as the basis for case intervention.⁵⁵ The focus on social diagnosis led to a focus on treatment and the birth of the social work profession.

Despite the efforts of early female social workers, their professional status was neither highly esteemed nor recognized. The caricature of social workers by journalist H. L. Mencken may have been typical of public sentiment:

The social worker, judging by her own pretensions, helps to preserve multitudes of persons who would perish if left to themselves. Thus her work is clearly dysgenic and anti-social. For every victim of sheer misfortune that she restores to self-sustaining and social usefulness, she must keep alive scores of misfits and incompetents who can never, for all her help, pull their weight in the boat. Such persons can do nothing more valuable than dying.⁵⁶

While COSs had provided meaningful activity for upper- and middle-class women, the new social casework agency found it necessary to ground their work in treatment techniques derived from science. The necessity of this was driven home in 1915 when during the National Conference of Charities and Correction, Abraham Flexner, a renowned authority on professional medical education, was asked whether social work was a profession. Much to the dismay of the audience, Flexner argued that social work lacked the requirements of a profession, particularly a scientifically derived and transmittable knowledge base.⁵⁷

The future of social casework depended upon it developing a scientific orientation, and social workers looked to the scientific grounding of other disciplines, such as psychology and psychiatry.⁵⁸ While a scientific basis for social work would improve the effectiveness of social work practice, thereby elevating the status of the new profession, it would also redefine the client from a “hopeless wretch” into someone with a specific and treatable affliction.

Funding for social service organizations was undependable due to the competition among agencies for donors’ funds and because these gifts depended on a largesse that fluctuated with the economy. The solution was a collective approach to philanthropic giving. As early as 1887, Denver’s Associated Charities pioneered the concept whereby a group of agencies appealed to the community for operating funds. By the 1920s, more than 200 cities had adopted community chests, thus reducing the need for individual social agencies to curry favors on their own.⁵⁹ While the collective approach to agency funding provided a relatively steady income, it also demanded a uniform and an efficient operation, a guiding principle of the Progressive Era.⁶⁰ This resulted in the centralization and reorganization of the core services of a previously haphazard array of social services.⁶¹

Sixteen executives and board members from six national organizations came together in 1923 to draft a document endorsing agency-based service delivery. The Milford Conference report provided an outline of the organizational structure by which professional caseworkers would deliver services. Social casework agencies were to be housed in a community and expected to derive their objectives and purpose from that community. These agencies were to be governed by a board of directors that hired the agency director and met monthly to monitor agency affairs. Caseworkers would be regularly supervised to ensure accountability. Agency workers would have a repertoire of more than 25 methods, including diagnosis, interviewing, prognosis, planning, treatment, and reeducation. Some or all of these methods were to be employed in counteracting a host of deviations from normal social life, such as alcoholism, delinquency, family problems, mental health problems, pauperism, and vagrancy. The social casework agency would be involved in child welfare, family welfare, medical social work, psychiatric social work, and probation. Agencies would execute these charges within administrative procedures “in accordance with accepted

business standards and practice, including audits of accounts at least annually by an accredited public accountant.”⁶²

This Milford Conference framework served as a model for human service delivery for decades. Child guidance clinics, probation departments, mental health clinics, family planning clinics, and public welfare offices resembled the casework agency envisaged by the Milford Conference. This framework provided a service delivery model through which the emerging profession of social work could apply its skills.

The Progressive Movement

The Progressive Era lasted from the early 1900s to World War I. Reflected in President Teddy Roosevelt’s administration, progressive era philosophy tried to inject a measure of public credibility and morality into America’s social, political, and economic affairs. It contained a blend of social reform that was skeptical of big business, held a belief that government was responsible for the public good, strongly emphasized ethics in business and personal life, was committed to social justice, and was concerned for the “common man.” Progressives believed that government was responsible for protecting the interests of the public, especially those who were vulnerable. This philosophy was supported by some of the nation’s most notable social workers at the time, including Jane Addams, Lillian Wald, and Paul U. Kellogg.

Important reforms were enacted during the Progressive period. For instance, Teddy Roosevelt made great strides in natural resource conservation and civil service reform, and strengthened the power of the Interstate Commerce Commission. President Woodrow Wilson’s administration enacted reforms in tariffs and banking, and curbed monopolistic practices through the Clayton Antitrust Act of 1914. Other reforms in Wilson’s administration included better credit facilities, agricultural education for farmers, better working conditions in the Merchant Marine, worker’s compensation for all federal civil service employees, the eight-hour day for workers on interstate railroads, excluding products made by child labor from interstate commerce, and federal aid to states for highway construction.⁶³

Progressive reformers also had some success in protecting the rights of working women. By 1912, 11 states had legislated a minimum floor

for women’s wages. Massachusetts passed a bill in 1919 that limited the number of hours for most working women to 44 hours a week. New York followed suit in 1919 with a 54-hour week for certain women workers.⁶⁴ Some states enacted laws that prohibited mandatory night work for women in certain industries, and restricted the maximum weight a woman could be required to handle.⁶⁵

The disillusionment following World War I diminished the liberal fervor of the Progressive Era. By the 1920s, progressive ideas were treated with skepticism and liberals were frequently accused of being Bolsheviks. The situation grew worse in 1924 as Congress curtailed immigration, the anti-child labor amendment was all but defeated, political repression became commonplace, foreign radicals were deported based on raids conducted by Attorney General A. Mitchell Palmer, the Ku Klux Klan was growing, labor unrest exploded everywhere, and corruption in high places was rampant.⁶⁶ An exaggerated patriotism and intolerance was normative as settlement houses lost much of their sway. Apart from the suffrage movement that gave women the right to vote, the 1920s was a highly conservative period in American history.

Although the 1920s is often viewed as prosperous, more than one-third of Americans were living in poverty.⁶⁷ While some states had enacted widows’ pensions and workers’ compensation laws by the 1920s, most charity continued to occur through voluntary social service organizations.

The Great Depression and the Modern Welfare State

In 1928, Herbert Hoover ran for President against New York state governor Alfred E. Smith. Known as a militant prohibitionist and a strong humanitarian (due to his relief activities at the end of World War I), Hoover handily beat Smith, and in the process, won the support of many social workers, including Jane Addams. While Hoover’s support by social workers was based on his humanitarian efforts, it was also because of his strong prohibitionist stance. Most social workers at the same strongly supported prohibition.⁶⁸

The stock market crashed in October 1929; a year later, 6 million men and women were walking the streets looking for work. By 1932, more than 600,000 people were jobless in Chicago and 1 million in New York City. In Cleveland, 50 percent of

workers were unemployed; in Akron 60 percent and in Toledo 80 percent. Three years after the crash, more than 100,000 workers were fired in an average week.⁶⁹ By the early 1930s, the gross national product (GNP) (i.e., the total of all the goods and services produced) dropped to half of its pre-Depression level. National income dropped from \$81 billion in 1929 to \$40 billion in 1932 and manufacturing output fell to 54 percent of 1929 levels, a little less than in 1913.⁷⁰ The automobile industry was operating at only one-fifth of its 1929 capacity, while steel plants operated at only 12 percent of their capacity. The output of pig iron was the lowest since 1896. Factory wages shrank from \$12 billion in 1929 to \$7 billion in 1932. Unemployment reached a high of 24 percent, and for the first time, more people emigrated from the United States, than migrated to it. Even the birthrate dropped from 30 births per thousand in 1929 to 18 births per thousand in 1940. Twenty million people were on the relief rolls by 1932.⁷¹ Thousands of Americans aimlessly wandered the country in search of work. In 1929, the Missouri Pacific Railroad reported 13,745 migrants; by 1931 it was 186,028.⁷² The unemployed who stayed home often frequented the thousands of soup lines that sprang up across the United States.

Relief payments that totaled \$5 a week for a family in 1929 were cut to \$2.39 in New York and still less elsewhere. Dallas and Houston refused relief to all Hispanic and African American families. Detroit slashed its relief rolls by one-third. New Orleans refused all new relief applicants and St. Louis cut its relief rolls in half. Except for New York, Illinois, Pennsylvania, New Jersey, and Wisconsin, state governments did almost nothing to aid the victims of the Depression. More than 100 cities had no relief appropriations in 1932.⁷³

Having seen little of the acclaimed prosperity of the 1920s, most U.S. farmers were devastated by the Depression. American foreign trade declined from \$10 billion in 1929 to \$3 billion in 1932. Wheat fell from \$1.05 a bushel in 1929 to 39 cents in 1932; corn from 81 cents to 33 cents a bushel; cotton from 17 to 6 cents a pound; and tobacco from 19 to 10 cents a pound. Gross farm income fell from nearly \$12 billion in 1929 to \$5 billion in 1932.⁷⁴ Farmers responded with angry protests. Across the country, they banded together to prevent banks and insurance companies from foreclosing on mortgages. When sheriffs attempted to break up these actions, some farmers brandished pitchforks and hangman's nooses to make their point.

Herbert Hoover's response to the Depression was largely inaction. Instead of fiscal activism, he relied on the voluntary sector to meet the needs of the poor. Hoover believed that federal relief would weaken the social and moral fiber of society, impair the nation's credit and fiscal solvency, and delay the natural forces at work to restore the economy. Like Franklin Pierce, he argued that federal relief was an illegal violation of states' rights.⁷⁵

The voluntary sector responded to the Depression by organizing massive fund drives that proved ineffective, given the scope of the problem. Agency coffers were soon exhausted. Although a segment of social workers and most voluntary social agencies agreed with Hoover's opposition to federal aid, he was forced to propose an unemployment assistance program in 1932 in which the federal government paid 80 percent of the costs. This was too little, too late. By the time Franklin D. Roosevelt defeated Hoover in 1932, Hoovervilles (shantytowns sarcastically named in Hoover's honor) had sprung up in most large cities. Soup lines were a part of the urban landscape, financially driven suicides were commonplace, and legendary bank robbers like John Dillinger, Baby Face Nelson, and Bonnie and Clyde were transformed into folk heroes.

When FDR took office in 1933, he faced a country divided between right and left political factions, an industrial system convulsing with violent labor strikes, a class society at the breaking point, and a banking system on the verge of collapse. One of FDR's first acts was to declare a "bank holiday." While the banks were closed, he started the Federal Deposit Insurance Corporation (FDIC), a program that guaranteed the federal government would insure bank deposits (up to a certain amount) if a member bank became insolvent. FDR's response to the Depression involved a massive social experiment whose objectives were relief, recovery, and reform.⁷⁶ Despite accusations, FDR was not a socialist and the **New Deal** approach reflected that. Faced with an economic system at its breaking point, Roosevelt plunged into relief activities to save capitalism—not to end it—through reform.

FDR's first task was to alleviate suffering by providing food, shelter, and clothing for millions of unemployed people and their families. In 1933, Congress established the Federal Emergency Relief Administration (FERA), which distributed over \$5.2 billion of emergency relief to states and local communities. In 1933, he implemented the National Recovery Act (NRA) that provided a comprehensive

series of public works projects. Under the umbrella of the NRA, Congress enacted the Public Works Administration (later changed to the Works Progress Administration or WPA) to coordinate the system of public works. Workers employed in this make-work program built dams, bridges, libraries, and other public structures. In 1935, the WPA was expanded to provide more employment through an emphasis on long-range projects that had a positive value for the country. WPA projects employed white- and blue-collar workers as well as unskilled laborers. The WPA ultimately cost about \$11.3 billion, employed 3.2 million workers in 1938, and built roads, public parks, airports, schools, post offices, and other public buildings. The white-collar division of the WPA provided work opportunities through the Federal Writers' Project, the Federal Arts Project, and the Federal Theater Project.⁷⁷

FDR created the Civilian Conservation Corps (CCC) and the National Youth Administration (NYA) to address the problem of unemployed youth. The CCC employed poor youths (ages 17 to 23) to replant forests and help conserve the soil. Apart from employment, the 2.5 million young men served by the CCC received a leisure-time educational program and vocational and academic training. The NYA, on the other hand, was designed for young adults who wanted to stay at home and for needy high school and college students. Begun in 1935, the NYA provided part-time employment for more than 1.7 million young people and assisted more than 1.8 million high school and college students.

In 1933, Congress created the Tennessee Valley Authority (TVA), a technological and social experiment that brought electricity to the South, helped control flooding, reclaimed land, improved river navigation, and produced nitrates. The Farm Security Administration (FSA) aided farmers and migratory workers by raising prices of farm goods; and in 1934, Congress established the Federal Housing Administration (FHA), an agency that provided first mortgages for residential property and insurance to lenders against losses on secured and unsecured loans for repairs and improvements.⁷⁸

Congress passed the Fair Labor Standards Act (FLSA) in 1937, which established a minimum wage (25 cents an hour) and a maximum work week (44 hours, and then time and a half for overtime). The FLSA also abolished child labor for those under age 16. The passage of the National Labor Relations Act (NLRA) gave workers the right to collectively bargain, organize, and strike.

The apogee of FDR's New Deal legacy was the Social Security Act of 1935, a comprehensive bill that included:

- (1) A national old-age insurance system
- (2) Federal grants to states for maternal and child welfare services
- (3) Public assistance for dependent children (ADC)
- (4) Vocational rehabilitation for the handicapped
- (5) Medical care for handicapped children
- (6) Aid to the blind
- (7) A plan to strengthen public health services
- (8) A federal-state unemployment system.

Conspicuously absent was national health care, a plan included in virtually every other social security measure adopted by industrialized nations. Despite its flaws, the 1935 Social Security Act became the framework for the current social welfare state.

The Post-World War II Welfare State

The end of WWII ushered in a repressive political climate similar to the aftermath of WWI. Progressive ideas were branded as Communist, and the House Un-American Activities hearings, chaired by Martin Dies, frightened away all but the most intrepid social reformers. Joseph McCarthy's witch hunts further frightened reformers. Welfare programs also suffered. The anti-welfare feeling was so strong that some major newspapers published lists of welfare recipients to shame them off the rolls. Virtually no new welfare programs were proposed, and the existing ones were in jeopardy. Many political leaders balked at the need for welfare programs, given the relatively healthy economy of the 1950s.

Americans believed that poverty had been largely eradicated by the 1950s, and although small poverty pockets remained, the age of affluence had arrived. In 1960, America could proudly claim to be the richest country in the history of the world.

John Kenneth Galbraith published *The Affluent Society* in 1958, where he argued that most Americans enjoyed a comfortable life that surpassed the comforts the rich enjoyed a century earlier.⁷⁹ The notion of a poverty-free America was criticized by Michael Harrington, whose classic book, *The Other America: Poverty in the United States*, dispelled that myth. Harrington pointed out a large subculture of American poverty. Hidden from view,

40 to 50 million Americans—African Americans, Hispanics, rural poor, single mothers, the unemployed, and the disabled—led impoverished lives in America’s cities, towns, and villages.⁸⁰ In 1959, about 22 percent of the United States lived below the poverty level.⁸¹

The relative quiet of the 1950s gave way to turbulent changes in the 1960s. In 1960, John F. Kennedy (JFK) tried to implement—with modest success—his New Frontier program. Despite some drawbacks, Congress passed the Manpower Development and Training Act, a major jobs training program. Most of his other legislative efforts failed.

The civil rights movement was well underway when Lyndon Johnson took office after Kennedy’s assassination in 1963. In 1954, the Supreme Court had ruled in *Brown v. Board of Education* that segregating public schools (i.e., separate but equal) was unconstitutional. The next year, Rosa Parks refused to relinquish her bus seat to a white passenger, resulting in the Montgomery boycott led by Martin Luther King, Jr. The boycott ended when the city complied with the Supreme Court’s ruling that segregation on buses was illegal.

The year 1963 was a year of bloodshed. NAACP field secretary Medger Evers was murdered in Jackson, Mississippi; a church was bombed in Birmingham, Alabama, killing four young African American girls; and the Mississippi Freedom Summer (a voter education and registration project) led to the murder of two white students, Andrew Goodman and Michael Schwerner, and James Chaney, an African American civil rights worker. By the late 1960s, peaceful protests gave way to riots in some urban areas. There were 164 racial disturbances (and 83 deaths) reported across the country in 1967.⁸²

Faced with domestic unrest, former President Lyndon Johnson took some of JFK’s ideas and declared a “War on Poverty” (later changed to the **Great Society**) that was composed of many programs targeted at poverty eradication. Driven by the unrest in African American communities, Johnson’s programs sought to empower poor communities. Operating under the **Office of Economic Opportunity (OEO)**, numerous programs were created, including Volunteers in Service to America (VISTA), a domestic peace corps; Upward Bound, a program that encouraged poor children to attend college; a Neighborhood Youth Corps for unemployed teenagers; Operation Head Start, a program that provided preschool training for low-income children; special grants and loans to

rural families and migrant workers; a Community Action Program (CAP) designed to mobilize community resources; Legal Services Corporation; the Model Cities Program; and Job Corps, a manpower program providing job training for disadvantaged youths from 16 to 21. The Economic Development Act of 1965 provided states with grants and loans for public works and technical assistance. A key phrase in Johnson’s Great Society was “the maximum feasible participation of the poor,” a concept whereby poor people—rather than welfare bureaucrats—would control local institutions and decide their needs. The Great Society was based on the belief that job creation, education, and other incentives would end poverty. Important pieces of legislation emerged from the Johnson administration.

1. Civil Rights Act of 1964 outlawed discrimination based on race, religion, sex, or national origin. It also ended discriminatory voter registration requirements and racial segregation in schools, the workplace, and in all public accommodations.
2. Civil Rights Act of 1965 (Voting Rights Act) targeted at barriers that prevented African Americans from voting. Parts of this act were struck down by the Supreme Court in 2013.
3. The passage of Medicare and Medicaid in 1965. Medicare provided health care coverage for senior citizens, while Medicaid provided it for the poor.
4. The Older Americans Act of 1965 created a national network of Area Agencies on Aging that coordinated and subsidized a range of programs targeted at the aged.
5. The Food Stamp (now called SNAP) program.⁸³

By 1968, the Great Society programs had become unpopular with the American public. Despite huge sums spent on poverty eradication, critics claimed the programs were wasteful and ineffective. At the same time, Johnson was prosecuting a costly war in Vietnam while pursuing his social agenda. It soon became increasingly clear that despite America’s wealth, it could not fight two simultaneous wars, one in Vietnam and the other around poverty. Confusion in Washington, inexperienced personnel, delays in funding, corrupt local politicians, intransigent bureaucrats, and ineffective community leaders all contributed to the demise of many, but not all, of Johnson’s programs. Despite the criticisms of the Great Society, the number of

people living in poverty fell from 25 percent of the population in the early 1960s to 12 percent by 1969.

The Languishing Social Welfare State

Richard Nixon assumed the U.S. presidency in 1969. Although Nixon was not a supporter of Great Society programs, he was pragmatic and realized that to maintain support among blue-collar workers, he had to address the social welfare concerns. Under the banner of a “New Federalism,” he shifted money and power from the federal bureaucracy to states and local authorities. He also closed the influential OEO that developed and drove the Great Society programs.

Despite Nixon’s dislike of welfare programs that “coddled the poor,” his administration pushed through Congress several important pieces of legislation, including the Supplemental Security Income (SSI) program that brought Old Age Assistance, Aid to the Blind, and Aid to the Disabled under the federal Social Security Administration. Nixon’s administration also enlarged the scope of the Food Stamp program by making it a federal responsibility and by making participation mandatory for all states. Social Security was indexed to inflation, which helped cut the poverty rate for seniors whose benefits had been stagnant. Other initiatives involved job training, community development, child care, domestic violence shelters, rehabilitation, family planning services, occupational health and safety, child abuse, health care, and juvenile justice and delinquency.⁸⁴

Determined to address the welfare problem, Nixon unveiled the Family Assistance Plan (FAP) in 1969. FAP was based on a guaranteed annual income for all poor persons that would have eliminated Aid to Families with Dependent Children (AFDC). Under Nixon’s plan, a family of four with no income would have received \$1,600 from the federal government. States would have to top up that amount to match their AFDC benefit level. To be eligible for assistance, the able-bodied (including women with children over the age of three) would be required to work or be enrolled in a job-training program. Criticism came from all corners. Welfare advocates argued that the proposed income support was insufficient. Conservatives disliked a guaranteed annual income for those not working. Labor saw it

a threat to the minimum wage, and welfare bureaucrats and caseworkers feared losing their jobs. Always the pragmatist, Nixon abandoned the idea.⁸⁵

In some sense, FAP was reborn in the Earned Income Tax Credit (EITC), a program originally designed as a temporary tax credit for lower income workers to offset the Social Security payroll tax and high food and energy prices. In 1978, the EITC was made permanent. Republicans liked the EITC since it only applies to working individual and families.

Despite Nixon’s ambivalence toward welfare issues, federal spending priorities were reversed between 1965 and 1975. In 1965, defense expenditures accounted for 42 percent of the federal budget, while social welfare expenditures made up only 25 percent; by 1975, defense expenditures were only 25 percent of the federal budget, while social welfare rose to 43 percent. Even in Ronald Reagan’s conservative budget of 1986 (with large increases in defense spending), only 29 percent was earmarked for defense, while 41 percent was allocated to social welfare.⁸⁶

Reagan’s election in 1980 led to a further deterioration in the welfare state safety net. His conservative domestic policy agenda involved large tax cuts, cuts in financial support for social welfare programs, and returning more power to the states. Reagan’s domestic views were based on the assumption that (1) federal government expenditures for social welfare should be minimal, (2) only the “truly needy” should receive welfare, and (3) welfare should be short-term.

Benefits for public assistance beneficiaries fell by \$20 billion in the first few years of Reagan’s presidency. Uneven economic development accelerated inequality, and while some people were better off, large segments of American society faced greater economic hardship. Homelessness grew at unprecedented rates for a non-depression period. Real income continued to fall as higher paying industrial jobs were lost and replaced by relatively low-wage service jobs. The number of people living below the poverty line rose from 26 million in 1979 to almost 33 million in 1988. Despite Reagan’s statement that “Government is not a solution to our problem—Government is the problem,” the federal government grew by 60,000 employees, while the national debt topped \$1 trillion, pushing the United States from a creditor to a debtor nation.⁸⁷

The Reagan-era policies continued throughout the George H. W. Bush administration. Bush called

for “a thousand points of light” that was based on a return to voluntary agencies to address social problems. Like Reagan, Bush’s inattention to social welfare resulted in the dramatic growth of hungry Americans and increased unemployment, homelessness, and racially based incidents. Not surprisingly, AFDC rolls rose 5 percent from 1989 to 1991.

Progressive rejoiced when Bill Clinton was elected in 1992 since for the first time in 14 years a Democrat was in the White House. This enthusiasm would soon wane as Clinton’s mixed legislative record emerged.

The Clinton administration’s progressive legislative record included fully funding the HIV/AIDS-related Ryan White Care Act, strengthening the Office of AIDS research at the National Institute of Health, and placing HIV/AIDS victims under the Americans with Disabilities Act.⁸⁸ He was also successful in passing a large urban stimulus package; AmeriCorps, a national service corps initiative; and a crime bill that added 100,000 police officers, locked up third-time felons for life, allocated \$10.5 billion for new prisons, and created new death penalty crimes.⁸⁹ Progressives balked at Clinton’s endorsement of the death penalty. Clinton’s most important defeat was the administration’s failure to pass a comprehensive health care reform bill in 1993. It would take another 17 years before Obamacare was passed in 2010.

Clinton’s most enduring—albeit conservative—domestic legacy was signing into law the Personal Responsibility and Work Opportunity Act of 1996. This bill capped public assistance benefits, ended AFDC, and eliminated recipients’ entitlement to public assistance. This bill was the most dramatic change to public assistance since the passage of the Social Security Act of 1935. Despite decades-long Republican efforts to end AFDC, it took a Democrat to realize that goal.

By the end of Clinton’s first two years in office, his reelection seemed all but doomed. In a remarkable comeback, he became the first Democratic president to be reelected to a second term since FDR. However, Clinton’s coattails were not long enough and Republicans took control of both houses, making it difficult to implement any far-reaching domestic programs in his second term.

George W. Bush became the 43rd president of the United States in 2001 and served two terms. Like Reagan, Bush’s domestic agenda was predictably conservative. He strengthened faith-based service organizations through his new Office of

Faith-Based and Community Initiatives, opposed gay marriage, was anti-abortion (he signed the Partial-Birth Abortion Ban Act in 2003), supported drilling in the Arctic National Wildlife Refuge, and opposed the Kyoto Protocol to curb carbon emissions.

Bush’s two signature domestic bills were No Child Left Behind and the Medicare Prescription Drug and Modernization Act (Part D). The No Child Left Behind Act initially received bi-partisan support with Democrat Ted Kennedy acting as chief sponsor. The goal was to drive up the academic achievement of students by helping individual schools and school districts to improve teacher and principal quality. In particular, the No Child Left Behind act was supposed to close the achievement gap between high-performing and low-performing schools by measuring student performance, providing options to parents with children in low-performing schools, and allocating more federal dollars to these schools. The act was attacked by critics who argued that schools were not given adequate resources to meet the policy goals.

Bush signed the Medicare Act of 2003 that added prescription drug coverage to Medicare. Apart from helping seniors, this controversial bill aided pharmaceutical companies by having the federal government—through Medicare—subsidize prescription drugs for the elderly. To ensure pharmaceutical company profits, the bill prohibited the federal government from negotiating discounts with drug companies and prohibited the import of identical but less expensive drugs from Canada and Mexico.

Bush faced considerable challenges in eight years in office, including Hurricane Katrina that devastated New Orleans and much of the Gulf Coast and the global financial crisis. However, the signature event of his administration was September 9, 2001, when terrorists hijacked four commercial airliners and crashed two of them into the World Trade Center and the third into the Pentagon, resulting in 3,000 deaths. The 9/11 attack led to costly wars in Iraq and Afghanistan that Harvard Professor Linda Blimes estimates at between \$4 trillion and \$6 trillion.⁹⁰ Even if Bush had wanted new social programs, there would be little fiscal room left after the war costs were added to an almost \$6 trillion national debt.

Barack Obama was elected president in 2008 and then again in 2012. Although Obama’s

presidency has been falsely characterized as ineffectual by some critics, much of consequence happened in his administration. The American Recovery and Reinvestment Act (stimulus package) helped stabilize the economy after the global financial crisis of 2007 to 2008. He helped pass the Dodd-Frank Act, a significant crackdown on Wall Street power. Obama used executive power to curb greenhouse gases; stop the deportation of almost 6 million undocumented immigrants; end the ban on gay and lesbian military service; and limit private sector involvement in student loans. On the international front, Obama cut U.S. military involvement in Afghanistan and Iraq, reopened relations with Cuba after a 50-year failed embargo; and worked to reach a deal to stop Iran from acquiring nuclear weapons. While Obama was initially slow in supporting same-sex marriage, he eventually came around and directed the Justice Department to support the plaintiffs in the successful Supreme Court case.⁹¹

The most important piece of domestic legislation in the Obama presidency was passage of the Affordable Care Act (called ACA or Obamacare) in 2010. Although not a single-payer system like Canada, for the first time it established the responsibility of the federal government to provide health insurance to most Americans. It also expanded Medicaid and provided billions in insurance subsidies.⁹² Obamacare is discussed in detail in Chapter 12.

Conclusion

Although the history of the welfare state is clearly linked to the social and economic history of the United States, there are nevertheless important actors, many of them social workers, who also helped shape the direction of social welfare.

Throughout the history of American social welfare, advocates of care for vulnerable populations have been instrumental in shaping social policies. If one looks beneath the surface of policy statements, one finds a rich and often exciting account of the skirmishes fought by advocates for social justice. In some respects, social policy innovations can be looked upon as individual and collective biography written in official language. In an age of mass populations, often manipulated by private and public megastructures, it is easy to forget how powerful some individuals have been in shaping American social welfare policy. Many of these leaders are known because they achieved national prominence;

yet some of the more heroic acts to advance social justice were performed by individuals whose names are not widely recognized. Not to be forgotten in this regard is Michael Schwerner, a social worker who was murdered while working in a voter registration drive in the South during the Civil Rights movement.⁹³

Early social welfare leaders emerged during the Progressive Era, a period when educated and socially conscious men and women sought to create structures that would advance social justice in America. The settlement house gained a reputation as the locus for reform activity, leading one historian to conclude that “settlement workers during the Progressive Era were probably more committed to political action than any other group of welfare workers before or since.”⁹⁴ From this group, Jane Addams quickly surfaced as a leader of national prominence. Through her settlement home, Hull House, she not only fought for improvements in care for slum dwellers in inner-city Chicago but also for international peace. Social work for Jane Addams was social reform. Instead of focusing solely on restoration and rehabilitation, Addams claimed that there was a superior role for the profession: “It must decide whether it is to remain behind in the area of caring for the victimized,” she argued, “or whether to press ahead into the dangerous area of conflict where the struggle must be pressed to bring to pass an order of society with few victims.”⁹⁵ In that struggle, Addams served nobly, receiving an honorary degree from Yale University and serving as president of the Women’s International League for Peace and Freedom. In 1931, Jane Addams was awarded the Nobel Peace Prize, a suitable distinction for a social worker who once had herself appointed a garbage collector in order to improve sanitation in the slums around Hull House.

Settlement experiences crystallized the motivations of other reformers as well. Harry Hopkins, primary architect of the New Deal and of the social programs that comprised the Social Security Act, had resided in New York’s Christadora House Settlement. Ida Bell Wells-Barnett led the Negro Fellowship League to establish a settlement house for African Americans in Chicago; Lillian Wald, with Florence Kelley, a co-founder of the U.S. Children’s Bureau, had earlier established New York’s Henry Street Settlement, an institution that was to achieve distinction within the African American community. Under the guidance

of Mary White Ovington, a social worker, the first meetings of the NAACP were held at the Henry Street Settlement.⁹⁶

Early social welfare leaders championed causes that improved the conditions of children and immigrants, but also of African Americans. When it became apparent that Booker T. Washington's program of "industrial education" was unable to contend effectively with ubiquitous racial discrimination, social reformers Jane Addams, Ida Bell Wells-Barnett, and John Dewey joined W. E. B. Du Bois in the Niagara Movement. The early organizations spawned by the Niagara Movement were later consolidated into the National Urban League, with George Edmund Haynes, a social worker, as one of its co-directors. Haynes was the first African American to graduate from the New York School of Philanthropy in 1910.⁹⁷ During the height of the Civil Rights movement, the National Urban League, under the direction of social worker Whitney Young, Jr., collaborated in organizing the August 28, 1963, march on Washington, memorialized by Martin Luther King, Jr.'s ringing words, "I have a dream!"⁹⁸

If the New Deal bore the imprint of social workers, the Great Society was similarly marked some 30 years later. Significantly, one leader of the War on Poverty was Wilbur Cohen, a social worker who was the first employee of the Social Security Board created in 1935. Cohen was credited with some 65 innovations in social welfare policy, but his crowning achievement was the passage of the Medicare and Medicaid Acts in 1965. The Secretary of Health, Education, and Welfare during the Johnson administration, Cohen was arguably the nation's most decorated social worker, receiving 18 honorary degrees from American universities.⁹⁹ The history of social work has often been bound up with the history of the American welfare state.

DISCUSSION QUESTIONS

1. Many commentators argue that the English Poor Laws continue to form the basis of current social welfare policy in America. How are the English Poor Laws reflected in modern social welfare policy? Specifically, which current values or social welfare policies can be traced to the influence of the English Poor laws?
2. Are there any residual social welfare policy values that have persisted from colonial times to the

present? Describe specific social welfare policies or values that have their origin in colonial times.

3. What lasting legacy, if any, did the Freedmen's Bureau have for contemporary U.S. policy toward African Americans? Describe any influence that the Freedmen's Bureau had on the U.S. welfare state in terms of specific policies and programs.
4. The concept of Social Darwinism clearly had a big following in the late 1800s. Is this concept dead today or does a modified form of it continue to exercise influence? If you answer yes, describe how this idea is expressed in terms of values or specific social welfare policies or programs.
5. Religion clearly played an important role in the development of professional social work, especially in the late 1800s and early 1900s. What role, if any, does religion currently play in the creation of social welfare policy? Specifically, are religious impulses an important factor in decisions relating to social programs and policies? If you agree, describe specific policies and programs that reflect religious influences.
6. Although the Charity Organization Societies and the Settlement Houses seemed to be at opposite poles, some commentators argue that there were strong similarities between the two. Describe the similarities, in terms of both values and approaches, between the two organizations.
7. The Progressive Era was an important period in American history. Describe the major contributions of the Progressive Era, in terms of its programs and policies, for the later American welfare state.
8. FDR is often thought of as the father of the American welfare state, and most policy analysts agree that his New Deal policies formed the basis for the current welfare system. What is the most enduring legacy of the New Deal and why?
9. Most pundits across the political spectrum agree that Ronald Reagan had a major impact on the American welfare state. His impact included both concrete fiscal cuts and new ideas about welfare. What, if any, permanent legacy was left by this president? Specifically, in what ways did Reagan help shape current values and attitudes toward social programs?
10. Evaluate Bill Clinton's contribution to the American welfare state? What are the successes and the failures of the first Clinton term?
11. The history of American social welfare is marked by the formidable accomplishments of key leaders in social work. These include Jane Addams, Lillian Wald, the Abbott sisters, and Wilbur Cohen, among many others. In effect, these leaders dominated the field of social welfare for more than a half-century. Some critics point out that there are no leaders on the social work scene today who have the stature of those mentioned. What do you believe are the reasons for this predicament?

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Social Welfare Policy Research

A Framework for Policy Analysis



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Policy analysts engage in the systematic investigation of a social policy or a set of policies. They are employed in a variety of settings, including federal, state, and local governments; think tanks; universities; social justice or public interest groups or community organizations; and larger social agencies. The goals of policy analysis can range from pure research to providing information to legislators (i.e., congressional researchers) and to advocacy research. This chapter examines one of the major tools used by the policy researcher, a systematic and structured framework for policy analysis. The authors also propose a model for policy analysis.

A **policy framework**—a systematic model for examining a specific social welfare policy or a set of policies—is one means analysts use to evaluate the congruence of a policy with the mission and goals of the welfare state. Analysts can also employ policy frameworks to assess whether key social welfare values—that is, social justice, redistribution, or equity—are incorporated within a given policy. Moreover, a policy framework can help determine if a policy is consistent with established social welfare values or historical precedents. For example, consider the proposal that foster care should be abolished and all children suspected of being abused or neglected should be placed in orphanages. The use of a policy framework would illustrate that this proposal reflects a clear break with the general drift of child welfare policy since the end of the twentieth century, and that it contravenes established values such as family reunification. In addition, a systematic analysis would reveal that this policy is not feasible economically, politically, or socially.

An analytic framework can also be used for comparing existing policies. For example, comparing the mental health policies of Missouri with those of Massachusetts and Minnesota would yield valuable information for all three states. A comparative analysis of the health systems of the United States, Canada, and United Kingdom would also provide useful information for decision makers. Finally, analytic frameworks are useful in evaluating competing policies and helping analysts make a recommendation as to which policy most effectively addresses a problem or need.

Policy analysis frameworks are useful for social work practitioners on several levels and can address micro and macro policies at the agency, statewide, or national levels. Agency policies dictate what a

social worker will do, with whom, and for how long. They also define who is (or is not) a client and what services will be offered. Social work practitioners can analyze agency policies on issues like flextime, merit pay, agency-based day care, job sharing, and so forth. Child welfare workers can use policy analysis to evaluate impending state or federal legislation and the concomitant fiscal allocations. Social workers in health care can analyze managed care policies in terms of equity, effectiveness, and a wide range of other issues. Social work practice is clearly influenced—if not driven—by social policy.

Policy analysis is also useful in the environmental scanning activities of non- and for-profit agencies. As more social services are delivered in the competitive marketplace, nonprofit social agencies are forced to replicate private sector corporate behavior. This includes being aware of changing demographics and market trends, and monitoring new legislation. In some sectors, events change so rapidly that agencies must continually modify their operating procedures to remain viable. Data-based environmental scanning allows agencies to make long- and short-term plans based on changing demographics, new competing organizations, or the effects of impending legislation. Environmental scanning also allows agencies to discover new markets and to protect existing ones.¹

Social welfare policies and programs are complex phenomena. For example, it is easy to propose a social policy such as mandatory drug testing for all governmental employees. On the surface, the policy appears simple: Illegal drug users are discovered through testing and then are terminated or forced to seek treatment. On closer scrutiny, the hidden issues become problematic. In states where marijuana is legal can employers fire workers or force them into treatment if they test positive for cannabis use? Is it constitutional to require drug treatment if a positive result is found? Is occasional marijuana use sufficient grounds for requiring drug treatment? Should an employer be permitted to refuse employment to an applicant that tests positive for marijuana in a state where it is legal? Since alcohol is a legal substance, drug tests do not detect it. Is alcohol less debilitating than marijuana? Can mandatory drug testing be misused by supervisors to harass employees? Without a systematic means to analyze the effects of a policy, decisions become arbitrary and may produce consequences that are worse than the original problem.



The policy analyst is expected to evaluate policies and make recommendations. To succeed, the analyst must accept his or her own values while basing the analysis on objective criteria.

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All well-designed policy frameworks are characterized by eight key elements:

- Policy frameworks systematically analyze a social policy or program.
- Policy frameworks reflect the understanding that social policy is context sensitive, and there are competing priorities in all policy options.
- Policy frameworks employ rational methods of inquiry and analysis. The data used in policy analysis is derived from scientific inquiry and legitimate sources, and it is objectively interpreted and analyzed.
- The analytic method is explicit and all succeeding policy analyses should approximate the same conclusion.
- Policy analysis is based on the commitment to derive the largest possible social and

economic benefit at the least possible social and economic cost. A good social policy is one that benefits at least one person (as he or she perceives his or her own self-interest) while at the same time hurting no one (a variation of the Pareto Optimality). Although rarely achieved in the real world of finite resources—and proliferating claims on them—policy analysts should nevertheless strive toward that optimality.

- Policy frameworks should attempt to take into account the unintended consequences of a particular policy or program. This could be aided by constructing multiple policy models or scenarios.
- Policy analysts should consider alternative social policies or alternative uses of present or future resources allocated to a given policy.
- Policy frameworks should employ models that examine the potential impact of a policy (or a series of policies) on other social policies, social problems, and the public good.

A policy framework can provide decision makers and the general public with information, an understanding of the ramifications of a policy on the target problem (and other problems and policies), and alternative policies that could more effectively address the problem. Untoward consequences occur more frequently when the decision-making process is not based on an analytic policy framework.

History is replete with examples of well-intentioned policies that proved catastrophic. For example, the U.S. Congress enacted a law in 1919 that prohibited the manufacture, sale, or transport of alcohol in an effort to cut down on crime, familial instability, unemployment, and sundry other alcohol-related social problems. Advocates of Prohibition, including many social workers, touted the end of alcohol as a major step in the social evolution of the United States. By the time Prohibition was repealed in 1932, most of its original supporters were silent. Despite their best hopes, Prohibition did not decrease crime or lead to more family stability and greater social order; instead, it led to the growth of an organized crime industry that fed the ongoing taste of Americans for alcohol. Instead of curbing alcohol-related nightlife, Prohibition was responsible for the creation of illegal and well-attended speakeasies. Many supporters conceded that alcohol was almost as abundant as before Prohibition. A systematic analysis of Prohibition before

the passage of the bill might have demonstrated the futility of this policy.

A similar argument can be made for the current policy of drug interdiction and enforcement. After almost a half-century of vigorous drug enforcement, marijuana, heroin, cocaine, and other drugs continue to be readily available. While there is little hard evidence that vigilant drug enforcement has resulted in lowered drug use, it has increased the prison population. In 1980, about 220 people were incarcerated for every 100,000 Americans; by 2010, that number had more than tripled to 731 or roughly 2.27 million people.² This represents the highest incarceration rate in the world, both in actual numbers and proportionate to the population. While United States represents only about 5 percent of the world's population, it has 25 percent of the world's inmates.³ Roughly 25 percent of inmates are in U.S. prisons for drug-related crimes.⁴ Since 1985, the money that states spend on prisons has risen six times faster than their rate of spending on higher education.⁵ The drug interdiction policy has also led to the creation of international drug cartels whose wealth and power rival many national governments and multinational corporations. From January to September 2011, the Mexican government reported that almost 13,000 people were killed in drug-related violence, including security forces and innocent bystanders. Roughly 60,000 Mexicans were killed in drug-related violence from 2006 to early 2012.⁶

The drug policy has also led to an increase in drug-related homicides and to the deterioration of inner-city neighborhoods racked by gang warfare that is driven by the lucrative drug trade. Drug enforcement policies have not stopped the flow of illegal drugs, but they have created a vibrant industry that includes government officials, contractors, private correctional corporations, police

departments, and large segments of the legal and judicial system. All of these groups have a vested interest in maintaining the status quo around drug policy, regardless of its effectiveness. In short, a social problem led to a social policy, which in turn, led to a powerful industry that depends on the continuation of this policy for its very existence.

As this example suggests, social policy is often driven by politics and rarely, if ever, systematically analyzed. Even a cursory examination of the impact of Prohibition would have informed policy makers of the potential pitfalls of a drug interdiction policy. Since policy analysis often occurs only after a bill or policy is enacted, analysts are asked to perform an autopsy to determine why a specific bill or policy failed.

The purpose of a policy framework is to provide the analyst with a model or a set of questions for systematically analyzing a policy. Consequently, the choice of a policy framework must fit the requirements of the project as well as the available resources. Every policy framework can be fine-tuned or modified, and the best policy framework is often a synthesis of existing models.⁷

A Proposed Model for Policy Analysis

Policy analysts are expected to evaluate a policy and make recommendations. As part of the process, the analyst must acknowledge their own values, while at the same time, basing the analysis on objective criteria. The proposed policy framework is divided into four sections: (1) the historical background of the policy, (2) a description of the problem(s) that necessitated the policy, (3) a description of the policy, and (4) the policy analysis (see Figure 3.1).

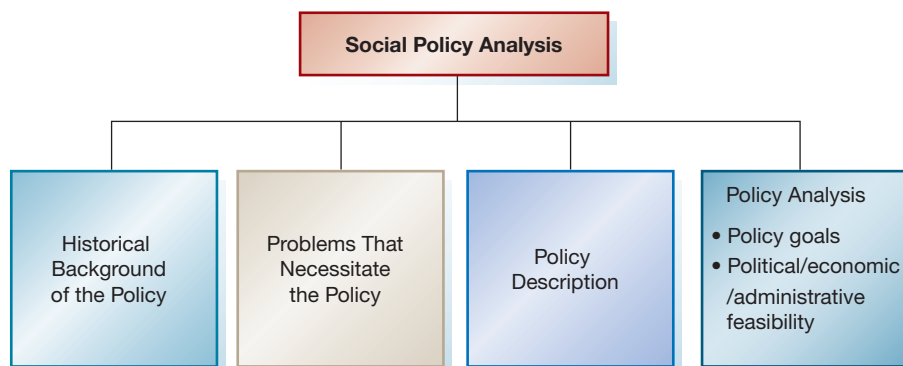


FIGURE 3.1 Proposed Framework for Policy Analysis

Historical Background of the Policy

The policy analyst should examine similar past policies and assess how they fared. First, the analyst should identify the historical problems that led to the original policy. Questions might include: What

historical problems led to the creation of the policy? In what way was the problem handled before? When did the policy originate and how has it changed over time? What is the legislative history of the policy?

Spotlight 3.1

A MODEL FOR POLICY ANALYSIS

The model for policy analysis can be divided into four sections—the historical background of the policy, a description of the problem that necessitated the policy, a description of the policy, and the policy analysis. Each of these sections requires the policy analyst to ask specific questions.

Historical Background of the Policy

- What historical problems led to the creation of the policy?
- How important have these problems been historically?
- How was the problem handled before?
- What is the historical background of the policy?
- When did the policy originate?
- How has the original policy changed over time?
- What is the legislative history of the policy?

Description of the Problem That Necessitated the Policy

- What is the nature of the problem?
- How widespread is it?
- How many people are affected by it?
- Who is affected and how?
- What are the causes of the problem?

Policy Description

- How is the policy expected to work?
- What are the resources or opportunities that the policy is expected to provide?
- Who will be covered by the policy and how?
- How will the policy be implemented?

- What are the short- and long-term goals and projected outcomes of the policy?
- What are the administrative auspices under which the policy will be lodged?
- What is the funding mechanism for the policy?
- What agencies or organizations will be charged with overseeing, evaluating, and coordinating the policy?
- What are the formal or informal criteria that will be used to determine the effectiveness of the policy?
- How long is the policy expected to be in existence?
- What is the knowledge base or scientific grounding for the policy?

Policy Analysis

- Are the goals of the policy legal?
- Are the goals of the policy just and democratic?
- Do the goals of the policy contribute to greater social equality?
- Do the goals of the policy help the redistribution of income, resources, rights, entitlements, rewards, opportunities, and status?
- Do the goals of the policy contribute to a better quality of life for the target population?
- Will the goals adversely affect the quality of life of the target population?
- Does the policy contribute to positive social relations between the target population and the overall society?
- Are the goals of the policy consistent with the values of professional social work?

A historical analysis helps to curb the tendency of decision makers to reinvent the wheel. For instance, an analyst may find that circumstances have changed, and a previously unsuccessful policy may now be viable. A historical analysis also helps the analyst to understand the forces that were mobilized to support or oppose a given policy. This part of the analysis locates a policy within a historical framework, thus helping to explain its evolution.

Problems That Necessitate the Policy

The second step in policy analysis is an examination of the problems that led to the creation of the policy. To fully assess whether a proposed policy will successfully remedy a social problem, the analyst must understand the parameters of the problem. Furthermore, the analyst must be familiar with the nature, scope, and magnitude of the problem and with the affected populations. In this way, the analyst can discern early whether the policy is appropriate. Specific questions might include the following: What is the nature and cause of the problem? How widespread is it? Who and how many people are affected by it?

Policy Description

The next step is the description of the policy. This section requires a detailed explanation of the policy, including a description of (1) how the policy is expected to work; (2) the resources or opportunities the policy is expected to provide (power, cash, economic opportunity, in-kind benefits, status redistribution, goods and services, and so forth); (3) who will be covered by the policy and how (e.g., universal versus selective entitlement, means testing); (4) how the policy will be implemented; (5) the expected short- and long-term goals and outcomes of the policy; (6) the administrative auspices under which the policy will be lodged; (7) the funding mechanism, including short- and long-term funding commitments; (8) the agencies or organizations that have overall responsibility for overseeing, evaluating, and coordinating the policy; (9) the criteria used to determine the effectiveness of the policy; (10) the length of time the policy is expected to be in existence (i.e., is it a “sunset law” designed to end at a certain date?); and (11) the knowledge base or science undergirding the policy.

Policy Analysis

This section is the heart of any policy analysis. It is where the analyst engages in a *systematic* analysis of the policy.

Policy Goals The policy goals are the criteria by which all else is measured. Often the analyst must interpret the overall goals of the policy since they may not be explicitly stated. The following questions may help to explicate the goals of a particular policy:

- Are the goals legal, just, and democratic?
- Do they contribute to greater social equality?
- Do the goals positively affect the redistribution of income, resources, rights, entitlements, rewards, opportunities, and status?
- Do the goals contribute to a better quality of life for the target population? Will they adversely affect the quality of life of the target group?
- Does the policy contribute to positive social relations between the target population and the overall society?
- Are the policy goals consistent with the values of professional social work (i.e., self-determination, client rights, self-realization)?

Analysts must understand the value premises of the policy as well as the ideological assumptions underlying it. To this end, several questions should be asked: What explicit and covert ideological assumptions are contained in the policy? How is the target population viewed? Does the policy encourage the status quo or does it represent a radical departure? Who are the major beneficiaries? Is the policy designed to foster real social change or to placate a potentially insurgent group? Does the policy contain a vision of society? Unearthing the hidden ideological dimensions of a policy is often the most difficult task for the policy analyst.

Despite good intentions, a prospective policy must be viable. American history is littered with worthy policy proposals that were not deemed viable at the time. For example, at the height of the Great Depression, a California physician named Francis Townsend proposed that all citizens over the age of 65 receive a flat governmental pension of \$200 a month. While this proposal might have received at least a cursory hearing in prosperous times, in the midst of the Great Depression policymakers dismissed it as nonviable. The overall

viability of a policy is based on three factors: political feasibility, economic feasibility, and administrative feasibility.

Political Feasibility The political feasibility of a policy is based on a subjective assessment. To fully evaluate a policy, the analyst must assess which group(s) will oppose or support it. They must also be able to assess the power and constituency base of each group. However, the size of the constituency base and its relative power are sometimes unrelated. For example, despite its relatively small numbers, the National Rifle Association (NRA) is a powerful lobby. Conversely, although more than 30 million Americans are poor, they have negligible political clout.

The political viability of a policy is subject to the public's perception of its feasibility. In other words, for a policy to be feasible, it must be perceived as being feasible. For example, although much of the public would welcome some form of gun control, it does not exist partly because of the power of the NRA and the gun lobby, but also partly because the public believes it cannot happen. A public belief is therefore born about what is and what is not possible.

In assessing the political feasibility of a policy, the analyst must examine the public sentiment. Does a large segment of the public care about the policy (i.e., do they believe they will be affected by it)? Do large numbers of people fear a policy, as in European-style universal health care? Does the policy threaten fundamental social values? Is the policy congruent with the current social and political climate? Can either side marshal substantial public support for or against the policy? The answers to these questions will help the policy analyst determine the political feasibility of the policy.

The analyst should consider the relationship between the policy and external factors in agencies and institutions. For example, which social welfare agencies, institutions, or organizations will support or oppose the policy? What is their relative strength? How strident is their support or opposition to the policy? Which federal, state, or local agencies would be affected by the policy? The world of social policy is highly political, and some governmental and private social welfare agencies have political power on par with elected officials. Groups of institutions often coalesce around policies that directly affect them, and through lobbying

efforts, they can defeat legislation. In cases where they cannot defeat a policy outright, administrative units can implement a policy in ways that ensure its failure. The analyst must therefore take into account whether key administrative units support or oppose a new or existing policy.

Economic Feasibility Most social policies require some form of direct or indirect funding. In assessing the economic feasibility of a policy, the analyst must question the minimum level of funding necessary for successfully implementing the policy. Is the policy adequately funded? If not, what is the public sentiment toward increased funding? What are the funding needs of the policy likely to be in the future?

Given the current political and economic climate, it appears unlikely that bold new social policy initiatives requiring large revenues will be successful. More likely, new revenue-based policy legislation will be based on the reallocation of existing resources (budget-neutral policies) rather than on new revenue sources. This approach—taking money from one program to fund another—is called **pay-go** financing. The danger is that thinning out fiscal resources among many programs may mean that none are adequately funded. The analyst must therefore decide whether a new policy initiative should be recommended based on the funding prospects. The parameters of this decision are complex. If a new policy is recommended despite insufficient resources, the chances of its failure are greater. If a policy is not recommended, there is no possibility that future fiscal resources will be allocated to it. As a result, many policy analysts lean toward incremental approaches, tending to recommend policies in the hope that future funding will become available.

Administrative Feasibility The analyst must also be concerned with the administrative viability of a policy. Regardless of the potential value of a policy, responsible administrative and supervisory agencies must possess the personnel, resources, skills, and expertise needed to effectively implement the policy. In addition, directors and supervisors must be sympathetic to the goals of the policy, have the expertise and skill needed to implement or oversee the policy, and have an understanding of the fundamental objectives of the policy. Policy analysts focus on two key aspects of administrative feasibility: effectiveness and efficiency.

Effectiveness is based on the likelihood that the policy will accomplish what its designers intended. This encompasses several questions: Is the policy broad enough to accomplish its stated goals? Will the benefits of the policy reach the target group? Is the implementation of the policy likely to cause new or other social problems? What are the ramifications of the policy on the non-target sector (i.e., higher taxes, reduced opportunities, diminished freedom)?

Oftentimes it is difficult, if not impossible, to gauge the effectiveness of a policy. In 1994, the U.S. Congress passed the Violent Crime Control and Law Enforcement Act, making it unlawful to transfer or possess a large capacity ammunition magazine not lawfully owned before the law's enactment. The law also banned semi-automatic assault weapons. The law "sunsetted" in 2004 and despite public support, it was not renewed. Proponents argued that banning high-capacity magazines and assault weapons would lessen mass shootings. However, 2012 data found that while the ban was in place from 1994 to 2004, the number of mass shootings actually increased slightly. Moreover, the Columbine shootings occurred in the middle of the ban, thus making it difficult to say with certainty that the bill achieved its ends.⁸

Another challenge facing policy analysts is the nature and extent of a policy's unintended consequences. Virtually all policies have unforeseen consequences. One example is methadone, a drug legally administered to addicts as a heroin substitute. When introduced in the 1960s, methadone was thought to be a safe way to wean addicts from heroin. However, by the mid-1970s, health experts realized that methadone was almost as addictive as heroin and that some addicts were selling their methadone as a street drug. Despite this unintended outcome, some addicts were able to withdraw from heroin, and on balance, the methadone program was a positive development. Because policy analysts cannot see into the future, recommendations are made on the basis of available data. Nevertheless, analysts must try to anticipate the possibility of adverse future consequences.

Policy analysts look closely at *efficiency*, or the cost-effectiveness of the proposed policy compared to alternative policies, no policy, or the present policy. Social policy always involves trade-offs. Even in the best of economic times, fiscal resources are always inadequate to meet the breadth of human need. For example, virtually everyone could benefit

from some form of social welfare, whether it be counseling, food stamps, or free health care. But because resources are finite, society must choose the primary beneficiaries for social allocations. Publicly financed services are often provided on the basis of two criteria: (1) the severity of the problem (those who most need it) and (2) means (those least able to afford it). As a result, adequately funding one policy can mean limiting the allocations of another. This is the essential trade-off in social welfare policy. When analysts evaluate a policy, they are cognizant that promoting one policy often means that needs in other areas may go unmet. Hence, the analyst asks, "Is the policy important enough to justify the allocation of scarce resources?" Are there other areas where scarce resources could be better used?

The policy analyst frequently compares the cost-effectiveness of a proposed policy with an alternative policy. Will the proposed policy yield better results than the present policy or no policy at all? Is it advantageous to enlarge or modify the present policy rather than create a new one? Will an alternative policy provide better results at a lower cost? Can an alternative policy achieve the same results? These questions are often addressed in a policy analysis.

Researching and Analyzing a Social Policy Assignment

Students tasked with a policy analysis assignment will face two major hurdles. The first is finding and focusing on a manageable social policy, and the second is finding or generating information relevant to it.

One of the most difficult tasks the student faces is choosing the policy. A manageable assignment requires selecting a policy that is both specific and limited. For example, it is virtually impossible to do a complete and exhaustive analysis of U.S. child welfare policy. For one thing, the United States lacks an integrated child welfare policy; instead, child welfare is characterized by a patchwork quilt of myriad programs. Second, differing child welfare policies on the national, state, and community levels would make such a policy analysis even more daunting. To succeed, the student would have to focus on a small segment of child welfare.

A second task involves locating policy-relevant information. There are seven major avenues for finding information. First, policy analysts may

choose to generate their own data through primary research, including surveys, opinion polls, experimental research, longitudinal studies, and so forth. Although this method can yield a rich body of information, time and cost constraints may prove an impossible obstacle. Moreover, the same research may already exist in other places, in which case the replication of the effort would be unwarranted.

Second, governmental or agency records are often useful sources for relevant data. These can include archival material, memos, meeting minutes, policy manuals, and departmental records. A large amount of information is available on government or agency websites.

A third avenue for policy research is the records and published minutes of legislative bodies and committees. On the federal level, these sources include the *Congressional Record* and the minutes of the various House and Senate committees and subcommittees. All state legislatures have similar record-keeping procedures, and most of these records are available in regional or university libraries or online.

A fourth source of information is governmental publications. For example, the U.S. Government Printing Office maintains catalogues of all published government documents. Other documents include the Census Bureau's population studies; publications of the Departments of Labor, Commerce, Housing, and Health and Human Services. The Green Book published by the Committee on Ways and Means of the House of Representatives provides background material and data on most social welfare programs. It is a valuable online resource.

A fifth source of policy-relevant information includes think tanks, advocacy organizations, and professional associations. All think tanks employ research staff who evaluate and analyze social policies. Examples of these include the Brookings Institution, the American Enterprise Institute, the Heritage Foundation, the Hoover Institute, the Urban Institute, the Center on Budget and Policy Priorities, the Reason Foundation, the Hudson Institute, the Progressive Policy Institute, and the Economic Policy Institute. Again, much of this information can be accessed through the Internet. Since most think tanks are affiliated with a particular political or economic ideology, their policy recommendations should be viewed through a critical lens.

Many national advocacy organizations retain research staff and publish helpful reports for the

policy analyst. Some of these organizations include the Urban League, the National Association for the Advancement of Colored People (NAACP), the Children's Defense Fund, the National Farm Organization, and the National Organization for Women. Many professional associations, such as the American Medical Association, the American Public Welfare Association, the **National Association of Social Workers (NASW)**, and the American Psychological Association, also publish policy-relevant information.

A sixth data source is professional journals, books, and monographs. Articles or books on specific policy topics can be found in various venues, including the *Social Science Index*, Google Scholar, and the subject headings in library catalogues. Commercial electronic databases such as Lexis/Nexis and Westlaw are also useful. Last, policy-relevant information can be derived from interviews with principals in the policy process, advocates, recipients of services, and government officials. Personal interviews may also help gauge public opinion and assess the opposition or support for a particular policy. Taken together, these sources can be a treasure trove for the student and the policy analyst.

Social Policy Research and the Internet

The Internet has made the job of the policy analyst immeasurably easier, more efficient, and even more fun. Policy analysts no longer have to wait months before their library receives the report they are seeking; no longer do they have to contend with lost, misplaced, or checked-out documents, or wade through library stacks filled with linear feet of government reports. Instead, analysts can now directly download the newest government data, legislative reports, and government legislation. The researcher is helped by major university libraries, which are making the full texts of articles available online. Plus, virtually all major newspapers and news services are now online. The ease of finding materials online also enhances the temptation of plagiarism.

Recruiting subjects and collecting data online for research requiring quantitative data collection has several advantages:

- Posting messages on e-mail lists or other online media (e.g., social network sites) to recruit and interview subjects is more efficient and less costly than traditional forms of data collection,

such as mailed questionnaires or paying field researchers.

- Data obtained online does not require transcription, which lowers research costs.
- Subjects living anywhere in the world can respond to questionnaires at their convenience.
- Principal investigators living in various parts of the country or the world can quickly and conveniently communicate with each other on issues pertaining to the research.⁹
- Survey Monkey and other online web-based programs make surveys easier to manage compared to traditional mail surveys.
- Qualitative research can even be done online using Skype, Face Time, Tango, Viber, or other video conferencing programs.

There are also disadvantages to online data collection. First, there is a danger that convenience and cost will drive sampling methods, resulting in a biased sample. Second, the pool of people available to the researcher is limited to those who possess (or have access to) and understand the technology.

Conclusion

The choice of a policy analysis framework depends on several factors, including (1) the kind of problem or policy that is being analyzed; (2) the resources available to the analyst, such as time, money, staff, facilities, and the availability of data; (3) the needs of the decision maker requesting the analysis; and (4) the time frame in which the analysis must be completed.

Because it is impossible to evaluate all of the data (data are essentially infinite) and to ask all of the possible questions, no policy analysis is ever complete. Policy analysis is always an approximation of the ideal, and therefore decisions are inevitably made on the basis of incomplete information. How incomplete the information and how close an approximation to a rational decision is provided to decision makers will depend on the skills of the analyst, the available resources, and the time allotted for the project.

Despite its reliance on an analytic framework, social policy analysis is largely subjective. Because a policy is analyzed by human beings, it is always done through the lens of the analyst's value system, ideological beliefs, and their particular understanding of the goals and purposes of the project. Subjectivity may be evident in the omission of facts or questions or in the relative weight given to one variable over another. Other forms of subjectivity

include asking the wrong questions about the policy, evaluating it on the basis of expectations that it cannot meet, or expecting it to tackle a problem the policy was not designed to remedy. Finally, political pressure can be put on the policy analyst to come up with recommendations that are acceptable to a certain interest group. Policy analysis always approximates the ideal and always involves an informed hunch as to the effects of a policy on people or on a set of policies.

DISCUSSION QUESTIONS

1. What are the main advantages of using an analytic framework for social policy analysis? Describe the benefits of using such a framework. What, if any, are the potential drawbacks?
2. Although the *unintended* consequences of a social policy are obviously unpredictable, what can a policy analyst do to minimize the risk? Describe a social policy that has produced unintended consequences that were either positive or harmful.
3. Can a policy researcher neutralize his or her personal values when conducting a policy analysis? If so, describe ways in which this can be done.
4. What components should be added to the proposed policy framework presented in this chapter? Which of the components provided in this framework are the most important, and why?
5. Are most social policies analyzed rationally? If not, why not? Describe the factors that stand in the way of a systematic and rational analysis of social policy in the U.S. context. How much value do decision makers place on social policy research? Why?
6. Because any analysis of social policy is by nature incomplete, should decision makers bypass policy studies? What alternatives, if any, can be used in lieu of a thorough and systematic policy analysis?
7. The Brookings Institution (www.brookings.edu) and the Cato Institute (www.cato.org) are two influential policy research think tanks that often take different views on specific social problems. Access these websites and compare the materials provided by each think tank on a research topic of your choice. How would you describe the different frameworks through which the problem is approached? Which would you access first when looking for new information? Why?

NOTES

1. Thanks to Brene Brown for her ideas on environmental scanning. Dr. Brown has repeatedly pointed out that environmental scanning is a concept that successful agencies must adopt.

2. Adam Gopnik, "The Caging of America: Why Do We Lock Up So Many People?" *The New Yorker*, January 30, 2012, pp. 20–28.
3. International Centre for Prison Studies, World Prison Brief, 2011. Retrieved February 2012 from http://www.prisonstudies.org/info/worldbrief/wpb_country.php?country=190.
4. Drug War Facts, Prison and Prison Offenders, 2011. Retrieved February 2012 from http://www.drugwarfacts.org/cms/Prisons_and_Drugs#Federal-State.
5. Gopnik, "The Caging of America," pp. 20–28.
6. BBC News, "Q&A: Mexico's Drug-Related Violence," January 25, 2012. Retrieved February 2012 from <http://www.bbc.co.uk/news/world-latin-america-10681249>.
7. Many social policy writers have developed excellent policy frameworks, among them Elizabeth Huttman, *Introduction to Social Policy* (New York: McGraw-Hill, 1981); Neil Gilbert and Harry Specht, *Dimensions of Social Welfare Policy*, 2nd ed. (Englewood Cliffs, NJ: Prentice Hall, 1986); Gail Marker, "Guidelines for Analysis of a Social Welfare Program," in John E. Tropman et al. (eds.), *Strategic Perspectives on Social Policy* (New York: Pergamon Press, 1976); David Gil, *Unraveling Social Policy* (Boston: Shenkman, 1981); and Charles Prigmore and Charles Atherton, *Social Welfare Policy* (New York: D.C. Heath, 1979).
8. Fox News, "Assault-Weapons Ban No Guarantee Mass Shootings Would Decrease, Data Shows," Foxnews.com, December 24. Retrieved August 2015 from <http://www.foxnews.com/politics/2012/12/24/history-gives-mixed-grade-to-impact-assault-weapons-ban/>.
9. Richard Lakeman, "Using the Internet for Data Collection in Nursing Research," *Computers in Nursing* 15, no. 5 (1997), pp. 269–275.

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Discrimination in American Society



Source: Bettmann/Getty Images

Discrimination occurs in virtually all countries and spans the spectrum from moderate to severe. Forms of discrimination include the following: (1) religious; (2) gender; (3) sexual orientation; (4) national origin, culture, or tribal; (5) racial (including “colorism” based on the skin tones); (6) class and caste; (7) political; and (8) personal characteristics such as disability, age, and appearance.

Discrimination and poverty are inextricably linked to the fabric of American social welfare. Economic, social, and political discrimination often leads to poverty, which in turn, results in the need for income maintenance and other social programs. Realizing that discrimination leads to poverty, some policymakers have tried to address this cycle by attacking discrimination. These policymakers hope that if discriminatory practices and attitudes are curtailed, the result will be equal opportunities for achievement and success. This chapter probes discrimination based on race, gender, sexual orientation, disability, and age.

Discrimination

The causes of discrimination in U.S. society are complex. The main theories reflected in the literature fall into three broad categories: psychological, normative-cultural, and economic.

Psychological interpretations attempt to explain discrimination in terms of intrapsychic variables.¹ For example, a theory called the frustration-aggression hypothesis, formulated by J. Dollard, maintains that discrimination is a form of aggression activated when individual needs become frustrated.² Dollard argues that when people cannot direct their aggression at the real sources of their rage, they seek a substitute target. Thus, relatively weak minority groups become an easy and safe target for the aggression and frustration of slightly stronger discontented groups. For example, poor Southern whites have been viewed as an outwardly racist group. Exploited by the rigid economic and social class system of the old South, they often focused their rage on African Americans, a group even weaker than themselves. African Americans therefore served a twin purpose for poor whites: On the one hand, they formed a lower socioeconomic group, making poor whites feel better about their own standing, and on the other, they were scapegoats for the frustrations of poor whites. Women, racial minorities, homosexuals, and other

disenfranchised groups often serve the same purpose for those on a slightly higher social rung.

The normative-cultural explanation suggests that individuals hold prejudicial attitudes because of their socialization. Through overt and covert messages, a society teaches discrimination and rewards those who conform to prevailing attitudes and behaviors. Because of strong societal pressures to conform to established norms, resistance to discriminatory practices becomes difficult.³ For example, special opprobrium in the old South was reserved for liberal whites who broke the norms governing interactions with African Americans.

Some discrimination is economically based. Economic discrimination occurs when a dominant group discriminates to maintain their economic and political advantages. For instance, male workers may discriminate against female workers because they perceive them as encroaching on their employment prospects. Employers may promote discriminatory attitudes because as long as women workers are stigmatized, they will remain a cheap labor pool. By pitting stigmatized groups (women, racial, and ethnic minorities) against each other, employers can suppress wage demands and force wage concessions by threatening to replace relatively well-paid employees with lower-paid workers from a disenfranchised group. Simply put, discrimination partly determines who will flip the burgers and supersize the drinks.

To maintain an air of legitimacy, discrimination must have moral, social, and theological underpinnings. To that end, some have used the Bible to explain the inferiority of women, the “sin” of homosexuality, and the necessity of separating the races. For example, some people maintain that menstrual cycles cause severe mood swings that make women incapable of holding positions of power. Others believe that African Americans are descended from Ham and have committed biblical sins that justify discrimination. Some people claim that African Americans are racially inferior, based on theories supported by dodgy anthropological theories supported by dubious intelligence testing.

Social stigma and discrimination can lead to the transformation of disenfranchised groups into a lower socioeconomic class. Alternatively, as in the case of gays, lesbians, and the aged, social stigma and discrimination can result in social marginalization without triggering statistically observable economic discrimination. For example, although the individual incomes of gays and lesbians (and the assets of the elderly) are higher than the national

averages, people in these groups often experience economic discrimination in the form of restricted career choices, including forced occupational clustering, limited access to upper managerial positions, discrimination in hiring practices, and forced retirement. The following sections will examine some core components of discrimination and social stigma, including racism, sexism, homophobia, ageism, and discrimination against people with disabilities.

Racism

Racism refers to discrimination against and prejudicial treatment of a racially different minority group. This prejudicial treatment may take the form of differential hiring and firing practices and promotions, differential resource allocations in health care and education, a two-tier transportation system, segregated housing policies, discriminatory behavior of judicial and law enforcement agencies, and/or stereotypical and prejudicial media images. A pattern of racial discrimination that is strongly entrenched in a society is called *institutional racism*.

Demographic data often divides society along the lines of whites and people of color. However, clumping white Americans into a single category is misleading. For example, after the Civil War, the vast numbers of immigrants who arrived from southern and central Europe were neither Protestant nor Anglo-Saxon. In addition, although many white Americans have a Hispanic surname, they may have no Hispanic cultural memory. Those from Arabic-speaking countries may experience discrimination, even though they are classified as white.⁴ This is especially true in light of the anti-Muslim feelings resulting from the threat of ISIS and domestic terrorist acts.

Throughout America's history, whites have been in the majority. Although whites represented 69 percent of the population in the 2000 Census, by 2010 their population declined to 64 percent. By 2050, whites will make up only 50 percent of the population with minority groups expected to account for the other half. This demographic shift is mainly attributable to low birth rates among whites and high immigration rates.⁵ The Census Bureau projects that Asians and Hispanics will see the most dramatic increases, as the U.S. population grows from 319 million in 2015 to 420 million in 2050.⁶ The next several decades will demonstrate how the United States addresses this changing demography and whether it will result in greater

equality. Regardless, major social institutions will be transformed because of this demographic shift.

The Minority Middle Class

The Census Bureau has no official definition of the middle class. Although the belief in a "middle class" is central to American life, there is no official income figure that connotes a middle-class lifestyle. Some analysts classify households with total annual incomes between \$40,000 and \$140,000 as middle class.⁷ Other analysts categorize middle-class families as having annual incomes between \$25,000 and \$75,000. Still others use annual incomes of \$50,000 as a marker. Since the cost of living differs so widely depending on location (e.g., geographic, urban or rural), a precise definition of "middle class" is virtually impossible.

Educational gains made by minorities, including African Americans, were significant. In 2014, 92 percent of blacks were high school graduates compared to 96 percent of whites. There is now no significant difference in high school graduation rates between whites and blacks.

From 1990 to 2014, the percentage of black 25- to 29-year-olds with a bachelor's or higher degree rose from 13 to 22 percent. The rate for Hispanic college graduates rose from 8 to 15 percent and for Asians/Pacific Islanders from 43 to 61 percent. At the same time, the rate for white college graduates rose from 26 to 41 percent. Although these numbers reflect the growth of tertiary educational attainment among nonwhite groups from 1990 to 2014, the educational gap between these groups and whites has widened.⁸



African Americans have made significant gains in professional employment.

Source: Pressmaster/Shutterstock

Spotlight 4.1

THE MINORITY MIDDLE CLASS

Minority groups have made concrete gains in entering the middle class. One of the most noticeable gains has been in the area of professional employment. A variety of media outlets offer business and financial information to the African American community. One of these outlets is *Black Enterprise* magazine, which has been in existence since 1970. The magazine provides information on financial management, entrepreneurship, and careers to professionals, corporate executives, entrepreneurs, and decision makers. To learn more about *Black Enterprise*, visit its website at www.blackenterprise.com.

The Hispanic population has also experienced significant economic successes. Hispanics have achieved impressive gains in household income, educational attainment, and homeownership. There has also been a significant uptick in Hispanic-owned business ownership, outpacing the national average. The U.S. Census Bureau website is an excellent source of information about the current economic trends in the Hispanic business community, with the article “Hispanic-Owned Business on the Upswing” providing in-depth statistics on the growth in Hispanic business ownership.

The minority middle class is nearly invisible to the general public partly because of the negative

attention paid to poor urban populations in terms of public assistance, child abuse, crime, teenage pregnancy, and drug use. However, since social researchers rarely focus on the other three-fourths who are solidly middle or upper class, we have relatively little information on this group.

Despite high rates of poverty—25 percent of African Americans and Hispanics lived below the poverty line in 2014⁹—until recently minority groups have made solid economic gains. Before the 2007 financial crisis, many African Americans had not only improved their socioeconomic position in recent years but they did so at a relatively faster rate than whites in professional employment, income (including two-earner family incomes), and homeownership. For example, the number of black professionals increased from 3.3 million in 2001 to over 4 million in 2004, with the percentage of black full-time workers earning over \$75,000 a year increasing from 4.4 to 5.7 percent.¹⁰ Unfortunately, many of these gains were reversed in the recession that began in 2008. Specifically, blacks experienced the largest decline in family income out of all the race groups. Although black household income had reached \$37,562 in 2000, by 2014 it had declined to \$35,398.¹¹

The U.S. Census has estimated a 3.5 percent increase in median household income for Hispanics between 2012 and 2013.¹² Economic achievements for Hispanics remain impressive. In particular, Hispanic high-income earning families have done especially well. In 1986, only 5.9 percent of all Hispanic households had incomes of over \$100,000; by 2011, this had nearly doubled to 11.5 percent.¹³ In 2015, 45.4 percent of *all* Hispanic households were homeowners. Much of this economic growth was fueled by Hispanic business formation, which increased 39 percent from 1992 to 1997.¹⁴ Despite this growth, however, Hispanic-owned firms only comprised 8.3 percent of all U.S. businesses in 2007.¹⁵

From 2002 to 2007, the number of black-owned businesses increased by almost 61 percent to 2 million, more than triple the national rate of 18 percent. During the same period, receipts generated by black-owned businesses increased from 55.1 percent to \$137.5 billion.¹⁶

While the growth of the minority middle class was a result of the prosperity of the middle 1990s, this changed with the recession that began in late 2007. In 2014, the Census Bureau reported that the overall median household income was up: Asian households had a median income of \$74,297, white households earned \$60,256, Hispanic households had \$42,491, and black households had \$35,398. The median income for all households was \$53,657 (see Figure 4.1).¹⁷

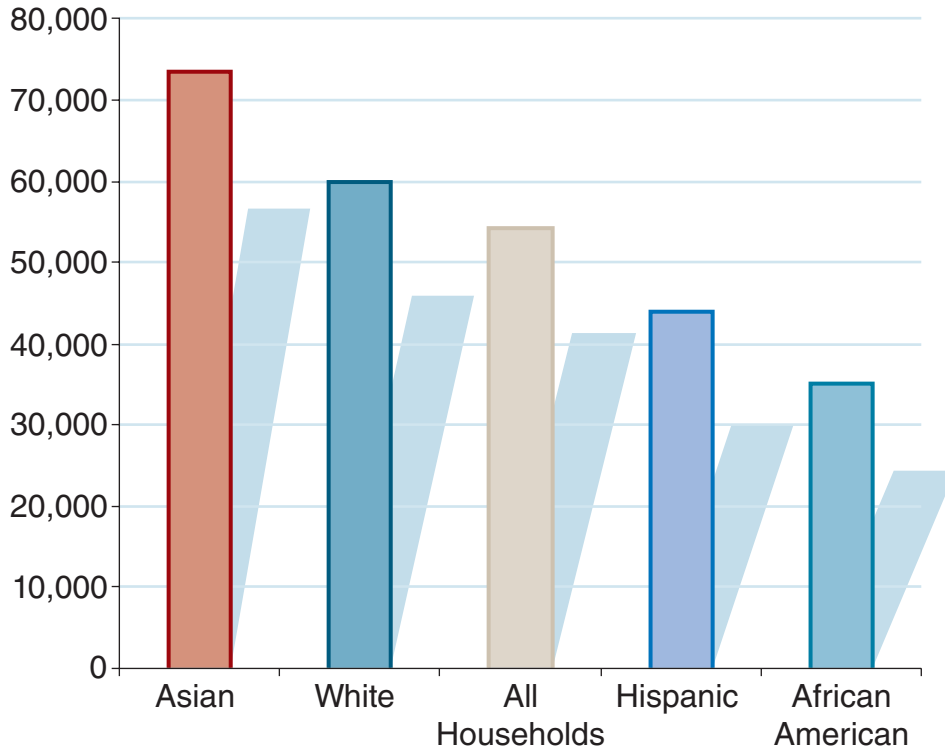


FIGURE 4.1 Median Household Annual Income, 2014

Source: Based on Carmen DeNavas-Walt and Bernadette D. Proctor "Income and Poverty in the United States: 2014," U.S. Census Bureau, September 2015. December 2015 from <https://www.census.gov/library/publications/2015/demo/p60-252.html>.

African Americans

The Demography of African Americans

The nation's African American population in 2015 was approximately 45.0 million and totaled 17.9 percent of the U.S. population. The African American population is expected to grow to 74.5 million by 2060.¹⁸

African Americans are concentrated in the South (60 percent) and are also more likely to live in metro areas than whites. The 10 states with the largest African American populations in 2013 were New York (3.7 million), Florida (3.2 million), Texas (3.2 million), Georgia (3.1 million), California (2.7 million), North Carolina (2.5 million), Illinois (2.0 million), Maryland (1.8 million), Virginia (1.7 million), and Ohio (1.5 million).¹⁹

African Americans in Poverty

After dropping in the mid-1990s, the number of poor African Americans rose from 9.9 million in 2009 to 10.7 million in 2010. By 2015, the poverty

rate for blacks was 27 percent nationally, and as high as 36 percent in Arkansas and the District of Columbia.²⁰ This is a significant change from 2001, when the Census Bureau recorded the lowest poverty rate for African Americans (22.7 percent or 8.1 million people) since they began collecting poverty data in 1959.²¹

The "Diswelfare" of African Americans

History was made on November 4, 2008, when Barack Obama was elected the first African American president in U.S. history. Despite this important victory, intense problems continue to plague the African American community, including poverty, police violence, high levels of incarceration, lower wages than comparable white workers, and high unemployment.

Poverty Despite a drop in the mid-1990s (and a rise in the early 2000s), black poverty rates continue to be much higher than for non-Hispanic whites. Some 10 percent of non-Hispanic whites were poor

in 2015, a rate almost one-third of that of blacks. Similarly, black median household income in 2014 was only 59 percent of their white counterparts.²²

Family Structure In 2011, 28 percent of African American households were married.²³ In 2010, 41 percent of African American households headed by a single female with no husband were living below the poverty line, a statistic which reflects the difficulties of single-income women raising families.²⁴

Labor Force Participation and Income Discrimination continues in the areas of employment and wages. Although laws and regulations addressing employment racial discrimination have had some success, they have not adequately prevented widespread discrimination against African Americans. For instance, black unemployment rates are still more than twice that of whites, reflecting significant inequality in the labor force.²⁵ By late 2015, the white unemployment rate was 4.3, while the black rate was 9.4 percent. This unemployment differential holds even for black college graduates. Discrimination influences income, and more the education, the greater the income gap (see Table 4.1).²⁶

Despite high tuition costs, the payoff from a college degree lifts lifetime earnings. However, that advantage is lessened for black and Hispanic college graduates due to high debt levels and lower wages.

In particular, blacks and Hispanics have less family resources to draw from and hence incur high debt levels. Although blacks and Hispanics with college degrees generally earn significantly more and have assets four times greater than their non-degreed counterparts, they suffer disproportionately in periods of financial trouble. From 1992 to 2013, the median net worth of blacks with college degrees dropped nearly 56 percent when adjusted for inflation. By comparison, the median net worth of whites with college degrees rose 86 percent.²⁷

Jared Bernstein offers several explanations for the African American wage gap, including employment discrimination (aided by the lax enforcement of antidiscrimination laws), jobs in vulnerable sectors of the labor economy, lower rates of overall unionization, and the general erosion of worker rights.²⁸ Another explanation suggests that as middle-class African Americans advance, greater unemployment occurs among those left behind. But this explanation does not address the large African American/white unemployment differential among college-educated men. According to Franklin Wilson, the cause of this discrepancy can be found in two factors: (1) the decline during the 1980s in the number of public sector jobs traditionally filled by college-educated African American men and (2) the failure of African Americans to penetrate the occupations

TABLE 4.1 Usual Weekly Earnings of Full-Time Wage and Salary Workers Aged 25 and Over by Educational Attainment, 2014 Annual Averages

Education Level	Total	Men	Women	White	Black or African American	Asian	Hispanic or Latino
Total, all education levels	\$839	\$922	\$752	\$864	\$674	\$991	\$619
Less than a high school diploma	488	517	409	493	440	477	466
High school graduates, no college	668	751	578	696	579	604	595
Some college or associate degree	761	872	661	791	637	748	689
Bachelor's degree only	1,101	1,249	965	1,132	895	1,149	937
Bachelor's degree and higher	1,193	1,385	1,049	1,219	970	1,328	1,007
Advanced degree	1,386	1,630	1,185	1,390	1,149	1,562	1,235

Source: U.S. Bureau of Labor Statistics, U.S. Department of Labor, The Economics Daily, Median weekly earnings by educational attainment in 2014 on the Internet at <http://www.bls.gov/opub/ted/2015/median-weekly-earnings-by-education-gender-race-and-ethnicity-in-2014.htm> (visited January 29, 2016).

that encompass managerial or supervisory responsibilities. Regardless of the causes, African Americans continue to earn less than whites, irrespective of household composition, education, region, or religion.

Crime In 2015, 2.2 million people were held in U.S. federal or state prisons or in local jails. The lifetime likelihood of going to state or federal prison for blacks is about twice that of Hispanics and five times higher than for whites. Based on the rates of first incarceration, about 32 percent of black males will enter state or federal prison during their lifetime, compared to 17 percent of Hispanic males and 5.9 percent of white males.²⁹ Moreover, the racial composition of the 3,297 people on death row in 2015 was 43 percent white, 42 percent black, and 13 percent Latino. Given that blacks make up only about 13 percent of the U.S. population, they are overrepresented on death row by more than three times their overall number.³⁰

In 2014, blacks were almost four times more likely than whites to be murdered.³¹ Although the homicide rate for blacks has fallen in recent years, it remains unacceptably high. Blacks aged 20 to 24 years old have the highest homicide rates (more than seven times that of blacks aged 13 to 16 years old). The homicide rate for blacks aged 25 to 29 is the second highest and is comparable to the 20 to 24 age bracket: 1,101 compared to 1,409 respectively. Overall, in 2014, the black male homicide rate was almost 32 per 100,000 compared to about 4 per 100,000 for white males.³²

Black Lives Matter was created in 2012 by Patrisse Cullors, Opal Tometi, and Alicia Garza after George Zimmerman was acquitted in the death of 17-year-old Trayvon Martin. The movement took hold after the media focus on police-caused deaths of unarmed black males, among them were 18-year-old Michael Brown in Ferguson, Eric Garner (died after being placed in a choke hold by NYPD officers), John Crawford III (killed in an Ohio Walmart while holding a toy BB gun), Tanisha Anderson (died after Cleveland police officers allegedly slammed her head into the pavement during custody), 12-year-old Tamir Rice (shot by Cleveland police due to a mistaken toy gun), Walter Scott (killed while running away from police), Freddie Gray (died due to a spinal injury incurred while in custody), and 17-year-old Laquan McDonald (shot 16 times as he was walking away from Chicago police). This is only a

partial list of police-related deaths of black males and females.³³

Housing Housing patterns reflect the economic gulf between African Americans and whites. Minority (African American, Hispanic, and Native American) households are both poorer and more likely to be renters than white households. The homeownership rate in 2014 for whites was 72.3 percent, for Hispanic or Latino households was 44.5 percent, and for African American households was 42.1 percent.³⁴ Many of these homes were purchased under less than desirable terms and are subject to foreclosure.

Health Racism is also evident in health issues. The 2013 black infant mortality rate in the United States was 11.1 per 1,000 live births, more than twice that of whites (5.1 deaths).³⁵ Taken by itself, the U.S. black infant mortality rate ranks high internationally (see Table 4.2). In 2013, African Americans had almost twice the sudden infant death syndrome (SIDS) mortality rate as non-Hispanic whites. In 2010, black men were nearly seven times more likely than white men to be diagnosed with HIV, and black women more than 20 times more than white women.³⁶ Not surprisingly, life expectancy is lower for African Americans. In 2014, black males had a life expectancy of 71.8 years compared to 76.5 years for white males. For African American women, the life expectancy was 76.5 years compared to 81.3 years for white women.³⁷

A major factor affecting infant mortality is low birth weight. Although the cause is unknown, low birth weight (less than 5.5 pounds) increases the chances of infant death during the first month by 40 percent. Overall, 8 percent of babies were born at low birth weight in 2013; for African Americans it was 13 percent.³⁸

Education African American and white students graduate high school at close to the same rate.³⁹ Although high numbers of black and white students graduate from high school, college completion rates significantly differ. In 2013, 67 percent of whites aged from 18 to 21 were enrolled in higher education compared with 57 percent of blacks. The percentage of whites aged 25 and over holding a bachelor's degree in 2014 was 41 percent compared to 22 percent of blacks.⁴⁰ As a high school diploma becomes less valuable in the marketplace, educational upgrading can help

TABLE 4.2 Infant Mortality Rates and Ranks*: Selected Countries, 2015 (Estimated Deaths per 1,000 Live Births)

Rank	Country	Rate	Rank	Country	Rate
222	Japan	2.08	178	Greece	4.70
221	Singapore	2.48	184	Taiwan	4.44
217	Sweden	2.60	176	Hungary	5.02
212	Italy	3.12	167	United States	5.82
211	Spain	3.30	168	Croatia	5.72
213	France	3.28	166	Serbia	6.05
218	Finland	2.52	182	Poland	4.50
208	Germany	3.43	175	Slovakia	5.27
198	Ireland	3.70	160	Chile	6.86
199	Switzerland	3.67	151	Costa Rica	8.46
204	Israel	3.55	146	Sri Lanka	8.80
194	South Korea	3.86			
191	Denmark	4.05	145	Botswana	8.93
206	Austria	4.45	134	Panama	10.41
188	Australia	4.37	113	Malaysia	13.27
187	United Kingdom	4.38	121	China	12.44
185	Portugal	4.43	122	Mexico	12.23
181	New Zealand	4.52	92	Venezuela	18.91
180	Cuba	4.63	95	Brazil	18.60
179	Canada	4.63	89	Peru	19.59

*Ranks are on a scale from 1 to 222, with 1 being the country with the highest infant mortality rate and 222 being the country with the lowest.

Source: Adapted from Central Intelligence Agency, The World Factbook: Country Comparison-Infant Mortality Rate (2015). Retrieved October 2015 from <https://www.cia.gov/library/publications/the-world-factbook/rankorder/2091rank.html>.

protect workers' incomes. In 2011, both blacks and whites with a bachelor's degree earned over \$100 more a week in comparison to those with only a high school diploma.⁴¹ The strong correlation between higher education and higher salaries implies that the disparity between African American and white college completion rates will have a long-term impact on the economic well-being of minorities.⁴²

Welfare Dependency Welfare dependency is another indicator of racism. Although welfare dependency is increasing for African Americans, in 2009, 11 percent received more than 50 percent of their total annual family income from means-tested public assistance programs compared to 2.7 percent of the non-Hispanic white population.⁴³

Hispanic Americans

In 2002, Hispanics became the nation's largest minority community.⁴⁴ By 2015, the U.S. Hispanic population totaled 55 million.⁴⁵ It is projected that from 2002 to 2050, the Hispanic population will increase by 188 percent to 102.6 million, or roughly one-quarter of the U.S. population. According to the 2010 U.S. Census, Hispanics accounted for 15.2 million, or more than one-half, of the total population increase of 27.3 million from 2000 to 2010.⁴⁶

The use of the term *Hispanic* masks a rich diversity within this group. The U.S. Hispanic population has roots in 22 different countries; their family histories are from Mexico, Puerto Rico, Cuba and other Caribbean islands, and Central and

South America as well as the United States. Latinos inject \$300 billion into the U.S. economy. Despite this fact, 26.6 percent of Hispanics lived below the poverty line in 2010 compared to just 9.9 percent of non-Hispanic whites and 27.4 percent of African Americans.⁴⁷

Due to the large number of undocumented workers coming from the Americas, the Hispanic population of the United States is difficult to accurately measure. Estimates of the number of undocumented workers in the United States (most of them from Spanish-speaking countries) are about 11.3 million, half of which are Mexican.⁴⁸ The rapidly growing Hispanic population is geographically concentrated, with over 50 percent residing in just three states: California, Texas, and Florida.⁴⁹

Hispanic Poverty and Income

The poverty rate among Hispanics compares to that of African Americans. Although the Latino economic position has improved in recent years (a slight drop in poverty and a lower unemployment rate), they remain significantly poorer than whites. In 2014, the median income for all households in the United States was \$53,657, with white median household income at \$60,256. For Hispanic households, it was \$42,491. The Hispanic poverty rate is almost triple that of non-Hispanic whites.⁵⁰

Diversity in the Hispanic Population

Although for statistical purposes the Hispanic population is often considered as a single group, the various Latino subgroups have distinct social and historical backgrounds. For example, Cubans living in Florida may have little in common historically or politically with Mexican Americans living in California, and Puerto Ricans living in New York may have little understanding of the culture of either Cuban or Mexican Americans. These differences are also reflected in income.

Mexican Americans constitute about 67 percent (17.1 million) of all Hispanics in the United States and are the fastest-growing Spanish-speaking subgroup. In 2000, there were 20.6 million Mexican Americans in the United States; by 2010, this number had grown to 31.8 million, comprising 75 percent of the current overall Hispanic population.⁵¹

The poverty of Mexican Americans is correlated in part to deficits in educational attainment. In

2013, 14 percent of Hispanics aged between 18 and 24 had never completed high school. In 2013, only 8 percent of Mexican Americans had earned a college degree or higher.⁵²

The total wealth in the Latino community has risen. The number of Hispanic businesses increased 31 percent (from around 1.2 million to 1.57 million) from 1997 to 2002.⁵³ By 2015, Hispanic businesses were growing at more than 15 times the rate of all U.S. firms and contributed \$600 billion in revenue to the national economy.⁵⁴ Despite these gains, the economic data suggests a difficult economic picture for most Hispanic communities in the United States, especially Mexican American and Puerto Rican.

American Indians

Despite the 562 federally recognized tribes (including 223 Alaskan Native groups) and 275 American Indian reservations, there is no single definition of an American Indian.⁵⁵ The Bureau of Indian Affairs (BIA) classifies a person as American Indian if he or she is a member of a recognized tribe and has one-fourth or more Indian blood. The Census Bureau uses self-identification. In 2013, American Indian and Alaska Natives numbered almost 5.2 million people or 2 percent of the U.S. population.⁵⁶ According to BIA estimates, about half of American Indians live on or near reservations.⁵⁷

American Indians, in some ways the poorest group in the United States, experience oppression similar to that of other disenfranchised populations. The history of American Indians is marked by hardship, deprivation, injustice, and genocide. Before white settlement, American Indians numbered somewhere between 1 million and 12 million.⁵⁸ This population was decimated as a result of disease, the westward expansion of white settlers, and the resulting wars and genocidal policies. By 1880, the census reported only 250,000 American Indians.⁵⁹ Moreover, American Indians were not granted citizenship until 1924; New Mexico prohibited them from voting until 1940.

The social and economic problems faced by American Indians are similar to those of other at-risk minority populations. After generations as the nation's poorest and most overlooked minority, American Indians continue to suffer from what a 2003 U.S. Commission on Civil

Rights report called a “quiet crisis” of discrimination, poverty, and unmet promises. Unemployment, substance abuse, and school dropout rates are among the highest in the nation, and American Indians face staggering health problems.⁶⁰ In 2004, American Indians were 770 percent more likely to die from alcoholism, 650 percent more likely to die from tuberculosis, 420 percent more likely to die from diabetes, 280 percent more likely to die from accidents, and 52 percent more likely to die from pneumonia or influenza than the rest of the United States, including other minority populations.⁶¹

American Indian tribes are sovereign nations under federal law, and states may not enforce their civil codes on reservations. The Indian Gaming Regulatory Act of 1988 allowed tribes to operate gambling operations within their borders. By 2008, there were nearly 450 gaming operations run by more than 235 tribes in 28 states. In 2008, the Indian gaming industry contributed \$8.5 billion in output; over 700,000 jobs; \$27 billion in wages; and \$10.8 billion in federal, state, and local tax revenue. Economists William Evans and Julie Topoleski found mixed results around gaming. While more young adults moved back to reservations and adult employment increased by 26 percent, most of that growth was among non-Indians. Four years after a casino opens, communities experienced a 10 percent increase in auto thefts, larceny, and violent crime, and an increase in bankruptcies. Their presence in many states diverts funds from taxable activities.⁶²

Caught in the paternalistic web of the BIA, American Indians continue to struggle to retain their identity. Having been robbed of their land, murdered indiscriminately by encroaching white settlers (as well as by the U.S. Cavalry), and treated alternately as children and pests by the federal government, for many years American Indians were further oppressed by having their children taken away by welfare officials. This widespread abuse by welfare workers, who evaluated American Indian child-rearing practices as neglectful in the context of white middle-class family values, was partially remedied by the **Indian Child Welfare Act of 1978**, which restored child-placement decisions to the individual tribes. As a result of this act, priority in placement choices for American Indian children was given to tribal members rather than white families.⁶³ In an

attempt to remedy historical injustices, the Indian Self-Determination Act of 1975 emphasized tribal self-government, self-sufficiency, and the establishment of independent health, education, and welfare services.⁶⁴ Despite these limited gains, the plight of American Indians serves as a reminder of the mistakes made by the United States in both past and its present policies toward disenfranchised minority groups.

Asian Americans

There are more than 25 Asian-Pacific subgroups in the United States, including Chinese, Filipino, Japanese, Asian-Indian, Korean, Vietnamese, Laotian, Thai, Cambodian, Polynesian (Hawaiian, Samoan, and Tongan), Micronesian, and Melanesian. By 2000, the six largest Asian population groups in the United States were: Chinese (2.7 million), Filipino (2.4 million), Japanese (1.1 million), Asian-Indian (1.9 million), Korean (1.2 million), and Vietnamese (1.2 million).⁶⁵ In 2010, the Asian population was 14.7 million.⁶⁶ According to the 2010 Census, Asians were the fastest-growing racial group, increasing by 43 percent between 2000 and 2010.⁶⁷ It is projected that because of above-average birthrates and accelerating immigration, the Asian population is expected to rise to 33 million by 2050.⁶⁸

While the social and economic data on Asians is mixed, perhaps the most striking feature is their high-median family income. Economic and social data point to a population that has made great strides, especially in the educational area.⁶⁹ While Asians made up only 5 percent of the population in 1914, they make up as much as 20 percent of students at many prestigious research universities. In 1997, the University of California system moved to a race-blind admissions system. Consequently, Asian American enrollment in the top California public universities has reached 40 or even 50 percent. On the other hand, the test scores needed for Asian American students to get into prestigious universities may be higher than other groups due to overrepresentation.⁷⁰

As impressive as these economic and educational statistics are, they conceal several phenomena:

- Most Asians live in expensive urban areas where salaries are higher than the U.S. average;

- many recent Asian immigrants work in low-wage sweatshops in urban Chinatowns;
- although many Chinese and Japanese Americans have achieved economic success, Southeast Asians are at higher risk of poverty than whites;
- Asians are underrepresented in the higher-salaried public and private career positions and their salaries tend to be lower than whites; and
- although a large number of Asian immigrants have become successful entrepreneurs and own and operate their own small businesses, for some this role was forced on them as a result of discrimination.

One of the most egregious examples of discrimination against Asian Americans was the internment of 127,000 Japanese Americans in detention camps during World War II. The fear was that Japanese Americans would remain loyal to their ancestral land. This was despite the fact that almost two-thirds of the interns were born in the United States and many had not even been to Japan. Even Japanese-American veterans of World War I were forced to leave their homes. Fearing that their homes and businesses would be gone when they returned, many sold them off at a fraction of their worth. Congress acknowledged this dark mark on U.S. history when in 1988 it awarded each surviving internee \$20,000.⁷¹

Asians also experience more subtle forms of discrimination. For example, because of their economic and educational achievements, they are often thought of as a “model minority.” Because of this status, many Asians experience internal and external pressure around achievement and success. Some Asian Americans complain they are discriminated against in colleges and universities because of their strong academic performance. Facing this backlash, some have redoubled their efforts to become mainstream U.S. citizens—to be in the center of the society rather than on the margins as “hyphenated Americans.”

Many Asians point to contributions resulting from Asian cultural values that include an emphasis on frugality, greater consideration for the feelings of others, and a balance between group and individual welfare. According to sociologist Tu Weiming, the less individualistic culture of Asians, their lower sense of self-interest, their less adversarial nature, and their less legalistic approach have valuable lessons for the United States.⁷²

Immigrants and Immigration

At current rates, the U.S. Census Bureau estimates that immigration will help swell the U.S. population from 310 million in 2010 to more than 420 million in 2050.⁷³ As noted earlier, it is difficult to determine exactly how many illegal immigrants are in the United States.⁷⁴ While Mexicans made up 52 percent of undocumented workers in 2014, another 23 percent came from other Latin American countries, including El Salvador, Guatemala, and Honduras.

Almost two-thirds of immigrants live in just six states: California, Texas, New York, Florida, Illinois, and New Jersey. However, much of the rapid growth in the immigrant population since the mid-1990s has been outside these states.⁷⁵ In 2014, the foreign-born population was 42.3 million, equal to 13.3 percent of the total U.S. population.⁷⁶ While poverty rates are higher for immigrants, they are lower for naturalized citizens (10 percent) compared to native-born Americans (12 percent).

The pace of economic growth in the 1990s would have been impossible without the influx of immigrants who contributed to job growth in three ways: (1) by filling an increasing share of jobs, (2) by taking jobs in labor-scarce regions, and (3) by filling the types of jobs shunned by native-born American workers. In fact, immigrants filled four of every 10 job openings at a time when the unemployment rate hit record lows in the mid-1990s.⁷⁷

Not only is the number of jobs filled by immigrants important, but the location and kind of jobs is also important. In earlier years, most new immigrants from Latin America and Asia clustered in a few large cities, such as Los Angeles, New York, and Chicago. However, the 1990s witnessed a spread to the western Midwest, New England, and the middle and south Atlantic regions. In some parts of the country, almost the entire labor force growth between 1996 and 2000 was due to immigration.

Because a large number of new immigrants have less formal education (33 percent of immigrants have not finished high school), new immigrants commonly fill low-skill, blue-collar, or service jobs. Many immigrants possess job and other skills that often do not translate into the U.S. workplace. Although immigrants overwhelmingly fill blue-collar jobs, they also account for as much as half the growth in categories such as administrative support and services.⁷⁸

Legal immigration to the United States underwent several important changes since the mid-1980s.

For one, while the number of immigrants remained relatively constant from 1985 to 1988, it rose sharply from 1988 to 1991 due to the impact of the Immigration Reform and Control Act of 1986 (IRCA), which granted legal status to undocumented immigrants who were in the United States continuously since 1982 or who had worked in agriculture. But by 1995, the total number of immigrants admitted to the United States was less than half that those admitted in 1991.⁷⁹

Other changes occurred in U.S. immigration since the 1950s. The most notable was the shift in immigration patterns from Europe to Asia (see Figure 4.2). Contrary to public myth, the largest immigrant group comes from Asia (40 percent), while Mexicans constitute only 14 percent of immigrants. Since 2009, more Mexicans left the United States (1.4 million) than migrated to it (870,000).⁸⁰

The other significant change in immigration patterns has been the increase of “Unaccompanied Alien Children” or UAC (the term used by U.S. Customs and Border Protection). The number of unaccompanied children swelled from 14,700 in 2012 to 57,500 in 2014, with a corresponding budget increase from \$278 million to \$868 million.⁸¹ Most of these children come from Guatemala, Honduras, and El Salvador and are sent to the United States to escape the widespread violence in those countries. These children are referred to the Office of Refugee Resettlement (part of the Department of Health and Human Services) until their immigration status is resolved.

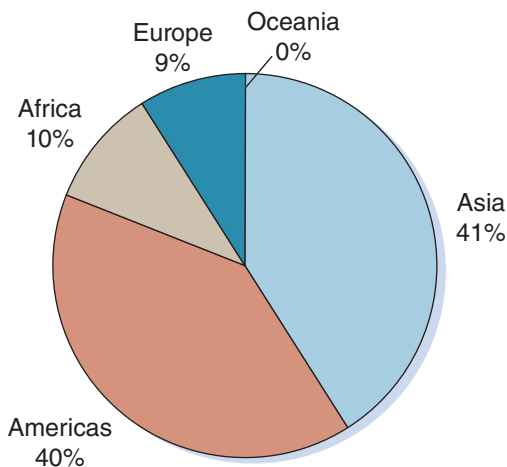


FIGURE 4.2 Immigrants by Place of Origin, 2013

Source: Based on Migration Policy Institute, U.S. Immigration Trends, 2015. Retrieved December 2015 from <http://www.migrationpolicy.org/programs/data-hub/us-immigration-trends>.

Finally, the United States is facing another immigration crisis. In 2015, former President Obama agreed to let in 10,000 Syrian refugees. Criticized as a token measure by immigration activists, this number pales in contrast to the 800,000 Syrian immigrants granted asylum by Germany in 2015. Despite this low number, several governors have stated they will refuse to allow Syrian immigrants to settle in their states.

In 1996, Congress passed the **Personal Responsibility and Work Opportunity Reconciliation Act (PRWORA)**, which contained important implications for legal and illegal immigration (PRWORA will be discussed more fully in Chapter 11).⁸² Specifically, the bill disentitled most legal immigrants (including many who had been living in the United States for years but were not citizens) from food stamps, public assistance, and Supplemental Security Income (SSI). (Illegal immigrants were never entitled to these benefits.) The only immigrants who were entitled to benefits were (1) those who had become citizens or who had worked in the United States and paid Social Security taxes for 10 years or more and (2) veterans of the U.S. Army who were noncitizens. In addition, states were allowed to deny Medicaid benefits to immigrants.⁸³ While some of the harsher provisions were later rescinded, anti-immigration sentiment remained.

Immigration policy changed dramatically after the World Trade Center attacks of September 11, 2001. After 9/11, three important acts were passed: (1) the USA Patriot Act, (2) the Enhanced Border Security Act, and (3) the Homeland Security Act of 2002. Although these acts do not directly shape immigration policy per se, they impact it by more carefully screening and monitoring foreigners who temporarily visit (and sometimes wish to settle in) the United States. For instance, in the Enhanced Border Security and Visa Entry Reform Act of 2002, the U.S. Congress mandated the use of biometrics in U.S. visas. In particular, the Homeland Security Council mandated that biometric screening (10 fingerprint scans) was to be collected at all U.S. Embassies and Consulates for visa applicants from non-Visa Waiver Program (VWP) countries.

In April 2010, Arizona adopted the nation's toughest bill on illegal immigration into law. Its aim was to identify, prosecute, and deport illegal immigrants. The law made the failure to carry immigration documents a crime and gave the police broad power to detain anyone suspected of being in the country illegally. For many, it signaled an open invitation for



Foreign students have been especially affected by stricter immigration procedures.

Source: Carlos E. Santa Maria/Shutterstock

harassment and discrimination against Hispanics regardless of their citizenship status. In 2011, the federal government filed a Supreme Court case around the legality of a state instituting its own immigration policies.⁸⁴ The Supreme struck down parts of the law while allowing Arizona to implement other parts. As of 2015, legal challenges were ongoing.

Surveillance in the post-9/11 areas also intensified. For instance, various foiled attempts at terrorist acts in the New York area led the New York Police Department (NYPD) to secretly infiltrate and spy on Muslim university students in the Northeast. The NYPD defended their actions by citing convicted terrorists worldwide who had once been affiliated with Muslim student groups.⁸⁵ The question brought up by these and other tactics is just how much freedom a democratic society is willing to forego for national security.

Immigration-Based Discrimination in Europe

The Syrian civil war and wars, repression, violence, and misery in Iraq, Afghanistan, Libya, Yemen, Sudan, and Somalia, to name a few countries, have

set into motion a powerful wave of immigration into developed and developing countries. Turkey is housing almost 2 million refugees, Lebanon 1.2 million, and Jordan 630,000. The migration from Syria has been particularly strong with at least 4 million refugees fleeing the country. In 2015 alone, a million or more refugees from the Middle East, Asia, and Africa had arrived in European Union (EU) countries seeking asylum. Germany was expected to take in almost 1 million refugees in 2015. This intense immigration has created untold problems for European nations accustomed to homogenous populations.

The intense flow of refugees has led to several different reactions across Europe. While Germany has taken in the largest share of refugees, other countries like Hungary built a barbed wire fence to keep immigrants out. Still other countries like Slovakia and the Czech Republic have stated their opposition to resettling large numbers of refugees. Even a liberal nation like Denmark ran an ad in a Lebanese newspaper discouraging immigration.

Most forms of discrimination are characterized by social exclusion. Although Western Europe prides itself on tolerance and multiculturalism,

recent events challenge that perception. Post-World War II Europe experienced an economic boom that required large numbers of workers, which in turn, led to high levels of immigration. The increase in immigration was also fueled by large numbers of migrants arriving from former European colonies or territories, such as the Caribbean, Algeria, and parts of Africa. Germany adopted a guest worker program based on the belief that Turkish workers would return home once their labor was no longer needed. After several generations, many Turkish immigrants viewed Germany as their home. Similarly, large numbers of immigrants came to the United Kingdom (UK) from former colonies, including India, Pakistan, the Caribbean, South Africa, Kenya, and Hong Kong.

One of the largest immigrant groups in Europe came from Muslim countries, which resulted in a Muslim population of about 20 million, out of 500 million European Union (EU) residents. In 2010, the EU countries with the largest Muslim populations were France (7.5 percent), Netherlands (6 percent), Belgium (5.9 percent), Germany (5.8 percent), Austria (5.4 percent), United Kingdom (4.8 percent), Sweden (4.6 percent), and Denmark (4.1 percent).⁸⁶ Muslims living in the EU are more likely to be poor and live in segregated high-crime neighborhoods.⁸⁷ Some analysts suggest that Muslims may self-segregate due to language and different cultural norms (i.e., prohibitions against drinking).

There are also calls—especially in the United Kingdom—for the formal institution of Sharia law in ethnic Muslim neighborhoods.⁸⁸ Parallel to the British legal system, Sharia or Muslim religious law has been operated by local Muslim councils since 1982 and is growing in use as thousands of Muslims use it to settle civil disputes, as well as family, financial, and commercial problems. Plaintiffs and defendants voluntarily choose to accept the court's rulings because the courts have no legal powers and cannot impose penalties. Despite its growth, women's groups and others oppose Sharia courts because they argue that they are male-dominated and discriminate against women.⁸⁹ By 2011, the leaders of Western Europe's three largest countries—Germany, France, and the United Kingdom—acknowledged that multiculturalism had failed. German Chancellor Andrea Merkel was disillusioned by the slow pace of Turkish immigration, British Prime Minister David Cameron by the tolerant attitude toward extreme Islam, and France's former prime minister Nicholas Sarkozy by Muslim

prayer in the street. Right-wing Dutch parties had long denigrated multicultural policies.⁹⁰

Anti-Islamic sentiment in Western Europe has been fueled by several events, including the 2004 Madrid commuter train bombings that killed 191 people and wounded 1,800. One year later, four coordinated terrorist suicide attacks on the London subway and bus system killed 52 people and injured more than 700. A Danish newspaper published cartoons depicting Muhammad, which resulted in worldwide riots and the deaths of 100 people. The anti-Muslim sentiment in Europe was further fueled by the 2015 terrorist attack on the satirical *Charlie Hebdo* magazine in Paris that left 12 dead. While supporters saw *Charlie Hebdo's* satire as free speech, others viewed it as Islamophobic and racist. These events were followed by the November 2015 ISIL-inspired terrorist act in Paris that left 130 dead, including 89 at the Bataclan theater. Almost 400 people were injured, with 100 seriously wounded.

The anti-Muslim sentiment was further aggravated by the rise in honor killings within Europe's Middle Eastern and Asian immigrant communities. Estimated at 5,000 deaths a year worldwide, honor killings typically involve the murder of a woman, usually by brothers and fathers (often with the collusion of mothers, sisters, aunts, or grandmothers), for acts perceived as dishonoring the family. These acts can include refusing an arranged marriage, having sex outside marriage, rebellious behavior (e.g., wearing Western clothes), disobeying male family members, dating someone the family disapproves of, attending the university, or even having been raped.⁹¹

In 2003 to 2004, the French government passed a law banning all clothing that indicates a student's religious affiliation from public schools. While the law was written in a religion-neutral way, its aim was to prohibit Muslim girls from wearing headscarves to class. In 2011, France took one step further by prohibiting the burqa (Muslim headgear that hides the full face) from being worn in public. Belgium passed a law in 2010 that prohibited wearing any clothing in public places that did not allow the wearer to be identified. Apart from security reasons (i.e., not being able to identify the wearer), the argument was that the burqa undermined the minimal demands of social life, degraded women, and was an affront to equality. Opponents of the burqa ban argue that the law is unjust and violates individual liberty. Moreover, the law prevents veiled

Muslim women from leaving home, thereby further isolating them. No reliable data exists as to how many Muslim women support or oppose the burqa ban.⁹² Still another negative reaction to the growth of Islam in Europe was expressed by Swiss voters who approved a ban on building new minarets on the top of mosques.⁹³

The EU maintains that successfully integrating immigrants is critical to maximizing their potential for development. The humane treatment of immigrants and the importance of human rights in the EU are reflected in the 1997 Amsterdam Treaty that reinforces the right to non-discrimination and provides for the suspension of a member state where there is a serious breach of human rights.⁹⁴

Women and Society

Hillary Clinton, former Secretary of State and 2016 presidential candidate, shattered an important glass ceiling for women. Although former Democratic Congresswoman Geraldine Ferraro ran for vice president in 1984, her candidacy was short-lived as Walter Mondale lost to Ronald Reagan in a landslide election. It took almost another three decades for a woman to run as a serious candidate for the U.S. presidency.

Sexism denotes the discriminatory and prejudicial treatment of women based on their gender. It is also a problem U.S. society has wrestled with since the beginning of the nation. Sexism can manifest itself in myriad ways. For example, women may be kept out of military academies (and combat duty), private clubs, and certain sports. Occupational boundaries can keep women from operating heavy construction machinery, or being involved in skilled trades such as bricklaying. Or, women reach a glass ceiling from where they cannot rise any higher. Sexism can take political forms, like appointing or electing token women to offices or cabinet positions or creating “special appointments” to placate feminist groups. Sexism reared its head high in the 2016 presidential election when male Republican candidates attacked Planned Parenthood and weighed in on women’s health and reproductive rights, including contraception.

Violence and Sexism

Sexism can also manifest itself in crime and family violence.⁹⁵ According to a study by Patricia Tjaden and Nancy Thoennes, nearly 25 percent of surveyed

women said they were raped and/or physically assaulted by a current or former spouse, cohabiting partner, or date at some time in their lifetime. Approximately 5 percent of U.S. women are raped and/or physically assaulted annually by an intimate partner.⁹⁶

The U.S. medical community treats millions of intimate partner rapes and physical assaults annually. Of the estimated 4.9 million intimate partner rapes and physical assaults perpetrated against women annually, about 2 million will result in serious injury to the victim, and 500,000 will result in some type of medical treatment. Most intimate partner victimizations are not reported to the police. Only one-fifth of all rapes, one-quarter of all physical assaults, and one-half of all stalking against women by intimates are reported to the police.⁹⁷

In 1994, Congress passed the Violence against Women Act (VAWA), which represents a comprehensive approach to domestic violence and sexual assault by combining a broad array of legal and other reforms. Specifically, the law was designed to improve the response of police, prosecutors, and judges; force sex offenders to pay restitution to their victims; and increase funding for battered women’s shelters. VAWA mandates a nationwide enforcement of protection orders and provides penalties for crossing state lines to abuse a spouse or violate a protection order. It also prohibits anyone facing a restraining order for domestic abuse from possessing a firearm.⁹⁸ VAWA, the Family Violence Prevention and Services Act (first authorized in 1984, it is the only federal funding source dedicated to domestic violence shelters and programs), and the Victims of Crime Act form the backbone of domestic violence services.⁹⁹

The Feminization of Poverty

Sexism also has an economic face. The median amount that a full-time female worker loses in wages over a 40-year period due to the gender or “career wage gap” is \$434,000, rising to \$713,000 if they have a bachelor’s degree or higher. Not only are women’s wages lower, but one-quarter to one-third of working women lack basic benefits. Four in 10 working women work evenings, nights, or weekends on a regular basis, and one-third work shifts different than their spouses or partners. Thirty-one percent of working women have no sick leave, and 34 percent do not have access to these benefits through their employers.¹⁰⁰

Because women's wages are generally lower than those of men, they more often have to resort to public welfare programs. The term *feminization of poverty* was coined by Diana Pearce in a 1978 article in *Urban and Social Change Review* in which she argued that poverty was rapidly becoming a female phenomenon as women accounted for an increasingly larger share of the economically disadvantaged.¹⁰¹

In 2014, 18 million women lived below the poverty line. The effects of poverty also impact families. In 2014, the poverty rate for children under the age was almost 22 percent. Of particular significance is the disparity in income between single female and single male householders who are raising families. In 2014, a single female householder's median annual income was \$32,031 in comparison to a single male householder at \$49,718. This wage gap means that it is more difficult for single women to support a family than it is for single men.¹⁰²

The causes of the feminization of poverty are complex. When women are deserted or divorced, many must find jobs immediately or go on public assistance. Those who choose welfare are held in poverty by the low benefits; those who opt to work are kept in poverty by the low wages that characterize service jobs. Low-paying service employment, such as fast-food outlets and retail stores, make welfare benefits seem attractive since staying at home involves no child care costs, transportation, clothing, and lunch costs.

Women frequently enter the workforce through the service and retail trades, clerical jobs, cleaning, food preparation or service, personal service work, or auxiliary health service work. These occupations are marked by low pay, a low level of unionization, little status, meager workplace benefits, and limited prospects for job advancement. Thus, much of the increase of women in the workforce is in the "secondary labor market" (see Chapter 5), a marginal area of employment.

The economics of low-wage service work are gloomy. For instance, if a single mother with one child in day care chooses to avoid the stigma of welfare and finds work at near the minimum wage, her prospects for economic survival are bleak.¹⁰³ The mock budget in Table 4.3 illustrates the dilemma of a single mother who finds full-time employment at \$8.25 an hour, \$1.00 above the federal minimum wage.

A 2010 USDA report found that an average middle-class family spent about \$12,000 on child-related expenses in their baby's first year; by age two, they spent more than \$12,500 a year.¹⁰⁴ Hence, a single mother with two children

TABLE 4.3 Monthly Budget for a Working Mother with One Child in School and One Infant in Day Care (Gross Monthly Income for a Full-Time Worker at \$8.25 an Hour Is \$1,320 a Month.)

Expenses	Monthly Total
Day care for 1 child (national average)	\$ 972.00 ^a
Rent	\$ 700.00 ^b
Health care	\$ 93.75 ^c
Utilities	\$ 60.00 ^c
Food	\$ 511.00 ^d
Clothing	\$ 62.00 ^a
Transportation	\$ 200.00 ^c
Entertainment	\$ 50.00
Sundry items (e.g., soap, cleaners, repairs, sheets, blankets)	\$ 100.00 ^c
Total approximate cost	\$2,748.75 ^c
Monthly deficit between income and budget	−\$1,428.75

^aChild care costs are calculated based on the national average. National Association of Child Care Resource & Referral Agencies (NACCRRA).

^bRent was calculated on the basis of the average of 2015 HUD determined fair market rents for a 2-bedroom apartment in Ohio (a median cost state).

^cCalculated from the Bureau of Labor Statistics, Consumer Expenditures, September 25, 2012. Retrieved December 2012 from <http://www.bls.gov/news.release/cesan.nr0.htm>.

^dThe amount for food is based on the maximum SNAP allocation that this mother would receive in 2015 if she had two children and no countable income.

Source: Based on Bureau of Labor Statistics, Consumer Expenditures, September 25, 2012. Retrieved December 2012 from <http://www.bls.gov/news.release/cesan.nr0.htm>.

earning \$8.25 an hour (\$1.00 above the federal minimum wage) would have a gross annual income of \$17,150, about \$2,940 below the federal poverty line of \$20,090 a year and \$6,850 below the estimated expenses for raising two young children. Neither welfare nor low-income work provides single female-headed households with viable economic choices.

Inequities in **public transfer programs** also aggravate the economic problems of low-income women. For example, Social Security and public assistance are often the only viable options for

women. Although median transfers to women are lower than to men, they constitute a higher share of the recipients' total income: about one-third for female-headed households compared to one-tenth for male-headed households. Moreover, Social Security is often the only source of income for elderly women. However, Social Security is riddled with pitfalls: Because women's wages are lower than men's, their retirement benefits are also lower, homemakers are not covered on their own unless they held a job in the past, widows do not qualify for benefits unless they are 60 years old or have a minor in the house, divorced women who have not worked are entitled to only one-half of their ex-husband's benefits, which is inadequate if it is their only source of income; and because of child care responsibilities, women typically spend less time in the workforce and their benefits are therefore lower.¹⁰⁵

Myths around Women and Work

Myths abound in attempting to explain why women consistently earn less than men:¹⁰⁶

Myth 1. A working mother's wages are not necessary to her family's survival; her job is a secondary activity that usually ceases with marriage or childbirth.

Fact. Most families require two paychecks to maintain the same standard of living as their parents. Twenty percent of working mothers are heads of households; two out of three working mothers report that they cannot decrease their working hours because of economic need. According to a 2004 AFL-CIO study, three in 10 working women reported making all or almost all of their families' incomes. Three in five earned half or more of their family's income.¹⁰⁷ The only families that realized an increase in their real incomes and living standards in the last two decades had two college-educated working parents. Three-fourths of these gains came from the longer hours women added to the labor force.¹⁰⁸

Myth 2. A working mother is unreliable, because her family is her primary concern.

Fact. Although some working mothers choose the "mommy track" (flexible and/or shorter work hours), others are forced into it by a lack of affordable quality day care. Seventy-five percent of mothers return to work within a year after childbirth.

Myth 3. Large numbers of working women leave the workforce to return home to raise their children.

Fact. Statistics show an opposite trend. In 1978, mothers of infants had a labor force participation rate of 36 percent; by 2011 that number rose to 64 percent.¹⁰⁹ The choice to become full-time homemakers is now the exception rather than the rule (see Figure 4.3).

Myth 4. Small businesses cannot afford to provide benefits and services such as child care, maternity leave, and flextime. For large businesses, these costs decrease their competitiveness.

Fact. Family assistance programs raise productivity, increase worker loyalty, decrease turnover, and curb absenteeism. The majority of employers report no change in costs owing to family leave legislation.¹¹⁰ An AFL-CIO study found that 64 percent of working women reported having no control over their work hours; 79 percent lacked child care benefits.¹¹¹

Myth 5. Women are doing better economically and rapidly closing the wage gap.

Fact. In 1979, women earned 64 percent of a male wage; by 2015, it was 79 percent, similar to the 77 percent in 1999.¹¹² At this rate, it would take about 100 years for the gender wage gap to close.

In 2012, approximately 57.7 percent of women were in the labor force. The unemployment rate for women in 2012 was 7.9 percent. Asian women had the lowest rate (6.1 percent), with white (7 percent), Hispanic (10.9 percent), and African American women (12.8 percent) following. From 1975 to 2012, the labor force participation rate of mothers with children under age 18 rose from 47 percent to 66 percent. Mothers with older children are more likely to be working than mothers with children under 6 years old. Unmarried mothers (75.8 percent) have higher participation rates than married mothers (59.5 percent). Only 20 percent of women were software developers and 31 percent lawyers, but 75 percent were involved in education and health services. Hispanic and African American women are more likely than white and Asian women to work in service occupations. Women who worked full time in 2012 had median weekly earnings of \$691. The average workweek for women has increased by about 2 hours since 1976.

FIGURE 4.3 Who Is in the Work Force?

Source: Bureau of Labor Statistics, *Women in the Labor Force: A Databook*, 2014.

Income and Job Disparities between Men and Women

Some of the income disparity between men and women can be traced to **occupational segregation**. Occupations such as cashier, receptionist, and home health aides are traditional female jobs in which 75 percent or more of the workers are women. But they are also typically among the lowest paying jobs. Women have gained ground in acquiring more managerial and professional positions, but the clerical and teaching fields are still among the most heavily female occupations.

The largest category of women workers in 2012 were teachers (3.2 million, excluding those in colleges and universities), and female secretaries and administrative assistants. About half of all working women are in pink collar ghettos where 80 percent of coworkers are women. In 2006, almost 97 percent of secretaries were women, as were 90 percent of bookkeepers, 89 percent of nurses, and 84 percent of librarians. In 2012, women still made up only 2 percent

of electricians, 6 percent of mechanical engineers, 12 percent of civil engineers, and 13 percent of police.¹¹³

The women with the highest earnings in 2012 were those employed as pharmacists, CEOs, lawyers, physicians and surgeons, legislators, and in financial services. Women's wages lag behind men's even in most female-dominated professions as Tables 4.4, 4.5, and 4.6 illustrate.¹¹⁴

Many women face extraordinary challenges in finding and securing employment at fair wages. Some of these obstacles include difficulty in finding high-quality and affordable day care, inadequate family leave policies, inflexible working conditions, poor or non-existent health insurance, and sexual harassment.

Day Care: A Barrier to Female Employment

A major barrier to female employment involves day care and subsidized child care leaves, which are a necessity for most working families. The

TABLE 4.4 Median Usual Weekly Earnings of Full-Time Wage and Salary Workers by Occupation and Sex, 2011 Annual Averages

Weekly Earnings Selected Occupations	Women	Men
Management, Professional, and Related Occupations	\$941	\$1,269
Management, Business, and Financial Operations	\$977	\$1,370
Service Occupations	\$433	\$551
Sales and Office Occupations	\$602	\$738
Office and Administrative Support Occupations	\$615	\$668
Natural Resources, Construction, and Maintenance Occupations	\$515	\$740
Installation, Maintenance, and Repair Occupations	\$751	\$807
Production, Transportation, and Material Moving Occupations	\$485	\$651
Number of Workers (in Thousands) Selected Occupations	Women	Men
Management, Professional, and Related Occupations	20,524	19,267
Management, Business, and Financial Operations	7,386	8,676
Service Occupations	6,991	7,387
Sales and Office Occupations	13,977	9,012
Office and Administrative Support Occupations	9,908	3,787
Natural Resources, Construction, and Maintenance Occupations	391	9,574
Installation, Maintenance, and Repair Occupations	146	4,013
Production, Transportation, and Material Moving Occupations	2,603	10,730

Source: Adapted from U.S. Bureau of Labor Statistics, "News Release: Usual Weekly Earnings of Wage and Salary Workers Fourth Quarter 2011," January 24, 2012. Retrieved February 2012, from www.bls.gov/news.release/pdf/wkyeng.pdf.

TABLE 4.5 Median Weekly Earnings, Selected Traditionally Female Occupations, 2014

Occupation	Earnings	
	Women	Men
Registered nurses	\$1,076	\$1,190
Elementary and middle school teachers	956	1,096
Cashiers	367	412
General office clerks	626	662
Food preparation and serving	419	463
Health support (non-nursing)	461	583

Source: Based on Bureau of Labor Statistics, Household Data Annual Averages, Median Weekly Earnings of Full-time Wage and Salary Workers by Detailed Occupation and Sex, 2014. Retrieved December 2015 from, www.bls.gov/cps/cpsaat39.pdf.

TABLE 4.6 Annual Median Earnings of Persons by Educational Attainment and Sex, Year-Round, Full-Time Workers, 2010

Level of Education	Women	Men
9th to 12th grade (non-graduate)	\$20,883	\$29,435
High school graduate	\$29,857	\$40,055
Some college, no degree	\$33,401	\$46,434
Associate degree	\$37,773	\$50,282
Bachelor's degree or more	\$51,942	\$71,778

Source: U.S. Census Bureau, "Income, Poverty, and Health Insurance Coverage in the United States: 2010," September 2011, Retrieved February 2012, from www.census.gov/prod/2011pubs/p60-239.pdf.

availability of day care has increased rapidly. By the early 1990s, there were three times as many child care centers as in the mid-1970s, and four times as many children were enrolled.¹¹⁵ By 2014, there were more than 580,000 child care centers (excluding family child care homes) offering 7.65 million spaces.¹¹⁶

The average annual fee for full-time child care varies dramatically from state to state and depends on the age of a child. In some of the more

TABLE 4.7 States with the Most and Least Expensive Child Care Costs, 2014

Most Expensive States	Four-Year-Old Child
Massachusetts	\$10,787
Wisconsin	10,524
New York	9,100
Colorado	9,765
Minnesota	9,350
Connecticut	9,111
Washington	8,804
Least Expensive States	
Mississippi	\$4,650
Arkansas	4,475
Louisiana	5,574
South Carolina	5,460
Alabama	4,888
Missouri	4,836
West Virginia	4,560

Source: Child Care Aware of America, 2015 Parents and the High Cost of Child Care Report.

expensive states like New York, the cost of yearly care for an infant was \$10,400 in 2015 and \$9,100 for a four-year-old. In Massachusetts, the cost was \$16,430 for an infant and \$10,787 for a four-year-old. In less expensive states like Mississippi, the cost for an infant and a toddler was \$4,650; in Louisiana it was \$5,574 for an infant and a toddler.¹¹⁷ These costs can put a severe financial strain on both single and two-parent families. As shown in Table 4.7, in 2014 a single mother in New York earning \$8.75 (the New York State minimum wage) an hour would have to spend half her salary just to pay for day care for one child.

First instituted in the 1970s, the Title XX Social Services Block Grant program is the largest federal program for child care services. In 1990, the U.S. Congress passed additional legislation that increased the supply of child care and expanded early childhood education. Through child care, development block grants, and amendments to Title IV-A of the Social Security Act, Congress provided new funds to help families with child care costs and to help states improve the quality and supply of these services.¹¹⁸

More than 120 countries provide paid maternity leave. *The United States is the only industrialized country in the world without paid maternity leave.*¹¹⁹ While the 1993 Family and Medical Leave Act (FMLA) allows for maternity leave, it is unpaid.

The American child care system is two-tiered: Those with means receive first-rate child care, or they can stay at home; those without the means are dependent on the ebb and flow of funding for publicly supported day care.¹²⁰

Other Obstacles Faced by Working Women

Many working mothers also require flexible family leave arrangements. Although 30 states had some form of parental or medical leave law on their books, no such law existed on the national level until 1993. With the election of former president Bill Clinton, the FMLA was rushed through Congress and was signed into law in 1993.

The FMLA requires public employers, and private employers with 50 or more workers to offer job-protected family or medical leave for up to 12 weeks to qualifying employees (i.e., who worked for the employer at least 1,250 hours in the previous year) who need to be absent from work for reasons covered by the law. These reasons include an employee's illness (including maternity-related) or the care for an ill family member. While the law does not mandate paid leave, it allows for health coverage to continue during the leave period. A worker who uses the FMLA is guaranteed the same or a comparable job upon returning to work. Many workers fail to take advantage of the FMLA because it is unpaid.

Another issue affecting working women is sexual harassment, which is defined as unwelcome sexual behaviors, including jokes, teasing, remarks, questions, and deliberate touching; letters, telephone calls, or materials of a sexual nature; pressure for sexual favors; and sexual assault. Although sexual harassment is illegal (Title VII of the 1964 Civil Rights Act has been interpreted as prohibiting sexual harassment), it remains all too common in the workplace.¹²¹

The battle for women's rights was also fought around **comparable worth**—the idea that workers should be paid equally for different types of work, if those jobs require the same levels of skill, education, knowledge, training, responsibility, and effort.

The desire to equalize incomes through comparable worth is based on the belief that the **dual labor market** has created a situation whereby “women's work” (e.g., secretarial, teaching, social work, nursing, child care) is valued less than traditional male occupations.

An illustration of the debate around comparable worth is provided in Table 4.8, which lists average wages for jobs typically occupied by women and men. Although controversial, 20 states have laws making comparable worth a requirement or goal of state employment. Nevertheless, comparable worth brings up a difficult question: What criteria should determine the comparability of different jobs? Some critics have rejected the idea of comparable worth on the grounds that the economic costs would be catastrophic.

In 1963, Congress passed the Equal Pay Act, requiring employers to compensate male and female workers equally for performing the same job under the same conditions (not all jobs were covered by the bill). Another protective measure was Title VII of the Civil Rights Act of 1964 that prohibited sex discrimination in employment and

TABLE 4.8 Comparable Worth and Average Annual Income, 2012

	2012 Mean Annual Income
Child care workers	\$ 19,560
Teacher assistants	23,640
Nursing aides	24,890
Hairdressers and cosmetologists	27,940
Preschool teachers	27,570
Home health aides	21,020
Social and human services assistants	28,850
Fitness trainers	55,638
Auto mechanics	36,610
Truck drivers	38,200
Maintenance and repair workers	35,210
Machinists	40,910
Auto body mechanics	35,000

Source: Based on U.S. Department of Labor, Bureau of Labor Statistics, “National Occupational Employment and Wage Estimates, All Occupations,” May 2012. Retrieved October 2015 from <http://data.bls.gov/oes/datatype.do#oes.f.5>.

provided the right of redress in the courts. In 1972, Presidential Executive Order 11375 mandated that employers practicing sex discrimination be prohibited from receiving federal contracts. Title IX of the Educational Amendments of 1972 prohibited discrimination in educational institutions receiving federal funds. Finally, the Equal Credit Act of 1975 prohibited discrimination by lending institutions on the basis of sex or marital status.

Abortion and Women's Rights

Feminists often point to the abortion issue as another area where women's rights are threatened. Pro-choice advocates argue that where abortion is concerned, some male legislators and judges have promulgated laws and regulations to control the behavior of women by denying them full reproductive freedom. They argue that the choice of an abortion is a personal matter involving a woman and her conscience. Anti-abortion forces claim that life begins at conception and that abortion is murder. Moreover, they point to the 1.06 million legal abortions performed in the United States in 2011, while at the same time pointing to the scarcity of adoptable infants.¹²²

In 1973, Sarah Weddington, a young attorney from Austin, Texas, argued *Roe v. Wade* before the U.S. Supreme Court. Though her client had long since relinquished her child for adoption, Weddington argued that a state could not unduly burden a woman's right to choose an abortion by making regulations that prohibited her from carrying out that decision. The Court ruled in favor of Weddington, and abortion was legalized in the United States, thereby nullifying all state laws that made abortion illegal during the first trimester of pregnancy. (Before 1970, four states—New York, Alaska, Hawaii, and Washington—had already made abortion legal contingent upon a physician's agreement.) Within a decade after *Roe v. Wade*, almost 500 bills were introduced in Congress, most of which sought to restrict abortions through a constitutional amendment outlawing abortion, by transferring the power to regulate abortion decisions to the states, or by limiting or terminating federal funding of abortions.¹²³

The abortion debate has been marked by Byzantine maneuvers and complex twists. In 1977, the Hyde Amendment prohibited the federal government from funding abortions except to save the life

of a mother. Also in 1977, the federal government lifted its ban on funding abortions for promptly reported cases of rape and incest and in cases where childbirth would result in severe and long-lasting harm to a woman. The 1980 Supreme Court decision in *Harris v. McRae* upheld the constitutionality of the Hyde Amendment. In 1981, the government again reversed its position, this time cutting federal funding of abortions except to save the life of the mother. By 1990, federal funds paid for only 165 abortions, a dramatic drop from the almost 300,000 federally funded abortions in 1977.¹²⁴ In 1994, the Freedom of Access to Clinic Entrances Act was signed into law that expanded the civil liberty of persons to have unobstructed access to any abortion or reproductive health services clinic. Any person or group that interferes with the right of such persons is subject to legal penalties, which range from a high fine to imprisonment.

The strategy of the anti-abortion movement has been to whittle away at *Roe v. Wade* by attempting to restrict abortions on the state level. For example, in a 1989 landmark decision, the Supreme Court upheld a Missouri law that prohibited public hospitals and public employees from performing an abortion (except to save the life of the mother), required physicians to determine whether a woman who is at least 20 weeks pregnant is carrying a fetus able to survive outside the womb, and declared that life starts at conception. In 1991, the Court ruled in *Rust v. Sullivan* that the United States can prohibit federally financed family planning programs from giving out abortion information (i.e., the gag rule). On former President Clinton's second day in office, he signed a bill overturning this "gag rule."

Despite the rise in population, the U.S. abortion rate dropped from 1.2 million in 2002 to less than 1 million in 2013. A substantial proportion of this was due to the use of emergency contraception. Medication abortions accounted for around 25 percent of all abortions performed before nine weeks gestation. Roughly 2 percent of American women aged 15 to 44 have an abortion in any given year. Thirty percent of women will have an abortion by age 45, 25 percent by age 30, and 8 percent by age 20. Sixty percent of women having an abortion already have a child, and more than 30 percent have two or more children.¹²⁵

Since 1973, abortion has proved to be one of the most divisive issues in American public life. While support for abortion has vacillated, a 2015 Gallup poll found that public opinion has shifted

with 50 percent of Americans identifying as pro-choice compared to 44 percent as pro-life.¹²⁶

The pro-life strategy has centered on imposing a web of restrictions on women, doctors, and clinics designed to discourage abortion. For instance, a 2011 Texas law required women seeking an abortion to make two visits to the abortion clinic, the first one was to receive an ultrasound. After the ultrasound, she has to wait 24 hours before getting an abortion. Another Texas law requires physicians to give women seeking abortions state-mandated information about medical risks, adoption alternatives, and developmental stages of the fetus. In 2009, North Dakota passed a similar law that required doctors to show an ultrasound to a pregnant woman before being going through with the procedure. Similarly, a bill was introduced in North Dakota that required an abortion provider to tell a woman considering an abortion that it “will destroy the life of an unborn child, a unique human being.” In 2012, the Virginia legislature passed a draconian law that would have required a woman seeking an abortion to have a transvaginal ultrasound before an abortion. She would also have been required to listen to the heartbeat of the fetus. After much state and national pressure, the governor refused to sign the bill, and it was referred back to the legislature. The revised bill was similar to the Texas legislation.¹²⁷

The most aggressive anti-abortion tactic came in the form of a “personhood amendment,” which defines a person as beginning at the moment the sperm fertilizes the egg. “Personhood” does not discriminate in terms of rape, incest, or saving the life of the mother, because the fertilized egg would have the same legal rights as the mother. This amendment would have not only made abortion illegal from the moment of conception, but also certain forms of birth control such as the morning after pill and intrauterine devices. Most physicians would likely abstain from performing in vitro fertilization because they feared criminal charges if an embryo doesn’t survive. Personhood referendums appeared in both Colorado and Mississippi and were defeated in both states. Proponents have vowed to return with a slightly modified amendment.¹²⁸ All of these laws are part of an unprecedented surge of anti-abortion legislation that has advanced through Republican-controlled legislatures since 2010.

Discrimination against women is problematic in much of the developed and developing world. For instance, women’s rights go from relatively high levels of protection in some Western nations to extreme

oppression and danger in others. Women also frequently face multiple forms of discrimination based on gender, religion, ethnicity, class or caste, and race.

Gender Discrimination and Violence in an International Context

One of the more sickening manifestations of gender violence occurred in ISIS controlled parts of Iraq where Yazidi and other women were sold as sexual slaves in marketplaces or given away to ISIS fighters as gifts. Women in ISIS-controlled territories were flogged, beaten, and killed for minor infractions. Yet another form of gender violence occurs in female genital mutilation (FGM), which involves injury or removal of parts of the external female genitalia for cultural or religious reasons. According to the World Health Organization, about 140 million girls and women worldwide are currently living with the consequences of FGM. This non-medical procedure is mostly carried out on young girls between infancy and age 15, and approximately 92 million girls 10 years of age and older have undergone FGM in Africa. This procedure (also practiced among some immigrant groups in industrialized nations) has no health benefits and can cause severe bleeding, urination problems, cysts, infections, infertility, childbirth complications, and the increased risk of post-natal deaths. Internationally recognized as a human rights violation, FGM has been documented in 28 African countries and several other countries in Asia and the Middle East (see Table 4.9).¹²⁹

Women face other forms of discrimination in patriarchal societies. For instance, Saudi Arabian women have fewer rights than men and face widespread discrimination. Their freedom of movement is restricted and often requires permission from a husband, son, or close male relative. For instance, Saudi women are not permitted to leave their neighborhood without the company of their husband or nearest male relative, nor are they permitted to drive cars. They require permission to travel by airplane, check into hotels, or rent apartments. Saudi Arabia has strict gender separation whereby women are forbidden to be in physical contact with unrelated males, therefore denying them unfettered access to employment, parks, museums, and libraries.¹³⁰ Discrimination is also evident in family law as Saudi children legally belong to their father. In a divorce or the death of a husband, the wife is granted custody

TABLE 4.9 Percentage of Girls and Women Aged 15 to 49 Who Have Undergone Female Genital Mutilation/Cutting (FGM/C), by Country (2004–2015)

Country	Estimated Prevalence of Female Genital Mutilation 15 to 49 Years (%)
Benin	9
Burkina Faso	76
Cameroon	1
Central African Republic	24
Chad	44
Côte d'Ivoire	38
Djibouti	93
Egypt	87
Eritrea	83
Ethiopia	74
Gambia	75
Ghana	4
Guinea	97
Guinea-Bissau	45
Iraq	8
Kenya	21
Liberia	50
Mali	89
Mauritania	69
Niger	2
Nigeria	25
Senegal	25
Sierra Leone	90
Somalia	98
Sudan	87
Tanzania	15
Togo	5
Uganda	1
Yemen	19

Source: United Nations Children's Fund, *Female Genital Mutilation /Cutting: A Global Concern*, UNICEF, New York, 2016. Retrieved March 2016 from http://data.unicef.org/corecode/uploads/document6/uploaded_pdfs/corecode/FGMC-2016-brochure_250.pdf.

of daughters until the age of 9 and sons until age 7. Older children are typically awarded to the father or the paternal grandparents. Although Saudi women can inherit from family members, their share is generally smaller than for men.¹³¹

One of the most extreme examples of gender oppression took place in Afghanistan from 1996 to 2001, when the ruling Taliban government implemented a strict interpretation of Sharia law. Before the Taliban, Afghanistan was under Russian influence, and gender discrimination—especially in urban areas—was less pronounced. Before the Taliban, women constituted 70 percent of school teachers, 50 percent of government workers, 60 percent of university teachers, 50 percent of students at Kabul University, and 40 percent of doctors.

By 1996, the Taliban had excluded women from government and public life. It implemented a severe system of gender apartheid that severely restricted women's access to education, health care, and employment. Women were effectively put under house arrest and prohibited from working or attending school, and they could not leave their homes without being accompanied by a husband or close male relative. The windows of their homes were painted, and they could only appear in public wearing the sanctioned full-body covering. Those with no male relatives were trapped in their homes. Moreover, women could not be heard and were beaten for showing a bit of ankle or wearing noisy shoes. They could not speak in public or to men who were not relatives; they were beaten or killed for minor infractions of these rules.

Because female doctors were generally prohibited from employment, and male doctors were forbidden to treat women, most Afghan women had no access to basic medical care, which led to a dramatic increase in preventable deaths. Although a few female doctors were allowed to work in a handful of female-only hospitals, most eventually fled the country. Those who stayed could not operate properly while wearing the burqa.

Women accused of prostitution or infidelity were hung in public squares or stoned to death. Those accused of homosexuality were buried alive. Brothels proliferated as educated women had few means of survival. Not surprisingly, the Taliban emphasis on modesty did not preclude the use of rape to terrorize the population and dishonor entire villages. Suicide rates and depression for women and girls skyrocketed under these conditions.¹³²

The U.S. invasion that overthrew the Taliban in 2001 raised hopes that Afghani women would soon

regain human rights. Although women now have greater rights than before, the government did not overturn all of the Taliban's edicts. Nor have they been able to provide the necessary security for women to attend school and participate in public life, especially in Taliban-controlled areas. In some areas, young girls who tried to attend school had acid thrown in their faces, or in the case of Malala Yousafzai, were shot because they spoke up for the right of girls to be educated.¹³³

Yet another form of gender discrimination and oppression occurs through trafficking—typically for sexual purposes—of women or young girls (or boys) within or outside of a specific country. These women or girls are commonly recruited from poor, oppressed, or stigmatized communities; and are often homeless or drug addicts. Tourists, widows, and job-seekers are also falsely lured abroad by promises of a steady income and a better life.

Traffickers may promise citizenship, employment, marriage, education, and a better quality of life. They may promise high wages that can be sent home to families or children. They may promise domestic employment with sufficient income to allow the victim to save money for education or buy a home in their native country. Or, they may threaten to harm or kill the women's family in their home country if they do not comply. In the end, victims are forced to become prostitutes or do other work in the sex industry. Domestic servants can be held in involuntary servitude with no pay, no passport, and no means of escape. They are sometimes sexually abused by their employers or lent out as prostitutes to acquaintances.

International trafficking in human beings requires complex business and organizational arrangements that include travel agents to arrange transit, brokers, agents to secure immigration clearances, real estate agents to find the property, accountants to ensure the business appears legal, and overseers to ensure the business functions smoothly. Although the exact number of victims of sex trafficking is unknown, it could be in the hundreds of thousands worldwide.¹³⁴

Gays and Lesbians: Two Populations at Risk

In today's culture wars, one of the most intense controversies surrounds the issue of whether homosexuality is an acceptable lifestyle and whether those who are openly gay and lesbian should enjoy

protected status under civil rights and all civil laws. Forty years ago, few people in public or religious life would have dared to raise the possibility that it is acceptable to be gay. In 1960, all 50 states had laws criminalizing (on a felony level) sodomy by consenting adults.

In recent decades, there has been a slow but dramatic shift in public attitudes toward homosexuality. In 1993, after years of concerted pressure by gay activists, the American Psychiatric Association removed homosexuality from its *DSM-III* list of "objective disorders" and declared it "a normal, if divergent lifestyle." Laws forbidding sodomy were repealed in state after state throughout the 1970s and 1980s. Although there is no federal law against discrimination based on sexual orientation by private sector employers, 14 states, the District of Columbia, and over 140 cities and counties have enacted antidiscrimination laws. Many of these laws also ban discrimination in housing and public accommodation.¹³⁵ In 2003, the U.S. Supreme Court in *Lawrence v. Texas* ruled Texas' sodomy statute was unconstitutional, which then invalidated any remaining state sodomy laws. In 2008, the California Supreme Court struck down two state laws that had limited marriage to be only between a man and a woman, making California the second state after Massachusetts to allow same-sex marriages.¹³⁶ These licenses were halted due to the passage of Proposition 8 that eliminated the right of same-sex couples to marry in California. Proposition 8 was later ruled unconstitutional by the courts.¹³⁷

Despite these successes, many homosexuals continue to be forced to live in the closet, concealing their sexual orientation in order to survive in a hostile world. Often the objects of ridicule and bullying, homosexuals have been denied housing and employment, harassed on the job, assaulted, and even killed because of their sexual orientation. Discrimination and violence remains commonplace.

In his 1999 State of the Union address, former President Clinton referred to the murder of Matthew Shepard: "We saw a young man murdered in Wyoming just because he was gay."¹³⁸ In 2009, former President Barack Obama signed into law the Matthew Shepard and James Byrd, Jr. Hate Crimes Prevention Act (known as the Matthew Shepard Act), which expanded the 1969 federal hate-crime law to include crimes motivated by a victim's actual or perceived gender, sexual orientation, gender identity, or disability. This is also

the first federal law to extend legal protections to transgender persons. There were 5,928 hate crimes reported in 2013. Of that number, 21 percent (1,253) were based on sexual or gender identity.¹³⁹ Represented in all occupations and socioeconomic strata, gays and lesbians make up anywhere from 1 to 10 percent (the original 1953 Kinsey Report) of the population. A Centers for Disease Control and Prevention study released in 2014 reported that less than 3 percent of Americans identify as gay, lesbian, or bisexual. It was the first large-scale government survey measuring Americans' sexual orientation. Other studies have pointed to higher figures at around 4 percent.¹⁴⁰

One group often overlooked is the transgendered population. For transgendered people, the gender identity assigned at their birth differs from their real gender identity. Gender identity is based on one's internal sense as to whether they are a man, woman, or have no gender identity per se. People in this community may describe themselves as transgender, transexual (implying that they may surgically change their gender), or genderqueer (identifies with neither gender, both genders, or a combination of genders). Through altering their physical appearance, many transgender people seek to align their bodies with their true gender identity. They may also change their name to better reflect their real gender identity. One of the most visible transgender celebrities has been Caitlyn Jenner of the Kardashians. Transgender people can be gay, straight, bisexual, or asexual (i.e., they have no sexual feelings or desires).¹⁴¹

Transgender people face high levels of discrimination and violence. The average life expectancy for a black transgender woman is 35 years old. In 2013, 72 percent of anti-LGBT homicide victims were transgender women.¹⁴² A survey by the National Center for Transgender Equality and the National Gay and Lesbian Task Force found that 63 percent of participants had experienced a serious act of discrimination, including:

- Losing job due to bias
- Eviction due to bias
- School bullying/harassment resulting in dropping out
- Teacher bullying
- Physical and sexual assault
- Homelessness due to expressions of gender identity
- Termination of relationships with partner or children due to gender expression

- Denial of medical service due to bias
- Incarceration due to gender identity expression

Twenty-three percent of respondents experienced catastrophic discrimination—that is, they experienced at least three of the above life-disrupting events. Overall,

- Transgender people are four times more likely to live in poverty and experience twice the rate of unemployment as the general population; people of color have four times the national unemployment rate.
- 90 percent of respondents reported harassment, mistreatment, or discrimination on the job.
- 22 percent of respondents who interacted with police reported police harassment; much higher rates of harassment were reported by people of color. Forty-six percent of respondents were uncomfortable asking for police assistance.
- 41 percent of respondents reported attempting suicide compared to 1.6 percent of the general population.¹⁴³

One question around transgender issues involves bathroom use. Specifically, should transgender people be allowed to use the restrooms where they feel the most comfortable? Is it discrimination when they are forced to do otherwise? Should there be separate transgender bathrooms? In 2013, California became the first state to allow transgender public school students to access whichever restroom and locker room they choose. By mid-2015, 17 school districts representing 600,000 students had adopted non-discrimination laws to protect transgender students. Thirteen states and the District of Columbia have adopted laws that prohibit discrimination on the basis of gender identity and sexual orientation in public schools.¹⁴⁴

Conservatives have introduced “bathroom bills” that require people to use the bathroom that matches their officially assigned gender. They argue that these bills are necessary to protect privacy and public safety. They further argue that allowing transgender women to use a women's bathroom would



Watch this video for a debunking of many of the arguments used by opponents of nondiscrimination bills who target transgender bathroom use.

www.youtube.com/watch?v=wCAWnbj9shc

invite sexual predators or curious teenage boys who would use this as a ruse to enter female spaces.¹⁴⁵ An alternative that is being more widely adopted in some public spaces and universities is gender-neutral bathrooms, which have been in use in Europe for decades.

Gay Rights

The self-perception of gays and lesbians has undergone a dramatic shift since the 1960s. Gays and lesbians have begun to identify as members of an oppressed minority, similar to other oppressed minority groups. As gays and lesbians have become more visible, they have organized support groups; religious groups, social service organizations; sub-chapters of professional associations; and political action groups. The political power of gays and lesbians had grown to the point that by 1984, activists were successful in inserting a gay civil rights plank in the Democratic Party platform.

However, when gays and lesbians have demanded equal rights under the law, the result has been mixed—and often negative on a statewide level. During the 1970s, Miami gays tried to pass a civil rights amendment that would have prevented discrimination based on sexual orientation. This referendum failed as did similar efforts in St. Paul and other cities. Houston voters defeated two gay rights proposals in 1986 and another in 2015. In Tampa and Portland, Maine, voters in the 1990s overturned city ordinances protecting gays and lesbians.¹⁴⁶ Conversely, 13 states had antidiscrimination laws by the late 1990s. Municipalities across the country have also adopted civil rights ordinances to cover lesbians and gay men in areas such as employment, public accommodations, housing, and credit.¹⁴⁷

The most recent attacks on gay rights came in the form of rights and religious tolerance issues. In 2014, the Arizona legislature passed a right-to-discriminate measure that would have allowed business owners to discriminate against LGBT customers. Facing boycott threats, Jan Brewer, the former Arizona governor, vetoed it. In 2015, Indiana's Republican Gov. Mike Pence signed a controversial "religious freedom" bill (Religious Freedom Restoration Act or RFRA) that sanctioned discrimination against LGBT people by prohibiting any governmental entity from burdening a person's religious beliefs, unless they can prove it is otherwise furthering a compelling governmental interest. Fearing

a boycott, the Indiana state legislature passed a "fix" that softened the bill. The Arkansas Senate approved a similar bill aiming to protect religious beliefs. Michigan Republican Gov. Rick Snyder signed a law that allowed any faith-based adoption agency to refuse to place children with same-sex couples if it violated their religious beliefs. That such measure was seen was proffered in conservative statehouses across the nation.¹⁴⁸

Gays and Lesbians in the Military

The U.S. armed forces have generally not tolerated homosexuality in their ranks. Draft board physicians in 1940 were ordered to screen out homosexuals on the basis of such characteristics as a man's lisp or a woman's deep voice. Because World War II created a desperate need for soldiers, these instructions were often overlooked. After the war, however, gay and lesbian military personnel were discharged, and exclusionary policies were again enforced. A similar situation occurred during the Vietnam War; once the conflict ended, gays and lesbians were again persecuted and their careers terminated.¹⁴⁹ The policy that excluded gays and lesbians from military service read as follows: "Homosexuality is incompatible with military service. The presence in the military environment of persons who engage in homosexual conduct or who, by their statements, demonstrate a propensity to engage in homosexual conduct, seriously impairs the accomplishment of the military mission."¹⁵⁰ Those who admitted to homosexuality at the time of enlistment were rejected; if homosexuality came to light later on, the individual was separated from the military. Although more liberal policies were enacted in the 1970s and 1980s, they were directed at the type of separation, not the legitimacy of it.¹⁵¹

Former president Bill Clinton promised to end discrimination against homosexuals in the military. As one of his first official acts, he signed an order prohibiting military recruiters from inquiring about the sexual orientation of potential recruits. Shortly afterward, he proposed wide-ranging reforms aimed at ensuring equal rights for gays and lesbians serving in the military. Facing criticism from the Joint Chiefs of Staff and many influential members of Congress, Clinton retreated and instead supported a "Don't Ask, Don't Tell, Don't Pursue, and Don't Harass" (DADT) policy that prohibited the military from asking questions as long as homosexuals behaved discreetly. Under this rule, service people

could be discharged if they state they are gay, lesbian, or bisexual or make a statement that indicates a propensity to engage in a homosexual act.

A 1999 report by the Servicemembers Legal Defense Network (SLDN), a gay rights group, noted that in the first five years of the DADT policy, there was a 92 percent increase in gay-related discharges. The Log Cabin Republicans claim that 13,000 men and women have been expelled from the military since the DADT rule came into effect. In a momentous event, DADT was repealed on September 20, 2011. In turn, the Department of Defense deleted the statutes around “homosexual conduct” being grounds for administrative separation. The acceptance of gays and lesbians in the military puts the United States in line with other countries, including Australia, Austria, Belgium, Canada, Czech Republic, Denmark, Estonia, Finland, France, Germany, Ireland, Israel, Italy, Lithuania, Luxembourg, Netherlands, New Zealand, Norway, Slovenia, South Africa, Spain, Sweden, Switzerland, United Kingdom, and Uruguay.

Gay and Lesbian Family Life

A long-standing issue for gays and lesbians has been the recognition of gay unions. Same sex marriage (SSM) became an issue in 1996 when it was feared that Hawaii would recognize these marriages. Conservative states feared that same-sex couples would relocate to a state allowing gay marriages and then move back to their state and demand it be recognized. By 1996, one-third of states had passed legislation prohibiting the recognition of SSMs from another state. The federal government entered the fray by passing the Defense of Marriage Act (DOMA), which effectively established that the only legitimate marriage was with a member of the opposite sex.

In 1999, the Vermont Supreme Court found no compelling reason to not recognize a civil union for same-sex couples and ruled that they be granted the same rights and responsibilities as married Vermonters. The Vermont governor signed into law a “civil union” bill in 2002 that granted Vermont gay and lesbian couples most of the rights and benefits available to married couples under the law, including insurance benefits, inheritance, medical decision-making rights, and state tax benefits and obligations.¹⁵² The one exception was the right to have a same-sex union legally called a marriage. Civil unions were criticized because they created a separate and thereby unequal set of rights.¹⁵³

SSM’s were performed in California for a short time in 2008 after the California Supreme Court ruled that limiting marriage to opposite-sex couples violated the state constitution. But in n 2008 the California electorate approved Proposition 8, which made a gay marriage ban part of California’s constitution. The legality of Proposition 8 was battled out in the courts.

The trend to legalize SSM’s was worldwide. The Netherlands became the first country to recognize same-sex marriages (SSM) in 2001. Belgium followed suit. By 2015, the United Kingdom, France, Argentina, Brazil, Canada, Finland, Iceland, Norway, Portugal, South Africa, Spain, New Zealand, and Sweden had legalized gay marriages.

In June 2015, the U.S. Supreme Court ruled in a 5 to 4 decision that states cannot keep same-sex couples from marrying and must recognize their unions. Although a monumental victory for



A long-standing issue for gays and lesbians has been the recognition of gay unions.

Source: Lisa F. Young/Shutterstock

gay rights, 37 states, including Connecticut, Iowa, Massachusetts, New Hampshire, New York, Vermont, Washington, New Mexico, Maryland, California, Oregon, Nevada, and Arizona had already legalized gay marriage by 2015. Moreover, a 2015 CBS News-New York Times Poll found that 54 percent of Americans supported marriage equality, while 55 percent of Republicans opposed it.

Not unexpectedly, the Supreme Court ruling opened a floodgate of fear, dire warnings, and a threat of anti-SSM state legislation. All major 2016 Republican presidential candidates were opposed to the decision. Conservative state legislatures debated ways to circumvent the law. Conservative heroes emerged. The owners of Memories Pizza (near South Bend, Indiana) closed their doors after declaring they would not cater pizzas to gay weddings, something which undoubtedly was a devastating blow for gay Indiana couples planning the perfect wedding. Within days Memories Pizza received \$840,000 in contributions and was back open. Masterpiece Cakes owner Jack Phillips used his Christian beliefs and religious freedom to justify why he would not make a cake for a gay wedding. The Colorado Court of Appeals thought otherwise and required that he comply with the law. In particular, he was prohibited from refusing to serve customers based on their sexual orientation if he wants to remain open to the public.¹⁵⁴

However, the most visible opponent of the Court's ruling was Kim Davis, County Clerk for Rowan County, Kentucky. As county clerk, Davis refused to issue marriage licenses to gay couples and defied a federal court order requiring her to follow the U.S. Supreme Court decision. Davis claimed that she was acting under "God's authority." To buffer her position, she also refused to issue marriage licenses to straight couples. Davis was jailed for contempt of court but released five days later. After returning to work, she stated that she would not interfere with her deputies who were issuing licenses. Not surprisingly, conservative politicians and religious leaders rushed to her defense and she even met the Pope when he visited the United States.¹⁵⁵

The question of parental rights is linked to the recognition of same-sex unions. Specifically, sexual orientation has often been a factor in issues of family law, including custody, visitation, and foster/adoptive parent eligibility. Since homosexuality is not a constitutionally protected status, it can be considered in legal actions surrounding the rights

of parents and would-be parents. In some jurisdictions, it effectively blocks adoption and foster care placement. Although most states have no statute explicitly prohibiting gay and lesbian adoption or foster care placement, several states unofficially do not permit it.¹⁵⁶

AIDS and the Gay Community

The AIDS crisis has been used to justify homophobic attitudes. In the early days of the epidemic (i.e., the 1980s), there were recommendations for quarantining AIDS victims, renewed attempts at punishing homosexual behavior, increased job discrimination, and a generally hostile climate for both gays and lesbians. At the federal level, former senator Jesse Helms successfully introduced a bill that prevented the Centers for Disease Control and Prevention from using AIDS education funds in ways that could foster homosexuality. Although in the U.S. AIDS still predominantly affects gay and bisexual men, intravenous drug users, and African American women, the movement of AIDS into the heterosexual community has fueled some sympathy around the gay population. But positive change has come only after the documentation of more than a million AIDS cases in the United States, resulting in about 658,000 deaths.¹⁵⁷

More than 1.2 million Americans are living with HIV infection, and almost 1 in 8 (12.8%) are unaware of their infection. Gay and bisexual men (particularly young African Americans) are the groups most impacted by the almost 50,000 new HIV cases detected each year.¹⁵⁸ The AIDS crisis has had a devastating effect on the lives of gay men, and there are few older gays in larger cities who have not lost either a lover or close friends to the disease.

Homophobia—the irrational fear of homosexuality—continues to limit the civil rights and legal protections of gays and lesbians. Justifications for this attitude have been found in religious texts that view homosexuality as a sin against God, and in psychological explanations that view homosexuality as a disease or symptom of arrested development or a fear of intimacy. Some forms of Christian counseling are based on "praying away the gay," or reparative/conversion therapies that claim clients can learn to be attracted to the opposite sex and rid themselves of gay urges through prayer and effort. The American Psychological Association in 2009 claimed that trying to change sexual orientation

is both ineffective and potentially harmful.¹⁵⁹ By 2015, four states (California, New Jersey, Illinois, and Oregon) and the District of Columbia have banned the use of conversion therapy on minors.

As gays and lesbians have become more visible, they have organized political action organizations and religious groups such as Dignity (Roman Catholics), Integrity (Episcopalians), Mishpachah Am (Jews), and Lutherans Concerned. This has encouraged some political leaders to openly court the gay vote. The Clinton administration was the first to openly (albeit erratically) endorse gay rights. For example, Clinton backed the Employment Non-Discrimination Act (a gay civil rights bill), appointed more than 100 openly gay and lesbians to administrative jobs, nominated the first-ever lesbian to the U.S. District Court, and mandated all federal agencies to add sexual orientation to their affirmative action policies. He also stopped the denial of security clearances based on sexual orientation; granted political asylum to gays and lesbians at risk of persecution in their home countries; and increased public health spending on AIDS.¹⁶⁰ Unfortunately, much of this progress was reversed in the two terms of the Bush administration.

Lesbian, gay, bisexual, and transgendered rights are an area of worldwide discrimination. On the positive side, seven of the 10 countries that have legalized same-sex marriage are in Europe, where another 14 have legalized civil unions. The laws of the European Union guarantee equal treatment in employment and vocational training, regardless of one's sexual orientation. EU policies and regulations not only guarantee equality and non-discrimination for all LGBT people, they also require EU member states to proactively fight discrimination. In general, the gay and lesbian community is likely to enjoy far greater rights and protections in the European Union than in many other parts of the world.¹⁶¹

Ageism

Ageism is a problem in a consumer-oriented society that idolizes youth. Like other minority groups, the aged face significant social and economic barriers. For example, workers over 50 often find it difficult to find equivalent employment if they lose their jobs, something that received wide publicity since the recession of 2007 to 2008. The aged in the United States are not culturally respected for their

wisdom and experience nor do they occupy high social positions protected by tradition. Instead, once they lose their earning potential, the aged are often viewed as financial albatrosses around the neck of an economically productive society. Socially isolated in retirement communities or low-income or senior housing, the aged often become invisible.

America is aging:

- The population 65 and over has increased from 35.9 million in 2003 to 44.7 million in 2013 (a 24.7 percent increase) and is projected to more than double to 98 million in 2060.
- Between 2003 and 2013, the population age 60 and over increased almost 31 percent from 48 million to almost 63 million.
- The 85+ population is projected to triple from 6 million in 2013 to 14.6 million in 2040.
- The number of Americans aged 45 to 64 who will reach 65 over the next two decades increased by almost 21 percent between 2003 and 2013.
- In 2013, one in every seven people (14 percent of the population) is an older American.
- People reaching 65 have an average life expectancy of an additional 19.3 years (20.5 years for females and almost 18 years for males).¹⁶²

This aging trend has significant consequences for health care costs, since disabilities and illness increases with age. For example, while only about 20 percent of people aged 65 to 74 need help with daily activities such as eating, bathing, and dressing, 53 percent of those over age 85 need assistance with daily activities.¹⁶³ Although only about 5 percent of all elderly people live in nursing homes, these numbers vary dramatically with age.¹⁶⁴

Elderly Poverty and Social Programs

Until recently, the proportion of older people living in poverty and low income had been declining. In 1959, 35 percent of people aged 65 and over lived below the poverty line; in 2013, it was 9.5 percent. The 9.5 percent represented an increase from 9.1 percent in 2012. Although not officially classified as poor, another 2.5 million or 5.6 percent of older adults were near-poor, with incomes above the poverty level but below 125 percent of it. The freezing of Social Security cost-of-living adjustments in 2015 will likely further impact the rising poverty rate among seniors.¹⁶⁵ Poverty rates differ by age and sex in the older population. Older women

(11.6 percent) were more likely to live in poverty than older men (6.6 percent). Race and ethnicity are also related to poverty among the elderly. About 7.4 percent of older whites were poor in 2013 compared to 17.6 percent of older African-Americans, 13.6 percent of older Asians, and almost 20 percent of older Hispanics. There were higher poverty rates in 2013 for older persons who lived within large cities (12.9 percent) and in southern states (11 percent).¹⁶⁶

In 2013, the median annual income was \$29,327 for males and \$16,301 for females. Median income of all households headed by older people rose a statistically significant 3.7 percent from 2012 to 2013. About 6 percent of family households with an older adult householder had incomes less than \$15,000, while 70 percent had incomes of \$35,000 or more. The major sources of income for older persons in 2012 were Social Security, income from assets, private pensions, government employee pensions, and earnings. In 2012, Social Security benefits accounted for 35 percent of the aggregate income of the elderly. The bulk of the remaining income was earnings (34 percent), asset income (11 percent), and pensions (17 percent). Social Security accounted for 90 percent or more of the income received by 36 percent of beneficiaries.¹⁶⁷ Other than the very poor, seniors are the only group with universal health coverage and the only group to receive non-means-tested government assistance. As such, policymakers must calculate the impact of increasing numbers of minority elderly in determining the future funding needs of health and income-based social programs.

Health Care and the Elderly

Aging trends have significant implications for social welfare policy. For one, as the numbers of elderly increase, their demands on society for housing, health, and recreational services become greater. The stresses put on the health care system by an increasingly aging population are already evident in the financial solvency of the Medicare trust funds. For instance, older (age 80 and above) Medicare beneficiaries comprised 24 percent of the recipients in 2011 but accounted for 33 percent of total Medicare spending. In contrast, Medicare recipients between 65 and 69 comprised 26 percent of the program's population but accounted for only 15 percent of total Medicare spending. Average Medicare per capita spending in 2011 more than doubled between age 70 (\$7,566) and age 96 (\$16,145).¹⁶⁸

While elderly women live longer than men, they have only about 57 percent of their income. Hence, more of elderly women's health care costs will have to be borne by the government or by their families, putting increasing burdens on both. In addition, as people live longer and require more care—especially in-home care—more pressure will be put on family members to provide or pay for that care. The government may be pressed to provide even more care for greater numbers of the elderly since families are already stressed by increasing workloads, myriad family pressures, and the often large geographic distances separating children and parents.

The elderly vote in large numbers, and over the years their voices have been heard more clearly by politicians than those of minorities. In the 1960s, policymakers responded to the needs of the elderly by passing the Older Americans Act (OAA) of 1965. The objectives of the OAA included (1) an adequate retirement income that corresponds to the general standard of living; (2) the promotion of good physical and mental health, regardless of economic status; (3) the provision of centrally located, adequate, and affordable housing; (4) the availability of meaningful employment, with the elimination of age-specific and discriminatory employment practices; (5) the provision of civic, cultural, and recreational opportunities; and (6) adequate community services, including low-cost transportation and supported living arrangements.¹⁶⁹

People with Disabilities

People with disabilities also experience the effects of discrimination (see Tables 4.10 and 4.11). In 2014, 17.1 percent of householders aged between 18 and 64 reported having a disability.¹⁷⁰

TABLE 4.10 Disabled Persons, Aged 21 to 64

Disability	Percent Employed*
Difficulty hearing	68.1
Difficulty seeing	55.3
Mental disability	47.2
Difficulty walking	40.8

*Persons may have more than one type of disability.

Source: Based on U.S. Census Bureau, Americans with Disabilities: 2010, Washington, DC: U.S. Department of Commerce, Economics, and Statistics Administration, October 2015. Retrieved November 2015 from <http://www.census.gov/prod/2010pubs/p70-107.pdf>.

TABLE 4.11 Characteristics of the Civilian Noninstitutionalized Population by Age, Disability Status, and Type of Disability, 2010

	Number (in Thousands)	Percent
Total all ages	303,858	100
With any disability	66,672	18.7
With severe disability	38,284	12.6
Total population 6 years and older	278,222	100.00
Needed personal assistance	13,349	4.4
Total population 15 years and older	241,682	100
With any disability	451,454	21.3
Visual	10,087	4.1
Hearing	8668	4.1
Mental	14,329	6.4
Total population 25 to 64 years	177,925	100
With any disability	29,479	16.6
Severe disability	20,286	11.4
Total population 65 years and older	38,599	100
With any disability	19,234	49.8
With a severe disability	14,138	36.6

Source: Based on Matthew W. Brault, *Americans With Disabilities: 2010*, U.S. Census Bureau, P70-131, July 2012. Retrieved November 2015 from Current Population Reports <http://www.census.gov/prod/2012pubs/p70-131.pdf>.

Disability is a difficult term to define. One medical definition is based on the assumption that a disability is a chronic disease requiring various forms of treatment. Another definition derived from the medical model—a definition used as a basis for determining eligibility in the Social Security Disability Insurance program—characterizes people with disabilities as those unable to work (or unable to work as frequently) in the same range of jobs as non-disabled people.¹⁷¹ People with disabilities are viewed as inherently less productive than other members of society. A third model defines disability based on what those with disabilities cannot do, seeing the disabled in terms of their inability to perform certain functions expected of the able-bodied population. As

William Roth maintains, “the functional limitation, economic, and medical models all define disability by what a person is not—the medical model as not healthy, the economic model as not productive, the functional limitation model as not capable.”¹⁷²

A newer definition—the psychosocial model—views disability as a socially defined category. In other words, people with disabilities constitute a minority group, and if a person with disabilities is poor, it is because of discrimination rather than personal inadequacy. This definition locates the problem in the interaction between disabled people and the social environment. Therefore, the adjustment to disability is not a personal problem but a social challenge requiring the adjustment of society. This definition requires that society reevaluates its attitudes and remove the physical and transportation barriers placed in the way of people with disabilities. It also requires the elimination of stereotypes. In part, this newer definition of disability was expressed in Section 504 of the Rehabilitation Act of 1973 (PL 93-112).

Although the potential range of disabilities is vast, people with disabilities share a central experience rooted in stigmatization, discrimination, and oppression. Like other stigmatized groups, those with disabilities experience poverty in numbers disproportionately larger than the general population. Perhaps not surprisingly, rates of disability are greatest among the aged, African Americans, the poor, and blue-collar workers.¹⁷³ Compared to the able-bodied, people with disabilities tend to be more frequently unemployed and underemployed and, as a consequence, often fall below the poverty line. In 2012, the median income of households containing a person with a disability was \$37,300, which is only 61 percent of households without a person with a disability (\$60,600).¹⁷⁴ Because disability is often correlated with poor education, age, and poverty, it is not surprising that African Americans are twice as likely as whites to have some level of disability (their representation is even greater in the fully disabled population) and that more women have higher instances of disabilities than men. The problems of low wages and unemployment are exacerbated because people with disabilities often need more medical care than others, may be less likely to have adequate health insurance, and depending on the disability they spend more of their own money on medical care than the able-bodied. The uninsured rate for

people with a disability increased from 16 percent in 2009 to 17.3 percent in 2010.¹⁷⁵

The greatest stride in disability rights occurred on July 26, 1990, when former president George Bush signed the Americans with Disabilities Act (ADA) (PL 101-336) into law (see Figure 4.4). This act is the most comprehensive legislation for people with disabilities ever passed in the United States. The ADA lays a foundation of equality for people with disabilities, and it extends to disabled people civil rights similar to those made available on the basis of race, sex, color, national origin, and religion through the Civil Rights Act of 1964. For example,

the ADA prohibits discrimination on the basis of disability in private sector employment; in state and local government activities; and in public accommodations and services, including transportation provided by both public and private entities.¹⁷⁶

In spite of some serious loopholes, the ADA represents an important step forward for disabled people. Nevertheless, some argue that while the ADA is a good law in principle, abuses are stirring up widespread resistance. Specifically, disability is often defined so broadly that virtually anyone with a problem, regardless of its extent, can claim protection under the ADA. Despite the ADA and other

I. Employment

- A. Employers may not discriminate against an individual with a disability in hiring or promotion if the person is otherwise qualified for the job.
- B. Employers can ask about one's ability to perform a job but cannot inquire if someone has a disability nor can employers subject a person to tests that tend to screen out people with disabilities.
- C. Employers must provide "reasonable accommodation" to employees with disabilities. This includes job restructuring and modification of equipment. Employers are not required to provide accommodations that impose an "undue hardship" on business operations.
- D. All employers with 15 or more employees must comply with the ADA.

II. Transportation

- A. New public transit buses and rail cars must be accessible to individuals with disabilities.
- B. Transit authorities must provide comparable para-transit or other special transportation services to individuals with disabilities who cannot use fixed bus services, unless an undue burden would result.
- C. Existing rail systems must have one accessible car per train.
- D. New bus and train stations must be accessible. Key stations in rapid, light, and commuter rail systems must be made accessible. All existing Amtrak stations must be accessible by July 26, 2010.

III. Public Accommodations

- A. Private entities such as restaurants, hotels, and retail stores may not discriminate against individuals with disabilities.
- B. Auxiliary aids and services must be provided to individuals with vision or hearing impairments, unless an undue burden would result.
- C. If removal is readily achievable, physical barriers in existing facilities must be removed. All new construction and alterations of facilities must be accessible.

IV. State and Local Government

- A. State and local governments may not discriminate against individuals with disabilities.
- B. All government facilities, services, and communications must be accessible, consistent with the requirements of Section 504 of the Rehabilitation Act of 1973.

V. Telecommunications

- A. Companies offering telephone service to the general public must offer telephone relay services to individuals who use telecommunications services for the deaf (TDDs) or similar devices.

FIGURE 4.4 A Summary of the Americans with Disabilities Act

Source: Fact Sheet, The Americans With Disabilities Act of 1990. Retrieved from <http://www.empowermentzone.com/adafacts.txt>.

federal laws, discrimination is still widespread against people with disabilities. For instance, most buildings still do not meet the needs of the physically handicapped in terms of access, exits, restrooms, parking lots, warning systems, and so forth. Many apartment complexes and stores continue to be built without allowing for the needs of people with disabilities. The struggle for full social, political, and economic integration of the disabled remains an ongoing battle.

Legal Attempts to Remedy Discrimination

Attempts to eliminate discrimination are a relatively recent development. Although the Fourteenth Amendment of the Constitution guaranteed all citizens equal protection under the law, it was also used to perpetuate discrimination on the basis of “separate but equal” treatment. Overt segregation existed in the South until the middle of the twentieth century, and separate but (supposedly) equal public facilities characterized much of the social and economic structure of the southern states. The extensive system of segregation encompassed public transportation, schools, private economic activities, and even public drinking fountains. In the mid-1950s, the U.S. Supreme Court overturned the *Plessy v. Ferguson* (1896) decision that had formed the basis for the separate but equal doctrine.

Desegregation and the Civil Rights Movement

In a landmark decision on *Brown v. Board of Education of Topeka, Kansas*, the U.S. Supreme Court ruled in 1954 that separate but equal facilities in education were inherently unequal, and that segregation hindered the educational and mental development of black children. Although the Supreme Court ruled against officially sanctioned segregation in public schools, *de facto segregation* was not addressed until the *Swann v. Charlotte-Mecklenburg Board of Education* ruling of 1971, which mandated court-ordered busing to achieve racial integration of school districts with a history of discrimination.

The gains made by African Americans were won through bitter struggle. Until the middle 1960s, Southern blacks enjoyed few rights, with total segregation enforced in almost all spheres of social, economic, political, and public life. Segregation

in the North occurred through unofficial *de facto* means rather than by legal mandate. The net effect was often the same.

In 1955 Rosa Parks, too tired to stand in the “colored” section in the back of a bus in Montgomery, Alabama, sparked a nonviolent bus boycott led by Martin Luther King Jr. Still another protest was begun when African American students in North Carolina were refused service at an all-white lunch counter. The Civil Rights movement grew rapidly and resulted in widespread demonstrations (in Selma, Alabama, one march drew more than 100,000 people), picket lines, sit-ins, and other forms of political protest. Gaining international publicity, the protests of the late 1950s and early 1960s attracted Northern religious leaders, students, and white liberals—some of whom would lose their lives. By the time the Reverend Martin Luther King Jr. was assassinated in 1968, many demands of the Civil Rights movement had been incorporated into the Civil Rights Act of 1964. Ironically, Congress exempted itself from complying with the act until 1988. Despite the high hopes, the 1964 Civil Rights Act did not live up to its promise. Power had not shifted, and African American and other minority groups continued to be disenfranchised economically, politically, and socially. It soon became apparent that other remedies were needed. One of those was affirmative action, a set of policies designed to provide equal opportunities for minorities and women.

Affirmative Action

Two basic strategies have been employed to address racial, economic, and other injustices. The first is non-discrimination, in which no preferential treatment is given to selected groups. The second is *affirmative action*, whose overall goal is to ensure that women and minorities are admitted, hired, and promoted in direct proportion to their numbers in the population. Affirmative action policies and legislation represent an aggressive step beyond the largely reactive stance taken by simple non-discrimination policies. As such, affirmative action policies are designed to give preferential treatment to minority and female applicants. The ostensible goal is to correct past wrongs done to groups of people throughout the country’s history. Rulings on affirmative action and civil rights cases have been inconsistent, characterized by two steps forward and one or two steps backward.

Opponents of affirmative action argue that it leads to racial quota systems, preferential treatment,

and reverse discrimination. They argue that it violates the equal protection under the laws guaranteed in the Fourteenth Amendment. Still others argue that affirmative action benefits minority group members who do not need the help while placing whites who are innocent of any wrongdoing at a disadvantage. These critics maintain that rights inhere in individuals not in groups. Other conservatives, such as Supreme Court Justice Antonin Scalia, argue that there never was a justification for affirmative action because the Constitution is “color-blind.”¹⁷⁷ Moderates such as former president Bill Clinton note that “affirmative action has been good for America. Affirmative action has not always been perfect, and affirmative action should not go on forever. . . . We should reaffirm the principle of affirmative action and fix the practices. We should have a simple slogan: Mend it, but don’t end it.”¹⁷⁸

William Julius Wilson, an African American sociologist, criticizes the ability of affirmative action strategies to help the most disadvantaged members of society:

Programs based solely on [race-specific solutions] are inadequate . . . to deal with the complex problems of race in America. . . . This is because the most disadvantaged members of racial minority groups, who suffer the cumulative effects of both race and class subjugation . . . are disproportionately represented amongst the segment of the general population that has been denied the resources to compete effectively in a free and open market. . . . On the other hand, the competitive resources developed by the advantaged minority members . . . result in their benefitting disproportionately from policies that promote the rights of minority individuals by removing artificial barriers to valued positions. . . . [If] policies of preferential . . . treatment are developed in terms of racial group membership rather than real disadvantages suffered by individuals, then these policies will further improve the opportunities of the advantaged without necessarily addressing the problems of the truly disadvantaged such as the ghetto underclass.¹⁷⁹

Affirmative action represents a controversial aspect of U.S. social policy that is subject to a wide array of moral questions. For example, while Asian Americans encounter significant discrimination, do they still need preferential treatment given their high income level and educational attainment? Should

poor whites from Appalachia be covered under affirmative action since they face class-based prejudice? Should women who grow up in a privileged upper-class family and attend an elite Ivy League university be covered under affirmative action?

Conclusion

Discrimination takes many forms. It can be targeted against African Americans, Hispanics, Native Americans, Asians, women, gays and lesbians, the aged, people with disabilities, and poor whites. Given that broad umbrella, the majority of the U.S. population experiences discrimination of one sort or another: Fifty-one percent of the population are women; almost 13 percent are over age 65; almost 20 percent are disabled; and roughly one in three Americans is Hispanic, African American, or Asian. Hence, discrimination in U.S. society is not about numbers per se but reflects the relative lack of political, social, and *economic* power of marginalized groups.

Because discrimination can lead to poverty, it often results in the need for income maintenance and poverty programs designed to curb its negative effects. Those who become beneficiaries of these programs soon find themselves with a second handicap—the stigma of being on public assistance, or just needing help.

Some policymakers have tried to reduce the need for long-term and expensive social welfare programs by trying to address the cycle of discrimination and stigma. They often undertake this effort by advocating for policies designed to attack discrimination at its roots. These can include antidiscrimination legislation, affirmative action policies, women’s rights legislation, city and state pro-gay and lesbian ordinances, and legislation to protect the rights of people with physical or mental disabilities. The hope is that by curtailing discrimination, U.S. society can offer at-risk populations equal opportunities for achievement and success. Similar efforts to curb discrimination have been instituted by the European Union and by some member nations and cities. At best, the scorecard on these well-intentioned antidiscrimination policies, both in the United States and abroad, has been mixed. Despite a strong start, affirmative action programs in the United States have not led to widespread economic success for women and minorities. Gays and lesbians continue to be discriminated against, even in places that have passed

civil rights ordinances. Although women have made significant gains over the past few decades, they continue to earn less than males in comparable jobs. Poverty and unemployment rates for minority groups are considerably higher than for the white population. The American dream has become all but attainable for many minorities and poor working class families.

Discrimination has a profound negative effect—not just on the human rights and economic well-being of a large number of affected groups—but on the character, culture, and economy of American society as a whole. Analysts and policy-makers in the United States have the challenging task of developing new strategies to effectively fight discrimination in all its forms.

DISCUSSION QUESTIONS

1. Racism can be manifested in many forms, including poverty, housing problems, underemployment, unemployment, wage differences, lowered educational opportunities, high crime rates, and welfare dependency. What are the causes of racism? Describe the primary causes of individual and institutional racism. How are these factors dealt with by society?
2. Over the past four decades, numerous legal and judicial decisions have attempted to eradicate racism, including the 1964 Civil Rights Act and various Supreme Court rulings. Were these attempts successful? Why or why not?
3. Sexism is a powerful and pervasive force permeating much of U.S. society. How is it manifested? What strategies can be employed to lessen the impact of sexism in society?
4. Most women in U.S. society work to either provide a necessary second income or as the family's primary wage earner. Describe some of the major obstacles faced by working women. What can be done to eliminate some of these obstacles?
5. What are some of the most important social, political, and economic hurdles standing in the way of full equality for gays and lesbians? What can be done to ameliorate them?
6. Being elderly in U.S. society is in many ways a social handicap. Why might that be true?
7. People with disabilities face significant discrimination. What is the evidence to support this belief?
8. The Americans with Disabilities Act (ADA) is the most important piece of legislation affecting people with disabilities. Why? What are its loopholes?
9. Based on the information you have read in this chapter, what are the major causes of discrimination? Why?

NOTES

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Poverty in America



Source: Brian Eichhorn/Shutterstock

This chapter examines the characteristics of poverty in the United States, focusing particular attention on demographic aspects and ways of measuring poverty; family constitution and poverty; child poverty and elderly poverty; the urban and rural poor; and the connections between poverty and work-related issues such as the minimum wage, structural unemployment, dual labor markets, job training programs, and the alternative financial sector or the fringe economy. Finally, strategies developed to combat poverty will be surveyed.

Poverty can be defined as deprivation—either absolute or relative. **Absolute poverty** refers to an unequivocal standard necessary for survival (e.g., the calories necessary for physical survival, adequate shelter for protection against the elements, and proper clothing). **Relative poverty** refers to deprivation that is relative to the standard of living enjoyed by other members of society. Although basic needs are met, citizens may be considered poor if they possess fewer resources, opportunities, or goods than others. Relative poverty (or deprivation) is inequality in the distribution of income, goods, or opportunities.

Theories on Poverty

Culture of Poverty

Poverty theories sometimes cycle in and out of fashion, albeit modified. The frequent underlying premise is that the poor are somehow different than the

rest of society. One such theory is the **Culture of Poverty** (COP), which maintains that poverty and poverty traits are transmitted intergenerationally in a self-perpetuating cycle. According to this theory, the COP transcends regional, rural/urban, and national differences and everywhere shows striking similarities in family structure, interpersonal relations, time orientation, value systems, and patterns of spending.

Writing in the 1960s, Oscar Lewis argued that the COP is characterized by hopelessness, indifference, alienation, apathy, and a lack of effective participation in or integration into the social and economic fabric of society. Key elements are a present-tense time orientation; cynicism and mistrust of those in authority; feelings of marginality, helplessness, dependence, and inferiority; a high incidence of maternal deprivation; lack of impulse control and the inability to defer gratification; a sense of resignation and fatalism; a widespread belief in male superiority; a high tolerance for psychological pathology; a high incidence of abandonment of wives and children; and a matriarchal family structure.¹

By the late 1960s, cultural explanations of poverty became a taboo subject among sociologists since it placed the blame for poverty squarely on the character of the poor. It was also seen in many quarters as being inherently racist. However, the older culture debate around poverty has resurfaced, albeit different form. Contemporary social scientists reject the notion of a monolithic and fixed

Spotlight 5.1

POVERTY

People living in poverty fall into three general categories:

1. Those making only minimum wage (the working poor)
2. The unemployed
3. Those with poor health or an occupational disability (e.g., a deficit in human capital such as poor education or a low quality and quantity of training and skills)

RESULTS is a nonprofit grassroots advocacy organization committed to creating the political will to end hunger and the worst aspects of poverty. RESULTS members lobby elected officials for effective solutions and key policies that affect hunger and poverty. To learn more about RESULTS, go to the organization's website at www.results.org.

COP. Instead, they attribute destructive attitudes and behavior to racism and isolation rather than the flawed moral character of the poor.

The critique of the old COP theory is grounded in the realization that many aspects of a poverty culture observed by Lewis are now commonplace among the middle class, such as high divorce rates, cohabitation outside of marriage, single-motherhood, and relaxed sexual mores. The prism by which the poor are viewed continues to vacillate between the poor as causes of their own poverty or as victims of a racist, market society.

Eugenics and Poverty

Theories based on genetic inferiority have been used as explanations for poverty, crime, and disease. Henry Goddard's *The Kallikak Family* was an account of a Revolutionary War soldier who had an affair with a feeble-minded servant girl before marrying a "respectable" woman.² Goddard meticulously listed the disreputable descendants of the servant girl and compared them with the respectable achievers of the wife's descendants. Generations of students were taught the dogma of eugenics. By the early twentieth century, the eugenics movement morphed into a pseudo-science that led to involuntary or forced sterilizations. Harry Laughlin, an officer in the Eugenics Research Association, proposed a model for the forced sterilization of people in institutions and others who were maintained at public expense. The law encompassed the "feeble-minded," mentally ill, criminals, epileptics, inebriates, and those with blindness, deafness, and deformities. It also included orphans, the homeless, and paupers. By the time Model Law appeared in 1914, 12 states had enacted sterilization laws.³

The eugenics movement lost favor when these theories were used by Hitler to justify genocide. However, the movement reemerged with the publication of Arthur Jensen's 1969 article "How Much Can We Boost IQ and Scholastic Achievement?"⁴ Jensen concluded that compensatory education was doomed to fail since 80 percent of intelligence (measured by IQ tests) was inherited.⁵ This argument was rekindled again in 1994 by Richard Herrnstein and Charles Murray's *The Bell Curve*, in which the authors argued that inequality was due to lower levels of genetic intelligence.⁶ They argued that affirmative action programs overlook intellectual meritocracy, and spending money to

educate the poor is wasteful, given their innate deficiencies. The scholarship in *The Bell Curve* was attacked by wide ranging critics who argued that Herrnstein and Murray exaggerated IQ as a predictor of job performance, attributed inaccurate validities to IQ scores, and substituted hypotheticals for reality.⁷

The Progressive Understanding of Poverty

For socialists and other left-wing thinkers, poverty results from the exploitation of the poor by an elite class of capitalists and elected representatives who look after their interests. David Gil observes that poverty can be understood in terms of status, resource allocation, and the division of labor.⁸ For instance, societies must develop resources (symbolic, material, life-sustaining, and life-enhancing goods and services) to maintain itself. They must also develop a division of labor, thereby assigning individuals or groups to specific tasks related to developing, producing, or distributing these societal resources. This division of labor is used to assign status to individuals and groups; that is, the more highly valued the function (by elites) an individual or group performs, the higher the status reward. This often has little to do with the actual value of a job. For instance, while teachers are more important to most of us than Wall Street stockbrokers who create risky financial instruments, the pay difference between them is staggering. By manipulating the division of labor, a society is able to assign individuals to specific statuses. In turn, poverty is correlated to the status a group or occupation occupies within a society.

The assignment of status roles is complemented by the distribution of rights. Higher-status roles implicitly involve greater compensation than lower-status roles, and such rewards come by way of the distribution of rights. Higher-status groups are also rewarded with more entitlements and rights to material and symbolic resources, goods, and services than their lower-status cohorts. Conversely, lower status groups are denied these resources or privileges by formal and informal constraints. For example, all things being equal, rich kids have a greater chance of being admitted into elite Ivy League schools than those from low-income families. This is rationalized by an expressed belief in the omniscience of the marketplace. In other words, rewards are based on the value of one's

contribution to the market. This ideology is rarely questioned. Socialists argue that poverty will exist as long as wealth and privileges are based solely on hereditary and market values. They believe the abolition of poverty cannot occur without a fundamental alteration of the economic and social fabric of society.

Who Makes Up the Poor?

Poverty is a fluid (rather than static) condition in which people cycle in and out. The University of Michigan's Panel Study of Income Dynamics (PSID) has followed thousands of U.S. families (9,000 in 2013) since 1968. The study has found that as people gained or lost jobs, as marriages were created or dissolved, or as offspring were born or left home, people were either pushed into poverty or escaped from it.⁹ PSID data points to a constellation of events that push people in and out of poverty. Some of these include changes in household composition (divorce, abandonment,

single-motherhood), loss or gain of employment, disability status, the shift from a two-adult household to a female-headed household and vice versa.¹⁰

Intergenerational economic mobility has traditionally been an important ticket out of poverty. Studying intergenerational mobility, Raj Chetty, Nathaniel Hendren, Patrick Kline, Emmanuel Saez, and Nicholas Turner found that intergenerational mobility had remained stable for the 1971 to 1993 birth cohorts. They found that children entering the labor market in 2014 had the same chances of rising (relative to their parents) in the income distribution ladder as those born in the 1970s. But, because inequality has risen, the effect of the "birth lottery" (i.e., the economic status of a child's parents) are larger today than in the past.¹¹ In another study, they found that a 10 percentile increase in parental income is associated with a 3.4 percentile increase in a child's income. Intergenerational mobility varied across the country. For instance, starting from a family in the bottom quintile, the probability

Spotlight 5.2

WHO ARE THE POOR?

- The official poverty rate in 2014 was 14.8 percent representing 46.7 million people in poverty. These numbers have been relatively stable for four years, going up or down a point or two.
- The 2014 poverty rate was 2.3 percentage points higher than in 2007, the year before the most recent recession.
- The number of people in poverty increased for unrelated individuals, people aged 18 to 64 with a disability, people with at least a bachelor's degree, and married-couple families. Poverty rates in 2014 rose slightly for only two groups: people with a bachelor's degree or more and married-couple families.
- The poverty rate in 2014 for children under age 18 was 21 percent. More than half

(55 percent) of related children under age six in families with a female householder were in poverty. This was more than four times the rate of their counterparts in married-couple families (11.6 percent).

- For people aged 18 to 64, the poverty rate was 13.5 percent; and for people aged 65 and older was 10 percent. These poverty rates were stable, going up only a point a two over several years.

Race and Hispanic Origin

- Non-Hispanic whites accounted for 62 percent of the total population and 42 percent of the people in poverty. The poverty rate for this group was 10 percent in 2014, lower than the poverty rates for other groups.

- The 2014 poverty rate for African Americans was 26 percent with 10.8 million people in poverty.
- The Asian American poverty rate was 12 percent in 2014, which represented 2.1 million poor people.
- For Hispanics, the 2014 poverty rate was almost 24 percent with 13 million people in poverty.

None of these estimates were statistically different from the 2013 estimates.

Age

- In 2014, 13.5 percent of people aged 18 to 64 (26.5 million) were in poverty compared with 10.0 percent of people aged 65 and older (4.6 million) and 21 percent of children under age 18 (15.5 million). While children made up 23 percent of the total population they represented 33 percent of people in poverty.
- For related children under age 18 in families with a female householder, 46.5 percent were in poverty compared with 10.6 percent of related children in married-couple families.

Sex

- In 2014, 13 percent of males and 16 percent of females were in poverty. The poverty rate for women aged 18 to 64 was 15 percent, while the poverty rate for men aged 18 to 64 was almost 12 percent.
- For children under age 18, the poverty rate for girls was not statistically different from the poverty rate for boys since both were at 21 percent.
- Gender differences in poverty rates were more pronounced for those aged 65 and older. The poverty rate for women aged 65 and older was 12 percent, while the poverty rate for men aged 65 and older was roughly 7 percent.

Foreign and Native-Born

- The 2014 poverty rate for the foreign born was 18.5 percent (almost 8 million) compared to 14 percent (39 million) for native-born Americans.
- Within the foreign-born population in poverty, 47 percent were naturalized U.S. citizens, while the remaining were not U.S. citizens.

- The poverty rates in 2014 were 11.9 percent for foreign-born naturalized citizens and 24 percent for those who were not U.S. citizens.

Region and Residence

- The 2014 poverty rate in the Northeast was 12.5 percent (7 million people) and 13 percent (8.7 million) in the Midwest (not statistically different).
- Poverty rates were considerably higher in the South at 16.5 percent (19.5 million) and in the West at 15 (11.4 million).
- The 2014 poverty rate inside metropolitan statistical areas (high population density) was 14.5 percent (almost 39 million). For those living outside metropolitan statistical areas, the poverty rate climbed to 16.5 percent (8 million).
- For those living inside metropolitan areas but not in principal cities, the 2014 poverty rate was almost 12 percent (20 million). The 2014 poverty rate for those who lived in principal cities was almost 19 percent (19 million).
- While 37 percent of all people living in metropolitan areas lived in principal cities, 49 percent of poor people in metropolitan areas lived in principal cities.

Work Experience

- In 2014, 7 percent of workers aged 18 to 64 were in poverty. The poverty rate for those who worked full time, year-round was 3 percent, while the poverty rate for those who worked less than full time, year-round was 16 percent. Presumably, those who worked less than full time were in the secondary labor market.
- The 2014 poverty rate for those who did not work at least one week in 2014 was almost 34 percent (16 million).

Disability Status

- The 2014 poverty rate for people aged 18 to 64 with a disability was almost 29 percent, more than double the poverty rate compared to people aged 18 to 64 without a disability (12 percent).

- Among people aged 18 to 64, those with a disability represented 8 percent of all people compared with 17 percent of people aged 18 to 64 in poverty.

Educational Attainment

- Almost 29 percent of people aged 25 and older without a high school diploma were in poverty in 2014. The poverty rate for those with a high school diploma (but no college degree) dropped to 14 percent.
- The poverty rate for those with some college but no degree was 10 percent.
- Among people with at least a bachelor's degree, the poverty rate dropped in half to 5 percent. This was a rise from the 4.4 percent poverty rate in 2013.
- People with at least a bachelor's degree in 2014 represented around 33 percent of all people aged 25 and older compared with almost 14 percent of people aged 25 and older in poverty. While the advantage of a

bachelor's degree may be slightly eroding, it continues to represent a significant hedge against poverty.

Families

- In 2014, the family poverty rate and the number of families in poverty in 2014 was 11.6 percent (9.5 million families).
- For married-couple families, both the 2014 poverty rate and the number of families in poverty increased to 6 percent (3.7 million), up from 5.7 percent (3.4 million) in 2013. This increase is especially troubling rise since two-parent families were traditionally seen as the economic backbone of the nation.
- The 2014 rate for families with a female householder was 30.5 percent in 2014, with the actual numbers in poverty being less than in 2013 (down from 5.2 to 4.8 million).
- In 2014, families headed by a single male householder had almost a 16 percent poverty (1 million) rate.



Watch this video for some insight into the reality of poverty.

www.povertyusa.org/the-state-of-poverty/poverty-usa-tour/

Source: Carmen DeNavas-Walt and Bernadette D. Proctor, *Income and Poverty in the United States: 2014*, Current Population Reports, September 2015, P60-252. Retrieved December 2015 from <https://www.census.gov/content/dam/Census/library/publications/2015/demo/p60-252.pdf>.

that a child will reach the top quintile of national income is 4.4 percent in Charlotte, but 12.9 percent in San Jose. In addition, the authors found that high mobility areas are characterized by less residential segregation; less income inequality; better schools; high levels of social capital; and greater family stability.¹²

Table 5.1 describes the characteristics and numbers of the poor over a 50-year period.

Measuring Poverty

There are two versions of the federal poverty measure: (1) the poverty threshold and (2) the poverty guideline. The poverty threshold, also called the **poverty line**, is the official federal

poverty measure and is used primarily for statistical purposes, such as estimating the number of Americans in poverty each year. All official population figures are calculated using the poverty threshold. The poverty guideline uses a slightly lower poverty level than the weighted poverty threshold. For example, the poverty threshold for a family of four in 2013 was \$23,834,¹³ while the poverty guideline was \$23,550 for the same family.¹⁴ The poverty guideline is used for determining eligibility for federal programs such as Head Start, Food Stamps, the School Lunch Program, and Low-Income Home Energy Assistance Program. Other federal programs, including TANF and Supplemental Security Income, use the poverty threshold. As noted earlier, the federal poverty index for a family of four was \$23,834 in

TABLE 5.1 Persons below the Poverty Line, Selected Years and Characteristics, 1959–2013 (number and percentage below poverty, in thousands)

Year	Overall	Aged	Children ^a	Individuals in Female-Headed Families ^b	Blacks	Hispanic ^c	White
2013	45,318 14.5%	4,231 9.5%	14,142 19.5%	15,606 33.2%	11,041 27.2%	12,744 23.5%	29,936 12.3%
2010	46,180 15.1%	3,520 9.0	16,401 22.0	15,895 34.2	10,675 27.4	13,243 26.6	31,650 13.0
2006	36,460 12.3%	3,394 9.4	12,827 17.4	13,199 30.5	9,048 24.3	9,243 20.6	24,416 10.3
+2002	34,570 12.1%	3,578 10.4	12,133 16.7	11,667 26.5	8,884 24.1	8,556 21.8	24,074 10.3
1999	32,258 11.8%	3,167 9.7	12,109 18.9	12,687 27.8	9,091 22.7	7,439 22.8	21,922 9.8
1995	36,425 13.8%	3,318 10.5	13,999 20.2	12,315 32.4	9,872 29.3	8,574 30.3	24,423 11.2
1990	33,585 13.5%	3,658 12.2	14,431 20.6	12,578 37.2	9,837 31.9	6,006 28.1	22,326 10.7
1986	32,370 13.6%	3,477 12.4	12,876 20.5	11,944 38.3	8,983 31.3	5,117 27.3	22,183 11.0
1980	29,272 13.0%	3,871 15.7	11,543 18.3	10,120 36.7	8,579 32.5	3,491 25.7	19,699 10.2
1978	24,497 11.4%	3,233 14.0	9,931 15.9	9,269 35.6	7,626 30.6	2,607 21.6	16,259 8.7
1969	21,147 12.1%	4,787 25.3	9,961 14.0	6,879 38.2	7,095 32.2	NA NA	16,659 9.5
1959	39,490 22.4%	5,481 35.2	17,552 27.3	7,014 49.4	9,927 55.1	NA NA	28,484 18.1

*These numbers may appear inconsistent because the U.S. Census Bureau changed the classification of races in the 2000 and 2010 census to include the designation of more than one race. In addition, the statistics were calculated slightly differently in different years.

^aAll children, including unrelated children.

^bDoes not include females living alone.

^cPeople of Hispanic origin may be of any race; it is an overlapping category.

Sources: U.S. Census Bureau reports, U.S. Census Bureau, from <https://www.census.gov/hhes/www/poverty/data/historical/people.html>.

2013, up from \$8,414 in 1980¹⁵ (see Table 5.2). These increases are due solely to the effects of inflation.

The poverty threshold used by the federal government was developed by taking the cost of the

least expensive food plan (the Thrifty Food Plan developed by the Department of Agriculture) and multiplying that number by three. This formula was based on 1955 survey data showing that the average family spent about one-third of its budget

TABLE 5.2 Changes in the Poverty Line Based on Income and Family Size, 1975–2013

Family Size	Income, Selected Years						
	1980	1985	1990	2003	2007	2010	2013
1	\$4190	\$5,250	\$6,652	\$9,393	\$10,787	\$11,139	\$11,888
2	5,363	7,050	8,509	12,015	13,884	14,218	15,142
3	6,565	8,850	10,419	14,680	16,218	17,374	18,552
4	8,414	10,650	13,359	18,810	21,386	22,314	23,834
5	9,966	12,450	15,572	22,245	25,791	26,439	28,265
6	11,269	14,250	17,839	25,122	29,664	29,897	31,925
7	13,955	16,050	20,241	28,544	34,132	34,009	36,384

Source: Compiled from U.S. Census Bureau, Technical Paper 56, ser. P-60, nos. 134 and 149 (Washington, DC: U.S. Government Printing Office, 1992); U.S. Census Bureau, "Poverty 1995," retrieved September 1996 from www.census.gov/hhes/poverty/pov95/thresh95.html; and Carmen DeNavas-Walt, Bernadette D. Proctor, and Robert J. Mills, U.S. Census Bureau, "Income, Poverty and Health Insurance Coverage in the United States: 2003," Current Population Reports, ser. p. 60–226, retrieved October 2004 from www.census.gov/prod/2004pubs/p60-226.pdf; U.S. Census Bureau, "Poverty Thresholds 2007," Retrieved June 2008 from <http://www.census.gov/hhes/www/poverty/threshld/thresh07.html>; U.S. Census Bureau, "Income, Poverty, and Health Insurance Coverage in the United States: 2010," September 2011. Retrieved February 2012 from www.census.gov/prod/2011pubs/p60-239.pdf.

on food. Formally adopted by the Social Security Administration (SSA) in 1969, the official poverty measure provides a set of income cutoffs adjusted for household size, the number of children under age 18, and the age of the household head. To ensure constant purchasing power, the SSA adjusts the poverty line yearly, using the consumer price index (CPI).

The poverty index is plagued by a variety of structural problems. A National Academy of Sciences (NAS) report noted that the poverty threshold disregards various in-kind benefits when counting family income,¹⁶ including the costs (i.e., clothing, transportation) of employment; regional variations in living costs (i.e., housing and food); sales, payroll, and property taxes; the value (and costs) of health care coverage and out-of-pocket medical expenses; and the change in consumption patterns (food costs now account for only one-seventh of household expenditures).¹⁷

Roosa et al., support this critique and raise the following issues around the poverty threshold:

- It does not reflect family structure and type (e.g., working mothers contributing to the household income or the health care costs of raising a child with a disability).
- It does not account for how families' needs change across lifespans (e.g. the different costs

associated with children at various ages and the costs of having one or more elderly parents requiring full-time care).

- It does not account for recent changes such as the casualization of the workforce (which has increased job and income insecurity).
- It is arbitrary for those close to the limit. For instance, there is no significant difference between a family living below the poverty line and one that earns \$1 more than the threshold.¹⁸

To partially offset the inherent flaws in the poverty index, Obama administration officials developed a new formula, the Supplementary Poverty Measure (SPM) that considers a wider range of factors in determining the poverty threshold. The SPM does not simply calculate a family's income and cost of food; instead, the new measure factors in modern expenses (e.g., high cost of health care, child care, housing, utilities and in-kind benefits), plus "a little more," a new category that provides a little extra padding. The new formula will also adjust for geographic location (a major flaw in the existing poverty measure) and take into account benefits like food stamps, housing subsidies, and tax credits. Consequently, the new poverty measure will likely increase the percentage of people classified as poor, especially among the elderly Americans who are subject to high medical expenses. Although the

SPM will be used in Census Bureau reports, it will not replace the existing poverty measure in determining eligibility for government programs since it would substantially increase the costs of those programs.¹⁹ Among other variables, an accurate poverty threshold would take into account important changes (and additional costs) in family structure and labor force participation.

Measuring the Depth of Poverty

Categorizing a person or family as poor is only one measure of economic well-being. The other measure is ascertaining the depth of poverty. The income-to-poverty ratio compares a family or individual income with the applicable poverty threshold. For example, a family with a 125 percent income-to-poverty ratio has income that is 25 percent above their poverty threshold. The income deficit illustrates how many dollars a family or an individual income is below their poverty threshold. It is an estimate of the dollar amount necessary to raise a family or individual income to their appropriate poverty threshold. Some social program, such as SNAP, allow for incomes at 130 percent above the poverty line.²⁰

In 2014, almost 21 million people (45 percent of those in poverty) reported family incomes that were below half their poverty threshold. Of that number, almost 7 million were children under age 18 and 1.5 million were over age 65. About 20 percent of families had incomes below 125 percent of their poverty threshold; 24 percent had incomes at 150 percent; and 33 percent had incomes below 200 percent. The income deficit for families in poverty averaged \$10,137 in 2014. The average income deficit was larger for families with a female householder (\$10,662) than for married-couple families (\$9,474).²¹

Families and Poverty

Family composition is strongly correlated to poverty, and U.S. families at greatest risk of poverty are those headed by single females. In 2013, the poverty rate for all families was 15.5 percent; for female-headed households with no husband present, it was 33.2 percent. These figures become even starker when disaggregated. Families headed by non-Hispanic white women had a poverty rate of 22.9 percent; families headed by African American and Hispanic women had poverty rates of 37.4 percent and 41.6 percent, respectively.²²



The families at greatest risk of poverty in the United States are those headed by single females, which is compounded when child support payments are withheld.

Source: saiyood/Fotolia

More than 1 million American children see their parents divorce or separate each year, and more than half of all children will spend some time in a single-parent family. A 2012 U.S. Department of Health and Human Services study found that compared to children in two-parent families, children in single-mother families were more likely to have learning disabilities and ADHD, (11 percent and 12 percent compared to 7 percent and 8 percent); were twice as likely to be absent from school for 11 or more days in a 12-month period due to illness or injury; were two and one-half times more likely to have two or more ER visits in a 12-month period; and more likely to have unmet dental needs.²³

Child Support Enforcement

According to the Census Bureau, 53.4 percent of mothers in 2011 had child support orders or agreements in place compared to 29 percent of fathers. Of those with orders, just under half received full payment and over half received partial or no payment.²⁴

Child support is important for families with only one custodial parent. According to the Census Bureau, 56.7 percent of custodial parents received some child support in 2011. The proportion of custodial parents receiving every payment they were due was 43.4 percent in 2011. Of the almost 14.4 million custodial parents in 2011, 7.2 million (50 percent) had some type of formal child support agreement in place. The most commonly cited reasons for custodial parents not entering into any

formal agreement were that they did not feel the need to go to court and make a formal agreement (11.2 percent), they felt that the other parent provided what they could in support (35 percent) or they felt that the other parent could not afford child support (32.4 percent).²⁵

Child support is critical for low-income families, and in 2011, it constituted 16 percent of the annual income of divorced families and reduced their poverty rate by 7 to 11 percent.²⁶ It is also a key income source for poverty-level families, since child support for parents who received full child support payments constituted 20 percent of their average income.²⁷ In 2004, child support payments lifted 500,000 children out of poverty.²⁸ One study of welfare leavers found that those with regular child support payments had a slower rate of welfare reentry, a faster rate of finding work, and a slower rate of job loss compared to families without steady child support income.²⁹

In 1996, former President Clinton signed into law the Personal Responsibility and Work Opportunity Reconciliation Act (PRWORA), which also addressed child support enforcement. Specifically, failure to meet child support obligations could result in the revocation of driver's and professional licenses; wage garnishment; liens; and/or denial, revocation, or limitation of passports. Delinquent support obligations could also be collected through unemployment and disability insurance benefits, as well as tax refunds.

Children in Poverty

In 2013, there were 14.1 million poor children in the United States, whose poverty rate is higher than for any other age group—23.5 percent compared to the overall poverty rate of 12 percent. For children under age six, the poverty rate was even higher at 13.6 percent (6,343,000 children).³⁰ In 2011, more than 50 percent of U.S. children lived in extreme (family incomes below 50 percent of the poverty line) poverty and 22 percent of children lived in or near poverty (family incomes below 200 percent of the poverty line).³¹ Compared to poverty in later childhood, research indicates that extreme poverty during a child's first five years is especially deleterious to their future life chances.³²

Minority children experience more instances of poverty: In 2013, 27 percent of African American and 23.5 percent of Hispanic children lived

below the poverty line compared to 9.6 percent of non-Hispanic white children.³³

Poverty and the Elderly

On the surface, the poverty picture for the elderly (once the poorest group in the country) seems to be growing less bleak. In 1959, the poverty rate for those over 65 was 35 percent, which decreased to 9.5 percent by 2013.³⁴

Elderly poverty is higher within minority groups. In 2013, almost 18 percent of elderly African Americans and Hispanics were poor compared to 7 percent of elderly non-Hispanics.³⁵

A 2015 study of senior poverty by the Kaiser Family Foundation found that:

- In 2013, 45 percent of those 65 and older had incomes well below the SPM poverty threshold compared to 33 percent of the elderly listed as poor under the official measure.
- Poverty rates were higher for females 65 and older than for elderly men. In 2013, official poverty rates for females 65 and over was 12 percent compared to 7 percent. Under the SPM, it was 17 percent versus 12 percent. Among those 80 and older, 23 percent of women lived below the SPM poverty thresholds compared to 14 percent of men.
- The 2013 official poverty rate was roughly three times higher for elderly Hispanics than for elderly whites (20 versus 7 percent). It was two and a half times higher for elderly blacks (18 percent).
- 2013 poverty rates were higher for all three groups under the SPM, with 28 percent of Hispanics, 22 percent of blacks, and 12 percent of whites living in poverty.
- The percentage of seniors living in poverty under the SPM guidelines is larger than the official measure in every state.³⁶

The Rural Poor

Almost 8 million rural Americans live in poverty. Those working in farming, fishing, and forestry have a poverty rate of 16.8 percent, which is higher than any other occupations profiled by the U.S. Bureau of Labor Statistics.³⁷

Many rural areas have poverty rates exceeding those of central cities, and in 2013, the rural poverty

rate of 16.1 percent was lower than the urban poverty rate of 17.7 percent. Broken down, the poverty rate of the South was 16.9 percent; 15.3 percent in the West; 13.9 percent in the Midwest, and 12.8 percent in the Northeast.³⁸ In 2013, five states (Arizona, Mississippi, New Mexico, Kentucky, and Louisiana) has poverty rates of 20 percent or above. Since the farming population accounts for only 10 percent of the rural population, the rural poor are therefore mostly nonfarmers.³⁹

Rural minority members experience considerable poverty; in 2010, the non-metro poverty rates for blacks and Hispanics were more than double the poverty rate of non-metro whites. Broken down, the 2010 non-metro poverty rate for blacks was 33 percent; 30 percent for non-metro Hispanics; and 13 percent for non-metro whites.⁴⁰ The rural poor are more likely than the urban poor to live in chronic long-term poverty. Despite higher unemployment in rural areas, the rural poor rely less on public assistance than the urban poor. Lack of information and access to services, fear of stigma, and reliance on informal employment may help explain some of this difference.⁴¹

Family structure has a significant bearing on poverty, with the highest poverty among single female-headed, non-metro families. In 2010, 41 percent of female-headed, non-metro families were poor compared to 21 percent of male-headed, non-metro families. For non-metro families headed by both husband and wife, the poverty rate was only 7 percent. In 2010, the child poverty rate in non-metro areas was 24 percent.⁴²

Several factors contribute to chronic rural poverty, including high levels of illiteracy, low levels of education, a shortage of highly trained workers, high numbers of low-skill and low-paying jobs, high levels of underemployment and unemployment, and a poor physical infrastructure. Since rural areas have difficulty in attracting major industries, it is not surprising that the lack of economic opportunity has resulted in the outmigration of many educated and skilled rural families.⁴³

Work and Poverty

Almost one in four U.S. workers lives in or around the edges of poverty. Thirty-five million Americans work full time but fail to make an adequate living. They are the nursing home aides, poultry processors, pharmacy assistants, child care workers, data

entry workers, janitors, and employees of the secondary and tertiary labor markets.

A Profile of the Working Poor

The **working poor** are defined as individuals who spend at least 27 weeks in the labor force (working or looking for work) but whose family or personal incomes fall below the poverty line.

This is a growing segment of the poverty population. According to data from the Bureau of Labor Statistics in 2013:

- 10.5 million or 23 percent of the total poverty population poor were classified as working poor.
- The working poor constituted 7 percent of the total work force.
- Of the working poor, 13 percent were black; 13 percent Hispanic; 6 percent white; and 5 percent Asian.
- 8 percent were women; 6 percent men.
- They represented 19 percent of the labor force with less than a high school diploma; 9 percent of high school graduates with no college education; 5 percent for those with an associate's degree, and 2 percent for those with a bachelor's degree or higher.
- Rates were highest for 16- to 24-year-olds (11 to 13 percent) and lowest for those over 65 (2 percent).
- 16 percent of part-time workers were classified as working poor compared to 4 percent of full-time workers.⁴⁴

Why Are There Working Poor?

The large numbers of working poor are related to several factors, including the replacement of high-paying industrial jobs with low-paying service jobs. Millions of Americans with families who are working full time, year-round earn less than the official poverty level.⁴⁵ This trend reflects a clear movement away from higher-paying manufacturing jobs to low-wage service employment. In the 1950s, 33 percent of all workers were employed in primary manufacturing industries (e.g., cars, radios, refrigerators, and clothing); by 1992 only 17 percent were employed in those industries. From 2001 to 2009, 2.5 million industrial jobs disappeared from the United States. In fact, General Motors employed about half as many workers in

2009 as it did in 2000. Many economists predict this trend will intensify as increased international competition leads to more outsourcing of industrial and white-collar jobs. The competition for jobs will also lower wages for lesser-skilled occupations and thus increase income inequality and the numbers of the working poor. In line with this, real wages have stagnated since the mid-1990s, while productivity has grown rapidly. The Economic Policy Institute reports wages in the industries in which jobs are being created are, on average, 21 percent lower than wages in those industries in which jobs are disappearing. Lower wage jobs in the expanding industries are also less likely to provide health insurance than those industries cutting jobs.⁴⁶ For instance, although the jobless rate fell to 5.1 percent in 2015, most of the jobs created were in health care, social assistance, and financial activities. The manufacturing and mining center lost jobs.⁴⁷

Underemployment and Unemployment

The failure of the labor market to meet the economic needs of the population has been the source of important distinctions in employment policy. For example, those over 16 who are looking for work are counted by the Department of Labor as unemployed. But the **unemployment** rate does not assess the adequacy of employment. For example, part-time workers who wish to work full time are counted as employed; and workers holding jobs below their skill levels are not identified, even though such workers are **underemployed**. Finally, **discouraged workers** who simply give up and stop looking for work, relying on other methods to support themselves, do not appear in the unemployment statistics because they are not actively seeking work.

A second set of distinctions relates to economic performance. In a robust economy, businesses start up and close down in significant numbers, leaving workers temporarily out of work until they find other employment. Such **frictional unemployment** is considered unavoidable and reflects the cost of a constantly changing economy. **Structural unemployment** refers to “deeper and longer-lasting maladjustments in the labor market,” such as changes in the technical skills required for new forms of production.⁴⁸ Because of swings in economic performance, some unemployment may be **cyclical unemployment**, as when recessions pitch the rate upward; but because certain groups of workers in certain regions have persistent difficulty finding

work owing to an absence of jobs, some unemployment may be **chronic unemployment**.

These distinctions are noteworthy, because when people are out of work, they frequently rely on welfare benefits to tide them over. Thus, welfare programs are often designed to complement the labor market, which is why some observers refer to welfare as a **social wage**, or the amount government pays to workers through welfare programs when they are unable to participate in the labor market. Logically, much of welfare could be eliminated if well-paying jobs were plentiful, but such has not been the case in the United States. Policymakers have tacitly accepted an unemployment rate of between 4 and 5 percent as normative, which means that at any given time 8 million workers are out of work if the unemployment rate hits 5 percent.⁴⁹ Yet in 1978, Congress enacted the Humphrey–Hawkins Full Employment Act, which set an unemployment rate of 3 percent—equivalent to frictional unemployment—as a national goal. Since then, many government programs to aid unemployed, underemployed, and discouraged workers have been reduced or eliminated, leaving many Americans dependent on welfare programs for support.

The outsourcing of U.S. jobs to low-wage countries may also exacerbate unemployment. A report by the U.S. Department of Labor (DOL) found that in 2010, a total of 7,247 mass layoff events resulted in a loss of 1.3 million jobs. (Extended mass layoffs refer to layoffs of at least 31 days duration that involve the filing of initial claims for unemployment insurance by 50 or more individuals from a single establishment during a consecutive five-week period.) It is difficult to determine how many workers actually lose employment due to outsourcing since employers may report reasons for layoffs in different terms (i.e., cost-cutting, changes in demand).⁵⁰ The trend of outsourcing jobs overseas raises concerns about the potential loss of the high-wage, white-collar jobs (especially in information technology) that were once considered safe from global competition. It also raises the specter that competition from less-expensive overseas workers will further slow the wage growth of American workers.⁵¹ The Economic Policy found that since 1973 the median male (full-time, year-round) wage has declined in constant dollars from \$53,291 in 1973 to \$51,902 in 2002 and to \$50,383 in 2014.⁵²

The absence of employment opportunities contributes to other social problems. Research

TABLE 5.3 Consequences of Increases in Unemployment

Pathological Indicator	Percentage Increase Due to Rise in Unemployment Increase	Rise in Incidence of Pathology
Total mortality	2.3	45,936
Cardiovascular mortality	2.8	28,510
Cirrhosis mortality	1.4	430
Suicide	1.0	270
Population in mental hospitals	6.0	8,416
Total arrests	6.0	577,477
Arrests for fraud and embezzlement	4.0	11,552
Assaults reported to police	1.1	7,035
Homicide	1.7	403

Source: M. Harvey Brenner, *Estimating the Effects of Economic Change on National Health and Social Well-Being* (Washington, DC: U.S. Government Printing Office, 1984), p. 2.

by M. Harvey Brenner demonstrated that a seemingly small increase in the unemployment rate is associated with an increase in several social problems. During the 1973 to 1974 recession, the unemployment rate increased to 14.3 percent, a change associated with the pathologies shown in Table 5.3. Brenner calculated that the combination of the 1973 to 1974 increase in the unemployment rate, the decrease in real per capita income, and an increase in the business failure rate was related to “an overall increase of more than 165,000 deaths [from cardiovascular disease] over a 10-year period (the greatest proportion of which occurs within three years).”⁵³ Overall, the total economic, social, and health care costs of this seemingly slight increase in unemployment cost \$24 billion.⁵⁴

The impact of the high unemployment rate that began in 2008 is particularly difficult for older workers in search of new employment. By 2014, the typical length of time a jobless worker in the United States was unemployed fell to 31.2 weeks. But the outlook was bleaker for older workers. The unemployment spells of older workers are longer and they are less likely to be employed. In 2011, the average jobless person over age 65 was looking for work for 43.9 weeks; for those between 55 and 64, it was 44.6 weeks. The longer that a worker is out of the workforce, the less employable they are.⁵⁵

Dual Labor Markets

According to Piore, the labor market can be divided into two segments, or “dual labor markets”—a **primary labor market** and a **secondary labor market**:

The primary market offers jobs that possess several of the following traits: high wages, good working conditions, employment stability and job security, equity and due process in the administration of work rules, and chances for advancement. The other, secondary sector, has jobs which, relative to those in the primary sector, are decidedly less attractive. They tend to involve low wages, poor working conditions, considerable variability in employment, harsh and often arbitrary discipline, and little opportunity to advance. The poor are confined to the secondary labor market.⁵⁶

Researchers calculated that in 1970, 36.2 percent of workers fell into the secondary labor market, a modest increase over 35 percent in the 1950s.⁵⁷ By the 2000s, two factors increased the proportion of workers in the secondary labor market. First, membership in labor unions—the best job security for nonprofessional workers—fell from 31 percent of nonagricultural workers in 1970 to 11.4 percent in 2015 (the lowest rate in 70 years), leaving millions of workers vulnerable to the vicissitudes of the secondary labor market. Second, a higher proportion of the newly

created jobs were in the service sector, consisting largely of secondary labor market jobs.⁵⁸

Wages and Poverty

The Minimum Wage The low minimum wage is another factor that explains the growth of the working poor. Congress passed the Fair Minimum Wage Act in 2007, which increased the minimum wage in three incremental steps from \$5.15 an hour (which had been stuck since 1997) to \$7.25 an hour in 2009. By 2015, the \$7.25 minimum wage had not risen since 2009.⁵⁹

It is difficult for minimum wage employment to provide an adequate household income. Under the 2015 minimum wage of \$7.25 an hour, a single parent with two children working full time would earn \$13,920 a year and be at 69 percent of the

\$20,090 poverty line. A four-person, two-parent family where both adults are working full time and earning the minimum wage would gross \$27,840 a year, or only \$3,590 above the \$24,250 poverty line. Roughly 1.3 million workers in 2014 earned exactly the prevailing federal minimum wage of \$7.25 per hour while 1.7 million workers earned below that. These 3 million workers make up almost 4 percent of all hourly workers.⁶⁰

Traditional labor economists maintain that minimum wage increases result in inflation and significant job losses for low-wage workers.⁶¹ This contention was disputed by the Economic Policy Institute, which did not find an increase in unemployment following the federal minimum wage increase in 1996 to 1997.⁶² The Fiscal Policy Institute concluded that states that enacted minimum wage increases higher than the federal standard experienced stronger small business and retail sector growth as a result.⁶³ These employment sectors have the highest proportion of minimum wage jobs:

- Food service workers (26.6 percent)
- Sales occupations (18.7 percent)
- Office and administration support workers (13 percent)
- Transportation and material moving workers (8 percent)
- Cleaning and building service workers (8 percent)
- Personal care workers (7.5 percent)⁶⁴

Unlike some countries, the federal minimum wage is not automatically adjusted to the cost of living. Congress must pass a bill, and the president must sign it into law for the minimum wage to rise. The minimum wage has risen only slightly during the past 18 years, increasing to \$4.25 an hour by 1991 and then remaining at that level for more than five years. By 1996, approximately 10 million American workers were earning between \$4.25 and \$5.14 per hour. The minimum wage was frozen from 1997 to 2007.

In 1950, the minimum wage brought a worker to 56 percent of the median wage. Throughout the 1950s and 1960s (see Table 5.4), the minimum wage hovered between 44 and 56 percent of the average wage. However, even the higher minimum wage hike of \$7.25 in 2009 brought a worker to only 34 percent of the \$40,711 U.S. national wage in that year. An increased minimum wage can result in a spillover effect, since some businesses hire at a “real minimum wage” that is \$1 to \$2 above the federal level.



The minimum wage has been criticized by both conservatives and liberals.

Source: Paul Vasarhelyi/Shutterstock

TABLE 5.4 Value of the Minimum Wage, Selected Years

Year	Percent of Poverty Line for a Family of Three	Percent of Average Wage	Minimum Wage, Nominal Dollars
1955	73	44	\$0.75
1960	88	48	1.00
1965	103	51	1.25
1968	120	56	1.60
1970	107	50	1.60
1975	101	46	2.10
1980	98	47	3.10
1985	81	39	3.35
1988	73	36	3.35
1989	71	35	3.35
1990	79	38	3.80
1993	75	39	4.25
1995	72	37	4.25
1997	83	42	5.15
2007	73	30	5.85
2013	71	37	7.25

Sources: Based on Lawrence Mishel, Declining Value of the Federal Minimum Wage is a Major Factor Driving Inequality, Economic Policy Institute, February 21, 2013. <http://www.epi.org/publication/declining-federal-minimum-wage-inequality/>.

The Living Wage Movement Started out as a push for local ordinances requiring private businesses that benefit from public money to pay their workers a living wage. Commonly, these ordinances cover employers who hold large city or county service contracts or receive substantial financial assistance from the city in the form of grants, loans, bond financing, tax abatements, or other economic development subsidies. Since 1994, community, labor, and religious coalitions have fought for and won similar ordinances in St. Louis, Boston, Los Angeles, Tucson, San Jose, Portland, Milwaukee, Detroit, Minneapolis, and Oakland. By 2000, there were more than 75 living wage campaigns under way in cities, counties, and states. The justification for these campaigns was that when subsidized employers pay workers less than a living wage, taxpayers pay in the increased taxes necessary to pay for food stamps and other income support services required by low-wage earners to support themselves and their families.⁶⁵

The Living Wage Movement morphed into the “Fight for 15,” campaign to raise the hourly minimum wage to \$15 an hour in cities. In April 2015, thousands of workers across the United States protested in front of fast-food restaurants and on major college campuses demanding \$15 an hour. Beginning as a fast-food worker’s issue (initiated by the Service Employees International Union—SEIU), it soon became something larger and encompassed workers like adjunct professors, home health care, child care providers, and Walmart employees.⁶⁶

In addition to the federal minimum wage, states can institute their own higher minimum wage (see Table 5.5). By 2015, 29 states and the District of Columbia set their minimum wages at rates higher than the federal level.⁶⁷ Several states annually adjust their wage rates according to changes in the cost of living.⁶⁸ Los Angeles County supervisors agreed to raise the minimum wage to \$15 an hour by 2020.

TABLE 5.5 States with Minimum Wages above the Federal Rate, 2014–2015

State	Minimum Wage (\$)
Alaska	\$8.75
Arizona	8.05
California	9.00
Colorado	8.23
Connecticut	9.15
District of Columbia	10.50
Florida	8.05
Illinois	8.25
Maine	7.50
Massachusetts	9.00
Michigan	8.15
Montana	8.05
New Mexico	7.50
Nevada	8.25
Ohio	8.10
Oregon	9.25
Rhode Island	9.00
Vermont	9.15
Washington	9.47

Source: U.S. Department of Labor, Division of Communications Wage and Hour Division, Minimum Wage Laws in the States – February 2014. Retrieved October 2015, from <http://www.dol.gov/whd/minwage/america.htm>.

Strategies Developed to Combat Poverty

For the vast majority of households, the road to ending poverty involves savings and the accumulation of assets. Assets are important when purchasing a home, sending a child to college, starting a small business, and reaching other economic goals. Moreover, when people begin to accumulate assets, their thinking and behavior changes, leading to important psychological and social effects that are not achieved when simply receiving and spending regular income.⁶⁹ Despite its importance, asset accumulation for the poor has been virtually ignored in welfare state policies. Moreover, the tax system does not reward asset accumulation for the poor in two primary areas: tax benefits for home mortgages and retirement pensions. These benefits

go largely to the better-off. In these two areas alone, the federal government spends well over \$100 billion each year, and the total is rising.⁷⁰ Even worse, many means-tested welfare programs count assets and savings as income in calculating eligibility, which means that the poor who have accumulated assets or savings must spend these down to qualify for assistance.

IDAs

The concept of individual development accounts (IDAs) was introduced in 1991 by Michael Sherraden in *Assets and the Poor*.⁷¹ IDAs are part of an asset-building strategy designed to reduce wealth inequality by enabling poor individuals to accumulate assets. Low-income individuals establish savings accounts matched by public and private resources, which are then used for home purchases, business start-ups, and postsecondary education.⁷²

In Sherraden's framework, IDA accounts would be established for all low-income individuals and would include tax benefits to foster asset accumulation. Individual savings would be matched by federal, state, or private contributions at varying rates. The matching system would be flexible and permit the government to supplement savings as the economic circumstances of individuals change. IDA withdrawals would be restricted to approved purposes such as home purchases, education, retirement, or the creation of a business. IDA assets could be transferred to children at death or prior to death.⁷³

Three Approaches to Combat Poverty

Policy analysts have identified three basic strategies for combating poverty. The first (used in the War on Poverty and Great Society programs) was to apply a curative or rehabilitative strategy to the problems of the poor. The **curative approach to poverty** aims to end chronic and persistent poverty by helping the poor to become self-supporting through changes in their personal lives and in their environment. By breaking the cycle of poverty, the curative approach strives to initiate the poor into employment and, later, the middle class. The second strategy is the **alleviative approach to poverty**, which is exemplified by public assistance programs that focus on easing the suffering of the poor rather than attacking the causes of poverty. The third strategy is the **preventive approach to poverty**, exemplified by social insurance programs such as Social Security. In this approach, people are required to utilize social

insurance to insure against the costs of accidents, sickness, death, old age, unemployment, and disability. The preventive strategy sees the state as a large insurance company whose umbrella shelters its members against the vicissitudes of life.

John Kenneth Galbraith's *The Affluent Society* identified two kinds of poverty: **case poverty** and **area poverty**. According to Galbraith, case poverty was the outgrowth of personal deficiencies (i.e., deficits in human capital). Area poverty was related to economic problems endemic to a region. "Pockets of poverty" or "depressed areas" resulted from a lack of industrialization or the inability of an area to adjust

to technological change. This kind of poverty was a function of the changing nature of the marketplace.⁷⁴

One example of a case poverty approach is the federal government's attempt to promote education as a means of increasing human capital. Poverty is highly correlated with educational deficits, and adolescent parenthood is strongly associated with low levels of basic skills and high school dropout rates.⁷⁵ To help address these educational deficits, the federal government initiated the Head Start program, targeted at poor children age three to five and their families.⁷⁶

Area poverty is illustrated by reviewing poverty rates on a state-by-state basis (see Table 5.6). While

TABLE 5.6 2013 Poverty Rates by State

Location	Total	Location	Total
United States	15%	Missouri	14%
Alabama	17%	Montana	15%
Alaska	11%	Nebraska	11%
Arizona	20%	Nevada	17%
Arkansas	17%	New Hampshire	9%
California	15%	New Jersey	11%
Colorado	11%	New Mexico	22%
Connecticut	11%	New York	15%
Delaware	14%	North Carolina	19%
District of Columbia	21%	North Dakota	10%
Florida	15%	Ohio	14%
Georgia	16%	Oklahoma	14%
Hawaii	11%	Oregon	15%
Idaho	13%	Pennsylvania	13%
Illinois	13%	Rhode Island	14%
Indiana	12%	South Carolina	16%
Iowa	11%	South Dakota	11%
Kansas	13%	Tennessee	18%
Kentucky	20%	Texas	17%
Louisiana	19%	Utah	8%
Maine	12%	Vermont	9%
Maryland	10%	Virginia	11%
Massachusetts	12%	Washington	12%
Michigan	15%	West Virginia	18%
Minnesota	12%	Wisconsin	11%
Mississippi	23%	Wyoming	12%

Source: Poverty Rate by Race/Ethnicity, The Henry J. Kaiser Family Foundation, 2013, <http://kff.org/other/state-indicator/poverty-rate-by-raceethnicity/>.

the 2013 national poverty rate was 15 percent, some states with extensive poverty pockets had much higher rates (e.g., Mississippi, New Mexico, District of Columbia, Alabama, Kentucky, Arkansas, Louisiana) compared to low poverty states like New Hampshire, Maryland, Alaska, Connecticut, and New Jersey.⁷⁷

These approaches to poverty are not merely hypothetical formulations; they form the basis for social welfare policy. Between 1965 and 1980, social welfare policies were grounded in the view that public expenditures should be used to stimulate opportunities for the poor. As a result, major social welfare legislation was enacted, and billions of dollars were earmarked for the remediation of poverty. However, beginning with the Reagan administration in 1980, there was a move away from reliance on social welfare expenditures and toward an emphasis on ending poverty through overall economic growth and tax transfers.

Consequently, public expenditures for poverty programs decreased and tax cuts—intended to give people incentives to work and save money—increased. The Reagan approach maintained that the poor should wait for gains coming from increased economic activity rather than relying on welfare programs. This perspective assumed that the trickle-down effect of economic growth would benefit the poor more than direct economic subsidies. According to analyst Kevin Phillips, “Low-income families, especially the working poor, lost appreciably more by cuts in government services [Under Reagan] than they gained in tax reductions.”⁷⁸ Despite the nation’s long-standing belief in eradicating poverty through labor market participation, the main factor responsible for the decrease in poverty rates from the 1960s to the late 1970s was governmental cash and in-kind transfers.⁷⁹ The cuts in income transfer and in-kind programs led to a more desperate search to make ends meet from one paycheck to the next.

America’s Fringe Economy⁸⁰

In 2008, America—and much of the world—experienced an economic crash not seen since the Great Depression of the 1930s. A massive credit bubble allowed Americans to spend more than they earned, and rising home prices and a robust stock market made it seem like the party would never end.⁸¹ From 1960 to 2008, the ratio of household

debt to personal disposable income more than doubled as Americans went on the longest sustained spending spree in history.⁸² By 2009, more than 83 percent of all consumer debt was secured by residential property that was rapidly declining in value.⁸³ In short, many average American middle-class households became accustomed to a lifestyle that could not be sustained solely by their income.

While much of the economic life of the middle class is rooted in credit,⁸⁴ many poor people encounter obstacles to basic credit and are thus vulnerable to exploitation by fringe businesses. The often shoddy storefronts of fringe financial businesses hide the true scope of this economic sector. For instance, 90 percent of payday lending revenues are based on fees from trapped borrowers who take out new payday loans to pay off existing ones (called rollovers). Other borrowers are repeat customers. Due to rollovers, the typical payday borrower pays back \$793 for a \$325 loan. In 2010 alone, payday lending fees cost poor American families upwards of \$4.7 billion a year in fees.⁸⁵

Bad economic times are good economic times for check cashers, payday lenders, pawnshops, rent-to-own stores, and the like. For instance, the number of payday loan outlets increased significantly from just 2,000 in 1995 to more than 20,000 by 2014. The total loan volume in 2009 was \$29.2 billion.⁸⁶ By 2010, 10 large corporations, including NASDAQ traded ones, owned or operate about 40 percent of payday loan stores in the United States.⁸⁷

America’s **fringe economy** is not a mom-and-pop operation composed of small storefronts that generate moderate family incomes; instead, it is a multibillion parallel economy that provides low-income people with a full range of cash, commodities, and credit lines. While pawnshops, check cashers, and payday lenders are a major part of the fringe economy, they are only the tip of the iceberg. This subeconomy also includes mainstream banks issuing high-interest credit cards and expensive check overdraft protection, high-interest home financing and refinancing loans, and deferred interest retail payments. Fringe economic services exist in every sector where people borrow or spend money.

The prices of commodities and financial services in the fringe economy have little relationship to their real value in the mainstream marketplace. Within this economic bubble, used cars can cost twice their book value, housing prices are

determined by the financial desperation of home buyers rather than the home's value, a 14-day \$200 payday loan can cost \$40 in interest/fees, and credit cards come with yearly and monthly service fees plus **annual percentage rates (APRs)** of 30 percent or more. In the fringe economy, economic distress and low credit scores translate into high costs that are justified by the supposedly higher risk of serving a poor or credit-challenged population.

The Unbanked and the Functionally Poor

The unbanked (e.g., individuals and families without accounts at deposit institutions) make up an important part of the fringe economy. About 12 million U.S. households (one-fourth of all low-income families) have no relationship with mainstream financial services, such as a bank, savings institution, or credit union.⁸⁸ The unbanked report lack checking accounts because (1) they do not write enough checks to warrant one, (2) they have almost no month-to-month financial savings to deposit, (3) they cannot afford high bank fees, (4) they cannot meet minimum bank balance requirements, (5) they want to keep their financial records private, and/or (6) they are uncomfortable dealing with banks. Almost 85 percent of the unbanked have yearly incomes below \$25,000.⁸⁹

One industry-funded study found that the average payday loan customer was female with children living at home, was between 24 and 44, earned less than \$40,000 a year, was a high school graduate, a renter, transient (most were in their residence less than five years), and had little job tenure.⁹⁰ This group represents the lower-and-moderate-income working class rather than the poorest of the poor.

The misconception that only the poor use the fringe economy overlooks the convergence of the traditional poor with growing segments of the middle class that are functionally poor. The functionally poor can include homeowners who are “over under” in their mortgages (i.e., they owe more than their house is worth), have huge credit card debt, have tarnished credit histories that precludes them from moderate interest loans, carry high-interest-rate credit cards, or finance their purchases through tricky time-deferred payments. In that sense, a burgeoning sector of the middle class is economically closer to the traditional poor than to the traditional middle class.

Credit and the Poor

Credit is a bridge between real household earnings and consumption decisions, offering relief during periods of economic distress and uncertainty.⁹¹ Payment options are flexible for those with good credit and collateral is not required. Hence, the middle class can purchase goods or services or borrow cash without losing their possessions. On the other hand, neither trust nor the presumption of goodwill exists in the fringe economy. A borrower with compromised credit typically must provide collateral such as a secured bank account, a post-dated check, household goods, or a car title. Those who manage to find unsecured credit are often charged high interest rates and face onerous loan terms.

Credit card use has become almost mandatory in the United States. Renting a car or reserving a hotel room or flight is almost impossible without a credit card. The average American credit card holder has 10 cards—four retail cards, three bank cards, one phone card, almost one gasoline card, and one travel and entertainment card. Not surprisingly, bank write-offs for uncollectible credit card debt was \$75 billion in 2010.⁹²

There are two basic types of credit cards: unsecured and secured cards. Unsecured credit cards are based on the cardholder's creditworthiness, past use of credit, and ability to repay debt. Conversely, secured credit cards require cardholders to guarantee their credit line by providing cash collateral, thereby making it almost impossible for the borrower to default.

Preloaded or Stored Value Debit Cards A variation of the secured credit card is the preloaded debit card. Using a Visa or MasterCard logo, these cards are similar to prepaid phone cards since they are preloaded with funds. When the funds are exhausted, the card is reloaded, or it becomes invalid. Unlike secured credit cards, pre-paid cards are not dependent on credit or banking history since they are not linked to a bank account. (Customers can use the entire amount loaded onto the card.) There is also no debt to repay. Like any fringe economy transaction, there are downsides. Usage fees are high and the cards do not build credit history since no credit or repayment is involved.

Telecommunications and the Alternative Services Market Consumer telecommunications are divided into two groups: post- and prepaid services. For postpaid (i.e., paying *after* a charge is accrued)

telephone services, customers are required to undergo credit checks and those without an acceptable credit rating are denied service and forced into the more expensive prepaid sector.

Postpaid cell phone service is also dependent on good credit scores, and clients with problematic credit are a lucrative part of the wireless industry. Like regular telephone service, prepaid cell phone plans are more expensive than post-paid plans.⁹³ The large difference in pricing between is perplexing since prepaying customers cannot default. The sole reason for the higher prices is the customer's economic vulnerability. Prepayment plans are also unfair because they allow corporations to use a customer's money without paying interest. Prepaid customers are therefore penalized twice—once by paying high prices and then again when the company uses their money without paying them interest. Consumers trying to build or rebuild their credit history also derive little benefit from prepaying because no credit is extended and therefore there is nothing to report to a credit agency.

The Furniture and Appliance Rental Industry The rent-to-own (RTO) sector targets low- and moderate-income consumers. RTOs advertise no credit checks, weekly or monthly payments, and a choice of appliances, furniture, and jewelry that would otherwise be unaffordable for these customers if bought outright.⁹⁴ The \$8.5 billion a year RTO industry employed 50,000 people in 9,800 storefronts and served almost 5 million customers in 2014. It is a major player in the fringe economy.⁹⁵ The RTO industry employs two approaches to transactions: (1) customers rent goods and pay weekly or monthly fees or (2) they rent-to-own with payments extending from 12 to 24 months. In either case, customers can usually cancel the agreement without further cost or obligation. They take ownership of the property if the contract is renewed a prescribed number of times (usually 12 to 24 months) or if they complete the lease agreement. No credit bureau reports are filed because RTO customers make advanced payments.⁹⁶

RTO rentals are expensive and can cost 150 to 300 times more than a normal retail sale. In 2014, one of the largest RTOs advertised a Hewlett-Packard 14" Android laptop for \$89.99 a month on a 12-month lease. The total price would be \$1,079.88. The laptop could be bought outright for \$809.99. Amazon listed the same laptop

for \$317.64 and Hewlett-Packard's MSRP was \$429.99. This RTO was charging 250 percent more than the MSRP for the rental and almost twice the MSRP. It is indeed expensive to be poor in America, especially if one shops at RTOs.

Successful fringe businesses long ago realized the importance of treating low-income customers well. Good customer service is important to poor consumers, especially those who have been humiliated by mainstream merchants after bad credit checks. Some poor consumers are so sensitive to poor treatment that they will pay more, sometimes much more, just to feel they are being respected. A former Rent-A-Center manager stated it succinctly, "If you treat the customer like royalty, you can bleed them through the nose."⁹⁷

Pawnshops Pawnshops are a high-growth industry, and at least five chains are publicly traded (i.e., EZ Pawn, Cash America Pawn, Express Cash, Famous Pawn, and First Cash Pawn). According to the National Pawnbrokers Association, the number of pawnshops in the United States has nearly doubled in the past 10 years from 6,900 in 1988 to more than 10,000 by 2014, a reflection of the tough economic times.⁹⁸

A pawnshop loan is a relatively simple transaction. A pawnbroker makes a fixed-term loan that is guaranteed by collateral. The customer is given a pawn ticket that includes their name, address, a description of the pledged good, the amount lent, the maturity date, and the amount of money needed to reclaim the property. The property is returned when the customer presents the ticket and pays the loan and fees in full by the agreed-on time. If the loan is not repaid, the collateral becomes the property of the pawnbroker, and the customer's debt is extinguished. Customers can repay a loan at any time during the loan term and redeem the collateral. However, many pawnshops have an open-ended policy, whereby the customer can extend the pawn indefinitely by paying only the loan interest. Pawnshops are typically regulated by state and sometimes by local governments; interest rates can range from 1.5 percent to 25 percent a month, depending upon the state regulation. While some state laws cap interest rates, loopholes often allow "lease back" agreements to add fees, sometimes effectively doubling interest rates.⁹⁹ The average pawn shop loan is about \$75 but can go as low as \$15.¹⁰⁰ Compared to payday loans, pawnshops are a relatively safe transaction

since there is no obligation to repay the loan and no ongoing financial responsibility beyond the loss of the item.

Car Title Pawns Car title loans are a cross between pawn loans (i.e., they involve secured collateral) and payday loans since they are single payment loans that can be churned or renewed several times. Higher dollar car title loans are usually set up as longer-term installment loans. Since most title lenders lend at 40 to 50 percent or less of the vehicle's value in rough condition, they will get their investment back even if the borrower stops making payments. If the lender repossesses the vehicle and resells for more than the loan value, the borrower receives nothing from the proceeds of the sale. Car title loans generate nearly \$2 billion in annual loans with borrowers paying more than \$4 billion in fees, or double the amount loaned.¹⁰¹

Car title pawns operate similarly to pawnshops. A consumer needs a short-term loan, but instead of using his or her television or stereo as collateral, they use their vehicle title. In most cases, this substantially increases the amount that can be borrowed since vehicles generally have more value than televisions, stereos, or microwaves. Unlike pawnshop transactions, the borrower does not forego the use of their property during the course of the loan, even though the vehicle is technically owned by the lender until the loan is repaid.

Car title pawns operate in the following way. A borrower provides the lender with a free and clear vehicle title and an extra set of keys. In return for a 30-day loan, the borrower either allows the title lender to keep the title or puts a lien on the vehicle. The borrower can ask for multiple 30-day extensions after paying the interest and fees. After several months, the loan will exceed the value of the vehicle.

Payday Loans These loans typically involve small loans that are supposed to be repaid within two to six weeks. More than 5 percent or 12 million Americans use payday loans as a pricey stopgap for financial problems.¹⁰² The average payday loan is \$430 but loan amounts can vary between \$250 and \$1,000, and lenders charge anywhere from \$15 to \$30 for every \$100 borrowed.¹⁰³ Higher dollar loans are called installment loans and involve a longer repayment period. Borrowers guarantee the payday or installment loans with a post-dated check or an authorization to debit their bank account.

If a borrower cannot afford to repay the loan, they can “roll it over” (extend the loan period) by paying an additional fee or they can secure a loan through another payday lender to repay the first loan. The Center for Responsible Lending estimates that this rollover or churning accounts for 75 percent of payday loans and for more than \$20 billion a year in industry revenues.¹⁰⁴ Despite industry claims that their loans are designed as one-off crisis cash, the research suggests that they are often used for day-to-day financial support.¹⁰⁵

Seventeen states and the District of Columbia cap interest rates on short-term loans at 36 percent, effectively discouraging payday lenders, given the profit margins expected in the industry. Military personnel and their families are protected by federal regulations that prohibit lenders from charging more than 36 percent interest on payday, car title, and tax refund loans. In response, payday lenders found and exploited loopholes in the law, which led the Obama administration to promulgate more comprehensive rules around lending to military personnel.¹⁰⁶

While the number of brick-and-mortar payday storefronts has declined slightly since 2009, the slack has been picked up by online lenders, the fastest growing segment of the payday lending industry. Online lenders saw their revenue grow from \$1.2 billion in 2006 to \$4.2 billion in 2012 with Internet sales making up 38 percent of all payday loans in 2012.¹⁰⁷ Online payday loans are typically more expensive than storefront loans and can result in unauthorized withdrawals that overdraw checking accounts, the release of personal information, and aggressive threats for nonpayment. They also have higher default rates. Payday loans are especially dangerous since they are not secured by physical collateral, and hence the buyer must repay the loans regardless of their circumstances.

Legislation around payday loans varies widely among states, with some jurisdictions limiting the APR that payday lenders can charge. Other jurisdictions outlaw payday lending entirely, while still others have few, if any, restrictions. Payday loans (and certain other financing) offered to military personnel and their dependents must include certain protections, such as a maximum 36 percent interest rate cap.

To qualify for a payday loan, the customer must have a valid checking account from which a check is issued (or electronically debited) to cover the interest and principal for the loan. Borrowers

must also provide recent pay stubs and valid identification. Many payday lenders consider benefits as income and will give loans to those on public assistance, recipients of child support or alimony, and Social Security beneficiaries.¹⁰⁸

Payday loans are typically issued 14 or 18 days. If a borrower took out a \$430 payday and paid \$17 per \$100, they would owe roughly \$473 in two weeks. This would translate into almost a 500 percent APR. (Nationally, the average APR for payday loans is 474 percent.¹⁰⁹) Because the lender may extend the loan (called a rollover) for additional 14- or 18-day periods (after the interest is paid), a \$430 payday loan extended to three loan periods will accrue almost \$220 in interest charges alone, elevating the full loan repayment to \$650 in less than two months. Payday loans can inflict potentially serious damage on borrowers, including criminal prosecution for writing a hot check.¹¹⁰

Collection tactics for payday loans can be aggressive. If a borrower cannot repay the loan, it may be turned over to a collection agency and result in the loss of a house or a car or the garnishing of wages. In some cases, the collection agency can add additional interest to the original debt.¹¹¹ Some payday loan companies require borrowers to agree beforehand to pay all fees related to the collection of their account, including attorney fees, collection fees, and court costs.¹¹² A default on a payday loan involves a worthless check, and some state credit laws allow for triple damages when a bad check is used in a retail transaction. Lenders may also require that customers sign statements authorizing them to go directly to the borrower's employer and ask for the amount owed to be deducted from their paychecks.

Check Cashing Outlets (CCOs) Behind these 11,000 plus storefronts lies an industry that cashes upward of 180 million checks a year with a face value of more than \$55 billion. The CCO industry generates nearly \$1.5 billion a year in revenues.¹¹³ In large urban areas, from 20 to 40 percent of the unbanked pay fees to cash their paychecks through CCOs.¹¹⁴ Many larger CCOs are one-stop financial service centers since they offer a wide range of services in one location: (1) check cashing; (2) utility and other bill pay services; (3) money transfers; (4) payday loans; (5) money orders; (6) telecommunications products (e.g., pre-paid long distance calling cards, pre-paid local phone service, cell phones, and beepers); and (7) other services such as fax

transmissions, copy services, stamps and envelopes, notary services, mailboxes, and lottery tickets.¹¹⁵

CCOs are an expensive way to cash checks. Most check cashing fees range from 1 to 10 percent (plus a service fee in some states) of the face value of a check.¹¹⁶ Check cashing charges are often based on a sliding scale, and Dollar—the second largest CCO—charges 3.5 percent, or \$35, to cash a \$1,000 payroll check.¹¹⁷ Fees for cashing personal checks can run as high as 10 to 12 percent of the value of the check. If a customer cashes twenty \$800 paychecks a year through Dollar, they will pay \$560—far more than the costs of even a deluxe checking account in a mainstream bank. High check cashing fees do not correspond to high risk since about 70 to 90 percent of all checks cashed at CCOs are relatively secure payroll checks with an average value of \$500 to \$600.¹¹⁸ Losses are also extremely low.

Transportation in the Fringe Economy

The path to car ownership for the poor is mined with high down payments, predatory financing, extortionate interest rates, and overpriced insurance. An understanding of the used-car industry is required to appreciate the obstacles faced by the poor. Used-car lots fall into two categories: franchised dealerships and independent lots. Franchised used-car lots are part of new-car dealerships and their cars tend to be newer, cleaner, and more expensive. Independent dealers frequently sell older and less-expensive vehicles, and hence a large number of poor buyers end up there, many of which are “here today and gone tomorrow.”

A major obstacle faced by the poor is that most mainstream lenders refuse to finance vehicles that are older than four to six years or have more than 100,000 miles on the odometer. Some lenders only finance vehicles purchased through franchise dealerships, and some further restrict that to dealerships that also sell new cars. These restrictions limit the choices for the poor since used cars sold by franchised dealers generally sell for top dollar.

Three tiers of financing exist for used-car buyers. The first is **prime lending**, which is offered to borrowers with a higher income and a good credit history. Interest rates are low because they are tied to the prime rate. The second tier is **subprime lending**, which is geared toward buyers with credit problems but who still have sufficient creditworthiness to secure a loan. Subprime loans carry higher

interest rates, involve a substantial down payment, and often require that a vehicle be purchased from a franchised dealership. The third tier is nonprime lending or dealer financing, whereby vehicles are financed in-house. This type of financing often carries the highest interest rates and can require weekly payments.

Buy-Here, Pay-Here used-car lots provide in-house financing and hence do not require buyers to undergo a credit check. Dealer-financed cars require a down payment and late payments can result in immediate repossession. In some cases, the buyer is expected to hand deliver their payments to the dealership weekly. Buy-Here, Pay-Here used-car lots are generally more profitable than franchised car dealerships.

Auto insurance is another area where the poor are hard hit. For example, many people with older cars insure them only for state-mandated liability rather than for collision (damage to their vehicle) coverage. It makes little sense to pay \$600 a year for collision coverage on a car worth \$1,000, especially with a \$500 deductible. Unfortunately, consumers who finance through Buy-Here, Pay-Here dealerships or subprime lenders must insure their vehicles for liability *and* collision, regardless of whether it is cost effective.

The poor are hard hit by high auto-insurance rates. According to one study, 92 percent of large insurance companies run credit checks on potential customers, which translate into insurance scores.¹¹⁹ These scores determine whether the carrier will insure an applicant and for how much. Those with poor or no credit may be denied coverage, while those with limited credit will likely pay high premiums. Although there is no evidence that residents in low-income or high minority zip codes are involved in more accidents, they pay more for basic auto insurance, even if they have been accident- and ticket-free for years.¹²⁰ Mounting consumer complaints have aroused the suspicion of some state insurance regulators that this may be a new form of redlining—a practice outlawed by the Fair Housing Act of 1968—since it discriminates against low-income, single-parent, young, and minority consumers whose credit histories may be less than perfect.¹²¹

While mainstream auto insurers calculate premiums based on six-month or one-year period, fringe auto insurers usually provide only monthly quotations. Many also require a sizeable down payment and a service fee. The premiums charged by

fringe insurers are exorbitant compared to mainstream auto insurers. These insurers get away with charging high premiums for several reasons: (1) they have captive consumers who were rejected by large insurance carriers; (2) the insurance requirements of Buy-Here, Pay-Here dealers and subprime lenders create a steady stream of buyers desperate for insurance; (3) in some states auto insurers are minimally regulated, and regulators are lax in rooting out predatory insurers, especially those serving the poor; and (4) many state vehicle inspections require a proof-of-insurance card before a vehicle can pass inspection. Some car owners pay the high monthly premium to get the card and pass inspection, after which they drop the coverage. This may explain why fringe auto-insurance rates are quoted monthly rather than biannually. Not coincidentally, fringe auto insurers take the pressure off mainstream carriers to cover the poor.

The poor pay more than the middle class for financial services in both absolute dollars and relative to their income.¹²² These costs are aggravated by a dual financial system, with one system for the poor and another for the middle and upper classes. This dual system leads to even greater inequality: Banks for the middle class and check cashers for the poor; access to savings tools for the middle class and barriers to savings for the poor; low-cost financial services for the middle class and high fee-based services for the poor. The fringe economy represents a financial exploitation that depletes the resources of poor families and communities. To better understand the poor, policy analysts must be aware of the myriad ways in which the poor are economically exploited.

To gain perspective on U.S. poverty, it is helpful to locate it within a wider international context. While the 15 percent rate of poverty in the United States is lower than in most developing countries, it is significantly higher than the 9.4 percent in Canada.

World Poverty

The good news about world poverty is that the number of people in developing countries living in extreme poverty (currently at \$1.90 per day) fell from 1.82 in billion in 1990 to under 1 billion in 2015. This trend could change if food prices rise due to climate change, and global economic conditions result in the reduced ability of rich nations to provide foreign aid along with cuts in charitable donations.

One indicator of severe poverty is the infant mortality rate, which is measured by the number of infant deaths per 100,000 live births. Developed countries with less than 10 deaths per 100,000 include most European countries, along with Qatar, Kuwait, Japan, Israel, Ireland, Australia, and New Zealand. In comparison, some of the poorer nations have extraordinarily high levels of infant deaths. Countries with more than 500 deaths per 10,000 live births include Afghanistan (1,400), Chad (1,200), Somalia (1,200), Guinea-Bissau (1,000), Liberia (990), Burundi (970), Central African Republic (850), Nigeria (840), Mali (830), Niger (820), Tanzania (790), Zimbabwe (790), Guinea (680), Angola (610), Cameroon (600), Lao (580), Burkina Faso (560), Mauritania (550), Mozambique (550), Kenya (530), Rwanda (540), and Malawi (510). Another indicator of poverty is life expectancy, which ranges from a high of 83.3 in Japan to a low of less than 49 years old in Guinea-Bissau, Sierra Leone, Afghanistan, and the Central African Republic. The average life expectancy in Sub-Saharan Africa is only 54 years old.

Global poverty is influenced by multiple factors. For one, most poor countries were part of the empires of European colonial powers. When the Europeans granted independence to their colonies, they often left nations with diminished resources and geographical boundaries based on political concessions and the self-interest of the colonizer.

Global poverty is often marked by food insecurity. The agricultural self-sufficiency of poor nations is hampered by cheap food imports from the United States, Europe, and Australia, which is partly driven by the subsidies given to Western farmers. This food problem is further exacerbated by the much touted “Green Revolution,” a series of agricultural technology transfers that began in the late 1960s. While the Green Revolution has helped feed much of the world, it has also led to the monoculture farming of cereals and grains (frequently used for livestock, export, and ethanol) rather than the traditional polyculture farming based on crop variety. While fewer people in the world die of starvation, many suffer from malnutrition and vitamin deficiencies as a result of high-starch and low-protein diets. The reliance on pesticides has also had a major impact on the ecology of farming areas and the quality of drinking water supplies.

Climate change in the form of droughts, floods, extreme heat, and violent storms is having a major impact on already marginal farming and grazing

areas in the developed and developing world. This change affects poor nations more than the richer ones since the very poor often live in climatically marginal locations with few resources for adaptation. For example, the population of Bangladesh and Burma live in densely populated deltas subject to flooding from storm and tidal surges. Sub-Saharan African countries—some of the poorest in the world—have populations that are highly dependent on unstable rains for their crops and grazing areas. Droughts have a devastating impact on these already semiarid areas. Although the poorest countries contribute the least to pollution and greenhouse gas emissions, they appear to suffer some of the harshest effects of climate change.

Another factor that drives world poverty is the graft and corruption that marks some governments in developing countries. This corruption drains economic growth and leads to bureaucratic institutions where widespread bribery is required to accomplish anything. Corruption also discourages foreign investment and ensures that resources and wealth remain in the hands of the small elite that run the country, thus diminishing the prospects for wider development. This system is reinforced by weak democratic structures that prevent an equitable distribution of resources and deny the poor a voice.

Attempts to remedy world poverty center around two major approaches to international development: the pro-growth and the pro-poor growth models. The pro-growth model maintains that aid strategies should focus exclusively on economic growth since poverty reduction will naturally occur as the economy expands. The latter model considers an economic policy as pro-poor if its main focus is on lessening inequality and reducing poverty. There is a debate about whether pro-growth policies skew income and inevitably lead to more inequality and higher poverty. Some development economists, like Martin Ravillion, have integrated both approaches by adopting a pro-growth policy as the center of a poverty reduction strategy.

Foreign or development aid is given by wealthier Western governments to assist poorer countries in poverty alleviation or for economic development. This aid is sometimes used as a cover to sell or provide arms to poorer countries. While humanitarian aid is one-time emergency assistance to address a natural catastrophe, the longer-term goal of foreign aid is used to alleviate structural poverty-related problems.

The largest donor nations in 2010 were (in billions): United States (\$30), United Kingdom (\$13),

Germany (\$13), EU (\$13), Japan (\$11), Netherlands (\$6), and Canada (\$5). The largest recipient nations were (in billions) were: Afghanistan (\$6); Ethiopia (\$4); Democratic Republic of the Congo (\$3); Haiti (\$3); Tanzania (\$3); Vietnam (\$3); India (\$3); Iraq (\$2); Nigeria (\$2); Sudan (\$2); Mozambique (\$2); Uganda (\$2); Kenya (\$2); and Rwanda (\$1).

Numerous secular and religious nonprofit organizations are also actively fighting world poverty. Some of these include the Bill and Melinda Gates Foundation, Christian Aid, Habitat for Humanity, Oxfam, World Vision, Save the Children, Heifer International, and Doctors without Borders. These organizations are classified as **non-governmental organizations (NGOs)**, nonprofit organizations that are independent of governmental control. NGOs try to achieve social, economic, and humanitarian goals within a nonpolitical environment. Although supposedly purely humanitarian, some religiously based NGOs proselytize and/or promote a conservative political and social agenda. There are an estimated 40,000 small and large NGOs (nongovernmental organizations) that operate worldwide, some of which originate within a developing country.

NGOs are typically involved in activities focused on health, farming, economic development, and education. For instance, the work of the Bill and Melinda Gates Foundation (gave almost \$4 billion in grants in 2014) includes support for developing a malaria vaccine and support for a large number of agricultural, health, financial, and educational institutions.

Numerous strategies have been proposed to curb or eliminate global poverty. For instance, at the 2000 United Nations Millennium Summit, world leaders agreed to eight development goals—now called the Millennium Development Goals (MDGs)—to be achieved by 2015. The MDGs are supported by major international organizations, such as the International Monetary Fund (IMF), the International Labour Organization (ILO), and the World Trade Organization (WTO), to name a few. The eight MDGs for poverty reduction were approved by all 193 UN member states and 23 international organizations. These goals include:

1. Eradication of extreme poverty and hunger
2. Provision of universal primary education
3. Promotion of gender equality and the empowerment of women
4. Reduction of child mortality rates

5. The improvement of maternal health
6. The reduction in the instance of HIV/AIDS, malaria, and other diseases
7. Achieving environmental sustainability
8. Developing a global partnership for development, including targets for aid, trade, and debt relief.

Microcredit or “microfinance” is another poverty reduction strategy that centers on providing financial services to low-income people who lack access to mainstream banking services. Microfinance is based on the idea that the poor can better their lives—perhaps even leave poverty—if given access to low-cost financial services and products. Those living in poverty are thus provided with a range of financial services to start or manage their businesses, to build assets, and to carry them in hard times. While a useful tool in the poverty reduction battle, microfinance is sometimes oversold as the panacea for eradicating poverty.

Despite impressive gains, it is too early to celebrate the decline in the numbers of people living in extreme poverty. For example, financial problems remain a salient feature of economic life, and while global food prices have been stable in recent years, they are high relative to income in the developing world. Writing in *The Observer*, John Vidal wonders how the world will feed an additional 2.5 billion people—an extra China and India—in 2050. The UN suggests that food production will have to nearly double to meet that need. At the same time, there are already 1 billion chronically hungry people in the world, a problem exacerbated by reductions in agricultural land. Climate change is also making food production more tenuous in marginal farming or grazing areas, especially those in already semiarid regions. Drinking water is also becoming increasingly scarce in some areas of the world, while overfishing is rapidly depleting an important food source. Vidal points out that while food production has nearly doubled in a generation, water use has increased threefold. He argues that unless dramatic changes occur, the current method of water and energy-intensive farming will exhaust the earth’s resources before the world is able to feed the additional people.

Conclusion

Poverty is one of the most intractable problems in the United States and elsewhere in the world. Because poverty is both a political and a social

issue, the policies surrounding it are often less than objective. For example, governments can halve the poverty rate simply by redefining the poverty threshold. In the United States, the official poverty rate can be lowered by placing a high dollar value on in-kind benefits such as food stamps and Medicaid. Conversely, governments can swell the ranks of the poor by moving the poverty line upward and increasing the income level at which people are defined as poor. Like all social policies, poverty-related policies exist in a context marked by political exigencies, public opinion, the economic health of a society, and the complex mask of ideology.

Although mountains of poverty-related data are available, policymakers remain uncertain as to the precise causes of poverty. What is known is that these causes are complex and involve, among other things, the effects of discrimination; the composition of family life, including the rise in single, female-headed families; teenage pregnancies; geographical location; and age. In large measure, the determination of whether a child is poor depends on chance—the family he or she was born into. Nobel Prize winning economist Joseph Stiglitz argues that “America has long prided itself on being a fair society, where everyone has an equal chance of getting ahead, but the statistics suggest otherwise: The chances of a poor citizen, or even a middle-class citizen, making it to the top in America are smaller than in many countries of Europe.”¹²³ Policymakers know that the skewed distribution of income and governmental tax and investment policies have a major impact on the numbers of people in poverty and on the extent of their poverty.

Most policymakers agree that employment is the best antipoverty program. Thus, work-related factors such as the value of the minimum wage (especially its relationship to mean incomes), the level of under- and unemployment, the rise or decrease in family incomes, and the general state of the economy all have a major impact on the level and extent of poverty. The availability of job training programs and the regulation of the dual labor market help determine the salaries workers will make. Taken together, these factors have caused poverty rates to remain higher in the United States than in many other industrialized nations. They have also helped make poverty seem like an intractable problem with few viable solutions.

DISCUSSION QUESTIONS

1. The measurement of poverty is both complex and controversial. Nevertheless, the way that poverty is measured has important consequences for the development of social policy in the United States. Describe some of the potential pitfalls in measuring poverty rates and discuss how the calculation of poverty rates affects the creation of social policy.
2. Working families make up an important and growing segment of the poor. What is causing the increase in the numbers of working poor? What specific policies should be implemented to reduce the number of working poor families?
3. Several theories have been advanced to explain why some individuals and groups of people are persistently poor while others are not. Theorists who have tried to tackle this problem include Daniel Patrick Moynihan, Oscar Lewis, and Edward Banfield, among others. Although all theories of poverty have intrinsic flaws, which theory or combination of theories described in this book (or elsewhere) do you think best explains the dynamics of poverty?
4. Many strategies have been developed to fight poverty, including the curative approach, the alleviative approach, and the preventive approach. Of these strategies, which is the most effective in fighting poverty and why? What alternative strategies, if any, could be developed that would be more effective in combating poverty?
5. Policy analysts have traditionally argued that jobs are preferable to welfare and that the lack of employment opportunities results in increasing needs for social welfare. Is this relationship apparent in your community? What is the evidence?
6. A commonly held belief is that government make-work jobs are inferior to private sector employment. Yet many New Deal jobs programs have made important contributions to the infrastructure of the nation's cities. What New Deal projects are evident in your community? If a new governmental jobs program were initiated, what community needs might it address?
7. The fringe economy is a high-growth sector that is adversely affecting the economic lives of the poor. What policies or programs would you propose that could regulate, constrain, or abolish the fringe economic sector? What legislative or policy reforms are needed?

NOTES

1. See Edward C. Banfield, *The Unheavenly City* (Boston: Little, Brown, 1966) and Oscar Lewis, *La Vida* (New York: Harper & Row, 1965).

2. Henry Goddard, *The Kallikak Family* (New York: Arno Publishers, 1911).
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The Voluntary Sector Today



Source: Brad Coville/The Wilson Times/AP Images

The heart of the **voluntary sector** is made up of private, non-profit organizations, which are ubiquitous at the local level (see Figure 6.1). Supporting the voluntary sector are philanthropic contributions that have been a mainstay for social service initiatives. Prominent human service agencies are embedded in communities nationwide, providing essential social services. Historically, voluntary organizations have been the platform for social justice movements. While voluntary sector organizations have been dependent on philanthropy from large foundations, individual contributors provide most of the donations to this sector of American social welfare. In recent decades, government purchase-of-service contracts have been an important source of revenues for nonprofit organizations. Finally, the responses to September 11, 2001, Hurricane Katrina in 2005, and the Great Recession of 2008 demonstrate the importance of the voluntary sector in responding to Americans in need.

During the last decade, welfare professionals have been reassessing the capacity of the voluntary sector to meet the nation's social welfare needs. As governmental expenditures for social welfare fail to increase in the face of rising demand for human services, the voluntary sector has been called on to shoulder more of the welfare burden. In the early 1980s, President Reagan appealed to the charitable impulses of Americans as a way for the nation to address human needs while reducing federal appropriations to social programs. The idea was restated by George H. W. Bush and affirmed by his reference to "a thousand points of light" during his 1988 presidential campaign. Although former president Clinton made no comparable gesture to the nonprofit sector, his statement that "the era of big government is over" had enormous implications for nonprofit agencies. The emergence of "compassionate conservatism" and its corollary, "faith-based social services," promoted by former president George W. Bush restated the conservative case for a reinvigorated voluntary sector. President Obama's continuation and expansion of the Office of Faith-Based and Community Service continue this trend.

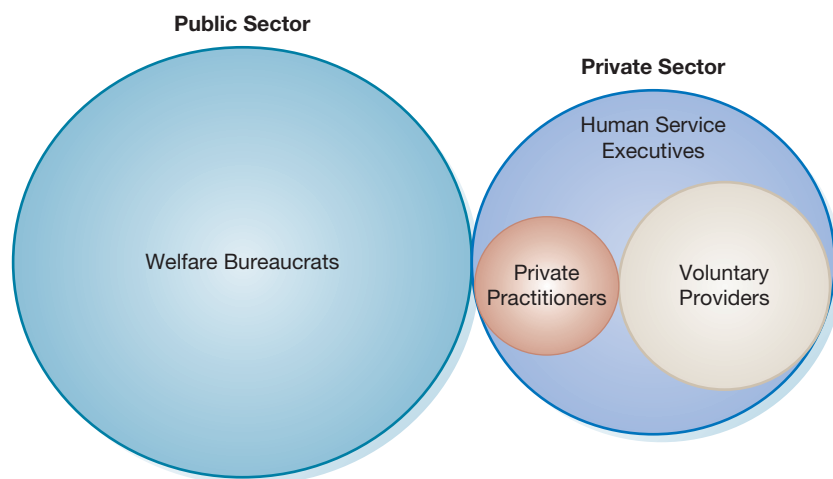


FIGURE 6.1 Dynamics of Structural Interests

Traditional Providers

The heart of the voluntary sector consists of professionals and laypersons seeking to maintain and enhance traditional relations, values, and structures in their communities through private, nonprofit agencies. Traditional providers see social welfare as tightly interwoven with other community institutions. According to traditional providers, voluntary nonprofit agencies offer the advantages of neighborliness, a reaffirmation of community values, a concern for community as opposed to personal gain, and freedom to alter programming to conform to changes in local priorities.

Much of the heritage of social welfare can be traced to early proponents, such as Mary Richmond of the **Charity Organization Society** (COS) movement and Jane Addams of the **settlement house** movement. Voluntary agencies routinized philanthropic contributions by socializing charity, through local initiatives that would become the United Way. Beginning with Denver's Associated Charities in 1887, the concept of a community appeal spread so rapidly that by the 1920s more than 200 cities had "community chests." The needs of social workers for effective treatment techniques and the economic imperatives for organizational survival functioned together to **standardize** the social agency. Perhaps the best description of the casework agency is found in the 1923 Milford Conference Report, *Social Casework: Generic and Specific*, which comprehensively outlined the organization through which professional caseworkers delivered services.¹ By the 1940s, the social casework agency had become a predominant

form of service delivery. Today, much social service provision exists in the form of the United Way—subsidized sectarian and nonsectarian agencies, whose member groups collected \$5.09 billion in 2010.²

The purpose of the voluntary sector is to advocate for and serve those populations not meeting the eligibility requirements for government programs or not representing a profit for commercial vendors. Marginal groups usually represent special populations that have been ignored, excluded, or oppressed by mainstream society. A partial list of marginal interest groups includes African Americans, poor women, Native Americans, members of the GLTBQ community, the disabled, residents of isolated rural areas, and Hispanic Americans. These groups are of concern to welfare professionals because typically they have not had the same opportunities as mainstream populations; in other words, they have been denied social justice. Within the context of democratic capitalism, elevating marginal groups and incorporating them in cultural mainstream remains extraordinarily difficult.

In U.S. culture, groups excluded from the mainstream are expected to gather their resources and identify leaders who will mount programs to serve their particular group. Although this expectation is congruent with traditional values such as self-sufficiency and community cohesion, the solidarity approach does not necessarily ensure success. Programs to aid victims of domestic violence and the homeless, for example, have often struggled

to mount and sustain minimal services because the United Way often has funding priorities consonant with the needs of clientele more highly regarded in the community. By the same token, mainstreaming GLTBQs has proceeded relatively rapidly, culminating in the 2015 Supreme Court decision validating same-sex marriage.

In a democratic polity, marginal groups can also make claims on the social order by seeking benefits through governmental programs; but doing so presents other problems. Government programs are likely to be managed by welfare bureaucrats, who have their own understanding of what is best for the marginalized. For example, during the 1980s, the effort to combat AIDS was plagued by volatile disagreements between gay advocacy organizations, such as Act Up! and public health bureaucrats who ran the Centers for Disease Control and Prevention.

The Independent Sector

When policymakers turned to the voluntary sector in the 1980s to compensate for reductions in the governmental welfare effort, little was known about nonprofit social service agencies. Such agencies include tens of thousands of organizations, many of which are not associated with any national umbrella association (see Table 6.1). Authorities on the voluntary sector observed, “despite their importance, these organizations have tended to be ignored in both public policy debates and scholarly

TABLE 6.1 Public Charities by Subsector, 2012

Field of Activity	Revenues (billions)	%	Number	%
Arts, culture, humanities	\$31.3	1.9	28,482	9.9
Education	\$284.9	17.2	49,070	17.1
Environment & animals	\$15.5	0.9	12,767	4.5
Health	\$978.8	59.3	37,374	13.0
Human services	\$206.6	12.5	101,803	35.5
International and foreign	\$29.3	1.8	6,100	2.1
Public & societal benefit	\$92.3	5.6	33,282	11.6
Religion related	\$13.4	0.8	17,542	6.1
Total	\$1,652.0	100	286,420	100

Source: Brice McKeever and Sarah Pettijohn, “The Nonprofit Sector in Brief 2014,” (Washington, DC: Urban Institute, 2014). Used with permission.

research.”³ As government assumed a dominant role in U.S. social welfare during much of the twentieth century, the voluntary sector receded in importance. If the Reagan and Bush presidencies rekindled interest in the voluntary sector, the 1994 Republican takeover of Congress created a small firestorm. Congressional conservatives had a strategy in mind when they critiqued federal social programs: Replace governmental welfare programs with private, voluntary sector initiatives. “The crisis of the modern welfare state is not just a crisis of government,” contended conservative scholar Marvin Olasky. “The more effective provision of social services will ultimately depend on their return to private and especially religious institutions.”⁴

By the end of the millennium, researchers had a portrait of the voluntary sector. The picture emerging from preliminary investigations revealed a sector that, if small compared to government and corporate entities, is extraordinarily rich socially. Perhaps the most convenient measure of the scale of the voluntary sector is to count the voluntary and philanthropic associations that received tax-exempt status as social service agencies from the IRS—1.44 million in 2012, an increase of 8.6 percent since 2002.⁵

Comprised of youth services, food banks, homeless shelters, and family services, Human Services involves over 100,000 nonprofits generating over \$200 billion in revenues, representing one-eighth of all charitable contributions. Coupled with related activities in health and education, the voluntary contribution to American social welfare is significant, indeed.

The vast majority of nonprofit organizations are small, 29.6 percent reporting revenues less than \$100,000 per year. Of more affluent nonprofits, several attributes are noteworthy. Elite philanthropy, often associated with symphonies, operas, and ballets (known by consultants as SOBs!), has visibility far beyond its share of revenues. By far, the largest components consist of colleges and universities (education) and hospitals (health). While human services show the largest number of nonprofits, their total revenue is far less than education and health, indicative of organizations with much smaller budgets.

Although nonprofits focusing on foreign affairs represent the smallest cohort of American voluntary sector organizations, their value is far greater

TABLE 6.2 Total Net Engagement by United States in Developing Countries

Component	\$Billions	%
U.S. official development assistance	30.4	9
U.S. private philanthropy	39.0	12
Foundations	4.6	12
Corporations	7.6	19
Voluntary organizations	14.0	36
Volunteerism	3.7	9
Universities and colleges	1.9	5
Religious organizations	7.2	18
U.S. remittances	95.8	29
U.S. private capital flows	161.2	49
Total	326.4	100

Source: Carol Adelman, “Global Philanthropy,” (Washington, DC: Hudson Institute, 2012), p. 8. Reproduced with permission of Hudson Institute.

than this number represents. According to the Hudson Institute’s Index of Global Philanthropy, international aid through private, voluntary sector organizations eclipses direct federal aid by a significant margin (see Table 6.2).

Contributions to the voluntary sector vary with prosperity, yet the performance of the sector has been mixed. Between 2007 and 2012, charitable contributions to nonprofits in general increased from \$1.55 billion to \$1.65 billion, and donations to Human Services specifically grew from \$193.9 billion to \$206.6 billion.⁶ Although the rich might be expected to contribute more to nonprofits as their share of wealth has increased, during the Great Recession they contributed less. Between 2006 and 2012, taxpayers reporting incomes at least \$200,000 annually decreased their donations by 4.6 percent, compared to those earning less than \$100,000 who contributed 4.5 percent more.⁷ In addition to financial contributions, volunteer hours represent a significant benefit to nonprofit organizations; in 2013, volunteers represented the value of 4.8 million full-time employees, or \$163.0 billion.⁸

Public support for nonprofit organizations has suffered somewhat from controversies affecting a

number of prominent nonprofits. A dispute about the use of contributions to aid victims of 9/11 led to the resignation of the director of the American Red Cross. Questionable practices in response to 9/11 were compounded by the Gulf Coast disaster of Hurricane Katrina. (Ultimately, the Red Cross would go through seven executives in as many years and have to cut up to one-third of its head-quarter staff to address a \$200 million deficit.⁹) And victims of the Oklahoma City bombing questioned why they were not able to get assistance for their losses comparable to that provided to victims of 9/11.¹⁰ Even though profound questions were raised during the last decades of the twentieth century about whether the voluntary sector could replace government social programs, there is little doubt that philanthropy will expand significantly during the twenty-first century. Aside from normal economic growth, the value of the four tax cuts, enacted by the administration of George W. Bush, were expected to have significant philanthropic consequences. According to Paul Schervish,

We estimate that an unprecedented \$45 to \$150 trillion in wealth transfer is in the offing over the next five decades, that this will produce at least \$21 trillion to \$55 trillion of charitable giving, and that between 52 percent and 65 percent of this amount will be contributed by households with \$1 million in net worth.¹¹

The extent to which such unprecedented wealth will be used to amplify *moral citizenship*—fulfilling the inclinations and obligations of caring—will be one of the challenges of the twenty-first century.¹²

Advancing Social Justice

In addition to providing social services, the voluntary sector has been important in U.S. social welfare because it has been the source of efforts to advance the rights of disenfranchised populations. In this respect, the voluntary sector is essential to the nation's culture, in that it is a correcting influence to the indifference often shown to marginal populations by governmental and corporate bureaucracies. John W. Gardner, former secretary of Health, Education, and Welfare and chairman of the board of Independent Sector, contended that the voluntary sector fostered much of the pluralism in the life of

this country, taking on those concerns that do not attract the broad spectrum of public support necessary for the legislation that mandates governmental programs, and concerns that do not represent the commercial prospects necessary to attract the interests of the business community. Indeed, much of what Americans would identify as central to their culture can be attributed to organizations of the voluntary sector: hospitals, schools, religious institutions, welfare agencies, fraternal associations, symphonies, and museums, to name a few. According to Gardner,

Institutions of the nonprofit sector are in a position to serve as the guardians of intellectual and artistic freedom. Both the commercial and political marketplaces are subject to leveling forces that may threaten standards of excellence. In the nonprofit sector, the fiercest champions of excellence may have their say. So may the champions of liberty and justice.¹³

Gardner's last reference here is not merely rhetorical but has its basis in history. Gardner noted that "... virtually every far-reaching social change in our history has come up in the private sector: the abolition of slavery, the reforms of populism, child labor laws, the vote for women, civil rights, and so on."¹⁴

Important social welfare initiatives have also originated in the voluntary sector. All social movements have been the product of voluntary sector organizations, nonprofits, such as the AFL-CIO, the backbone of the labor movement, the National Association for the Advancement of Colored People, the platform for the Civil Rights Movement, and the National Organization for Women, the basis for the Women's Movement. More recently, services to battered women, patients with AIDS, and the homeless have been pioneered by voluntary sector organizations. Given the public apathy toward these groups, for many years the voluntary sector was the only source of service for these groups.

That social change begins in the voluntary sector has a particular lesson for human service professionals: The openness of democratic American culture means that anyone is free to organize for purposes of rectifying past injustices. Wendy Kopp's Teach for America (TFA) is a good illustration. In 1988, armed with a vision of a Peace Corps-like

program for inner cities and rural areas, the then 23-year-old Kopp began to hustle corporate contributions to match idealistic professionals to disadvantaged communities. In 1996, TFA boasted a \$5.5 million budget and was graduating 500 volunteers annually¹⁵; by 2012 9,000 volunteers were teaching 600,000 students through a TFA budget augmented by a \$100 million endowment.¹⁶ In one of the most distressed communities in America, Harlem, Geoffrey Canada established the Harlem Children's Zone, in the process becoming an exemplar for the Obama administration's America's Promise initiative.¹⁷

The United Way

Perhaps the best recognized of voluntary sector organizations is the United Way. Local United Ways, as well as the United Way of America, are nonprofit organizations. The purpose of 1,800 local United Ways is to raise funds that are then disbursed to nonprofit agencies in the community, most of which are United Way members. Local United Ways also contribute a small percentage of funds to the national headquarters, the United Way of America. Because the United Way is a confederation of organizations, power resides within the local United Ways. The United Way of America provides support services nationwide but has no direct authority over local United Ways. In 2012, the total revenue for the United Ways of America was \$5.273 billion, making it the largest private nonprofit in the United States.¹⁸

United Way contributions have oscillated over time. Revelations of managerial improprieties on the part of United Way CEO William Aramony in the early 1990s precipitated a reduction in donations (adjusted for inflation) to United Ways.¹⁹ After the Aramony scandal, administrative problems within the United Way continued. In 2003, Oral Suer, head of the United Way of the National Capital Area (Washington, DC), was found to have misappropriated \$1.5 million through various financial arrangements. After the Suer scandal, contributions cratered, causing the organization to lay off half of its staff, close several satellite offices, and forfeit its lucrative fundraising contract with federal employees.²⁰ In a sector where credibility is a virtue, such damage requires years to repair. Fortunately for the United Way, contributions have



One of the most well-known nonprofit human service organizations is the United Way.

Source: Mark Richard/PhotoEdit, Inc.

modestly rebounded in the recent years, despite the effects of the Great Recession.²¹

Elite Philanthropy

The United Way offices that dot the nation's landscape, which pursue relatively modest goals, can be classified as "bourgeois" philanthropy. "Elite" philanthropy, on the other hand, has much grander ambitions. During the late 1990s, Ted Turner announced an unprecedented \$1 billion contribution to the United Nations; this gift was matched by Bill Gates, whose foundation pledged \$1 billion for various disease prevention initiatives in the Third World, notably work on HIV/AIDS in sub-Saharan Africa.²² By 2004, the Gates Foundation was investing \$600 million annually for prevention of diseases that ravaged developing nations, significantly outspending other charities.²³ In 2006, Warren Buffett, reluctant to establish his own foundation, donated over \$6 billion to the Gates Foundation, raising its assets to \$44 billion.²⁴ As a result of their collaboration, Gates and Buffett announced an initiative to encourage the wealthiest Americans to commit half of their wealth to philanthropy; by 2010, 40 families had subscribed, representing \$600 billion²⁵; a year later the number had grown to 69. The major philanthropic foundations are identified by the names of the titans of U.S. capitalism (see Table 6.3).

Spotlight 6.1

Despite the substantial philanthropic activity of foundations—gifts of \$43 billion in 2007²⁶—critics questioned the motivation behind their generosity. Accusations of impropriety, fraudulent activity, and stinginess have plagued the foundation world. In addition to criticisms of fund misuse, many people question the motives of large philanthropies and the individuals who support these organizations. “Elite American philanthropy serves the interests of the rich

to a greater extent than it does the interests of the poor, disadvantaged, or disabled,” argued Teresa Odendahl; it is “a system of ‘generosity’ by which the wealthy exercise social control and help themselves more than they do others.”²⁷ Odendahl calculated that about one-half of all gifts itemized on income tax returns were made by the wealthy.²⁸ For what purposes do the rich demonstrate such beneficence? They give to institutions that “sustain their culture, their education, their policy formulation, their status—in short, their interests.”²⁹ In other words, wealthy Americans usually used the tax code to maintain institutions of elite culture. The use of funds that have been withdrawn from public use by the tax code in order to reproduce the social institutions of the rich was unacceptable to Odendahl, who proposed reforms that would require philanthropy to be put to public benefit.



The Gates Foundation has committed billions of dollars to eradicate polio, malaria, and HIV/AIDS in developing countries. Watch this video in which Bill Gates explains his philanthropic motivation.

www.youtube.com/watch?v=ejgjQP1T-iA

TABLE 6.3 Largest Philanthropic Foundations

Rank	Name/(State)	Assets	As of Fiscal Year End Date
1.	Bill & Melinda Gates Foundation (WA)	\$41.3 billion	12/31/13
2.	The Ford Foundation (NY)	\$12.3 billion	12/31/13
3.	J. Paul Getty Trust (CA)	\$11.1 billion	06/30/13
4.	The Robert Wood Johnson Foundation (NJ)	\$10.2 billion	12/31/13
5.	W. K. Kellogg Foundation (MI)	\$8.6 billion	8/31/14
6.	The William and Flora Hewlett Foundation (CA)	\$8.6 billion	12/31/13
7.	Lilly Endowment, Inc. (IN)	\$7.7 billion	12/31/13
8.	The David and Lucile Packard Foundation (CA)	\$6.9 billion	12/31/13
9.	Gordon and Betty Moore Foundation (CA)	\$6.4 billion	12/31/13
10.	John D. and Catherine T. MacArthur (IL)	\$6.3 billion	12/31/13
11.	The Andrew W. Mellon Foundation (NY)	\$6.2 billion	12/31/13
12.	Bloomberg Philanthropies (NY)	\$5.4 billion	12/31/13
13.	Silicon Valley Community Foundation (CA)	\$4.7 billion	12/31/13
14.	Helmsley Charitable Trust (NY)	\$4.2 billion	3/31/13
15.	The Rockefeller Foundation (NY)	\$4.1 billion	12/31/13
16.	Tulsa Community Foundation (OK)	\$3.7 billion	12/31/12
17.	The California Endowment (CA)	\$3.7 billion	3/31/14
18.	The Kresge Foundation (MI)	\$3.5 billion	12/31/13
19.	The Duke Endowment (NC)	\$3.4 billion	12/31/13
20.	The John Templeton Foundation (PA)	\$3.4 billion	12/31/13

Source: Top 100 U.S. Foundations by Asset Size, Top Funders, Copyright © 2015 Foundation Center. Used by permission.

A unique development in foundation giving has been the emergence of Patient Assistance Foundations (PAF), established by major pharmaceutical companies as a way to provide aid to indigent Americans needing assistance to purchase medication. Eight of the top 10 foundations by amount donated are PAFs, each donating between \$600 million and \$800 million annually.³⁰

The Future of the Voluntary Sector

Despite the visibility of elite philanthropy, most Americans voiced preference for voluntary efforts to address social problems. Of respondents to a poll of the public's perception of problem solving, 56 percent ranked sectarian agencies as important for "solving social problems in their communities," compared to 53 percent for local nonprofits, 39 percent for the United Way, 33 percent for state government, and 28 percent for the federal government.³¹ The ability of the nonprofit sector to meet these public expectations is problematic, however. Several forces have transformed the nonprofit sector, including the intrusion of commercial firms, the rise of faith-based social services, and the emergence of social entrepreneurship.

Commercialization

In the mixed welfare economy, the voluntary sector coexists with government but also competes with commercial providers. This places nonprofit organizations in an odd position: Having secured a tax-exempt status from government in order to provide charity care, many voluntary sector agencies find themselves competing with for-profit corporations in the same market. The commercialization of health care, described in Chapter 7, is a case in point. In order to compete with for-profit hospitals, nonprofit hospitals have adopted similar billing practices, including dunning patients whose bills are in arrears. A controversy erupted when nonprofit hospitals threatened to sue poor patients in order to collect on outstanding bills, a contradiction of the basis for their tax exempt status: serving the indigent.³² Subsequently, the Affordable Care Act includes provisions for reviewing nonprofit hospitals every three years to assure they serve the poor; economists calculate tax exemption

benefits nonprofit hospitals about \$12 billion annually.³³

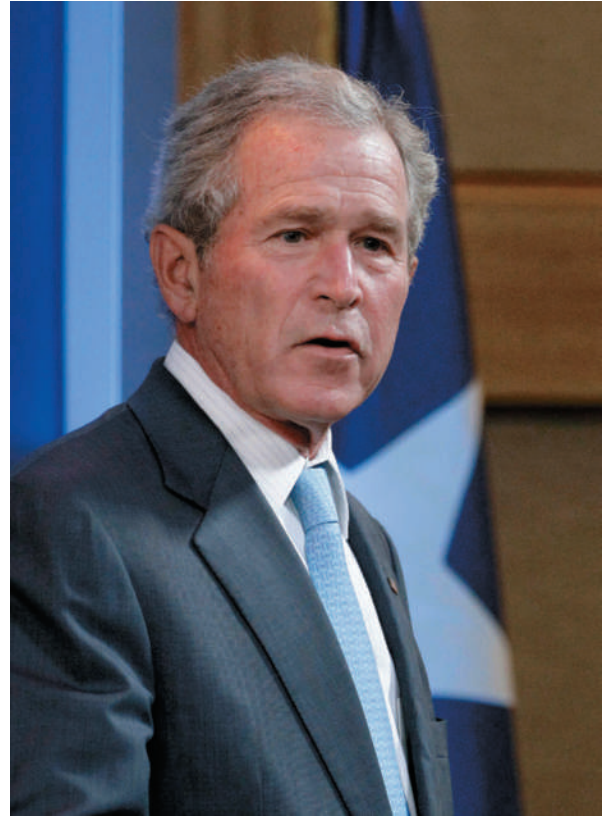
The **commercialization** issue is particularly important for nonprofit agencies desiring to enhance their incomes. Faced with static, and sometimes declining, revenues, some voluntary sector agencies have experimented with commercial activities in order to supplement income derived from traditional sources (contracts, contributions, grants, and fees). That nonprofit organizations should be allowed to engage in commercial endeavors without restriction is "unfair competition," according to business operators, who note that nonprofits do not ordinarily pay taxes on their income. Business operators fear that, exempt from taxation, nonprofits could lower their prices, increasing pressure on tax-paying businesses operating in the same market, eventually driving them out of business. This issue was resolved by a celebrated legal case involving the alumni of the New York University Law School purchasing the Mueller Macaroni Company as a subsidiary of the law school in order to claim company profits as alumni contributions. Since the law school was a nonprofit, competing pasta companies feared that their prices could be undercut and challenged the purchase. They won, and Congress quickly established the Unrelated Business Income Tax in order to regulate unfair competition.³⁴ As a result, in 2004, 38,000 nonprofits collectively were required to pay \$368 million in federal taxes on unrelated business income of \$9.5 billion.³⁵ As a result, nonprofits can operate commercial ventures, but only so long as they are congruent with the main mission of the agency; unrelated commercial activities are subject to the Unrelated Business Income Tax.

Finally, the unfair competition issue presents other disturbing questions for administrators of nonprofit organizations. "We can't ask nonprofits to be more like for-profits in the ways that we like—efficient, responsive, aggressive—without expecting that they will also become more like for-profits in the ways that we don't: rapacious, hardheaded and, yes, sometimes selfish," noted Jacob Hacker.³⁶ By the same token, because they are tax exempt, nonprofits enjoy an advantage, perhaps as high as 25 percent, over commercial rivals, so there is little reason for them not to provide high-quality services through state-of-the-art management; yet, nonprofit programs are often perceived as inferior to their for-profit rivals.

Faith-Based Social Services

The idea of government's contracting with voluntary sector agencies is not a novel concept; through purchase-of-service agreements dating to the late 1960s, government has negotiated arrangements for delivery of a range of social services, often with sectarian agencies. This liberal mechanism for decentralization, however, has given conservatives an entrée to pose more fundamental questions about the proper role of government in social welfare. Since the 1980s, conservative think tanks have, with increasing urgency, sought an alternative to federal social programs.

A primary proponent of sectarian nonprofit agencies has been Marvin Olasky, a journalist who has argued that voluntary religious agencies provided efficient and effective services to the needy well before the New Deal and the deployment of the welfare state.³⁷ Another source of support has come from evangelical Christians, who have objected to the absence of moral standards that they see as characterizing traditional social service agencies managed by liberal welfare professionals. Since the election of Ronald Reagan, conservatism had been in opposition to liberal social policy; however, it lacked a positive stance with respect to a large segment of the electorate—the middle class, the elderly, and the ill, among others. Invoking the virtues of the voluntary sector provided a way to address this ideological deficit, an approach that George W. Bush enthusiastically proclaimed during his 2000 presidential campaign as “compassionate conservatism.”³⁸ Eventually, Bush proposed a five-year, \$5 billion federal program for faith-based organizations; however, Democrats objected because of the possibility of discrimination in hiring, and the proposal failed. Undaunted, Bush instituted part of the plan through executive order. The Bush administration's designs for faith-based services received a boost when a federal judge determined that sectarian agencies could dismiss employees who refused to adhere to religious precepts of their sectarian employers.³⁹ Ultimately, even conservatives expressed reservations about the “faith-based” policy initiative. David Kuo, deputy director of the White House Office of Faith-Based and Community Initiatives, questioned the sincerity of the Bush administration in *Tempting Faith: An Inside Story of Political Seduction*, a book that noted the manipulation of initiative in order to curry the support of religious conservatives.⁴⁰



Former President George W. Bush advanced faith-based social services as an alternative to traditional social programs.

Source: Tony Gutierrez/AP Images

Regardless, the Bush faith-based initiative diverted \$10.6 billion to religious organizations.⁴¹ President Obama continues Bush's faith-based social policy under the renamed Office of Faith-based and Neighborhood Partnerships.

Proponents of faith-based social services, including many traditional sectarian agencies with a long history of helping the needy, such as the Salvation Army, insisted that government programs were too bureaucratic and impersonal, and that secular nonprofit agencies were populated by welfare professionals who failed to infuse services with adequate moral content to be effective. Although Bush's “faith-based” policy initiative may not have generated the revenue promised, it not only demonstrates how appeals to the voluntary sector resonate with conservative Republicans, but Obama's continuation of the policy shows how important nonprofit organizations are to Democrats as well.

Social Entrepreneurship

Advocates of the voluntary sector also pursued innovation through capital and technology. A new generation of social entrepreneurs emerged as experiments with markets revealed untapped opportunities for program innovation, both locally and nationally. For example, the Ben & Jerry's ice cream company and Working Assets Long Distance (WALD) [now CREDO] represent an ethic in which business praxis is inextricably suffused with social consciousness.⁴² In 2007, CREDO contributed \$3.5 million to progressive organizations advocating in several areas: social justice, the environment, civil rights, and international justice.⁴³ It is worth acknowledging that an iconic institution that has lifted millions of peasant women out of poverty, the Grameen Bank, is a commercial enterprise.⁴⁴ Within the American context, social enterprise may provide a productive, middle ground for putting profit to social purpose.⁴⁵

Concerned about the decline of civic institutions, particularly in urban areas, sociologists proposed "social capital" as a vehicle for revitalization. For Robert Putnam, the term *social capital* referred to "features of social organization, such as networks, norms, and trust, which facilitate coordination and cooperation for mutual benefit. Social capital enhances the benefits of investment in physical and human capital."⁴⁶ At the nexus of social capital, community-based nonprofit organizations can facilitate the development of poor neighborhoods, provided they exploit new markets and technologies. An illustration of the social capital approach to community development is the work of former congressman Floyd Flake, pastor of the Allen African Methodist Episcopal Cathedral in Jamaica, New York.⁴⁷ Flake has used his position as church pastor to construct a \$34 million network of community development ventures. Seeing the future as a social entrepreneur, Flake recognizes the ideological implications of recent changes in social policy: "Those of us who have made a commitment to stay in an urban community have decided that *this* is our paradise," he observed. "We are going to rebuild that paradise—and we understand that it means some paradigm shifts, even politically, because the majority of statehouses today are in the hands of Republican governors and the majority of the assemblies are in the hands of Republicans. So we can either continue in a protest mode or we find ways to have entrée to deal with who is in power now."⁴⁸

Although the social entrepreneur movement has been slow to evolve in relation to social

welfare, it has captured the imagination of international development advocates. David Bornstein championed their cause in his best-selling book, *How to Change the World: Social Entrepreneurs and the Power of New Ideas*.⁴⁹ The latent capacity of the commercial sector to provide micro-credit loans to low-income families has spawned financial institutions worldwide.⁵⁰ As the social entrepreneur gospel spread, its proponents characterized it as qualitatively different from traditional nonprofit or commercial organizations. David Bornstein and Susan Davis argue that social entrepreneurship represents a new generation of organizational activity, the use of collective agency to address pressing social problems:

Social entrepreneurship represents a fundamental reorganization of the problem-solving work of society: a shift from control-oriented, top-down policy implementation to responsive, decentralized institution building. It draws on a core insight of the twentieth century: namely, that a dynamic marketplace of ideas and initiative is the basis of a thriving economy.⁵¹

In addition to nonprofit organizations, social entrepreneurship has been endorsed by major businesses. Goldman Sachs, a leading investment house, announced a \$250 million investment in social impact bonds, an arrangement through which investors commit funds in exchange for future returns providing outcomes are met, while Morgan Stanley promised \$10 billion for public projects.⁵²

Within social entrepreneurship lurks a critique of government. Public social programs may anchor the welfare state, but they are hardly champions of innovation; legislated by a majority of elected representatives, government social programs, of necessity, are the product of a cultural consensus. Invariably, public policies take decades to be enacted, and many suffer from inadequate resources upon implementation. Finally, public programs evolve stakeholders who become dependent upon them. "Once a program is rolled out, with a budget and a constituency to defend it, it will remain there almost regardless of its effectiveness."⁵³ By contrast, social entrepreneurs are opportunistic, flexible, innovative, and accountable. In contrast to the skepticism of traditional welfare advocates toward capitalism, social entrepreneurs embrace its advantages. "The new breed of philanthropic and social

entrepreneurs want to see measurable results, and soon; they embrace business and businesslike methods; they want projects to be sustainable and scalable, capable of living and growing on their own.”⁵⁴ Michael Bloomberg, former Mayor of New York City and founder of Bloomberg/Businessweek, has stated that philanthropy should “embolden government” by addressing issues such as gun control, global warming, and obesity.⁵⁵

Issues Facing the Voluntary Sector

Minimal IRS requirements for nonprofit status coupled with the openness of American society have placed the voluntary sector under scrutiny for what many perceive as abusive practices.

Frivolity Stanford University’s Center on Philanthropy and Civil Society has taken the IRS to task for indiscriminate approval of nonprofit applications, authorizing tax exemption for organizations such as The Sisters of Perpetual Indulgence (an organization of drag nuns), Ghostface Ryderz (family-oriented motorcyclists), and the Metemprion Foundation (encouraging telepathic communication).⁵⁶

Politicization Loosening of regulations around campaign contributions have resulted in private groups using nonprofit provisions of the tax code, such as section 527, to allow unlimited, although indirect, candidate advocacy. While the IRS rejected nonprofit status for organizations training candidates for office, it failed to take action on Super-PACs, which expanded after the *Citizens United* decision rendered by the United States Supreme Court.⁵⁷ Perversely, some Super-PACs have relied on the “social welfare” provisions of the tax code, sections 501(c)(4-6), to shield the identity of donors, a practice aptly called, “dark money.” In 2015, the tens of millions contributed to the Clinton Foundation, to address world poverty, presented an issue of undue influence by foreign parties, when Hillary Clinton declared her candidacy for the presidency.⁵⁸

Property Tax Exemptions Tax-exempt institutions, particularly churches, colleges, and hospitals, fail to pay local taxes to reimburse cities for essential services, such as water, sanitation, and public safety.⁵⁹ For example, an analysis of the New York City budget revealed that \$13.5 billion was forfeited

due to nonprofit exemptions from paying the property tax.⁶⁰

Religious Precepts A primary motive for sectarian nonprofits is to promote religious values; however, these may conflict with public policy. Catholic Charities, for example, has terminated services in several jurisdictions rather than comply with rules to which it objects, such as allowing homosexuals to adopt children.⁶¹

Celebrity Philanthropy Seeking public visibility, nonprofits often solicit support from celebrities, some of whom adopt a signature cause, such as Bono and African development and George Clooney and Sudan. Global Philanthropy Group has developed this as a business model, charging celebrities \$25,000 per month to promote their causes.⁶² Doubtless, nonprofits appreciate benefiting from such exposure, yet celebrity philanthropy raises the prospect that its real value is less social welfare and more entertainment.

Salaries Large nonprofits, such as universities and hospitals, have paid executives salaries comparable to those managing corporations of comparable size. The adoption of a business model for executive compensation raises questions when the purpose of the nonprofit is clearly social welfare. If the CEOs of the American Cancer Society and the American Heart Association claim salaries of \$1 million, should the leader of Boys & Girls Clubs be similarly compensated?⁶³ A journalist investigating care of the disabled in New York uncovered a network controlled by two brothers whose nonprofits earned them million dollar salaries, paid primarily by Medicaid.⁶⁴ Certainly, management of nonprofit organizations can be lucrative for CEOs, as evident Table 6.4.

Conclusion

There are a number of important challenges facing the voluntary sector: How will the nonprofit preserve its mission of caring for the disenfranchised without succumbing to the bottom-line ethos of the corporate sector? How will it respond to the range of diverse populations without favoring the mainstream? How will it adapt to the latest innovations in technology while putting forth a human face on social welfare? Finally, how will the nonprofit sector respond to national and international challenges since its creation over a century ago?

TABLE 6.4 Salaries of Nonprofit Executives

Category	2012 Median Salary	2012 Maximum Salary
Animals	\$105,865	\$3,394,434
Arts, Culture, Humanities	150,000	840,000
Education	158,847	1,150,927
Environment	120,101	616,146
Health	135,000	2,731,016
Human Services	110,100	3,206,088
International	120,000	942,089
Public Benefit	133,011	3,733,930
Religion	85,387	533,819

Source: Charity Navigator: 2014 CEO Compensation Study, p. 5. Used with permission.

DISCUSSION QUESTIONS

1. What are the most prominent nonprofit human service agencies in your community? Are they members of the United Way? What do agencies perceive to be the advantages of United Way membership? Has the United Way in your community failed to achieve its goals in contributions in recent years? If so, what are the causes?
2. What are the most prominent foundations in your community? What has been the source of revenues for their activities? How would you rank them with respect to the volume of their gifts versus assets? Which foundations have been most active with respect to social welfare? Have their gifts increased or decreased in recent years?
3. If there are unmet needs in your community, how would you create a nonprofit agency to meet them? Whom would you recruit for your board of directors? Where would you solicit resources, both cash and in-kind? Whom would you recruit for staff? What would you name your agency? Would your agency focus on providing services or on advancing social change?
4. In response to diminishing resources, many nonprofit social agencies have resorted to entrepreneurial strategies to raise money. What innovative projects have agencies deployed in your community? What entrepreneurial strategies might be successful for nonprofit agencies in your community?
5. Thinking as a social entrepreneur, how would you apply a business ethic to address a social problem in your community? Are there local nonprofits that have undertaken commercial ventures to increase their revenues? How successful have they been?

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Privatization and Human Service Corporations



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This chapter reviews the privatization of social welfare and its correlates: welfare capitalism, corporate social responsibility, the emergence of social markets, and corporate influence in social policy. As a function of public dissatisfaction with governmental social programs, increasing reliance on the private sector to finance and deliver social services has emerged as an important theme in American social welfare. Insofar as the political economy of the United States is democratic-capitalism, the for-profit sector is unavoidable in public affairs, including social welfare. Thus, **privatization**, addresses the problem of the proper relationship between the public and private spheres of the national culture. In this case, the concept of privatization has come to involve “the idea that private is invariably more efficient than public, that government ought to stay out of as many realms as possible, and that government should contract out tasks to private firms or give people vouchers rather than provide them services directly.”¹

Logically, proponents of privatization have two options: the nonprofit, voluntary sector (Chapter 6) and the for-profit, **corporate sector**. Both sectors are well established in U.S. social welfare. Since the advent of former president Lyndon Johnson’s Great Society programs, government has been contracting out services through both nonprofit and for-profit providers. The for-profit corporate sector capitalized on the contracting-out provisions of the Medicare and Medicaid programs. At the same time, through the purchase-of-service concept introduced in Title XX Social Services Block Grant, nonprofit agencies became contractors, providing a range of social services on behalf of public welfare departments.

Advocates of *voluntarization*, or reliance on the voluntary sector to assume more of the responsibility for welfare, point to the voluntary sector’s historical contribution to the national culture, the rootedness of its agencies in the community, and the altruistic motives behind its programs. Advocates of voluntarization received a boost when the second Bush presidency advocated “faith-based” initiatives in social welfare. Proponents of *corporatization*, or dependence on the corporate sector to provide welfare, argue that corporations offer more cost-effective administration, are more responsive to consumer demand, and pay taxes, a strategy endorsed by the Obama administration. Corporatists scored major victories in the 2003 Medicare reforms that introduced the prescription drug benefit that designated pharmaceutical companies

as the conduit for medicines for the elderly as well as the 2010 Affordable Care Act, which mandated that citizens purchase commercial health insurance. Whether voluntarists or corporatists prevail in the privatization debate rests largely on the ability of each party to manipulate the social policy process in their favor. Whatever the outcome, this process is certain to be lengthy and complex, as one might expect with the remaking of an institutional structure that has become as essential as social welfare is in the United States.

Historically, much social welfare has been provided by the private nonprofit sector, and many nonprofits contract with government to provide social services. Early in the industrial era, corporations subsidized **scientific philanthropy**, providing essential resources for social welfare organizations that preceded the establishment of the governmental welfare state. Toward the end of World War II, **welfare capitalism** assured health and pension benefits to workers through private employers, subsidized by tax expenditures. More recently, proprietary firms have become well established in several **human services markets**: health care, child care, and corrections as well as child welfare and education. As corporations flex their economic muscles, they use trade associations to hire lobbyists who leverage social policy toward corporate ends, which may be at odds with public welfare, broadly understood.

The emergence of for-profit provision of social welfare has been controversial, pitting leftists who insist that government be the basis of service provision against conservatives who view markets as optimal for distributing goods and services. Rather than either-or, a pragmatic middle ground is to recognize the complementary role played by government in relation to markets. Even liberal policy analysts entertained ways in which the private sector could complement governmental welfare initiatives.² Child welfare is illustrative: Federal and state child welfare agencies have contracted with nonprofit agencies for decades; however, with the privatization of child welfare in many states, companies such as Camelot Community Care have emerged as providers as well.

Liberal proponents of social programs are often willing to concede a viable role to the private sector—even an innovative role—but insist that government must be the primary instrument to institutionalize social justice. Conservatives see the proper balance as one in which the private sector is

the primary source of protection against social and economic calamity, and the government activity should be held in reserve as the secondary source. According to conservative doctrine, government can deploy the “safety net” of social programs, but these should provide benefits only as a last resort. Since the 1980s, however, conservatives have called for downsizing government, in the process shifting service responsibility to the private sector, providing opportunities to the for-profit sector. Liberals have watched with dismay as public welfare has been cut, while corporate profits and CEO salaries have risen.

For liberals, **collective bargaining**, contract agreements between workers and employers, is a strategic response to privatization. While non-professional workers have joined unions, health and human service professionals have been reluctant to engage in collective bargaining, primarily because they fear that doing so would taint their independence. Yet, continuing privatization means that unions may be the only aggregate defense for professionals who have become employees of profit-making health and human service corporations. In other industrialized nations, professionals belong to unions in order to increase their political influence. American social workers have not been enthusiastic about joining unions; however, they have been eager to establish **private practice**, the for-profit provision of services, paradoxical in light of social work’s criticism of privatization.

Privatization Issues

Many health and human service professionals have trepidations about privatization, citing risks associated with the for-profit sector for certain activities. The irony is that the private, *nonprofit* sector, which has been an institution in every American community, is often excluded in discussions about privatization. Regardless, privatization is a profound issue: To the extent that social workers work in the private sector, which often serves people with less severe problems, there are fewer human service professionals in the public sector to work with people presenting more serious disorders. As a result, what limited influence social work possesses is invested in promoting its self-interest at the expense of the poor—a point underscored by the late Harry Specht and Mark Courtney in *Unfaithful Angels*:

... there has been an increasing tendency of the [social work] profession to use its political power to support licensing of clinical social workers and third-party payments for social workers who are so licensed, to the relative neglect of efforts to improve the lot of social workers employed in the public social services and their clients.³

For many, human service professionals private practice, in particular, represents a retreat from a service ethic that transcends self-interest: the public’s welfare.

Opponents to privatization voice several criticisms. First, if a corporation’s profits are linked

Spotlight 7.1

TECHNIQUES FOR THE PRIVATIZATION OF SERVICE DELIVERY

The President’s Commission on Privatization identifies three techniques for the privatization of service delivery:

1. Selling government assets
2. Contracting with private firms to provide goods and services previously offered by the government
3. Using vouchers, whereby the government would distribute coupons authorizing private providers to receive reimbursement from the government for the goods and services they have provided



Watch this video explaining privatization of service delivery.

www.youtube.com/watch?v=-1EF0E2WdGA

to reducing services, the incentive to deny aid will be significant. According to Henry Freedman of the Center for Social Welfare Policy and Law, “No company can be expected to protect the interests of the needy at the expense of its bottom line, least of all a publicly traded corporation with a fiduciary duty to maximize shareholder profits.”⁴ Clearly, corporations will have strong incentives to ration services to improve their profits and performance.⁵ Second, corporations are apt to reduce personnel costs. They may cut staff and replace full-time professionals and managers with contingent, lower-wage workers. Accordingly, corporations may try to dislodge public sector unions where they exist and prohibit them where they do not, as a way to control costs. Third, preferential selection of clientele and market differentiation frequently conspire to create dual levels of care where higher quality of service for the well-off contrasts with inferior services for poorer citizens. Fourth, privatization encourages corporations to save money by using technology such as automated service centers to replace staff, which would have enormous implications for people who are not technologically sophisticated or who would benefit from face-to-face services. Fifth, government would find itself in a quandary if commercial providers bailed out of contracts. At worst, corporations could declare bankruptcy, effectively voiding their contractual obligations, leaving government to pick-up the pieces. Finally, corporations with a major presence in a given market are in a strong position to shape social policy, defining such factors as eligibility for program benefits and allocations. All of these corporate strategies would have a major impact on the quality of services delivered.

In the absence of definitive results showing the advantages of one sector over another, the privatization debate has become heated.

Commercialization

For welfare professionals, the idea of subjecting human need to the economic marketplace is often problematic. In one of the earliest treatments of the matter, Richard Titmuss’ *The Gift Relationship* explored the differences in the ways nations manage their blood banks. Unlike the United Kingdom, blood in the United States is “treated in laws as an article of commerce”; that is, rules of the market affect the supply and quality of blood. Titmuss observed the growth of blood and plasma

businesses with alarm, because these businesses bought blood from a population that was often characterized by poverty and poor health. Quite apart from the health hazard posed by a blood supply derived from such a population—a hazard highlighted by AIDS—Titmuss was concerned that the profit motive would disrupt the voluntary impulses of community life. “There is growing disquiet in the United States,” he observed, that “expanding blood programs are driving out the voluntary system.”⁶

With the passage of Medicare and Medicaid in 1965, the federal government was placed in the position of reimbursing and regulating a private industry that evolved rapidly. In both programs, scandals involving fraudulent billing of billions of dollars and neglecting of patients, sometimes with fatal consequences, were exposed.⁷ Once for-profit providers gain a significant share of a market, even nonprofit providers are likely to adopt commercial practices. In 2006, Congress convened hearings about some nonprofit hospitals, which, instead of providing charity care congruent with their tax-exempt status, hired collection agencies to harass former patients into paying for their care.

A poignant problem with commercialization relates to those tax-exempt providers who transition from nonprofit to for-profit status. Before the commercialization of health care, the largest health insurer was Blue Cross–Blue Shield; however, many state Blue Cross–Blue Shield plans have converted to commercial status. The transition from nonprofit to for-profit status requires the approval of state regulators, often elected officials who are concerned that low-income citizens might be left uninsured. Consequently, states often require such companies to establish health care foundations as a condition for becoming corporate. When Wellpoint and Anthem merged in California, for example, the companies offered \$265 million, much of it to assure the continuation of health care to underserved communities.⁸ Such foundations become an important resource for those who wish to reduce inequities in health care.

Preferential Selection

The application of market principles to client service introduces strong incentives for providers to differentiate clients according to their effect on organizational performance. Such selection can be at variance with professional standards, which emphasize the client’s need for service over

organizational considerations. But the marketplace tends to penalize providers that are imprudent about client selection, at the same time rewarding providers that are more discriminating. The practice of choosing clients according to criteria of organizational performance—as opposed to client need—is known as **preferential selection**. Under marketplace conditions, providers that do not practice preferential selection are bound to serve a disproportionate number of clients with serious problems and with less ability to pay the cost of care, thereby running deficits. By contrast, providers that select clients who have less serious problems and who can cover the cost of care often claim surpluses. For example, an analysis of psychiatric patients admitted to a public and a private hospital found that the latter selected patients of higher social status. The researchers concluded, “patients in the marginal/uncredentialed social class were comparatively more likely to be admitted to state mental hospitals than to private hospitals.”⁹

Critics of privatization complain that creaming the client population through preferential selection is unethical and should be prohibited. Simply ruling out the practice is, however, easier said than done. Accusations of preferential selection are not new; private nonprofit agencies were accused of denying services to welfare recipients long before proprietary firms became established.¹⁰ In Denver, where public hospitals have sustained heavy deficits by caring for a disproportionate burden of the medically indigent, the problem has become critical. Jane Collins, director of clinical social work for the Denver Department of Health and Hospitals, described public hospitals in the city as having become “social dumps.”¹¹

Preferential selection on the part of a large number of providers is likely to be adopted by others who wish to remain competitive. In analyzing the practice in health care, a team of researchers from Harvard University and the American Medical Association noted that “in the same way that competition from for-profit providers leads to reduction in access, the more competitive the market for hospital services generally, the more likely are all hospitals in that market to discourage admissions of Medicaid and uninsured patients.”¹² In other words, even nonprofit providers—who are exempt from taxes because they contribute to the community’s welfare—are compelled to adopt the discriminatory practices of for-profit providers in a competitive market, unless the nonprofits are

willing to underwrite the losses that more costly clients represent to for-profit providers.

Dual Levels of Care

Preferential selection by service providers is frequently complemented by specialized care for affluent clients, resulting in dual levels of care. For example, life care—the service offered by the continuing-care retirement community—has emerged as an attractive alternative to traditional nursing home care that has been standardized to reduce costs. Under life-care plans, residents can purchase cottages or apartments in self-contained communities, at prices comparable to that of a new home, which provide a range of human services. In many respects, the life-care community provides more affluent residents an opportunity to purchase a higher level of long-term care. Many continuing-care facilities boast such amenities as wall-to-wall carpeting, maid service, and designer landscaping. The prospect of extensive proprietary involvement in life-care troubles some analysts. Lloyd Lewis, director of a nonprofit life-care community, feared that “well-funded proprietary interests” would “drain off the more financially able segment of our older population, widening the gap between the ‘haves’ and the ‘have nots.’”¹³

To a significant extent, this has already occurred. The late Robert Ball, former commissioner of the Social Security Administration, noted that even life-care communities operating under nonprofit auspices are often beyond the means of “the poor, the near poor, or even the low-income elderly.”¹⁴ As human service corporations divert capital to care for those who represent profit margins, economic and political support for the care of those less fortunate diminishes. Left unchecked, this pattern is likely to divide long-term care into two clearly demarcated systems, with the affluent enjoying the generous care of completely—some would say excessively—provisioned life-care communities and the elderly poor dependent on the squalid institutions willing to accept government payment for their care.

Cost-Effectiveness

Proponents of privatization frequently cite the discipline imposed on organizational performance by a competitive environment as a rationale for market reforms in social welfare. A competitive



A competitive environment provides strong incentive for organizations to adopt cost-effective practices that reduce waste.

Source: Gromovataya/Shutterstock

environment provides strong incentives for organizations to adopt cost-effective practices that reduce waste. This claim has led to a handful of studies of for-profit versus nonprofit service providers. In a comprehensive review of the issue, the Institute of Medicine of the National Academy of Sciences concluded that there was “no evidence to support the common belief that investor-owned organizations are less costly or more efficient than are nonprofit organizations.”¹⁵ Later, Robert Kuttner summarized analyses conducted by an association of nonprofit hospitals and found that “investor-owned hospitals . . . were 13.7 percent more expensive on a charge basis than nonprofit and public hospitals.” Again, not only were for-profit hospitals more expensive but they also provided less care to the poor, admitting only half as many Medicaid patients as nonprofit hospitals. Another analysis of proprietary hospitals by the same association of nonprofit hospitals found that investor-owned hospitals were 30 percent more expensive than nonprofit hospitals.¹⁶ A study of 14,423 nursing homes found that even though public facilities had a significantly higher proportion of Medicaid patients, they, along with nonprofit providers, performed better than their for-profit counterparts.¹⁷

Oligopolization

The privatization of human services invites **oligopolization**—the control of a market by few providers as organizations seek to reduce competition by buying their competitors. Within the health

care sector, three waves of acquisition can be identified: acquisitions affecting long-term care, hospital management, and health maintenance organizations (HMOs). As firms gain control of major shares of markets, they are in a strong position to leverage influence through trade associations to shape social policy. Oligopolization of human services presents a daunting specter in that a small number of wealthy and powerful trade associations are in a strong position to shape social policy to conform to their interests. Within health care, this development led the late Arnold Relman, former editor of *The New England Journal of Medicine*, to voice alarm at the growing influence of the “new medical-industrial complex” in defining health policy in the United States.¹⁸

One provision of the Affordable Care Act of 2010 called for the creation of **accountable care organizations**, regional networks of providers that would provide high quality of care at lower costs by integrating services. Within a year of the passage of health reform, “there is a growing frenzy of mergers involving hospitals, clinics and doctor groups eager to share costs and savings, and cash in on the incentives,” noted a journalist. In Baltimore, for example, Johns Hopkins Medicine acquired Sibley Memorial Hospital in Washington, DC, and Suburban Hospital in Bethesda, Maryland, an exurb of Washington.¹⁹

The Challenge of Privatization

For many health and human service professionals, privatization is contrary to social welfare. Advocates of government’s responsibility for ensuring social and economic equality contend that privatization is nothing less than a retreat from a century of hard-won gains in social programs. This case is argued cogently by Pulitzer Prize-winning sociologist Paul Starr:

A large-scale shift of public services to private providers would contribute to further isolating the least advantaged, since private firms have strong incentives to skim off the best clients and most profitable services. The result would often be a residual, poorer public sector providing services of last resort. Such institutions would be even less attractive as places to work than they are today. And their worsening difficulties would no doubt be cited as confirmation of the

irremediable incompetence of public managers and inferiority of public services. Public institutions already suffer from this vicious circle; most forms of privatization would intensify it.²⁰

Most profoundly, privatization reinforces a tendency in market economies to evolve dual structures of benefits, services, and opportunities: adequate and varied services for the affluent, substandard and uniform services for the poor. As Robert Kuttner has pointed out, “in a purely for-profit enterprise or system, there is no place for uncompensated care, unprofitable admissions, research, education, or public health activities—all chronic money losers from a strictly business viewpoint.”²¹ For many human service advocates, the purpose of social welfare is to correct for the inherent tendency of markets to direct resources toward the affluent and away from the poor.

Yet, increased reliance on the private sector at a time when public social programs are under assault is a reality that must be faced by those concerned about social welfare. Insofar as corporations have assumed ever-larger responsibilities for social program activities, such as Medicaid, State Children’s Health Insurance Program (SCHIP), child welfare, and mental health, welfare professionals have little choice but to concede privatization as a basis for welfare provision. For example, Molina and Centene have contracted with several states to manage Medicaid and SCHIP programs.

For-profit firms have also reported success with long-term public welfare recipients. Celebrating its 25th anniversary, America Works places TANF recipients, ex-felons, and the long-term unemployed in jobs that pay on average \$15,000 to \$18,000 per year. America Works has won awards for its entrepreneurial model: Its placement fee of \$5,000 is paid only after a client has been a successful employee for four months. Since its inception, America Works claims to have saved taxpayers millions of dollars in welfare benefits.²² Each year the company finds jobs for more than 700 of the state’s hard-core unemployed, 68 percent of whom are (as a result) permanently weaned from the welfare rolls. All told, America Works is saving taxpayers approximately \$4.5 million annually and providing many of the state’s hard-core unemployed with meaningful work.²³

Upon closer inspection, there are strong arguments in favor of privatization as a strategy for promoting social welfare. Through commercial loans

and issuance of stock, for-profit organizations have faster access to capital than does the governmental sector (which requires a lengthy public expenditure authorization process) or the voluntary sector (which relies on arduous fundraising campaigns) for purposes of program expansion. The private sector has also been the source of important program innovation²⁴; thus, welfare administrators have missed opportunities for promoting social welfare by ignoring privatization. How health and human service professionals choose to respond to the challenge of privatization—whether reactively or innovatively—will be critical for the future of U.S. social welfare.

Unions and the Private Sector

Unions of health and welfare professionals are one response to privatization. Since the Great Depression, social welfare professionals have organized collectively in order to obtain better wages and benefits, to enhance working conditions, and to improve services to clients. Social workers in the public sector often hold memberships in unions, which are private organizations—most often in the American Federation of State, County, and Municipal Employees (AFSCME), with 1.6 million members in 2012, or in the Service Employees International Union (SEIU), with 2.1 million members in 2012. Altogether, about one in four social workers belong to a collective bargaining unit.²⁵ Because of the dispersion of social work activities, however, social workers have been less successful than nurses or teachers in using unions to achieve their ends.

Collective bargaining is the foundation of the union process, face-to-face negotiation between unionized employees and management for the purpose of arriving at a union contract. Such bargaining is supposed to be done in good faith, and the legal rights of workers are protected by the National Labor Relations Act. If these rights are attenuated, workers can petition the National Labor Relations Board to address grievances. The ultimate power of unions is to exercise the right to strike when the bargaining process breaks down. Theoretically, both parties have an interest in a successful collective bargaining process, because strikes hurt both union members and their employers. Collective bargaining can also address professional issues such as caseload size, educational benefits like

tuition reimbursement, conference release time and reimbursements, payment for professional dues and subscriptions, and flexible work hours. One union leader noted, "To the professional—the teacher or caseworker—things like class size and caseload size become as important as the number of hours in a shift is to the blue collar worker."²⁶

A significant question before human service professionals has been the extent to which union objectives are consistent with professional values. Despite the constructive influence that unions could potentially exert in response to workplace problems in social welfare, social workers have approached unions with great apprehension. Opponents to collective bargaining contend that (1) unions cost employees money; (2) strike losses are never retrieved; (3) even when they are not purposely kept uninformed by union leadership, members have little voice in union affairs; (4) bureaucratic union hierarchies control the economic destiny of members; (5) union corruption is rampant; (6) union opposition to management attempts to increase productivity arrests organizational growth; (7) union featherbedding results in unneeded employees and unnecessary payroll expenses; (8) union activities foster conflict rather than collaboration; and (9) unions fail to extrapolate the effects of wage increases on future employment, inflation, and taxes.

A fundamental concern among social welfare professionals revolves around the ultimate tactic that unions can bring to employer-employee relations: job actions, particularly strikes. For social workers pledged to make client welfare a priority, the prospect of denying services as a result of job actions makes union membership and professional commitment contradictory. In covering a strike by Local 1199 of the Retail Drug Employees Union, which included the social work staffs of more than 50 hospitals and nursing homes, Dena Fisher wrote:

Standards for professional practice conflict with the [NASW] Code of Ethics with regard to behavior during a labor strike when the prescribed behavior includes withholding service, failing to terminate clients properly, and picketing activity directed toward consumers. The problem is that participation in a strike is a nonprofessional activity. Standards of professional behavior conflict with union membership requirements.²⁷

Yet, proponents of union membership cite ways in which collective bargaining can complement

professional objectives. In an attempt to encourage a better relationship with professional social workers, Jerry Wurf, former AFSCME president, explained that to improve public services and programs, the commitment of AFSCME is important to show how labor organizations are tied to social work. AFSCME grew over the last decade as a result of more social workers in the field.²⁸ As unions have lost membership, dropping to its lowest level in 70 years at 11.9 percent of the labor force, they have become more receptive to non-confrontational methods to advance their objectives.²⁹ Thus, Andy Stern, a caseworker who rose to the Presidency of SEIU, found job actions against employers inferior to alternative methods to advance his union's agenda. After losing a strike with the Beverly nursing home corporation, Stern convinced the company to sign a contract improving staff working conditions if SEIU convinced government to increase reimbursement to patients cared for by Beverly. In New Jersey, SEIU negotiated novel contracts with employers that were not binding until a majority of employers had also agreed, spreading the contract costs across a wide spectrum of companies.³⁰

The few studies that examined the issue found little incongruity between the loyalties of social workers belonging to unions. Leslie Alexander and his colleagues studied 84 union members with MSW degrees and found that "they view their work as solidly professional and, for the most part, do not see unionism and professionalism as incompatible."³¹ Ernie Lightman reported similar findings when studying 121 randomly chosen professional social workers in Toronto, Canada. According to Lightman, "the vast majority saw no incompatibility; indeed, many felt unionization may facilitate service goals, offsetting workplace bureaucracy."³² Reporting on child welfare agencies in Pennsylvania and Illinois, Gary Shaffer found that "workers did not find unionism incompatible with their educational or professional goals."³³

Such complementarity notwithstanding, the concept of social work "exceptionalism" pervades the debate about professionalism and unions. The exceptionalism premise implies that tasks performed by social workers are more important than those performed by many other workers, especially nonprofessionals, and that normal labor relations principles are therefore not applicable. Proponents of social work exceptionalism must address two matters: First, how is it that other

semiprofessionals, such as teachers and nurses, have reconciled their professional priorities with union activities and become more powerful in the contexts of their work as a result? Are social service activities to be considered more essential than education or health care? Second, does the idea of social work exceptionalism contribute to the powerlessness of social workers? If social work places such value on the empowerment of clients, why should social workers not also be so empowered?

Social Contributions of Business

The business community influences social welfare in several important ways. Benefit packages for employees, which are usually available to dependents, provide important health and welfare benefits to a large segment of the working population. Historically, this form of “welfare capitalism” has been an important complement to public social programs. “For much of the twentieth century, indeed, the development of U.S. social policy has followed an identifiable second track of intervention, one to which scholars of the welfare state, orthodox or revisionist, have paid only limited attention,” noted Jacob Hacker. “The legislative milestones along this track have not been large and highly prominent social programs, but public policies of diverse form—tax breaks, regulations, credit subsidies, and government insurance—designed to encourage and shape private responses to public social problems.”³⁴ The implications of welfare capitalism are momentous. Once private sector activities are calculated as part of the national welfare effort, provision of welfare services in the United States is similar in scale to that of European nations. Politically, any welfare state with competing public and private sectors can develop separate solutions to citizens’ needs, evident in the liberal defense of Social Security contemporaneous with conservative preference for private alternatives, such as 401(k) plans.³⁵

At this point, many welfare theorists began to reexamine the role of the corporate sector in U.S. social welfare. The concept of the “mixed welfare economy” combines the corporate proprietary sector with the governmental and voluntary sectors as primary actors in social welfare.³⁶ And the issue of privatization provoked a vigorous argument about the proper balance between the public and private (including corporate) welfare sectors.³⁷ Although

Neil Gilbert’s *Capitalism and the Welfare State* provided a timely review of the issues posed by “welfare capitalism,”³⁸ empirical investigations of for-profit human service corporations are in their infancy.³⁹

Business has also reevaluated its position in relation to social welfare as a result of accusations that it has been insensitive to the needs of minorities, the poor, women, and consumers. These problems contributed to a public relations crisis, as a leading business administration text noted:

The corporation is being attacked and criticized on various fronts by a great number of political and citizens’ organizations. Many young people accuse the corporation of failing to seek solutions to our varied social problems. Minority groups and women contend that many corporations have been guilty of discrimination in hiring and in pay scales.⁴⁰

Melvin Anshen, a professor of public policy and business responsibility bemoaned the fact that “profit-oriented private decisions are now often seen as antisocial.”⁴¹

In order to improve their public image, many businesses established policies on **corporate social responsibility**. Corporations that were reluctant to take seriously the social implications of their operations ran the risk of inviting the surveillance of public interest groups. As an example, in 2010, Human Rights Campaign published its report, *Corporate Equality: Rating American Workplaces on Lesbian, Gay, Bisexual and Transgender Equality*, calculating that 338 businesses achieved a rating of 11 percent.⁴² Although public relations facades frequently gloss over businesses’ abuses, specific corporate social responsibility policies have advanced social welfare. Under the title, Public Interest Director, Leon Sullivan assumed a position on the General Motors board of directors, from which he presented principles governing ethical practices for U.S. corporations doing business in South Africa.⁴³ Ultimately, U.S. firms’ adherence to the “Sullivan principles” contributed to the fall of apartheid in South Africa. More recently, financial consultants pioneered the concept of socially responsible investing. The practice of excluding certain industries from mutual fund portfolios because their activities are contrary to those of investors gained considerable ground during the 1980s. An illustration of socially responsible investing is the Domini Social Index (DSI), an investment strategy that screens out

businesses in five areas: military contracting, alcohol and tobacco, gambling, nuclear power, and, prior to the end of apartheid, South Africa.⁴⁴

Corporate practices have also been applied directly to social problems, as evident in initiatives undertaken by the Enterprise Foundation and the Ford Foundation. In 1981, the late developer James Rouse established the Enterprise Foundation, a foundation that supported charitable projects. In 2009, Enterprise announced a \$4 billion investment in retrofitting multi-family housing units to make them more energy efficient.⁴⁵ Recognizing the tendency of community institutions in poor areas to become dependent on government or philanthropy for continuing operations, the Ford Foundation sought contributions from corporations for a program to apply business principles to social problems. By 2010, the Local Initiatives Support Corporation (LISC) had invested \$1.1 billion through its sustainable communities initiative.⁴⁶ In the light of such ventures, some business leaders have become enthusiastic about the activist responses to social problems on the part of the corporation.

Concerned about public perceptions that corporations have worsened economic inequality while

making only token philanthropic contributions, CEOs have begun to make capitalism more socially responsible.



Watch this video to learn more about corporate social responsibility.

<https://www.youtube.com/watch?v=dvJSK4viVMs>

Corporate Influence on Social Welfare Policy

Corporate social responsibility notwithstanding, it would be naive to think that the corporate sector is above self-interest in its orientation toward social welfare. The conservative political economist Irving Kristol stated as much when he wrote that corporations do not donate based on noble goals or even because it is required but only in their own interests. Philanthropy serves a purpose for the corporation; it contributes to the corporation's larger goals.⁴⁷ And corporate influence in social welfare is not exerted simply through myriad corporations acting independently. Special interest organizations, such as the National Association of Manufacturers and the United States Chamber of Commerce, have routinely pressed for public policies that clearly reflect

the priorities of the business community. The influence that the business community brings to public policy is discussed in greater detail in Chapter 8. Since the 1970s, certain foundations have made sizable contributions to conservative policy institutes, constructing the ideological infrastructure that would challenge liberalism in social policy.

At the turn of the twentieth century, conservative foundations continued to support think tanks associated with the Right. By the end of the twentieth century, such contributions placed conservative policy institutes at the center of the social policy debate. Not only were conservative think tanks well-endowed with resources but they were also dispersed throughout the United States. As a result of these investments in the “marketplace of ideas,” the conservative imprint on social policy has become indelible, and many of the most prominent conservative think tanks—American Enterprise Institute for Public Policy Research (AEI), Heritage, and CATO—have become household names. In response to conservative think tanks, liberal-oriented donors and businesses established the Center for American Progress to press progressive changes in social policy. Think tanks address the complaint voiced by Lawrence Fouraker and Graham Allison of Harvard's Graduate School of Business Administration: “Public policy suffers not simply from a lack of business confidence on issues of major national import, but from a lack of sophisticated and balanced contribution by both business and government in the process of policy development.”⁴⁸

Corporate influence in social welfare is dynamic, involving a range of organizations, evident in crafting health policy. The Medicare Modernization Act of 2003 not only diverted \$400 billion over 10 years to pharmaceutical companies but also prohibited the federal government from negotiating drug prices as has been the practice of commercial health insurers as well as the Veterans Administration. Similarly, the Affordable Care Act of 2010 established a health insurance exchange and mandated that citizens purchase health insurance, a \$1.2 trillion windfall for the health insurance industry over 10 years. In anticipation of the 2010 health care reform, an unprecedented \$200 million was spent on attempts to influence members of Congress who found themselves confronted with 4,500 lobbyists. Prominent among them were the representatives from for-profit hospitals, the pharmaceutical industry, and the health insurance industry.⁴⁹

The Future of Corporate Involvement in Social Welfare

The corporate sector is apt to exert greater influence on social policy as the twenty-first century unfolds. The emergence of health and human service companies demonstrates that activities once reserved for government can be converted to markets. As health care has been followed by child care, corrections, and education, investors consider new markets to exploit; given prohibitions against restraint of trade, investors are free to explore profit-making opportunities anywhere, including social welfare, so long as they act lawfully. Thus, alternative financial services, such as auto title and payday lending, provide credit to the welfare- and working-poor, who are not served by banks or credit unions.⁵⁰

The corporate sector also influences social policy through industrial policy—periodic meetings of representatives of business, labor, and government to define mutually agreed objectives. Indeed, the European welfare states are being transformed by a **social investment strategy** through which government negotiates with workers and employers around social objectives. Americans' skepticism about a strong federal government makes such a strategy unlikely in the United States. To the contrary, international trade agreements, such as the North American Free Trade Agreement, the Transatlantic Trade and Investment Partnership, and the Trans-Pacific Partnership, have been opposed by many liberals and labor leaders because they subvert wages of American workers. Such concerns notwithstanding, the globalization of industry is being followed by the globalization of services. The fluidity of capital makes it increasingly difficult for sovereign nations to honor their social program commitments since these are dependent on taxes; once taxation becomes too burdensome, companies can decamp to countries offering a more favorable business climate.

The application of business principles to social welfare has been an important source of innovation, evident in **social entrepreneurship**.⁵¹ The Grameen Bank has been an iconic example, a commercial bank that has lifted millions of peasant women in developing nations out of poverty. The micro-finance model, pioneered by the Grameen Bank, has been replicated in Africa and South America; its founder, Muhammad Yunus, was awarded the 2006 Nobel Peace Prize. Subsequently, Yunus

proposed “social business” as a means to capitalize businesses that serve social purposes.⁵² Because schools of social work have been wedded ideologically to governmental and nonprofit sectors, they have limited experience with social entrepreneurship, but some business schools offer courses in the subject.

Finally, business affects social welfare through the polity, lobbying, and election campaigns. The 2010 *Citizens United* Supreme Court decision removed limits on contributions to large Political Action Committees, “super PACs,” so long as they are not coordinated with candidate campaigns. Subsequently, the Supreme Court’s *McCutcheon* ruling permitted contributions to any number of candidates. Together, these decisions unloosed a tsunami of cash during the 2014 midterm election, which was soon eclipsed by the 2016 presidential election. Because corporations and wealthy individuals have made larger contributions than organized labor, these decisions have benefited conservatives more than liberals. Congressional gridlock along with the inability of federal regulators, including the Federal Election Commission and the Securities and Exchange Commission, to limit corporate political activity means it will likely increase in future elections.

Human Service Corporations

Continued demand for human services in the postindustrial period has drawn the corporate sector directly into social welfare in the United States. Corporate exploration of the growing human services market has proceeded apace. In contrast, shackled by debt and denied revenue because of tax cuts, government social programs struggle to keep up with demand for services. Dependent on government support and on contributions of middle-income Americans who have experienced a continual erosion of their economic position, the voluntary sector is unlikely to meet future service demands. Relatively unfettered by government regulation and with easy access to capital from commercial sources, the corporate sector has made dramatic inroads into service areas previously reserved for governmental and voluntary sector organizations.

Significantly, the incentives for corporate entry into human services were initially provided through government social programs, especially with passage of Medicare and Medicaid in 1965. As a

percentage of **gross domestic product (GDP)** government social expenditures more than doubled, from 8.8 percent in 1970 to 20.5 percent in 1991. Health care allocations figured prominently in the expansion of government expenditures over this period. In 1970, government spent \$24.9 billion on health care; by 1991 that figure had grown to \$317.0 billion,⁵³ and \$2.6 *trillion* in 2010.⁵⁴ The potential profits for corporations entering the rapidly expanding health and human services market were unmistakable.

Concomitantly, public policy decisions encouraged proprietary firms to provide health and human services. By using a market approach to ensure the availability of health care for the medically indigent and the elderly, Medicaid and Medicare avoided the costs of constructing a system of public sector facilities and, in so doing, contributed to the restructuring of health care in the United States. What was essentially a haphazard collection of mom-and-pop nursing homes and small private hospitals was transformed, in a short period, into a system of corporate franchises, complete with stocks traded on Wall Street. And almost a decade later, incentives offered through Medicaid and Medicare to encourage the corporate sector to become involved in hospital care were replicated in the health maintenance industry. The Health Maintenance Organization Act of 1973 stimulated a sluggish health maintenance industry that has since grown at an explosive rate. As Pulitzer Prize-winning journalists Donald Barlett and James Steele noted, “Wall Street reasoned that a portion of America’s hospitals could be assembled into national chains, much

like department stores and auto-parts distributors, and investors could make a fortune.”⁵⁵

Initially dependent on government welfare programs, the corporate sector developed a life of its own. Exploitation of the nursing home, hospital management, and health maintenance markets has led to corporate interest in other markets. By the 1980s, human service corporations had established prominence in child care, ambulatory health care, substance abuse, psychiatric care, and home health care, as well as in life care and continuing care. By the 1990s, managed care firms were restructuring health and mental health services so as to batten corporate profits; by the end of the twentieth century, they had established beachheads in welfare and corrections, as well. Increasingly, proprietary firms were able to obtain funds for facilities through commercial loans or sales of stock, and to meet ongoing costs by charging fees to individuals, companies, and nongovernmental third parties. Insofar as resources for human service corporations are not financed by the state, firms are free to function relatively independent of government intervention.

How big is the corporate sector in U.S. social welfare? Early in the twenty-first century, more than 30 of the largest firms reported annual revenues that were far greater than *all* contributions to the United Ways of America.⁵⁶ These corporations employed thousands of workers, some more than the number of state and local workers for public welfare programs in any state in the union. Some salient statistics on the larger human service corporations are listed in Table 7.1.

TABLE 7.1 Leading Human Service Firms⁵⁷

Firm	Market	Revenues, 2013/14	Employees
UnitedHealthcare	HMO	\$132.0 billion	156,000
HCA	Hospital Management	40.0 billion	162,000
Humana	Hospital Management	41.3 billion	52,000
Centene	State Medicaid plans	15.7 billion	8,800
Magellan	Behavioral health	3.7 billion	5,949
MAXIMUS	Service consulting	1.3 billion	13,000
CCA	Corrections	1.6 billion	16,750
Cash America	Payday lending	1.8 billion	7,637
University of Phoenix	Higher education	1.2 billion	39,000
ITT	Vocational training	1.3 billion	4,900

Consolidation and Growth in New Human Service Markets

Human service corporations have become prominent, if not dominant, in several areas of social welfare: nursing homes, hospital management, HMOs, child care, and home care. The descriptions of the following subsectors are somewhat arbitrary because many companies diversify their operations, functioning across several subsectors. In health care, for example, a hospital management firm will operate nursing homes as well as an HMO. Over time, these holdings can change significantly, reflecting the dynamic nature of the evolving health and human services market. Human service markets have also become global; Sunrise Senior Living generated \$1.4 billion for elder care services in Britain, Canada, and the United States; the GEO Group earned \$1.6 billion providing correctional services in Australia, Canada, South Africa, and the United Kingdom as well as the United States.

Nursing Homes

Among corporate initiatives in social welfare, expansion into the nursing home industry is unparalleled. Between 1965 and 1978, expenditures for nursing home care increased 16.9 percent *annually*.⁵⁸ By the early 1980s, nursing homes had become a \$25-billion-a-year industry, and the number of nursing home beds exceeded those in acute care facilities for the first time.⁵⁹ At that time, 70 percent of nursing homes were under proprietary management. Market conditions such as these led a writer in *Forbes* magazine to observe, “This is a guaranteed opportunity for someone. How the nursing home industry can exploit it is the real question.”⁶⁰ Beverly Enterprises, a leader in the nursing home industry, encountered labor problems and regulatory difficulties, which resulted in the firm’s acquisition by Fillmore Strategic Investors in 2005 for \$1.6 billion. The nursing care industry would be shaped largely by Manor Care, which owned 500 nursing homes and had been sold to the Carlyle Group for \$6.3 billion.⁶¹ By 2011, Tenet Healthcare Corporation had rebounded from its legal difficulties, posting revenues of \$8.8 billion, but had divested most of its nursing homes.

Hospital Management

The growth of the nursing home industry has been matched by corporate involvement in hospital management. Between 1976 and 1982, the

number of investor-owned or investor-managed hospitals increased from 533 to 1,040, accruing gross revenues of approximately \$40 billion.⁶² The mid-1990s witnessed the largest mergers among health care corporations that had occurred up to that point. The merger of National Medical Enterprises and American Medical International to form Tenet Healthcare Corporation effectively approximated the market share controlled by Humana. This transaction paled, however, in comparison to the acquisition of HCA by Columbia. The Columbia/HCA merger created a \$20 billion behemoth that dwarfed Tenet and Humana. By the mid-1990s, the holdings of Columbia/HCA included 292 general hospitals, 28 psychiatric hospitals, and 125 outpatient and auxiliary facilities.⁶³ Being the largest health and human service corporation was not without its problems, however. An intensely competitive health market invited irregularities, the scale of which matched the size of the perpetrator.

In June 2003, HCA agreed to pay \$631 million in civil penalties and damages growing out of false claims to the government health care programs. That came on top of \$840 million in criminal fines, civil restitution, and penalties the company paid in 2000. That same year, the company agreed to pay \$250 million to settle other Medicare overbilling claims. In all, HCA has paid \$1.7 billion as a result of its fraudulent practices.⁶⁴

Despite such setbacks, HCA was well positioned with respect to federal programs that accounted for one-third of its revenue. The son of the company’s founder, Thomas Frist Jr., was none other than Bill Frist, who would become Senate Majority Leader. After flirting with a presidential campaign, Bill Frist joined his father and brother in buying HCA for \$33 billion, executing the largest purchase of a publicly held company in the nation’s history.⁶⁵ In 2010, HCA was publicly traded once again; a year later the company reported revenues of \$29.5 billion. By 2012, HCA was eclipsed by Humana, a company whose revenue had tripled from \$12.2 billion in 2004 to \$36.8 billion in 2011.

Health Maintenance Organizations

Pioneered by the nonprofit Kaiser Permanente in California, the concept of **health maintenance organizations (HMOs)** was slow to attract the interest of the corporate sector. However, from 1973 to 1981, the Health Maintenance Organization Act of 1973 authorized funds for the establishment of

these membership health plans in a large number of favorable markets. This funding, coupled with the growth in the nursing home and hospital management industries, reversed investor apathy. By 1983, 60 HMOs were operating on a proprietary basis.⁶⁶ By the end of the century, the HMO industry was a commercial enterprise, save Kaiser Permanente, which had to adopt for-profit strategies in order to remain competitive.

HMOs such as Kaiser Permanente offer prepaid medical services. Members pay a monthly or yearly fee for all health care, including hospitalization. Because costs to patients are fixed in advance, preventive medicine is stressed to avoid costly hospitalization. One criticism of HMOs is that patients can use only doctors and specialists who are associated with the organization.

At the beginning of the twenty-first century, the insurance giant CIGNA dominated the industry, controlling 46 HMOs in which 3 million people were subscribers.⁶⁷ CIGNA's reign was not to last, however. In November 2004, WellPoint and Anthem received permission from California regulators to consummate their merger announced a year earlier. As a condition to the merger, the

health providers agreed to pay \$265 million to upgrade health care in California. Combined, WellPoint and Anthem eclipsed CIGNA as the nation's largest health care insurer.⁶⁸ By 2012, Wellpoint and UnitedHealth Group International vied for dominance in corporate health care. Although Wellpoint reported \$60.7 billion in revenues compared to UnitedHealth's \$102 billion, it remained the nation's largest health insurer with 78 million members. But UnitedHealth overtook Wellpoint (renamed Anthem) as a result of its acquisition of PacifiCare for \$8.1 billion, representing 3.3 million enrollees,⁶⁹ to become the largest health care company in the world.

The largest managed care company providing behavioral health services, Magellan, emerged from bankruptcy in 2004, having consolidated its position, reporting revenues of \$2.8 billion in 2011. A primary competitor is Psychiatric Solutions, which was purchased by Universal Health in 2010. Operating a number of specialty hospitals including psychiatric clinics, Universal reported revenues of \$7.5 billion in 2011. Community Health Systems, which also operates psychiatric hospitals, reported revenues of \$13.6 billion in 2011.



Health maintenance organizations such as Kaiser Permanente offer prepaid medical services. Members pay a monthly or yearly fee for all health care, including hospitalization. Because costs to patients are fixed in advance, preventive medicine is stressed to avoid costly hospitalization. One criticism of HMOs is that patients can use only doctors and specialists who are associated with the organization.

Source: PhotoEdit Inc

Child Care

The day care market expanded as the number of mothers entered the labor market. Begun in 1969, KinderCare demonstrated prodigious growth, claiming approximately 825 “learning centers” representing \$128 million in revenue in 1983. During the 1990s, KinderCare dominated the child care industry with more than 1,100 centers, but new competitors emerged to serve a seemingly infinite need for organized child care. Within a few years, KinderCare’s command of the child care industry was challenged by several upstarts that showed explosive growth. Bright Horizons Family Solutions managed more than 500 workplace centers in 37 states, reported revenue of \$423 million, and had 16,000 employees; La Petite Academy boasted 650 centers, \$383 million in revenue, and 12,000 employees; Learning Care Group reported \$206 million in revenue and 7,500 workers. By 2007, KinderCare had been acquired by Knowledge Learning Corporation, which boasted annual revenues of \$1.6 billion, almost double what KinderCare had reported in 2004. Many of the smaller child care businesses were acquired by Learning Care Group, which operated 1,049 centers in the United States and abroad.

Home Health Care

Several companies in the home health care market replicated the success of corporations in the nursing home industry. Home Health Care of America, later renamed Caremark, began in 1979. A leader in the field, the company grew particularly quickly. By the mid-1990s, Caremark was reporting annual revenues of \$2.4 billion and claimed health care facilities in Canada, France, Germany, the United Kingdom, the Netherlands, and Japan.⁷⁰

Home health care expansion continued during the early 1990s, when home health companies adjusted their services to meet earlier hospital discharges resulting from implementation of Medicare’s prospective payment system: As patients went home sooner, the need for a range of specialized in-home health services grew. Quickly, home health companies began to offer a variety of these services to patients. In markets where conventional home health companies failed to offer such specialized care, new firms entered the market and expanded rapidly.

The diversification of elder care began to accelerate in anticipation of the retirement of the baby

boomers, providing consumers for companies such as Apria Healthcare Group and American Home Patient. Although Caremark had been acquired by the CVS drugstore company, the home health industry was comprised of smaller providers. In 2007, the industry consisted of Tender Loving Care Health Care Services reporting \$112.6 million in revenues; Assisted Living Concepts, \$229.4 million; Altria Senior Living Group, \$183.6 million; and National Home Health Care Corporation, \$110.8 million. Odyssey, a commercial hospice, reported revenues of \$404.9 million in 2007.

Corrections

Among the more ambitious of human service corporations is the Correction Corporation of America (CCA), founded in 1983 by Tom Beasley with the financial backing of Jack Massey, founder of the Hospital Corporation of America. CCA officials noted that many states were unable to contend with overcrowding of prison facilities and proposed contracting with state and local jurisdictions for the provision of correctional services. By 1986, CCA operated nine correctional facilities with a total of 1,646 beds.⁷¹ By 2012, CCA reported revenues of \$1.7 billion from holdings that had expanded to 66 correctional facilities, totaling 91,000 beds. CCA’s chief competitor is the GEO Group, which operates 116 prisons in the United States, the United Kingdom, South Africa, and Australia, totaling 80,000 beds and earning the company \$1.6 billion in 2012. Undaunted, other firms were entering the market: Avalon Correctional Services reported revenue of \$26 million in 2010 for community reentry programs for inmates. Other companies serving inmates include Correctional Medical Services and MHM Services, Inc., which provided mental health services in several state prisons.

Public Welfare

Several companies have become profitable through contracts with state and local government. The largest consulting group, MAXIMUS, increased its revenues from \$738.7 million in 2007 to \$930 million in 2011. Within five years of passage of the 1996 welfare reform act, for-profit firms claimed 13 percent of \$1.5 billion contracts for services, among them Lockheed Martin and MAXIMUS. Much of the profit generated by MAXIMUS was due to replacing unionized workers with less costly nonunion

employees.⁷² During the 1990s, MAXIMUS contracted to provide a range of welfare-related services through Wisconsin Works (W-2), the welfare waiver granted to then governor, and later Secretary of HHS, Tommy Thompson. Since W-2 mandated work, MAXIMUS hired Financial and Employment Planners to find jobs for welfare recipients; however, their caseloads were double what had been intended and only half of recipients were referred for employment. But the \$100 million contract had been intended as a national model for MAXIMUS, so the company redoubled its efforts to comply with provision of its contract. MAXIMUS met its caseload reduction objectives and was not only recognized with a \$100,000 Innovation in American Government Award, which was cosponsored by Harvard's Kennedy School of Government and the Ford Foundation, but also used its success in W-2 to secure a \$100 million welfare-to-work contract in New York City.⁷³

Several companies provide managed health care for populations eligible for Medicaid and SCHIP. Centene Corporation generated \$5.3 billion, and Molina Healthcare reported \$4.7 billion serving low-income people receiving Medicaid and SCHIP. Revenues of these firms will increase in those states that have adopted the Medicaid expansion under the Affordable Care Act.

Private Practice

Despite their antipathy toward privatization, many social workers engage in private practice, a commercial activity. Indeed, mental health care has been an area in which social workers have emulated the success of psychiatrists and psychologists. As a form of independent practice, private practice is influenced by the policies of the states regulating it, by professional associations, by the insurance companies that pay clinicians for their services, and by managed care companies, such as Magellan, which have become established as providers of mental health services. Private social work practice in mental health has been controversial within the social work profession, as reflected in this depiction:

In increasing numbers, social workers are flocking to psychotherapeutic pastures, hanging out their shingles to advertise themselves as psychotherapists just as quickly as licensing laws will permit. For the most part, professional

associations of social workers and schools of social work are active participants in the great transformation of social work from a professional corps concerned with helping people deal with their social problems to a major platoon in the psychotherapeutic armies.⁷⁴

Despite its detractors, private practice continues to be an attractive vehicle for delivering clinical social services. By 2001, 23 percent of social workers reported that their primary activity was in solo or group private practice; another 37 percent indicated that they were “involved” in private practice. The upsurge of social workers’ interest in private practice is such that today a large portion of students entering graduate programs in social work—as many as 80 percent⁷⁵—do so with the expressed intent of establishing a private practice. Professional schools of social work are specifically equipped to prepare graduate students for private practice. “M.S.W. programs appear to offer more to the practitioner bound for private practice than to the social worker who would prefer to work in an agency setting,” concluded researchers in a study of private and agency-based social workers.⁷⁶

The current enthusiasm for private practice can be attributed to several factors. First, private practitioners often enjoy a prestige and an income that sets them apart from salaried professionals.⁷⁷ While the median salary of social workers in private practice is about \$40,000,⁷⁸ those with an Academy of Certified Social Work credential (ACSW) and who specialize in family therapy can earn over twice that.⁷⁹ In addition, private practitioners work significantly fewer hours per week compared to colleagues in public and nonprofit settings.⁸⁰ In 2002, 41 percent of social workers in private practice worked between 10 and 29 hours per week, a greater number than colleagues in public and nonprofit settings who worked 30 hours or more, 32 percent.⁸¹ It is not surprising that social workers, who are mostly female and often underpaid—social work salaries are significantly less than those of nurses or teachers⁸²—would see private practice as a way to increase their earnings and status. In fact, women are more likely to engage in private practice than to work in traditional social service agencies; two-thirds of private practitioners are women.⁸³

Private practitioners also have a degree of autonomy that is not available to professionals who are bound by the personnel policies of traditional agencies. Researchers found that “whereas

55 percent of the private practitioners report a high level of congruence between their expectations and their activities, only 18.3 percent of the agency practitioners do so.”⁸⁴ This autonomy is important for experienced professionals who find continued supervision unnecessary or intrusive and who desire some flexibility in their work schedules to make room for other priorities. Finally, private practice allows professionals to specialize in activities at which they are best instead of having to conform to organizational requirements of the private agency or governmental bureaucracy. Again, 66.5 percent of private practitioners reported that they were able to do the things at which they excelled, whereas only 22.9 percent of agency practitioners said they could do so.⁸⁵

The image of private practice that has emerged is one of freedom and opportunity, sans rules and regulations, but this is misleading. Although private practice may involve comparatively fewer compliance requirements compared to salaried employment, it is anything but unfettered. In actuality, private practice involves many policies with which practitioners must be familiar if they are to be successful. The policies that affect private practice originate primarily from three sources: the professional community (a private entity); a government regulatory authority (a public entity); and, because of the role of managed care, corporate firms (commercial entities). This situation is complicated by the provision of service through the marketplace of a capitalist economy that traditionally discriminates against groups that do not participate fully in the labor market—minorities, women, elderly people, and people with disabilities. These groups frequently lack the resources to purchase the services provided by private practitioners. For this reason, private practice is not easily reconciled with the traditional values of social work, which emphasize service to the community and to the disadvantaged. So it is paradoxical that private practice has become a popular method of social work practice.

Despite the popularity of private practice, it has provoked controversy within the professional community. There are several aspects to this controversy, not the least of which is that many practitioners who have committed themselves to helping the disadvantaged by working in voluntary and governmental sectors view the popularity of private practice as antithetical to everything that is “social” about social work. One critic impugned the motives of social workers in private practice, noting the

increase in the numbers of social workers in private practice and admonishing those who do not admit that the primary lure of private practice is the promise of higher income.⁸⁶

Defenders of private practice emphasize the benefits of the method for practitioners and clients. Why should social workers not enjoy the same professional freedom and responsibility as other professions that use private practice extensively, for instance, law, medicine, and psychiatry? Moreover, “some clients prefer the opportunity to choose their own practitioner and a service they consider more personal and confidential.”⁸⁷ Concern for the client’s perceptions means that practitioners must be concerned about their image. This is evident in one privately practicing social worker’s description of her office:

It is decorated with comfortable chairs, built-in book cases, soft lighting, etc., and is arranged in such a way as to offer several different possibilities for seating. It is commensurate with most of the socio-cultural levels of my client group and provides him or her the opportunity for free expression without being overheard. Dealing with only one socio-cultural client group allows me to provide physical surroundings which facilitate the client’s identification with the worker.⁸⁸

On the surface, then, private practice often provokes strong responses from human service professionals who perceive private practitioners as avoiding efforts by the voluntary and governmental sectors to advance social justice. On the other hand, some private practitioners believe that their work offers them an opportunity not only to enhance their status but also to provide mental health services to a middle class that the profession has neglected.

Beneath this surface issue, there are more substantive problems raised by private practice. Perhaps the most important of these is preferential selection. In an era of specialization, professionals will refer to other providers those clients with problems that are inappropriate for their practice. An important finding of research on private practice was that private practitioners do not perceive their clients in the same way that agency-based social workers do. The latter “were significantly more likely to agree with the statement that ‘my personal values and those of my clients differ greatly’ than those in private practice.”⁸⁹ Preferential selection becomes an issue when private practitioners elect to serve less troubled clients (who are able to pay

the full cost of care) while referring multi-problem clients (who are unable to pay the practitioner's **fee for service** directly or through insurance) to agencies of the voluntary sector. Such "creaming" of the client population places an enormous burden on public agencies, which are left to carry a disproportionate share of chronically disturbed and indigent clients. In effect, then, the public sector absorbs the losses that private practitioners would suffer if they served this population. Preferential selection has become so pronounced that researchers have facetiously identified it as a syndrome. According to Franklin Chu and Sharland Trotter, the commercialization of private practice contributes to the **YAVIS syndrome**—the tendency of clients of private practitioners to be young, attractive, verbal, intelligent, and successful. One might add W to the syndrome, because the clients also tend to be disproportionately white.⁹⁰ Consequently, clients of private practitioners are less likely to be poor, unemployed, old, and uneducated.

The Future of Private Practice

In response to the limitations imposed by traditional practice settings, many human service professionals have turned to private practice as a way of securing their economic and professional objectives. Private practice gives program administrators a chance to maintain their direct service skills, educators the opportunity to continue contact with clients, and clinicians with families the freedom to combine professional practice and attention to family life. More important, private practice may prove an adjunct to agency activities. "By fostering part-time practice," researchers have noted, "the profession can keep its main focus on agency services where there is a commitment to serve persons without regard to their ability to pay and where there can be a basis for social action and reform."⁹¹

As the growth of managed care illustrates, much of private practice is a result of larger social forces. The commercialization of social work came under scrutiny as the profession became more immersed in private practice. "There is concern that too many social workers are turning their backs on their traditional casework among the poor to practice therapy," reported a national news magazine, speculating about the consequences of "an apparent middle-class therapy explosion at the expense of public welfare and grassroots service."⁹² How social work will reconcile its commitment to social

justice with the new opportunities presented by private practice remains a central question before the professional community.

Conclusion

Despite the proliferation of human service corporations, health and human service professionals have been slow to adopt the corporate sector as a setting for practice. Considering that organizations under traditional auspices—the voluntary and governmental sectors—are increasingly limited in their capacity to provide services, it is unfortunate that health and human service professionals have been loath to consider corporations as a suitable vehicle for delivering services and benefits. Actually, for-profit firms can be advantageous for several reasons. Proprietary firms may provide access to the capital needed for expanding social services, a primary explanation for their rapid growth. And human service corporations can reduce the cost of commercially derived capital by depreciating assets and writing off interest payments against income during the first years of operation. These present obvious advantages for human service administrators who are faced with diminishing revenues derived from charitable or governmental sources.

In some instances, too, the corporate sector offers more opportunities for program innovation than are possible under other auspices. Governmental programs must be mandated by a public authority, which requires a consensus on how to deal with particular concerns, while voluntary sector agencies are managed by boards of directors that reflect community norms in organizational policy. When human service issues are controversial, welfare professionals can encounter stiff opposition to needed programs. Some of this difficulty can be obviated by a corporate structure that is not so directly wedded to the status quo. The corporate sector offers greater organizational flexibility than that usually found in governmental agencies and a level of sophistication in managerial innovation rarely found in the voluntary sector. Paralleling the expansion of the health care market has been the growth in medical schools offering dual MD/MBA degrees, numbering 65 universities in 2011.⁹³ Whether or not schools of social work adopt similar programs will determine the competitiveness of their graduates in the health and human services market that is dominated by the corporate sector.

DISCUSSION QUESTIONS

1. Privatization is a hotly debated issue in social welfare. To what extent are some of the major concerns about privatization (e.g., competition between nonprofit agencies and commercial firms, preferential selection and dumping of clients, superior performance of private providers, the emergence of an oligopoly of private providers) evident in your community? Should human service professionals practice in for-profit firms?
2. If you were inclined to establish a business providing a human service, what population would you focus on? How would you get capital to start the business? Would you own the business, or would you share ownership with stockholders? How would you market your service? What would you do with the profits—provide stockholders with dividends, enlarge the business, or make contributions to nonprofit agencies? What would be the name of your business?
3. In order to attain job security, many human service professionals join unions. What is the largest union to which social workers belong in your community? Do issues relating to service delivery figure in the union's negotiations with management? Under what conditions would human service professionals in your community engage in a strike?
4. The debate between private practice and agency-based practice continues as a heated issue within social work. What are the advantages and disadvantages of each? Is there a common base of social work practice? Can you foresee some ways to resolve the issue and bring private practitioners and agency-based practitioners together?
5. Private practice continues to be a focus of students in schools of social work. How many of your classmates are planning to become private practitioners? What are their motives? Do faculty members in your social work program who also have private practices serve as role models to students? Is there an opportunity in your studies to discuss the implications of private practice for social work?
6. How does your state regulate the practice of social work? What governmental unit is responsible for regulating social work? How is it constituted? Does your state have reciprocity arrangements with other states, honoring licenses granted in other jurisdictions? Has your state's social work licensing unit expelled professionals for unethical practices?
7. What is the position of your state chapter of the National Association of Social Workers on regulating professional practice? Are licensing and vendorship still high on the state chapter's priority list? If so, which social workers in your state remain concerned about the regulation of professional practice? What are the main concerns within

your professional community about private practice—fees, misdiagnosis, licensing, image, competition with other professionals, vendorship? How are disagreements arbitrated, formally or informally?

NOTES

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The Making of Governmental Policy



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Public policy is the product of forms of government that have changed over time. Monarchies, once common in Europe, were transformed when King John of England ceded power to lords through the Magna Carta in 1215. Among influential nations, only Saudi Arabia remains a monarchy. During the Reformation, instigated in 1517 by Martin Luther, the power of the Catholic Pope was curbed. Today, Iran approximates a theocracy where religious leaders reign. Representative democracy, as outlined in the Constitution of the United States in 1789, is often contrasted with parliamentary democracy, where the executive and legislative branches of government are integrated, expediting passage of legislation, as in England. The Russian revolution of 1917 installed Communism in the Soviet Union, which collapsed in 1991, leaving only North Korea and Cuba as nations where the state rules supreme. The recent economic prosperity of China has introduced a new form of political economy, state capitalism where the communist party controls the economy, but without citizen political input. In contrast, democratic socialism, dominant in northern Europe during the twentieth century, allowed voters influence in public utilities, such as education, energy, and transportation. In 1993, the European Union was formed, which harmonized social and economic policies of its 28 nation members, many being democratic socialist. Among industrialized nations, democratic governance has become dominant. Either form of democracy—representative or parliamentary—has an overarching benefit: As Amartya Sen has observed, there are no instances of famines within truly democratic governments, nor wars fought among them.¹

In an open, democratic society, **public policy** should reflect the interests of all citizens to the greatest extent possible. For a variety of reasons, however, this ideal is not realized. Although many Americans have the right to participate in the establishment of public policy, they often fail to do so. When the have-yachts are more politically engaged than the have-nots, social policy reflects the priorities of elites. For example, between 1952 and 2008, the lowest quintile of voters exercised the franchise between 50 percent and 60 percent, while the highest quintile consistently voted 90 percent of the time.² Beyond voting, public policy may be perceived as being too far removed from the daily activities of citizens, or too complicated to warrant the type of coordinated and persistent efforts necessary to alter it. Moreover, many Americans

are not in a position to shape public policy, as in the case of children, illegal immigrants, and those with mental impairments, who must rely on others to speak on their behalf. Consequently, governmental policy does not necessarily reflect the interests—indeed, the welfare—of the public, even though it is intended to do so. Because social workers and their clients tend to be comparatively powerless, a critical analysis of the policy process is essential.

Technical Aspects of the Policy Process

Public policy in the United States is made through a deliberative process that involves the two bodies of elected officials that make up a legislature. This applies both to the federal government and to the states—with the singular exception of Nebraska, which has only one deliberative body, a unicameral legislature. Initially, a policy concern of a legislator is developed into a legislative proposal and usually printed in the *Congressional Record*. Because every legislator has a party affiliation and a constituency, legislators' proposals tend to reflect the priorities of those stakeholders. Often, several legislators will prepare proposals that are important to similar constituencies. Increasingly, policy institutes are instrumental in developing and vetting legislative options. Through a subtle interaction of ideas, the media, and legislative leadership, one proposal—usually a synthesis of several—is presented as a policy alternative. Other legislators are asked to sign on as cosponsors, and the measure is officially introduced. After the proposed bill is assigned to the appropriate committee, public hearings are held, and the committee convenes to “mark up” the legislation so that it incorporates the concerns of committee members who have heard the public testimony. Under propitious circumstances, the committee then forwards the legislation to the full body of the chamber that must approve it. While it is being approved by the full body of one chamber, a similar bill is often introduced in the other chamber, where it begins a parallel process. Differences between the measures approved by the two chambers are ironed out in a conference committee. The proposed legislation becomes law after it is signed by the chief executive, or, if the executive vetoes the bill, after it is passed by a two-thirds vote of each legislative chamber. This process is always tortuous and usually unsuccessful. The eventual enactment

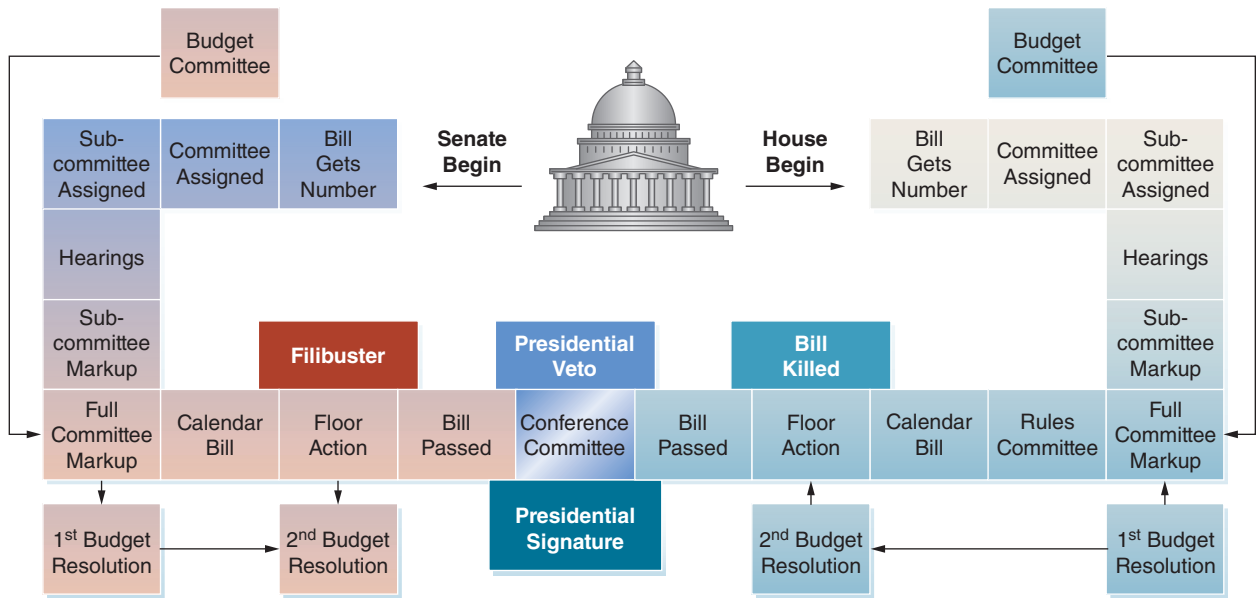


FIGURE 8.1 The Steps Necessary to Get a Policy Proposal Enacted into Law

of legislation under these conditions is a true testament to legislative leadership. A third branch of government, the judiciary, assesses legal challenges to existing legislation. In the upper levels of the judiciary, members can hold their posts for life. The primary features of the policy process of the federal government are illustrated in Figure 8.1.

The Constitution establishes a system of checks and balances designed to thwart speedy decision making. As a result, efficacy requires command of the legislative process. Decision making follows *Robert's Rules of Order*,³ which lays out in detail the rules for democratic deliberation. Although *Robert's* can appear obtuse, its value should not be underappreciated. Those who have mastered “the means of deliberation” are one step ahead of the rest of the crowd in seeing their ideas become public policy. Social activists who are optimistic that their proposal is working its way steadily through the legislative minefield may find their hopes dashed by an adroit procedural move on the part of an opponent who sidetracks a bill until the next legislative session. The elaborate rules of decision making can be compounded by the traditions of deliberative bodies. In recent years, the U.S. Senate has effectively required a 60-vote majority to pass legislation, even though such a super-majority is not explicated in the Constitution.⁴ Understandably, such procedures may tend to deter citizens from participating in the democratic process. Yet, there

are means that public policy novices can employ to become better acquainted with the ways in which elected officials go about the public's business. Citizen advocacy organizations, such as the League of Women Voters and Common Cause, can be helpful in making social policy more responsive to the public. Legislative advocacy is not for the faint-of-heart: of the tens of thousands of bills presented in Congress, only 10 percent were reported out of committee, and only 5 percent became law.⁵

Congress and the legislatures of the larger states employ staff members as technical experts to aid in decision making. Reports researched as part of committee deliberations can be valuable in that they often provide the most up-to-date data on particular programs or issues. A good example of this type of resource is the *Overview of Entitlement Programs*, also known as the *Green Book*,⁶ used by the U.S. House Ways and Means Committee in its consideration of social programs. Begun in the early 1980s to help committee members comprehend the vast number of social programs under their jurisdiction—Social Security, Medicare, child welfare, Temporary Assistance for Needy Families (TANF), Supplemental Security Income (SSI), and Unemployment Compensation, among others—this resource is essential reading for social program analysts. In recent years, the *Green Book* has been available online, making it more accessible.

The federal budget process begins each February when the president presents his or her proposed budget and culminates months later when he or she signs or vetoes the Congressional budget reconciliation bill that authorizes the budgets for 13 separate agencies. Typically, the ideal federal budget process is compromised in two ways: (1) a budget is not agreed upon, in which case a continuing budget resolution is passed extending funding at current levels until a complete budget is enacted and (2) the amount of funds appropriated is less than that authorized, leaving programs with revenue shortfalls. This means that many welfare programs that receive discretionary funds are often beset with budgets that are not timely or adequately funded. The exceptions are social entitlements, such as Social Security, Medicare, and Medicaid, which are not subject to the budget cycle.⁷

A Critical Analysis of the Policy Process

The public policy process often frustrates social activists. Despite the most urgent of needs, the best of intentions, and the most strategic of proposals, the social welfare program output derived from the legislative process often falls far short of what is required. Yet to conclude from this that public policy simply does not work would be a serious error. A critical approach to public policy not only helps explain some of the limitations of the technical approach but also suggests ways to make the legislative process a more effective strategy for those concerned with advancing social justice.

From a critical perspective, the policy process consists of a series of discrete decisions, each heavily influenced by money and connections—in other words, by power. The extent to which governmental policy reflects the concerns of one group of citizens while neglecting those of others is ultimately a question of power and influence.

In 2014, researchers analyzed 1,779 policies between 1981 and 2002 and found that interest groups were “heavily tilted toward corporations and businesses and professional associations,” which advantaged elites versus the voting majority, assuring business groups about twice the influence of “mass groups.”⁸ The status quo, in other words, favored elites. When the popular majority opposes a policy change, favoring the status quo, it is likely to prevail; however, “when a majority—even a

very large majority—of the public favors change, it is not likely to get what it wants.”⁹ The analysis essentially relegated the pluralism school to political nostalgia: “Not only do ordinary citizens not have *uniquely* substantial power over policy decisions; they have little or no independent influence on policy at all.”¹⁰ The people, in other words, are influential only when they work in tandem with business elites.

Such research supports an *elitist* orientation, positing that individuals representing a “power structure” control social policy in order to maintain a status quo that advantages them and, in the process, excludes marginal groups. In contrast, a *pluralist* orientation assumes that social policy in a heterogeneous democratic polity is the sum total of trade-offs among different interest groups, all of which have an equal opportunity to participate. At the program level, advocates of *incrementalism* have proposed that the more important questions about social policy are the product of bit-by-bit additions to the public social infrastructure. As Charles Lindblom observed, the great bulk of legislative decision making is “incremental,” representing only marginal improvements in social policy already in place.¹¹

As counterpoint, other scholars have focused on “paradigm shifts” through which major changes, such as the inception of Social Security in 1935 and the devolution of welfare to the states through the Personal Responsibility and Work Opportunity Reconciliation Act of 1996, have altered the very foundation of social policy. It can be argued that, in the final analysis, there are no new ideas, but there *are* new governmental policies that have enormous implications for certain groups. Few could dispute that the Social Security Act and the Civil Rights Act were radical departures from the status quo and substantially changed the circumstances of older people and African Americans, respectively.

Researchers presume that *rationalism* can be used to assess the extent to which policy changes bring about intended outcomes. Historically, two basic forms of rationality have served to justify social policy: bureaucratic rationality and market rationality.¹² **Bureaucratic rationality** refers to the ordering of social affairs by governmental agencies. According to bureaucratic rationality, civil servants can objectively define social problems, develop strategies to address them, and deploy programs in an equitable and nonpartisan manner. Bureaucratic rationality takes its authority from the

power vested in the state, and bureaucracies have become predominant in social welfare at the federal (through the Department of Health and Human Services) and state levels. **Market rationality** refers to a reliance on the rule of “supply and demand” as a means for distributing goods and services. While on the surface this may appear to be antithetical to the meaning of rationality, a high degree of social ordering in fact occurs within capitalism. Such organization is implicit in the very idea of a market, entailing a large number of prospective consumers that businesses seek to serve. In a modern market economy, the success of a business depends on the ability of managers to survey the market, merchandise goods and services, shape consumer preferences through advertising, and reduce competition by buying or outmaneuvering competitors. Of course, market rationality is not a panacea for providing social welfare because the marketplace is not particularly responsive to those who may not fully participate in it, such as minorities, women, children, the elderly, or people with disabilities. Yet market rationality cannot be dismissed as a rationale for delivering social welfare benefits. Approximately half of Americans get their health and welfare needs met through employer-provided benefits that are ultimately derived from the market.¹³

By contrast, *social activists* use the political process as the measure of program performance, assuming that the optimum in program assessment is continued recertification and refunding by public decision makers. While activism has been traditionally associated with liberals who advocated for the rights of workers, women, minorities, and the disabled, more recently conservatives have deployed the same methods to oppose abortion and same sex marriage. Most recently, *populists* who were concerned about how public policy affected typical Americans registered their influence through the Tea Party movement during the 2010 elections, achieving victories that continued with the 2014 midterm election. By comparison, the liberal Occupy Wall Street movement attained fewer comparable electoral victories.

Thus, the policy-scape of social welfare is quite dynamic. Two orientations help simplify the objectives of various interests. The first orientation, the **institutional conception of social welfare** holds that public policy reflects steady progress toward a desirable condition of social welfare for all. Most liberals who have promoted the welfare state as an ideal have adopted an institutional perspective,

expecting that the national government will progressively expand social programs until, eventually, the basic needs of the entire population are guaranteed as rights of citizenship. A competing orientation is the **residual conception of social welfare**, which holds that people in need should first rely on the labor market, then on family and friends, and next on voluntary organizations in the community but should rely on government social programs only as a last resort and only temporarily. The residual conception, favored by conservatives, emphasizes self-sufficiency, neighborliness, and limited government assistance. To the extent that ideology is congruent with the primary political parties, it follows that the institutional conception has been advanced by Democrats, while the residual conception has been favored by Republicans.

The Policy Process

A critical analysis of the policy process highlights the social stratification of the society, the phases through which policy is formulated, and the organizational entities that have become instrumental in the decision-making process.

Social Stratification

Social stratification differentiates the influential from the marginal. A stratification system familiar to Americans defines three parts: an upper class, a middle class, and a lower class. Placement of individuals in the appropriate class is usually made on the basis of income, education, and occupational status. This three-part stratification is limited in its capacity to explain very much about U.S. social welfare, since most Americans identify themselves as middle class, even if by objective criteria they belong to another stratum. Further, the designation *lower class* is not particularly informative about the social conditions of a large portion of the population with which human service professionals are concerned.¹⁴

A more informative stratification consists of six social groups, which are differentiated according to wealth, internal solidarity, and control over the environment, as elaborated in Table 8.1.¹⁵ As this stratification model indicates, some groups—the wealthy and executives—are able to influence the environment, but other groups—the working/welfare poor and the underclass—have virtually

TABLE 8.1 Social Stratification of the Population

Name of Group	Examples	Characteristics
The wealthy	Upper elites, the independently wealthy, large stockholders	Ownership of resources is the main source of power; control over goals is very high, but control over means is through executives.
Executives	Top administrators in business, government, the military	Organizational solidarity facilitates effective policy implementation; some control over goals and a high degree of control over means.
Professionals	Middle-level managers, technical experts, private practitioners, community leaders	Environment encourages limited solidarity; control over means is high, and goal setting can be influenced if collective action is undertaken.
Organized workers	Semiskilled workers, civic and political clubs, social action organizations	Environment encourages solidarity; groups have some control over the means by which goals are realized.
Working/welfare poor	Temporary and part-time workers earning minimum wage and who use welfare as a wage supplement	A subjugated position with no control over the environment; frustration is shared and irrational, explosive behavior results.
Underclass	Unemployables and illiterates; disabled substance abusers; itinerants, drifters, migrant workers	A subjugated position with no control over the environment; a sense of failure coupled with limited mobility reduces social interaction and leads to retreatism.

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no influence. This distinction has important implications for social welfare, because those who are of lower status tend to be the recipients of welfare benefits that are the product of a social policy process in which they do not participate. The way in which these various groups influence the social policy process will be discussed in greater detail in the following section.

With these clarifications in mind, the policy process can be divided into four stages: formulation, legislation, implementation, and evaluation.

Formulation

Before the nineteenth century, it would have been accurate to state that policy formulation began with the legislative phase. With the advent of industrialization, many complexities were injected into the society, and in time special institutions emerged to

assist the legislature in evaluating social conditions and preparing policy options. Eventually, even constitutionally established bodies, such as Congress, lapsed into a reactive role, largely responsive to other entities that formulated policy.¹⁶ Initially, institutions of higher education provided technical intelligence to assist the legislative branch, and some still do. For example, the University of Wisconsin's Institute for Research on Poverty provides analyses on important welfare policies, while Harvard University's Kennedy School of Government promotes innovations in social policy.

That legislators at the federal level, as well as those in the larger states, would rely on experts to assess social conditions and develop policy options is not surprising, given the fact that each legislator must attend to multiple committee and subcommittee assignments requiring expertise in particular matters, while at the same time contend with

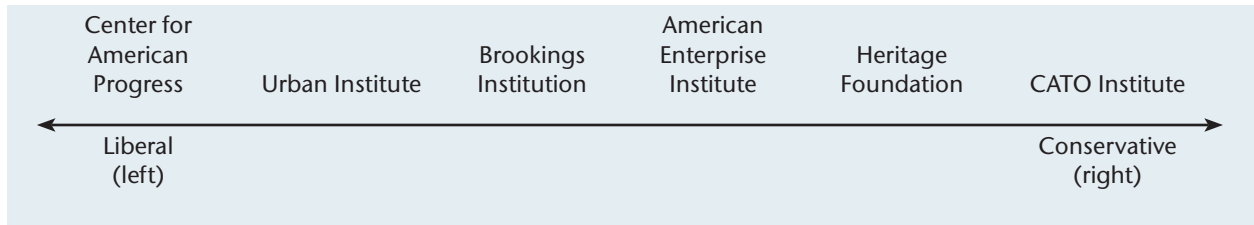


FIGURE 8.2 Place on the Ideological Continuum of Six Policy Institutes

the general concerns of a large constituency. As a result of competing demands, legislators pay somewhat less attention to the policy process than their public image would suggest, leaving much of the work to their staffs. Even then, public policy tends to get short shrift. Because reelection is a primary concern for legislators, their staffs are frequently assigned to solve the relatively minor problems presented by constituents. In fact, placating unhappy constituents has become so prominent a concern that one legislative observer notes that constituency services—called “casework” by elected officials—have become “more important than issues” for representatives.¹⁷

Gradually, institutions have begun to specialize in providing the social intelligence necessary for policy formulation. These policy institutes, sometimes called **think tanks**, now wield substantial influence in the social policy process. Not unlike prestigious colleges, think tanks maintain multidisciplinary staffs of scholars who prepare position

papers on a range of social issues. With multimillion-dollar budgets and connections with national and state capitals, think tanks are well-positioned to shape social policy. Generally, financial support for these institutes comes from wealthy individuals and corporations with particular ideological inclinations that are evidenced by the types of think tanks they support. Several prominent policy institutes are located on the ideological continuum in Figure 8.2.

Legislation

At the federal level, public policy is made by members of Congress who are appointed to committees and subcommittees based on their particular interests. An important and often unappreciated component of the legislative phase is the role played by the staffs of committees and subcommittees. Former legislative staffers are definitive experts in the subject area of a committee and are prized as lobbyists for

Spotlight 8.1

THE POLICY PROCESS

Public policy in the United States is made through a deliberative process that involves the Senate and House of Representatives. You can learn more about each of these bodies of elected officials at their respective websites: www.senate.gov and www.house.gov. For a detailed explanation of the legislative process and how laws are made, visit <http://thomas.loc.gov/home/lawsmade.toc.html>. Political action committees (PACs) play an influential

part in decisions on public policy. In the United States, PACs represent almost every group or interest imaginable. To review the various PACs, go to the Google search engine (www.google.com) and type in the words *political action committee*.



Watch this video of Lawrence Lessig discussing the role of PACs in public policy.

www.youtube.com/watch?v=UnOn60YtcGo

special interest groups.¹⁸ As a result of the increasing complexity of the policy process, the number of legislative staff members has multiplied. By 2010, 24,000 staff members served Congress, more than double the number in the late 1970s.¹⁹ Committees are the locus of testimony on issues, and legislative hearings provide an opportunity for the official and sometimes the only input from the public on some matters. Accordingly, representatives of advocacy groups make it a point to testify before certain committees in order to ensure that their views are heard. At the federal level, the primary Senate committees are Finance, Agriculture, Appropriations, Health, Education, Labor, and Pensions, while those in the House are Ways and Means, Economic and Educational Opportunities, and Appropriations.

Throughout the process, representatives of special interests attempt to shape any given proposal so that it is more congruent with priorities of their members. Special interests can be classified according to the nature of their activities: During legislative sessions, **lobbyists** are hired to influence legislation valued by clients. As noted in Table 8.2,

TABLE 8.2 Top Spenders, 1998–2015

Sector	Total
Pharmaceutical/Health Products	\$3.1 billion
Insurance	2.2 billion
Electric Utilities	2.0 billion
Electronics Mfg & Equip	1.8 billion
Business Associations	1.8 billion
Oil & Gas	1.7 billion
Misc Manufacturing & Distributing	1.4 billion
Education	1.4 billion
Hospitals & Nursing Homes	1.3 billion
Securities & Investment	1.3 billion
Real Estate	1.2 billion
Civil Servants/Public Officials	1.2 billion
Telecom Services	1.2 billion
Health Professionals	1.2 billion
Air Transport	1.1 billion

Source: Based on Top Spenders, 1998–2015, Center for Responsive Politics, OpenSecrets Retrieved from <https://www.opensecrets.org/lobby/top.php?indexType=s>.

TABLE 8.3 Top Eight Lobbying Firms, 1998–2015

Firm	Total
Akin Gump	\$9.6 million
Squire Patton Boggs	6.6 million
Brownstein Hyatt	5.7 million
Podesta Group	5.6 million
Van Scoyoc Assoc.	4.7 million
K & L Gates	4.7 million
Holland & Knight	4.7 million
Williams & Jensen	4.4 million

Source: Based on Top Lobbying Firms, 1998–2015, Open Secrets /Center for Responsive Politics, Retrieved from <http://www.opensecrets.org/lobby/top.php?indexType=l>.

business interests dominated in recent decades, especially in regards to health care. By contrast, **Political Action Committees (PACs)** divert campaign funds to candidates for elected office.

The beneficiaries are lobbying firms who have done exceedingly well during the same period, as shown in Table 8.3.

During elections, PACs campaign on behalf of specific candidates. Their contributions are limited and must be reported to the Federal Election Commission. In recent decades, substantial funds have been expended in order to influence elections for the purpose of shaping public policy. Contributions are made to political parties and candidates who take positions that are congruent with a donor's interests. As a result, larger contributors tend to prefer one political party over another by channeling contributions through PACs, as evident in Table 8.4. In national politics, the key to influence is choreographing the resources of PACs, soft money, and lobbyists to attain party objectives.

Advocacy groups are able to influence policy indirectly by establishing 527 Committees (named for the Internal Revenue Service code that sanctions them) that advocate for specific causes. As such, 527s are prohibited from funding specific candidates or political parties (see Table 8.5).

Over time, lobbying and campaign contributions are used to redefine politics and policy. Well-choreographed, significant changes in social policy have been achieved, and these have affected social welfare. For example, repeating their takeover of Congress in 2002, Republicans again won a majority of both

TABLE 8.4 Top 20 PAC Contributors to Federal Candidates, 2013–2014

Name	Total Amount	Percent Democratic	Percent Republican
National Realtors Assoc.	\$3.8 million	48%	52%
National Beer Wholesalers	\$3.2 million	44%	56%
Honeywell International	\$3.2 million	44%	56%
National Auto Dealers Assoc.	\$2.8 million	29%	71%
Lockheed Martin	\$2.6 million	42%	58%
American Bankers Assoc.	\$2.5 million	23%	76%
AT&T	\$2.5 million	40%	60%
Operating Engineers Union	\$2.5 million	80%	20%
Credit Union National Assoc.	\$2.5 million	49%	51%
Intern'l Brotherhood of Electrical Workers	\$2.4 million	97%	3%
Blue Cross Blue Shield	\$2.4 million	39%	61%
Northrup Grumman	\$2.4 million	43%	57%
National Assoc. of Insurance and Financial Advisors	\$2.3 million	38%	62%
American Assoc. for Justice	\$2.2 million	96%	4%
Boeing	\$2.2 million	42%	58%
National Rural Electric Coop Assoc.	\$2.2 million	28%	72%
Laborers Union	\$2.1 million	85%	15%
Comcast	\$2.1 million	48%	52%
Plumbers/Pipefitters Union	\$2.1 million	97%	3%
American Crystal Sugar	\$2.1 million	56%	44%

Source: Based on Top 20 PAC Contributors to Candidates, 2015–2016, Open Secrets/Center for Responsive Politics. Retrieved from <https://www.opensecrets.org/pacs/toppacs.php>.

TABLE 8.5 Top Five 527 Organizations, 2014

Organization	Contributions
NextGen Climate Action	\$21.9 million
Service Employees International Union	\$19.4 million
American Federation of Teachers	\$14.0 million
Plumbers/Pipefitters Union	\$4.7 million
Carpenters & Joiners Union	\$4.4 million

Source: Based on Top 50 Federally Focused Organizations, Open Secrets/Center for Responsive Politics, Retrieved from <https://www.opensecrets.org/527s/527cmtes.php>.

chambers in 2014. Ultimately, their objective was an era of political dominance that would rival—and reverse—the legislative triumphs of the New Deal coalition that engineered the American welfare state.²⁰ Two British journalists, John Micklethwait and Adrian Wooldridge, noted in 2004 that for at least 40 years, conservatives have consistently been better organized and better able to sell their point of view. They do not attribute the right's success to higher intelligence or better funding, citing the resources of liberal universities and foundations. Instead, they point to the persistence and focus of right-leaning organizations such as the Heritage Foundation, which they call “ruthless” in pursuing their goals.²¹

Scrambling to regain influence in domestic policy, a group of philanthropists established the

Phoenix Group to encourage innovative thinking among Progressives. Tycoons George Soros and Peter Lewis “had come to view progressive politics as a market in need of entrepreneurship, served poorly by a giant monopoly—the Democratic Party—that is still doing business in an old, Rust Belt kind of way.”²²

An open, democratic polity provides ample opportunities for groups to mobilize members to advance their cause. EMILY’s List, founded by Ellen Malcolm in the mid-1980s, is illustrative. Malcolm established this PAC to increase the number of pro-choice members of Congress and to raise funds to encourage Democratic women to enter politics. Coining an acronym for “early money is like yeast, it makes the dough rise” EMILY’s List focused on primaries, figuring that “if we could raise enough early money to get women started, then they could convince the traditional fundraisers that they were viable, and they could then raise the money they needed to win,” Malcolm recalled. Forging strong relationships between female candidates and female donors augmented the focus on primaries and vaulted EMILY’s List to the forefront of progressive politics. Three decades after its inception, EMILY’s List had leveraged \$86 million for pro-choice candidates, increasing the number of Democratic women in Congress fourfold.²³

Compared to business, labor, and lobbyists, most social advocacy groups bring few assets to bear on the political process. Limited by meager resources, social advocacy groups usually rely on volunteer lobbyists. There are several advocacy groups within social welfare that have been instrumental in advancing legislation to assist vulnerable populations—among them the American Public Human Service Association, the Child Welfare League of America, the Children’s Defense Fund (CDF), the National Association for the

Advancement of Colored People, the National Urban League, the National Assembly of Voluntary Health and Welfare Associations, and the National Organization for Women. Of these advocacy groups only one, AARP, ranks among the top 100 lobbyists on Capitol Hill. Despite the number of welfare advocacy organizations and their successful record in evolving more comprehensive social legislation, changes in the policy process hamper their work. Increases in the number of governmental agencies as well as in their staffs make it difficult to track policy developments and changes in administrative procedures. Worse, the escalating cost of influencing social policy, evident in the number of paid lobbyists and in the contributions lavished by PACs, is simply beyond the means of most welfare advocacy organizations. As one Democratic candidate for the Senate lamented, “only the well-heeled have PACs—not the poor, the unemployed, the minorities or even most consumers.”²⁴

In 2010, the Supreme Court ruled in *Citizens United v. the Federal Election Commission* that campaign contributions in any amount were protected under the First Amendment, effectively neutralizing federal campaign finance limits. The result increased the influence of wealthy interests while diminishing those of less well endowed.²⁵ Almost instantly, “super PACs” emerged and pumped unprecedented amount into the 2012 election, as shown in Table 8.6.

Citizens United and a subsequent Supreme Court decision, *McCutcheon v. Federal Election Commission*, which permitted contributions to an unlimited number of candidates, upended campaign regulations. By 2015 an avalanche of cash was diverted from 527 committees and traditional PACs to “super PACs.” Exploiting section 501(c)(4) of the IRS Code, intended for nonprofit “social

TABLE 8.6 Top Five Super PACs, 2014

Name	Ideology	Expenditures
Senate Majority PAC	Liberal	\$46.6 million
House Majority PAC	Liberal	\$29.4 million
Freedom Partners Action Fund	Conservative	\$23.4 million
Ending Spending Action Fund	Conservative	\$22.6 million
American Crossroads	Conservative	\$21.9 million

Source: Based on Super PACs, Open Secrets/Center for Responsive Politics, Retrieved from <https://www.opensecrets.org/pacs/superpacs.php>.

welfare” organizations, super PACs were able to keep their donors secret, aptly referred to as “dark money.” Quickly, contributions from billionaires and corporations flooded into the coffers of presidential candidates. In the run-up to the 2016 presidential election, candidates sought the support of super PACs supported by billionaires: Foster Friess supporting Rick Santorum, Sheldon Adelson interviewing Jeb Bush, Chris Christie, Scott Walker, and John Kasich, while Hillary Clinton courted Alice Walton of Wal-Mart, in her attempt to build a \$2.5 billion campaign war-chest.²⁶ The Supreme Court decisions thus altered the campaign environment so that politically motivated billionaires became their own political parties.²⁷

Compounding the shift in the polity toward wealthy individuals and large corporations, changes in voter registration and legislative redistricting favored conservative Republicans. While Democrats benefited from demographic shifts favoring minority populations, which should attain a national majority of the population about 2040, Republicans sought to shore-up a dwindling number of aging, White voters by passing state voter ID laws. Redistricting, constitutionally mandated after each census, has also provided an opportunity for state legislatures to craft Congressional districts so as to marginalize minority candidates, a practice that has been blatant in the South. In combination, these developments led long-time political observer, Elizabeth Drew to lament, “In recent years, we’ve been moving further and further away from a truly democratic election system.”²⁸

The Fiscal Cliff and Sequestration

Assuming the presidency in 2008 as the nation sunk into the Great Recession, Barack Obama sought to bolster a flagging economy through \$787 billion in stimulus spending over 10 years while correcting financial abuses via the Wall Street Reform and Consumer Protection Act and extending health care to the uninsured by the Patient Protection and Affordable Care Act. However, a nascent Tea Party movement used this legislation to mobilize Republicans who took over the House of Representatives in the 2010 midterm elections. In short order, conservatives sought to block Obama’s liberal designs in social policy. Budgetary brinkmanship would surface in 2010 when several major policy changes loomed, including the expiration of the Bush tax cuts, extended unemployment benefits, and the 2 percent reduction in the withholding tax.

Using what had been an automatic increase in the federal debt ceiling, House Republicans succeeded in passing the Budget Control Act (BCA) of 2011, which required dollar-for-dollar reductions in federal spending, totaling \$1.1 trillion over eight years. A provision of BCA mandated automatic spending reductions but exempted social entitlements, such as Social Security, Medicare, and Medicaid. In targeting discretionary spending, BCA would, if spending was not brought under control, cut programs dear to conservatives, such as defense, as well as those held close by liberals, such as child care.

Countervailing forces—liberal Democrats insisting on tax increases versus conservative Republicans demanding spending cuts—came to a head on December 31, 2012, when Congress went to the 11th hour before passing legislation that resolved the conflict. This episode, called the “fiscal cliff” because it threatened to place the federal government in default and push the economy back into recession, was resolved by a deal that increased revenue through tax increases but postponed spending cuts. The payroll tax, taxes on individual earning more than \$400,000 and couples more than \$450,000, and several taxes on investments all increased; however, spending cuts remained a source of bitter contention. As a result, automatic cuts on non-entitlement discretionary programs, the “sequester,” was triggered. Beginning March 2013, federal spending was cut \$84 billion for the year. Although critics criticized sequestration as “dumb policy” because it was indiscriminate, response to its initial effects were muted. To some extent, the political parties achieved their objectives, although at the other’s expense: Democrats saw cuts in defense spending, while Republicans applauded reductions in Head Start. On the other hand, both parties were unhappy about cuts to the research budget of the National Institutes of Health, which was on the cusp of important breakthroughs in health care, especially individualized genetic treatments for cancer.

Eventually, the stalemate that produced the fiscal cliff and sequestration began to dissipate due to political and economic developments. In 2012, President Obama won a resounding reelection victory, while the Congressional ranks of the Tea Party were reduced sufficiently to give the movement pause. This left Republicans in the House with fewer options to control spending, short of refusing to authorize an increase in the federal debt ceiling, which would throw the federal budget into default, brinkmanship that had disastrous consequences for

Republicans during the Clinton presidency. Slowly, the economy began to improve. Economic growth reduced the federal deficit as a percent of expenditures, taking some of the fire from Right's obsession with deficit spending. While these augured well for liberals, developments after the 2012 were hardly cause for celebration. The 2014 midterm elections precluded expanding social entitlements, indeed the Obama administration would be preoccupied with defending accomplishments of its first two years unless Democrats retake control of Congress in 2016. Products of economic austerity, the fiscal cliff and sequestration reflect a conundrum in social policy: how to honor commitments to social programs without raising taxes.

In the highly charged climate of modern American politics, the pursuit of fiscal responsibility is a proxy for a larger ideological agenda. The "no compromise" tactic of Tea Party and Libertarian Republicans intends to shred the social safety net that they characterize as a "hammock," create a laissez faire business environment with little or no regulation and roll back government at every level except for the Pentagon. On the defensive, Democrats argue for maintaining basic social welfare and other governmental services. The battle over deficits, sequestration, and the threat of pushing the United States into default are likely to continue given political polarization.

Implementation

Six months after sequestration had taken effect, the federal government braced for its first shutdown since the mid-1990s. The slow motion train-wreck of 2013—a sequester leading to a government shutdown culminating in a possible federal debt default—imposed the equivalent of a Tea Party tax that cost the nation between \$3.1 billion and \$24 billion, threatening not only the economic recovery but also the benefits of millions of Americans as well. Resolved at the 11th hour by a deal that expired in early 2014, raising the specter of further economic wreckage, the future of the American welfare state remained in doubt. While the federal shutdown and prospect of debt default dominated the headlines, sequestration continued to cut into discretionary funding, including many social programs. Public opinion faulted Republicans for their brinkmanship, strengthening Obama and the Democrats, yet the prospect of future battles over spending, the debt, and taxes seemed inevitable.

The fact that a policy has been enacted does not necessarily mean it will be implemented. Often governmental policies fail to provide for adequate authority, personnel, or funding to accomplish their stated purposes. This has been a chronic problem for social welfare programs. It is also possible that a governmental policy initiative will not be enforced even after it has been established. Many local jurisdictions have correctional and child welfare institutions now operating under court supervision because judges have agreed with social advocates that these institutions are not in compliance with state or federal law.

Implementation is that much more problematic when the public is ambivalent about governmental institutions. Albert O. Hirschman, differentiated between "private interest and public action" in depicting the episodic nature of public support for governmental institutions. According to Hirschman, public endorsement of governmental institutions is a fundamental problem for industrialized capitalist societies, which emphasize individual competitiveness while generating social and economic dislocations that require collective action. "Western societies," Hirschman observed, "appear to be condemned to long periods of privatization during which they live through an impoverished 'atrophy of public meanings,' followed by spasmodic outbursts of 'publicness' that are hardly likely to be constructive."²⁹ Disenchantment with governmental solutions to social problems, most recently evident in the Tea Party movement, makes public welfare programs vulnerable to their critics, leading to reductions in staff and fiscal support, often followed by an escalation in the social problem for which the social program was initially designed. Thus, the episodic nature of public support for programs designed to alleviate social problems further impedes effective implementation.

Evaluation

The expansion of governmental welfare policies has spawned a veritable industry in program evaluation. Stung by the abuses of the executive branch during Watergate and the Vietnam War, Congress established additional oversight agencies to review federal programs.³⁰ As a result, multiple units within the executive and legislative branches of government have the evaluation of programs as their primary mission. At the federal level, the most important of these include the Government Accountability Office (GAO), the Office of Management and Budget (OMB), the Congressional Budget Office (CBO),

and the Congressional Research Service (CRS). For example, OMB developed the Program Assessment Rating Tool (PART) to evaluate federal programs, concluding that one-third were inadequate. Between 2002 and 2006, the number of programs determined to be “ineffective” or “results not demonstrated” was reduced from 55 percent to 25 percent.³¹ State governments have similar units. In addition, departments have evaluation units that monitor program activities for which they are responsible.

Federal and state levels of government commonly contract with nongovernmental organizations for evaluations of specific programs. As a result, many universities provide important research services to government. The University of Wisconsin’s Institute for Research on Poverty is distinguished for its research in social welfare, Chapin Hall at the University of Chicago evaluates child welfare programming, and Harvard’s Kennedy School of Government has a reputation for policy innovation. In the past three decades, private consulting firms, such as MDRC, Abt Associates, MAXIMUS, and Mathematica Policy Research, have entered the field, often hiring former government officials and capitalizing on their connections in order to secure lucrative research contracts. Of course, any politicization of the research process is frowned upon, because it raises questions about the impartiality of the evaluation.

On the eve of the 2012 election, for example, the Congressional Research Service released a report indicating that tax cuts for the wealthy did not contribute to economic growth, but it used language to which Republicans objected. The report was later withdrawn when it was revealed that its author had made campaign contributions to several Democratic organizations.³² Questions about the closeness between governmental agencies, research firms, party politics, and the validity of evaluation studies have become common; indeed, the consulting firms located near the expressway surrounding Washington, DC, are often referred to as “the beltway bandits.” Whether conducted in-house or contracted out to universities or commercial firms, evaluation research has become a major industry.

Marginalization

If the governmental decision-making process is somewhat irregular and irrational, it is also unrepresentative. As illustrated in Figure 8.3, groups in the upper levels of the social stratification populate the institutions through which policy is made. Significantly, many members of Congress are wealthy; the average net worth of Republicans in 2012 was \$881,786 while that of Democrats was \$862,508.³³ In the case of welfare policy, welfare beneficiaries must adjust to rules established

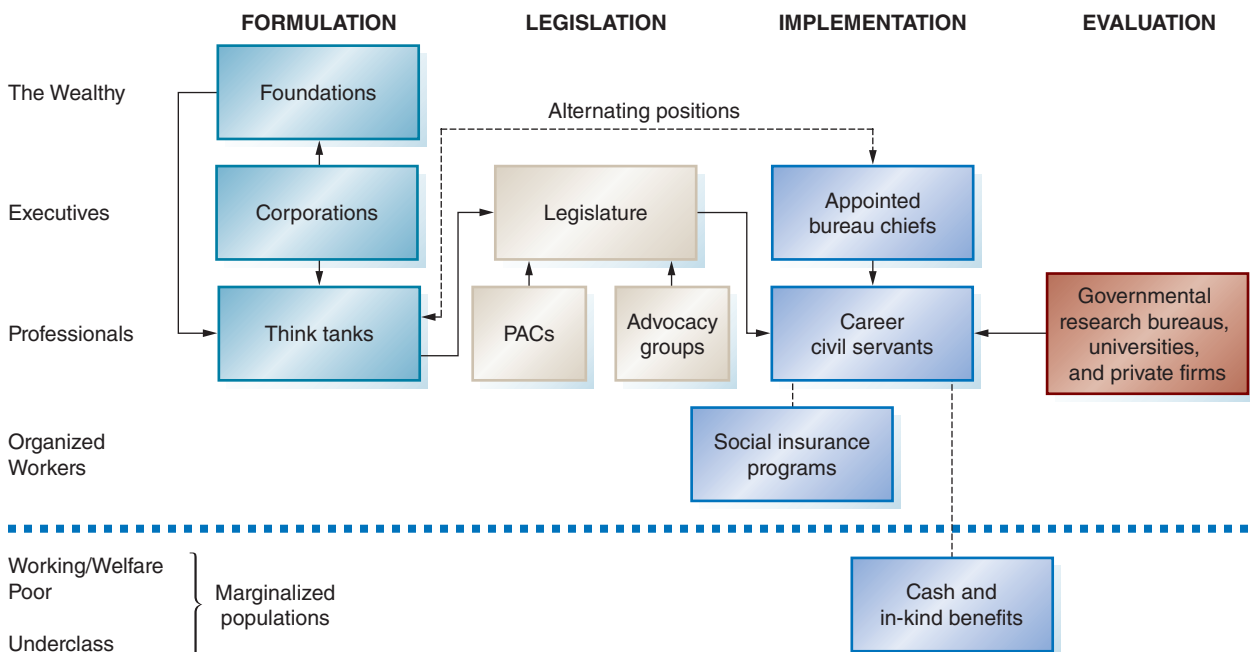


FIGURE 8.3 Stratification and the Public Policy Process

by other social groups. The primary players in the social policy game are executives and professionals. Lower socioeconomic groups' lack of influence in the social policy process is virtually built into governmental decision making. The term *non-decision making* has been coined to describe this phenomenon—the system's capacity to keep the interests of some groups off the decision-making agenda.³⁴ Marginalization has a long history in the United States; generations of African Americans and women were legally excluded from decision making prior to emancipation and suffrage.

The governmental policy process also poses problems for administrators and practitioners. Policies frequently reflect assumptions about the human condition that may seem reasonable to the upper socioeconomic groups that make them but bear little resemblance to the reality of the lower socioeconomic groups that are supposed to be beneficiaries. For example, Child Support Enforcement (CSE) policy assumes that fathers of children on TANF have the kind of regular, well-paying jobs that would allow them to meet the amounts of their court orders, whereas often their jobs are intermittent and offer low wages. Consequently, support payments to children who are dependent on welfare have been disappointing. In 2006, \$21.8 billion in child support was collected through the CSE program, but most of this was for non-TANF families; only

\$2.1 billion of TANF benefits were recovered through CSE.³⁵ For non-welfare households, CSE's revenues exceeded administrative costs, but for welfare families, those for whom the program was initially justified, CSE lost money.³⁶

Making the public policy process more representative is a primary concern of welfare advocates. Since the Civil Rights movement, African Americans and the poor have recognized the power of the ballot, and voter registration has become an important strategy for advancing the influence of these groups. The mobilization of marginal groups was widely understood to have vaulted Barack Obama to the White House and provided Democrats control of Congress in 2008, while their failure to vote in comparable numbers helped Republicans win an unprecedented number of victories in the midterm elections only two years later, a victory repeated in 2014.

Political parties and campaign managers who contend with the growing ranks of independent voters have come to appreciate segmentation of the voting public. The Pew Research Center for the People and the Press has divided the electorate into nine groups, according to their affinity for different issues. In an important book, Jonathan Haidt has identified six primary values that voters articulate. These are depicted in Figure 8.4. According to Haidt, four of the six primary values held

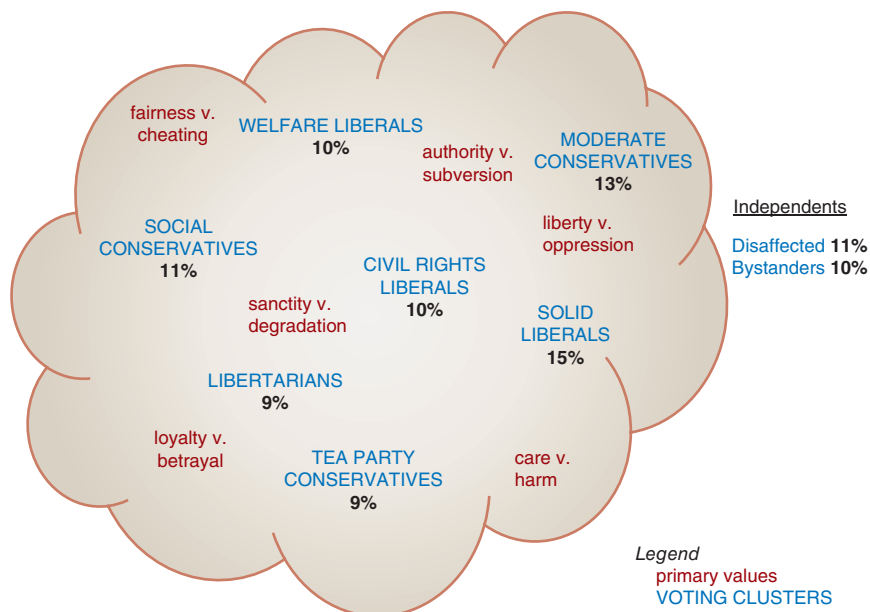


FIGURE 8.4 Primary Values and Voting Clusters

by Americans favor conservatives, leaving two to liberals.³⁷ Conservative groups reflected 44 percent of voters, while liberals 35 percent. This means that the victorious presidential candidates need to make a concerted effort to appeal to the 21 percent of independent voters in order to obtain a majority. Congressional candidates have to appeal to a similarly fluid pool of voters, although the dynamic varies with each state for senators and districts for House members. Increasingly, Congressional elections are held in jurisdictions that have been gerrymandered into liberal or conservative enclaves, contributing to political gridlock.³⁸ Clearly, advocates of social and economic justice cannot afford to rest on their laurels if they wish to be competitive in the electoral marketplace.

Logically, increasing the involvement of apathetic voters would not only make inroads against marginalization but also make public social programs more responsive to their circumstances. In 2010, only 37.8 percent of the voting-age population voted in the national election.³⁹ The advent of voter ID laws makes that more difficult. Subsequent to passage of the National Voter Registration Act in 1993—aka the motor voter law—True the Vote, founded by a Tea Party activist, worked to reverse the expansion of the number of low-income, minority voters. Alleging that voter fraud was pervasive, True the Vote disseminated a model statute, developed by the American Legislative Exchange Council, to require photo ID in order to vote. By 2011, 37 states had passed voter ID laws. Former President Clinton inveighed against the requirement, likening it to poll taxes that suppressed the black vote during Jim Crow. While courts in several states postponed implementation until after the 2012 election, proponents of voter ID allied with conservative legal groups in order to challenge the legitimacy of voters in battleground states if the election is close.⁴⁰ In 2012, President Obama won reelection by mobilizing a coalition of minorities, the well-educated, women, students, and blue-collar workers, leaving the Republican party with the task of appealing to a voting public that was no longer dominated by white men.

Social Work and Advocacy Organizations

The formulation of social welfare policy in the United States is a complicated, arduous, and often discouraging process. Much of this can be attributed to the nature of U.S. culture: to the competing

interests inherent in a pluralistic society; to the multiple systems of government that make social policy at the same time; to public and private bureaucracies that serve large numbers of consumers; and to economic and technological developments that lead to specialization. Under these circumstances, changing social welfare policy to improve the circumstances of disadvantaged groups can be a daunting task. Regrettably, few human service professionals consider social policy advocacy an enterprise worthy of undertaking. Most social workers prefer direct service activity, in which they have little opportunity for direct involvement in social policy. Some social workers attain important positions in federal and state human service bureaucracies and are close to the policy process. Unfortunately, these managers are often administering welfare policies that have been made by legislators who do not necessarily represent either clients or human service professionals. Perhaps most troubling is the fact that the involvement of social workers in the formulation of social policy has been diminishing in recent years. In a provocative statement, June Hopps, former dean of the Boston College School of Social Work and former editor-in-chief of *Social Work*, acknowledged that “since the late 1960s and early 1970s, the [social work] profession has experienced a dramatic loss of influence in the arenas where policy is shaped and administered.”⁴¹ Though opportunities to enter the upper levels of the federal civil service have become available through the Presidential Management Fellowship, social work has not found a way to systematically exploit this.

One indicator of good social policy is the correspondence between the policy and the social reality of its intended beneficiaries. Thus, it's clear that social welfare policy would be enhanced by the input of social workers. However, social workers have left much of the decision making about social welfare to professionals from other disciplines. “There are increasing numbers of non-social workers, including psychologists and urban planners,” observed Eleanor Brilliant, “taking what might have been social work jobs in service delivery and policy analysis.”⁴² The consequences of welfare professionals opting to leave social policy in the hands of others are important. For direct service workers, these consequences can mean having to apply eligibility standards or procedures that, although logical in some respects, make little sense in the social context of many clients. For the public, they may mean a gradual disenchantment with social programs that do not seem

to work. The causes of the retrenchment affecting social programs are complex, of course; however, it is worth noting that public dissatisfaction with social programs has escalated as social welfare professionals have retreated from being actively engaged in public policy.

The rebuilding of a role for social workers in social policy will take concerted effort. Individual leadership is a necessary, but no longer sufficient, precondition for this objective. Essential to the undertaking will be social workers' ability to understand and manage complex organizations and programs. In fact, this skill may be the most critical for welfare professionals to acquire if they are to advance social justice, for it addresses a question central to the postindustrial era. During the Industrial Revolution, Karl Marx suggested that the central question was "who controls the means of production?" A mature industrial order and the expansion of civil bureaucracy led Max Weber to ask, "who controls the means of administration?" The evolution of a postindustrial order, in which primary economic activity occurs in a service sector dependent on processed information, raises a new question: "who controls the means of analysis?" If social workers are to shape social policy as effectively as they had in the past, they will have to learn to control the means of analysis. This means conducting research on social problems, surveying public opinion about welfare programs, analyzing existing social policy for opportunities to enhance welfare provision, and winning elected office in order to make decisions about proposed social welfare policies.

Advocacy Organizations and the New Policy Institutes

Policy analysis organizations have been instrumental in shaping social policy since the New Deal. Subsequently, policy institutes have had liberal or conservative labels ascribed to them, with the liberal organizations, such as the Brookings Institution and the Urban Institute, achieving dominance up until the late 1970s when conservative institutes, such as the American Enterprise Institute, the Heritage Foundation, and the CATO Institute, ascended. In response, social reformers established a new group of policy analysis organizations.

Children's Defense Fund Begun by Marian Wright Edelman in 1974, the CDF sought to address the health, income, and educational needs of the

nation's children.⁴³ By the mid-1980s, CDF had become a major voice in children's policy and had successfully advocated programs at the federal and state levels. CDF advocated for the 1998 State Children's Health Insurance Program (SCHIP). CDF is noted for its educational publications, particularly for eye-catching posters depicting injustices inflicted on poor, minority children. Its annual report, *The State of America's Children*, is an authoritative compendium of issues and programs concerning children.

The Center on Budget and Policy Priorities (CBPP) Established in 1981 by Robert Greenstein, former administrator of the Food and Nutrition Service in the Agriculture Department, CBPP has fought to defend social programs for low-income people against budget cuts. CBPP distributes its analyses to Congressional staffs, the media, and grassroots organizations, providing much of the program analysis to refute arguments presented by conservative officials in their efforts to cut programs for the poor. Significantly, CBPP and CDF have developed a close working relationship. CBPP regularly provides data to CDF on the health and income status of children.⁴⁴

Economic Policy Institute (EPI) In 1986, Jeff Faux convinced labor-oriented liberals to establish a policy institute to represent "the perspective of working families and the poor."⁴⁵ During the subsequent decade, EPI expanded rapidly and published reports depicting a widening chasm between the economic circumstances of working families and the affluent. Notable among EPI publications is its annual volume *The State of Working America*, which contains current data on income, taxes, wages, wealth, and poverty.

Institute for Women's Policy Research (IWPR) Created in 1987 by Heidi Hartmann, IWPR quickly attained a respected position in Washington, DC. The IWPR research and advocacy agenda relating to women is extensive, including retirement, family leave, health insurance, welfare reform, and pay equity. In 1994, Hartmann received a MacArthur "genius" fellowship award for her advocacy of gender equity.

New America Foundation In 1999, Ted Halstead launched the New America Foundation in order to develop policy options congruent with the

information age. With Michael Lind, Halstead wrote *The Radical Center*, a book that outlined the need for an ideological orientation that was post-liberal as well as post-conservative.⁴⁶ New America has promoted important policy innovations, such as Individual Development Accounts.

Center for American Progress Smarting from reversals attributed to conservative think tanks, John Podesta convinced other liberals to establish the Center for American Progress in 2003. Drawing on an annual budget of \$10 million and many policy experts who had served in the Clinton administration, the Center for American Progress soon competed with policy institutes from the Right.⁴⁷ Many CAP staff joined the Obama administration.

Political Practice

The capacity of social workers to reassert their role in social welfare policy depends on the willingness of individuals to consider public office as a setting for social work practice. Although many welfare pioneers began their careers advocating for social welfare policy and then assumed administrative positions managing social programs, others used elected office to advance social reform. The first woman elected to the House of Representatives was Jeannette Rankin, who won a seat in 1916 running as a Republican in Montana. As a social worker who had studied under Frances Perkins, Rankin voted for early social welfare legislation and against military expansion. More recently, social workers in **political practice** include five social workers elected to national office:

- Barbara Mikulski received her MSW degree in 1965 and then served on the Baltimore City Council and in the U.S. House of Representatives. In 1986, Mikulski became the first Democratic woman to be elected to the U.S. Senate in her own right. Through appointments to the powerful Appropriations and Labor and Human Resources



Barbara Mikulski

Source: Harry Hamburg /AP Images

Committees, Mikulski was a tireless advocate for programs in health and social services.⁴⁷ In 2015, she announced her retirement.

- Debbie Stabenow was awarded her MSW degree from Michigan State University, after which she was elected to the Ingham County Commission, the Michigan House, and the Michigan Senate. In 1996, Stabenow ran to represent the Eighth Congressional District of Michigan on a platform opposing the extremes of the Republican Congress elected in 1994. Having won election to the House of Representatives, in 1999 Stabenow challenged an incumbent to become the second social worker to be elected to the U.S. Senate.



Debbie Stabenow

Source: Manuel Balce Ceneta/AP Images

- Barbara Lee was elected in 1998 to fill the House seat of Ron Dellums, a social worker on whose staff she had served. Lee received her MSW from the University of California, Berkeley, and established a community mental health center while completing her graduate studies.



Barbara Lee

Source: J. Scott Applewhite /AP Images

Subsequently, she was elected to the California State Senate. Congresswoman Lee now serves on the House Committee on Banking and Financial Services and on the House Committee on International Relations, the Congressional Progressive Caucus, the Congressional Black Caucus, and the Congressional Women's Caucus.

- In 2000, Susan Davis, MSW University of North Carolina, defeated an incumbent to represent California's 49th Congressional District. Davis attained a



Susan Davis

Source: Scott J. Ferrell/ Congressional Quarterly/Alamy Stock Photo

coveted seat on the Armed Services Committee. Among her primary concerns are education and campaign finance reform.

- In 2012, Kyrsten Sinema who received her MSW from Arizona State University was elected to the House of Representatives where she acts as a supporter of the DREAM Act as well as same sex marriage. Endorsed by EMILY's List, Sinema was reelected in 2014.



Kyrsten Sinema

Source: dpa picture alliance/Alamy Stock Photo

An analysis of 250 social work elected officials—69 percent in local office, 29 percent in state office, and 2 percent in federal office—revealed that most found their social work education instrumental in their public service.⁴⁸

Defying predictions, Donald Trump was elected the 45th president of the United States in 2016. With the support of a Republican Congress, Trump's agenda promised at least one Supreme Court appointment, repeal of the Affordable Care Act, severely restricting immigration and deporting millions of illegal immigrants, and exacerbating economic inequality by reducing taxes. The effect of Trump's victory on social policy is unknown since he offered few details during the presidential campaign; however, he promised to support Social Security and Medicare. At the same time, a Republican Congress is likely to exact punishing cuts on discretionary social programs and attempt entitlement reform.

Conclusion

Perhaps the best indicator of social work's potential future influence on social policy appears at the local level. For example, social workers have lobbied successfully on behalf of nonprofit agencies facing threats to their tax-exempt status,⁴⁹ encouraged students to engage in election campaigns and to become more knowledgeable about politics,⁵⁰ and managed a campaign for the election of a state senator.⁵¹ In each of these instances,

social workers were gaining the kind of experience that is essential to political involvement at higher levels. More than 200 social workers had been elected to state, county, municipal, and judicial offices. In response to increasing political activity at the state level, social work professor emeritus Bob Schneider established Influencing Social Policy in order to empower human service professionals.

Social workers disinclined to engage in high-visibility activities such as campaigning for public office could make their imprint on politics through legislators' "constituent services." Using a term familiar to most social workers, politicians call constituent services "casework"—that is, "having your staff track down missing Social Security checks, inquire about sons and husbands in the armed services, help veterans get medical care, pursue applications for small-business loans."⁵² The importance of political "casework" has been noted by political scientists, who attribute up to 5 percent of the vote to such activities, a significant amount in close elections. David Himes of the National Republican Congressional Committee claimed that "our surveys have shown that constituency service, especially in the House, is more important than issues."⁵³

The cultivation of practice skills in the political arena at the local level offers, perhaps, the most promising way for social workers to regain influence in social welfare policy. On a volunteer basis, social workers would find few politicians who would turn down their professional assistance in the provision of constituent services. With experience, enterprising social workers might find that political practice could be remunerative, provided that they possessed skills needed by elected officials—such as conducting surveys, maintaining data banks of contributors, organizing public meetings, and keeping current on legislation important to constituents. From another perspective, however, the prospect of political practice should be taken seriously. If social workers are sincere about making essential resources available to their clients—a responsibility stated in the NASW Code of Ethics—then some form of political practice is a professional obligation. "To do less," noted Maryann Mahaffey, "to avoid the political action necessary to provide these resources, is to fail to live up to the profession's code of ethical practice."^{54, 55}

DISCUSSION QUESTIONS

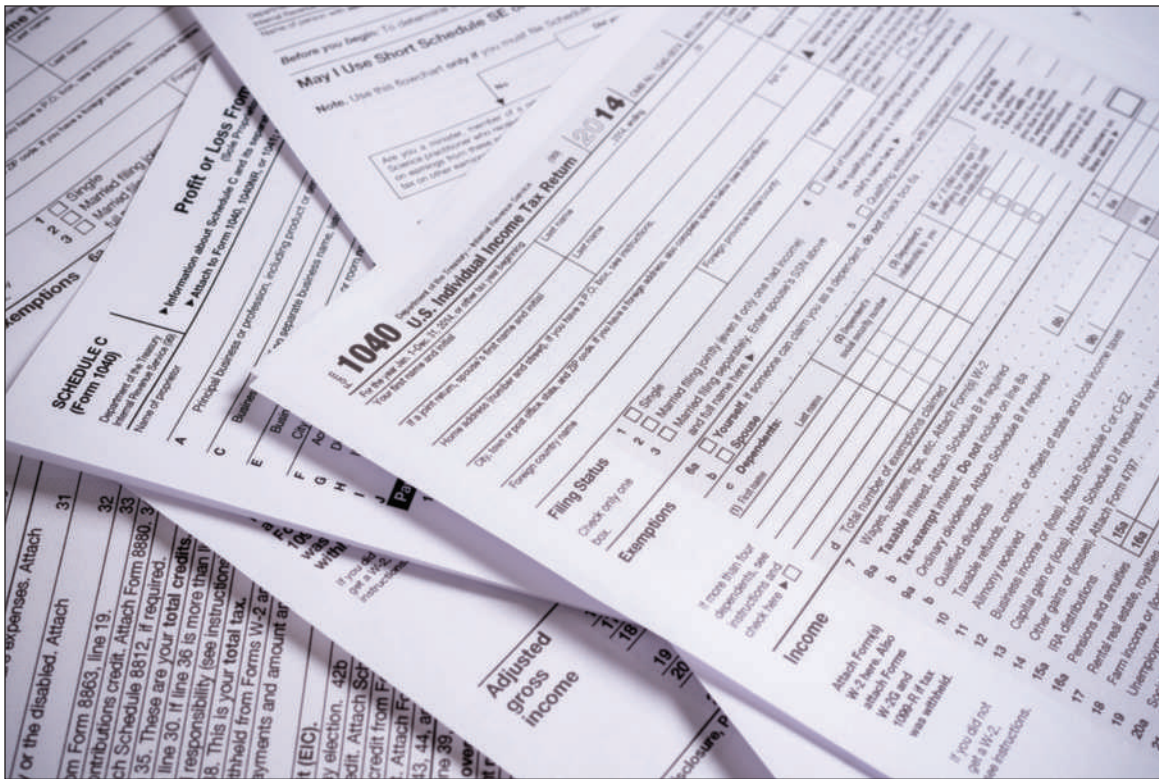
1. Clicking on the Congress.org website, select a social welfare policy and identify its primary components. Which elected members of Congress were cosponsors of the legislation? What committees were instrumental in its passage? Which special interests benefited from its passage?
2. Identify your representative and senators. E-mail them about an issue that is important to you. Do any of your members of Congress have any important health and human service committee assignments? Do your representatives have committee assignments that could make them influential on issues important to you? Do your representatives have position statements available to constituents about specific social programs?
3. Politicians elected to your state legislature have responsibilities similar to members of Congress. Identify your state representatives. Do they have assignments on health and human service committees? Do they have position statements they could send to you on health and human service issues?
4. Select a health and human service bill of interest to you and follow it in your state legislature. Which committees and interest groups supported or fought the proposed legislation? How was the bill changed to make it more acceptable to special interests? Have local interests, such as a major newspaper, endorsed or objected to the proposed legislation? Why?
5. Does your state chapter of the National Association of Social Workers make legislation a high priority for the professional community? What are the legislative goals of the state NASW chapter? How are those goals reflected in the resources allocated? How would you prioritize health and human services in your community?
6. If marginalization leaves many clients of social programs impotent in the public policy process, how could these people be made more influential? How could the local professional community assist in empowering beneficiaries of social programs? What could you do?
7. Identify social workers who serve in local elected offices. What led them to pursue elective office? What are their future political plans? Could you provide constituent services for these persons or help with their reelection?
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Tax Policy and Income Distribution



Source: Michael Flippo/Fotolia

Taxes are the price we pay for a civilized society,” pronounced Supreme Court Justice Oliver Wendell Holmes. Thus, tax policy, the use of legislation to define how revenues are generated in order to achieve public objectives, is fundamental to American society. At base, taxes are necessary for the **regalian functions** of government: printing currency, establishing a treasury, managing foreign relations, maintaining an army, and overseeing a postal service. The advent of democracy and industrialization added **social welfare functions**, the Social Security Act of 1935 establishing the American welfare state to protect citizens against risks associated with poverty, unemployment, illness, disability, and discrimination. Today, a volatile debate continues over the relationship between these functions and taxation, evident in “strict constructionist” interpretations of the U.S. Constitution, contending that the original text should be the basis of law and that social welfare activities are inherently unconstitutional, a position that has been professed by the Federalist Society, conservative Supreme Court justices, and the Tea Party.

Nations vary considerably in their tax rates, which reflects their commitments to social welfare. As Table 9.1 shows, the welfare states of northern Europe report much higher tax rates than less developed welfare states, such as the United States.

Taxes, by nature, are compulsory, while charitable contributions and employee benefits are voluntary. As noted in Chapter 18, the addition of these private sector efforts brings the American welfare state to parity with its European counterparts. Regardless, the low taxation rate of the United States reduces governmental social welfare initiatives.

History of U.S. Tax Policy

Taxation has been the flash point of major historical events, as evident in the Boston Tea Party when colonists objected to British taxation without American representation in Parliament. No sooner had the Constitution been ratified than the Whiskey Rebellion erupted during George Washington’s presidency. In 1791, Appalachian distillers revolted against a federal tax on their whiskey, which required federal enforcement. The new nation adopted a precept of American conservatism: minimal taxation restrained government while leaving

TABLE 9.1 Tax Revenue of OECD Countries as a Percent of GDP, 2009

Rank	Country	Total Tax Rate %
1	Denmark	48.1
2	Sweden	46.7
3	Italy	43.4
4	Belgium	43.2
5	Norway	42.9
6	Finland	42.6
7	France	42.4
8	Hungary	39.9
9	Netherlands	38.2
10	Luxembourg	37.6
11	Slovenia	37.4
12	Germany	37.3
13	Estonia	35.9
14	Czech Republic	34.7
15	United Kingdom	34.3
16	Iceland	33.9
17	Poland	31.8
18	New Zealand	31.5
19	Israel	31.4
20	Portugal	30.6
21	Spain	30.6
22	Greece	30.0
23	Switzerland	29.7
24	Slovak Republic	29.0
25	Ireland	27.8
26	Japan	26.9
27	Australia	25.9
28	Korea	25.5
29	United States	24.8
30	Turkey	24.6
31	Chile	18.4
32	Mexico	17.4

Source: Based on data from Table 2. Total tax revenue as percentage of GDP, 2004–2009 (cont.) (p. 82) from OECD (2011), *Revenue Statistics 2011*, OECD Publishing, Paris, http://dx.doi.org/10.1787/rev_stats-2011-en-fr accessed on November 26, 2015.

taxpayers with more disposable income. Speaking about the American colonies, Edmund Burke observed that taxes, by their very nature, tended to be imprudent: “To tax and to please, no more than to love and to be wise, is not given to men.”

Unprecedented dislocations, attendant with the Great Depression, challenged the preference for low taxes. The private sector proved unable to address the social and economic upheavals following the Wall Street crash of 1929, leaving government with the task of putting the nation on the track to recovery. With the passage of the Social Security Act of 1935, social program expenditures have grown, paid for by increasing taxes. The optimal welfare state, as liberals conceived it, would provide essential benefits as a right of citizenship. These benefits would be funded by progressive taxes—taxes that derived their revenues disproportionately from the wealthy. Implicit in this vision was the political calculus that was captured by Harry Hopkins, whose synopsis has become part of welfare folklore: “Tax, tax; spend, spend; elect, elect!” For half a century, this strategy produced solid electoral support for liberal social programs: The wealthy were taxed at higher rates, the revenues were diverted to social benefits for the middle and working classes through social programs, and social program beneficiaries expressed their gratitude by voting Democratic.

Ultimately, however, liberal Democratic hegemony in social policy foundered on the shoals of its own success. As working families rose into the middle class, they tended to individualize their achievements, discounting the role of social programs and, in the process, becoming more receptive to conservative proposals to reduce social programs and their tax burden. By the 1980s, this scenario led to the election of Ronald Reagan, who had a visceral dislike for federal social programs. The Reagan presidency was revolutionary in several respects, one being profound changes in tax policy, increasing economic inequality.

While the Clinton interregnum restored a measure of fairness in tax policy, the presidency of George W. Bush viewed tax cuts as the centerpiece of its domestic agenda, achieving one cut for each year of his first term. The intent of the Bush White House was summarized by Grover Norquist, who as president of Americans for Tax Reform proposed that the Bush White House implement annual tax cuts: “I don’t want to abolish government. I simply want to reduce it to the size where I can drag it into the bathroom and drown it in the

bathtub.”¹ Accordingly, conservatives have viewed tax cuts as essential to rolling back social programs, “starving the beast.” As Paul Krugman observed, “starving-the-beasters believe that budget deficits will lead to spending cuts that will eventually achieve their true aim: shrinking the government’s role back to what it was under Calvin Coolidge.”²

For intellectuals of the Right, tax reform is ultimately the vehicle for dismantling the U.S. welfare state. “Instead of sending taxes to Washington, straining them through bureaucracies and converting what remains into a muddle of services, subsidies, in-kind support, and cash hedged with restrictions and exceptions,” argued Charles Murray, “just collect the taxes, divide them up, and send the money back in cash grants to all American adults.” According to Murray, a \$10,000 grant to each non-incarcerated U.S. citizen, of which \$3,000 would be for health care, would cost \$355 billion more than current expenditures. Murray’s thought experiment, revenue neutral in 2011, would save a half trillion dollars by 2020.³

How this dispute between social entitlements and tax revenues will be worked out has implications far beyond beneficiaries of social welfare programs. If social program obligations exceed revenues, the only option for the federal government is to increase the national debt, a solution that had been pursued by both Democratic and Republican presidents. Massive tax cuts of the George W. Bush administration provoked dire warnings by liberals and conservatives alike about the fiscal health of the republic and contributed to the emergence of a new Tea Party movement. In 2010, the prospect of further indebtedness was effectively checked when candidates representing the Tea Party won Congressional elections on a platform that opposed to further deficit spending. In 2013, the federal government shutdown when conservative legislators refused to pass a budget, a problem that morphed into a threatened debt payment default that would have been unprecedented. As these events indicate, tax policy has become a volatile issue within social policy.

Federal Taxes

A creation of the legislative process, tax policy is most visible in the passage of major bills. Over time, these become the basis of the federal tax code, a notoriously confounding labyrinth of accounting

rules. Periodic attempts to reform tax policy emerge, such as the 1986 federal tax reform, simplify tax code and eliminate—at least temporarily—provisions for special interests. Thus, tax policy is dense as well as dynamic. Historically, three tax policies have been central to social policy: the income tax, the withholding tax, and the Earned Income Tax Credit (EITC).

- The federal income tax was instituted after approval of the Sixteenth Amendment to the Constitution in 1913. A **progressive tax**, in that the wealthy were taxed at a higher rate, the income tax was initially levied on less than 1 percent of the population and had a top rate of only 7 percent. In 2009, 7.5 percent of wealthy taxpayers were affected by the highest tax rate, 35 percent, while 25.0 percent of the lowest income households were exempt from paying any federal income tax.⁴ By 2015, the highest tax bracket had increased to 39.6 percent for individuals earning more than \$400,000 and couples earning more than \$450,000 annually.
- The Social Security withholding tax, the **payroll tax**, was established in 1935. For employed workers, the withholding tax was initially set at 2 percent of the first \$3,000 in wages, split equally between employers and workers. Since then Social Security withholding has increased to 12.4 percent of the first \$97,500 in wages in 2007, increasing to \$106,800 in 2011. The wage cap was established at the outset, under the presumption that Social Security was a public pension plan for workers who would not have recourse to retirement provisions available to the wealthy. The withholding tax, established under the **Federal Insurance Contribution Act (FICA)** is a regressive tax, in that lower-income workers pay the same rate as higher-wage employees. Most taxpayers now pay more in Social Security withholding than they do in income taxes.
- The **Earned Income Tax Credit (EITC)** was enacted in 1975 after the failure of a “negative income tax” plan advanced by the Nixon administration. A **refundable tax credit**, the EITC instructs the IRS to send a check to low-wage workers, especially those with children, whose earned income is below a certain level. In 2010, for example, a worker with two children could have received a maximum refund of

\$5,666.⁵ Since the creation of the EITC, other tax credits have been introduced: A child care tax credit allows low-wage workers to deduct the costs of day care, and 22 states and the District of Columbia have introduced tax credits for low-income workers, some of which are refundable.

The interaction of these basic tax policies is complex. For instance, although taxpayers who fall in the top quintile (top 20 percent) of income earners, as evident in Table 9.2, claim 54.6 percent of income, they also bear the weight of paying most of the income, payroll, corporate, and estate taxes. Almost 70 percent of the federal tax burden is shouldered by the top quintile of taxpayers. On the other hand, the effective tax rate of the top quintile—32.1 percent—is considerably less than its share of income. The lowest income earners often pay “negative income tax” because of rebates paid to them through EITC for low-wage families with children.

Taxes, Spending, and the Debt

Tax provisions fund social programs that exist within an economy shaped by economic policy; thus, economic policy, through tax policy, influences social programs. A classic example has been the conservative enthusiasm about “supply-side economics” since the 1980s. As advocated by Arthur Laffer, optimal economic policy would consist of minimal taxation so as not to impede capital formation and economic expansion. Given the relatively higher tax rates that preceded his presidency, Ronald Reagan endorsed tax reform that incorporated a one-third cut in the income tax, assuming that the cut would reinvigorate a sluggish economy.⁶ But although the tax cut of 1981 jolted the economy out of recession, it also cut off tax revenues to the Treasury, which then had to sell bonds to service the federal debt. Soon the federal government plunged further into debt, the depth of which was unprecedented for peacetime. By 1983, the annual deficit was \$207 billion, 6.3 percent of **gross national product (GNP)**, and growing. If they are not paid-off, annual budget deficits accumulate over time to comprise the debt, on which the federal government is obligated to pay interest. Debt service on government bonds grew commensurately, so that by the end of the 1980s, annual interest payments on the debt were \$150 billion, the second largest item in the federal budget.⁷

TABLE 9.2 Federal Tax Rates, 2012

	Individual Income Tax	Payroll Tax	Corporate Income Tax	Estate Tax	All Federal Taxes
Top 0.1 %	18.4	0.7	10.9	0.6	30.6
Top 1 %	18.1	1.7	7.9	0.4	28.1
Top Quintile	14.2	6.1	4.0	0.2	24.4
Fourth Quintile	7.2	10.4	0.7	*	18.3
Middle Quintile	3.3	10.0	0.7	*	14.0
Second Quintile	-2.7	9.4	0.6	*	7.3
Bottom Quintile	-6.0	7.1	0.4	*	1.6

*less than 0.05 percent

Distribution of Income and Federal Taxes, 2011

	Pre-Tax Income		Federal Tax Burden		Average Federal Tax
	Average \$	% Total	Average \$	% Total	%
Top 0.1 %	6,859,873	7.7	2,113,515	15.1	30.8
Top 1 %	1,530,773	16.8	422,727	25.6	27.6
Top Quintile	251,746	54.6	58,040	69.5	23.1
Fourth Quintile	79,524	19.9	13,168	18.2	16.6
Middle Quintile	44,639	13.5	5,592	9.3	12.5
Second Quintile	24,603	8.5	1,420	2.7	5.8
Bottom Quintile	9,187	3.8	77	0.2	0.8

Source: Table T12-0008, Tax Policy Center, Washington, DC: Urban Institute and the Brookings Institution, 2012. Used with permission.

Annual debt service overshadowed domestic policy discussions during the early 1990s. The congressional response was to impose a cap on domestic spending, an effort to stem the hemorrhaging of cash leaving the Treasury during a period when inflowing revenues had been stemmed by tax cuts. Liberal Democrats insisted that entitlement spending for social programs be exempt from the spending cap, with the result that federal budget decisions subsequently penalized discretionary programs—those with fixed budgets that are subject to the appropriation process annually—disproportionately. Because discretionary programs include research and development, student loans, public transportation, and the entire defense budget, the spending cap created intense pressure between social entitlements and discretionary programs. Thus, several years after a sharp cut in

federal taxes, the effects took the form of pressure to cut an array of discretionary programs.

Looming federal deficits cast a pall over the incoming Clinton administration. After campaigning on a platform that emphasized investments in human capital, former president Clinton was confronted with the massive deficits left over from the Reagan and Bush administrations. Alan Greenspan, chair of the Federal Reserve, convinced President Clinton to postpone public works and national health insurance, arguing that the economy in general, and financial markets in particular, would respond negatively to new social programs that carried high price tags, because such programs would either (1) worsen the federal debt or (2) require significant tax increases. Although Clinton balked at Greenspan's position, economic reality was making short work of the new president's campaign

rhetoric. Capitulating to Greenspan's insistence on deficit reduction, Clinton put his social investment plans on hold, a decision that contributed to an unprecedented economic expansion that promised to make possible the eventual elimination of the federal debt by generating a surplus projected at \$2 trillion over 10 years.

After the election of George W. Bush in 2000, and with Republicans in control of Congress, the White House was positioned to move aggressively on tax policy. During its first term, four tax cuts were passed, reducing federal revenues \$1.9 trillion over 10 years. Because the federal government had to honor its obligation to social entitlements while increasing spending for national security and mounting a war on terrorism, the federal government not only spent the surplus inherited from the Clinton presidency but also slid rapidly back into debt. As a result, the federal debt skyrocketed. With passage of the 2004 tax cut, the federal debt was projected to increase from \$4.3 trillion to \$19 trillion by 2016.⁸ Conservatives minimized the fiscal consequences of the federal deficit, noting that as a percentage of gross domestic product, the Bush deficit was only 1.8 percent in 2007, while the Reagan deficits were much higher, 6 percent.⁹ Adherents to supply-side economics in the Bush White House bet that economic growth would shrink the deficit—in the words of former vice president Cheney: “Reagan proved that deficits don’t matter.”

Liberals were less sanguine. “Bush’s policies may, in fact, best be explained by another, more radical agenda. Extensive tax cuts will require Congress to limit the growth of social programs and public investment and undermine other programs altogether,” argued Jeff Madrick, “Rising deficits will inevitably force Congress to starve those ‘wasteful’ social programs. The prospective high deficits may even make it imperative to privatize Social Security and Medicare eventually.”¹⁰ In 2008, the first of 77 million baby boomers began collecting Social Security, incurring future obligations that current social insurance programs will not be able to meet. The Bush tax cuts, scheduled to expire in 2012, became permanent for individuals earning less than \$400,000 and couples earning less than \$450,000; for those earning more, the marginal tax rate would be 39.6 percent. Deficits incurred during the Bush presidency plunged deeper with the advent of the Great Recession in late 2008. Reductions in government revenues accompanied by increased demand for social program benefits

increased the federal deficit to 9 percent of GDP, a level not seen since World War II. Subsequently, a bipartisan commission on deficit reduction, impaneled by President Obama, proposed solutions, such as eliminating the mortgage interest deduction on second homes, increasing the retirement age for Social Security, and freezing pay increases for federal employees and military personnel for three years.¹¹ Nominal income tax increases on the wealthy along with a gradual economic recovery and reduced Medicare expenditures due to the 2010 Affordable Care Act, reduced deficit spending; however, the larger question of how to address the long-term costs of retiring baby boomers while honoring discretionary commitments remained.

Tax Policy and Special Interests

Tax policy has always contained provisions that benefit specific interests. Bending the tax code in response to lobbying is a long-standing practice in the United States, though today it is most often associated with corporate influence. Actually, the exclusion of pension plans from taxation began in 1921, and these provisions have been updated to include provisions such as individual retirement accounts. Tax expenditures that benefit individuals have now grown to the point that they exceed allocations for many prominent social welfare programs. For 2010, for example, tax expenditures for pension contributions were projected to be \$117.7 billion, for health insurance \$131.0 billion, and for mortgage interest deductions \$88.5 billion. By comparison, in 2007 allocations for Supplemental Security Income were \$41.2 billion and Temporary Assistance for Needy Families (TANF) were \$16.5 billion.¹² Tax expenditures represented a “submerged” welfare state where most of the benefits accrue to the affluent.¹³

Realizing that the tax code can be manipulated to serve the interests of the affluent, many social justice advocates have targeted “corporate welfare,” that is, the special provisions that provide advantage to specific industries, for reform. The libertarian CATO Institute reported that the federal government spent \$92 billion, directly and indirectly, to subsidize private sector activities each year, including over 11,600 earmarks that members of Congress inserted in bills to aid organizations in their districts in 2008.¹⁴ Such pedestrian examples of corporate welfare were eclipsed by the creation

of the Troubled Asset Relief Program (TARP) designed to fund banks and other financial institutions to prevent a depression in 2008. Funded at \$700 billion, TARP generated controversy not only by the scale of assistance to investment firms, the leaders of which were largely responsible for the subprime housing bubble but also for the seven-figure bonuses paid them while receiving the federal bailout.¹⁵

Efforts to downsize corporate welfare have often been frustrated. As long as there is pork in politics, there is the opportunity to customize tax policy and budget bills to serve the concerns of individual legislators—who, after all, are often influenced by constituent requests. Thus, for example, tax policy is crafted both to meet the Appalachian Regional Commission's need for roads and to respond to Sonoma Valley vintners' desire to export their products to France. Tax law also has a significant influence, directly and indirectly, on the revenues of nonprofit organizations. By allowing taxpayers to deduct charitable contributions from taxable income, tax law directly encourages support of philanthropy. Lower tax rates work indirectly, at least in theory, by leaving taxpayers with more discretionary income and assets, which they may then donate to nonprofit causes. Despite changes in tax policy that prevent taxpayers who do not itemize tax deductions to deduct charitable contributions, the charitable impulse of Americans has continued. "Giving as a percent of income has remained remarkably constant in the face of increases in the cost of giving," concluded tax policy analysts.¹⁶

As noted in Chapter 6, increases in charitable giving due to tax reduction are less evident than conservatives might wish. Deep tax cuts introduced during the 1980s significantly reduced federal social welfare funding; and indeed, until the early 1990s, these were made up for by increases in philanthropic donations. During the 1990s, however, although both private giving and federal support of nonprofits increased significantly, these sources never compensated for the total revenue losses attributed to direct federal funding cuts. After the election of George W. Bush, conservatives proposed repeal of the estate tax, the consequence of which the Congressional Budget Office predicted would be between \$13 billion and \$25 billion in reduced charitable contributions because the wealthy would no longer count gifts against their tax obligations.¹⁷ Unlike earlier

economic downturns that prompted increases in charitable contributions, the Great Recession was so profound that philanthropic giving actually dropped 13 percent in 2011.¹⁸

Income Distribution

As shown in Table 9.1, the tax burden of the United States was lower than that in most industrialized nations, which contributes to the nation's skewed **income distribution** (see Table 9.3). A tenet of the welfare state has been the progressive taxation of income and its redistribution to the poor through social programs. By definition, a welfare state with a low tax rate is unable to generate revenues sufficient to level the differences between rich and poor; thus, the question of income distribution has become integral to the discussion of tax policy. Income distribution changes over time; most recently, wealthier families have benefited significantly.

Economic growth during the latter decades of the twentieth century exacerbated income inequality; while the lowest 40 percent treaded water with respect to family income, the top 20 percent fared significantly better. The fortunes of the wealthiest 20 percent of families were battered by the tax cuts during the presidency of George W. Bush, which favored the rich. By 2006, the top 10 percent of American families claimed almost half of all family income. By one calculation, the top 1 percent claimed 53 percent of the growth in income in 2004.¹⁹

The evidence suggests that top income earners are not "rentiers" deriving their incomes from past wealth but rather are "working rich," highly paid employees or new entrepreneurs who have not yet accumulated fortunes comparable to those accumulated during the Gilded Age. Such a pattern might not last very long. The possible repeal of the federal tax on large estates in coming years would certainly accelerate the path toward the reconstitution of the great wealth concentration that existed in the U.S. economy before the Great Depression.²⁰

While income for the top fifth has increased significantly during the past three decades, lower quintiles have lost share of income, especially the lowest. Not surprisingly, the "hollowing-out" of the middle class has become a hot political issue.

Income is only one component of economic justice; another measure of affluence is assets.

TABLE 9.3 Income Distribution, 1968–2013²¹

Quintile	Year								
	1973	1978	1983	1988	1993	1998	2003	2008	2013
Top	43.9	44.1	45.1	49.2	48.9	49.2	49.8	50.0	51.0
Fourth	24.5	24.7	24.6	24.2	23.5	23.2	23.4	23.3	23.0
Middle	17.0	16.8	16.5	16.0	15.1	15.0	14.8	14.7	14.4
Second	10.4	10.2	9.9	9.6	9.0	9.0	8.7	8.6	8.4
Bottom	4.2	4.2	4.0	3.8	3.6	3.6	3.4	3.4	3.2

Although considered in discussions of social policy less often than income, assets are valuable insofar as they are an indication of real wealth. Consisting of savings, real estate, stocks and bonds, and related property, assets can not only be liquidated during periods of adversity, thus offering the owner a buffer against poverty but can also appreciate in value, thus generating additional wealth. The distribution of assets is even more skewed than income distribution, with the highest quintile owning more than 80 percent. By contrast, the wealth of the lowest quintile is negative, indicative of debt. As has been the case with income, the distribution of assets has become more skewed during 1962 to 2004, the wealthiest quintile controlling more wealth, while the lowest reported negative assets. Consider the period following the Great Society efforts of the mid-1960s: Despite a major expansion of social programs for the poor, the lowest quintile still showed negative wealth, remaining mired in debt.

Wealth is also skewed in relation to race and ethnicity. The fact that assets are consistently negative for the lowest quintile reflects the difficulty of poorer families to buffer themselves from economic shocks. Thus, asset poverty, the wealth needed to survive for three months at the poverty level, exceeds income-based poverty. By 2000, the asset poverty rate was 25.5 percent, twice the conventional poverty rate of 12.7 percent.²² What meager assets the minority poor were able accumulate were wiped-out by the Great Recession, as noted in Table 9.4.

While the wealth of whites dropped significantly, that of minorities of color imploded, falling by more than half. The losses for minorities were “twice the size of the ratios that had prevailed between these three groups for the two decades prior to the Great Recession that ended in 2009,”

TABLE 9.4²³ Median Net Worth of Households, 2005 and 2009*

Race/ethnicity	2005	2009
Whites	\$134,992	\$113,149
Hispanics	18,359	6,325
Blacks	12,124	5,677

*2009 dollars.

noted researchers. The consequences of the tax cuts engineered during the second Bush presidency compounded by the Great Recession thus further widened the chasm between the rich and the poor.

State Tax Policy and the Poor

Federal taxes are important in social welfare policy because they subsidize the major social entitlements, but states also levy taxes in order to meet their legislative obligations. Historically, states have held major responsibility in social programs, areas such as mental health, child welfare, and corrections, so to a great degree the adequacy of a state’s social programs depends on its tax collections. Unlike federal taxation, which is uniform across the nation, state tax policy varies significantly. By way of illustration, consider the income tax: 43 states have income taxes, but seven do not (Florida, Nevada, South Dakota, Alaska, Texas, Washington, and Wyoming) and two states only tax investment income (Tennessee and New Hampshire).

State income taxes provide general revenues that can be used for a range of social programs,

TABLE 9.5 State Income Tax at Poverty Line of \$23,018 for Two-Parent Families of Four, 2011

Rank	State	Tax	Rank	State	Tax
1	Alabama	\$ 548	16	North Dakota	0
2	Illinois	509	16	Pennsylvania	0
3	Hawaii	331	16	South Carolina	0
4	Oregon	274	16	Utah	0
5	Georgia	273	16	Virginia	0
6	Iowa	251	27	North Carolina	(21)
7	Montana	240	28	Rhode Island	(182)
8	Ohio	162	29	Oklahoma	(242)
9	West Virginia	151	30	Wisconsin	(342)
10	Missouri	120	31	Nebraska	(485)
11	Arkansas	112	31	New Mexico	(485)
12	Indiana	108	33	Massachusetts	(530)
13	Mississippi	103	34	Kansas	(592)
14	Kentucky	94	35	Michigan	(664)
15	Louisiana	48	36	New Jersey	(717)
16	Arizona	0	37	Maryland	(943)
16	California	0	38	District of Columbia	(1400)
16	Colorado	0	39	Connecticut	(1454)
16	Delaware	0	40	Vermont	(1551)
16	Idaho	0	41	Minnesota	(1788)
16	Maine	0	42	New York	(1873)

Source: Adapted from Phil Oliff, Chris Mai, and Nicholas Johnson, *The Impact of State Income Taxes on Low-Income Families in 2011*, TABLE 2B: 2011 State Income Tax at Poverty Line, Two-Parent Family of Four (Washington, DC: Center on Budget and Policy Priorities, <http://www.cbpp.org/>, 2012), p. 14.

but this is not the only reason that state tax policy is important. State tax policy can establish an income floor for taxation or exempt low-income families from any tax liability altogether, thus allowing them to keep more of their income. Disparities among states are striking; in 2011, the state income threshold in Alabama was only \$9,800, for example, while that of California was \$46,900. Of states that levy an income tax, the more progressive jurisdictions actually provide a rebate, similar to the federal EITC. As Table 9.5 indicates, some states have been much more generous with respect to low-income families, while others have been downright punitive. To some extent, adverse state tax policy on the part of some states is offset by the EITC, which reimburses workers according to a standardized schedule nationwide.

The Efficiency of Tax Policy in Reducing Poverty

In the larger context of social policy, tax policy is one of several strategies that apportion societal resources. Within social welfare, more traditional benefits have consisted of social insurance such as Social Security, cash public assistance (means-tested cash benefits) such as TANF, in-kind public assistance (means-tested noncash benefits) such as Food Stamps, and the EITC (tax rebates). These different strategies vary in terms of their efficiency in poverty reduction over time. For elders, social insurance—compulsory contributions to social programs like Social Security and Medicare—makes the biggest dent in poverty, and means-tested in-kind benefits and tax policies are more important for children

TABLE 9.6 Impact of Safety Net on Poverty Reduction, 2006

Category	Individuals, Over 65 (in thousands)	Percent over 65	Children Under 18 (in thousands)	Percent under 18	Persons in Unmarried Households (in thousands)	Percent in Unmarried Households
Number of poor	36,035		73,727		40,749	
Number removed due to:						
Social insurance	13,042	77.7	1,301	8.9	1,398	8.7
Means-tested cash benefits	349	2.1	545	3.7	728	4.5
Means-tested in-kind	433	2.6	1,626	11.1	1,800	11.2
Federal taxes and refunds	—4	0	1,782	12.1	1,539	9.5
Total removed	13,820	82.3	5,254	35.8	5,465	33.9

Source: House Ways and Means Committee, Overview of Entitlement Programs (Washington, DC: U.S. GPO, 2008), adapted from Tables E-29, E-30, and E-31.

and families. Regardless, more than twice the number of elders is removed from poverty than are children and families by these different strategies (see Table 9.6).

Tax Expenditures as AntiPoverty Policy

The use of federal tax policy to alleviate poverty and the increase in states' use of tax policy to augment the income of poor families are relatively new features of U.S. social policy. In the past quarter century, a significant shift in social welfare policy has been witnessed: Gradually, direct welfare transfers are being augmented with indirect expenditures through tax credits. While the family welfare allocation was restricted to \$16.5 billion when Aid to Families with Dependent Children (AFDC) was replaced by TANF, the EITC expanded to over \$66.5 billion in 2015.²⁴ This created a major disconnect in poverty policy insofar as many of the families transitioning from welfare to work do not claim EITC benefits even though they would be eligible for them. One researcher found that while 61.6 percent of TANF/AFDC families had heard of the EITC, only 33.3 percent had received an EITC refund.²⁵ If welfare departments were truly concerned about poverty, as opposed to simply distributing public assistance benefits, they would make certain that all families with earned income claimed the EITC, but few welfare workers are familiar with

the program. As a result, the welfare poor who are entering the labor market underutilize the EITC.

Tax expenditures in the form of deductions for families' housing and health insurance costs have been enjoyed by the middle class for more than a half century, but it has not been until relatively recently that tax expenditures have been targeted for low-income families. The list of tax credits available to the poor has grown to include credits for earned income, child care, the welfare-to-work transition, care for the elderly and disabled, and adoption expenses. As the number of tax credits targeted to the poor has increased, tax credits have emerged as a contender to replace, at least partially, direct income transfers to aid the poor. As might be expected by a transition of such magnitude, the replacement of welfare transfers with tax credits raises several policy issues:

- Tax and revenue agencies replace welfare departments as the source of benefits, a role that many departments of social services are either unprepared for or may resist outright.
- Beneficiaries of tax credits must participate in the tax system in order to claim benefits, a status that is unfamiliar for many or avoided as in the case of undocumented workers.
- Low-income workers may fall prey to unscrupulous, commercial tax preparers, particularly those advancing a refund as a loan.

- Tax credit refunds are almost always paid after the fact, requiring the recipient's willingness to wait until late spring, unlike traditional welfare that arrives at monthly intervals during the year.
- As tax expenditures, tax credits are no less consequential for the federal and state treasuries than are traditional welfare transfers, both representing cash outflows.

An important advantage of tax credits is that their allocations are not fixed; like open-ended entitlements, the amount refunded is determined by the volume of valid claims, not a predetermined budget figure.

Tax credits offer new opportunities in areas historically understood as “welfare.” In order to accelerate the upward mobility of the poor, for example, tax credits could be connected directly to asset-accrual strategies, such as Individual Development Accounts, by encouraging low-income taxpayers to split their refund, directing a portion to a savings instrument. The tax preparation necessary for people to access tax credits could be one of several basic services—checking, savings, financial planning—offered by community financial services that could replace current welfare departments and serve as alternatives to marginal financial outfits that exploit the poor. Integrated with other capital formation strategies such as electronic benefit transfer, deposits by commercial banks to meet their Community Reinvestment Act obligations, and deposits by government and nonprofit agencies, tax credits could be part of a broad community development initiative that would finance projects in poor neighborhoods, thereby providing jobs to residents. In this respect, tax credits may not only begin to replace traditional welfare transfers but in so doing may well introduce a new era of basic supports for poor families.²⁶

Other tax credits have been proposed to augment poverty policy. Martha Ozawa and Baeg-Eui Hong suggested using the EITC to establish a \$1,000 income floor for poor children and adjusting the benefit according to family size. “The modified EITC and children’s allowances combined would improve the income status of all EITC-recipient children by 23.6 percent, with black and Hispanic children benefiting more than white children,” Ozawa and Hong concluded.²⁷ Robert Cherry and Max Sawicky proposed a universal unified child credit that would augment the EITC and child care

tax credit with an additional child credit.²⁸ Thus, the integration of refundable tax credits and calibrating them according to family size could reduce poverty significantly.

Increasing refundable tax credits and adjusting them to family size would not only address poverty but also accelerate upward mobility for low-income households. Unfortunately, the United States lags behind other industrialized countries with respect to economic mobility; too often children born into poor households grow up to be poor, like their parents. This is not to say that none escape poverty; indeed, some move quickly upward; the problem is that upward mobility in the United States is “sticky”—the lowest income households tend to have children who are also poor. This stickiness affects minority children more than others. A pre-recession report on mobility sponsored by prominent policy institutes across the ideological spectrum concluded “it is fairly hard for children born in the bottom fifth to escape from the bottom: 42 percent remain there and another 42 percent end up in either the low-middle or middle fifth. Only 17 percent of those born to parents in the bottom quintile climb to one of the top two income groups.”²⁹ Many minority children actually experience downward mobility. Of black children with middle-class parents, 45 percent fell to the bottom of the income distribution, compared to 16 percent of white children. Poor black children fared the worst: 54 percent of children in families in the bottom quintile remained there, compared to 31 percent of white children.³⁰ The recession further slowed the upward mobility of minorities. A post-recession analysis of economic mobility concluded that one-fourth of middle-class children had fallen out of the middle class as adults, a prospect that affected 38 percent of African American men.³¹

The Anti-Tax Movement

Proponents of publicly funded social programs assume that tax-generated revenues are prudent investments toward the public good. Within the larger policy context, there are differing views on this assumption, however. It is worth acknowledging that some of the most egregious violations of personal decency, to say nothing of civil rights, have occurred under the auspices of public programs—such as the sterilization of “feebleminded” people during the eugenics movement, the Tuskegee

“experiment” on syphilitic African American men, and the warehousing of chronically mentally ill patients as well as of prison inmates. To the extent that government enriches the powerful and mistreats citizens, a logical reform strategy is to defund the state by cutting off its tax revenues. By way of illustration, Michael Tanner of the CATO Institute calculated that \$3.5 trillion has been spent on poverty programs since the Great Society period, yet with little success. “We are not going to solve our welfare problems by throwing more money at them,” he concludes. “It is time to recognize that welfare cannot be reformed. It should be ended.”³²

The obvious question raised by conservatives is, why tax at all? Beyond regal functions of the state, conservatives contend that citizens should be allowed to retain earned income and use it as they see fit. The liberal rebuttal to this suggestion has been that unregulated capitalism inevitably skews the distribution of resources and opportunities, leaving subgroups vulnerable to insecurity with respect to income, employment, and health. The result, liberals have contended, is that specific populations suffer disproportionate and protracted poverty, thus providing the rationale for social programs. For more than half a century, they note, social insurance and public assistance programs have buffered low-income families from poverty.

Conservatives have long objected to progressive taxes as a means for funding social programs.³³ At best, the Right contends, government consumes

tax revenues that could be otherwise used for personal purposes; at worst, social programs inflict not only acute damage on individuals but also long-term harm on society. The inverse relationship between citizen autonomy and government social programs is at the heart of the conservative social policy strategy. By favoring autonomy, tax cuts provide individuals with discretionary income with which they can choose the services they desire. This is shrewd politics, as Jonathan Rauch noted, “by repudiating the Washington-knows-best legacy of the New Deal, Republicans will empower the people, and the people will empower Republicans.” And the consequences for welfare liberalism are equally profound:

If the Democrats dig in their heels and fall back on stale rants against greed, inequality, and privatization, so much the better. The voters will know whom to thank for empowering choices that Republicans intend to give them. As for which is the “party of nostalgia,” the voters will also remember who defended, until the last dog died, single-payer Medicare, one-size-fits-all Social Security, schools without accountability, bureaucratic government monopolies, static economics, and Mutually Assured Destruction.³⁴

The success of the anti-tax movement seems evident with the relatively wide support that the Bush tax cuts have received, even if they benefit the wealthy disproportionately. In this respect, voter support of tax cuts can be interpreted as a referendum on the welfare state; given the choice, rather than divert income to public programs through taxes, many voters prefer to keep their income for themselves.



Despite support for publicly funded social programs, there are some who believe that Americans should not pay any taxes.

Source: Brad Coville/The Wilson Times/AP Images

The Debate over Economic Inequality

Stagnant wages, growing federal debt, and the emergence of insurgent groups, such as the Tea Party and Occupy Wall Street, fueled a debate over economic inequality that raged as the recession dragged on. In February 2009, President Obama appointed a bipartisan deficit reduction committee to propose reductions in federal expenditures; however,

the commission proposed reductions in social entitlements as well as popular tax expenditures, and its recommendations failed to attract the interest of Congressional leaders.³⁵ If federal expenditures could not be trimmed significantly, the expiration of the Bush tax cuts, scheduled for the end of 2011 promised to increase revenues, but President Obama could be criticized for overseeing a major increase in federal taxes.³⁶ Momentarily defusing the issue, Obama received Congressional support for continuing the Bush tax cuts but cutting the payroll tax while continuing unemployment benefits.³⁷ Procrastination on tax policy not only served to invigorate the Tea Party, which sought major reductions in federal spending in order to reduce the debt but also a nascent Occupy Wall Street movement determined to tax the wealthy at higher rates.

Public opinion found Americans divided in their understanding of how to proceed: Surveys show that while most respondents object to increasing economic inequality, they presume that such differences are inevitable given the nation's economic system.³⁸ Providing respondents with more data changed their perceptions, however. For example, of respondents to Pew Charitable Foundation surveys, 57 percent thought the wealthy were not paying enough in taxes, and 86 percent objected to bonuses paid to executives of investment firms.³⁹ In attempting to reconcile these differences, pundit Charles Lane suggested that Americans believed in the work ethic and an opportunity society but rejected the means by which the wealthy accrued riches: "What annoys Americans is not that some

of us are rich—it's that some of us get rich just through connections."⁴⁰ The possibility of such a consensus on tax policy dissolves once political party affiliation is factored into the equation. While 39 percent of Republicans and 36 percent of Democrats thought they paid more than their fair share of taxes in 2012, only 37 percent of Republicans thought the wealthy should pay more, compared to 70 percent of Democrats.⁴¹

Although public opinion varies considerably on tax policy, data show increasing economic inequality. Although wages increased only 26 percent from 1970 to 2005, median executive compensation increased 430 percent, far above corporate profits, 250 percent.⁴² If executives gained most in recent decades, professionals were not far behind, as shown in Table 9.7. By 2010, the number of American millionaires had more than fivefold since 1972, but their tax burden had almost been halved, as shown in Table 9.8. Logically, long-term income inequity affects generations; in the absence of opportunity structures that assure upward mobility, economic inequality persists. Using data from Canadian economist Miles Corak, Alan Krueger, Obama's Chairman of the Council of Economic Advisors, presented the "Great Gatsby Curve"; when generational mobility is plotted in relation to economic inequality, the United States more resembled developing nations, such as Singapore and China, than those of Europe.⁴³

Interpretation of data about economic inequality conforms to conventional ideology, with Democrats favoring leveling the economic playing field and Republicans finding justifications for economic

Spotlight 9.1

ECONOMIC INEQUALITY CAN BE REDUCED THROUGH TAX POLICY

There are many views on how to reduce economic inequality in the United States. Like Senator Elizabeth Warren and Professor Thomas Piketty, many believe that tax policy can be used to reduce economic inequality.



Watch this video of a discussion between Senator Elizabeth Warren and Professor Thomas Piketty to understand how progressive taxation can reduce economic inequality.

<https://www.youtube.com/watch?v=P4bb2AA8K6A>

TABLE 9.7 Professional Paydays, in 2010 Dollars

	1985	2010	% Increase
All Employed Americans	\$39,044	\$41,919	7
Radiologists	334,400	448,900	34
Urologists	226,200	380,400	43
Harvard University Professors	132,000	193,800	47
University of Chicago Professors	112,100	190,400	70
Attorneys at Wachtell Lipton	1,669,000	4,345,000	160
Attorneys at Kirkland & Ellis	850,000	3,075,000	262
CEO at Union Pacific	3,825,000	23,060,000	502
CEO at Occidental Petroleum	2,440,000	20,130,000	825

Source: We're More Unequal Than You Think, New York Review of Books, February 23, 2012, p. 36. Used with permission from Andrew Hacker.

TABLE 9.8 Millionaires and Their Taxes, 2010 Dollars

	1972	1985	2010
\$1 Million+ returns	22,887	58,603	236,893
Per 100,000 returns	30	58	162
Share of all income	1.3%	3.0%	9.5%
Average tax paid	47%	37%	25%
Average after-tax income	\$1,159,595	\$1,465,947	\$2,319,236

Source: We're More Unequal Than You Think, New York Review of Books, February 23, 2012, p. 34. Used with permission from Andrew Hacker.

disparities. Liberals, such as Paul Krugman, contend that the rich should pay more, as they do in other industrialized nations, in order to upgrade America's degraded physical and social infrastructure, in the process generating jobs that are badly needed during recession.⁴⁴ Indeed, billionaire Warren Buffett has complained that his tax rate of 17.4 percent is about half that of other staff in his office, including his secretary.⁴⁵ Conservatives have argued that there is much more to economic inequality, and thus poverty, than measures related to income. James Q. Wilson, for example, contended that focusing on the rich was the wrong end of the affluence continuum; the poor should be examined for self-defeating behaviors that consigned them to penury. Since very few adults who finish high school, work full-time, and marry before having children are poor, the key to rectifying economic inequality is not to punish the affluent by taxing them more but encouraging the poor to be more responsible: "the problem facing the poor is not too little money, but too few skills and opportunities to advance themselves."⁴⁶

Ideological polemics subside during periods of economic expansion, which permit taxing the wealthy moderately and diverting revenue to incremental augmentation of social programs; however, during a recession, the situation resembles a zero-sum game where the benefit to one group subtracts from the wellbeing of another. Slow recovery from the Great Recession has left many Americans confronted with adversity: static wages, unemployment, or foreclosure. "This is truly a lost decade," observed Harvard economist Lawrence Katz. "We think of America as a place where every generation is doing better, but we're looking at a period when the median family is in worse shape than it was in the late 1990s."⁴⁷ Conventional welfare philosophy suggests expansion of government social benefits under these circumstances, but Congressional conservatives emboldened by members identified with the Tea Party have negated that option. Precluding the infusion of federal support for social welfare leaves the nation contending with a "politics of austerity." According to Thomas Edsall, the current stalemate pits taxpayers, the most influential of whom are wealthy Republicans, against tax consumers, mostly lower-income minorities who tend to be Democrats. Instead of a virtuous circle through which government funds spark economic activity and minimize economic dislocation, the result is a vicious circle where federal assistance

is choked-off, the needy are left to fend for themselves, and antagonism increases across class, racial, and ethnic lines.⁴⁸ How to employ tax policy in order to create a virtuous circle under conditions of significant federal debt and during a protracted recession remains a signal challenge to contemporary social policy.

Economic inequality became headline news with the publication of Thomas Piketty's *Capital in the Twenty-First Century*, which depicted social equality among industrialized nations as a U-curve. During the decades following the Great Depression and World War II, welfare states expanded in North America and Europe; however, after the recession of the 1970s, nations reduced social expenditures, the result being increasing economic inequality. Piketty indicated that, short of a reversal of government policy, inequality comparable to the Gilded Age of the 1920s was likely. Due to capital flight, Piketty thought that only a tax on global wealth was the only way to regain social equality through public policy.⁴⁹ Subsequently, Anthony Atkinson assessed the problem differently, proposing a series of actions that nations could take to reduce economic inequality, including making taxes more progressive, assuring the long-term unemployed a government job, and replacing means-tested programs with social insurance.⁵⁰ The American welfare state is likely to diverge from such prescriptions, however. Economic solutions require political support to pass public policies, and a waning liberalism provides an unlikely platform for reversing economic inequality. Limits on spending—total government expenditures at 35 percent of GDP and federal social spending at 22 percent of GDP—are probably at maximum. This, compounded by social entitlements consuming 60 percent of the federal budget, indicates that restructuring the American welfare state is in order if taxes are to be increased.⁵¹

Conclusion

Tax policy, often undervalued in discussions of social welfare, serves a vital function because it provides the revenues through which public programs operate. Increasing fluency in tax policy has significant benefits for advocates of social justice. At the national level, for example, introducing progressive features to the Social Security withholding tax, adjusting the tax rate for income, and lifting

the cap on taxable income, would generate significant new revenues that could make minimal Social Security benefits more adequate.

Leveraging tax policy to advance social justice requires sophistication in social policy, however. Historically, two streams of poverty policy have evolved in the United States. The first—public welfare—was legislated through the Social Security Act of 1935 and consists of TANF, SSI, Medicaid, Food Stamps, and the like. Accordingly, the federal Department of Health and Human Services (HHS) along with parallel agencies at the state level manages much of social welfare. The second stream—tax credits—was enacted through the 1975 Earned Income Tax Credit and consists of an array of tax credits for individuals as well as businesses. The Treasury Department and the Federal Reserve System manage the tax credit stream, quite independent of HHS. By understanding poverty policy exclusively through the public welfare paradigm, human service professionals omit from the realm of possibility a set of policy options that have enormous promise. Thus, if social welfare professionals are to enhance their role in the domestic policy debate, they will have to master the financial, procedural, and accounting nuances of tax policy. These are daunting fields, to be sure, but the potential payoff makes the effort worthwhile.

DISCUSSION QUESTIONS

1. Should the federal government expand its spending on social programs despite the large and ongoing federal debt? What are the effects of increased spending, and what are the effects of stable or even decreasing spending, on social programs?
2. What are the positive and negative effects of increasing taxes on corporations and the rich? How would it affect the poor in both the short term and the long term?
3. How does your state's income tax affect the poor? To what extent can a state refundable tax credit address economic inequality?
4. Some welfare advocates concerned with income inequality argue that the function of the welfare state is to equalize incomes and assets through social welfare programs. Others believe it is unrealistic to expect that welfare state programs can do more than alleviate human suffering by providing resources to those in need. Should the goal of social welfare programs be to reduce income and asset inequality, or should that function be relegated to tax and labor policy?

5. Has the social work profession been successful in lobbying efforts and in promoting a more just society? If not, why? What strategies should social workers employ to move society toward more equitable income and asset redistribution?

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Social Insurance Programs



Source: Mark Richard/PhotoEdit, Inc.

This chapter explores the major forms of social insurance in the United States: Old-Age, Survivors, and Disability Insurance (OASDI); Unemployment Insurance (UI); and Workers' Compensation. In addition, this chapter explores some of the major issues and problems surrounding social insurance programs.

Definition of Social Insurance

Social insurance is the cornerstone of U.S. social welfare policy. Specifically, it is a system whereby people are compelled—through payroll or other taxes—to insure themselves against the possibility of their own indigence, such as might result from the economic vicissitudes of retirement, the loss of a job, the death of the family breadwinner, or physical disability. Based in part on the same principles used in private insurance, social insurance sets aside a sum of money that is held in trust by the government and earmarked for use in the event of workers' retirement, death, disability, or unemployment. The major goal of social insurance is to help maintain income by replacing a portion of lost earnings. It is

a pay-as-you-go system whereby the workers and employers of today pay for those who have retired, are ill, or who have lost their jobs. Although originally designed to replicate a private insurance fund, the original Social Security program has been broadened to include a variety of programs that provide a minimal level of replacement income. Because the benefits paid to some retired workers exceed their contributions to the system, Social Security has taken on some of the characteristics of an income redistribution and/or public assistance program.

Unlike Social Security, public assistance programs are subject to **means tests** and are based entirely on need. The rationale for public assistance is grounded in the concept of *safety nets* designed to ensure that citizens receive basic services and do not fall below a given poverty level. Social insurance, on the other hand, requires beneficiaries to make contributions to the system *before* they can claim benefits. Because social insurance affects a larger number of people and benefits are generally higher, expenditures for social insurance are far greater than for public assistance. Social insurance is also universal; that is, people receive benefits as legal entitlements regardless of their personal wealth. Table 10.1 shows

TABLE 10.1 Past and Current Federal Spending for Major Social Insurance Programs, Selected Years (expenditures in billions)

Programs	2002	2004	2006	2008	2010	2014
OASDI	\$452	\$494	\$539	\$598	\$670	\$851
Medicare	254	287	319	366	421	511
Unemployment Insurance	51	46	43	45	49	44
Total	757	827	901	1,009	1,140	1,406

Source: Based on Committee on Ways and Means U.S. House of Representatives, *2014 Green Book* (Washington, DC: U.S. Government Office, 2014). Online at <http://greenbook.waysandmeans.house.gov/2014-green-book>.

Projected Federal Spending for Social Security, Medicare and Medicaid, Selected Years (expenditures in billions)

Programs	2012	2014	2018	2020	2022
Social Security	\$770	\$857	\$1,063	\$1,197	\$1,345
Medicare	560	629	769	890	1,041
Medicaid	262	330	456	522	605
Total	1,592	1,816	2,288	2,609	2,991

Source: Adapted from Congressional Budget Office (CBO), "Budget and Economic Outlook Fiscal Years 2012 to 2022," January 2012. Retrieved April 2012 from HeinOnline at <http://heinonline.org/>.

federal spending for major social insurance programs. Because social insurance benefits are linked to occupationally defined productive work, most programs are not stigmatized compared to public assistance programs. In part, this stigma occurs because public assistance programs are financed from general tax revenues and are not occupationally linked or based on a previous work record, and the eligibility for these programs is determined by means tests.

Although conservatives complain about the costs of public assistance programs, social insurance programs consume many times more resources as public welfare. In 2014, the expenditure for OASDI alone comprised 24 percent of the federal budget, while safety net programs consumed only 11 percent. The average OASDI benefit in 2015 was \$1,127 a month, while the median monthly Temporary Assistance for Needy Families (TANF) payment for a family of three with no income was around \$429.¹

The Background of Social Insurance

The first old-age insurance program was introduced in 1889 by Chancellor Otto von Bismarck to curb the growing socialist trend in Germany. By the onset of World War I, nearly all European nations had old-age assistance programs of one sort or another. In 1920, the U.S. government began its own Federal Employees Retirement program. By 1931, 17 states had enacted old-age assistance programs, although these were often harsh with stringent eligibility requirements.²

Spurred on by the Great Depression of the 1930s, President Franklin Roosevelt championed a government assistance program that would cover both unemployed and retired workers.³ The result was the Social Security Act of 1935 and the establishment of the modern American welfare state.

Relatively modest compared to its present scope, the original Social Security Act of 1935 provided assistance to the elderly poor, dependent children, the blind, and some disabled children. Title VII established a Social Security Board, whose job was to monitor the fund. Title XI gave the Congress the right to alter any part of the act.⁴

The current Social Security Act provides for (1) OASDI; (2) UI programs under joint federal and state partnership; (3) federal assistance to aged,

blind, and disabled under the Supplemental Security Income (SSI) program; (4) public assistance to families with dependent children (now TANF); (5) federal health insurance for the aged (Medicare); and (6) federal and state health assistance for the poor (Medicaid). Although all these programs are under the Social Security Act of 1935, not all are true social insurance programs (e.g., Medicaid, TANF, and SSI).

The Social Security Act of 1935 has been repeatedly modified, almost always in the direction of increased coverage. The original Act provided retirement and survivor benefits to only about 40 percent of workers, farm and domestic workers; the self-employed, and state and local government employees were excluded. In 1950, farmers and the self-employed were added, thereby covering more than 90 percent of the labor force. Congress made survivors and dependents of insured workers eligible for benefits in 1939, and in 1956, disability insurance was added. In 1965, Medicare—a health insurance plan for the aged—became law. In later years, workers were allowed to retire at age 62, provided they agreed to accept only 80 percent of their benefits. In 1977, an automatic cost-of-living index was attached to Social Security payments.

By 1996, more than 43 million Americans and more than one-quarter of all U.S. households depended on a Social Security check.⁵ As Figure 10.1 illustrates, only 64 percent of Social Security beneficiaries were retired workers in December 2010.⁶

The Financial Organization of Social Insurance

Social insurance is relatively straightforward. Covered workers pay taxes to Social Security, and when they retire or become disabled, they (or their family) collect monthly benefits. General revenue dollars are used for SSI, which despite its name, is actually a means-tested public assistance program.

Social Security and Medicare taxes are divided among several trust funds, including (1) the Old-Age and Survivors Insurance (OASI) trust fund (pays for retirement and survivor benefits) and (2) the federal Disability Insurance (DI) fund (provides benefits to the disabled and their families). There are also two Medicare trust funds: (1) the federal Hospital Insurance (HI) trust fund pays for services covered under the hospital insurance (Part A) provisions of Medicare and (2) the federal

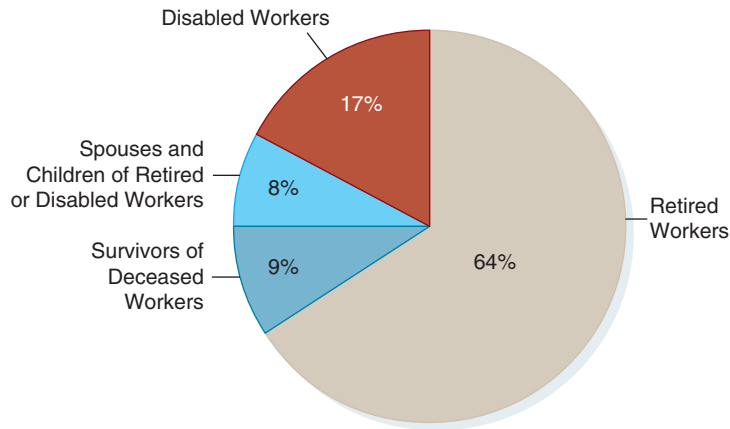


FIGURE 10.1 Number of Social Security Beneficiaries by Type, June 2012 (total 56.2 million)

Source: Social Security Administration, Office of Research, Evaluation, and Statistics. "Fast Facts & Figures about Social Security, 2015," SSA Publication No. 13-11785, Washington, DC.

Supplementary Medical Insurance (SMI) trust fund pays for services covered under the medical insurance (Part B) provisions of Medicare.

The trust funds are governed by a board of trustees made up of the Secretary of the Treasury, the Secretary of Labor, the Secretary of Health and Human Services, the Commissioner of Social Security, and two public trustees who serve four-year terms. The board of trustees reports annually to Congress on the condition of the funds and on estimated future operations.

Before 1983, the system operated on a pay-as-you-go system, with taxes flowing into one end of the pipeline and flowing out the other end. However, since 1983, Social Security has operated under a partial reserve system in which inflows exceed outflows, thereby building up a reserve to help pay benefits to an increasingly large number of retiring baby boomers.

Surplus income (i.e., income not needed to pay benefits) is invested in U.S. government bonds that earn the prevailing rate of interest and whose principal and interest are guaranteed by the federal government. Social Security trust funds are invested in sound Treasury securities similar to those held by investors across the globe. They are also considered one of the safest investments in the world. The amount of earned interest can be substantial. In 2013, the Social Security trust funds earned \$103 billion in interest.⁷

Key Social Insurance Programs

OASDI

OASDI is a combination of OASI and DI. Commonly referred to as Social Security, it is the largest social program and covers approximately nine out of ten workers. OASDI is a federal program administered by the Social Security Administration, which in 1994 became an independent agency headed by a commissioner and a board appointed by the U.S. president for a six-year term.

Social Security is a stellar example of a program that has worked.

- In 2014, Social Security served more than 60 million beneficiaries and paid out \$851 billion in benefits.⁸
- Social Security provided at least half of the income for nearly two-thirds of the aged.⁹
- 85 percent of women were fully insured, up from just 63 percent in 1970.¹⁰ They accounted for 56 percent of adult Social Security beneficiaries.¹¹
- Before the Social Security **cost-of-living adjustments (COLAs)** took effect in 1969, the poverty rate for the elderly was double that of the general population, 25 percent versus 12 percent.¹² According to the Social Security Administration, the poverty rate for the elderly would have been 52 percent higher without

Social Security.¹³ Social Security has a strong redistributive effect, transferring resources from those with high lifetime earnings to those with lower lifetime earnings.¹⁴

Unlike prefunded private annuity plans, where a worker draws off money already invested, Social Security benefits are paid by today's workers rather than by the retirees. Benefits under OASDI are entitlement based; that is, they are based on the beneficiaries' earnings rather than the amount of the revenue in the trust fund. They are also not means-tested. If benefits in a given period exceed revenues, the difference is made up from the reserves in the trust funds. OASDI operates in the following manner:

- On an employee's first \$118,500 in earnings in 2015 (the maximum tax payable by each employee is \$7347), the employee and the employer each pays an OASDI tax equal to 6.2 percent and a Medicare HI tax equal to 1.45 percent of earnings. There is no upper income limit on the Medicare tax. Employees therefore pay a total of 7.65 percent.¹⁵ Self-employed persons pay 15.3 percent (12.4 percent for OASDI and 2.9 percent for Medicare). (Half of that tax is deductible for income tax purposes.) In general, increases in the wage base are automatic and are based on the increase in average wages in the economy each year.
- Based on their age at retirement and the amount earned during their working years, workers receive a monthly benefit payment. The current full retirement age is 66 (increasing to 67 for those born after 1959), and how close a worker is to this age is significant in calculating their monthly benefits. Retired workers aged 62 receive a reduction of 5/9 of 1 percent for each month of entitlement before age 66 (depending on their year of birth), with a maximum reduction of 20 percent.¹⁶ Benefits are modest; the average retired worker received \$1,336 in 2014 if they were a current Social Security recipient, and \$1,193 if they were a new recipient.¹⁷ The maximum benefit for a retired worker in 2015 at age 66 was \$2,663 a month—if they waited to retire at age 70, it would jump to \$3,501.¹⁸
- Under the OASI program, a monthly payment is made to an unmarried child or eligible dependent grandchild of a retired worker or a deceased worker who was fully insured at the time of death if the child or grandchild is (1) under age 18, (2) a full-time elementary or secondary school student under age 19, or (3) a dependent or disabled person aged 18 or over whose disability began before age 22. A grandchild is eligible only if the child was adopted by the insured grandparent.¹⁹
- A lump-sum benefit of \$255 is payable to a spouse who was living with an insured worker at the time of his or her death, or minor children who meet certain criteria.²⁰
- Under the DI program, monthly cash benefits are paid to disabled workers under the age of 65 and to their dependents. Monthly cash benefits are computed similar to the OASI program in terms of past earnings; however, the age at which the individual becomes disabled is also taken into account.²¹ (Medicare benefits are treated in depth in Chapter 12.)
- In 2015, retired worker beneficiaries were required to have earned at least 40 credits (approximately 10 years) before they were eligible to draw benefits.²² For workers applying for DI, the amount of required credits depended on the age at which the recipient became disabled.²³

Social Security, especially OASDI, has been a heated topic for much of its history. Political conservatives and free market economists argue that Social Security socializes a portion of the national income, discourages savings, and causes retired people to become dependent on a supposedly fragile governmental system. The Social Security system, on the other hand, is extremely popular



Social Security is the key social insurance program in the United States.

Source: Bradley C. Bower/AP Images

with the elderly who rely on it for much of their income, and with their grown children, for whom it helps provide peace of mind.

Unemployment Compensation

The first line of defense for workers fired or laid off from their jobs is unemployment insurance (UI). Part of the Social Security Act of 1935, UI is a federal/state program whose objectives are (1) to provide temporary and partial wage replacement to involuntarily and recently unemployed workers and (2) to help stimulate the economy during recessions by providing the unemployed with the necessary cash to purchase goods and services. Although the U.S. Department of Labor oversees the general program, each state administers its own UI program.²⁴ The current guidelines of the UI program require employers to contribute to a trust fund that is activated if an employee loses his or her job. The average weekly number of workers claiming UI in 2014 was roughly 2.2 million. In 2015, the maximum federal cap on unemployment compensation was 26 weeks from the regular state-funded unemployment compensation program; however, 8 states (North Carolina, 12 weeks; Florida and Georgia, 14 weeks; Kansas, 16 weeks; Michigan, Missouri, and South Carolina, 20 weeks; and Arkansas, 25 weeks) provided fewer weeks and two (Massachusetts, 30 weeks; and Montana, 28 weeks) provided more. No additional weeks of federal benefits are available since the temporary Emergency Unemployment Compensation (EUC) program expired in late 2013.²⁵

The UI system consists of two basic parts: (1) regular state-funded benefits, generally provided for a maximum of 26 weeks, and (2) a federal/state Extended Benefits (EB) program. The permanent EB program provides an additional 13 or 20 weeks of compensation to jobless workers who have exhausted their regular benefits in states where the unemployment situation is severe. In 2015, no state triggered the EB program. The total number of additional weeks depends on the unemployment rate in a state and its UI laws.²⁶

To be eligible for UI benefits, a worker must (1) be unemployed, (2) be ready and willing to work, (3) be registered for work with the state employment service, and (4) have been working in covered employment during a base eligibility period. A worker who is fired for misconduct, quits a job without a legally acceptable reason, fails to register

with the state employment service, refuses a job equal to or better than the one previously held, or goes on strike is ineligible for unemployment benefits. States cannot deny benefits to workers who refuse to be strikebreakers or who refuse to work for less than the prevailing wage rate.²⁷

Basic decisions concerning the amount of benefits, eligibility, and length of benefit time are made by the states. In general, state laws typically try to replace about half of a worker's previous wage up to a maximum benefit level. In 2015, the average weekly benefit was \$326.00. State unemployment benefits varied widely, ranging from a weekly average of \$235.00 in Mississippi to \$551.00 in Hawaii.²⁸ Benefits are generally based on the wage earned by the beneficiary, while some states adjust that amount based on dependents.

Problems in UI Problems endemic to the UI program include the following:

1. *Part-time workers* were 59 percent less likely than full-time workers to collect UI benefits in 2007. In 23 states, people looking for part-time work were not considered to be looking for "suitable work" and were therefore ineligible for benefits.²⁹ This changed with the introduction of the Recovery Act in 2011, which provided incentive payments to states who met a set number of requirements (see Figure 10.2). One of the requirements was that states cannot decline benefits to unemployed persons seeking part-time work.³⁰
2. *Temporary workers* are less likely than all other workers to receive UI benefits. The number of temporary workers has been increasing, and by 2014 they totaled 3.24 million. Those who work at temporary agencies face several difficulties when applying for UI benefits. First, they may not have worked enough hours or earned enough to qualify for benefits. Second, since they were employed at temporary agencies, they have difficulty proving they did not voluntarily quit the job. Plus, to remain eligible for benefits, a worker cannot refuse "suitable work," or work of the same nature at or above the same pay rate that the worker was earning before. Some temporary workers have been denied benefits because they refused a different type of temporary assignment than they had previously been doing.³¹

3. *Low-wage workers.* Throughout the 1990s and early 2000s, low-wage workers were less likely to be eligible for and receive UI benefits. This trend persisted throughout the recession. Between 2009 and 2013, workers whose earnings were in the bottom 30 percent were half as likely to receive UI as those in the top 70 percent. This is because low-wage workers often fail to meet minimum earning requirements, particularly if they have been employed part time.³² Another reason low-wage workers are often ineligible is that not all of their earnings are considered when determining eligibility. In most states, up to six months of a worker's earnings do not count toward the earnings requirements because the state ignores the last completed quarter and the current quarter of earnings. A number of states have adopted low-wage worker eligibility reforms based on an "alternative base period," which expands eligibility for workers who need their most recent earnings to qualify for UI.³³ By 2014, 29 states had adopted low-wage worker eligibility reforms based on an "alternative base period," which expands eligibility for workers who need their most recent earnings to qualify for UI.³⁴
4. *Limited coverage.* In 1975, 75 percent of jobless workers received benefits; by 1986, it dropped to 33 percent.³⁵ Between 2005 and 2007, the number of UI recipients was consistent at about 36 percent; however, the recession drove that up to 49 percent by 2010.³⁶ Hence, just over half of the U.S. unemployed are still not receiving UI benefits. This is particularly problematic since in February 2015 the number of long-term unemployed (for 27 weeks or more) accounted for almost 2.7 million unemployed.³⁷ Long-term unemployment is a particularly acute problem for older workers. While the length of unemployment for a jobless worker in the United States was 27 weeks in April 2015, the average jobless person from age 55 to 65 was looking for work for about 54.3 weeks, or just short of a year.³⁸
5. *Benefit decline.* In 1986, total UI benefits were 59 percent lower than in 1976 (after adjusting for inflation). By 2010, only about half of the average worker's lost wages were replaced by UI benefits.³⁹ As such, many families cannot afford to live on these benefits.
6. *Longer spells of unemployment.* UI was based on the assumptions that (1) layoffs are temporary and employers will recall workers or (2) suitable jobs would be found that match the skills and experience of the unemployed. Now, however, many people are losing their jobs because of long-term structural changes in the economy. The median duration of unemployment in July 2007 was 8.9 weeks; by 2014, it had risen to 27 weeks.⁴⁰
7. *Women and unemployment.* From 2007 to 2010, 2.7 million women lost their jobs. Since the recession of 2007–2008, women have experienced less growth in employment opportunities than men. Between 2011 and 2012, men took 67 percent of the 1.9 million jobs created and women only 33 percent. By 2012, women regained 34 percent of the jobs lost during the recession, while men regained 45 percent.⁴¹ Sixty-seven percent of part-time workers are women, and one-third of all women work part time. Women are also more likely to be employed by temporary agencies than are men, and their lower wages make them less likely to meet minimum UI earnings requirements. Finally, many women leave their jobs for voluntary reasons that are not considered "good cause" in many states. In some states, a woman is ineligible for UI benefits if she quits to escape a violent domestic partner, because of a schedule change that makes child care arrangements impossible, to take care of a sick child, or to follow a spouse to another location.
8. *Meeting the UI system's financial obligations.* The UI system has difficulty in meeting its financial obligations during economic downturns without federal aid or deficit spending. The huge increase in the number of UI recipients during the recession put enormous pressure on the UI finances. By late 2009, states had borrowed nearly \$40 billion from the federal Unemployment Trust Fund to maintain their UI programs. States are required to fully repay the loans, with interest, within two to three years of borrowing the funds. If a state cannot repay the full amount, the federal government recoups its funds by effectively raising the federal tax on employers within the state.⁴²

9. *Regressive nature of UI.* Because UI benefits are capped, they replace a smaller share of previous earnings for higher-wage workers than lower-wage workers. As noted earlier, benefit levels vary widely from state to state. In addition, the federal tax is set by the Federal Unemployment Tax Act (FUTA) and is equal to 0.8 percent of the first \$7,000 paid annually to each employee. This tax is regressive because most workers earn more than \$7,000 per year and are effectively paying the same \$56 flat tax a year regardless of their income. Hence, these taxes represent a much smaller share of the wages of high-wage workers than low-wage workers.⁴³

Workers' Compensation

In 2014, the U.S. Bureau of Labor Statistics reported that 4,679 workers were killed on the job, and there were about 3 million non-fatal work-related injuries or illnesses that required sick leave.⁴⁴

Workers' Compensation (WC) programs began in 1911 in Wisconsin and New Jersey. By 1948, every state operated some form of WC program. WC programs provide cash, medical assistance, rehabilitation services, and disability and death benefits to persons (or their dependents) who are victims of industrial accidents or occupational illnesses.⁴⁵ By 2009, 97 percent of all UI-covered workers were protected by Workers Compensation.⁴⁶ Although laws vary from state to state, the basic principle is that employers assume the costs of occupational disabilities without regard to fault.⁴⁷ Although most workers are covered in the majority of states, business owners and independent contractors are often excluded, as are volunteer, farm, railroad, maritime, and domestic workers. Federal employees are covered under a special program.

The specific laws governing WC vary from state to state, and there is little consistency either in benefit levels or in the administration of the programs. For example, some states require employers to carry WC insurance, some provide a state-sponsored insurance fund, others allow employers to act as self-insurers, and still others do not require compulsory WC coverage. Some state programs do not cover employees of nonprofit, charitable, or

religious institutions. Because of the potential for large claims, most employers transfer their responsibility by purchasing insurance from private companies that specialize in WC.

WC programs are problematic in several ways. First, benefit levels are established on the basis of state formulas and are usually calculated as a percentage of weekly earnings (generally about 66.66 percent). As such, each state sets its own annually adjusted benefit level, and these levels vary widely across states.⁴⁸ The cost to employers for providing WC insurance is significant, and they paid almost \$83.2 billion in 2014 to insure their workers.⁴⁹

There is great variability among states in the way claims are handled. Workers are often encouraged to settle out of court for attractive lump sums, even though the amounts may not equal their lost wages. Often, benefits are uneven. For example, the price attached to the loss of a body part has been interpreted differently from state to state.⁵⁰ There are often long delays between the time an injury occurs and the start of benefits. Finally, in some states employers are exempt from the Workers' Compensation tax if they can demonstrate that they are covered by private insurance. Unfortunately, private insurance coverage may prove inadequate in meeting a disability benefit. Despite receiving \$61.9 billion in benefits and medical payments in 2014, injured workers or their survivors may not receive adequate protection.⁵¹

The Social Security Dilemma

Because of its widespread public support, Social Security was long perceived by policy analysts and legislators as the third rail of politics—touch it, and you die. For decades, the immediate focus of conservative Republicans and Democrats was on the more vulnerable public assistance programs that enjoyed less public support. Nevertheless, emboldened by the successful passage of the Personal Responsibility and Work Opportunity Reconciliation Act of 1996, conservatives increasingly turned their attention to the massive social insurance programs. Opponents argued that Social Security was depressing private savings (and thereby providing less capital for investment) by giving people a retirement check financed by the working population rather than interest on accumulated savings.

Spotlight 10.1

MAJOR SOCIAL INSURANCE PROGRAMS

In the United States, there are four major social insurance programs:

- OASDI (also known as Social Security) covers the elderly, survivors, and those people unable to work due to a disability.
- UI covers workers who have been fired or laid off from their jobs.
- WC provides case, medical assistance, rehabilitation services, and disability and death benefits to persons (or their dependents) who are victims of industrial accidents or occupational illnesses.
- Medicare

Arguments against the Current Social Security System

Social Security in the United States is a pay-as-you-go system with birthrate and longevity determining its solvency. The U.S. birthrate dipped below 20 per 1,000 people in 1932 (at about the time Social Security was created) but rose in the 1950s and 1960s. The general fertility rate in the United States dropped to a record low in 2014 with 1.86 babies, well below the 2.1 needed for a stable population.⁵² This has important implications for the future of the Social Security system.

The average life expectancy in 1935 was 63; today it is about 78. As a result of these demographic factors, the number of workers paying Social Security payroll taxes has gone from 16.5 workers for every retiree in 1950 to just 3.3 for every Social Security beneficiary in 2009. According to the Social Security Board of Trustees, that ratio is expected to decline to just 2.2 by 2025.

Social Security is fueling intergenerational tensions. Younger workers are skeptical about the ability of Social Security to support them upon retirement. They are nervous that Social Security will buckle now that the baby boomers have begun to retire. Some are also anxious because they are saving less than their parents. Ted Dimig sums up the dilemma: “So, where does this leave my generation? First of all, it leaves us with a huge resentment over the idea that our elders might saddle us with the debt for their retirement . . . while shortchanging us on our own retirement.”⁵³

Although OASDI is an essential part of economic security for the nation’s retired, serious problems threaten its future viability. The original strategy of the Social Security Act of 1935 was to create a self-perpetuating insurance fund, with benefits to the elderly being in proportion to their contributions. That scenario did not materialize. Instead, Social Security took on some public assistance characteristics in terms of providing a minimum benefit level regardless of a worker’s contribution.⁵⁴ Moreover, both the Social Security tax and the wage base upon which the tax is determined have increased so dramatically since the 1970s that many current workers may not recover their entire investment, depending on their longevity, their marital status, and whether they earned a high or low wage.

Arguments for the Current Social Security System

The Social Security program serves four main functions.

- It supports millions of survivors of deceased workers and disabled Americans.
- It is the nation’s most successful antipoverty program.⁵⁵ Largely because of Social Security, the nation’s elderly poverty rate has fallen over the last 50 years from more than 40 percent to 9 percent.⁵⁶ By 2014, only 80 percent of elderly Americans had access to public or private pension plans or annuities.⁵⁷ Without

Social Security, about half of elderly Americans would live below the poverty line.

- In 2008, 53 percent of married couples and 74 percent of nonmarried persons aged 65 or older received Social Security benefits. It provided at least 50 percent of total income to the aged beneficiary couples and aged nonmarried beneficiaries. Social Security benefits accounted for 90 percent or more of income for 22 percent of aged beneficiary couples and 47 percent of aged nonmarried beneficiaries.
- Social Security is important to working as well as retired Americans (about one in three Social Security recipients is not a retiree). Financially solvent seniors are consumers and therefore good for businesses. Moreover, few children would trade the burden of payroll taxes for the onus of supporting destitute retired parents.

Social Security is a special kind of “investment” unavailable in the private sector. Specifically, it is a lifetime retirement annuity with benefits that rise with inflation. Almost all corporate pensions (many which run out after 20 years) are not indexed to the erosion of purchasing power caused by inflation.

Social Security has been accused of overpaying the elderly, slighting younger workers (who could get a better return if they invested privately), and leading the nation to financial collapse. Merton and Joan Bernstein challenge this criticism and claim that rather than discouraging private savings, Social Security actually stimulates financial planning for retirement and thus encourages savings.⁵⁸ Others dispute the overpayment argument by noting that in 2010, the median income for households aged 65 and over was \$31,408, which was \$18,037 below the national median household income of \$49,445.⁵⁹ This would hardly suggest an elderly population growing wealthy by exploiting an overly generous Social Security system.

Social Security in Trouble

Social Security began to show signs of fiscal trouble in the mid-1970s. Between 1975 and 1981, the OASI fund suffered a significant net decrease in funds and a deficit in the reserve. This imbalance between incoming and outgoing funds threatened to deplete the reserve by 1983. The prospects for Social Security seemed bleak in other ways. While the ratio of workers to supported beneficiaries (the **dependency ratio**) was three to one in the 1970s,

it was estimated that by 2025 this ratio would approach two to one. In short, the long-term expenditures of Social Security would exceed its projected revenues. This crisis was fueled by demographic changes (a dropping birthrate coupled with an increase in life expectancy), liberal benefits paid to retiring workers, the effects of inflation, high levels of unemployment, and the COLAs passed by Congress in the mid-1970s.

Facing these short- and long-term problems, Congress quickly passed p. 98–21, the Social Security Amendments of 1983. Among the newly legislated changes was a delay in the COLAs. Specifically, if trust funds fall below a certain level, future benefits will be keyed to the consumer price index (CPI) or the average wage increase, whichever is lower. A second change was that Social Security benefits became taxable if taxable income plus Social Security benefits exceeded \$25,000 for an individual or \$32,000 for a couple. A third change increased the 2027 retirement age to 67 to collect full benefits. Although workers could retire at age 62, they would receive only 70 percent of their benefits instead of 80 percent. Finally, new federal employees were covered for the first time, as were members of Congress, the president and vice president, federal judges, and employees of nonprofit corporations. In the mid-1990s, the Social Security rules were again changed, this time easing the historic Social Security penalty for about 1 million beneficiaries aged 65 to 67 who were still working. The new rules meant that for those reaching their full retirement age, the earnings limit was \$36,120 with a loss of \$1 of benefits for every \$3 earned.⁶⁰ Moreover, the retirement age for receiving full Social Security benefits was raised to 66 for those born between 1943 and 1954 and to 67 for those born after 1959.

The Long-Term Prospects for Social Security

The long-term financial prospects for Social Security depends on several factors, including the general health of the economy, the unemployment rate, immigration, the life expectancy of beneficiaries, and the birth rate. Despite the reforms noted above, the fiscal viability of the OASDI system remains in question as deficits in funding are predicted to grow rapidly as the baby boom generation’s retirement causes the number of beneficiaries to increase more rapidly than the number of employed workers.

Some analysts suggest that Social Security is on sound footing. They point to the fact that the assets reserve in the combined OASDI trust funds was almost \$2.8 trillion in 2014, with receipts accounting for \$884 million and expenses at \$859 million. This was a \$25 million increase in reserves over 2013. However, it was also the slowest rate of increase since 1988, coming down from a high of \$190 million in 2007. The rate of increase has been falling every year since 2007.⁶¹ It is projected to keep on falling until 2019 when program costs will exceed tax revenues and the asset reserves will need to be drawn on. In their 2015 report, the Social Security and Medicare Boards of Trustees predicted that the total trust funds reserves will be depleted in 2034, but after 2035 the growth will slow as the population of baby boomers will decline. After 2038, tax income is projected to be sufficient to pay about three-quarters of scheduled benefits through the end of the projection period in 2089⁶² (see Figure 10.2). The viability of OASDI represents only part of the picture since

Social Security funding also relies on additional trust funds, including the Medicare HI Trust Fund, which pays for inpatient hospital and related care; and the SMI Trust Fund, which pays for physician and outpatient services; and Part D prescription drug benefit.

As noted in Table 10.2, while the Social Security retirement fund will be depleted in 2038, the disability fund is in immediate danger of insolvency. According to the Trustees, “Social Security’s Disability Insurance (DI) Trust Fund now faces an urgent threat of reserve depletion, requiring prompt corrective action by lawmakers if sudden reductions or interruptions in benefit payments are to be avoided.”⁶³ Unless legislative changes are implemented immediately, automatic reductions in payments to DI beneficiaries is predicted to begin in late 2016.⁶⁴



Watch this video, in which the National Academy of Social Insurance claims that Social Security is “built to last”.
www.youtube.com/watch?v=IHAcY4SNVbo

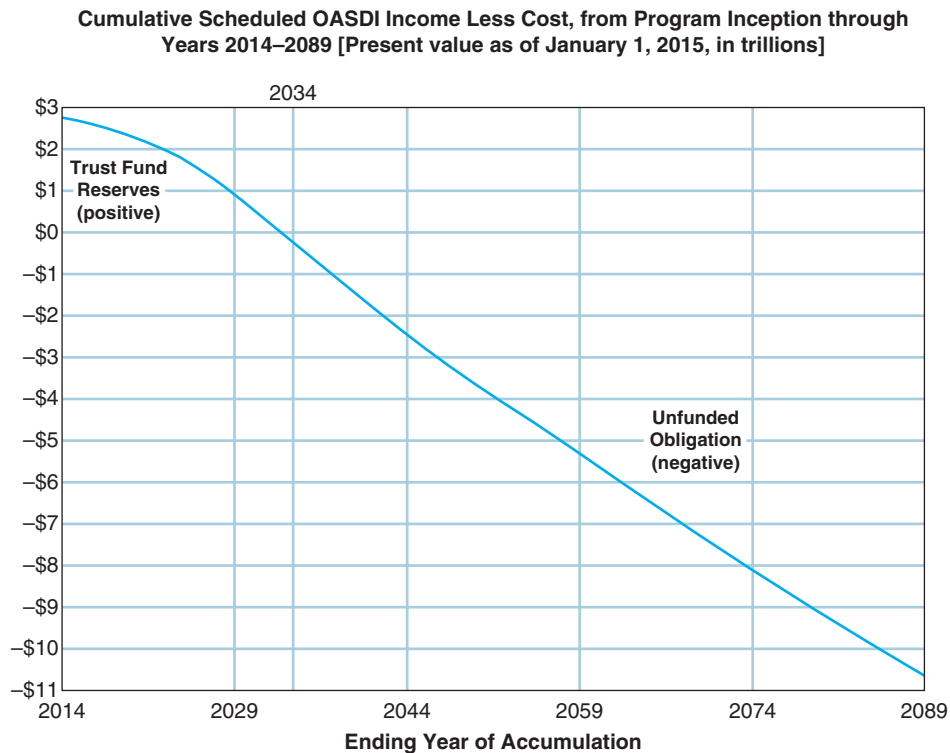


FIGURE 10.2 Projected Exhaustion of the Social Security Trust Fund

Source: 2015 OASDI Trustees Report, Projections of Future Financial Status, Washington, D.C. Retrieved December 2015 from, https://www.socialsecurity.gov/OACT/tr/2015/II_D_project.html#105057socialsecurity.gov/OACT/TR/2011/tr2011.pdf.

TABLE 10.2 Key Dates for the Trust Funds

	OASI	DI	OASDI	HI
First year outgo exceeds income, excluding interest	2017	2005	2010	2008
First year outgo exceeds income, including interest	2025	2009	2023	2008
Year trust funds are exhausted	2038	2018	2036	2024

Source: Social Security Administration, "Status of the Social Security and Medicare Programs: A Summary of the 2014 Annual Social Security and Medicare Trust Fund Reports," Last Revised May 2014. Retrieved November 2015 from <http://www.socialsecurity.gov/OACT/TRSUM/index.html>.

Medicare (Hospital Insurance and Supplementary Medical Insurance)

The Trustees estimate that the Medicare HI Trust Fund will be depleted in 2030. At that point, revenues will only cover 86 percent of HI costs. The Trustees maintain that the proportion of HI costs financed by HI revenues will decline to 80 percent in 2050 and will then rise gradually to 84 percent in 2089. HI non-interest income less HI expenditure was negative in 2015, as it has been every year since 2008. According to the Trustees, the HI fund—like OASDI—fails the test of financial adequacy.⁶⁵

The Trustees predict that Part B of SMI, that pays physician and outpatient costs, and Part D (which pays for prescription drug coverage) will continue to be adequately funded into the future since it is based on financing from general revenues and beneficiary premiums. However, the aging population and rising health care costs will cause SMI costs to grow steadily, eventually rising to 3.5 percent of the GDP from 2 percent in 2014. General revenues will finance roughly 75 percent of the costs with beneficiary premiums funding the other 25 percent.⁶⁶

Lingering Problems in the Social Security System

Although some Social Security reforms have provided a short-term solution, structural problems continue to plague the system. As already discussed, one problem is the "graying of America." Since 1900, the percentage of Americans 65 years and older has more than tripled (from 4.1 percent in 1900 to 12.3 percent in 2006). Between 2010 and 2012, the number of Americans aged 65 and older increased

from 40 million to 43.3 million. Older Americans now represent 13 percent of the total population.⁶⁷ Demographic projections suggest that by 2050, the number of persons over age 65 will increase to 83.7 million.⁶⁸ Furthermore, the elderly are living longer. Between 2012 and 2020, the 85- to 94-year-old age group is projected to increase by 2 percent, and the 95 and older age group increased by 2 percent.^{69,70} These demographic trends suggest that the dependency ratio (the number of current workers it takes to support a retired worker) will significantly increase, as will the pressures on the Social Security system. For example, in 1960 the worker/beneficiary ratio was 5 to 1; by 2007, it had dropped to 3.4 to 1. By 2025, the dependency ratio is expected to be 2 to 1.⁷¹ A question arises whether two workers in 2040 will be able to support one retired person, and whether the Social Security system—as presently structured—can support 20 percent of the U.S. population.

Social Security predications are predicated on the belief that certain economic and demographic factors will be in play for the foreseeable future. However, economic and demographic could easily shift, invalidating the most earnest predictions. For example, a recession (such as the one in 2007), changing demographic trends (i.e., the current low birth rate), or major changes in immigration would have profound consequences for the future of Social Security. However, there is nothing inviolate about the way Social Security is currently funded. An act of Congress could easily eliminate the self-financing social insurance feature of Social Security and replace it with general revenue taxes. Given the widespread dependence on Social Security, it seems highly unlikely that policymakers and the public will allow the system to go bankrupt.

Another issue in Social Security involves the competition between public (compulsory) and private (voluntary) pension plans. Some critics argue that private pension plans are preferable to public plans because they are not dependent on the government and have greater potential for higher returns. Private pensions originated as a means of encouraging employee loyalty and as a way of easing out aging workers. However, only about one-third of all workers and one-fourth of current employees are covered under private pension plans. Moreover, only a small fraction of these plans are indexed for inflation.

Critics of private pension plans argue that they are basically unreliable. Employees can switch jobs and thus lose their pension rights; companies can go bankrupt; corporations can raid pension funds; many are based on the vicissitudes of company stock (e.g., Enron); workers risk losing their entire retirement if the corporate stock drops; and corrupt or incompetent pension managers can wreak havoc on pension plans. Supporters of Social Security argue that unlike the riskier private pension plans, OASDI benefits are portable and indexed for inflation, and benefits are not contingent on the financial condition of the employer.

Although originally intended to supplement private pension funds and to operate as a pay-as-you-go insurance plan, OASDI has taken on many characteristics of a public welfare program. Specifically, some retirees are realizing benefits in excess of their contributions. Should social insurance be modified to reflect social assistance and income redistribution goals? If the answer is yes, then benefits must be structured to reflect the current needs of retired workers rather than their past contributions. Furthermore, if Social Security is viewed as a public welfare program, then its regressive tax structure must be modified along more progressive lines. For example, Social Security is the single largest tax paid by a low-income worker, yet that same worker receives the lowest benefits when they retire. In short, the workers hurt most by the tax receive the fewest benefits. Using that same line of reasoning, if Social Security is designed for social assistance, then should everyone, regardless of income, be eligible? More particularly, should the very wealthy be allowed to be beneficiaries? Unfortunately, there are no simple answers to these questions.

Reforming Social Security

Several proposals have emerged to reform Social Security and extend the fiscal viability of the retirement trust fund.

- Raise the full retirement age to 68. Increasing the normal retirement age amounts to a benefit cut of 6 to 8 percent for each year the retirement age is increased. However, raising the retirement age is a de facto benefit cut since beneficiaries will receive less during their lifetimes.
- Initiate Longevity Indexing whereby the age for retirement would be correlated with the average life expectancy of males and females.
- Recalculate COLA by using an alternative price index. In 2015, the federal government announced that there would be no COLA increases for that year. Since the current method for COLA adjustments are based on the overall rise in the price of goods and services, it does not take into account the “Elderly Index”; namely, that elderly people spend more on health care and pharmaceuticals than the general population, and those costs are rising faster than the general level of inflation.
- Increase the payroll tax cap from \$118,500 to a higher cap. More dramatically, the government could eliminate the cap altogether.⁷² The downside is that it only buys time and would result in massive monthly payments once a highly paid employee retires.

Alternatively, a combination of the above approaches could be adopted.

Privatizing Social Security

Proposals have emerged from several quarters—including the former Bill Clinton administration—to fully or partially privatize Social Security with individual or private fund managed accounts. Supporters argue that privatization would provide individuals with control of the Social Security portion of their retirement plan and that successful investors would likely achieve higher rates of return and higher benefits than Social Security provides. It would also remove the governmental obligation to provide retirement benefits.

By early 2005, the largely theoretical debate about privatizing Social Security came to a head when the former George W. Bush administration released its Social Security reform plan. In one of the greatest challenges to Social Security since its inception in 1935, the Bush plan would have diverted some payroll taxes into private accounts that workers could invest in the stock market. At the same time, the guaranteed Social Security benefit would be cut by up to 40 percent. The Bush administration asserted that since Social Security payroll taxes belong to the people, they should decide what to do with them. Because these accounts would be individually owned, they could be passed along to heirs. Workers would continue to get benefits from the Social Security system since the private accounts would account for only a percentage of benefits.

Critics argued that privatization would replace the guaranteed economic security of Social Security with risky stock market accounts. They maintained that even limited privatization would open the door to full privatization, thereby creating a voluntary system that would destroy Social Security. Opponents claimed that privatization was a draconian solution to the problems of Social Security.⁷³

The Bush plan was virtually dead on arrival since even many stalwart Republicans distanced themselves from such a radical departure in social policy. Most Republicans knew full well that messing with Social Security was extremely dangerous: Older people vote, and they don't like their income support fiddled with. Also, the presumed vast gains in a privatized Social Security system were predicated on a robust stock market. The 2001 stock market crash made many people wary about trusting their future retirement to the market. Any discussion on privatizing Social Security was extinguished by the stock market crash of 2007–2008.

Retirement income and other forms of social insurance are addressed differently in different nations. Social insurance in some countries—similar to the U.S. system—largely depends on a guaranteed governmental benefit to compensate for lost wages due to retirement or disability. Other countries relegate social insurance benefits to a private insurance market model or a combination of private and public. The following section will examine how several industrial countries manage retirement-based social insurance benefits.



Critics argue that privatization would replace the guaranteed economic security of Social Security with risky stock market accounts.

Source: Richard Drew/AP Images

Pension Systems in Selected Industrialized Countries

Canada's Retirement Income System

The Canadian retirement system is a multifaceted system that includes tax-funded benefits, low-income benefits, employment-based pensions, and private tax-subsidized retirement savings. The following make up Canada's retirement system:

1. Old-Age Security (OAS) provides the first level of Canadian retirement income. An eligible applicant needs to be a resident and 65 years or older. Monthly payments are modest: In 2015, the average payment was \$640.23, while the maximum was \$1,065.
2. The Guaranteed Income Supplement (GIS) provides additional money on top of the OAS pension. In 2015, a single eligible retiree received a minimum of \$310.52 and

a maximum of \$639 on top of their OAS pensions.⁷⁴

3. The Canada Pension Plan (CPP) is another level of the system and operates similarly to U.S. Social Security in that it is universal and contributory (i.e., based on mandatory employer/ employee contributions to the system). The CPP is a joint federal and provincial scheme that is administered at the federal level. Like U.S. Social Security, the CPP is designed to be self-financing.
4. Yet another level consists of private pension plans and savings. The Canadian public pension system was designed to provide a modest base to augment private savings and employer pension plans. The self-employed or those with no employer pension plan can supplement their retirement income through the Registered Retirement Savings Plans (RRSPs) and Registered Pension Plans (RPPs), which assist retirement savings.

The Canadian system initially appears more complex than the U.S. system of retirement income based on Social Security, private pension plans, and tax-deferred savings (e.g., IRAs).

Retirement in the United Kingdom

The United Kingdom (UK) has a workforce of around 30 million people, of which about 23 million are employed in the private sector. Only 3.2 million of these workers contribute to a workplace pension plan that includes an employer contribution. (Like in the United States, many private employers have dropped employee pension plans because of the cost and future obligations to the worker.) Most of the UK public sector workers are enrolled in a workplace pension plan. There are also roughly 6.5 million self-employed people paying into personal pensions, which have no employer contribution. Thirty-two percent of private sector workers (including the self-employed) contribute to a personal pension plan that lacks a guaranteed level of retirement income.⁷⁵

Similar to Canada, the UK publicly-funded pension system is based on a two-tiered framework: Basic State Pension (BSP) and the Additional Pension (AP). The BSP is a flat-rate pension paid to anyone who has accrued sufficient National Insurance Contributions (NI). A retiree receives the

full rate if they have 30 qualifying years on their NI record. Those with fewer years receive a proportionately lower payment. Spouses and civil partners without sufficient contributions can receive a pension based on the contribution of their partners, which could total 60 percent of their respective partner's BSP.

The State Second Pension (S2P) is paid to certain groups in addition to the BSP. In 2002, it was modified to provide more generous assistance for low and moderate earners, caretakers and those with a long-term illness or disability. In effect, the S2P is a top-up to the BSP based on actual or assumed earnings. All employees contribute to the AP unless they contribute instead to an alternative pension plan that has contracted out of S2P.

Pension Credit (PC) is a means-tested social security benefit designed to provide qualifying moderate income for the elderly with a minimum income level. PC is composed of two parts (a recipient can receive one or both): the Guarantee Credit (GC) and the Savings Credit (SC). The GC provides a guaranteed minimum level of weekly income for elderly singles and couples. The SC functions as a monetary reward for modest income retirees who have made additional plans for their retirement beyond the BSP. Some people may be eligible for the PC if they or their spouse are severely disabled, are caretakers for someone who is severely disabled, or if they have excessive housing costs.⁷⁶

In 1984, Margaret Thatcher's conservative government was swept into power for a second term. Thatcher viewed the victory as a mandate to address what she saw as the last vestige of socialism—social insurance for the elderly. The Thatcher administration began the assault by declaring that the social insurance system would be unaffordable in the future. Instead, she proposed privatizing a portion of old-age benefits. Workers were asked to forego some future benefits in exchange for a tax rebate that could put into a retirement-based investment account. The result was a shrinking state pension system and a partly privatized retirement system. Unfortunately, fund management costs could reduce lump-sum pensions by up to 30 percent.⁷⁷

Many British retirees were sold dodgy and poorly designed retirement plans on false pretenses. In turn, guilty companies were forced to return about \$20 billion to investors. Britain's Pensions Commission also warned that 75 percent of those

with private investment accounts would not have sufficient savings for retirement.⁷⁸

Policymakers and legislators against privatizing the U.S. Social Security would have discovered a potent critique by observing the abuses resulting from privatizing Britain's system.

The Chilean Experiment in Privatizing Social Security

In 1924, Chile was the first country in the Americas to enact a Social Security system that provided coverage for old-age, survivors, disability benefits, and cash sickness and medical benefits. Chile's original plan was similar to the U.S. system enacted 11 years later.

By the 1970s, Chile's public pension program was foundering for several reasons, including being overly complex with more than 100 different retirement programs with various contribution rates, retirement ages, and benefits. The predictable result was administrative inefficiency.

The funding for the system was similarly bleak. Despite high payroll taxes, Chile's Social Security system was generating insufficient income to pay retirees and they received fewer benefits than promised. For instance, while some workers expected to receive benefits that would replace 70 percent of their wages, their actual replacement rate was closer to 20 percent. These problems were heightened by widespread tax evasion, employers who skirted contribution requirements, and workers in the gray or underground economy who were paid in cash.⁷⁹

By the late 1970s, the Chilean Social Security system was effectively broke. In 1981, military dictator Augusto Pinochet replaced Chile's publicly-funded Social Security program with a mandatory system of privately managed individual accounts. Under this scheme, workers contributed 10 percent of their salary (up to a specified ceiling) to a government-approved investment fund. Employees were required to pay another 3 percent to cover term life and disability insurance. The self-employed were required to set up investment accounts with the same basic features. These accounts were managed by private investment firms. On retirement, workers could purchase an annuity or withdraw their money based on a schedule.⁸⁰

By 2005, the first generation of Chilean workers enrolled in the new system began to retire and

found that Pinochet's promises fell far short of expectations. Some middle-class workers who regularly contributed to a pension fund found that hidden fees ate up almost 30 percent of their original investment. Many felt that they would be better off in the old system. Moreover, the government was forced to direct billions of dollars into a safety net for retirees whose contributions did not even reach the minimum \$140 monthly pension. Pension spending made up more than a quarter of Chile's national budget and accounted for almost as much as health and education spending combined. In addition, many workers remained outside of the system, including those in the underground economy, the self-employed, and seasonal workers. (These groups make up about half of the Chilean workforce.) Another factor was the vulnerability of retirement accounts to extreme market fluctuations.⁸¹ Like the British reforms, the expectations of retired Chilean workers in their retirement pension fell far short of the reality.

Germany's Social Security System

The German retirement system is built on governmental Social Security pensions, occupational pensions, and individual retirement accounts (relatively minor). Similar to the U.S. system, German Social Security provides survivor and disability benefits, and is mandatory and universal (about 85 percent of German workers are enrolled). Civil servants (about 9 percent of the work force) have their own pension system and although the self-employed (also about 9 percent of the work force) are mostly self-insured, they can also participate in the public pension system.

Contributions are based on annual earnings, and the almost 20 percent contribution rate is split evenly between workers and employers, with low-wage earners paying less. The government subsidizes contributions for a worker raising a child (for up to three years) or providing care to a relative. Also similar to the U.S. system, it is a pay-as-you-go system whereby retirement benefits are paid for by current workers. Not surprisingly, the German system is financially stressed by an aging population, low birthrates (Germany surpassed Japan in 2015 as having the lowest birthrate in the world), unemployment, workers having fewer years in the labor force (partly due to more years of education) and early retirements. As a result, 2007 changes in German law raised the minimum retirement age

from 65 to 67.⁸² However, in 2014 the German legislature passed new rules allowing a select group of workers to retire at age 63, while continuing to phase in the higher retirement age of 67.⁸³

About three-fifths of Germany's working population is covered by occupational pension plans that augment Social Security. Employer-sponsored pension plans are a long-term fixture in German industry, and more than 17 million people are covered under these agreements.⁸⁴

The Greek Pension System

Until recently, Greece had one of the most liberal pension systems in Europe. For example, the Greek government had identified at least 580 job categories thought to be sufficiently hazardous to merit early retirement (age 50 for women and 55 for men). These hazardous industries included radio and television presenters (presumed at risk from the bacteria on their microphones) and musicians playing wind instruments (at risk for gastric reflux). All told, the Greek government had promised about 700,000 employees (14 percent of its work force) early retirement, which would give the country an average retirement age of 61.⁸⁵ The conservative business press was particularly critical regarding the Greek retirement system:

- Years of work to earn a full pension—Greece 35, Germany 45
- Proportion of wages as pension—Greece 80 percent, Germany 46 percent
- Number of pension payments a year—Greece, 14 (bonus payments); Germany 12
- Pension increase in 2006—Greece 3 percent, Germany 0 percent
- Maximum payment—Greece \$3,333, Germany \$2,758
- Minimum pension age for men—Greece 65, Germany 65–67
- Minimum pension age for women—Greece 60, Germany 65–67
- Average pension entrance age—Greece 62.4, Germany 63.2⁸⁶

In its 2009 annual report on Greece, the International Monetary Fund warned that left unchecked, the government's excessive old-age pension and health payments would result in a debt level of 800 percent of Greece's total output by 2050.⁸⁷ In part, Greece's generous retirement is assumed to be partly responsible for its massive

debt and near-bankruptcy status. Moreover, Greece (like many other countries) had had a problem in assessing and collecting tax revenues; hence, Harry Hopkins's recipe for staying in power (tax, tax, spend, spend, elect, elect) was missing a vital part.

The Australian Retirement System

The Australian retirement system is based on superannuation (called Super) and differs from the U.S. and other European systems because of its heavy reliance on privately managed investment funds. The compulsory superannuation (private investment funds) system was introduced as part of a reform package arising from the fear that an aging population would stress an Australian economy facing low birth rates coupled with high pension responsibilities. The solution was a three-pronged approach:

- A modest means-tested governmental pension system designed as a safety net for low-income workers who fell through the superannuation cracks.
- Compulsory contributions to a superannuation fund that would force private savings.
- The promotion of voluntary savings through the use of superannuation-based tax breaks. Superannuation forms part of an overall employment package whereby employers are required to deposit contributions calculated at least 9 percent of an employee's wages into a retirement account. (Some employers, like universities, can deposit as much as 17.5 percent into an employee's Super account.) The government offers tax breaks and other incentives to help grow Super accounts. Super funds invest in stocks, bonds, and other investments.

A worker's Super is quarantined until retirement and cannot be withdrawn before a certain age, usually 55 or older. In some cases, employees can choose their own Super fund. Workers over age 55 can save an additional limited amount without accruing a tax liability.⁸⁸ In many cases, workers can choose a defined benefits plan whereby they get a specified monthly benefit upon retirement. The defined benefit is usually determined by a formula based on earnings, length of employment, and age. The amount of benefits can be reduced if the superannuation fund finds itself in financial distress brought on by a recession or drop in the stock market.

Perhaps the most compelling criticism leveled against superannuation is that it exposes the retirement income of Australians to the vicissitudes of the stock market. This was particularly problematic when stock markets plummeted in the global financial crisis, and Australia's superannuation funds lost about \$430 billion.⁸⁹ Consequently, workers who planned to retire had to forestall their plans and remain in the workforce. Others were forced back into work as their Super accounts were rapidly depleted.

In addition to superannuation, approximately 80 percent of retired Australians also receive a part or full age pension from the government. The pension is a governmentally-funded retirement income scheme for people who cannot fully support themselves. The single rate pension is set at roughly 28 percent of male average weekly earnings. In early 2015, the maximum amount was approximately \$788.40 a month for a single and \$1,188.60 for a couple.⁹⁰

Conclusion

When compared to several others systems in industrialized nations, the U.S. Social Security stands out as one of the best in the world for protecting workers and retirees from the economic vicissitudes that could dramatically reduce a lifetime of contributions. While retirement benefits for American workers may be initially lower than for workers in privatized pension systems, the benefits are more predictable, which allows U.S. workers to better plan for their retirement.

Social insurance programs, especially OASDI, have become a mainstay of the American social welfare state. Despite the original intent of its architects, Social Security has become a primary source of financial support for elderly Americans. Moreover, OASDI has demonstrated the ability to not only curb the poverty rate for the elderly but also to actually reduce it. A majority of Americans have come to view Social Security as a right and to count on its benefits.

Social insurance programs represent a major source of security for both elderly Americans and present-day workers. Over the past 50 years, Americans have come to believe that regardless of the ebb and flow of economic life, Social Security and UI embody a firm governmental commitment to care for workers and the elderly. Economic gains made by elderly people since the mid-1960s have

validated this belief. Because Social Security is clearly linked to past contributions, beneficiaries experience little stigma. This is not true for the highly stigmatized beneficiaries of public assistance programs, and it is to this population that we will turn in Chapter 11.

DISCUSSION QUESTIONS

1. Social Security is the most popular social program in the nation. Part of the reason for this popularity is that unlike income maintenance programs, it is not stigmatized. What are other reasons for its popularity?
2. There are serious concerns about the future of Social Security. Some critics argue that Social Security is doomed since the trust funds are expected to be depleted within the next 30 years. Other observers argue that Social Security is sound, because the federal government is backing it. Is Social Security on solid ground? What can we expect in the future? What can be done to make the system more stable?
3. The Social Security system is currently plagued by several different problems. What are the three most important problems facing the system?

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Public Assistance Programs



Source: J. Pat Carter/AP Images

This chapter examines key public assistance programs, including the former Aid to Families with Dependent Children (AFDC); AFDC's replacement, Temporary Assistance to Needy Families (TANF); Supplemental Security Income (SSI); and general assistance (GA). The chapter also investigates and analyzes the problems and issues inherent in public assistance programs.

The U.S. social welfare state is a complex mixture of programs, policies, and services. Perhaps few people, including many policymakers, fully appreciate the complexity of the welfare system. One reason is that unlike many European countries that operate under a comprehensive and integrated welfare plan, the United States relies on a patchwork quilt of social welfare programs and policies. Hence, public assistance in the United States is a disorganized mix of programs and policies, rather than a comprehensive, integrated, and non-redundant system of social welfare services.

Public assistance programs are one of the most misunderstood components of the U.S. welfare state. Although expenditures for public assistance programs are far less than for social insurance programs, they tend to be more controversial. Unlike social insurance, public assistance programs are based entirely on need and are means-tested (see Figure 11.1).

Public assistance programs that offer cash, medical, and other forms of assistance are based on the concept of safety nets, which are designed

to ensure that citizens receive basic services to keep them above a federally defined poverty level. There are 51 separate safety nets—one in each state and in the District of Columbia. States differ with respect to benefit levels, and the vast majority lack an adequate safety net to help the poor and the jobless.¹ Public assistance programs are based on a minimalist approach designed to ensure that beneficiaries receive the necessary resources to meet their subsistence needs, but not enough to lift them out of poverty.

Assumptions and Myths about Public Assistance

American attitudes toward public assistance can be characterized as a mixture of compassion and hostility. This ambivalence plays out in a series of harsh and often conflicting assumptions about public assistance recipients. The struggle around public assistance programs is symbolic, reflecting the tensions in Americans' ideas about wealth, opportunity, and privilege.

On the hostility side, the argument goes like this: If privilege is earned by hard work, then people are poor because they are lazy and lack ambition. Those driven by competitiveness see the inability of the poor to compete as a serious character flaw. On the other hand, only a few paychecks separate the

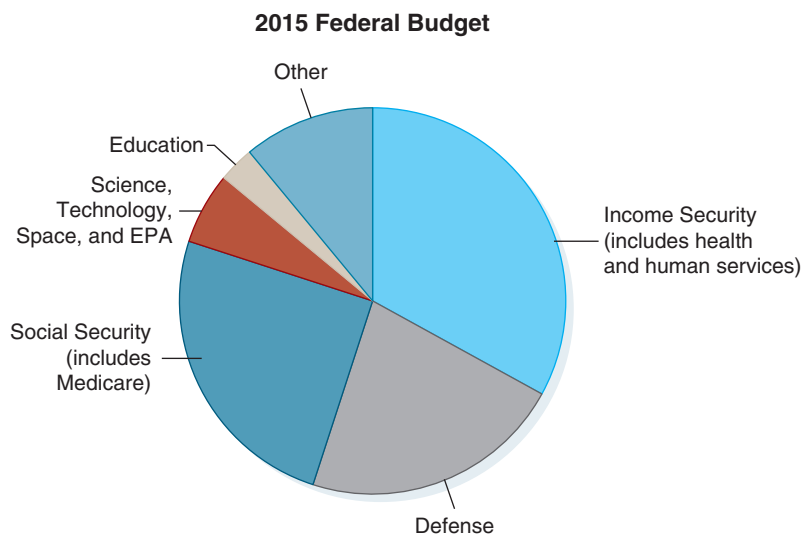


FIGURE 11.1 2015 Budget Outlays by Category

Source: Based on The White House, Budget of the U.S. Government, Fiscal Year 2015, Office of Management and Budget, Washington, DC. Retrieved December 2015 from www.budget.gov.

Spotlight 11.1

KEY PUBLIC ASSISTANCE PROGRAMS IN THE UNITED STATES

In the United States, there are four major public assistance programs:

1. Temporary Assistance to Needy Families (TANF): a block grant based on workfare, time-limited benefits (a maximum of 5 years), and strict work participation rates.
2. Supplemental Security Income (SSI): provides cash assistance to the impoverished elderly and the disabled poor, including children.
3. General Assistance (GA): state or locally run programs designed to provide basic benefits to

low-income people who are ineligible for federally funded public assistance programs.

4. Supplemental Nutrition Assistance Program (SNAP): Originally known as the Food Stamp Program and renamed in 2008.² The program provides benefits to low-income households for food assistance.³



Watch this video of Congresswoman Barbara Lee speaking on the House Floor about TANF.

www.youtube.com/watch?v=BC_WN8r0Nlc

welfare recipient from the average citizen—thus the compassion.

Although democratic capitalism is rooted in the belief that hard work guarantees success, real life often tells a different story, especially given the rapid growth of income inequality. The following assumptions, among others, underlie much of the discussion around public assistance:

- Because generous benefits create a disincentive to work, recipients must always receive less cash benefits than if they worked at a minimum wage job.
- Welfare recipients need prodding to work because they lack internal motivation.
- Work is the best antipoverty program.
- Public assistance programs must be highly stigmatized lest people will all too readily embrace them.
- Women receiving public assistance should work, and their children should not have the luxury of being raised by a full-time homemaker.

These assumptions about public assistance—many of which are remarkably similar to those found in the Elizabethan Poor Laws—are rife with serious misconceptions. The many myths surrounding public assistance make it important to discriminate between fact and fiction. (Note that in the items that follow,



One of the most commonly held myths about public assistance is that recipients don't want to work.

Source: Albert H. Teich/Shutterstock

AFDC and TANF are used interchangeably since they served, and continue to serve, the same clientele.)

Myth 1. Many families on public assistance include an able-bodied father who refuses to work.

Fact. In 2015, only about 66,000 of the monthly average of 1.3 million TANF families (1/2 percent) were two-parent families.⁴ This was a significant drop from the 342,000 two-parent TANF families in 1995.⁵ The majority of TANF

families are headed by mothers who are unmarried, divorced, widowed, or separated.⁶ About 90 percent of TANF children live with their mothers and 10 percent with their fathers.⁷

Myth 2. Most poor people are on public assistance and the number is growing.

Fact. In 2015, only 1.3 million families were receiving TANF assistance on average in any given month.⁸ This represents only 14 percent of the 9.2 million families that lived below the poverty line in 2014. Clearly, many low-income families do not turn to TANF for support or are denied income support.

Many states have developed policies that attempt to divert assistance from applicants needing the least amount of state help to encourage them to become self-sufficient. As of October 2015, 48 states had created formal “diversion programs” that included job search requirements for eligibility or provided a one-off, lump-sum payment to deal with immediate needs instead of providing applicants with a monthly TANF benefit.⁹ Families that accept diversion payments are typically barred from applying for monthly TANF benefits for a stated period of time.¹⁰

Myth 3. Recipient mothers have more children in order to collect greater benefits; therefore, families on public assistance are large and steadily growing in size.

Fact. The average size of families receiving TANF benefits in 2013 consisted of one child.¹¹ This represents a significant shift from 2009, where the average size of TANF families was three persons. In 2013, over 50 percent of TANF families had only one child, with 30 percent of child recipients between 6 and 11 years of age and 27 percent between 2 and 5 years of age. In 2015, at least five states (California, Washington, South Carolina, Wisconsin, and New Mexico) had cut monthly cash assistance that prohibited TANF families from receiving additional assistance for any child born after the mother has enrolled in the program.

Myth 4. Once on welfare, always on welfare.

Fact. This has become a moot point, since the 1996 TANF regulations instituted a five-year lifetime benefit cap for recipients.¹² The number of welfare recipients who return after a period of employment is relatively modest: In 2002, 25 percent of people who received welfare between 2000 and 2002 had returned.¹³

Even before TANF, more than half of all AFDC recipients left the rolls within one year; by the end of two years, that number increased to 70 percent. However, over the course of seven years, more than 75 percent of those who left welfare returned at some point.¹⁴

Myth 5. Welfare programs create intergenerational dependency.

Fact. **Welfare dependency** theories are predicated on the belief that welfare weakens traditional values around nuclear families, individualism, initiative, and self-determination. Given the lifetime cap on TANF cash assistance, the dependency issue has become less relevant. Even before TANF, the vast majority of AFDC recipients were on public assistance for less than four years; those who stayed eight years or more accounted for half of the people on the rolls at any given point in time.¹⁵ The research is inconclusive regarding the existence of welfare dependency.

Myth 6. Most welfare recipients are African Americans and Hispanic Americans.

Fact. In 2015, whites constituted 61 percent of recipients, while 33 percent were black, and 9 percent Hispanic. The remaining recipients were Asian, American Indian, or of another ethnicity.¹⁶ Although the percentages of African Americans and Hispanics on the welfare rolls were larger than these groups’ representation in the population, this is not surprising, since people of color are statistically poorer than whites.

Myth 7. Public assistance benefits provide a disincentive to work; people on welfare either do not want to work or are too lazy to work.

Fact. In 2012, 17.6 percent of TANF recipients were employed part-time, 33.5 percent were unemployed, and 25 percent were not in the labor force.¹⁷

Myth 8. Public assistance recipients are doing better than ever.

Fact. The reverse is true. Four states had lower TANF benefit levels in 2010 than in 1996; 16 states had the same benefit levels as in 1996. Even those states that raised TANF benefits have generally not kept pace with inflation. When adjusted for inflation, 48 states have lower real-dollar benefit levels in 2010 than in 1996.¹⁸ Today, the maximum benefit amount in 30 states comprises 30 percent or less of the federal poverty level.¹⁹

Myth 9. Never-married teen mothers constitute the bulk of welfare recipients.

Fact. In 2015, teen parents made up less than 5 percent of the TANF caseload. Of the overall TANF caseload, only 5 percent of adult beneficiaries were under age 20; 29 percent were aged 20 to 29 years; and 27 percent were aged 30 to 39 years.²⁰

Myth 10. It is easy to get on public assistance and too many undeserving people are receiving benefits.

Fact. David Morris noted that in the early 1990s, about 75 percent of those who walked into the welfare office to apply ended up on welfare rolls.²¹ Under TANF, states have broad discretion to determine eligibility and benefits. Nationally, TANF caseload have fallen by more than 60 percent since its inception in 1996, even as the numbers living in deep poverty have risen. Sixty-eight families received TANF for every 100 families in poverty in 1996; by 2013, only 26 families obtained TANF for every 100 families in poverty.²² Some states drop entire families from the welfare rolls after the first instance of non-compliance (e.g., the refusal to accept an assigned job or identify a paternal parent) by the adult recipient.

Myth 11. Public assistance recipients migrate to states where benefits are high.

Fact. Although research by the Wisconsin's Institute for Research on Poverty concluded that some poor people migrate across state lines to receive higher benefits,²³ it failed to demonstrate that high benefits per se cause migration. According to Henry Freedman of the Center for Social Policy, "Census data on migration show that poor people move in the same direction as those who are not poor, usually toward states with jobs and booming economies rather than those offering higher welfare benefits."²⁴ Economist Bruce Meyer concluded that "The different methods all point toward the same result: there is welfare induced migration, but it is modest in magnitude."²⁵

Myth 12. Welfare spending consumes a large portion of state budgets.

Fact. After adjusting for inflation, the amount states are required to spend (at the 80 percent level required by the federal government) in 2013 was about half of the amount they spent on AFDC-related programs in 1994. This suggests that the TANF spending burden is modest compared to other state expenditures.²⁶

Myth 13. TANF benefits influence decisions relating to family composition (i.e., childbearing, marriage, divorce, and living arrangements) by encouraging women to head their own households.

Fact. Under the TANF guidelines, teenage mothers under age 18 are not entitled to benefits unless they are living at home or in a supervised facility. Although some empirical studies found a small correlation between public assistance benefits and the number of women who choose to head households or remarry, most researchers believe that the evidence does not support the view that generous benefits are responsible for high illegitimacy rates or the growth of single female-headed households. For example, although total welfare benefits have declined since 1975, the number of single female-headed households has remained relatively constant.²⁷

Myth 14. TANF families are largely made up of a mother who refuses to work.

Fact. The bulk of TANF recipients are children. Out of 4 million TANF recipients in 2005 almost 3 million were children. The TANF caseload is varied and diverse. The stereotypical TANF family with one unemployed adult constituted only 45 percent of all beneficiary families in 2013. Moreover, 17 percent of families receiving TANF had an employed adult, while 38 percent were "child-only" (there was no adult recipient). This category included families with a disabled adult receiving Supplemental Security Income (SSI), non-parent adults (e.g., grandparents, aunts), and families with an ineligible noncitizen parent.

Aid to Families with Dependent Children

Replaced by TANF, AFDC was arguably the most controversial program in the U.S. welfare system. AFDC was designed to provide financial assistance and care to needy dependent children in their own homes or in the homes of responsible caregivers. Despite these modest goals, AFDC served as a symbol in the ideological battle between liberals and conservatives and caused recipients to be victimized in two ways: (1) by their own poverty and (2) by ideologically motivated assaults upon their character and motives.

The requirement for receiving AFDC was that a child be deprived of the parental support of one parent because of death, desertion, separation, or divorce. (In the case of the AFDC-Unemployed Parent program, the criterion was deprivation of parental economic support because of unemployment or illness.)

Originally called Aid to Dependent Children (ADC), the AFDC program was part of the Social Security Act of 1935 and was designed to provide support for children by dispensing aid to their mothers. In 1950, the adult caregiver (usually the mother) was made eligible for ADC benefits.²⁸ Also in the 1950s, medical services paid for in part by the federal government were made available for ADC recipients. Beginning in the late 1950s, some critics argued that ADC rules led to desertion by fathers, because only families without an able-bodied father were eligible. In 1961, a new component was added that allowed families to receive assistance in the event of a father's incapacity or unemployment. The new program, Aid to Families with Dependent Children-Unemployed Parent (AFDC-UP), was not made mandatory for the states, and until the welfare reform act of 1988, only 25 states and the District of Columbia had adopted it. In 1962, ADC was changed to AFDC to emphasize the family unit.

By 1962, the focus of the AFDC program had shifted to rehabilitation and new policies mandated casework and treatment services. Beginning in 1972, federal policy dictated that the AFDC program be divided into social services and **income maintenance programs**. As AFDC rules became more liberalized, the number of AFDC recipients tripled from 1960 to 1970. From 1971 to 1981, that number rose another 50 percent, and in 1992 it reached 13.6 million recipients. In 1950, AFDC recipients constituted 1.5 percent of the population; by 1992, it was 5 percent.

One notorious chapter in AFDC history involved the man-in-the-house rule. This policy mandated that any woman with an able-bodied man in the house would be terminated from AFDC because, regardless of whether the man was the father of her children, it was his responsibility to support the family. This policy was manifested in "midnight raids" whereby social workers made late-night calls to determine if a man was present. Even a piece of male clothing found on the premises could justify cutting off aid. In some states, the man-in-the-house rule was extended to include rules on dating. In 1968, the U.S. Supreme Court

struck down the rule in Alabama, and the Court later reinforced its decision in a California case.²⁹

The Family Support Act (FSA) of 1988 was an important piece of welfare legislation.³⁰ The FSA (budgeted at only \$3.3 billion over a five-year period) attempted to change AFDC from an income support to a mandatory work and training program. To accomplish this, the bill established the **Job Opportunities and Basic Skills (JOBS)** program that required recipient women with children under age three (or, at state option, age one) to participate in a work or training program. By 1990, each state was to enroll at least 7 percent of its recipients (increasing to 20 percent by 1993) in a basic state education program, job training, a work experience program, or a job search program.³¹

Adoption of the AFDC-UP program became mandatory for all states, although they could choose to limit enrollment for two-parent families to 6 out of 12 calendar months in a year. Moreover, one family member of an AFDC-UP household was required to participate at least 16 hours a week in a community or make-work job in return for benefits. In addition, the FSA called for mandatory child support payments to be automatically deducted from the paycheck of an absent parent.³²

The FSA's promise soon faded. Instead of declining, AFDC caseloads actually rose by 2.1 million from 1990 to 1992. By the end of 1991, the states had spent less than half of the available federal JOBS funds. Underappreciated at the time was the FSA component that permitted states to request waivers to existing AFDC rules, an option that opened the door for the radical welfare reforms that emerged in 1996.³³ In his 1992 presidential campaign, Bill Clinton promised to "end welfare as we know it." In 1993, he appointed a high-profile working group on welfare reform that came up with far-ranging suggestions for reforming AFDC. Unbeknownst to him at the time, this rallying cry would get out of hand and lead to the most comprehensive reform of public welfare since its inception in 1935.

The Personal Responsibility and Work Opportunity Reconciliation Act of 1996

In 1996, former President Bill Clinton signed the Personal Responsibility and Work Opportunity Reconciliation Act (PRWORA) (H.R. 3734), a complex 900-page document that confused even seasoned welfare administrators.³⁴ A basic principle underlying the PRWORA was the transference of

responsibility for managing social programs from Washington to the states. Promoted early by the Reagan administration, this ideological perspective has been labeled the “devolution revolution” or the “new federalism.” Energized by the cry of “states’ rights,” the new federalism traded off long-term stable federal funding for increased state control through block grants.

The PRWORA replaced AFDC, JOBS, and the **Emergency Assistance Program** with the Temporary Aid for Needy Families (TANF) program. One of the most radical features of the PRWORA was among the least understood. Under the PRWORA, federal **entitlement** to public assistance was ended. In contrast, the former AFDC program operated under the principle of entitlement, which meant that states must provide assistance to anyone eligible under the law. This did not mean that states were required to provide something for nothing. In fact, states could have required recipients to participate in work, education, training, or job search programs as a condition for receiving aid. Under TANF, no family or child is *entitled* to assistance.³⁵ In effect, the TANF **disentitlement** rescinded the 60-year-old federal entitlement for support to poor children and families.

- **Funding.** To receive a TANF grant, each state must submit a plan to the Department of Health and Human Services (HHS). HHS determines whether the plan conforms to the law.

TANF provides lump-sum federal **block grants** for states to operate their own welfare and work programs. States must also spend their own money on programs for poor families lest they face high financial penalties. This requirement, known as “maintenance of effort” (MOE), mandates that for states to receive their full block grant, they must spend their own funds at 80 percent of their historic spending level based on 1994 spending. This requirement is reduced to 75 percent for a state that meets the act’s **work participation rate** (percentage of TANF recipients in the workforce). A state that does not maintain the required spending level risks a reduction in its block grant funding. Small adjustments in the federal block grant can occur by way of bonuses or penalties. Under limited circumstances, a state experiencing an economic downturn may qualify for additional federal

funding. Only about half the states received the contingency funding since 2008, and since the entire federal allocation was only \$2 billion a year, most states did not receive their full request since the fund runs out of money mid-year. TANF block grants to states has been frozen at \$16.5 billion since 1996.³⁶

- **PRWORA goals.** TANF and MOE funds must meet four primary goals of the PRWORA: (1) the funds must be used for cash and other assistance to low-income families so that children are looked after in their own homes or in the homes of relatives; (2) promote self-sufficiency and end welfare dependency through job preparation, work, and marriage; (3) reduce the number of out-of-wedlock pregnancies through quantifiable goals; and (4) encourage the growth and maintenance of two-parent families.

Because the four goals are so general, states have latitude to use TANF and MOE funds for a variety of services, such as cash income assistance (including supplements for working-poor families), child care, job training and education, transportation, child protection, and other services. Some states use a share of funding for other services and programs that may be only tangentially related to benefitting very-low-income families. In 2013, only about half of federal and state TANF dollars went toward core welfare reform activities.³⁷

- **Restrictions on the use of TANF funds.** States are prohibited from using TANF funds to assist certain categories of families and individuals. The most notable involves prohibiting the use of TANF funds to assist families where an adult has received assistance for 60 months or more. States can provide hardship exceptions for up to 20 percent of their caseloads. They can also choose to pay beneficiaries with their own money. The law does not impose a 60-month **time limit** on child-only families where no adult is receiving benefits. Although the PRWORA mandates a maximum five-year lifetime limit on cash assistance, it allows states to set a shorter time limit, and about a third of the states have done so.³⁸

Other restrictions include a prohibition on assisting minor parents unless they are attending school and living at home or in an adult-supervised living arrangement (subject

to limited exceptions), and a requirement to reduce or eliminate assistance if an individual does not cooperate with child support-related requirements, such as identifying the father.

The PRWORA also prohibits states from using federal TANF dollars to assist most legal immigrants until they have been in the United States for five or more years. States can, however, use MOE funds to provide benefits to recent immigrants, but less than half have taken that option. Neither federal nor state TANF funds can go toward undocumented immigrants.³⁹

States must deny cash assistance and Food Stamp Program benefits to recipients convicted of felony drug possession, use, or distribution. Other members of the family can continue to receive benefits. States may opt out of this provision or limit the period of denial by passing legislation.

- **Federal work requirements.** For states to meet federal work rules, 50 percent of families receiving TANF assistance must work for at least 30 hours a week (20 hours for single mothers with young children). Ninety percent of two-parent families on assistance must work at least 35 hours a week. States can get credit against these work rules based on declines in their TANF caseloads.

When parents participate in required work activities, the state may (but is not required to) provide child care assistance. However, a state may not reduce or terminate a family's assistance if a single parent of a child under age six refuses to comply with work requirements because of a demonstrated inability of the state to provide needed child care.

- **Detering out-of-wedlock pregnancies.** The TANF program attempted to address the dramatic increase in non-marital births (especially teen births). State TANF plans must establish goals and take action to prevent and reduce out-of-wedlock pregnancies. The act provides financial incentives to states to reduce their out-of-wedlock birth rates, while at the same time lowering their abortion rates. In addition, child support collection efforts were strengthened through a number of provisions.⁴⁰

The PRWORA allows states to impose a **family cap** that denies assistance to children born into families already receiving public assistance.

Other Reforms In addition to TANF, the PRWORA included other reforms, including strengthening child support collection efforts. Families must assign child support collection rights to the state; states must operate centralized collections units; noncustodial parents who have \$5,000 or more in arrears are subject to passport revocation; states must accept out-of-state child support orders and liens; states must have laws in effect that establish authority to withhold, suspend, or restrict drivers and professional, occupational, and recreational licenses of individuals who owe overdue support.⁴¹

Responding to concerns about intergenerational welfare dependency, states began implementing “diversion” programs to keep families whose needs could be met through other means from ever entering the welfare rolls. In response to higher effective work participation rates that followed the reauthorization of the TANF program in 2005, states added new policies and programs that divert TANF-eligible families from the TANF system. Three kinds of diversion strategies are employed: (1) lump-sum payment programs provide applicants who are employed or have a job offer with the option of accepting a one-time cash or voucher payment, in lieu of receiving TANF; (2) applicant work requirements target applicants likely to be subject to the TANF work requirements and require them to participate in work-related activities during the 30- to 45-day application certification period; and (3) temporary support programs provide TANF applicants with up to four months of assistance, which do not count toward TANF time limits or work participation rates. All but three states have implemented at least one diversion strategy, with 35 states adopting a lump-sum payment program.⁴²

As opposed to strict means-tested programs like TANF, some industrialized countries have opted for other ways to transfer money to low-income families. In countries like France, Denmark, Sweden, and Germany, family benefits are universal, while in others like the United States, Canada, Czech Republic, and Italy, they are means-tested (e.g., the EITC and Child Tax Credit). The majority of large Western industrialized countries have universal family benefits and some form of single-parent assistance. For instance, Australia has a Baby Bonus that is paid to families following the birth (including stillborn) or adoption of a child. About half of the industrialized countries require

some form of employment activity (i.e., **workfare** or welfare-to-work) as a condition for receiving public assistance benefits.

Has the PRWORA Worked?

By 2000, both Democrats and Republicans had pronounced the PRWORA a raging success. They pointed to the dramatic decline in welfare caseloads—60 percent for individuals and 53 percent for families—since the passage of the PRWORA in 1996. The number of white families on welfare showed the steepest decline, falling by 63 percent; the number of African American families fell by 52 percent; and the number of Hispanic families by 44 percent.⁴³ Nevertheless, important questions remain. First, caseloads had already begun to shrink—from a high of 5 million in 1994 to 4.5 million in 1996. These declines occurred before the PRWORA was fully implemented. Second, other factors, such as the strong labor market in the 1990s helped explain the drop. By the late 1990s, the nation was in the midst of the longest peacetime expansion in its history, with low unemployment and rising wages.

Gains in employment and wages were experienced by all groups during this period, including those with typically high rates of welfare use. Given that the PRWORA was conceived and passed by Congress in a strong economy with rising wages and low unemployment, its real test would occur in the global financial crisis that began in 2007. In 2006, there were 1.78 million families (4.2 million recipients) and 3.2 million children receiving TANF. By 2015, there were 1.3 million families, 3 million recipients, and 2.3 million children on TANF. The poverty rate in 2006 was 12.5 percent with 37 million in poverty; by 2015, that rate had increased to 14.8 percent with almost 47 million people in poverty. Not coincidentally, the 10-million increase in the poverty numbers was not reflected in higher caseloads, but in lower ones.

There are three primary avenues for leaving TANF: (1) voluntary termination because of employment, marriage, and so forth; (2) reaching the end of the federal or state time limit; and (3) being terminated for failure to comply with work or administrative rules. According to a U.S. Census report, 36.5 percent of former welfare recipients surveyed in 2009 cited that their reason for leaving TANF was due to their income becoming too high. A further 20 percent of participants stated that

they no longer needed to receive benefits. However, 12 percent of participants reported that they no longer received benefits because they had exceeded their time limit, and 5 percent had received the maximum amount of assistance they were allowed.⁴⁴

Most recipients leaving public assistance (50 to 60 percent) take jobs that pay just over minimum wage. Former TANF recipient families have not done well when it comes to average family income. In 2013, the total median monthly income for TANF families of a family consisting of at least two children ranged anywhere from \$789 (New York) a month to \$170 (Missouri) a month.⁴⁵ Because of this, many low-income families continue to receive some form of public assistance, such as food stamps. In 2014, 4.7 million people receiving food stamps were lifted out of poverty.⁴⁶

The U.S. Census Bureau data shows that there is a significant percentage of TANF families experiencing health barriers to work. In 2009, 34 percent of TANF families had a parent with a work-limiting disability, and 21 percent of TANF families received SSI for one or more months. The majority of TANF families have no private health insurance.⁴⁷

A 2015 report by the Center on Budget and Policy Priorities found that TANF is a weak safety net that has failed on multiple measures compared to the former AFDC program.

- Sixty-eight families were TANF beneficiaries for every 100 families below the poverty line in 1996; only 26 families received TANF for every 100 families in 2013.
- Since the recession, nearly all of the employment gains made by single mothers in the 1990s disappeared. The number of never-married recipients with a high school diploma or less went from 51 percent in 1992 to 76 percent by 2000, before falling to 63 percent in 2013.
- While poverty has worsened, the national TANF caseload has declined by over 60 percent since 1996. This is especially troubling given the increase in the number of families living in deep poverty, which rose from 2.7 million to 3 million between 1996 and 2013. TANF brought only 629,000 children out of deep poverty in 2010 compared to the 2 million children helped by AFDC in 1995.
- Benefit levels for TANF recipients are extremely low. In 2015, a family of three in

the median state had a benefit level of only \$429 per month; they received less than \$300 a month in 14 states. TANF benefits are below 50 percent of the federal poverty line in all 50 states, and in 16 states, it is below 20 percent of the poverty line.

- In all but two states, the inflation-adjusted value of TANF cash benefits has fallen since the PRWORA came into effect. In today's dollars, TANF cash benefits are worth 20 percent less than in 1996.
- In 2014, states spent only 8 percent of TANF and MOE funds on work activities. In contrast, they spent 16 percent of their funds on child care. About one-third of TANF and MOE funds were spent on other services such as child welfare and education (i.e., early education, after-school programs, and college financial aid). Arguably, a portion of this spending aided families with higher incomes.⁴⁸

Teenage pregnancy and never-married mothers has historically been strongly associated with public assistance and welfare dependency.

Teenage Pregnancy

By 2013, teen birthrates in the United States had reached a historic low at 26.5 births per 1,000 teens aged 15 to 19 years. Teen birthrates in the United States nevertheless remain high, exceeding those in most developed countries. For example, U.S. rates are nearly twice as high as those in England and Wales or Canada, and nine times as high as those in the Netherlands or Japan:

- In 2014, 273,105 infants were born to adolescent women 15 to 19 years old. Despite this high number, the teenage birth rate is declining. Between 2012 and 2013, teenage pregnancy decreased by 10 percent. The current rate for this age group is 24.2 live births per 10,000 women, which represents a decline from the 1991 rate of 61.8 per 1,000 women.⁴⁹
- Between 20 to 37 percent of teenage mothers have a second child within 24 months of the birth of their first child. The risk for preterm labor and low birth weight increases for second children born to teenage mothers.⁵⁰
- Teen mothers are more likely than mothers over age 20 to give birth prematurely (before

37 completed weeks of pregnancy). In 2014, preterm birth rates averaged 10.4 percent for women under age 20. Babies born too early face an increased risk of newborn health problems, long-term disabilities, and even death.

- Teen mothers are more likely to drop out of high school than those who delay childbearing. In 2010, only 50 percent of teenagers who had children received a high school diploma by the age 22, compared to 90 percent of teenagers who did not give birth until age 20 or over.⁵¹
- Teen mothers are more likely to live in poverty than women who delay childbearing, and more than 75 percent of all unmarried teen mothers go on welfare within five years of the birth of their first child.⁵²



Although teenage birthrates in the United States have declined, they still remain high, exceeding those in most developed countries.

Source: S. Borisov/Shutterstock

Out-of-wedlock births have grave economic consequences. For one, teenage mothers are twice as likely to be poor as are non-teen mothers, and a teenage mother earns only half the lifetime wage of a woman who waits until 20 to have her first child. Second, as mentioned above, a strong correlation exists between young single motherhood and welfare dependency. Teen mothers who grew up on public assistance were more than twice as likely to be dependent on welfare themselves.⁵³ Estimates are that teenage childbearing cost U.S. taxpayers \$9.4 billion in 2010.⁵⁴ The costs of developing and running teen pregnancy and prevention programs can also be substantial. In 2011, nine states reported a total cost of almost \$13 billion in outlays for teen pregnancy programs. California's "Cal-Learn case management" had the highest expenditure at \$8.85 million.⁵⁵

Although numerous programs have been developed to reduce teenage pregnancy, evaluations of these attempts have shown mixed results. One early teen pregnancy prevention program was Project Redirection. From 1980 to 1982, 805 AFDC-eligible mothers aged 17 or younger received intensive services to optimize educational, employment, and life management skills. The outcome was disappointing. Project Redirection teens fared no better than the control group in obtaining a high school diploma or GED certificate. Their weekly earnings five years later were only \$23 more than those in the control group; five years later, the household income of the control group exceeded the Project Redirection group by \$19. Two years after participation, 7 percent of Project Redirection teens were on welfare; five years later, 10 percent fewer of the control group were on welfare. Regarding childbearing, Project Redirection teens reported fewer pregnancies in the first and second years after the program; yet five years later, they exceeded the control group in their number of pregnancies and live births. Researchers concluded that "the program impacts were largely transitory."⁵⁶

From 1989 to 1992, researchers randomly assigned 2,322 poor young mothers, ages 16 to 22, either to New Chance—a program through which the women received health, education, and welfare assistance coordinated by a case manager—or to a control group that received no special services. At the 18-month follow-up, the experimental group fared worse than the control group in two important respects. First, New Chance mothers were less likely to be using contraception, were more

likely to become pregnant, and were more likely to abort their pregnancies than the control group. Second, participants were less likely to be working after entering the program; were earning less; and, during the fourth to the sixth months, were more likely to be on public assistance.⁵⁷ To compound matters, New Chance cost \$5,073 per participant, excluding child care.⁵⁸

Not surprisingly, the anti-teen pregnancy movement is highly politicized. On one side are those who support comprehensive sex education that includes information about contraception. On the other side are those who favor abstinence-only-until-marriage programs, which provide no information about contraception beyond failure rates. Proponents of abstinence-only programs believe that providing information about contraception contradicts and undermines the message.

The 1996 PRWORA had a provision (later set out in Title V of the Social Security Act) that appropriated \$250 million over five years for state initiatives that promote abstinence. From 1998 to 2003, almost a half billion dollars in state and federal funds were used to support this initiative. The findings of 11 states whose evaluations were available in 2003 showed few short-term benefits and no lasting, positive impact. Specifically, abstinence-only programs showed little evidence of sustained (long-term) impact on attitudes and intentions. Worse, they showed some negative impacts on the willingness of participants to use contraception, including condoms, to prevent untoward sexual health outcomes.⁵⁹ The apparent failure of abstinence-based programs illustrates how little policymakers understand the motivation and behaviors associated with teenage pregnancy. It also illustrates how little is known about the interpersonal, social, and cultural components that lead to teenage pregnancy.

Supplemental Security Income (SSI)

SSI is often confused with Social Security in part because they are both administered by the Social Security Agency. Despite being under the same umbrella agency, they are vastly different. Social Security is a social insurance that requires a contribution combined with 40 quarters of employment. SSI is a means-tested program that does not require a contribution or former workforce participation. SSI is a public assistance program funded by general

revenue taxes, while Social Security is a social insurance program funded largely by contributions.

The SSI program began during the Nixon presidency as a substitute for a number of state-operated disability programs. When Richard Nixon took office in 1972, he attempted to streamline the welfare system by proposing a guaranteed annual income that would replace various income maintenance and disability programs. Although Congress rejected the overall plan, various disability programs were federalized in 1972 under a new program—SSI—with the federal government taking over the operation of those programs from state governments. In 1974, there were 3.25 million SSI recipients; by 2009, that number had more than doubled to almost 8 million. Concomitantly, SSI expenditures increased from \$5 billion in 1974 to more than \$48 billion in 2010.⁶⁰ The number of SSI recipients grew by 14 percent from 2000 to 2008 alone.⁶¹ In large part, these increases resulted from the rapid growth in the numbers of disabled persons receiving SSI, a population that grew from 2.4 million in 1984 to over 6.5 million in 2010.⁶²

In 2015, the SSI program served approximately 8.3 million people and cost more than \$50 billion. The average monthly payment was about \$539.⁶³ Unlike TANF, the basic SSI payment level is adjusted annually for inflation. States have the option to supplement federal SSI benefits, and they can set their own requirements for those payments. Hence, these supplementary benefits could limit certain disabilities.

A portion of elderly people receive SSI in conjunction with Social Security. Age is not an eligibility criterion for SSI, and children may receive benefits under the disabled or blind portion of the act.⁶⁴ Among others, the following people are eligible for SSI: (1) mentally retarded individuals, (2) individuals at least 65 years old who have little or no income, (3) the legally blind, (4) disabled adults with a physical or mental impairment expected to last for at least 12 months, (5) the visually impaired who do not meet the criteria for blindness, (6) drug addicts and alcoholics entering treatment, and (7) children under age 18 who have a severe impairment comparable to that of an eligible adult.⁶⁵

Of those receiving federally administered SSI payments in 2015, approximately a little over 2 million were over the age of 65. The majority of recipients (85 percent) had some form of disability, and 6 out of 10 recipients under the age of 65 had

a mental disorder. Approximately 7 million of the 8.4 million in 2015 had a disability.⁶⁶

To qualify for SSI, an applicant must have limited resources. In 2015, SSI applicants were allowed to own resources valued at \$2,000 or less for an individual and \$3,000 for a couple (excluding, e.g., a house, a car, depending on use and value; burial plots; and certain forms of insurance). These resource limits are set by statute and have not changed since 1989.⁶⁷

Two groups of SSI recipients that have shown dramatic growth in their numbers are children with disabilities and adults with disabilities related to drug addiction and alcoholism. One reason for this growth was the Supreme Court's decision in the 1990 *Sullivan v. Zebley* case that made children eligible for SSI if they had a disability that was comparable to that of an eligible adult.⁶⁸ Children are now the fastest-growing population on SSI, and by 2015, almost 1.3 million children under age 18 had become recipients. Of those children, about two-thirds had some type of mental or developmental disorder.⁶⁹

Problems in SSI

A 2014 report by the Center on Budget and Policy Priorities identified several problems in SSI:

- SSI benefits bring an individual to 75 percent of the poverty line (80 percent for a couple). Although these benefits fail to bring individuals and families out of poverty, unlike TANF, these benefits do significantly reduce the number of those in extreme poverty.
- Bring SSI benefits to the poverty threshold. If benefits were brought up to 100 percent of the poverty threshold, the 2014 SSI benefit for individuals would have risen from \$721 to \$973 and the SSI award for couples from \$1,082 to \$1,311.
- Readjust SSI benefits. Although SSI benefits rise based on a cost-of-living adjustment, they are not adjusted to reflect real wage growth.
- Raise the asset limits. The asset limit of \$2,000 for individuals and \$3,000 for couples has not changed since 1989 and stops many potential beneficiaries from receiving service.
- Reassess SSI status for immigrants. Before 1996, legal permanent immigrants qualified for SSI like most U.S. citizens. The 1996 welfare law curtailed SSI eligibility immigrants unless

they fell into one of several groups: lawful immigrants who entered the United States by 1996; refugees and other humanitarian immigrants who entered after 1996 can only receive SSI on a temporary basis; and legal immigrants who entered after 1996 and have earned 40 quarters of qualifying Social Security coverage.⁷⁰

Stringent eligibility requirements and red tape have kept many people from applying for and obtaining SSI benefits. For example, SSI cases are reviewed every three years (a process that usually involves a medical review), and “continuing disability reviews” may be required. Some critics believe that the federal government has purposely made entry and maintenance in SSI difficult to discourage participation. The street wisdom suggests that it is normative to be rejected for SSI at least two times, which has led some potential recipients to hire attorneys who specialize in SSI cases. These lawyers often operate on a contingency fee basis since successful claims on SSI benefits can be paid retroactively, which can translate into a significant sum of money.

General Assistance

General assistance (GA) programs, also known as General Relief programs in some states, are cash and in-kind assistance programs financed and administered entirely by the state, county, or locality in which they are located. These programs of last resort are designed to meet the short-term or ongoing needs of the severely poor without children, who are ineligible for (or awaiting approval for) mainstream federally funded programs, such as TANF or SSI. In many ways, GA programs serve some of the poorest of the poor.

Only 11 states with GA programs provide benefits to adults without a disability—the other states require beneficiaries to be unemployable due to a physical or mental condition. Remarkably, some states using the SSI disability criteria require GA recipients to sign an agreement requiring them to repay the state once they begin receiving SSI payments.⁷¹ There are no federal programs for childless adults who are poor but not disabled.

GA programs have weakened over the last 30 years, and the number of states with programs has dropped from 38 in 1989 to 26 in 2015. Benefits are lower in nearly every state compared to 1989.

While some states have raised benefit levels, they are generally insufficient to make up for purchasing power lost since the 1990s. Although income eligibility varies across states, the majority of state GA programs limit assistance to only those with incomes less than half the poverty level.⁷² In virtually every state, maximum GA benefits are below 50 percent of the poverty line; in half of those states, they are below 25 percent of the poverty line. For instance, maximum benefit levels in New Jersey are \$140 a month for a recipient deemed employable and \$210 per month for a recipient that is unemployable.⁷³

Seven states with GA programs limit the length of assistance and three states (Colorado, Delaware, and New Jersey) have lifetime caps. Three other states (Maryland, Utah, and Nevada) limit benefit claims to a certain number of months per year. Employable beneficiaries in California can only receive benefits for nine months a year.⁷⁴

Although states are generally responsible for GA, some devolve responsibility and administration to their counties. For instance, GA in each one of California’s counties is established and 100 percent funded by its own board of supervisors. Hence, benefits, payment levels, and eligibility requirements vary among each of the 58 counties.⁷⁵

While the 1935 Social Security Act covered many groups of vulnerable citizens, it left poor unemployable (but not disabled) and childless adults to the largesse of the states. Historically, most states did not live up to their social welfare responsibilities, which is why that was taken away from them and lodged in federal legislation. More than 80 years later, the same refusal of states to assume social welfare responsibility continues in their refusal to serve poor adults, or in their provision of extraordinarily paltry benefits to some of society’s most vulnerable groups, including workers who have exhausted their unemployment benefits and face severe hardships.

Trends and Issues in Public Assistance

The Transformation of Welfare Policy into Labor Policy

The transformation of public assistance policy into labor policy reached fruition with the passage of the PRWORA, essentially a labor policy clothed in

welfare terminology. AFDC had been a sore spot for conservatives since its inception. This antagonism persisted even though in 1995 combined AFDC and food stamps accounted for only 3 percent of the federal budget compared to 22 percent for Social Security, 18 percent for defense, and 10 percent for Medicare.⁷⁶ The real concern about public assistance was not about excessive spending; instead, it was a reaction to AFDC as a symbol of governmental responsibility to provide cash proxies for labor market earnings. The very existence of AFDC was a thorn in the side of economic conservatives since it implied that the marketplace was incapable of meeting the needs of all Americans.

A unique confluence of events emerged in 1996 to complete the transformation of public assistance into labor policy. Frustrated by repeated failures of federal welfare-to-work efforts, the newly elected conservative 104th Congress was determined to not repeat past mistakes. First and foremost was the disentanglement of public assistance by limiting the fiscal responsibility of government to provide income support beyond a five-year lifetime cap. Under the PRWORA, the only option for those who exhausted their benefits was labor market participation or penury.

The PRWORA was passed in a period of robust economic growth. By 1996, the unemployment rate had dropped to 4.5 percent and the economy was growing at more than 2.5 percent a year.⁷⁷ The low unemployment rate combined with a strong economy meant that the 138-million-member U.S. labor force could easily absorb the 4.2 million AFDC recipients without driving down wages or increasing unemployment. The economic giddiness of the middle 1990s reinforced the conservative belief that a job existed for anyone that wanted it. It was in this climate that the PRWORA was passed.

Removing the federal responsibility for providing long-term cash assistance to the poor shifted the problem of poor support away from social welfare and into labor policy. The PRWORA represented the culmination of the long-standing conservative goal of deracinating public assistance: When the poor exhaust time-limited public assistance benefits they become a labor market rather than a welfare problem. With that change, U.S. public assistance policy became a short-term, transitional step in the march toward the full labor market participation of the poor.

The conservative push toward labor policy was not entirely without merit. One would be

hard-pressed to argue that a well-paying job with full benefits is not the best antipoverty program. Moreover, Sweden—historically considered a progressive welfare state—has frequently used labor policy as a proxy for aggressive public assistance programs.⁷⁸ The push toward workforce participation is also congruent with current labor market trends for women. For instance, in 1955 only 18 percent of all U.S. mothers with children under six were in the labor force. By 2010, that number had risen to 64 percent. By 2011, almost 61 percent of U.S. mothers with children younger than three years old were in the labor force full or part-time.⁷⁹ Given these trends, welfare advocates were hard-pressed to justify non-employment-based public assistance programs when the majority of all mothers with children under age six were in the labor force. Despite these conservative arguments, the absence of sound federal labor policy, the historic lack of guaranteed health care insurance, and few workplace protections resulted in most former recipients going from the frying pan into the fire. Many former recipients ended up in secondary labor market jobs where they faced an even more uncertain economic future than under AFDC.

Despite some positive gains, the substitution of labor policy for public assistance policy has led to serious problems. For one, the absence of sound labor policy rewards low-wage employers while punishing low-income workers forced into poorly paid dead-end jobs that provide little, if any, benefits. Specifically, low-wage employers are rewarded by not being required to provide employees with a minimum number of hours, benefits, or employment perks. These employers are subsidized by not having to pay the requisite salary to support a worker and his or her family. In effect, low-wage employers are subsidized by taxpayers through the Earned Income Tax Credit (EITC), child tax credit, food stamps, and Medicaid, which partially makes up the shortfall between low salaries and the real cost of living. Moreover, compelling former recipients to become engaged in the workforce using a lifetime benefit cap did not ensure permanent labor force attachment (as seen during the 2007 to 2008 recession), especially in a weak economy.

The transition from public assistance to labor policy does not bode well for the “hard to employ,” a group of recipients that have significant barriers to employment and difficulty in finding and sustaining work. Some of these barriers include substance abuse, physical disabilities, domestic

violence, learning disabilities, mental health issues, language problems, chronic health problems, and multiple other barriers.⁸⁰ No single policy will ensure that former recipients maintain a viable attachment to the workforce. Nor can any single measure compensate for the lack of a comprehensive labor policy for low-income workers. David Stoesz sums up the dilemma: “Welfare reform that offers the welfare-poor an opportunity to become the working-poor is no *real* reform at all. The challenge that remains is to devise policies that will accelerate the upward mobility of welfare families so that they can partake in the American dream.”⁸¹

Welfare to Work (Workfare)

One of the knottiest issues in public assistance has been the view that recipients should be moved off the rolls and into work as quickly as possible. Better yet, they should go right into work thereby bypassing the welfare rolls altogether. As noted above, TANF is predicated on the long-standing belief that recipients should be moved into private employment posthaste.

Workfare programs have been a constant feature of the welfare landscape since 1967, when new AFDC amendments were added to pressure recipient mothers into working. As part of those new rules, work requirements became mandatory for unemployed fathers, mothers, and certain teenagers. AFDC recipients who were deemed employable and yet refused to work could be terminated. Partly because of a lackluster federal commitment, the performance of work programs was generally disappointing. In addition, many states were reluctant to enact mandatory job requirements because they believed that enforcing them would prove costlier than simply maintaining families on public assistance.

Often, former welfare recipients who get jobs join the ranks of the working poor. If the reality of welfare-to-work diverges from the rhetoric of welfare reform, it is because of the assumption that TANF mothers are so welfare dependent that they have no experience with the labor market. In fact, many mothers have worked and have, as a result, come to see welfare benefits as a form of unemployment or underemployment assistance. To explain the relationship between work and welfare, labor economist Michael Piore split workers into primary and secondary labor markets: Workers in the primary labor market hold down salaried jobs that include health and vacation benefits, are full time,

and incorporate a career track; workers in the secondary labor market work for hourly wages—often at the minimum wage—without benefits in jobs that are part time or seasonal and are not part of a career track.

The success of welfare reform would have been to catapult TANF mothers into the primary labor market. The sticking point is that substantial investments are necessary to achieve such program performance. But to do so creates two problems: First, a “moral hazard” emerges as welfare beneficiaries become recipients of benefits unavailable to the working poor not on welfare. Second, a “political hazard” is created for any elected official who states a willingness to support welfare recipients over the working poor. In such circumstances, the prudent politician favors the most expedient option—push welfare recipients into the labor market and celebrate doing so with paeans to the work ethic. Welfare reform is therefore limited to elevating the welfare poor only up to the level of the working poor.

TANF work-to-welfare programs fail to take into account barriers to employment. A study of welfare recipients by Sandra Danziger and her colleagues found high levels of barriers to work, such as physical and mental problems, domestic violence, and lack of transportation, but relatively low levels of other barriers, such as drug or alcohol dependence or a lack of understanding of work norms. Their study also found that recipients commonly have multiple barriers and that the number of barriers is strongly and negatively associated with employment status. Almost two-thirds of the study respondents had two or more potential barriers to work, and more than one-quarter had four or more.⁸²

Welfare Behaviorism

Behavioral Poverty The PRWORA was predicated on a form of **welfare behaviorism**—an attempt to reprogram the behaviors of the poor. Despite data demonstrating the marginal economic benefits of making welfare conditional, conservatives effectively leveraged a moral argument that public policy should change the behavior of the welfare poor. By the mid-1980s, conservative theorists had arrived at a new consensus on poverty; namely, that although the liberally inspired public assistance programs might once have been appropriate for the “cash poor,” they were counterproductive for the “behaviorally poor.”⁸³ As poverty programs expanded, conservatives contended, the social dysfunctions

of the behaviorally poor metastasized: Beginning as teen mothers, women dominated family life, ultimately becoming generationally dependent on welfare; young men dropped out of school, failed to pursue legitimate employment, and resorted to sexual escapades and repetitive crime to demonstrate prowess; children lacking adult role models of effective parents promised to further populate the underclass.

Conservatives differed in how to respond to behavioral poverty. In *Losing Ground*, Charles Murray suggested “scrapping the entire federal welfare and income support structure for working-aged persons.”⁸⁴ Lawrence Mead argued for making public aid contingent on conventional behaviors, particularly work.⁸⁵ Eventually, both prescriptions were incorporated in welfare policy.⁸⁶ Die-hard conservatives justified the termination of entitlement under TANF by adopting George Gilder’s view that the poor needed “the spur of their own poverty.”⁸⁷ “Compassionate conservatives” found a rationale in “tough love.” Either way, conservative welfare reformers conceded that terminating benefits would probably worsen deprivation but argued that stopping benefits was necessary.

Personal and Parental Responsibility The welfare behaviorism inherent in the PRWORA reflects a belief in the importance of personal and parental responsibility. This philosophy harks back to supposedly traditional “main street” values of self-reliance, independence, and individual responsibility. It also includes a de facto belief in the limited role of government.

The view of parental responsibility reflected in the PRWORA focuses on several themes: (1) fathers have a responsibility to financially support their children; (2) mothers have a responsibility to disclose paternity before receiving public assistance (a policy called **establishment of paternity**); (3) mothers who have physical custody of their children have a responsibility to financially provide for them through work efforts; (4) custodial parents are responsible for ensuring that their children receive educational opportunities; and (5) custodial parents are responsible for providing basic public health protection for their children, including immunizations. Significant controversy surrounds the question of whether attempts at engineering appropriate social behaviors are successful or whether they simply function as punishment.

Another issue underlying the welfare reforms involves (re)marriage. According to several studies, many—if not most—mothers voluntarily leave public assistance due to a change in marital status.⁸⁸ Yet public assistance programs punish marriage in two ways: (1) how they treat a married couple with children and (2) by how they treat families with stepparents. First, a needy two-parent family with children is less likely to be eligible for aid than a one-parent family. Although the non-incapacitated two-parent family can apply for TANF, many states require work expectation and time limits that make the program inaccessible to many poor families. Second, a stepparent has no legal obligation to support the children of his or her spouse in most states. Nevertheless, TANF cuts or limits benefits when a woman remarries by counting much of the stepparent’s income when calculating the family’s countable income, thus jeopardizing the mother’s eligibility status.⁸⁹ This creates a strong economic disincentive for a low-income woman to marry a low-earning male.⁹⁰

Conclusion

The U.S. welfare system has undergone a dramatic two-parent transformation over the last 20 years marked by several important themes: (1) the conversion of public assistance policy into labor policy, (2) the conversion of public assistance policy into tax policy, and (3) the increased privatization of social welfare services. By converting AFDC into workfare through TANF, the PRWORA eliminated the boundary between public assistance and labor policy and thus became a feeder system to supply the secondary labor market with manpower. The result is fewer protected islands left for those who cannot compete in the labor market.

The conversion of public assistance policy into labor policy is shored up by tax policy through the expansion of EITC benefits, the only public assistance program that has been purposely enlarged in the last 20 years. By expanding EITC benefits through the tax code while at the same time eliminating the entitlement to AFDC, Congress shifted the focus from the nonworking poor to the working poor, a group more marginally acceptable to conservatives and the larger society. If the past trajectory continues, public assistance benefits will be increasingly routed through tax policy for low-income workers rather than through direct cash or in-kind subsidies for the nonworking poor.

Privatized social services have encroached upon most welfare domains, including health care, public education, mental health and chemical dependency services, nursing home care, corrections, and child care. Many of these markets are already saturated by the influx of corporations seeking to increase their profits and their corporate reach.

All developed economies have some form of income support for poor families and individuals. Within the range of income transfer approaches, some countries opt for separate means-tested programs like TANF; others choose a universal and more far-ranging approach. Still other industrial countries like France use a Guaranteed Minimum Income (GMI), which guarantees that all families will have sufficient income provided they meet certain conditions, such as citizenship and/or a means test. To receive benefits, other countries require recipients to be in the labor market and/or perform community services. While the United States is one of the few industrialized countries without a GMI, the tax system—largely through the EITC and other tax-related programs—provides some assistance to low-income working families.

Several U.S. approaches to public assistance have been adopted by other countries. Notably, welfare-to-work policies and strict means tests were taken on board in the United Kingdom, Australia, Ireland, and other countries faced with fiscal difficulties. Given the ongoing economic lethargy of many European nations, it is likely that more countries will opt to limit public assistance and other social spending.

This is clearly a difficult time for the American welfare state. Even after huge expenditures on social welfare services, basic human needs remain unmet and poverty numbers remain high. Given the present economic trend—in which vast numbers of service jobs are produced, many of which are part time, have few if any benefits, and pay the minimum wage—long-term welfare benefit packages will likely be required for an increasing number of citizens. How much help these income benefit packages will contain, and at what cost, will be a matter for future public policy discussions.

DISCUSSION QUESTIONS

1. Public assistance programs are arguably the most controversial programs in the U.S. welfare state. Critics frequently lambaste them for encouraging everything from welfare dependency to teenage

pregnancy. Supporters argue that public assistance programs are poorly funded and barely allow recipients to survive. Why is public assistance so controversial? What, if anything, can be done to make the TANF program less controversial?

2. Numerous myths have arisen around the TANF program. In your opinion, what myths have been the most harmful to the programs and to recipients? Why?
3. Since the 1970s, most strategies for reforming public assistance have revolved around implementing mandatory work requirements. This strategy was evident in the programs of Presidents Carter and Reagan and was the centerpiece of the 1988 Family Support Act. The TANF program mandated that benefits should last for a maximum of five years, after which a recipient would be ineligible for continuing public assistance. Is establishing a mandatory work requirement a viable strategy for reforming public assistance? Why? Why not? What would be a better strategy for reforming public assistance?
4. TANF benefit levels vary widely from state to state because the federal government has refused to establish a national minimum benefit level. Moreover, there is little federal pressure on states to increase benefit levels and thereby curtail the erosion of public assistance benefits that has taken place since the 1970s. Should the federal government establish a minimum national benefit level for TANF and compel states to meet that level? If not, why not? If you agree, what should that level be?
5. Public assistance programs are increasingly being designed to change the behavior of poor recipients. Can and should the behavior of poor people be changed through social policy legislation? Will this legislation work? If not, why?
6. Although administered by the Social Security Administration, the SSI program carries a stigma similar to public assistance programs like food stamps or the TANF. Why does this stigma exist? What can be done to diminish it?

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The American Health Care System



Source: Rocketclips/Fotolia

This chapter examines the U.S. health care system—specifically, the organization of medical services; key governmental health programs such as Medicare and Medicaid; the crisis in health care, including attempts to curb health care costs; the large numbers of uninsured people; the impact of the American Medical Association on health care; and that of managed care in the American health care system. The chapter also examines gun violence (a major health policy issue) and surveys various proposals designed to ameliorate the problems in U.S. health care. The chapter also investigates the organization of medical services in the United Kingdom, Canada, and Australia.

The Uninsured

Health care in the United States is marked by several contradictions. According to Census Bureau data, the number of people *with* health insurance rose to almost 87 percent in 2014; however, more than 13 percent or 33 million people were *without* health. Of that number, 8.6 percent of all children were uninsured. The percentage of the nation's population covered by government health insurance programs (e.g., Military, Medicaid, and Medicare) was at 40 percent in 2014.¹ The medically uninsured have the following characteristics:

- The uninsured are more likely than the privately insured to not receive needed medical care and more likely to need hospitalization for avoidable acute conditions like pneumonia or uncontrolled diabetes. They are also more likely to rely on emergency room services or to have no regular source of care.² Millions of Americans seeking medical care are turned away each year because they cannot pay; millions more forego preventive services.³ This exists even though every major city has at least one major medical center.⁴
- Uninsured children have a higher instance of developmental delays than those with health coverage.⁵
- They are more likely to put off seeking care, to not receive care when needed, and to not fill a prescription or get treatment because of the expense. In 2014, 27 percent of uninsured adults aged between 19 and 25 delayed seeking medical care due to the expense. This is in comparison to 7 percent of adults within the same age bracket who had private health insurance.⁶

- They are more likely to be contacted by a collection agency for overdue medical bills.
- Uninsured adults hospitalized for heart attacks are 25 percent more likely to die while in the hospital than privately insured adults. Even after controlling for the severity of the injury, uninsured adults hospitalized for a traumatic injury are more than twice as likely to die in the hospital as insured adults.
- The diagnosis of a serious new health condition, including cancer, diabetes, heart attack, chronic lung disease, or stroke, reduced the wealth of uninsured households by 20 percent. Insured households with a similar diagnosis suffered a 2 percent decline in overall wealth.
- Insured households paid about \$26,957 in total medical spending after the diagnosis of a serious new health condition; uninsured households paid \$42,166.
- Americans who lack health insurance cost the economy billion a year in lost productivity.⁷ The cost of medical care for the uninsured totaled almost \$85 billion in 2013.⁸

The Organization of Medical Services

Most health care costs in the United States are paid for by private insurers, public plans, and the direct public provision of health care. Only 28 percent of health care costs was paid for directly by consumer households in 2009, a significant decline from the 1987 rate of 37 percent. Although the dominant form of health care coverage in the United States is private insurance, employer-based health care accounted for only 21 percent of national health care spending in 2009. Conversely, government spending has increased significantly, from 32 percent in 1987 to 44 percent in 2009.⁹

Medical services in the United States consist of five major components:

1. Physicians in solo practice (e.g., traditional physicians who may employ a nurse and receptionist), which is becoming increasingly rare in light of group practices and managed care.
2. Group practice settings, including physicians that share facilities. This setting is becoming common as physicians are forced to pool resources—capital, equipment, office staff,

Spotlight 12.1

THE NATIONAL CENTER FOR HEALTH STATISTICS

The National Center for Health Statistics (NCHS) is a key resource of information about the health of Americans. As the principal health statistics agency in the United States, the organization compiles statistical information to guide action and policies to improve the health of people living within the United States. The information provided by NCHS helps to:

- Document the health status of the population and of important subgroups
- Identify disparities in health status and use of health care by race/ethnicity, socioeconomic status, region, and other population characteristics
- Describe various experiences with the health care system
- Monitor trends in health status and health care delivery
- Identify health problems
- Support biomedical and health services research
- Provide information for making changes in public policies and programs

Source: National Center for Health Statistics. Retrived from http://www.cdc.gov/nchs/data/about/nchs_50th_brochure.pdf.

Evaluate the impact of health policies and programs. To learn more about NCHS, go to its website at www.cdc.gov/nchs.

- Children under 18 in poverty were more likely to be uninsured—15.4 percent in 2010 compared to 9.8 percent of all children. Adults between the ages of 19 and 44 made up approximately three-quarters (75 percent) of the uninsured.
- Young adults (19 to 25 years old) were the least likely of any age group to have health insurance in 2010. Nearly 30 percent of uninsured persons were in the six-year age bracket.
- The poor are three times more likely to be uninsured as those who are not poor. Just over one-quarter (26.9 percent) of people in households with annual incomes under \$25,000 had no health insurance in 2010 compared with only 8 percent for those with incomes over \$75,000.
- The uninsured rate for African Americans in 2015 was 12.7 percent, for non-Hispanic whites was 9.1 percent, and for Hispanics was 20.9 percent.
- The proportion of the foreign-born population without health insurance (21 percent) was about two-and-a-half times that of the native population (79 percent) in 2014.
- The South (19.1 percent) and West (17.9 percent) have a higher uninsured rate than either the Northeast (12.4 percent) or the Midwest (13 percent).
- The number of people who received health insurance coverage through their employers rose slightly from 55.3 percent in 2010 to 57 percent in 2015, while the number of people covered by government health insurance programs (e.g. Medicare and Medicaid) was 95 million or 31 percent.¹⁰ The relatively high number of uninsured Americans is not surprising, given that family employer-based health insurance premiums cost an average \$17,545 annually in 2015—a high cost for a family trying to make ends meet, especially in a deep economic recession.¹¹ Purchasing affordable, accessible insurance is a particular challenge for many older people not eligible for Medicare, for workers in transition between jobs, and for small businesses and their employees. For example, premiums for COBRA (transitional health insurance allowed a worker when they terminate employment) average almost \$1000 a month for family coverage and \$400 for individual coverage, a huge expenditure given the average monthly unemployment check.¹²

and so forth—in order to compete in the health care marketplace. Group outpatient settings may also include health maintenance organizations (HMOs), physicians in industrial Employee Assistance Plan (EAP) settings, or doctors operating under university auspices. During the past few decades, physicians have increasingly worked in group practices or other organized settings.¹³

3. Physicians employed in corporate-owned for-profit clinics or in nonprofit clinics.
4. Hospitals—private, nonprofit, or public.
5. Public health services delivered on the state, local, regional, national, or international level. These services include health counseling, family planning, prenatal and postnatal care, school health services, disease prevention and control, immunization, referral agencies, STD (sexually transmitted diseases) services, environmental sanitation, health education, and maintenance of indexes on births, deaths, and communicable diseases. Government-sponsored health services include the Veterans Administration Hospitals (the largest network of hospitals in the United States), Community and Migrant Health Centers, and so forth.
6. Corollary health services, which includes home health services, physical rehabilitation, group homes, nursing homes, and so forth.

Major Public Health Programs: Medicare, Medicaid, and S-CHIP

Health care spending is the second fastest-growing component of the federal budget, overshadowed only by the growth in the public debt.¹⁴ In 2014, 54 million people were enrolled in Medicare (a 183 percent increase over 1966), and about 65 million (children accounted for 45 percent) were covered by Medicaid.¹⁵

Medicare

After Social Security, Medicare is the largest social insurance program in the United States with expenditures of \$748 billion in 2014.¹⁶ It is also the largest public payer of health care, financing close to 20 percent of all health care spending. Medicare was added to the Social Security Act in 1965

and was designed to provide elderly people with prepaid hospital and optional medical insurance. The modern Medicare system is composed of four parts: compulsory Hospital Insurance (HI), known as Part A; Supplemental Medical Insurance (SMI), known as Part B; the Medicare Advantage program, known as Part C; and the Medicare Prescription Drug, Improvement, and Modernization Act of 2003, known as MMA or Part D. Although traditionally consisting of two parts (HI and SMI), Part C (established by the Balanced Budget Act of 1997) expanded beneficiaries' options for participation in private-sector health care plans. The MMA or Part D was added in 2003.

Surrounded by controversy, the MMA was signed into law in 2003. As a 700-page document, this complex bill proved difficult for Medicare beneficiaries to understand. Apart from prescription drug coverage, Part D also included other changes for beneficiaries, including premium hikes. Like SMI, participation in the MMA is voluntary.

Part D provides subsidized access to prescription drug insurance coverage on a voluntary basis, upon payment of a premium. Some financial assistance is available for low-income enrollees. People may enroll in either a stand-alone prescription drug plan (PDP) or an integrated Medicare Advantage plan that offers Part D coverage.¹⁷

The standard benefits for the MMA include:

- The Part D monthly premium varies by plan, with some higher-income consumers paying more.¹⁸
- Beneficiaries can enroll in a stand-alone prescription drug plan for Part D (PDP) or a Medicare Advantage plan that includes Part D coverage. Most beneficiaries choose to enroll in an alternative Part D coverage plan. These alternative plans can provide lower or no deductibles, lower set payments for drugs and additional coverage for gaps in cost.¹⁹
- Medicare beneficiaries receive preventive benefits, including an initial routine physical examination, cardiovascular blood screening tests, and diabetes screening and services.
- Medicare provides additional assistance to beneficiaries who qualify based on low incomes and limited assets. For Social Security beneficiaries enrolled in Part B, a “hold harmless” provision was introduced whereby the premium cannot be increased beyond Social Security cost-of-living increases. Since there

was no Social Security cost-of-living increase for 2015, beneficiaries continued to pay the 2014 premium amount.²⁰

- MMA plan providers are permitted to offer an alternative benefit design provided it is equivalent and does not raise the Part D deductible or out-of-pocket limit. Plan providers are also required to provide drugs in each therapeutic category but have flexibility to establish preferred drug lists. In addition, companies offering drug plans can change formularies at any time, while a beneficiary can only change providers once a year. Plans may utilize a preferred network of pharmacies to reduce beneficiary cost-sharing or offer supplemental benefits for an additional premium. Although Medicare has guidelines for the type of drugs the plans must cover, they do not require all plans to offer the same drugs. Beneficiaries therefore need to determine whether a plan covers the drugs they need.

Critics charge that the MMA may actually increase drug costs by prohibiting Medicare from using its potentially huge purchasing power to negotiate lower drug prices. Although private plan providers seek discounts for enrollees, they lack Medicare's potential purchasing clout. This stands in contrast to the Veterans Administration, which has successfully negotiated lower prices with drug companies. The "no negotiation" clause increases program costs, which in turn, adds to the overall costs of Part D. The bill also prohibits consumers from benefiting from cheaper drugs sold in Canada and Mexico. Under the Medicare legislation, drugs can only be reimported from Canada, and then only if the Secretary of Health and Human Services certifies that the re-importation is necessary, safe, and would significantly reduce costs. Not only are there no mechanisms to effectively control rising drug costs, but the new drug benefit means much larger sales volume.²¹

Perhaps the most important part of the MMA is a little-noticed component called health savings accounts (HSAs), which offers a tax-free shelter for those with high-deductible insurance. When money is put into an HSA, it is not taxed. Nor is it taxed when the money is removed to pay for medical costs. Critics charge that this adds to the federal budget deficit (i.e., it functions as a tax shelter), while the HAS removes the owners of these accounts from the shared risk that is at the core

of the health insurance system. Specifically, many younger and healthier workers will opt for the HSA, leaving the older and sicker employees in traditional health insurance plans. Drawing healthier workers out of health care plans will likely raise the cost of health insurance for those that remain. Conservatives claim HSAs encourage people to more closely monitor their health care spending, and it therefore brings down medical costs.²²

Medicare Coverage HI or Part A is provided free to persons age 65 or over who are eligible for Social Security or Railroad Retirement benefits. Workers and their spouses with a sufficient period of Medicare-only coverage in federal, state, or local government employment are also eligible beginning at age 65. HI is a compulsory **inpatient care** (hospital) insurance plan (it also includes some nursing and **home health care**) with premiums derived from a payroll tax that is part of the Social Security deduction.²³ The following health care services are covered under the HI program:

- Inpatient hospital, including the costs of a semi-private room, meals, regular nursing services, operating and recovery rooms, intensive care, inpatient prescription drugs, laboratory tests, X-rays, psychiatric hospitals, inpatient rehabilitation, long-term care hospitalization, and all other medically necessary services and supplies.
- Skilled nursing facility (SNF) care is covered if the treatment occurs within 30 days of a hospital stay of three days or more, and if it is deemed a medical necessity. Covered services are similar to those for hospitalized patients but also include rehabilitation services and medical appliances.
- Home health agency (HHA) care may be provided for a home-bound beneficiary if deemed medically necessary. Certain medical supplies and durable medical equipment may also be provided requiring a co-payment. Full-time nursing care, food, blood, and drugs are not provided as HHA services. Home health agency (HHA) care is covered by both HI and SMI and requires no co-payment or deductible.
- Hospice care is available to terminally ill persons with life expectancies of six months or less who refuse standard Medicare benefits. Hospice care includes pain relief, supportive

medical and social services, physical therapy, nursing services, and symptom management. Medicare also covers treatment for conditions unrelated to the terminal illness. Beneficiaries pay no deductible for the hospice program, but they do pay a small coinsurance for drugs and inpatient respite care.²⁴

By paying a monthly premium, U.S. citizens (and approved legal aliens) over age 65 and all disabled persons entitled to HI are eligible to enroll in the SMI program. Almost all persons entitled to HI enroll in SMI, which covers the following²⁵:

- Physicians' and surgeons' services, including some covered services furnished by chiropractors, podiatrists, dentists, and optometrists. Also covered are services provided by non-Medicare-approved health care practitioners such as nurses in collaboration with a physician, clinical psychologists, clinical social workers (other than in a hospital or skilled nursing facility), and physician assistants.
- Services in an emergency room or outpatient clinic, including same-day surgery, and ambulance services.
- Laboratory tests, X-rays, and other diagnostic radiology services, as well as certain preventive care screening tests.
- Ambulatory surgical center services in a Medicare-approved facility.
- Most physical and occupational therapy and speech pathology services.
- Outpatient rehabilitation facility services and mental health treatment in a partial hospitalization psychiatric program.
- Radiation therapy, renal dialysis, and certain organ transplants.
- Approved durable medical equipment for home use, such as oxygen equipment, wheelchairs, prosthetic devices, surgical dressings, splints, and casts.
- Drugs that cannot be self-administered.²⁶

Gaps in Medicare Coverage Beneficiaries without private supplemental coverage pay for charges not covered by Medicare and for various cost-sharing aspects of both HI and SMI. These charges may be paid by beneficiaries out-of-pocket, by a third party (an employer-sponsored retiree health plan or private "Medigap" insurance), or by Medicaid, if the person is eligible. "Medigap" insurance refers

to private insurance plans that cover most of the Medicare co-pays.

For HI hospital care, the beneficiary co-pay includes a one-time deductible at the beginning of each benefit period for the first 60 days of each instance of inpatient hospital care. If inpatient care is needed beyond the 60 days, additional coinsurance payments (\$289 a day in 2012) are required through the 90th day of a benefit period. Each HI beneficiary also has a one-time lifetime reserve of 60 additional hospital days that may be used when the covered days have been exhausted. Although Medicare covers the first 20 days of skilled nursing facility (SNF) care, a co-payment is required for days 21 to 100. Medicare's obligation ends after 100 days of SNF care per benefit period. Home health care has no deductible or coinsurance payment.²⁷

Reimbursements to Providers Medicare payments for most inpatient hospital services are made under a reimbursement mechanism known as the **prospective payment system (PPS)**. Under PPS, a predetermined amount is paid for each inpatient hospital stay based on a diagnosis-related group classification. In some cases, the payment to the hospital is less than the actual costs, while in other cases it is more. The hospital absorbs the loss or makes a profit. Adjustments are made for unusual or costly hospital stays. Payments for skilled nursing care, home health care, inpatient rehabilitation, long-term hospital care, psychiatric hospitalization, and hospice care are made under separate PPSs.²⁸

If a doctor or medical supplier agrees to accept the Medicare-approved rate as payment in full (which they often refuse to do), they may not request any additional payments from the beneficiary. If the provider does not take the assignment, the beneficiary is charged for the excess (sometimes paid by Medigap insurance). Limits exist on the excess that doctors or suppliers can charge. Medicare reimbursements to health care providers are subject to maximum payments by level of service, which are often less than some physicians will accept. In these cases, patients pay the difference between the physician's charge and the Medicare reimbursement. Patients are entitled to choose their own doctor, and consequently may elect to go to a "participating physician" who will accept the Medicare rate as payment in full. However, due to paperwork and limited reimbursement, the number of physicians who choose to participate is lower than patient demand.²⁹

Although Medicare provides important services, the gaps in coverage are extensive, which is why many beneficiaries opt for HMOs or supplement Medicare with private **Medigap** insurance.

Funding Medicare finances are handled by two trust funds in the U.S. Treasury, one for the HI program (Part A) and the other for SMI (Parts B and D). The HI program is financed primarily through a mandatory payroll tax for all employed and self-employed people. In 2015, the HI tax rate was 1.45 percent of earnings (paid by each employee with a matching amount paid by the employer) and 2.90 percent for the self-employed. An additional HI tax of 0.9 percent was added in 2013 for those with annual incomes above \$200,000. Unlike Social Security, there is no earnings cap on taxable wages. The SMI program is financed by beneficiary payments and contributions from general tax revenues. Beneficiary premiums are generally set at a level that covers 25 percent of the average expenditures for aged beneficiaries; general fund payments are calculated to match estimated program costs for the next year.³⁰

Total Medicare expenditures more than doubled from 1989 to 1998 and are rising faster than the wages on which the payroll tax is based.³¹ Medicare's annual cost in 2011 equaled 3.7 percent of the gross domestic product (GDP) but is expected to reach almost 6.7 percent by 2086. In turn, the projected date of the HI trust fund exhaustion is 2024. Part B of the SMI trust fund and the MMA are both expected to remain adequately funded into the future because current law automatically sets financing to meet the next year's expected costs (not true for the HI program). The outlook for Medicare improved substantially in 2010 because of program changes made in the 2010 Patient Protection and Affordable Care Act (ACA). As a result of the ACA, the HI Trust Fund is expected to remain solvent longer than projected. The ACA is also expected to substantially reduce costs for the SMI program. Despite this potential improvement in Medicare's short-term future, further reforms will be needed, given the retirement of the baby boomer generation that began in 2010.³² See Figure 12.1 for Medicare program costs as a percentage of the GDP.³³

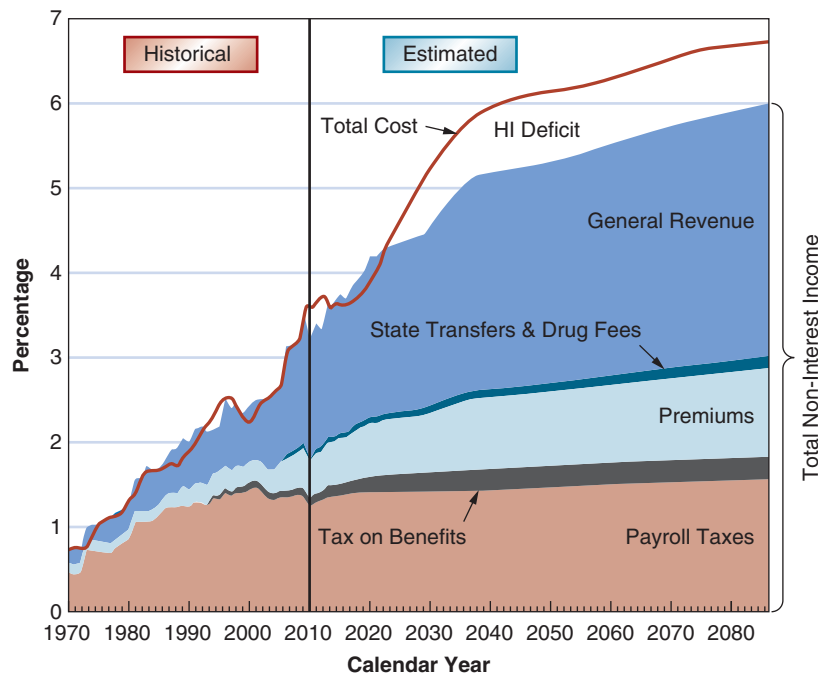


FIGURE 12.1 Medicare Cost and Non-Interest Income by Source as a Percentage of GDP

Source: Social Security Administration, "A Summary of the 2012 Annual Social Security and Medicare Trust Fund Reports," Retrieved June 2012 from <http://www.socialsecurity.gov/OACT/TRSUM/index.html>.

Deciding on Medigap or Medicare supplemental policies for Part B has been a source of confusion for those turning 65. After applying for Medicare, seniors are barraged with flyers and various advertisements promoting the dozens of supplemental plans available. Although many of these plans are similar, subtle differences could mean the difference between being fully or only partially covered, being covered only for certain services, or landing in a network with few providers that may be located a large distance away. The decision process becomes even more confusing when seniors are asked to choose a prescription plan under Part D. Costs of both of these services could vary wildly among plans.

Medicaid

Before 1965, medical care for those unable to afford it was primarily a responsibility of charitable institutions and state and local governments. In 1950, the federal government authorized states to use federal/state funds under the Social Security Act of 1935 to provide medical care for the indigent. In 1957, the Kerr-Mills Act provided for a federal/state matching program to provide health care for the elderly and the poor. However, Kerr-Mills was not mandatory, and many states chose not to participate. As a compromise to ward off more far-reaching health reforms, former President Lyndon Johnson signed into law the Medicaid and Medicare programs in 1965.³⁴ Replacing all previous governmental health programs, Medicaid became the largest public assistance program in the nation.³⁵ The 2014 expenditures for Medicaid was \$492.3 billion.³⁶

Medicaid is a means-tested public assistance program. Eligible people receive services from physicians who accept Medicaid patients (often a minority of physicians) and other health care providers. These providers are reimbursed by the federal government on a per-patient basis. To contain costs, several states require Medicaid recipients to enroll in state-contracted HMOs.

Medicaid is a federal/state program. States determine eligibility within broad federal guidelines. As such, each state establishes its own eligibility standards; determines the type, amount, duration, and scope of services; sets the rate of payment for services; and administers its own program. Medicaid policies can be complex and vary widely among states. For instance, a person eligible for Medicaid in one state may be ineligible in

another, and the services provided by one state may differ in the amount, duration, or scope of services compared to other states. State legislatures may change Medicaid eligibility, services, and/or reimbursement during the year. To be eligible for federal funds, states are required to provide Medicaid coverage for most individuals who receive federally assisted income maintenance payments, as well as for related groups not receiving cash payments. Some examples of the mandatory Medicaid eligibility groups are:

- Limited and low-income families with children who meet certain eligibility requirements.
- Supplemental Security Income (SSI) recipients.
- Infants born to Medicaid-eligible pregnant women (within certain parameters).
- Children and pregnant women whose family income is at or below 133 percent of the federal poverty level. The maximum mandatory income level may be higher if the state has established a higher-income threshold.
- Recipients of adoption assistance and foster care under Title IV-E of the Social Security Act.
- Certain Medicare beneficiaries and special protected groups.
- Other “categorically needy” groups as decided by the state³⁷ (Table 12.1 shows the numbers and eligibility categories of the Medicaid population).

The medically needy (MN) option allows states to extend Medicaid eligibility to persons who would not ordinarily be eligible because their income and/or resources are above the state’s eligibility level. People may have to “spend down” through incurring medical expenses that reduce their income to their state’s medically needy level. By 2012, 33 states and the District of Columbia had elected to have a program for the medically needy. All remaining states utilize the “special income level” option to extend Medicaid to the “near poor.” in medical institutional settings.

Medicaid is funded by federal/state matching funds. In every state, the federal government pays at least half, and in some states far more, of state Medicaid costs. The following services are covered:

- Inpatient and outpatient hospital services
- Prenatal care and 60 days’ postpartum pregnancy services
- Vaccines for children
- Physician services

TABLE 12.1 Medicaid Recipients by Category, 1985 to 2009, Selected Years (in thousands)

Year	Total	Age 65 or Older	Blind/Disabled	Children	Adults	Other
1985	21,814	3,061	3,017	9,757	5,518	1,214
1988	22,907	3,159	3,487	10,037	5,503	1,343
1990	25,255	3,202	3,718	11,220	6,010	1,105
1993	33,432	3,863	5,016	16,285	7,505	763
1997	34,872	3,955	6,129	15,791	6,803	2,195
2000	42,886	4,289	7,479	21,086	10,543	862
2005	57,643	4,396	8,210	26,337	12,529	6,171
2007	56,825	4,043	8,424	26,584	12,371	5,402
2009	62,458	4,188	8,998	28,634	13,907	6,731
2010	65,562	4,286	9,287	30,236	15,094	6,659
2011	69,179	4,337	9,759	31,480	16,109	6,694
2012	69,334	4,320	9,856	31,466	16,522	5,996

Source: Social Security Administration, *Annual Statistical Supplement, 2014*, "Unduplicated number of recipients, total vendor payments, and average payment by type of eligibility category, fiscal years 1985–2012," Released April 2015. Retrieved November 2015 from <https://www.socialsecurity.gov/policy/docs/statcomps/supplement/2014/8e.pdf>.

- Nursing facility services for persons aged 21 or older
- Family planning services and supplies
- Rural health clinic services
- Home health care for persons eligible for skilled nursing services
- Laboratory and X-ray services
- Pediatric and family nurse practitioner services
- Nurse midwife services
- Early periodic screening, diagnostic, and treatment (EPSDT) services for children under 21³⁸

States may also receive federal matching funds to provide certain optional services, such as:

- Diagnostic and clinic services
- Intermediate care facilities for the mentally handicapped (ICFs/MR)
- Prescribed drugs and prosthetic devices
- Optometrist services and eyeglasses
- Nursing facility services for children under age 21
- Transportation services
- Rehabilitation and physical therapy services
- Home and community-based care to eligible people with chronic impairments³⁹

Program expenditures differ for different groups. For example, in 2014, children (under 21) comprised about 51 percent of all Medicaid beneficiaries; non-disabled adults were the second largest group, with 39 percent of all beneficiaries. Those over 65 represented less than 10 percent (5.4 million) of the 66 million Medicaid beneficiaries, but they accounted for almost 20 percent of the total Medicaid costs.⁴⁰ Payment for long-term nursing home care alone accounted for more than one-third of the Medicaid budget in 2012.⁴¹

Not surprisingly, the growth of the nursing home industry parallels the creation of Medicaid. By 2015, there were 15,700 nursing homes with 1.7 million beds. Because 68 percent of nursing homes are for-profit facilities, Medicaid is a de facto subsidy for the nursing home industry. Medicaid also functions as a subsidy for the middle class. Namely, when elderly parents spend down their assets, they become eligible for Medicaid. Without Medicaid, the middle-class children of elderly parents would be responsible for paying the 2015 average national cost of \$80,300 a year for a semi-private room and \$91,250 for a private room.⁴²

Despite federal guidelines, four important gaps exist in Medicaid coverage: (1) the low eligibility

limits set for Medicaid, (2) the refusal of many states to adopt most or all of the Medicaid options, (3) the gaps in coverage for the elderly and disabled, (4) the ineligibility of some poor single persons and childless couples if they are not elderly or disabled, and (5) state cutbacks in Medicaid coverage.⁴³

For all its shortcomings, the Medicaid program has led to important improvements in the nation's health. In 1963 (pre-Medicaid), 54 percent of low-income people did not see a physician, and only 63 percent of poor pregnant women received prenatal care. Between 1964 and 1975, the use of physicians' services by poor children increased by 74 percent. The increased utilization of health care helped infant mortality drop by 49 between 1965 and 1988. For African American infants, the drop was even sharper: Infant mortality fell by only 5 percent in the 15 years before Medicaid; it fell by 49 percent in the 15 years after the program began. Ongoing preventive care also cut program costs for Medicaid-eligible children.⁴⁴ Medicaid is one of the most significant governmental health programs in the United States.

The Children's Health Insurance Program (CHIP)

As part of the Balanced Budget Act of 1997, Congress enacted the State Children's Health Insurance Program (S-CHIP), a federal-state partnership that allocated \$48 billion over 10 years to expand health care coverage to low-income, uninsured children under age 19 who are ineligible for Medicaid.⁴⁵ As part of the eligibility requirements, children generally must be below 200 percent of the federal poverty level. In 2010, \$11.4 billion was spent on CHIP.⁴⁶ By 2015, over 8 million children were enrolled in the CHIP program.⁴⁷

The CHIP program gives states three options for covering uninsured children: design a new children's health insurance program, expand current Medicaid programs, or a combination of both. CHIP is a block grant program financed by federal/state matching funds. Each state with an approved plan receives enhanced federal matching payments for its S-CHIP expenditures up to a fixed state allotment.⁴⁸ The largest share of CHIP funds come from the federal government, and in 2011, federal matching funds averaged 71 percent.⁴⁹

Before CHIP, Medicaid eligibility was largely linked to welfare receipt. In large measure,

Medicaid-eligible children had to be in an SSI or an AFDC-eligible family. Perhaps the most important policy change in CHIP is that it delinked state-subsidized child health care from welfare receipt, and from working families with incomes too high to qualify for Medicaid but too low to afford private health insurance.⁵⁰ By 2011, 46 states and the District of Columbia covered children in families with incomes up to 200 percent of the federal poverty line, and 24 states covered children in families up to 250 percent of the federal poverty line. Several states also expanded eligibility categories and some now cover legal immigrant children and/or pregnant women.⁵¹

The Health Care Crisis

Health care in the United States is plagued with problems such as eroding coverage, rising and **shifting costs**, and an increasing number of anxious citizens fearful about getting and keeping health insurance. The following sections will explore some parameters of the health care crisis, including health care spending and cost efficiency, the effectiveness of the U.S. health care system, attempts at cost cutting, the growing role of managed care, and the impact of AIDS on the health care budget.

Overview of U.S. Health Care Expenditures

U.S. health care costs slightly more than \$3 trillion in 2014, up from \$27.4 billion in 1960. This translates into 17.6 percent of the total gross domestic product (GDP) compared to less than 5 percent for education. (Health expenditures as a percentage of the GDP measure the proportion of all resources devoted to health care.) It is projected that by 2020, health care spending will rise to \$4.6 trillion and account for almost 20 percent of the GDP. In comparison, health care spending in 1960 was only 5.2 percent of the GDP.⁵² From 1970 to 2007, per capita health care costs rose from \$341 to \$7,439.⁵³ By 2013, health expenditure equated to \$9,925 per person.⁵⁴

The costs of providing health care have risen faster than the rate of inflation. From 1980 to 1992, annual increases in health care expenditures were roughly 5 points above the yearly inflation

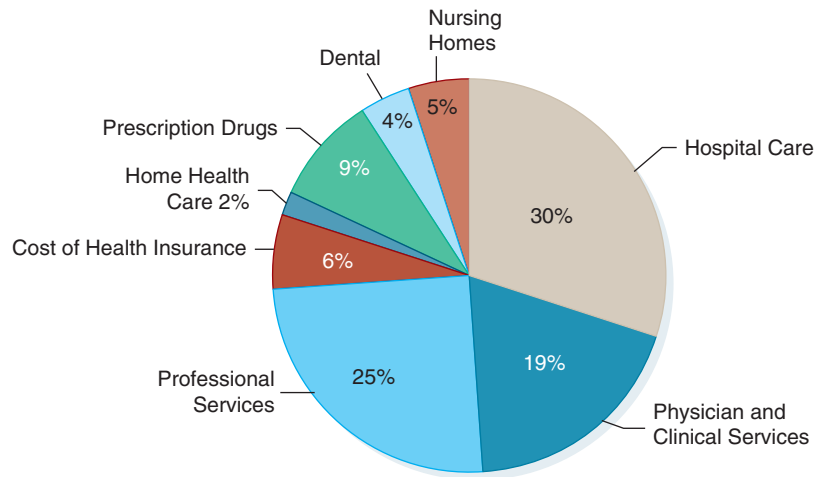


FIGURE 12.2 The Nation's Health Care Expenditure by Percentage, 2013

Source: Based on Table 103. National health expenditures, average annual percent change, and percent distribution, by type of expenditure: United States, selected years 1960–2013. CDC /National Center for Health Statistics/Office of Analysis and Epidemiology, May 2015. Retrieved January 2016 from <http://www.cdc.gov/nchs/has/healthexpenditures.htm#total>.

rate. Although the growth rate in health care expenditures declined somewhat in 1992 (registering less than 3 points above the rate of inflation),⁵⁵ it accelerated again in the late 1990s. The Centers for Medicare and Medicaid Services estimated that health spending is projected to grow by 5.8 percent from 2012 to 2022, which is 1 percent faster than the projected average annual growth in the GDP.⁵⁶

When health care expenditures are broken down, the largest share (32 percent) goes to hospitals⁵⁷ (see Figure 12.2). An increase in the cost of hospital care has been a key factor in driving up health care costs, and between 2012 and 2013 that rose 4.3 percent.⁵⁸ In 1965, the average cost per hospital stay was \$315; by 2013, it had risen to \$1,878 per day.⁵⁹

Household and private business spending has declined since 1987, and spending by government has increased from 32 percent in 1987 to around 44 percent. This is largely due to Medicare, Medicaid, and CHIP, which comprise more than one-third of total national health care expenditure.⁶⁰ Health care spending is expected to rise 5.8 percent a year, which is several times above the rate of inflation.

Single and family premiums for employer-sponsored health insurance rose about 4 percent from

2014 to 2015, continuing a 10-year period of relatively manageable rises, especially when compared to the 11 percent annual growth from 1999 to 2005. However, the modest rises since 2005 were still several times above the rate of inflation. By 2015, the average annual premium for a single employee was \$6,251, with employees paying about \$1,071 toward it. The average family premium was \$17,545, with workers contributing \$4,955.⁶¹ These employee contributions are high and can take a big bite out of a paycheck. For some employees, it has resulted in a net stagnation (or decrease) in wages, especially if the raises were based on a 2 percent or less rate of inflation.

The moderate increase in health care premiums is a kind of bait and switch. For instance, although insurance premiums have risen modestly, the number of employees with deductibles and the size of those deductibles have increased dramatically. Since 2010, the share and size of deductibles has increased 67 percent, much faster than the 24 percent rise in single employee premiums and the 10 percent rise in wages.⁶² Hence, workers have not experienced a slowdown in health care expenditures as their insurance coverage has eroded and their out-of-pocket expenses increased (see Figure 12.3).

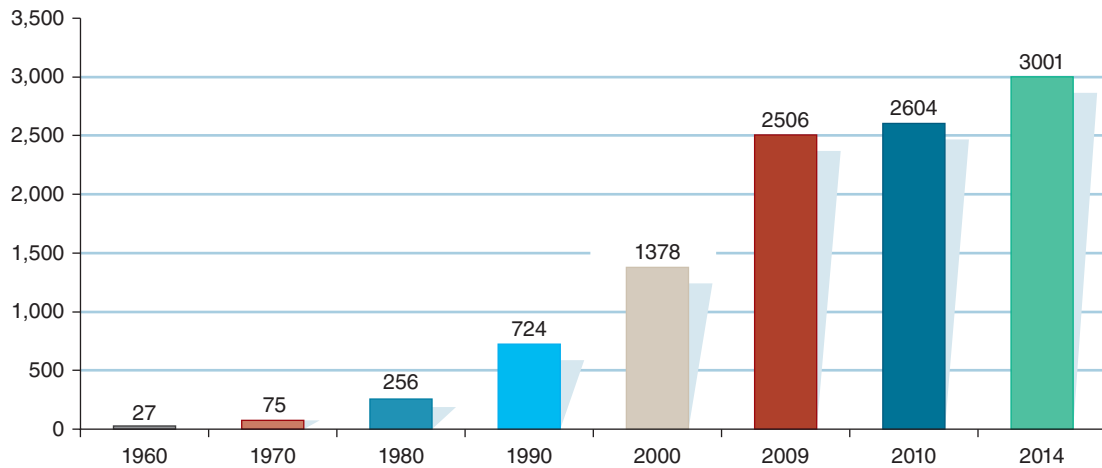


FIGURE 12.3 U.S. Health Care Expenditures Selected Years, 1960–2014 (in billions)

Source: Based on table 102. Gross domestic product, national health expenditures, per capita amounts, percent distribution, and average annual percent change: United States, selected years 1960–2013. CDC/National Center for Health Statistics/Office of Analysis and Epidemiology, May 2015. Retrieved January 2016 from <http://www.cdc.gov/nchs/hsu/healthexpenditures.htm#total>.

Explaining the High Cost of U.S. Health Care

Although accounting for the enormous costs of the U.S. health care system is complicated, some policy analysts attribute at least some of these costs to the following factors.

- Although malpractice insurance costs are dropping for some medical specializations, the costs are still high. In 2010, the total costs of medical malpractice (hospitals, physicians, etc.) was \$10 billion. Although it may seem high, it is only a small percentage of the \$3 trillion in total medical costs. The fear of medical malpractice suits results in some doctors practicing “defensive medicine” by ordering largely unnecessary tests for patients to avoid a possible lawsuit. The former G.W. administration argued that malpractice suits could be curtailed by legislation, limiting what injured patients could collect in lawsuits. The Obama administration rejected the “cap” legislation, instead choosing a preventative approach that supports research into the provision of better medical care and reduced incidents of malpractice.⁶³
- The United States leads the world in the development and use of medical technology, but these advances have come at a high price. By 2009, doctors had performed 500,000 coronary artery bypass operations each year, and the number of stent operations (an alternative to bypasses) were well over a million.⁶⁴ One study found that defensive medicine increased annual health care costs by \$650 to \$850 billion.⁶⁵
- Expensive new pharmaceutical drugs drive up costs considerably. The higher prevalence and treatment of chronic diseases, such as diabetes, has also driven up health care. Longer lifespans for a chronically ill patient means more care and treatment, which helps to drive up medical costs.
- The administrative costs involved in processing millions of insurance claims add to rising health care expenditures. Government health care administration cost \$30 billion in 2010, a \$27 billion increase from 1980.⁶⁶ The National Academy of Sciences estimates that \$190 billion a year is wasted on excessive administrative costs.⁶⁷
- According to the National Academy of Sciences, \$765 billion in 2009 (30 percent of 2009 health care costs) was wasted in health care costs: (1) \$210 billion in unneeded services, (2) \$75 billion in fraud, (3) \$190 billion in excessive administrative costs, (4) \$130 billion in inefficient services, (5) \$105 billion in excessive prices, and (6) \$55 billion wasted through missed prevention opportunities.⁶⁸

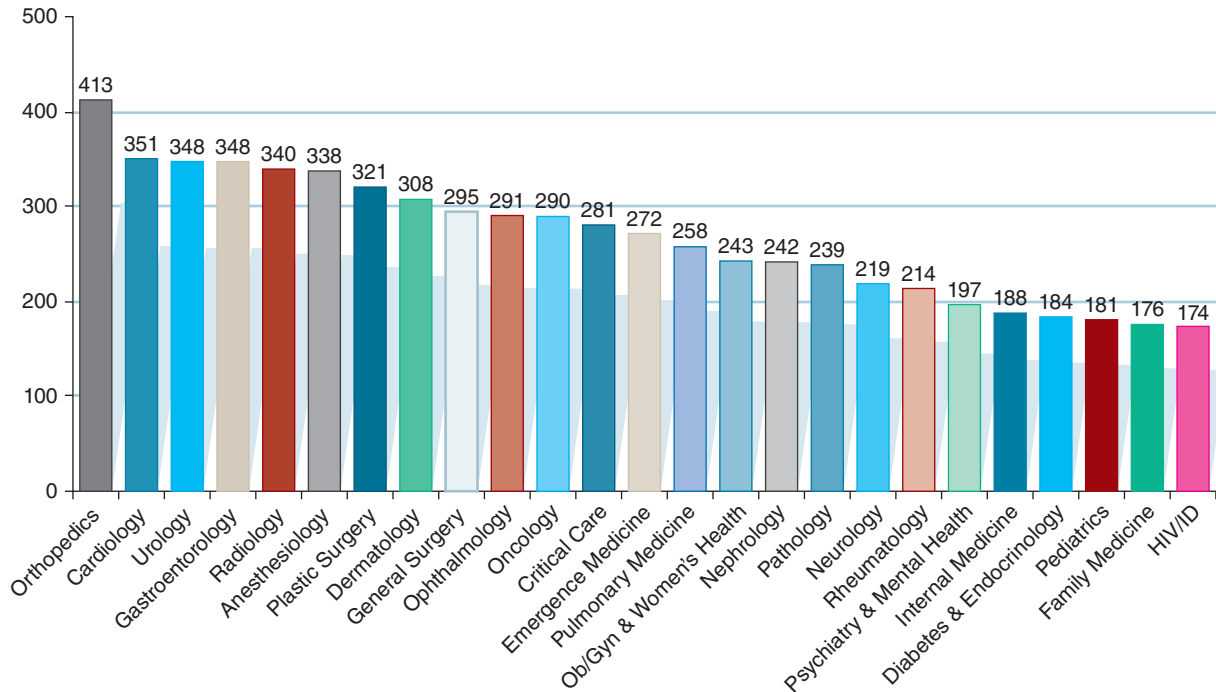


FIGURE 12.4 Selected Physician Yearly Salaries, 2013 (in thousands)

Source: Based on Leslie Kane and Carol Peckham, Medscape Physician Compensation Report 2014, Medscape, April 15, 2014. Retrieved January 2016 from <http://www.medscape.com/features/slideshow/compensation/2014/public/overview#2>.

Hospital Costs

As noted earlier, more than 31 percent of all health care expenditures in 2015 went toward hospital costs.⁶⁹ The total costs of hospital care reached \$972 billion in 2014, up from \$27.2 billion in 1970.⁷⁰ According to the American Hospital Association, nearly 60 percent of hospital costs go to wages and benefits of caregivers and others. The costs of technology, construction, and regulatory compliance account for a growing share of hospital costs.⁷¹

Physicians' Salaries

The fifth largest health care expenditure is physician services.⁷² From 1970 to 1999, the cost of physicians' services rose by almost 1,800 percent. From 1980 to 1990 alone, the cost of physicians' services rose by 300 percent.⁷³ By 2008, the average yearly salary for a number of specialist physicians, such as orthopedic surgeons, was approaching \$400,000.⁷⁴ Since then, physician salaries have experienced an overall leveling off. However, 2013 yearly salaries for specialists continued to remain high: orthopedic surgeons earned \$413,000; cardiologists, \$351,000; urologists, \$348,000; gastroenterologists,

\$348,000; radiologists, \$340,000; anesthesiologists, \$338,000; and plastic surgeons, \$321,000⁷⁵ (see Figure 12.4). Nevertheless, U.S. physician salaries are significantly higher than their counterparts in most OECD countries, such as the United



The increasing costs of malpractice suits against health care providers are felt most directly by consumers, who must pay higher medical costs to offset these suits.

Source: Julie Smith/Jefferson City News Tribune/AP Images

Kingdom, Canada, and Switzerland. This is true even when taking purchasing-power parity into account.⁷⁶

Physician organizations argue that high salaries are necessary to repay the huge debts incurred by medical students. Roughly 87 percent of medical students carry outstanding loans, with the average educational debt of 2014 medical school graduates at \$140,000. Added to that is the substantial difference between private and public medical school tuition fees: The 2014 average annual fee for public medical school was \$48,578 compared to \$54,862 for private colleges.⁷⁷

Despite this debt, the American Medical Association claims that “Even though the cost of medical school has risen faster than physician salaries, physicians still earn a decent return on their investment, roughly 6 percent for the \$1.2 million a physician bears for the assorted costs of a public education and the wages lost while in school. Also, becoming a doctor is typically a higher paying investment when comparing average educational costs and salaries of business and law school graduates.”⁷⁸ The problem of high physician salaries is aggravated by the growth in the number of expensive medical specialists. Although **primary care** physicians declined steadily as a percentage of the total physician population from 1970 to 2010 (from 40 to 33 percent), there was nevertheless a growth of 124 percent in absolute numbers. By 2009, approximately 70 percent of physicians were specialists and 30 percent engaged in primary care (12.3 percent in family medicine and general practice)—a reversal of the ratio that existed 30 years ago.⁷⁹

The problem of high physician salaries is aggravated by the growth in the number of medical specialists. Majority of medical students were in a primary care track developed in the 1970s.⁸⁰ By 2012, 19.1 percent (59 million people) of the U.S. population were living in one of the 5,796 regions that were experiencing a primary care physician shortage.⁸¹

The Pharmaceutical Industry

From 1997 to 2010, the share of health care expenditures spent on prescription drugs in the United States increased from 6.8 percent to almost 10 percent.⁸² Although prescription drugs account for less than 10 percent of total health care expenditures, their high cost is helping to drive up health care costs. From 1996 to 2005, pharmaceutical drug costs outstripped the general inflation rate and the

overall rise in hospitals and physician’s services, sometimes by a margin of two to three times. A 2010 study by the Association for Retired Persons (AARP) found that despite the recession, drug prices had risen an average of 8.3 percent in 2009, eight times higher than the rate of inflation and faster than in preceding years. Moreover, the average retail cost of brand-name medications in 2013 (the year before Medicare Part D) was \$3,000; prices have risen from \$1,382 in 2006.⁸³ There is scant evidence that Medicare Part D pharmaceutical plans resulted in better prices.

The United States is the largest pharmaceutical drug market in the world, representing 43 percent of global sales. Despite the size of the market, American consumers pay more for medicine than consumers in any other country. In 2013, Americans spent roughly \$1,000 per person on drugs, followed by Canada, Greece, and Ireland.⁸⁴ Two-thirds of doctor visits result in a drug being prescribed, and overreliance on drugs is a major factor contributing to medical care being the third leading cause of death in the United States.⁸⁵ Although many low-income consumers are exposed to financial catastrophe as a result of highly inflated drug prices, manufacturers of the top 20 drugs have increased their profits at rates much higher—often around 20 percent—than the Fortune 500 average.⁸⁶

Harvard physician Marcia Angell disputes the drug industry’s reputation as an “engine of innovation,” arguing that the top U.S. drug makers spend 2.5 times as much on marketing and administration as they do on research. In contrast to the carefully cultivated image of the drug industry as a research-driven, lifesaving enterprise, it is estimated that many U.S. pharmaceutical companies spend almost twice as much on promotion as on research and development.⁸⁷ According to Marcia Angell, at least a third of the drugs marketed by the industry are discovered by universities or small biotech companies but are sold to the public at inflated prices. Angell attacks the pharmaceutical industry—whose top 10 companies make more in profits than the rest of the Fortune 500 combined—for using free market rhetoric while opposing competition.⁸⁸

Price gouging in the pharmaceutical industry is becoming more commonplace. Although drug costs have been partially kept down by the increased use of lower cost generic drugs, the price of about 27 percent of generics have risen, sometimes dramatically. According to the AARP, some examples include Doxycycline Hyclate (a widely used

antibiotic) that went from \$20 for 500 capsules in 2013 to \$1,849 in 2014. Pravastatin sodium (a cholesterol medication) jumped from \$27 to \$196 for a one-year supply. The already high prices of brand-name drugs continued to climb. The average retail price of brand-name drugs commonly used by seniors rose 13 percent in 2013. About 97 percent of the 227 non-generic prescription drugs had price increases in 2013. Pharmaceutical companies have employed varied strategies to maintain a monopoly in certain profitable drugs, including slightly changing formulas and touting it as a new drug, creating time release versions of the drug, and even paying generic drug companies tens of millions of dollars to not make a generic version.⁸⁹ What has helped keep drug prices high is the refusal of FDA to allow the importation of lower cost drugs from Canada and elsewhere. This includes the importation of biosimilars (drugs that are similar to ones previously licensed). One glaring example of extreme price differentials involves the drug Sovaldi, which is used to treat Hepatitis C. In the United States, a 12-week course of Sovaldi costs \$84,000 or \$1,000 a day. In India, that same drug costs \$4 a day.⁹⁰

Direct-to-consumer advertising (DTCA) is the promotion of prescription drugs through newspaper and magazine ads, brochures, videos, television commercials, and Internet marketing targeted at consumers and doctors. By 2013, spending on advertising stood at \$8 billion.⁹¹ The only two developed countries where DTCA is currently legal are the United States and New Zealand, and in limited form in Canada. Although banned elsewhere, the drug industry is mounting major lobbying campaigns to have DTCA allowed in Europe and Canada.

Drug companies have aggressively marketed their wares to doctors by advertising and placing favorable studies in medical journals, conducting frequent visits by sales representatives, organizing junkets, subsidizing conferences, providing free lunches and dinners, and by giving doctors free drug samples for their patients. However, in recent years, they have focused their marketing attention directly on consumers through intensive television and print advertising. Although the drug industry maintains that DTCA helps educate consumers, critics argue that the ads are emotional and are misleading since they understate adverse side-effects. Studies reveal that consumers who have watched DTCA ads will often request (and be prescribed) expensive brand-name drugs as opposed to generic

versions. As a result, DTCA drives up the cost of health care without improving the health of the consumer or the efficiency of the system.⁹² There is a strong evidence suggesting that DTCA is also partially responsible for an overreliance on drugs. According to one study, 71 percent of American Academy of Family Physician members believed that direct-to-consumer advertising created more pressure on physicians to prescribe unnecessary medication.⁹³

Cutting Health Care Costs

There are two aspects to cutting health care costs. The first involves cutting costs for governmental health care programs and the second involves lowering health care costs in the insurance, physician, pharmaceutical, and hospital sectors.⁹⁴ The rising costs of Medicare have led the federal government to seek alternative ways to lower hospital costs, including the Prospective Payment System (PPS). In this system, the federal government specifies in advance what it will pay for the treatment of 468 classified illnesses or **Diagnostic Related Groups (DRGs)**. There are separate PPSs for various services, such as inpatient hospitals, home health care, hospice, inpatient psychiatric facilities, rehabilitation facilities, and skilled nursing facilities. If a patient requires less hospitalization than the maximum DRG allocation, the hospital keeps the difference.⁹⁵ Hence, patients not yet ready for discharge (e.g., patients who do not have appropriate aftercare services available) may be discharged—a situation that can result in patient dumping.

Managed Care

Managed care became a household word in the 1990s. By 2014, almost 86 million Americans were enrolled in HMOs⁹⁶ (see Figure 12.5). This represents a significant increase from the 3.3 million enrolled in 1990.⁹⁷ Managed care and HMOs have been touted by the nation's large employers as a way to control health care costs. However, although managed care cut costs for most of the 1990s, by the end of the decade, costs started climbing again, increasing by more than 7 percent in 2000.⁹⁸ By 2014, the Affordable Care Act allowed states to expand Medicaid eligibility, resulting in managed care covering more people, thereby increasing overall expenditures.

- **Health Maintenance Organizations (HMO)**—a prepaid or capitated insurance plan in which individuals or their employers pay a fixed monthly fee for services rather than a separate charge for each visit or service.
- **Preferred Provider Organizations (PPO)**—a type of HMO whereby an employer or insurance company contracts with a selected group of health care delivery providers for services at pre-established reimbursement rates. Consumers have the choice of who to contact to provide the service. If a doctor is not on the provider list, higher out-of-pocket expenses will result.
- **Exclusive Provider Organization (EPO)**—a type of HMO where members must get care only from the EPO doctors who may only treat members of the plan.
- **The Independent Practice Association (IPA)**—a type of HMO that has large numbers of independent doctors in private practice. Physicians are paid a fixed fee for treating IPA members but can also treat patients who are not members of the plan.
- **The Network Model**—multispecialty groups of doctors who have contracts with more than one HMO. Doctors work out of their own offices.
- **Point of Service (POS)**—an option that can be offered by any type of HMO. If patients use doctors in their HMO network, and if referrals are made only by the primary care physician, only nominal fees are charged. If patients use doctors outside their HMO network, the cost is higher.
- **Physician-Hospital Organization (PHO)**—organized groups of doctors affiliated with a particular hospital who provide services to patients enrolled in their plan as they would in an HMO.

FIGURE 12.5 Types of Managed Care Plans

Source: Naomi Brill, Joanne Levine, *Working With People: The Helping Process*, 7e Ed., © 2002. Reprinted and Electronically reproduced by permission of Pearson Education, Inc., Upper Saddle River, New Jersey.

Proponents of managed care argue that the system has effectively lowered health care costs without reducing the quality of health care services. They maintain that it encourages more efficient and less expensive medical care and that it can stress prevention over treatment. Because doctors reimbursed by managed care companies have little incentive to over-treat patients or recommend unnecessary medical care, the health care system is predicted to be more efficient.

Critics of managed care (including HMOs) point out serious problems. For one, managed care has not won the hearts and minds of the public. The 2010 Commonwealth Fund survey found that out of 11 countries, the United States experienced the most difficulties with their health insurance providers, with 31 percent of participants reporting that they spent a lot of time on paperwork or disputes, and/or their insurance provider declined to pay for treatment or did not pay as much as was expected. Only 58 percent of Americans were confident that they would be able to afford needed health care.⁹⁹

Managed care plans generally gatekeep the access to specialists. This is done by pressuring primary physicians to not refer or by limiting specialist

care to one or two visits. Some managed care plans are reluctant to cover costly procedures or experimental treatments. Still other plans refuse to pay for medical care that clients receive while out of state, even if it was an emergency. Some enrollees complain that managed care forces them to use only primary care physicians, hospitals, and specialists that are on an approved list, which restricts their freedom of choice. And some managed care plans do not provide the same level of benefits as Medicare, especially when it comes to home health care, physical therapy, and nursing home care.¹⁰⁰ Critics also note that the size of managed care operations has led to greater bureaucratization and impersonality.¹⁰¹

During the late 1990s, many HMOs were in financial trouble and the industry posted large losses. To compensate, HMOs raised premiums, cut services, or left the health insurance business altogether. Several large managed care companies went bankrupt, leaving state officials and consumers scrambling to find replacement coverage. Beginning in the mid-1990s, the health insurance marketplace consolidated at a rapid pace; by 2001, there were over 350 mergers of health insurers and managed

care corporations. Consequently, more than two-thirds of all commercially insured Americans are now covered by the 25 largest health insurers:

- #1 UnitedHealth Group
- #2 Kaiser Foundation Group
- #3 Wellpoint Inc. Group
- #4 Aetna Group
- #5 Humana Group
- #6 HCSC Group
- #7 Cigna Health Group
- #8 Highmark Group
- #9 Blue Shield of California
- #10 Independence Blue Cross
- #11 Centene Corp Group
- #12 HIP Insurance Group
- #13 BCBS of New Jersey Group
- #14 BCBS of Michigan Group
- #15 Guidewell Mutual Holdings
- #16 California Physicians' Service
- #17 Wellcare Group
- #18 Carefirst Inc. Group
- #19 Health Net of California, Inc.
- #20 Molina Healthcare Inc. Group
- #21 UHC of California
- #22 Lifetime Healthcare Group
- #23 BCBS of Massachusetts
- #24 Metropolitan Group
- #25 Cambia Health Solutions¹⁰²

Managed care companies are boxed in. For one, they cannot reprise their 1990s role as gatekeepers where they restricted care to boost the bottom line. Regulations on the state and federal levels have curbed their options, and after bad publicity in the 1990s, these companies became more flexible by paying for more care. Instead of gatekeeping and cutting costs, managed care companies simply passed along premium hikes. However, once their belt-tightening role is lost, their reason for existing is called into question.

The Underinsured

The U.S. medical system provides high quality care for most people in the upper and upper-middle

classes who are protected by good health insurance. Although most Americans are covered by private insurance plans, gaps in private health insurance may include high deductibles and copayments, limits on hospital stays, dollar limits on payments to hospitals and physicians, exclusion of certain laboratory tests, and so forth. Because privately purchased health insurance have become almost unaffordable, the availability of health insurance may be a major factor in an individual's job search or decision to continue in a job.¹⁰³ Moreover, employers may be reluctant to hire people with high-risk conditions because of the negative impact on insurance premiums. Finally, one of the most dramatic gaps is the frequent failure of some private health insurance plans to cover catastrophic medical costs—those costs that could reduce a middle-class family to “medically indigent” status within only a few months.

Gun Violence and Health Care Policy

One of the knottiest issues in health policy is gun violence. There are about 300 million guns in the United States, and one-third of homes with children have guns. About 1.7 million children live in a house with an unlocked, loaded gun.¹⁰⁴ Guns and



According to a Rand study, not all people with HIV are treated equally. Among adults with HIV, women receive inferior care compared to men, as do African Americans and Hispanics compared to non-Hispanic whites.

Source: David Weintraub/Science Source

cars are the leading causes of non-medical deaths in the United States, but while cars have gotten safer (vehicle deaths dropped 22 percent from 2005 to 2010), gun fatalities have been relatively steady. The Centers for Disease Control predicts that in 2015, gun-related deaths will rise to 33,000, while vehicle deaths will decline to 32,000. Gun violence is a critical public health issue that needs to be depoliticized to address the pressing health issues.¹⁰⁵

Supporters of gun control, such as the Brady Campaign to Prevent Gun Violence, point to the following 2014 data:

- Forty-eight children and teens (0–19) are shot daily in murders, assaults, suicides, accidental shootings, and through police action. Seven of those children and teens die: Five are murdered and 2 commit suicide.
- Each day, almost 300 Americans are shot in murders, assaults, suicides/suicide attempts, accidental shootings, and through police action. Of that number, 208 people survive.
- The results are even more startling when examined over a year. More than 108,000 Americans are shot in murders, assaults, suicides/suicide attempts, accidental shootings, or by police intervention. Of that number, 75,962 survive.¹⁰⁶

Led by former U.S. Representative Gabrielle Giffords (seriously wounded in a mass shooting) and former astronaut Mark Kelly, Americans for Responsible Solutions maintains that:

- Americans are 20 times more likely to be murdered in a gun-related incident than those living in other developed countries.
- American women are 11 times more likely to die in a gun-related incident than women in other developed countries. From 2001 through 2012, 6,400 women were murdered by an intimate partner using a gun—more than the total number of American soldiers killed during the Iraq and Afghanistan wars.
- American children are 11 times more likely to die in gun-related deaths than children in other high-income countries.¹⁰⁷

There have been at least 130 school shootings in America from 1999 to 2012. From 1982 to 2015, there were 73 mass shootings (defined as murdering four or more people unrelated to the shooter), including 36 since 2006. In 2012, there were seven

mass shootings alone.¹⁰⁸ An FBI report found that active shooter incidents have risen dramatically; from 2007 to 2013, there were 16.4 shootings a year.¹⁰⁹ An example of some of the more well-publicized mass shootings includes:

San Bernardino (2015), Inland Regional Center, 14 dead, 21 wounded
 Umpqua Community College (2015), Roseburg, OR, 9 dead, 9 wounded
 Chattanooga Military Recruitment Center (2015), 5 dead, two wounded
 Charleston, Church shooting (2015), 9 dead, 1 wounded
 Marysville-Pilchuck High School shooting (2014), 5 dead, 1 wounded
 Alturas, tribal shooting (2014), Alturas, CA, 4 dead, 2 wounded
 Washington (DC) Navy Yard shooting (2013), 13 dead, 8 wounded
 Santa Monica rampage (2013), 6 dead, 3 wounded
 Newtown (Sandy Hook Elementary School) school shooting (2012), 28 dead, 2 wounded
 Sikh temple shooting (2012), Oak Creek WI, 7 dead, 3 wounded
 Aurora (CO) theatre shooting (2012), 12 dead, 58 wounded
 Tucson shooting (2011), 6 dead, 13 wounded
 Hartford Beer Distributor shooting, Manchester, CT (2010), 9 dead, 2 wounded
 Fort Hood massacre (2009), 13 dead, 30 wounded
 Virginia Tech massacre (2007), 33 dead, 23 wounded
 Red Lake massacre (2005), 10 dead, 5 wounded
 Columbine High School massacre, CO (1999), 15 dead, 24 wounded¹¹⁰

Pro-gun advocates dispute these figures and point to the drop in gun murders. By 2010, gun murders were at their lowest point since 1981. Total gun deaths fell from 15.3 in 1993 to 10.6 in 2013. Pro-gun advocates attribute those drops to the rise in gun ownership and the almost doubling of gun production from 2007 to 2011.¹¹¹ Despite these lower numbers, comparing America's gun deaths to those of other industrialized nations (see Figure 12.6) is sobering.

The Debate around Gun Control

Gun control is one of the most divisive issues in American society. Advocates for gun control argue for what they consider to be sensible restraints on

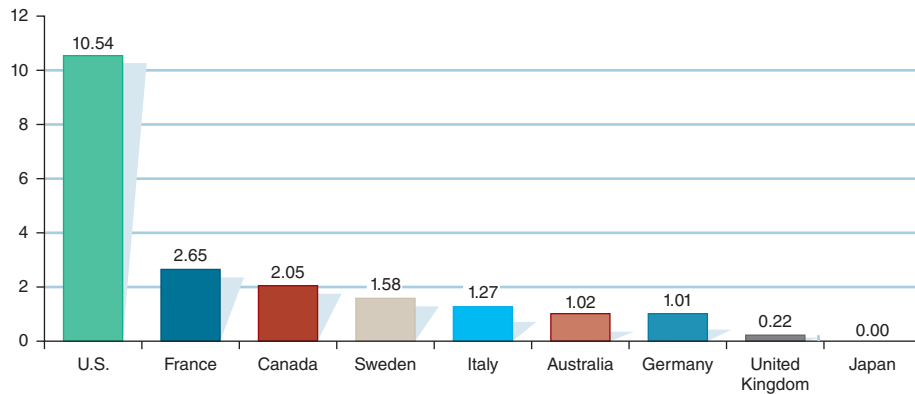


FIGURE 12.6 Rate of All Gun Deaths Per 100,000 Population

Source: Based on University of Sydney, School of Public Health (gunPolicy.org), Compare the United States, 2015. Retrieved January 2016 from, http://www.gunpolicy.org/firearms/compare/194/rate_of_all_gun_deaths_per_100_000_people/10,31,66,69,88,192,91,177

NB: U.S. data is from 2013; France, 2012; Canada, 2011; Sweden, 2010; Italy, 2010; Germany, 2011; Australia, 2013; United Kingdom, 2011; and Japan, 2014.

gun ownership, including more thorough background checks, prohibiting domestic abusers from purchasing guns, and eliminating the gun show loophole by mandating that all gun sales involve a background check on buyers. There are also calls for reinstating the 10-round magazine ban that expired in 2004.

Pro-gun advocates, supported by the National Rifle Association (NRA), have been successful in promoting their Second Amendment (the right to bear arms) agenda. By 2015, 45 states had adopted the “castle doctrine,” which states that a person does not have to retreat when their home is attacked. Twenty-two states went even further and passed stand-your-ground laws. The traditional expectation was that an armed person must first try to retreat to escape the danger. Only if there is no escape is the use of lethal force permitted. However, the “stand-your-ground” law removes the duty to retreat before using force in self-defense. The law permits an armed person to not retreat and use lethal force in any location they are legally allowed to be. This was the basis of the acquittal of George Zimmerman in the Trayvon Martin case in Florida.

Building on these victories, pro-gun advocates have argued for further loosening the law, including allowing concealed carry in bars, and more recently, allowing students to carry concealed guns onto college campuses. Texas became the eighth state to allow concealed weapons on college campuses, following Colorado, Idaho, Kansas, Mississippi, Oregon, Utah, and Wisconsin.¹¹²

Another issue pushed by the pro-gun lobby has been “open carry,” which is the legal right to openly carry a firearm in public. The open carry movement frightened many people as fully armed people with assault rifles and pistols flocked into supermarkets and fast food restaurants. Their motto was “If we don’t exercise our rights we will lose them.” Licensed gun owners in Texas will be allowed to openly carry a handgun in most places beginning in 2016. However, private businesses will retain the right to ban open carry on their premises, and many businesses have stated they would exercise that right. Those businesses not permitting open carry include H-E-B Grocery Stores, Randall’s Food Markets, Whole Foods, Starbucks, Jack in the Box, Whataburger, Chili’s, Sonic, and Chipotle.¹¹³

NRA’s motto is “The only thing that stops a bad guy with a gun is a good guy with a gun.” The problem is that NRA’s positions allow a bad guy to own a gun in the first place. For instance, the NRA has virulently defended the rights of the thousands of people on the government’s terrorist watch list, including those on the “no-fly list,” to buy guns. Seven hundred people have bought guns while being on the list. The NRA also remains opposed to expanded background checks and to closing the gun show loophole.¹¹⁴

The NRA fought to keep gun ownership as a political rights issue rather than a public health issue. One way to accomplish this was to block research into gun violence by health-related federal agencies like the Centers for Disease Control.

In 1996, Congress approved a budget restriction mandating that none of the funds made available for injury prevention at the CDC could be used to promote gun control. The ban, which remains in effect, was later extended to the National Institutes of Health (NIH). In turn, the CDC was so fearful of losing its funding that it effectively halted all major research on gun violence.¹¹⁵

What Can Be Done

Even if an anti-gun law were passed, eliminating the 310 million guns in the United States would be impossible. One way to lower the number of guns in circulation is through a gun buyback program. The results were inconclusive when voluntary gun buyback programs were attempted in the United States.¹¹⁶ On the other hand, Australia's gun buyback program was successful largely because it piggybacked onto legislation that severely restricted the ownership of firearms.

Another potential solution involves building "smart guns" using biometrics. By using fingerprint or radio frequency technologies, smart guns would be operable only by the owner. This would help curb accidental shootings, especially those involving children. Not surprisingly, the NRA is opposed to mandatory "smart gun" requirements based on the fear that it is a step toward gun control.¹¹⁷ Still another reform would be to require all gun sales to include a background check. At present, 40 percent of guns sold in the United States involve no background check whatsoever.¹¹⁸

These are obviously modest changes. What is needed is a cultural shift where rights are redefined. In particular, gun owners have rallied around freedom and their constitutionally protected right to own a gun. But what about the freedom to feel safe without being armed? Or the right to feel safe without owning a gun. Do we want to return to a Wild West where everyone was armed and "stand-your-ground" meant exactly that? Like many social issues, gun rights has become a symbol that reflects differing views of society and the role of government.

U.S. Health Care in International Perspective

The United States spends more than other countries on health care, both in absolute dollars and in the share of total economic activity. Per capita health

spending in the United States was \$55,200 in 2014, more than twice the average for the other industrial nations in the Organization for Economic Cooperation and Development (OECD).¹¹⁹

As a percentage of the GDP, health spending in the United States is nearly twice that of Australia and the United Kingdom (see Table 12.2), and the highest of 30 OECD countries.¹²⁰ Although high expenditures are often equated with positive outcomes, the health of the average American—as measured by life expectancy and infant mortality—is below that of other Western industrialized nations. In reviewing the American health care system, the Commonwealth Fund issued a report in 2010 that compared the U.S. health care system to economically comparable countries. The report found that the United States has a relatively low number of hospital beds and physicians per capita, and patients in the United States have fewer hospital and physician visits than in most other industrial countries. At the same time, spending per hospital visit is highest in the United States where patients are among the most likely to receive procedures requiring complex technology. Despite high per capita health care spending, the United States ranks in the bottom quartile in life expectancy among OECD countries and has seen the smallest improvement in this metric.¹²¹ Factors that impact America's problematic health indicators include diet, obesity, low levels of physical activity, high levels of births to teenage mothers, and the large number of violent deaths.¹²²

TABLE 12.2 Total Health Care Expenditures as a Percentage of GDP, 2013

Country	1975	2013	Percentage Point Change
United States	7.9	17.1	9.2
France	6.4	11.7	5.3
New Zealand	6.7	9.7	3.0
Germany	8.4	11.3	2.9
Canada	7.0	10.9	3.9
Australia	6.5	9.4	2.9
United Kingdom	5.5	9.1	3.6

Source: Health Expenditure in Relation to GDP April, 2013. Retrieved November 2015 from <http://data.worldbank.org/indicator/SH.XPD.TOTL.ZS>.

The United States is the only major industrialized country that does not provide health coverage for all its citizens. This is partly due to the emphasis the U.S. places on the private market to control costs. Yet, health care costs are growing more rapidly in America than in more governmentally regulated health care systems, such as Britain and Australia. Private spending as a share of total health care expenditures is higher in the United States than in other industrialized nations, as is out-of-pocket per capita spending.¹²³

A common perception is that other industrialized countries control costs by rationing patient care. As noted above, Americans receive fewer days of hospital care than residents of other industrialized nations, and they have about the same number of physician visits.¹²⁴ Infant mortality rates also illustrate troubling trends. In 2010, the U.S. infant mortality rate of 6.1 deaths per 1,000 live births put it under 46 other nations, including Cuba, Hungary, Greece, Canada, Portugal, Slovenia, Israel, Czech Republic, and Singapore. The United States has the highest infant mortality rate of any industrialized country. Longevity rates in the United States are also problematic, ranking at the bottom of comparable OECD nations.¹²⁵ In short, Americans are neither healthier nor live longer than people in similar industrial nations where health care spending is 50 percent lower.

Comparative Analysis: Health Care in Canada, the United Kingdom, and Australia

Rooted in “American exceptionalism” (i.e., the belief that the United States is so unique that policies outside its borders have little relevance), many U.S. citizens and policymakers miss the opportunity to learn from the health care systems in other nations. This section briefly explores health care systems in Canada (a single-payer system), Great Britain (National Health Service), and Australia (a hybrid public–private system).

The Canadian Health Care System In 1984, Canada passed the Canada Health Act (Medicare), a publicly funded nationwide federal/provincial health insurance system that is privately delivered and free at the point of access.¹²⁶ Although British Columbia, Alberta, and Ontario require health care premiums, the act prohibits provinces from denying health

services due to financial inability to pay premiums. Although each province is responsible for administering its own health care plan, each plan is essentially universal and comprehensive, covering all residents for inpatient and outpatient hospital and physician services. Six major principles underlie the Canada Health Act:

- **Public Administration:** All administration of provincial health insurance must be carried out by a public authority on a nonprofit basis and be subject to audits.
- **Comprehensiveness:** All approved hospital and physicians’ services must be covered, including medical and hospital care, mental health services, and prescription drugs for those over age 65 and for those with catastrophic illnesses. In addition to the mandated requirements, provinces typically provide additional services such as physiotherapy, dental coverage, and prescription medicines.
- **Universality:** Every provincial resident must be covered. All insured residents must be entitled to—and receive—the same level of health care. There must be free medical care at the point of access and services must contain no financial barriers.
- **Portability:** Residents who relocate to a different province or territory are entitled to coverage from their home province during a minimum waiting period. This also applies to residents who leave the country.
- **Accessibility:** All insured persons must have reasonable access to health care facilities. All health care providers must receive reasonable compensation for the services they provide.
- **Freedom of Choice:** A Canadian is free to choose his or her provider. After obtaining health coverage, the beneficiary registers with a primary care physician.¹²⁷

In addition to public providers, Canada has many private clinics that offer specialized services. The major draw for these clinics is the reduced wait time for voluntary surgeries and procedures, such as MRIs. Private clinic costs are often covered by private insurance plans, which typically pick up around 80 percent of the bill. Some Canadians worry that private clinics will lead to a two-tiered health system that relegates the poor to an inferior public sector.

Unlike the United States, the Canadian system is grounded in universal entitlement rather than being

linked to employment. Except for people covered by other federal programs such as the military, all 33 million Canadian residents are eligible for provincial health insurance, regardless of their employment status. Canada's Medicare system, however, does not include prescription medication, dental, prescription glasses, long-term care or home health care. Provinces provide partial coverage for these services for vulnerable populations, including low-income children. Canadians pay for these services either out-of-pocket or through private insurance plans.¹²⁸

General practitioners (GPs) make up more than 50 percent of the physicians in Canada and provide most of the nation's health care. Specialists are used only if a referral is received from the GP. Although patients may choose their primary care physician, the choice is contingent upon whether the physician has openings for new patients. Patients also use the hospital in which the physician has admitting privileges. Although covered health care is free at the point of access, some elements that are not covered include out-of-hospital drugs, dental care, eyeglasses, physical therapy, and chiropractic care not ordered by a medical doctor.

Contrary to some misconceptions, the Canadian health care system is not socialized medicine; instead, it is based on a social insurance model that mixes public funds with private health care delivery.¹²⁹ Approximately 67 percent of Canada's population elect to buy private insurance coverage that contains extra benefits.¹³⁰ The basis of Canada's single-payer model is the idea that provincial government functions as a single-source payer of health care with a centralized locus of control. Private physicians' fees are negotiated between the provincial governments and the medical associations. The salaries of physicians in Canada are generally lower than their U.S. counterparts.

There are more than 1,250 hospitals in Canada, of which 57 percent are run by religious orders or nonprofit organizations. Hospital reimbursements are made on a global prospective basis. In other words, hospitals operate on a negotiated but fixed yearly budget. As such, they must stay within the budgetary allotment granted by the province regardless of the number of patients seen in a year.¹³¹

Canada's Medicare system is funded by a mixture of federal and provincial funds. Federal funds are directed to the provinces in the form of block grants and transfer payments. The provinces obtain

funds to operate the medical system from general revenue taxes and, in the case of Alberta and British Columbia, from employer-based insurance premiums. Provinces that charge premiums provide exemptions or subsidies for the aged, the unemployed, and the indigent.

Critics of Canadian health care point to numerous problems facing the system, including less-than-adequate funding, increasing per capita health care costs, increased demand for services, and the high cost of technology. Taken together, this has made cost containment the number one issue facing the Canadian health care system.

Some critics have charged that Canada's prospective global budgeting system for hospitals has caused health care rationing. They argue that excessively long waiting lists exist for elective surgeries, rushed medical procedures lead to inaccuracies in diagnosis and treatment, there is widespread use of cheaper medical materials, and hospitals are not investing in technology or capital improvements. Critics also charge that prioritizing illnesses into "urgent," "emergent," and "elective" categories causes artificial queues for treatment.¹³² Accordingly, health care cuts are having a dramatic effect on Canada's medical system.¹³³

Canada's health care system is the subject of much political controversy and debate in the United States. Some question the ability of the system to deliver timely treatment and contain costs. Although in the minority, this group often advocates the adoption of a market system similar to the United States.

In 2012, Canada spent only US \$4,602 per capita on health care costs in comparison to the \$8,745 spent by the United States.¹³⁴ Despite these lower expenditures, the World Health Organization ranks Canada above the United States in the performance of their health care system, with the overall health of Canadians being ranked 35th and Americans 72nd. Despite lower health care expenditures, life expectancy in Canada was 81.76 in 2015 compared to 78.8 in the United States. Infant and child mortality rates are also higher in the United States.¹³⁵ One indicator of the success of Canada's health care system is that a majority of Canadians are generally satisfied with it and show no inclination of giving it up. According to a 2010 Harris/Decima poll, Canadians prefer their own health care system over the U.S. system by a margin of 10 to 1 (82 percent versus 8 percent). Seventy percent of Canadians are either fairly satisfied, or very satisfied, with their system.¹³⁶

The United Kingdom's National Health Service The direct inspiration for the British National Health Service was a 1944 white paper written by Sir William Beveridge, which argued that a "comprehensive system of healthcare was essential to any scheme for improving living standards."¹³⁷ After initial resistance from the British Medical Association, the National Health Service Act took effect in 1948. In the words of the act, the aim was to promote "the establishment of a comprehensive health service designed to secure improvement in the physical and mental health of the people . . . and the prevention, diagnosis and treatment of illness."¹³⁸ The principle of freedom of choice was upheld in that people could either use the NHS or seek outside doctors. Doctors were guaranteed that there would be no interference in their clinical judgment, and specialists were free to take private patients while participating in the service. The NHS was based on a tripartite system: (1) hospital service with specialists; (2) general medical doctors, dentists, and eye doctors that are maintained on a contractual basis; and (3) prevention and support systems, provided by local health departments.

The NHS is currently the world's largest publicly funded health service and its 2015 budget is \$171 billion. In 2014, the NHS employed almost 1.4 million people, of whom more than 150,000 were doctors, 377,000 were qualified nursing staff, 156,960 were scientific, therapeutic, and technical staff; and 37,078 were managers. NHS per capita health care expenditures was roughly \$3,000 compared to close to \$10,000 for Americans.¹³⁹

Each GP sees on average 140 patients a week. The NHS Act does not prohibit private medicine, and a small percentage of NHS hospital beds are reserved for private patients. GPs and specialists are permitted to treat private patients while working in the NHS. Moreover, affluent patients can purchase private health insurance and private care. The major advantages of private care are more attractive hospital rooms and quicker service for elective surgery. About 11 percent of Britons currently have some form of private health insurance, which businesses often provide as a fringe benefit for upper-level management.¹⁴⁰

The backbone of the NHS is the GP, and every patient in Britain is registered with one. GPs are paid by the NHS on the basis of an annual **capitation** (per-person) fee for each registered patient. They have wide professional latitude and equip their own office, can hire staff, and can choose to

work singly, in pairs, in groups, or in a government health center. Since the role of the GP is to provide primary medical care, they are forbidden to restrict their practice to any special client group. Individuals can register with any GP provided they live in the catchment area and the GP is willing to accept them.¹⁴¹

The second tier of the British health care system is the physician consultant (specialist). Most referrals to consultants—except for accidents or emergency care—are made through GPs. Although employed by the government and under contract to a public hospital, a physician specialist is allowed a small private practice. In effect, patients in the community are served by GPs, whereas in the hospital they are under the care of specialists. Similar to the United States, NHS specialists are accorded greater prestige and remuneration.¹⁴²

Health services under the NHS are comprehensive, with free hospital and primary medical care. However, there are patient charges for adult dentistry, eyeglasses, and a small charge for prescriptions. Drug prices are agreed upon between the health department and the pharmaceutical industry according to a specified pricing formula based on company profits. In addition, government subsidies for medical education lower students' direct educational costs.¹⁴³

Several criticisms have been leveled against the NHS, especially in the early 2000s. These included poor sanitary conditions that led to outbreaks of antibiotic-resistant infections, cost overruns on hospital construction, the growth of "health tourism" (people traveling to the United Kingdom to use the NHS without cost), long waiting times for elective surgeries (urgent surgeries are promptly done), and the non-availability of some treatments due to perceptions of poor cost-effectiveness. According to the 2010 Commonwealth survey, 21 percent of British participants reported waiting four months or more for an elective procedure, while 19 percent reported waiting two months or more to see a specialist.¹⁴⁴ Another criticism leveled against the NHS is that hospitals receive nothing extra for efficiently treating more patients at less cost; therefore, they have little incentive to improve efficiency.¹⁴⁵

Still another criticism of the NHS is that its funding is not based on the medical needs of consumers, but on how much the British treasury believes it can afford to spend on health care. The result is de facto health care rationing. Some consumers complain of long waits in GP offices and

hospital buildings in poor repair. Other critics charge that despite government efforts, there are serious shortages of doctors in certain areas of the United Kingdom. Critics complain about the lack of accountability of doctors and about strong unions that have supported restrictive practices and fought attempts to privatize support services.

Much of the reporting on the NHS in the U.S. press has emphasized its flaws. Despite the criticisms, the British people continue to use the NHS in large numbers, and it appears to be serving the majority of the British population well, and in many ways better, than the U.S. health care system. A 2014 Commonwealth Fund study found the NHS to be superior to health care systems in other major industrialized nations, including Australia, Canada, France, Germany, Netherlands, New Zealand, Norway, Sweden, Switzerland, and the United States. The Commonwealth study rated NHS as the top system in terms of efficiency; safe, effective, and coordinated care; and in dealing with cost related problems. A 2014 satisfaction survey of more than 56,000 inpatients in NHS hospitals found that 84 percent rated their overall experience from 7 to 10 on a 10-point scale. Eighty-one percent felt they were treated with dignity; 69 percent felt that their room or ward was very clean.¹⁴⁶ Despite lower per capita expenditures, most health indicators in the United Kingdom, such as life expectancy and infant mortality rates, are equivalent to or better than those in the costlier U.S. health care system.

The Australian Health care System The Australian health care system is a hybrid public-private system. Introduced in 1984, Australia's Medicare system provides universal health care and subsidized pharmaceuticals for all citizens. The Medicare system was predicated on the idea that all Australians should contribute to the cost of health care according to their ability to pay. As such, it is financed by a progressive income tax and an income-related Medicare levy.¹⁴⁷ As a supplement to Medicare, Australians are strongly encouraged to purchase private health insurance, which in 2015 could cost up to \$400 a month for a family at top-level coverage. In 2015, approximately 50 percent of Australians had private health insurance to gain access to private hospitals and to receive services not covered by Medicare.¹⁴⁸ Private health insurance only covers hospital care and some supplemental services such as eyeglasses, minor dental, chiropractic, physiotherapy, and so forth.

The publicly funded Medicare system provides comprehensive benefits such as consultation fees for doctors, including specialists; medical tests like X-rays, MRIs, and pathology tests; eye tests performed by optometrists; most surgical and therapeutic procedures; and some surgical procedures performed by approved dentists. Mental health services paid for by Medicare have broadened to include services to families and individuals.

One can enter an Australian hospital either as a public or private pay (using health insurance) patient. Public hospital patients are not charged for care and treatment by attending physicians or specialists, or aftercare by the treating doctor. However, the treatment they receive is from doctors and specialists nominated by the hospital. Those admitted to a public or private hospital as a private patient (i.e., using private health insurance) have a choice of doctors.¹⁴⁹

Medicare does not cover most dental examinations and treatment (except for children), ambulance services, home nursing, acupuncture (unless part of a doctor's consultation), glasses and contact lenses, hearing aids and other appliances, prostheses, medicines (except those subsidized by the Pharmaceutical Benefits Scheme, which covers most classes of medicines), and cosmetic surgery.

Criticism leveled against Medicare is similar to that directed at the British NHS—namely, long waits for elective surgery as a public patient (private patients have little or no wait), inefficiencies, staff shortages, and underfunding.¹⁵⁰ The 2010 Commonwealth Fund survey found that 18 percent of Australian participants waited four months or more for an elective procedure, and 28 percent waited two months or more to see a specialist, which is comparable to the waiting periods in the United Kingdom.¹⁵¹ Other criticisms involve co-payments and gaps in services, such as dental coverage. Despite these criticisms, Australians are pleased with their system and their health indicators are positive (i.e., they have one of the longest life expectancies in the world). Only 8 percent of the 2010 Commonwealth Fund survey participants experienced difficulty paying their medical bills within the last year; 76 percent of Australians were confident that if they were seriously ill they would receive the best care.¹⁵²

Although the U.S. health care system is vastly more expensive than other Western countries, its health outcomes are roughly equivalent to—and in some cases inferior to—that of countries such

as Britain, Canada, and Australia. For affluent or middle-class Americans with good health insurance, the U.S. system provides some of the best medical care in the world. And the United States is unequaled when it comes to complex medical procedures involving sophisticated equipment and technology. Unlike the long queues for elective surgery in the United Kingdom, Canada, and Australia, the wait for surgeries, tests, and other procedures is relatively short in the United States. Finally, U.S. physicians are among the best trained in the world. However, the emphasis on costly equipment and technology does not come without a price. A medical approach that emphasizes disease treatment over primary care and preventive medicine usually results in good care, but for fewer people. Health care systems that emphasize personal and primary care, accessibility, and free or inexpensive services typically reach more people. Hence, the Canadian, British, and Australian systems distribute health resources more equitably than the U.S. system.

Reforming U.S. Health Care

The American health care system is partly driven by ideological values. Primary among these is whether **health care access** should be a right or a privilege. Conservatives believe that access to health care is a privilege that must be earned through past or present labor force participation. Progressives believe that health care should be a universal right not tied to labor force participation, and a birthright granted to all citizens.

An important issue in the health care debate is the role of the private market. Conservatives believe that medical care is like other commodities that benefit from increased competition resulting in lower prices and higher quality. As such, health care should be lodged squarely in the private marketplace and that medical institutions and drug companies should be free to establish prices. In that vein, government regulation of health care is seen as leading to inefficiencies, lower quality, poorer service, and higher prices.

Although some liberals believe that the private marketplace is the proper venue for health care delivery, they also recognize that it does not respond to market conditions like other commodities. For example, a person in cardiac arrest does not shop around for the lowest hospital prices. Likewise, most patients facing serious surgery will

not choose a surgeon based on price point. For these liberals, the health care marketplace must be regulated by government to ensure quality, price, access, and equity. Other progressives argue that health care is too important to be left to the vicissitudes of the marketplace, and as such, it should be removed from a context where decisions are based solely on economics. For these thinkers, health care must be free or heavily subsidized at the point of access, universal in its coverage, and administered and controlled by government.

Most proposals to reform or transform U.S. health care have fallen into three basic categories: (1) removing health care from the marketplace (i.e., National Health Service), (2) maintaining the private health care system while providing broad or universal coverage through national health insurance, and (3) small incremental reforms designed to soften the abrasive features of the system.

National Health Service

The most radical proposal for reforming the U.S. health care system involved the creation of a National Health Service (NHS), similar to the British National Health Service. Proposed in the mid-1970s, the NHS would have established health care as a right of citizenship and provided free and comprehensive health care coverage, including diagnostic, therapeutic, preventive, rehabilitative, environmental, and occupational health services. To reduce the maldistribution of medical services, the NHS would have provided free medical education in return for required periods of service in medically underserved areas.¹⁵³ NHS supporters argued that it would allow for the coordination of health services and reduce profiteering by corporations. Moreover, the experience of other countries demonstrates that a system incorporating strict budgeting, nationalization, and elimination of profit arrests the growth of health care costs and, in the end, is less expensive than a privatized system.

National Health Insurance

Single-Payer System The National Healthcare Act of 1992 proposed restructuring the U.S. health care system along the lines of a **single-payer system** similar to Canada. As such, states would have the responsibility for ensuring the delivery of health

services, for paying providers, and for planning in accordance with federal guidelines. Although this plan would have allowed private medicine, the act would have discontinued private health insurance coverage.¹⁵⁴ Supporters claimed that a single-payer system would immediately cut health care costs by up to 18 percent and reduce the fraud endemic to a multiple-payer system.¹⁵⁵

Another strategy to reform the U.S. health care system is **national health insurance** (NHI). According to Paul Starr, the United States was on the brink of establishing national health insurance several times during the twentieth century, but each time factors unique to the United States' political and social institutions prevented its adoption.¹⁵⁶ For example, NHI plans began to proliferate in the 1930s as part of Roosevelt's New Deal, but the idea was abandoned because of the strident opposition of the AMA and the fear that its inclusion would jeopardize passage of the 1935 Social Security Act. Former president Harry Truman took up the NHI in the days following World War II, but by that time most middle class and unionized workers were covered by private insurance plans. Moreover, the AMA again set its powerful lobbying machine into motion, this time equating national health insurance with socialized medicine and communism.¹⁵⁷

The most recent incarnation of NHI occurred in the Health Security Act proposed by former president Bill Clinton in 1993. Although the proposal was not an NHI per se, it contained important components of national insurance. Under the proposed plan, all citizens and legal immigrants would get a card guaranteeing them a comprehensive lifelong package of health care benefits. Coverage would be continuous regardless of employment status, and all participants would buy into large purchasing pools called Healthcare Alliances. Charges for low-income self-insurers would be on a sliding scale, and those on public assistance would have their fees paid by Medicaid. Employers would pay 80 percent of the benefit package and workers the additional 20 percent.

The bill was doomed almost from its inception. Almost immediately, the Health Insurance Association of America (HIAA) broadcast \$2 million worth of "Harry and Louise" commercials, attacking the act as rationing health care under socialized medicine. Small business lobbyists argued that the employer costs would bankrupt them.¹⁵⁸

Liberal critics charged that NHI plans modified payment mechanisms rather than leading to major changes in the health care system. Although NHI schemes would equalize the ability of patients to pay, they would not improve the accessibility or the quality of services. Furthermore, most NHI proposals called for coinsurance (copayments) in amounts that many poor people could not afford.¹⁵⁹ Contrary to critics, NHI schemes are not socialized medicine since hospitals and physicians remain private, and most plans preserved a major role for private insurance companies.

Incremental Reform

Incremental health care reform is often more acceptable to the American public than sweeping reforms. This approach has generally focused on rectifying the more troubling aspects of the health care system and on fine-tuning public health care programs.

One conservative approach to incremental health care reform is based on individual medical savings accounts (MSA), a form of self-insurance whereby individuals purchase high-deductible health insurance while setting aside pretax dollars to pay for their medical expenses. Opponents of MSAs argue that these plans benefit only the healthy and wealthy, leaving those with less money and more health problems behind in an increasingly costly insurance pool.

Other reforms have been enacted on state levels. In Hawaii, employers are required to insure employees who work more than 20 hours a week.¹⁶⁰ A Health Rights program in Minnesota extends health care coverage to all non-covered, low-income residents and charges them on a sliding-scale basis. Under then Governor Mitt Romney, Massachusetts enacted health care legislation in 2006 that required nearly every resident of the state to obtain a state-government-regulated minimum level of health care insurance coverage. The bill provided free health care insurance for residents earning less than 150 percent of the poverty threshold (and a partial subsidization for those earning up to 300 percent of the poverty threshold) who do not receive Medicaid. The legislation also incorporated tax penalties for those failing to obtain an insurance plan. The Massachusetts act formed the basis of the 2010 Patient Protection and Affordable Care Act, the most expansive piece of social legislation enacted in decades.

The Patient Protection and Affordable Care Act (P.L. 111-148)

On March 23, 2010, former president Barack Obama signed into law the 2,000-page Patient Protection and Affordable Care Act (ACA). This legislation was incremental—yet in some ways also comprehensive—and was based on an NHI model that required everyone to be covered by a health care plan. In particular, the legislation extended coverage to the 32 million people in 2010 without health insurance coverage. The ACA contained the following features:

- **Enrollment and Employer Requirements:** The ACA required most U.S. citizens and legal residents to have health insurance. Almost everyone by 2014—with the exception of some low-income people—must have purchased health insurance or face a \$695 annual fine. Those without health insurance would pay a tax penalty of the greater of \$695 a year up to a maximum of three times that amount (\$2,085) per family or 2.5 percent of household income. The penalty would be increased annually after 2016 by a cost-of-living adjustment. Exemptions were granted for financial hardship, religious objections, American Indians, those without coverage for less than three months, undocumented immigrants, incarcerated individuals, those for whom the lowest cost plan option exceeded 8 percent of their income, and those with incomes below the tax filing threshold. Companies with more than 200 employees were expected to automatically enroll them in a health insurance plan. Employees could opt out of coverage.
- **Cost and Funding:** According to the Congressional Budget Office (CBO), the ACA was expected to cost \$940 billion over 10 years, while reducing the deficit by \$143 billion in the first 10 years and \$1.2 trillion in the subsequent decade. The ACA is to be partly paid for by the Medicare Payroll tax on investment income. Beginning in 2018, insurance companies will pay a 40 percent excise tax on high-end insurance plans worth more than \$27,500 for families or \$10,200 for individuals. Dental and vision plans are exempted in the total cost of a family's plan.
- **Health Insurance Exchanges (HIE):** The uninsured and self-employed could purchase insurance through state-based HIEs. Separate exchanges were to be created in 2014 for small businesses to purchase coverage. The Secretary of HHS was to set up a new website to make it easy for people in any state to find affordable health insurance options. This website would also include information for small businesses. Illegal immigrants were not permitted to purchase health insurance in the exchanges, even if they pay with their own money.
- **Subsidies:** Low-income individuals and families (i.e., those with incomes between 100 and 400 percent of the poverty line) who purchased health insurance in an exchange were to be eligible for subsidies if (1) they are ineligible for Medicare and Medicaid and (2) they are not covered by an employer. A sliding scale and cap was established for how much enrollees would have to contribute to their premiums. Businesses with fewer than 50 employees would receive tax credits covering up to 50 percent of employee premiums. The bill also established a temporary program to reduce the cost to companies providing early retiree health benefits for those aged 55 to 64 years.
- **Regulation of the Health Insurance Industry:** (1) By 2014, all lifetime caps on insurance coverage were to be banned, as were annual caps; (2) insurance companies could not deny children or adults coverage based on a pre-existing condition; (3) insurers must allow children to stay on their parent's insurance plans until age 26; (4) rescissions (arbitrarily cutting off consumers with expensive medical conditions) was prohibited, and insurers could no longer drop seriously ill consumers; (6) greater transparency was required as insurers had to reveal their overhead costs; (7) any new plans had to include a customer appeals process for coverage and claims; (8) new plans had to include free checkups and other preventative care; (9) new screening procedures were devised to help curb fraud and waste; and (10) nonprofit Blue Cross organizations were required to maintain a medical loss ratio (i.e., money spent on procedures over incoming money) of 85 percent or higher to take advantage of IRS tax benefits.
- **Public Health:** Chain restaurants were required to provide a nutrient content disclosure statement alongside menu items.
- **Medicare:** The ACA was to close the Medicare prescription donut hole by 2020. Medicare

payment protections were to be extended to small rural hospitals and other health care facilities with a small number of Medicare patients.

- **Medicaid:** Starting in 2014, the bill expanded Medicaid to all individuals under age 65 with incomes up to 133 percent of the poverty line. The federal government would pay 100 percent of costs for covering newly eligible individuals through 2016, and they would be guaranteed a package that provided essential health benefits. Illegal immigrants were ineligible for Medicaid.¹⁶¹
- **Abortion:** Health care plans were not required to offer abortion coverage. States could pass legislation choosing to opt out of offering abortion coverage through the exchange. No federal funds could be used to pay for abortions, except in the case of rape, incest, or the health of the mother.¹⁶²

One is hard-pressed to understand the extreme Republican vitriol toward Obamacare, especially given the absence of an alternative proposal. By early 2016, the Republican Congress had voted more than 50 times to repeal all or parts of the ACA. In none of those instances was an alternative to the ACA proposed. Opponents took the issue to the U.S. Supreme Court, specifically focusing on the constitutionality of the ACA's "individual mandate"—the part of the bill that requires the purchase of health insurance.¹⁶³ On October 28, 2012, the U.S. Supreme Court delivered its long-anticipated ruling on the constitutionality of the ACA. Overall, the Court upheld the ACA. Of particular importance, the Court's decision left in place the individual mandate. Also retained were Health care Exchanges (designed to offer cheaper health care plans), the requirement that insurers cover the children of those they insure up to age 26 (affecting about 2.5 million people in 2012 from age 19 to 25), the requirements around insuring pre-existing medical conditions, and the rule that firms with more than 50 or more full-time employees must provide health care coverage or face fines. Despite upholding most of the ACA, the Court ruled that the federal government could not require states to expand their Medicaid coverage. In effect, it struck down the provision that permitted the federal government to remove existing Medicaid funding from states that refuse to participate in the expansion of Medicaid to formerly

non-covered groups.¹⁶⁴ By early 2016, 17 states had opted to not expand Medicaid coverage. In June 2015, the Supreme Court upheld a key part of the Affordable Care Act that provides health insurance subsidies to qualifying Americans.



Watch this video for more information on how the Affordable Care Act has affected both the previously insured and the previously uninsured.

www.youtube.com/watch?v=EQG8eZs8AJc

Conclusion

Important issues remain unresolved in the American health care system. For instance, what is the extent of the government's responsibility to provide health care to all its citizens? How much high-tech medicine can society realistically afford? How should U.S. medical resources be allocated? How much emphasis should be placed on preventive care as opposed to treatments with questionable results? What, if any, limitations on personal freedom are permissible in the name of promoting health and preventing disease? These and other questions require thoughtful answers.

The U.S. health care system is facing an acute crisis partly based on the failure of the marketplace to curb health care costs, the system's overreliance on medical technology at the expense of primary care, huge administrative costs, and the large number of working families unable to afford health insurance or the ACA. Moreover, compared to industrialized countries with less-costly health care systems, the expensive U.S. system does not result in greater longevity, lower rates of infant mortality, or other indicators of improved public health.

The United States is one of the few industrialized countries without a universal health care system. Moreover, it is also one of the few industrialized nations where medical expenses can cause poverty. At minimum, comprehensive health care reform should incorporate principles that include (1) universality (making health care a right rather than a privilege), (2) removing health care from the marketplace (or at least subjecting it to stricter government oversight), (3) curbing the rise in pharmaceutical prices by creating a national formulary that negotiates the best price, (4) assuring that the quality and accessibility of health care is not dependent on income (equity), (5) placing more emphasis on providing accessible primary care over specialized

care, and (6) limiting administrative costs. Accessible health care should be a universal right for all U.S. citizens, not a privilege to be purchased or earned through workplace participation.

DISCUSSION QUESTIONS

1. In 2015, U.S. health care expenditures were roughly \$3 trillion a year. This cost has risen dramatically over the past 25 years in terms of the amount spent and the percentage of the GDP it consumes. Identify the main factors that have driven up health care costs, and discuss how they can be controlled.
2. Medicare has experienced large increases in costs. What are the major factors contributing to this steep rise? What, if anything, can be done to curb costs?
3. Some critics argue that the costs of Medicare and Medicaid cannot be brought under control without radically reforming the entire health care system. They argue that incremental reforms have only a minimal impact on the rise in federal and state health care expenditures. Are they correct?
4. Evidence of the effectiveness of cost-controlling mechanisms, such as PPS, has been mixed. Critics charge that not only has the PPS system failed to substantially reduce health care costs, but it has also led to a reduced level of patient care. What, if anything, can be done to reform the system.
5. Critics argue that America is experiencing a severe health care crisis. Describe the main characteristics and symptoms of that crisis.
6. Some health care analysts are calling for radical reform in the U.S. health care system. Many insist that the nation's free market health system should be replaced by a more cost-effective and comprehensive approach. Assuming that these analysts are correct, which of the health care systems described in this chapter would be the best model for the United States? Why?
7. Why has the United States not developed a health care system that is universal and publicly funded like those of its industrialized counterparts?
8. Anyone who watches television has seen the dozens of advertisements for prescription drugs. Many drugs have their own website offering discounts or even free products. Critique the aggressive marketing for new medications by pharmaceutical firms. How does this information affect the confidence that you have in federal oversight of the prescription drug industry?

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Mental Health and Substance Abuse Policy



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Despite the nation's abysmal record in mental health and substance abuse, American psychiatry largely determined the methods for intervening in mental disorders, primarily through the *Diagnostic and Statistical Manual (DSM)*, a publication of the American Psychiatric Association. Although the reliability of the *DSM* has been challenged frequently,¹ it remains a system for classifying emotional disorders, if only because of the absence of a more valid competitor. As shown in Table 13.1, the lifetime prevalence of psychiatric disorders varies significantly from country to country. It is notable that among these nations, the United States not only leads with respect to types of disorders but comorbidity as well.

Mental Health Reform

Begun early in the twentieth century, the National Mental Health Association (NMHA) pioneered efforts to provide social support and treatment for the mentally ill. NMHA was frequently critical of the custodial institutions operated by state governments. For instance, during the Progressive Era, the eugenics movement contributed to the institutional abuses that were condemned by NMHA. Proponents of eugenics argued that mentally impaired patients often suffered from hereditary deficiencies that should be prevented by sterilization. Adherents of eugenics were less concerned about the civil rights of individual mental patients than they were about the future of civilization—the fact that some patients might object was merely an inconvenience.

In such instances, eugenicists obtained court permission to sterilize patients without their consent. Approved by the Supreme Court in the 1927 case *Buck v. Bell*, in which the critical determination of Carrie Buck's social degeneracy was made by a social worker, 65,000 Americans would ultimately be sterilized involuntarily.² Many patients, of course, lacked the mental capacity to comprehend sterilization and had no idea that the surgical procedures to which they were subjected would terminate their ability to reproduce.

The issue of mental health attracted even wider public attention during World War II, when approximately one in every four draftees was rejected for military service because of psychiatric or neurological problems.³ In response to public outcry about mental health problems immediately after the war, Congress passed the Mental Health Act of 1946, which established the National Institute of Mental Health (NIMH). Accompanying the Mental Health Act was an appropriation for an exhaustive examination of the mental health needs of the nation. In 1961, NIMH released *Action for Mental Health*, a report that called for an ambitious national effort to modernize the U.S. system of psychiatric care.⁴

Action for Mental Health was a utopian vision of mental health care, the idealism of which coincided perfectly with a set of extraordinarily propitious circumstances. First, the postwar economy was booming, and, with cutbacks in military expenditures, a surplus existed that could be tapped for domestic programs. Second, a new generation of drugs—psychotropic medications—showed promise of being able to stabilize severely psychotic

TABLE 13.1 Percentage Prevalence of Psychiatric Disorders

Lifetime Estimates	Brazil	Canada	Germany	Mexico	Netherlands	Turkey	United States
Any anxiety	17.4	21.3	9.8	5.6	20.1	7.4	25.0
Any mood	15.5	10.2	17.1	9.2	18.9	7.3	19.4
Any substance	16.1	19.7	21.5	9.6	18.7	0.0	28.2
Any disorder	36.3	37.5	38.4	20.2	40.9	12.2	48.6
One disorder	21.2	21.2	25.5	14.8	23.0	7.9	21.3
Two disorders	8.8	9.3	8.1	4.0	9.3	3.0	13.1
Three disorders +	6.3	7.0	4.8	1.4	8.6	1.3	14.3

Source: WHO International Consortium in Psychiatric Epidemiology, "Cross-National Comparisons of the Prevalences and Correlates of Mental Disorders," *Bulletin of the World Health Organization*, 2000, 78(4): 417. Used by permission from WHO.

patients who before had been unmanageable. Third, literature was emerging that was critical of the “total institution” concept of the state hospital and implied that non-institutional—and, presumably, community—care was better. Finally, because of his experience with mental retardation as a family problem, a sister who was seriously impaired after a lobotomy, former president John F. Kennedy was supportive of programs that promised to improve mental health care.⁵

These political and social circumstances did not go unnoticed by Dr. Robert H. Felix, a physician who had grown up with the Menninger family in Kansas and had developed a sharp critique of the state mental hospital as an institution for the care of emotionally disturbed patients. Felix was later to become director of NIMH. A primary architect of the community mental health movement, Felix was able to draw on his extensive experience in the Mental Hygiene Division of the U.S. Public Health Service as well as on the breadth of professional and political contacts that three decades of public service afforded.⁶ Felix’s objective was as simple as it was radical. Through the community mental health movement, Felix would use federal legislation to reform the archaic state mental hospitals. The laws that enabled NIMH to reform mental health care were the Community Mental Health Centers Acts of 1963 and 1965.

The Community Mental Health Centers Acts

Under the propitious circumstances of the postwar era, the first Community Mental Health Centers (CMHC) Act was passed by Congress and signed by President Kennedy on October 31, 1963. The enactment of CMHC legislation was not, however, without obstacles. To allay the American Medical Association’s fears that the act represented socialized medicine, the CMHC Act of 1963 appropriated funds only for construction purposes. It was not until 1965, when the AMA was reeling from governmental proposals to institute federal health care programs for the aged and the poor, that funds were authorized for staffing CMHCs. Advocates of the CMHC Acts of 1963 and 1965 maintained that a constant target of the legislation was “to eliminate, within the next generation, the state mental hospital, as it then existed.” The strategy of the mental health leadership and its allies was

to “demonopolize” the state’s role in the provision of mental health services and to attempt to establish a triad of federal, state, and local support for mental health services. At this time, federal bureaucrats planned to blanket the whole country with comprehensive community mental health services. Their intention was not to federalize the total program through its financing but to obtain a degree of control through the resulting federal regulations and standards.⁷

The philosophical basis for transferring mental health care from the state hospital to the community was borrowed from public health, which had developed the concept of prevention. Proponents of community mental health presumed that services provided in the community would be superior to the warehousing of patients in state institutions. According to the CMHC Acts, the United States was to be divided into catchment areas, each with a population of 75,000 to 200,000 persons.⁸ Eventually, NIMH planned a CMHC for each catchment area, some 2,000 in all.⁹ Programmatically, each CMHC was to provide all essential psychiatric services to the catchment area: inpatient hospitalization, partial hospitalization, outpatient services, 24-hour emergency services, and consultation and education for other service providers in the community. Soon after initial passage of the CMHC Act, child mental health as well as drug abuse and alcoholism services were added to the array of services provided. To assure that patients were not lost among programs within the CMHC network, *case management* was included, whereby every case was assigned to one professional who monitored the patient’s progress throughout treatment. Financially, NIMH provided funding to disadvantaged catchment areas through matching grants over an eight-year cycle. At the end of the cycle, the catchment area was supposed to assume financial responsibility for the CMHC.¹⁰ With this framework, mental health reformers believed that the CMHC was an effective alternative to the state hospital.

Deinstitutionalization

Enthusiasm for community mental health reform ebbed when a series of circumstances that were beyond the control of the CMHC architects began to subvert the movement. By the end of Carter’s presidency, 691 CMHCs received

federal assistance. With the Omnibus Budget and Reconciliation Act of 1981, however, the Reagan administration collapsed all mental health funding into a block grant available to states for any mental health services they deemed fundable. As a result, the designation of CMHCs for direct receipt of federal funds ceased in 1981.¹¹

In the meantime, however, many states had planned to shift responsibility for the mentally ill to the CMHCs. In fact, the community mental health movement had proved a timely blessing for officials in states where the maintenance of archaic state hospitals was an increasing economic burden. As states discharged patients from state institutions, they realized immediate savings. By the end of the 1990s, 93 percent of the state psychiatric beds that had existed in 1955 were lost to **deinstitutionalization**.¹²

Deinstitutionalization was further confounded by a series of judicial decisions in the mid-1970s that enhanced the civil rights of mental patients while at the same time requiring states to provide them with treatment. In *Wyatt v. Stickney*, Alabama District Court judge Frank Johnson ruled that the state of Alabama was obliged to provide treatment to patients in state hospitals, a judgment with which the state subsequently failed to comply. Shortly thereafter, in *Donaldson v. O'Connor*, the Supreme Court determined that “the state could not continue to confine a mentally ill person who was not dangerous to himself or others, who was not being treated, and who could survive outside the hospital.” Finally, in *Halderman v. Pennhurst*, the Third District Court established that institutionalized patients deserved treatment in the “least restrictive alternative.”

As a group, these rulings had a profound effect on institutional care for patients with mental impairments. Only persons dangerous to themselves or others could be hospitalized involuntarily. For those hospitalized, involuntarily or otherwise, states were obliged to provide adequate treatment in the manner that was least restrictive to the patient. These decisions promised to be enormously costly to state officials who were trying to curb mental health expenditures. To comply with the court decisions, states would have to pump millions of dollars into the renovation of institutions that had been slated for closure. The solution, in many instances, was to use a narrow interpretation of *Donaldson* to keep emotionally disturbed people out of state institutions. In other words, judicial

decisions, coupled with the fiscal concerns of state officials, provided a convoluted logic that served to justify first emptying state hospitals of seriously disturbed patients and then requiring the manifestation of life-threatening behavior for their rehospitalization. If people were not hospitalized in the first place, the states bore no obligation to provide the adequate, but expensive, treatment demanded by *Wyatt*. The criteria for hospitalization specified the most serious self-destructive behaviors; once admitted, however, patients were stabilized as quickly as possible and then discharged. As a result, those in greatest need of mental health services—patients who were seriously mentally ill—were often denied the intensive care they needed.

The consequences of court decisions for the mentally ill were substantial. In his interpretation of the legal decisions influencing mental health services, Alan Stone, a psychiatrist and a professor at Harvard Law School, observed that the true symbol of the Supreme Court *Donaldson* decision was a bag lady.¹³ Thus, legal decisions favoring the mentally ill often proved illusory; in the name of enhancing the human rights of people with mental illness—but with no corresponding improvement in services—they offered those people nothing more than the right to be insane.¹⁴

As a result of a foundering desinstitutionalization movement, mental health advocates became more assertive. In 1979, former mental patients and their families established the National Alliance for the Mentally Ill (NAMI) to advocate on behalf of the seriously mentally ill. In 2000, NMHA changed its name to Mental Health America (MHA), but was soon engulfed in a controversy when it was revealed that donations from pharmaceutical companies were made in exchange for support for psychoactive medications to treat mental disorders. The politicization of mental health care thus compromised the best interests of care for those suffering from mental disorders.

The shortfall of the community mental health movement, states' transfers of patients from mental hospitals, judicial decisions assuring patients of their civil rights, and the deinstitutionalization movement, all combined to leave tens of thousands of former mental hospital patients adrift. Although some former mental hospital patients were able to deal with community agencies in order to obtain mental health care, many of the seriously mentally ill were left to themselves.¹⁵ In and out of mental institutions, patients received treatment only when they



The high incidence of readmissions for psychiatric hospital patients is a chronic problem in mental health care.

Source: Jim Noelker/The Image Works

were dangerous to themselves or others, were stabilized on psychoactive medication, and quickly discharged, a process that occurred so frequently that it became known as “the revolving door.”

The Advent of Psychotropic Medication

As state hospitals converted from long-term custodial care to short-term patient stabilization, psychotropic medication became a routine form of treatment. But the psychopharmacological revolution, though congruent with the relatively orderly march toward deinstitutionalization in the late 1960s, actually exacerbated the psychiatric chaos of two decades later. Shown to stabilize psychotic patients until interpersonal treatment methods could be employed, the major tranquilizers—Prolixin, Thorazine, Haldol, and Stelazine, to name a few—seemed clinically indicated within the controlled environment of the hospital. In a community setting, however, psychotropic medication became problematic. Once stabilized on major tranquilizers, patients frequently found the side effects of the medication—dry mouth, nervousness, torpor, lactation in women, and impotence in men—unacceptable and stopped taking the medication. Yet, without medication such patients frequently decompensated, and

without the regular supervision of psychiatric personnel, patients disappeared into inner-city ghettos or rural backwaters, adding to an already growing homeless population.

Discharged from state hospitals but having few prospects of care in the community, thousands of the seriously mentally ill populated skid-row, sought shelter beneath bridge overpasses, panhandled in urban centers, and often turned to petty crime to gain income, thus clogging local courts. In the absence of mental health care, increasing numbers of desperate former mental hospital patients thus became entangled in a criminal justice system that was ill prepared to manage them. Commenting on the surge in arrests of the mentally ill, one mental health worker became exasperated: “These people are forced to commit crimes to come to the attention of the police and get help.”¹⁶

By 2000, the Justice Department reported that 283,800, or 16 percent, of inmates in local and state correctional facilities suffered from mental disorders¹⁷ and another 550,000 were on parole.¹⁸ In 2003, Human Rights Watch reported that U.S. prisons contained three times more psychiatric patients than mental hospitals. This institutional mismatch had come to the point that the largest psychiatric facility in the nation was the Los Angeles County Jail, which held 3,400 mentally ill inmates; second was New York’s Rikers Island with 3,000 mentally ill inmates.¹⁹

As noted in the Criminal Justice chapter, police are often called to deal with deviance associated with mental disorders. Since 2011, Albuquerque police have killed several mentally ill citizens,²⁰ including James Beard, 38, who was homeless. Mr. Beard's death sparked protests in that city in response to police brutality.²¹ Once incarcerated, mentally ill inmates pose problems that correctional officers are ill prepared to serve. Chicago and New York are just two cities with jails housing thousands of mentally ill offenders.²² With capital offenses, the mentally ill present a conundrum: should they be executed for irrational acts? In 2007, the Supreme Court ruled in *Panetti v. Quarterman*, that offenders must have a "rational understanding" of the state's plan to be executed.²³

As the number of mentally ill miscreants grew, the local jail replaced mental health clinics as the source of care.²⁴ In the first census of the incarcerated mentally ill, the Treatment Advocacy Center counted 356,268 inmates with serious mental disorders, compared with 35,000 in state hospitals. Because jails and prisons are poorly equipped to care for the mentally ill, the behavior of disturbed inmates frequently resulted in assaults by other inmates, and some of the mentally ill committed suicide. Incarceration would prove catastrophic for many of the mentally ill:

- A schizophrenic Georgia man, jailed for walking into traffic and knocking on doors, gouged out his eyes in his cell.
- A schizophrenic man in North Carolina slashed his genitals with a razor while waiting to go to trial.
- A mentally ill Florida man cut open his stomach and, after it had been stapled, removed and ate the staples; subsequently, inmates gave the man objects that he shoved into the wound.
- A schizophrenic Texas man gouged out one eye in county jail, then four years later gouged out the other eye in state prison and ate it.
- A mentally ill Minnesota man, found shouting gibberish while standing in his own feces, later stabbed out both his eyes with a pencil in county jail.

Solitary confinement is a common disposition for the mentally ill when their aberrant behavior is either unmanageable by correctional officials or their safety is threatened by other inmates. In New York, one-fourth of inmates in solitary confinement are mentally ill, while the number in Colorado is over half.²⁵

As a result of the horrific conditions encountered by the mentally ill in jail, reformers sought to alter judicial policy by creating special courts to deal with nonviolent offenders who had mental disorders. Paralleling special drug courts that had emerged in several metropolitan areas during the 1990s to contend with nonviolent drug abusers, the mental health court concept was supported by mental health advocates.²⁶ In 2000, Congress passed America's Law Enforcement and Mental Health Project Act. The Mental Health Courts Grant Program subsequently supported the creation of 23 mental health courts in 2002 and 14 more in 2003. By the end of 2004, a consortium of mental health advocacy organizations reported that 99 mental health courts were operating.²⁷ Federal funding lagged behind the initiative, however; for 2012, only \$4.4 million in federal funds was available for special courts dealing with mental health and substance abuse problems.²⁸

The Psychopharmacological Scandal

Meanwhile, a second generation of psychoactive medication was being aggressively marketed to the middle class by the pharmaceutical industry. Psychopharmacologists viewed the brain as the focus of their work, discerning those chemicals that altered mood and temperament, testing dosages of new preparations, and evaluating their effects on a larger array of disorders. The Food and Drug Administration validated the biomedical model by approving psychoactive medications through Randomized Clinical Trials (RCTs) that had been conducted by the drug companies.

By century's end, biomedical psychiatry might have been wedded to talk therapy of psychoanalysts, had not managed care intruded into American health care. Health care inflation, increasing by double digits, prompted reformers to advocate for greater efficiencies at the same time focusing on prevention of illness, virtues that were evident in health maintenance organizations, which spread across the nation subscribing tens of millions of members. The possibility of a psychiatric ideal, integrating psychopharmacology with psychoanalysis, was precluded as health insurance companies pared reimbursements to hospitals and clinics, cutting the number of patient sessions that would be paid for as well as their value, in the process

reducing the revenue of mental health providers. Biomedical psychiatrists who could demonstrate relatively quick patient improvements benefitted from short-term accountability, compared to psychoanalysts whose method was more protracted. As cost considerations came to dominate American health care, biomedical psychiatrists were encouraged to abbreviate their sessions further, effectively minimizing talking therapy in favor of dispensing medication.²⁹ The reorganization of psychiatry by managed care instructed mental health professionals that the real money was in short-term psychopharmacology. This, of course, did not prohibit patients from seeking care from psychoanalysts, but they would have to pay out of pocket. Since only a small slice of mental patients had such resources, mainstream psychiatry became biomedical almost exclusively, the 15-minute medication session dominating the field.

The ascendancy of the biomedical model was facilitated by the publication in 1980 of the *DSM III*, which replaced the ambiguous Freudian descriptions of 83 mental disorders of the previous edition with a behavioral classification system of 265 diagnoses. Each diagnosis listed behaviors, which, if added in sufficient numbers, would justify a psychiatric label. As critics would contend, this is medicine determined not by etiology, but by committee. “Not only did the *DSM* become the bible of psychiatry, but like the real Bible, it depended a lot on something akin to revelation,” observed Marcia Angell, former editor of *The New England Journal of Medicine*, “There are no citations of scientific studies to support its decisions.”³⁰ By 1994, the *DSM IV* had expanded to 297 disorders; at 886 pages, the “manual” was so large that the American Psychiatric Association (APA) and the National Institute of Mental Health (NIMH) convened a group assigned to simplify the *DSM V* through a numerical system that also included new evidence from neurobiology and genetics, a “paradigm shift.” When it was learned that the drafting group had been sworn to secrecy about its work, a complaint by Dr. Robert Spitzer, who was largely responsible for earlier versions of the *DSM*, resulted in the termination of the venture. Spitzer’s critique of a broadening of categories in the *DSM V* was sharp: the consequence “would be a wholesale imperial medicalization of normality that will trivialize mental disorder and lead to a deluge of unneeded medication treatment—a bonanza for the pharmaceutical industry but at a huge cost

to the new false positive ‘patients’ caught in the excessively wide *DSM V* net.”³¹ Unfazed, the APA published the *DSM IV-TR* (text revised) in 2000, including 365 diagnoses.³²

By then, American psychiatry had become a virtual subsidiary of the drug companies, Big Pharma. In 1987, Eli Lilly introduced the first blockbuster psychoactive drug, Prozac, and received approval by the FDA in 1994 to market the drug for depression and bulimia in 1996, resulting in annual sales of \$2.6 billion, one-fourth of the firm’s revenues.³³ An SSRI (selective serotonin reuptake inhibitor), Prozac proved so successful that similar antidepressants, such as Paxil, Zoloft, and Celexa were soon introduced. In approving new drugs, the FDA only requires two clinical trials demonstrating a drug’s efficacy; however, the approval process was fraught with deceit. Upon discovering that 40 percent of the clinical trials for SSRIs had been withheld from public view, psychologist Irving Kirsch used the Freedom of Information Act to obtain the results of all of the clinical trials for antidepressants, which revealed that “82 percent of the response to medication had also been produced by a simple inert placebo,” an almost insignificant difference of two points on the 51-point Hamilton depression scale, insignificant for clinical purposes.”³⁴

Even more alarming is the failure of the psychopharmacological industry to demonstrate the chemical process by which SSRIs actually worked, since biomedical research failed to show the causal effect. The “chemical imbalance” theory, a simplification holding that deficiencies of chemicals in the brain cause diagnosable disorders, is fundamental to antidepressants. Absent the causal process, the SSRIs enjoy the same status of aspirin, a remedy for symptom management. As behavioral pediatrician Lawrence Diller observes, “Aspirin fixes headaches, yet no one claims that headache sufferers have an ‘aspirin deficiency.’”³⁵

All this would be an exercise in psychopharmacological frivolity, except for the quite real side effects attributed to SSRIs. “Symptoms of serotonin syndrome include restlessness, hallucinations, loss of coordination, a racing heart, rapid changes in blood pressure, fever, nausea, vomiting, and diarrhea,” noted Kirsch, “SSRIs can provoke an agitated restless state called akathisia, which some people describe as feeling like jumping out of their skin. It is often in this state that people on SSRIs become violent and aggressive toward themselves and others.”³⁶ In the absence of confirmatory

research, the biomedical model of psychiatry was sold to the public by Big Pharma in collaboration with the APA. In order to expedite marketing, drug companies went beyond the usual method, relying on detail reps, by hiring psychiatrists to promote their products. The employment of “hired guns” by drug companies became systematized through “key opinion leaders,” KOLs, many from prestigious academic centers.³⁷ Among the most celebrated was Dr. Joseph Biederman at Massachusetts General Hospital who, along with Timothy Wilens and Thomas Spencer, raked in \$4.2 million over seven years from drug companies but reported only a fraction of that to their universities. Biederman and Wilens were later reprimanded for their conduct. The self-serving circle between psychiatry and the psychopharmacological industry was closed in more pedestrian ways: Fifty percent of the drafters of the *DSM V* reported significant income from drug companies.³⁸ Some critics of the psychiatric sell-out were less charitable: psychiatrist E. Fuller Torrey suggested, “the present system is approaching a high-class form of prostitution.”³⁹

Psychiatrists failing to get the message directly from colleagues would likely find the evidence of drug efficacy in research studies published in professional journals. A 1999 lawsuit against Pfizer revealed that the company had hired Current Medical Directions (CMD), a medical education company, to write articles favorable to its blockbuster drug, Zoloft. CMD paid ghostwriters to draft articles, located psychiatrists to sign them, then published the pieces in prestigious journals, such as the *American Journal of Psychiatry* and the *Journal of the American Medical Association*. Ultimately, the CMD articles comprised a majority of all research on Zoloft published between 1998 and 2000. As Carlat observed, “for at least one antidepressant, the bulk of medical literature was literally written by the drug company that manufactured the drug.”⁴⁰

Covering all bases, the psychopharmacological industry also made substantial contributions to organizations advocating for the mentally ill. Organizations, such as NAMI, not only appreciated philanthropy from drug companies because it addressed chronic budget shortfalls but also because the biomedical model promised to destigmatize mental disorders. With the biomedical model ascendant early in the 1990s, NAMI joined with NIMH to educate the public about how chemical imbalances caused mental disorders. Between 1993

and 1996, 18 drug companies contributed \$11.7 million to NAMI. In 2009 alone, one company, Eli Lilly, contributed \$551,000 to NAMI, \$465,000 to Mental Health America, \$130,000 to an Attention Deficit Hyperactivity Disorder (ADHD) advocacy organization, and \$69,250 to a suicide prevention organization.⁴¹ In exchange for corporate beneficence, mental health advocacy organizations validated an explanation for mental disorders that could be corrected by medication, just like other diseases, in the process providing moral support to the psychopharmacological industry.

If American psychiatry has become captive to the biomedical model, other countries have flirted with the idea only to reject it. Whitaker cites a region of Finland, western Lapland, which has adopted an alternative to psychopharmacology in caring for the seriously mentally ill. Five years after receiving “open dialogue therapy,” 73 percent of 75 patients (30 schizophrenics) were working or in school, while only 20 percent were on disability. Whitaker reports, “Only two or three new cases of schizophrenia appear each year in western Lapland, a 90 percent drop since the early 1990s.”⁴²

Children’s Mental Health

Reductions in federal funding, a cut of \$29 million for 2013 for children’s mental health, occurred at a time when psychiatric care for children was at a crisis point. The FDA does not require RCTs for medication unless it is specifically intended for children, and no clinical trials are targeted at minority children. As a result, a common practice in psychiatry is to prescribe medication “off-label” for children; in other words, psychoactive drugs approved for adults are often prescribed for children at lower doses. Among children aged 6 to 17, between 1996/98 and 2010/12, psychotherapy increased from 4.2 percent to 6.0 percent, while psychotropic medication increased from 5.5 percent to 8.9 percent, of which stimulants increased 4.0 percent to 6.6 percent, while anti-depressants rose from 1.5 percent to 2.6 percent and antipsychotics from 0.2 percent to 1.2 percent.⁴³

Researchers have long debated the value of Ritalin, which is often prescribed for children with ADHD, for example.⁴⁴ In 1990, 600,000 children were diagnosed with ADHD, but a quarter century later, the number had increased to 3.5 million, an increase that Dr. Keith Conners likened to a false

epidemic, “a concoction to justify the giving out of medication at unprecedented and unjustifiable levels.”⁴⁵ ADHD also justified the medication of 10,000 two- and three-year olds, according to the Centers for Disease Control and Prevention, a practice that one consultant described as “absolutely shocking.”⁴⁶ Subsequently, child mental health practitioners proposed “sluggish cognitive tempo” to describe children inclined to daydream and lag in mental processing.⁴⁷ Whether or not daydreaming warrants medication, the compelling question becomes, Has childhood become medicalized? Two professors from Berkeley proposed expanding pre-K programs as an antidote to ADHD diagnoses.⁴⁸

Recently, atypical antipsychotics, such as Risperdal, have been prescribed for ADHD, a problem not only for which the medication was not approved but also for causing side effects, such as weight gain and permanent muscular tics.⁴⁹ Indeed, the adverse effects of SSRIs when prescribed off-label to children were associated with suicides at twice the number in the control group, resulting in “a black box” warning for Celexa, an SSRI produced by Forest Pharmaceuticals, a drug company fined \$313 million by the Justice Department in 2010 for irresponsible off-label marketing to children.⁵⁰ By 2008, 60 percent of antipsychotic medication was prescribed “off-label” often to children for such problems as anxiety, sleep disorders, ADHD, and behavioral problems.⁵¹

Off-label prescriptions for children came under the scrutiny of child welfare advocates when it became evident that foster children were being administered high doses of psychoactive medication. A study of children on Medicaid revealed that foster children were often prescribed powerful antipsychotics such as Risperdal, Seroquel, and Zyprexa, drugs that were intended to treat schizophrenia.⁵² Subsequently, the Government Accountability Office conducted a study of foster children in five states and found that four times as many foster children received antipsychotic medications compared to non-foster children. Not only were foster children prescribed higher doses, but they also received multiple prescriptions. In some cases, children under age one received antipsychotic medication.⁵³

Meanwhile researchers documented an 80 percent increase in autism during the previous decade.⁵⁴ Increasing prevalence of autism raised questions about diagnosis, and planners for the *DSM V* suggested an umbrella classification, Autism Spectrum Disorder that incorporated classic autism, Asperger

Syndrome, and Pervasive Developmental Disorder Not Otherwise Specified.⁵⁵ Exactly how such ambiguous developmental disorders among children will be addressed in the Affordable Care Act (ACA) remains to be seen.

Mental Health and Substance Abuse Funding

The Substance Abuse and Mental Health Services Administration (SAMHSA) of the Department of Health and Human Services oversees the federal alcohol, drug abuse, and mental health block grants. Since 1981, all mental health expenditures have been in block grants to states. By using a block grant strategy, the federal government removed the power from federal agencies and transferred it to the individual states. Since the creation of mental health block grants, federal funding for mental health and substance abuse services has decreased to \$3.4 billion for 2013, \$142 million less than for 2012.⁵⁶ It is important to recognize how funding has shifted during the past two decades: Although private funding for mental health services had diminished, state and local funding has imploded, leaving Medicaid and Medicare primary sources of revenues. Substance abuse services, on the other hand, continue to rely heavily on state and local revenues (see Tables 13.2 and 13.3).

TABLE 13.2 Distribution of Mental Health Expenditures, %

Source	1986	2003	2006	2014
Private				
Out of pocket	18	14	14	12
Private insurance	21	24	24	26
Other private	7	3	4	3
Public				
Medicare	6	7	11	11
Medicaid	16	26	24	27
Other federal	6	4	3	1
Other state and local	26	21	20	16

Source: Based on “Projections of National Expenditures for Mental Health Services and Substance Abuse Services, 2000–2014,” U.S. Department of Health and Human Services, Substance Abuse and Mental Health Services Administration, Washington, DC: 2008, p. 21.

TABLE 13.3 Distribution of Substance Abuse Expenditures among Payers, %

Source	1986	2003	2008	2014
Private				
Out of pocket	14	8	8	6
Private insurance	30	10	9	7
Other private	6	5	5	4
Public				
Medicare	4	4	4	5
Medicaid	10	18	18	20
Other federal	7	15	14	14
Other state and local	29	40	42	45

Source: Based on “Projections of National Expenditures for Mental Health and Substance Abuse Services, 2000–2014,” U.S. Department of Health and Human Services, Substance Abuse and Mental Health Services Administration, Washington, DC, 2008, p. 32.

Static funding has prompted innovation in service delivery. An example of how a capitation method of payment could be used in mental health service delivery was the integrated mental health (IMH) concept pioneered in New York State and Philadelphia. Capitated mental health care under IMH would have three major features: First, current **categorical grants**—Medicaid, Supplemental Security Income, Food Stamp Program, local funding—would be aggregated into a common fund from which capitation “premiums” would be paid. Second, a nonprofit planning and coordination agency would be established to oversee mental health care and in so doing negotiate contracts with providers, monitor performance, and evolve innovative programs. Third, particularly high-usage clients would be targeted for provision of less costly services in order to generate surpluses for less-intensive services.⁵⁷

Static funding provided the rationale for subsidizing private treatment innovations in substance abuse, such as Phoenix House. Established in 1967, Phoenix House has not only expanded to over 120 locations across the country but has also introduced electronic medical records in compliance with ACA. Of particular interest to Phoenix House has been PTSD experienced by military veterans. In addition, Phoenix House has been diligent about tracking the evolution of addictive drugs.⁵⁸

Among the 10 basic provisions that health insurance plans must include under ACA are behavioral health and substance abuse services. The adequacy of these services will vary with the plan selected by a consumer as well as their state of residence. States that have declined to expand Medicaid are likely to continue to rely on the mental health and substance abuse block grant for care. For states embracing ACA, mental health and substance abuse services should expand significantly.⁵⁹

Parity for Mental Health Care

In 1996, Congress delighted mental health advocates by agreeing on legislation establishing parity for mental health care. Effective January 1, 1998, employers with more than 50 employees who offer any health insurance coverage must include mental health benefits that are comparable to physical health benefits.⁶⁰ The consequences of legislatively mandating parity in mental health coverage were immediately disputed. Opponents warned of significant increases in health insurance premiums and the likelihood that employers would eliminate mental health coverage in order to dodge the parity mandate.⁶¹ Defenders, on the other hand, minimized the implications for premium increases, noting that mental illness was, for the first time, being interpreted as a physiological disorder.

An evaluation of the Mental Health Parity Act released by the General Accounting Office in May 2000 tempered the enthusiasm of mental health advocates. Although 86 percent of employers surveyed were in compliance with the legislation, most—87 percent—had altered employee benefits so that mental health benefits were more restrictive. Significantly, only 3 percent reported that compliance had resulted in increased costs and none had dropped mental health coverage altogether in response to mandated mental health parity.⁶² Achieving parity in mental health may have been an important development in social policy, but its provisions were quite modest. Parity was predicated on employment so it did nothing for those who were not in the labor market; moreover, because it applied only to employers with more than 50 workers, provisions were evaded by small employers. Thus, parity did little for those with mental illness who were not working and even less for those hired by small businesses.

Post-Traumatic Stress Disorder

A decade of war in Iraq and Afghanistan resulted in serious mental health problems among many of the 2.5 million service members returning from deployments in Iraq and Afghanistan.⁶³ Operation Iraqi Freedom, Operation Enduring Freedom, and New Dawn exposed soldiers to mental and physiological trauma that were qualitatively different from earlier military ventures. Unlike the Vietnam conflict, the prevalence of Traumatic Brain Injury (TBI) and Post-Traumatic Stress Disorder (PTSD) was significantly higher.⁶⁴ As a psychiatric diagnosis, PTSD generated controversy when large numbers of veterans were unable to function normally at home and work.⁶⁵ Critics, such as psychiatrist Sally Satel contended that “a culture of trauma has blossomed,” largely attributable to bonuses for veterans with PTSD.⁶⁶ Between 1999 and 2004, for example, claims for PTSD increased 79.5 percent, accounting for one-fifth of all compensation benefits.⁶⁷ As these wars wound-down, 195,000 veterans had been screened for TBI, the majority of which rated as mild to moderate in severity.⁶⁸ Equally disturbing, the incidence of suicide among members of the uniformed services increased. In 2010, 295 service members took their own lives; 48 percent having visited military treatment prior to suicide.⁶⁹

While PTSD and TBI attracted public attention, VA services as well as those provided by the Department of Defense addressed a range of mental health problems. The Family Advocacy Program, for example, reported 16,000 incidents of spouse

abuse in 2005.⁷⁰ Substance Use Disorder, often co-occurring with PTSD, increased from affecting 5 percent of active duty personnel in 2005 to 12 percent in 2008.⁷¹ As American military presence in Iraq and Afghanistan wound-down, mental health services were stretched to the breaking-point.

As increasing numbers of veterans sought care for mental disorders, the Veterans Administration (VA) came under scrutiny for poor care at Walter Reed Army Medical Center.⁷² Mental health analysts reached an alarming conclusion: Not only were mental disorders among returning soldiers delayed but were also actually more prevalent among reservists (see Table 13.4). Increases in war trauma coincided with loss of mental health personnel within the uniformed services, about 10 percent between 2003 and 2006. In response, Congress authorized \$600 million for enhanced mental health services for veterans.⁷³ To prevent PTSD and related disorders, military researchers introduced “battlemind debriefing and training” to a random sample of platoons, then evaluated soldiers’ post-deployment. Soldiers who had participated in “battlemind” showed lower incidence of PTSD, depression, and sleep disorders than those in the control group.⁷⁴



Watch this video that illustrates how Post-Traumatic Stress Disorder is manifested by panic attacks, with self-imposed isolation being a frequent response. It also shows how group support through the Veterans Administration can assist veterans.

www.youtube.com/watch?v=NkWwZ9ZtPEI

TABLE 13.4 Mental Health Disorders among Returning Soldiers

Type of Disorder		Return from Iraq (%)	Three to Six Months Later (%)
Interpersonal conflict	Active duty	3.5	14
	Reserves	4.2	21.1
Post-traumatic stress disorder	Active duty	11.8	16.7
	Reserves	12.7	24.5
Depression	Active duty	4.7	10.3
	Reserves	3.8	13
Overall mental health disorder	Active duty	17	27.1
	Reserves	17.5	35.5

Source: Based on Ann Scott Tyson, “Troops’ Mental Distress Tracked,” *Washington Post* (November 17, 2007), p. A3.

Although far from ideal, mental health services to veterans have been at least coordinated by national organizations that could be held accountable, unlike care for civilians, which remained fragmented under state authority. Speaking for many civilians, Pete Earley, an award-winning author whose son was diagnosed with mental illness, advocated for better inpatient care: “What is missing in our system today are modern, long-term treatment facilities where the chronically mentally-ill can receive good medical attention and, if necessary, can live safely until they can be moved to less restrictive facilities.”⁷⁵ Similarly, Andrew Solomon called for a network of university-based research centers that would study mental disorders, such as depression.⁷⁶ Building on the adoption of mental health parity, Art Levine advocated for “parity-plus,” expanding mental health care through a five-part strategy: implement mental health parity, promote proven treatments for mental illness, fund patients not programs, provide early screening for children, and encourage mainstream activities such as work.⁷⁷ A half-century after the community mental health movement, the question of mobilizing mental health advocates around a coherent, comprehensive reform remained.

Substance Abuse

The interaction of mental disorders, alcoholism, and substance abuse is reflected in social welfare policy and has been institutionalized at SAMHSA. The consolidation of categorical grants into a federal block grant program under SAMHSA reflected the preference of many human service professionals for preventive programs that apply generically to all forms of substance abuse. What is the logic in having separate preventive programs for tobacco, alcohol, and illegal drugs when effective prevention programs can be developed for all of them? Block grants place significant responsibility on professionals who must engage at the state level to advocate for those impaired by alcohol and drugs.

Mental health professionals know well that mental health is compromised by substance abuse. Human service professionals in direct services are familiar with clients who have chosen to anesthetize themselves from stress or misery with alcohol, tobacco, and other substances. Individuals’ psychological problems are of course compounded by reliance on such substances, and these problems not only affect the families of substance abusers but also become more severe

when addiction is manifested. Ordinarily, addiction is associated with alcohol and drugs, and less often with tobacco. Substance abuse has become an important area of public policy, not only because of the necessity for appropriations for treatment programs but also because of the enormous costs that substance abuse extracts from society. As these costs have escalated, substance abuse policy has attained a higher profile in domestic affairs.

Alcohol Abuse

Americans steadily increased their consumption of alcohol from the end of World War II until the 1980s, when drinking began to decrease. By 2013, average per capita consumption was a little more than 2.3 gallons of wine, 1.5 gallons of spirits, and 20.7 gallons of beer a year.⁷⁸ However, that amount was not evenly distributed throughout the population. One-third of the adult population abstains from alcohol consumption, one-third of people who do drink consider their consumption to be light, and the remaining third are considered moderate to heavy drinkers. The 2013 National Survey on Drug Use and Health revealed that 60.1 million Americans were binge drinkers on occasion and that 16.5 million were heavy drinkers.⁷⁹

Substance abuse is directly related to serious social problems. Forty-eight percent of all convicted criminals used alcohol just before committing a crime, and 64 percent of offenses against public order are alcohol related.⁸⁰ Yet, substance abuse services are often not provided to drinkers who are subsequently incarcerated. Substance abuse services were available at 93.8 percent of federal correctional facilities but at only 60.3 percent of state prisons, 33.5 percent of jails, and 36.6 percent of juvenile facilities.⁸¹ Perhaps the most significant adverse consequence of alcohol consumption is highway accidents. The tragic death toll on U.S. highways provoked not only the establishment of Mothers Against Drunk Driving (MADD) but also the demands for stronger penalties for drunk drivers and public education campaigns to dissuade people from drinking while driving. This combination of motivators seemed to have a positive effect: Between 1982 and 1986, the number of inebriated drivers involved in fatal accidents dropped significantly. By 2013, the percentage of people aged 12 and over who drove under the influence dropped to 10.9 percent, down from 14.2 percent in 2002.⁸²

Among the most pernicious effects of alcohol consumption is **fetal alcohol syndrome** (FAS),

a physiological and mental deformation in infants caused by their mothers' ingestion of alcohol during pregnancy. The National Institutes of Health estimated in 1990 that the incidence of FAS among children of heavy-drinking women was as high as 25 per 1,000 births and that the annual cost of coping with the disorder was almost one-third of a billion dollars.⁸³ In 2012, the Pine Ridge Reservation sued several American brewing companies, which distributed beer at outlets in Whiteclay, Nebraska, adjacent to the reservation in order to reduce incidents of FAS, drunk driving fatalities, and alcohol-related homicides.⁸⁴ In 2013, 2.3 percent of pregnant women reported binge drinking, compared with 0.4 percent heavy drinkers.⁸⁵

Drug Abuse

By contrast with alcohol abuse, the prevalence of drug abuse is more difficult to ascertain because the use of controlled substances—the focus of drug abuse—is illegal. It appears that general drug abuse has begun to decline after peaking during the 1979 to 1980 period. In 2006, almost 20.4 million Americans reported use of illicit drugs, a significant decrease from the 25 million abusers estimated in 1979 (see Table 13.5). Among younger Americans aged 12 to 17, drug abuse has been declining (from 11.6 percent in 2002 to 8.8 percent in 2013); drug abuse by young adults aged 18 to 25 increased somewhat (from 20.2 percent in 2002 to 21.5 percent in 2013). Drug abuse has changed little during the past decade; most illegal usage might be categorized as “recreational” as

contrasted with addictive “hard” drugs, as evident in SAMHSA's 2013 survey of drug usage.

Types of illegal substances vary considerably. Next to marijuana, cocaine is the most prevalent illicit drug, accounting for 1.5 million users; heroin is far less prevalent with 323,000 addicts.⁸⁶ Even if heroin is less prevalent, it remains a major problem if only because of the health problems associated with its use: As many as 25 percent of people who contract AIDS in this country are intravenous drug users (IDUs).⁸⁷ A haunting scenario takes shape: IDUs can no longer be thought of solely as tortured souls in the slow process of self-destruction; they have become transmitters of an epidemic that promises to be as costly as it is deadly.

The federal response to illicit drug use has been twofold, involving both interdicting the supply of illegal substances and reducing the demand through treatment and public education. Government strategies have oscillated wildly between the interdiction and prevention approaches. Before the Reagan presidency, federal policy emphasized treatment and public education, assuming that these strategies would diminish demand. During the early 1970s, for example, two-thirds of federal appropriations for drug abuse were for treatment and education. A decade later, however, supply interdiction had superseded demand reduction as the prime strategy, consuming 80 percent of federal drug funds. Illegal drug use is considered in greater detail in Chapter 14.

In the absence of major prevention initiatives, intervention strategies focus on treatment (see Table 13.6). Generally, employees with generous health insurance

TABLE 13.5 Types of Illegal Substances

Type of Substance	Millions of Users
Illicit drugs	24.6
Marijuana	19.8
Psychotherapeutics	6.5
Cocaine	1.5
Hallucinogens	1.3
Inhalants	0.5
Heroin	0.3

Source: Based on the source Results from the 2013: National Survey on Drug Use and Health: Summary of National Findings, retrieved from <http://www.samhsa.gov/data/sites/default/files/NSDUHresultsPDF-WHTML2013/Web/NSDUHresults2013.pdf>.

TABLE 13.6 Location of Substance Abuse Treatment

Location	Numbers (thousands)
Self-help group	2,292
Outpatient rehabilitation	1,753
Outpatient mental health center	1,176
Inpatient rehabilitation	1,042
Hospital inpatient	879
Private doctor's office	770
Emergency room	603
Jail or prison	263

Source: Based on the source Results from the 2013: National Survey on Drug Use and Health: Summary of National Findings, retrieved from <http://www.samhsa.gov/data/sites/default/files/NSDUHresultsPDFWHTML2013/Web/NSDUHresults2013.pdf>.



The federal response to illicit drug use has been twofold: cutting off the supply of illegal substances and reducing the demand through treatment and public education.

Source: Threerocksimages/Shutterstock

have been able to gain ready admission to drug abuse treatment programs. The poor, by contrast, have found treatment available irregularly, often through self-help groups. Treatment eludes large numbers of substance abusers. In 2013, over 22.7 million Americans 12 and older would have received substance abuse treatment had it been available.⁸⁸

Although treatment lags behind demand, research continues to demonstrate the wisdom of investing in rehabilitation. Columbia University's Center on Addiction and Substance Abuse has reported that 32.3 percent of Medicaid hospitalization days were due to neonatal complications attributed to substance abuse. Cardiovascular and respiratory disorders associated with substance abuse accounted for another 31.4 percent of Medicaid hospitalization days. Significantly, when substance abuse was noted as a secondary diagnosis, the length of hospitalization doubled.⁸⁹ A cost-benefit analysis of school-based prevention programs,

indicated every dollar invested in prevention returns 18 dollars in savings.⁹⁰

Unfortunately, as is often the case with addiction services, the subsequent question of long-term abstinence has not been thoroughly evaluated.

Conclusion

Mental health and substance abuse policies have been complex in part because they have addressed behaviors that are not well-understood. Compounding this have been major policy disputes that have made it more difficult to reach a consensus about optimal programming. Mental health care has been compromised by a fundamental disconnect in American social welfare policy: Should responsibility for the aberrant rest with the federal government or the states? Since the 1980s, mental health care has been devolved to the states; yet, the states have been unwilling to pick up the costs for an adequate array of mental health services. Moreover, the aftermath of deinstitutionalization is still evident in the prevalence of homelessness and substance abuse. Substance abuse services have been compromised by a different disconnect: Should the focus be on treatment or interdiction? Since the advent of the War on Drugs, the emphasis has been on law enforcement, resulting in underinvestment in treatment. Advocates of the mentally ill and addicts were heartened with the passage of the Mental Health Parity Act; however, further expansion of mental health and substance abuse services through the Affordable Care Act has yet to be determined.

DISCUSSION QUESTIONS

1. Click on Congress.org and identify a mental health or substance abuse policy that is of interest to you. Are the provisions of the policy adequate? What could be done to make services more effective?
2. In the early 1980s, funding for community mental health centers (CMHCs) was converted to mental health block grants. To what extent did your community develop CMHCs? What has happened to them since the 1980s? What priorities have been established through the mental health block grant system? How has this changed mental health services in your community?
3. The misuse of psychoactive medication has been implicated in several undesirable consequences. Have side effects of psychoactive medications become a significant problem among mental health

- patients in your community? If so, what is being done to prevent it? Are more or fewer mental patients going through the “revolving door”?
4. The effects of substance abuse on innocent people present difficult policy dilemmas for decision makers. What policies could be put in place to prevent infants being born with FAS or AIDS? How could the rights of mothers be protected? What should be the role of human service professionals in such cases?
 5. Prevention and treatment of substance abuse vary from locality to locality. What has your community done to dissuade young people from substance abuse? Have these initiatives been successful? According to what indicators?
 6. What resources has your community committed to dealing with substance abuse? Have these resources been adequate? Which organizations support or oppose increasing treatment for substance abuse?

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Criminal Justice



Source: Jeffrey Zalesny/Fotolia

All societies respond to norm-defying behavior through sanctions, although there is considerable variation among the penalties that societies apply to specific behaviors. In Western cultures, irrational conduct is usually understood as a mental health disorder, whereas deviant behavior on the part of a rational actor falls under the purview of law enforcement. In either case, deviance is of interest to human service professionals because clients often engage in such activity. Deviance is also an issue of social justice because poor people and minorities of color are disproportionately represented among those incarcerated in mental as well as correctional institutions.

Table 14.1 shows that the incarceration rate per 100,000 population is far higher in the United States than that of virtually all other countries. Notably, most developed nations have incarceration

rates that are a fraction of that of the United States. Among the nations of the world, the incarceration rate of the United States is second only to that of Russia. Russia imprisons 687 inmates per 100,000 population; the United States imprisons 682. The rates for other nations are strikingly lower: South Africa, 321; Canada, 115; France, 90; and Japan, 39.¹

By incarcerating large numbers of citizens, the United States not only incurs the costs of imprisonment but also further disadvantages minority communities that represent a large portion of inmates.

History of U.S. Criminal Justice

Modern criminal justice in the United States can be traced to the faith in social science that originated in the West in the eighteenth century. Before the advent of classical criminology, justice was predicated on vengeance: Illegal acts brought the wrath of authority on the deviant. In pre-modern societies in which deviance was understood to be a product of evil influences, vicious and barbaric methods were justified as ways to exorcise Satan or to maintain archaic social structures. Thus, mutilation, torture, and capital punishment were often employed, sometimes in grotesque public displays, in order to rid society of malevolent influences.

Modern criminology dates from the Age of Enlightenment in the eighteenth century and its notion that humankind was capable of producing the methods for its own perfectibility. The radical jurist and philosopher Jeremy Bentham (1748–1832) contended that scientific methods could be the vehicle for “the rational improvement of the condition of men.” Accordingly, Bentham successfully advocated a series of reform laws in Great Britain, including several pertinent to penal institutions.² In the United States, Cesare Beccaria applied Bentham’s utilitarian philosophy to corrections, arguing that crime could be measured in its severity, that prevention was more important than punishment, that the purpose of punishment was deterrence (not revenge), and that incarceration should segregate prisoners so as not to exacerbate lawlessness.³ Liberal, humanistic values in criminal justice can thus be traced to the earliest thinkers in criminology.

Nevertheless, the early American colonists imported traditional European thinking about crime and its control. Jails were a fixture of all settlements of any size, and justice was often swift and

TABLE 14.1 Incarceration Rates, Select Countries

Country	Rate/100,000 Population
Japan	62
Sweden	78
Ireland	78
France	88
Germany	95
South Korea	97
Italy	102
Canada	107
China	118
Australia	126
Spain	145
United Kingdom	145
Brazil	191
Mexico	196
Israel	209
South Africa	335
Cuba	447
Russian Federation	607
United States	738

Source: Based on Christopher Hartney, “US Rates of Incarceration: A Global Perspective,” Research from the National Council on Crime and Delinquency (November 2006), p. 2.

uncompromising. In 1776, in response to the dungeons that typified colonial America, the Quakers established the Philadelphia Society for Alleviating the Miseries of Public Prisons.⁴ During the following decades, institutional reformers such as Dorothea Dix sought to make jails and almshouses more humane. Unfortunately, the reformer's accomplishments were often subverted. An influx of immigrants, many of whom were unable to adjust to America, became incarcerated in mental and correctional institutions. The American ethos of rugged individualism left little room for compassion, particularly when adults were concerned.

Midway through the nineteenth century, an unlikely pioneer in U.S. corrections emerged. In 1841, a Boston shoemaker, John Augustus, negotiated an arrangement with the courts to use his own money in order to supervise petty criminals, post bail, and report to the courts on the offenders' rehabilitation. The services provided by Augustus were less expensive than prison, and many of his charges seemed to benefit from rehabilitation. Thus, a modest shoemaker began what was to become a nationwide system of probation.⁵ Some early criminologists sought more direct applications of emerging sciences to the study of crime. The Italian psychiatrist Cesare Lombroso, for example, proposed the existence of a "criminal type," a construct of inferior intelligence, exaggerated physical features, and a taste for amoral activities, including tattooing! The idea that a criminal could be physiologically identified, and criminal behavior was genetically transmitted subsequently preoccupied some criminologists. For example, in 1913 Charles Goring studied English convicts, and during the 1920s Earnest Hooton evaluated American criminals; both concluded that prisoners were "organically inferior" to their law-abiding compatriots.

Before World War II, the eugenics movement popularized the notion that deviant behavior had a genetic origin. Proponents of natural selection, some of them esteemed scientists and jurists, convinced state legislators to pass legislation allowing for involuntary sterilization of "mental defectives." By the mid-1950s, more than 58,000 mental patients and convicts had been forcibly sterilized.⁶ The practice of involuntary sterilization abated during the Civil Rights movement when it became recognized that many of the victims were women and minorities of color. The suggestion that crime was organically determined generated a firestorm of criticism during the 1960s when it was pointed

out that an increasing number of criminals were minorities of color, the same populations that had been victimized by discriminative social policies.

The Criminal Justice System

The U.S. criminal justice system is similar to education, and mental health programs in that states and localities provide a significantly larger portion of services than the federal government. The Constitution reserves public functions to the states unless they are ceded to the federal government; in the case of criminal justice, this means that state and local government expenditures exceed those of the federal government by a factor of four.⁷ Between 1980 and 2003, criminal justice expenditures increased 417.5 percent.⁸ Moreover, there was considerable variation in state expenditures. In 2007, for example, the per capita cost of criminal justice in the District of Columbia was \$1,373; that of nearby West Virginia was \$412.⁹ Such cost disparity encourages higher-cost correctional systems to contract-out incarceration with lower-cost systems; however, this contravenes family life when urban inmates are housed in rural areas, making visits difficult and phone calls expensive.

The last decades of the twentieth century witnessed a boom in prison construction, a development that drew the wrath of opponents of incarceration. Prison is an expensive way to manage deviants, costing \$22,000 on average per inmate annually.¹⁰ Since states pay most of the costs for incarceration, prisons vie with schools for funding, a competition that is decidedly skewed. "While the state government of Virginia spends upward of \$70,000 per year to incarcerate each child in a juvenile prison," noted critics of imprisonment of juvenile offenders, "it contributes only about \$3,400 to provide for the education of that child."¹¹ Such disparities have led liberals to argue for greater funding of education and youth services as a way to prevent incarceration, and some conservatives have come to appreciate the enormous expense of a criminal justice system in which prisons are dominant. In 2015, the most unlikely of partners, the conservative Koch brothers and the liberal Center for American Progress met to reverse the effects of high incarceration rates on minority communities.

Between 1995 and 2003, state correctional budgets increased by 50 percent, primarily because of

TABLE 14.2 Crime and Crime Rates, by Type of Offense: 1980 to 2009, thousands

Year	Total Personal	Murder	Rape	Robbery	Assault	Total Property
1980	1,345	23	83	566	673	12,064
1985	1,328	19	88	498	723	11,103
1990	1,820	23	103	639	1,055	12,655
1995	1,799	22	97	581	1,099	12,064
2000	1,425	16	90	408	912	10,183
2005	1,391	17	94	417	862	10,175
2009	1,318	15	88	408	807	9,321

prison construction and maintenance, leading several states to revise more punitive sentencing practices, such as mandatory minimums, “three strikes” provisions, and abolishing parole¹² (see Table 14.2 for categories of crime and crime rates).

In the criminal justice system, the disposition of offenders varies, not just by jurisdiction but also by type of offense. Those who have not been convicted of violent offenses and who have no previous criminal record are likely to be granted **probation**. Those who have been convicted of offenses under local or state jurisdiction may be jailed or imprisoned in facilities that have often been determined to violate minimal humane standards of care. Federal convicts, on the other hand, may be incarcerated in prisons that are qualitatively better, some of which have a decidedly pleasant ambiance. Having served time for good behavior, many prisoners earn early release and

go on **parole**, during which time they must report regularly to a supervising officer. These various statuses are depicted in Table 14.3.

A cursory examination of the tables above reflects one of the great contradictions in American criminal justice: while both personal and property crimes have fallen, incarceration and its alternatives—probation and parole—have increased dramatically. In 1980, the crime rate was 5,950 per 100,000 population; by 2009, the rate was 3,466, a drop of 41.7 percent; yet, the number of inmates *increased* by a factor of 3.9.

Prisons have been a signal concern in criminal justice, both because of their cost as well as what incarceration rates say about precipitating offenses. The United States passed a benchmark in 2000, when 2 million people were behind bars.¹³ In this population, gender and race emerge prominently: Roughly 90 percent of inmates are men¹⁴ and two-thirds are minorities of color. Virtually half of all

TABLE 14.3 Adults on Probation, in Jail or Prison, or on Parole

Year	Total	Probation	Jail	Prison	Parole
1980	1,840,400	1,118,097	182,288	319,598	220,438
1985	3,011,500	1,968,712	254,986	487,593	300,203
1990	4,348,000	2,670,234	403,019	743,382	531,407
1995	5,335,100	3,077,861	499,300	1,078,541	679,421
2000	6,437,400	3,826,209	613,534	1,316,333	723,898
2005	7,045,100	4,166,757	740,770	1,448,344	780,616
2009	7,225,800	4,203,967	760,400	1,524,523	819,308

Source: U.S. Census Bureau, *Statistical Abstract of the United States*, 2010 (Washington, DC: U.S. GPO, 2012), p. 217 Table 348.

inmates are African American¹⁵; in 2002, about 12 percent of African American men between ages 20 and 34 years were behind bars.¹⁶ In light of the fact that two-thirds of inmates return to prison within a few years of release,¹⁷ recidivism poses an enormous burden on the African American community.

Traditionally, crimes have been classified according to personal injury or property damage. Recently, the designation of hate crimes can add to punishment adjudicated for crimes against persons and property. Hate crimes are classified in five groups, those motivated by race, religion, sexual orientation, ethnicity, and disability. For 2013, the Federal Bureau of Investigation reported 5,922 hate crime offenses and classified them as follows: 48.5 percent were racially motivated, 20.8 percent were motivated by sexual orientation, 17.4 percent resulted from religious bias, 11.1 percent stemmed from ethnicity/national origin bias, 1.4 percent were prompted by disability, and 0.8 percent were due to gender bias.¹⁸

Juvenile Justice

Juvenile justice is a significant feature of U.S. criminal justice for several reasons. Young deviants are good candidates for becoming adult deviants, in which case they become subject to the adult criminal justice system. Because the majority of adult offenders were also known to the juvenile justice system, it stands to reason that diverting youngsters from juvenile crime may well keep them out of the adult criminal justice system. From a crime prevention standpoint, reaching young people early, when their understanding of themselves in relation to social norms is still being formed, is a more plausible strategy than attempting to intervene with adult offenders, who have a less malleable sense of themselves vis-à-vis social institutions.

The first institution for juvenile delinquency was the New York City House of Refuge established in 1825. Massachusetts followed suit with a facility for boys established in 1847 and one for girls in 1854.¹⁹ Institutional care for adolescent deviants remained virtually unchanged until the 1960s, when critics of warehousing children advocated for community alternatives. In 1972, Congress passed the Juvenile Justice and Delinquency Prevention Act, a measure that intended, in part, to remove children who were “status offenders” from the more serious deviants who had actually

committed felonies or violent crimes. According to Jerome Miller, a youth service advocate, the act ultimately failed. However, it gave impetus to the activities of those who were proponents of non-institutional care for delinquents.

In 1969, Miller, a social worker, assumed the directorship of the Massachusetts Division of Youth Services (DYS), a position that was to become symbolic of correctional reform in the United States. His initial plan, to seek incremental reforms in Massachusetts, faded as the structural flaws of state juvenile justice became more apparent. Staff routinely resorted to cruel and inhumane treatment—such as breaking fingers of escapees, smearing feces on incorrigibles, and placing miscreants in solitary confinement—that would have been unconstitutional in an adult institution. As Miller learned during his first months on the job, rather than instituting changes, staff and administration subverted attempts to reform DHS. At the same time, Miller was becoming aware of a compelling research finding: The longer children were institutionalized in DHS facilities, the *more* likely they were to become more serious adult offenders. As Miller saw it, not only were his reforms being disregarded but he was also managing an institution that made youth worse. This seemed especially counterproductive when it became evident that the vast majority of adolescents in DHS institutions were not violent felons but teens who had a knack for getting into trouble.

Miller responded to the intransigence of DHS staff with the most radical of plausible solutions: If DHS was not amenable to reform and continued to damage adolescents, the most conscionable act would be simply to close it down. Thus, over a period of months, Miller and a handpicked staff discharged the children to alternative community-based care programs. With the exception of a handful of violence-prone adolescents, all of the youth were removed from DHS by the end of Miller’s tenure in the 1970s.

Law-and-order activists condemned Miller for turning loose on the Massachusetts public the next generation of felons. Researchers recorded a different experience. Using recidivism as an outcome measure, Lloyd Ohlin and Bob Coates of Harvard’s Center for Criminal Justice found that in districts of the state in which strong community-based alternatives had been deployed, recidivism was markedly lower.²⁰ Eventually, the Massachusetts experiment was to become a benchmark in juvenile justice in the United States, a case study used by several prominent professional schools.



Because the majority of adult offenders were also known to the juvenile justice system, it stands to reason that diverting youngsters from juvenile crime may keep them out of the adult criminal justice system.

Source: Lannis Waters/POOL The Palm Beach Post/AP Images

Incarceration as the intervention of choice for young offenders continued to be a source of dispute within juvenile justice. Early proponents of boot camps, for example, promised that discipline, exercise, and routine would deter adolescents from future offenses. At boot camp “inmates would receive physical training, military discipline, and drug abuse treatment, all under the direction of military personnel and with the aim of preparing them for a life that would combine . . . the requirement of regular drug tests and the opportunity for gainful employment.”²¹ By 1993, boot camps had been established in 25 states, but just as rapidly enthusiasm waned. A study of Georgia facilities commissioned by the Justice Department concluded that boot camps failed to deter young offenders.²² In 1994, Connecticut closed its National Guard boot camp, citing “rampant gang activity, assaults on weaker inmates, marijuana use, sexual activity, and gambling.”²³ An analysis of boot camps in

eight states concluded that alumni of boot camps were neither more nor less likely to commit future offenses.²⁴

Eventually, child fatalities provoked a federal inquiry. In 2007, the Government Accountability Office released a report of 10 children who had died in private juvenile detention facilities. The agency reported “thousands of allegations of abuse, some of which involved death, at residential treatment programs across the country. . . . During 2005 alone, 33 states reported 1,619 staff members involved in incidents of abuse in residential treatment programs.”²⁵ Subsequently, Congressional hearings led to proposals to regulate such facilities, which were rejected by the National Association of Therapeutic Schools and Programs, the trade association for residential correctional facilities.²⁶

Despite the lack of success of boot camps, elected officials redoubled their efforts to contain what was perceived as an explosion in the number

of juveniles who were engaged in violent crime. Many resorted to treating juvenile offenders as adults. Since 1992, 45 states have altered adjudication of juveniles in order to facilitate treating them as adults. “As a result, the number of youths under 18 held in adult prisons, and in many cases mixed in with adult criminals, has doubled in the last 10 years or so, from 3,400 in 1985 to 7,400 in 1997. Of the juveniles incarcerated on any given day, 1 in 10 are in adult jails or prisons.”²⁷ The 1994 crime bill added draconian measures to corrections, including expanding the death penalty, encouraging longer sentences, and defunding inmate education.²⁸

Many youth advocates objected to increasing the severity of punishment for adolescents, citing evidence that some violent juvenile crimes were abating. In 2007, the Task Force on Community Preventive Services reported that the juveniles incarcerated as adults were “33.7 percent more likely to be rearrested for a violent or other crime” compared to juveniles residing in non-adult facilities. The Task Force concluded, “transferring juveniles to adult courts does harm to them, which diminishes their life chances, thereby increasing their likelihood of committing crimes in the future.”²⁹ Within a few years of the Task Force report, 37 states, the District of Columbia, and the federal government had set the age of adult criminal responsibility at 18, and 11 states had set it at 17.³⁰ As Table 14.4 shows, the number of juvenile cases has dropped dramatically since 1980 for all groups except blacks.

In search of alternatives to incarceration, states began piloting evidence-based interventions. For example, Functional Family Therapy (FFT) returns

\$10.46 for every dollar invested and produces better outcomes. FFT provides juveniles and their families up to 30 hours of intervention over a three-month span, in three phases: engagement and motivation, behavior change, and generalization. The objective of FFT has been to encourage delinquent youth and families to identify strengths, mobilize resources, and build constructive networks. The cost of FFT is about \$3,750 per youth. In 2011, Washington State reported that Functional Family Parole, a modification of FFT, produced a 48 percent reduction in re-arrest of juveniles as well as a 49 percent increase in employment compared to a control group. Independently, the Pew Charitable Trusts and the MacArthur Foundation associated FFT with a 22 percent reduction of criminal activity in Washington State, producing a benefit of \$34,549 per family, primarily due to reduced crime.³¹

African American youth incarcerated for drug offenses are good candidates for evidence-based interventions, such as FFT. In 1980, the arrest rate of juveniles for drug offenses was the same for white as for blacks; yet, by 1996 black youth were arrested 3.5 times as often as white youth. Although the racial disparity in arrest rates between blacks and whites diminished, by 2011 blacks were arrested 45 percent more often than whites.³² Arrest disparities between black and white youth would affect adults as well, the black to white ratio of drug arrests would increase from 2.8 in 1981, peak at 5.5 in 1988, before falling to 3.6 in 2007.³³

The fate of youth in detention would be tragic both with respect to the damage inflicted on inmates and the cost to the public. The United States locks up more than 66 thousand youth at an average annual cost of \$88,000 per detainee. “We

TABLE 14.4 Delinquency Cases by Gender and Race

	1995	2000	2005	2008
Males	1,396,000	1,275,600	1,229,300	1,203,600
Females	404,200	434,700	466,700	449,700
Whites	1,209,700	1,164,300	1,085,500	1,043,600
Blacks	541,400	496,300	561,300	563,500
American Indian	26,200	26,000	25,000	23,500
Asian	22,900	23,600	24,100	22,700

Source: Statistical Abstract of the U.S., 2012, Table 341.

Spotlight 14.1

JUVENILE INJUSTICE

In 2015, the case of Kaleif Browder, a teen wrongfully incarcerated at Rikers Island for three years (two in solitary confinement), came to public attention after he committed suicide.



Watch this video for an overview of juvenile injustice at Rikers Island. It includes an interview with Browder after his release.

www.youtube.com/watch?v=gJMR56H6MA0

still fill our youth prisons primarily with young people who pose little or no threat to public safety,” noted Nell Bernstein. “And we persist in sending them to places where they are likely to be victimized and certain to be isolated, and from which they consistently emerge further disadvantaged—a formula custom-made to send them back to prison again and again.”³⁴

The toll of incarceration on adolescent development would be steep, as evident in the deplorable conditions of institutions serving minority youth. Detention facilities for Native American adolescents are nonexistent for those committing felonies, so they are incarcerated in federal prisons, while misdemeanor offenders are often jailed hundreds of miles from their families and communities.³⁵ Detention institutions serving disproportionately African American youth are so rife with abuse that they have been investigated by the federal Department of Justice, as in two Mississippi institutions modeled on paramilitary, boot camps: Oakley and Columbia.³⁶

Nor would trauma during detention be confined to jails in the backwoods South. In 2014, the Department of Justice released its investigation of Rikers Island, the detention facility that in 2013 housed 682 New York City juveniles, finding that “adolescent inmates at Rikers are not adequately protected from harm, including serious physical harm from the rampant use of unnecessary and excessive force by DOC [Department of Corrections] staff.”³⁷ DOC staff not only physically abused adolescents with a frequency that was “staggering” but also placed them in solitary

confinement for long periods. In 2013, 565 incidents involving staff use of force on juveniles resulted in 1,057 injuries that were documented; however, since some of the abuse occurred in areas not covered by video cameras, the incidence was undoubtedly higher.³⁸

Inmates involved in altercations, including assaults by DOC officers, were often placed in solitary confinement. “Punitive segregation” varied with different Rikers units housing adolescents, but the time in isolation was often protracted. In February 2013, 22 inmates in one unit of 57 teens had sentences of 90 to 188 days. In March 2013, 10 inmates in another unit of 26 inmates had sentences of 90 to 197 days. In April 2013, 15 adolescents in a third unit of 25 teens had sentences of 90 to 194 days. Of these inmates in solitary, 16 were confined more than 200 days, the better part of a year.³⁹

The New Jim Crow

Because minorities of color have been disproportionately incarcerated, race and ethnicity have been fundamental to understanding criminal justice. After the Civil War, the passage of “Jim Crow” laws intended to deny African Americans their civil rights, institutionalized racism in public policy for a century and, in the process, subjugated Blacks. Recently, the disconnect between a falling crime rate and yet increasing incarceration of African Americans led a black attorney, Michelle Alexander, to conclude that the prison system replicated a caste

system that permanently marginalized minorities, not unlike slavery.

The current system of control permanently locks a huge percentage of the African American community out of the mainstream society and economy. The system operates through our criminal justice institutions, but it functions more like a caste system than a system of crime control. Viewed from this perspective, the so-called underclass is better understood as an *undercaste*—a lower caste of individuals who are permanently barred by law and custom from mainstream society.⁴⁰

Following this logic, the association between incarceration, social disadvantage, and dropping out of high school is striking. “More than half of all black men without a high school diploma go to prison at some time in their lives,” observed Adam Gopnik, “over all, there are now more people under ‘people under ‘correctional supervision’ in America—more than six million—than were in the Gulag Archipelago under Stalin at its height.”⁴¹ Because a criminal record disqualifies virtually anyone from legitimate employment, incarceration automatically results in exclusion from the labor market, irregular family life, disqualification from voting, and economic marginalization.

The process by which so many minority youths become inducted into the criminal justice system has been described as the “school to prison pipeline,” suggesting that exclusionary experiences in school are followed by immersion in the juvenile justice system, a precursor to adult corrections. According to the Department of Education’s Office of Civil Rights, black males are twice as likely as white males to be arrested after altercations at school, resulting in a suspension rate of 20 percent, compared to 7 percent for white boys.⁴² The use of police to sort-out disciplinary problems has complemented “zero tolerance” policies through which students are intercepted and expelled outright from school for acts, including bringing over-the-counter medication to school, disrespecting school staff, and failing to abide by strict school behavioral codes. One eight-year-old boy who had brought a toy gun to a Miami school was not only expelled from his home school but also assigned to a correctional school for problem children in another city.⁴³ Indeed, the problem prompted the Supreme Court to suggest that Miranda rights are indicated for children who encounter the police at

school.⁴⁴ The expulsion of problematic students benefits schools insofar as such students tend to have weak academic skills so that they are less likely to take tests that reflect on school instructional performance.

School difficulties are congruent with a concept developed by John Hagan: the “social embeddedness” of crime. In reviewing research on juvenile delinquency and adult offenders, Hagan noted the interaction of family and environment in the transmission of criminal behavior. Poor neighborhoods generated delinquents, often from families in which a parent had been an offender. In poor communities, two developmental paths evolve for children. Kids from working families with effective adult supervision are socialized in school and through early employment into a world in which there is the likelihood of success. By contrast, children from families in which an adult is an offender are more likely to encounter the juvenile justice system, and not only does that system label them as delinquent but the punishment of juvenile offenders—incarceration or its surrogates—also interferes with kids’ completing school or building a sound foundation for future employment.

Given this double burden, it is understandable how delinquency-prone youth could find gang involvement a natural progression in socialization. Drug-related gang activities, frequently targeted by law enforcement, bring arrests and sentences that label youngsters and disrupt education and employment. Before the end of high school, a delinquent has accumulated sufficient negative attributes to subvert any interest in conventional rewards. In place of education and employment, gang activity alternates with incarceration, producing a vicious circle. According to the National Longitudinal Survey of Youth, almost one-third of young people have been arrested, an experience that can prove perilous for their employment prospects.⁴⁵

Measures of the intractability of a delinquent’s circumstance can be found in the “oppositional culture” that flourishes among poor minority youth. The popularization of vulgar “gangsta” rap, a pattern of claiming the loyalty of young women by impregnating them, and “dissing” enemies (including authority figures)—even at the risk of bodily harm—are all part of the capitulation that young minority males make in the face of overwhelming odds militating against traditional means of success. Oppositional culture reproduces itself in children born to young minority women whose aspirations have been sabotaged. Such children

experience little that is nurturing or dependable, least of all fathers—given that their fathers, knowing the tentativeness of life on the streets, expect to be dead or incarcerated well before their offspring reach adolescence. The scenario is as tragic as it is “embedded” in the social reality of its perpetrators and victims.

The War on Drugs

By the 1980s, events had propelled the control of drug abuse to a top priority in U.S. criminal justice. The deterioration of inner cities had been accompanied by an alarming degree of social dysfunction. Illegal drugs were not only prevalent in the poorest minority neighborhoods but had also become an essential, if not the predominant, part of the local economy. As gangs vied over turf and ever more profitable drug peddling, violence exploded. Drive-by shootings became commonplace in larger cities. Gang-bangers invaded previously neutral territory—hospitals, mortuaries, cemeteries—in pursuit of enemies. Innocent people became victims of a rash of holdups, carjackings, and seemingly random shootings. In response, former President Reagan declared a war on drugs. Despite unrelenting efforts to contain drug abuse, during the 1990s the Department of Justice witnessed skyrocketing increases in drug seizures. Between 1990 and 2000, the number of drug seizures increased fourfold from 745,002 to 2.98 million.⁴⁶

Two strategies dominated the War on Drugs: government interdiction of supplies, aimed at

eliminating the substance, and treatment programs deployed to diminish the demand for illegal drugs. Appropriations for drug control increased substantially during the 1980s, but much of the funding was allocated for interdiction and comparatively less for treatment.⁴⁷ Despite massive infusions of funds for the Drug Enforcement Administration (DEA) and the Coast Guard, by the late 1980s most analysts agreed that supply interdiction had failed. Experts contended that emphasizing law enforcement would not solve the nation’s drug problem. “It would be naive to assume that this well-meant legislative effort will be an end to our drug dilemma,” concluded the late Sidney Cohen, former director of the Division of Narcotic Addiction and Drug Abuse of the National Institute of Mental Health. If efforts to reduce the propagation of coca in South and Central America proved futile, attempts to reduce street trafficking were similarly unsuccessful. The price of cocaine was so low that crack houses were able to offer cocaine free to new customers, charging regulars as little as \$2.⁴⁸

To compound the problem, attempts at street-level interdiction led to arrests of users and petty distributors, swelling already overcrowded prisons. In 2000, drug abuse violations accounted for one of eight arrests, more than any other offense. In every metropolitan area, a majority of arrestees tested positive for illegal drugs.⁴⁹ Thus, the focus on interdiction and enforcement proved perverse. To contain drug use through law enforcement, the country was imprisoning thousands of peddlers and addicts at enormous cost, yet funding for prevention and treatment lagged far behind allocations for incarceration. Most ironic, drug treatment for incarcerated addicts was virtually nonexistent.

During the 1980s, the combination of reductions in governmental assistance to cities through social programs and the prevalence of drug trafficking had a pronounced effect on inner-city neighborhoods. Gradually, once squalid but quiet urban neighborhoods began to echo with gunfire as rival gangs fought over turf; areas in many industrial cities virtually imploded.⁵⁰ Gang-related murders in the nation’s capital, which reached a three-year high in 1990, led a police department spokesperson to quip, “at the rate we’re going the next generation is going to be extinct.”⁵¹

In *Search and Destroy: African-American Males in the Criminal Justice System*, Jerome Miller, who had closed juvenile prisons in Massachusetts, identified the frequency with which young African



Two strategies dominate the War on Drugs: government interdiction of supplies and treatment programs to diminish the demand for illegal drugs.

Source: Joey Ivansco/The Journal & Constitution/AP Images

American males were likely to run afoul of the law, reporting that:

on an average day in 1991, more than four in ten (42%) of all the 18–35-year-old African-American males who lived in the District of Columbia were in jail, in prison, on probation/parole, or being sought on arrest warrants; on an average day in Baltimore, 56% of all its young African-American males were in prison, jail, on probation/parole, on bail, or being sought on arrest.⁵²

According to Miller, incarceration was damaging enough, but arresting all these young men inflicted substantial harm simply because most employment applications inquire about an arrest record, and research demonstrates that employers avoid candidates who have been arrested.

The social consequence of the arrest and incarceration of so many African American youths, Miller alleged, was the cultivation of an oppositional culture, replete with violence, in which engaging the criminal justice system was “something of a puberty rite, a transition to manhood.”⁵³ Drawing an Orwellian conclusion, Miller predicted that jails would soon morph into “simple internment camps” for minority youth who were refugees from an alienating culture. Other observers also documented the grim circumstances of poor minority families in urban centers.

The Underclass and “Moral Poverty”

Jerome Miller contended that “crime” was largely an attempt to control minority youth, which was challenged by conservative crime theorists. Conservatives argued that the chronic poor suffered from problems qualitatively different from those of the temporarily poor. The latter lacked basic resources, and the provision of cash benefits was a prudent response to their circumstances. The chronic poor, however, evidenced a “behavioral” poverty that differentiated them from the transient poor. The behaviorally poor engaged in habits—teen pregnancy, drug abuse, petty theft, unmarried parenthood, and welfare dependency—that not only ensured continual destitution but also subverted the objectives of well-intended social programs. Behavioral poverty sustained oppositional culture, and, in doing so, fostered the underclass.

Accordingly, William Bennett, John DiIulio Jr., and John Walters argued that the upsurge in crime could be attributed to “moral poverty, the poverty of growing up severely abused and neglected at the hands of deviant, delinquent, or criminal adults.”⁵⁴ During the 1990s, moral poverty accounted for a crime wave caused by a new breed of felons.

At the core of the crime wave is a new category of “super-predators”—radically impulsive, brutally remorseless youngsters, including ever more preteenage boys, who murder, assault, rape, rob, burglarize, deal deadly drugs, join gun-toting gangs, and create serious communal disorders. They do not fear the stigma of arrest, the pains of imprisonment, or the pangs of conscience. They perceive hardly any relationship between doing right (or wrong) now and being rewarded (or punished) for it later.⁵⁵

The conservative prescription for moral poverty relied extensively on incarceration, especially recidivists, evident in “three strikes” laws and mandatory minimum sentences. Following conservative thinking that contributed to passage of welfare reform in 1996, Bennett, DiIulio, and Walters advocated reinforcing work as opposed to unconditional welfare, removing young children from dysfunctional homes, and encouraging adoption as an alternative to foster care. Subsequently, DiIulio moderated his position about juvenile delinquents, recognizing the environmental deficits that contributed to their misconduct. In researching programs that served delinquents, DiIulio found that religious groups were often the most effective in treating delinquents⁵⁶ and became an advocate of “charitable choice,” the contracting out of social services through community-based religious organizations.

To the extent that social and economic factors—particularly the skewing of opportunities of minority youth who become adult offenders—are associated with crime, an argument can be made for environmental influences in criminal conduct, an argument congruent with a liberal orientation to criminal justice. William Julius Wilson, for example, has advocated liberal policies that divert youth from delinquency and subsequently prevent adult lawlessness. Mainstreaming the underclass, Wilson argues, requires enhanced educational opportunity; the realization of educational standards in inner-city schools; an increase in earned income tax credits for poor working families; and, in the end, a WPA-type public employment program.

Eventually, improved educational and employment opportunities will overcome current inducements to engage in lawlessness. “As more people become employed, crime, including violent crime, and drug use will subside; families will be strengthened and welfare receipt will decline significantly; ghetto-related culture and behavior, no longer sustained and nourished by persistent joblessness, will gradually fade.”⁵⁷ Thus, Wilson reiterated a major finding from the research on opportunity and crime: Lawlessness declines as rates of employment and marriage rise.

Other African Americans were less sanguine about the role of public programs in reversing the damage attributed to oppositional minority youth. Journalist Juan Williams veered away from traditional social programs, criticizing African American leaders in the process:

The nation’s leading civil rights groups . . . are locked into arguing that “the system” is causing the continued high level of poverty in black America. Their goal seems to be to get government money for programs, grants, and scholarships for the black community. That money has done some good. But at this point a blind pursuit of government money to help the poor is a tired, failed strategy. It encourages patterns of dependency among black people.

Rather than expand discredited social programs, Williams called for new initiatives through which “the poor can be empowered to help themselves.”⁵⁸

The Prison Industrial Complex

Since the mid-1980s, the United States witnessed a significant increase in prison construction, the passage of new laws requiring incarceration of repeat offenders, and the reinstitution of capital punishment in some states. Approaching the end of the twentieth century, the United States boasted an incarcerated population that exceeded 2 million⁵⁹; during 2010, the total number of people under the supervision of correctional authorities reached 7.1 million. In 1970, the number of inmates in state and federal correctional facilities was 96.7 per 100,000 population; by 1993, the number had more than tripled, to 352.9⁶⁰; by 1997, it had more than quadrupled, to 445.⁶¹ In 1994, the Violent Crime Control and Law Enforcement Act introduced the

“three strikes” penalties for repeat offenders and increased the number of federal crimes to which the death penalty applied.⁶² The number of prisoners under the death sentence soared. In 1980, 688 inmates were on death row; by 2000, the number had increased fivefold to 3,593.⁶³ Of these, 43.7 percent were nonwhite⁶⁴; the number of people of color with death sentences was thus more than double their proportion of the general population.

The rapid expansion in correctional facilities led criminologists Malcolm Feeley and Jonathan Simon to identify the advent of a “new penology.” In contrast to the old penology that focused on individual rehabilitation (via probation or parole) or deterrence (via incarceration), the new penology eschewed these for the efficient management of large populations of high-risk offenders. The mission of criminal justice, they contended, was “managerial, not transformative”⁶⁵; the social function of criminal justice was “rabble management,”⁶⁶ or controlling “unruly groups.”⁶⁷ As a managerial phenomenon, the new penology required a method for classification and incarceration, and the federal sentencing guidelines incorporated in the 1984 Comprehensive Crime Control Act (implemented on November 1, 1987) served just that purpose. The U.S. Sentencing Commission established an elaborate system of 258 categories for determining the punishment for an offense according to its severity and the offender’s previous convictions.⁶⁸

Although the Supreme Court would rule that the guidelines were advisory, not mandatory, their decision was consistent with the new penology, because the purpose of criminal justice was not to arbitrate punishment in relation to lawlessness but to incapacitate large numbers of high-risk offenders selectively. Accordingly, prisons became warehouses for the offenders who were judged to represent the highest risk to society. Because a high-risk designation is positively associated with race and poverty, the new penology functioned to regulate the most troublesome—and allegedly dangerous—elements of the underclass, and drug involvement served to identify those among the underclass who presented the highest risk. Thus, within the contemporary urban milieu, the new penology became an instrument of racial and class oppression, as Miller and Alexander have contended.

It could be argued that in the United States, prisons have historically served as institutions of last resort for incorrigible populations that have proved difficult to socialize. In that respect, the

large numbers of African Americans who dominate the prisons today are little different from the Irish or Italian immigrant inmates who disproportionately populated correctional facilities decades ago. If there is a qualitatively different aspect to contemporary corrections, however, it lies in the merging of systems management with the rapid expansion of the correctional corporations. The fundamental difference between the old, government-maintained penology and the new, corporate-managed penology is that the latter has a financial stake in an expanding correctional market, whereas the incentives under the old penology were such that the state attempted to limit criminal justice expenditures.

Thus, an analysis of corrections produced by Steven Donziger for the National Criminal Justice Commission targeted the emerging “prison-industrial complex.” According to Donziger, the prison-industrial complex consisted of an “iron triangle” of “government bureaucrats, private industry leaders, and politicians who work together to expand the criminal justice system.”⁶⁹ By 2012, the GEO Group claimed correctional programs in the United States, United Kingdom, South Africa, and Australia. GEO’s American operations included over 100 owned or managed programs, including contracts with Immigration and Control Enforcement, state prisons, and county jails. The company employed 18,788 workers and reported revenues of \$1.5 billion.⁷⁰ In 2014, the Corrections Corporation of America managed or owned 64 facilities in 19 states and the District of Columbia, employed 14,040 employees, and reported revenues of \$1.6 billion.⁷¹ Aging state prisons as well as need for federal immigration facilities are reasons for expansion of the for-profit correction industry.

The passage of the “three strikes” legislation targeting repeat offenders, the War on Drugs, and the consequences of the sentencing guidelines provided the prison-industrial complex with a more-than-adequate supply of inmates. Because most inmates were nonviolent offenders, incarceration in the minimum- and medium-security facilities that were favored by for-profit firms proved ideal. Contrasted to the archaic prisons typical of the old penology, the facilities managed by for-profit firms were not only more adequate with regard to basic amenities, but they were also more likely to provide educational and other services as methods to maintain a compliant population.

Although crime has fallen, incarceration rates remain high in large part because of prosecutorial

procedures. The professionalization of the police has removed them from the very communities they are sworn to protect; in earlier periods, Irish cops patrolled Irish neighborhoods and Italian cops patrolled “Little Italy,” but this historical process did not occur with respect to the African American community where police officers did not emerge from it organically. In order to reduce the cost of jury trials, prosecutors offer plea bargains to miscreants at the same time admonishing them that insisting on their constitutional rights will not only result in a much stiffer sentence but also a delayed trial, in the process attaining high conviction rates.⁷² Forced confessions, admissions of guilt under duress, have been disputed by DNA evidence in 24 percent of 289 sentences reviewed by the Innocence Project.⁷³ The entire system is so flawed that Supreme Court Justice John Paul Stevens wrote: “criminal statutes have limited the discretionary power of judges and juries to reach just decisions in individual cases, while the proliferation and breadth of criminal statutes have given prosecutors and the police so much enforcement discretion that they effectively define the law of the street.”⁷⁴

The get-tough mindset that typified corrections during the 1990s was shaken when increasing numbers of inmates were exonerated from crimes for which they had been convicted. In 1992, Barry Scheck and Peter Neufeld started the Innocence Project, reviewing convictions in light of recent developments in DNA technology. By 2012, the Innocence Project marked the release of 300 wrongfully convicted inmates. Of those released, 187 were African Americans, and their average length of incarceration was 13.6 years; yet, only 60 percent have been compensated for their wrongful convictions.⁷⁵ Civil rights loomed large in the Innocence Project because a disproportionate number of inmates scheduled for execution were minorities of color. Because most wrongful convictions were based on questionable confessions and evidence, police departments began to videotape confessions and take precautions to retain DNA evidence.⁷⁶ Despite such exonerations, the prison-industrial complex remained intact.

What moral indignation failed to accomplish with respect to an overbuilt and costly prison system, the Great Recession achieved. Financially distressed state and local governments reconsidered their commitments to incarceration, primarily because of the costs involved. The Pew Charitable Foundation calculated that a day in prison cost \$79 compared

to \$3.50 per day for probation or parole. Texas, the leader in executions among the states, projected the need for 8 prisons costing \$1 billion if alternatives were not found.⁷⁷ In an attempt to reduce costs, Arizona instituted a \$25 fee for visitors of inmates, which critics charged was a hardship, especially for those traveling from out-of-state.”⁷⁸ Riverside County, California Supervisors announced a plan to charge inmates for their incarceration, expected to generate \$8 million from their stay.⁷⁹

Although government attempted to maintain high rates of incarceration, problems became evident, especially when the number of inmates exceeded institutional capacity. About 4.5 percent of state and federal inmates reported being raped while incarcerated, most often by fellow inmates.⁸⁰ Two administrators of a remote Texas youth correctional facility routinely roused residents from their rooms at night in order to rape them, the reports of youth prompting retaliation when uncovered by the perpetrators.⁸¹ By 2005, 44 states had built “super-max prisons” in order to jail the most incorrigible offenders, many placed in solitary confinement that permitted only an hour each day outside a bleak cell.⁸² At any given time between 25,000 and 100,000 prisoners are held in solitary confinement, sometimes for months if not years, a disposition that is twice as costly as routine incarceration. Finally, get-tough sentencing has resulted in tens of thousands of inmates residing in prisons well into their senior years, posing the extra costs associated with nursing home care for age-related conditions such as dementia.⁸³

Legalization of Drugs

Seemingly endless retreats in the War on Drugs led some analysts to propose legalization of controlled substances.⁸⁴ Drug legalization had been a standard demand among Libertarians, who argued that individuals should be free to engage in any activity so long as it does not harm others. They contended that the current policy was little more than a replication of Prohibition’s futile effort to ban alcohol from American culture. A more mature and pragmatic policy would be to admit that certain substances were part of contemporary lifestyles and simply to regulate them, they contended, much as tobacco and alcohol are regulated. The proponents believed that legalization would have several

benefits. Legalizing drugs would enable substantial sums to be freed from law enforcement and put toward abuse prevention and treatment programs. Legalization would decriminalize drug use, thereby cutting the prison population significantly and, by destigmatizing users, making it more likely that users would enter treatment. Additional revenues could be raised by government because illegal substances would be available legitimately and taxed accordingly. Advocates of legalization questioned the claims of the defenders of the status quo who argued that prevention, treatment, and interdiction were effective strategies that would show positive results in the long run. Legalization of drugs, claimed its adherents, could produce substantial results immediately.

Momentum toward legalizing drugs provoked a volatile debate. Although proponents of drug legalization argued that people dependent on marijuana, heroin, and many psychedelics could use their drugs discreetly without disrupting society, opponents contended that the proliferation of crack cocaine and crystal meth presented a completely different picture. Not only were crack and meth implicated in violent incidents, but the craving for them was also more intense than that for many other drugs. Methamphetamine in rural states was implicated in a significant increase in the number of children placed in foster care.⁸⁵ The prospect of legalizing a substance over which users seemed to have so little control, and which, moreover, was connected to homicides, addicted infants, and family destruction, seemed inconsistent with the vision implied by drug legalization.

At the same time, momentum built for legalizing marijuana. By 2015, Colorado, Oregon, Alaska, Washington, and the District of Columbia had legalized marijuana with several more states legalizing cannabis for medicinal use. Legalization proponents noted that European countries were less punitive in dealing with drug addiction. Since 2001, Portugal has treated drug addiction as a public health problem, as opposed to a criminal justice issue, allowing addicts a 10-day supply of drugs.⁸⁶ By contrast, the United States continues to criminalize drugs, the only policy that lightened sentences being the Fair Sentencing Act, signed by President Obama in 2010, which reduced the penalties for selling crack cocaine, a substance preferred by minorities. Ultimately, 12,000 federal prisoners stood to have their sentences reduced.⁸⁷

Police Violence

On August 9, 2014, Michael Brown, an 18-year-old black youth was shot to death by a Ferguson, Missouri police officer after a dispute involving an alleged convenience store theft, setting off weeks of disorder. Subsequent police shootings underscored the lethality that police used in apprehending black men.

- On November 11, 2014, Cleveland police killed 12-year-old Tamir Rice who had displayed a replica of a pellet gun in a public park.
- On April 2, 2015, an Oklahoma police officer killed unarmed, 44-year-old Eric Harris, explaining that he had intended to use his Taser.
- On April 4, 2015, a South Carolina police officer shot unarmed, 50-year-old Walter Scott in the back after a traffic stop.
- On April 12, 2015, unarmed, 25-year-old Freddie Gray died from injuries while restrained in a Baltimore Police van.

During the first five months of 2015, 385 police killings were recorded, 49 of the victims were unarmed; about half were minorities.⁸⁸

Excessive force by police resulted in multiple court settlements with victims and survivors. For example, the death of Freddie Gray revealed that between 2011 and 2014, the City of Baltimore paid-out \$5.7 million in settlements for police brutality.⁸⁹ Ultimately, the public pays for police violence not only through individual injury but also by higher taxes assessed to the public to reimburse victims. When local courts are unwilling to charge officers, the Department of Justice can be engaged on the basis that civil rights of victims may have been violated.

Police misconduct has been addressed through several strategies. Police civilian review boards have been established in several cities, although police tend to oppose such oversight. Improved training of police with regard to non-lethal apprehension of suspects, especially the inebriated and mentally ill has also been undertaken. More recently, video cameras have been added to police cars as well as officers' vests in order to record interactions with suspects. While technology can be helpful in recording police-citizen interactions, it is inherently limited. Police can engage in unlawful conduct outside the view of the camera or they can simply turn the camera off. Clearly, police engagement with

citizens, some of whom are seriously impaired, as in substance abuse or mental illness, or may threaten officer safety, as in gangs, presents a challenge to the nation's criminal justice system.

Conclusion

Because criminal justice has such profound implications for individuals and their communities, advocates continue to press for reform. After three decades of more rigorous policing, higher incarceration rates, and longer sentences, the cost of corrections has come to crowd-out such governmental obligations as public education and physical infrastructure. Consequently, reduced governmental revenues have caused public officials to seek more cost effective methods of crime control. Not surprisingly, liberal reformers have seen this as an opportunity to restore a measure of rehabilitation within a system that has focused on punishment. Massachusetts, for example, has attempted to reduce the stigma of crime by limiting employers' access to records on misdemeanors to 5 years and felonies to 10 years, at the same time providing protection to those who hire someone who commits a future crime.⁹⁰

Predictably, some reformers have proposed more humane facilities for inmates. Norway's Halden Prison, designed by Henrik Hoiland, resembles a college dormitory with individual rooms that include kitchenettes, complemented by a gym that includes a climbing wall.⁹¹ In Austria, Josef Hohen-sinn designed Loeben prison similarly in order to provide inmates with a more normal, home-like environment where inmates' rooms are arranged in 15-unit clusters around a common kitchen, with extensive educational and recreational facilities.⁹² Although the adoption of European reforms is unlikely in the United States, reformers have seized on opportunities to make correctional facilities more congruent with the needs of detainees. The Karnes County Civil Detention Center in Texas, for example, is an immigrant detention facility that more resembles a school than a prison, replete with a gym, library, computers with Internet access, a medical center, and sports areas for soccer, basketball, and volleyball.⁹³ Aesthetic considerations aside, humanizing corrections so that it is more congruent with community reintegration of inmates remains a primary challenge for advocates of prison reform.

DISCUSSION QUESTIONS

1. Navigate to the Congress.org website, identify legislation related to criminal justice, and trace its implications for social welfare. Has the policy been effective? What are its costs? How might it be made more adequate?
2. Crime varies considerably in terms of both the nature of offenses and the characteristics of offenders. How prevalent is crime in your community? What are the primary offenses? Who are the offenders?
3. State and local governments vary regarding the management of juvenile delinquency. Does your community have a juvenile court? Are its deliberations open to the public? What are the usual offenses that bring a youth to juvenile court?
4. Many communities are building new correctional facilities. Is your community upgrading corrections? Are for-profit correctional firms active in your state? If not, should they be? Should they be allowed to contract to manage probation and parole?
5. Legalization of drugs has become a heated issue. What are the implications of drug legalization for substance abuse programs? How could drug legalization be structured in your state? How would substances be taxed? How would tax revenues from legalization be allocated?
6. In many poor urban communities, drug-related street violence has escalated to unprecedented heights. How has your community balanced resource allocations for supply interdiction versus resources for demand reduction? To what extent is substance abuse treatment available to inmates in local correctional facilities?
7. How prevalent are hate crimes in your community? Is there any attempt to organize initiatives encouraging citizens to become more tolerant? Are victims of hate crimes in your community encouraged to take advantage of victim assistance benefits?
8. To what extent are social workers involved in community reentry activities? Does the local chapter of the National Association of Social Workers have a position on reentry programs? If not, what should it consist of? How would you encourage NASW to take up a reentry initiative?
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Child Welfare Policy



Source: Jack.Q/Shutterstock

Child protective services, foster care, adoption, and Head Start have been the focus of child welfare policy since the 1960s. In America, the condition of children is inextricably linked to the status of their families. Because the United States has failed to establish a family policy that ensures basic income, employment, and social service supports to parents, parents frequently have difficulty in caring for their children. As families are less able to care for their children, the demand for child welfare services escalates. According to six variables presumed to define the well-being of children, the United States ranks second to last among industrialized countries, as shown in Table 15.1.

The low ranking of the United States can be attributed to two factors: the high incidence of poverty among American children and the absence of universal health care for their families. One-fifth of children in the United States live in poverty, a condition that is associated with inadequate nutrition and shelter.¹ In addition, one-third of children lack access to health care due to absence of health insurance, which may improve due passage of the Affordable Care Act.² By contrast, nations ranking higher than the United States make more financial benefits available to families, and health care is universal providing all children access to health care.

In the United States, child welfare services are often controversial because they sanction the intervention of human service professionals in family affairs that are ordinarily assumed to be private matters and the prerogative of parents. This dilemma places extraordinary demands on child welfare professionals, who are mandated to protect the best interests of the child while not intruding on the privacy of the family.³ Recently, this conundrum has become more pronounced: Advocates for child welfare services demand more programs, but conservative, traditionalist groups attempt to cut programs that they see as designed to subvert the family. Ironically, much of this argument could be defused if the United States adopted a family policy that helped parents care for children more adequately, thus reducing the need for the more intrusive child welfare interventions. For the moment, however, the introduction of a comprehensive family policy is unlikely, leaving services to children fragmented.

TABLE 15.1 Ranking Child Well-Being in Developed Nations

Nation	Ranking
Netherlands	4.2
Sweden	5.0
Denmark	7.2
Finland	7.5
Spain	8.0
Switzerland	8.3
Norway	8.7
Italy	10.0
Ireland	10.2
Belgium	10.7
Germany	11.2
Canada	11.8
Greece	11.8
Poland	12.3
Czech Republic	12.5
France	13.0
Portugal	13.7
Austria	13.8
Hungary	14.5
United States	18.0
United Kingdom	18.2

Source: Child poverty in perspective: An overview of child well-being in rich countries, Unicef Innocenti Research Centre Report Card 7, Unicef. Retrieved from link http://www.unicef-irc.org/publications/pdf/rc7_eng.pdf.

History of U.S. Child Welfare Policy

Although many states established orphanages during the eighteenth century, current child welfare policy in the United States has its origins in the 1870s.⁴ The large number of child paupers led Charles Loring Brace, founder of New York's Children's Aid Society, to move thousands of children from deleterious urban conditions in New York City to farm families in the Midwest. Eventually, criticism of Brace's methods, which were divisive of family and community, contributed to more preventive approaches to children's problems. By the beginning of the twentieth century, most



Child welfare policy in the United States began in the 1870s. An early program led by Charles Loring Brace, founder of New York's Children's Aid Society, moved thousands of children from deleterious urban conditions in New York City to farm families in the Midwest. Under intense criticism, this program would later be replaced with programs that were less divisive to family and community.

Source: NMPFT/Kodak Collection/SSPL/The Image Works

large cities had children's aid societies that practiced "boarding out" of children (the payment of a fee for child rearing) to sponsors in the community.⁵ The boarding out of children until adoption (except in the case of children with disabilities, who were unlikely to be adopted) was the forerunner of today's foster care and adoption programs in the United States.

Protective services for children began with one of the more unusual incidents in U.S. social welfare. In 1874, a New York church worker, Etta Wheeler, discovered that an indentured nine-year-old child, Mary Ellen, was being tied to a bed, whipped, and stabbed with scissors. On investigating what could be done for Mary Ellen, Wheeler spoke with the director of the New York Society for the Prevention of Cruelty to Animals (NYSPCA) on behalf of the child. Although it was subsequently misreported that intervention on behalf of Mary Ellen was predicated on her status as an animal, rather than as a child, a careful review of the case indicated that Mary Ellen's case was adjudicated consistently with legal precedents involving abused children.⁶ The following year, the New York Society for the Prevention

of Cruelty to Children was established.⁷ By 1922, 57 societies for the prevention of cruelty to children had been established to protect abused youngsters.⁸

Child welfare proved an effective rallying issue for Progressives, who advocated intervention on the part of the federal government. In 1909, James E. West, a friend of former president Theodore Roosevelt and later head of the Boy Scouts of America, convinced Jane Addams and other welfare leaders to attend a two-day meeting on child welfare. This first White House Conference on Children focused attention on the plight of destitute families, agency problems with the boarding out of children, and the importance of home care. One significant product of the White House Conference on Children was the call to establish a federal agency to "collect and exchange ideas and information on child welfare." With an initial appropriation of \$25,640, the U.S. Children's Bureau was established in

1912 under the auspices of the Department of Commerce and Labor.⁹

Because of the economic circumstances of poor families, child labor emerged as a primary concern of early child welfare advocates. The absence of public relief meant that families were compelled to work at whatever employment might be available. Children worked full shifts in coal mines and textile mills; women labored in sweatshops. Neither group was protected from dangerous or unhygienic working conditions. Before the Great Depression, welfare advocates could boast of a series of unprecedented initiatives designed to improve the conditions of poor families in the United States. The Children's Bureau Act of 1912 established a national agency to collect information on children. The Child Labor Act of 1916 prohibited the interstate transportation of goods manufactured by children. The Maternity and Infancy Act of 1921 assisted states in establishing programs that dramatically reduced the nation's infant and maternal mortality rates. Yet, these hard-won successes were constantly at risk of being subverted. The Supreme Court ruled the Child Labor Act unconstitutional in 1918, and the Maternity and Infancy Act was

terminated in 1929 when Herbert Hoover and Congress refused further appropriations.¹⁰ Child and family welfare initiatives remained unsuccessful until the Social Security Act of 1935 ushered in an array of welfare policies.

The Social Security Act of 1935 addressed child welfare through two provisions. Title IV introduced the Aid to Dependent Children program, which provided public relief to needy children through cash grants to their families. Cash benefits were to be authorized and overseen by welfare case-workers who also provided social services to welfare families. Title V reestablished Maternal and Child Welfare Services and expanded the mandate of the Children's Bureau to oversee a new set of child welfare services "for the protection and care of homeless, dependent, and neglected children, and children in danger of becoming delinquent."¹¹ Significantly, both family relief and child welfare services were administered by the states through public welfare departments. As a result, as of 1935 the provision of child welfare services shifted largely from the private, voluntary sector to the public, governmental sector.

Since 1935, the state and federal governments have shared responsibility for social services provided to families, but the role of the federal government has changed with respect to income maintenance benefits to poor families. From 1935 until 1996, federal and state government shared the funding and administration of the Aid to Families with Dependent Children (AFDC) program. With devolution of public assistance to the states through welfare reform in 1996, however, the federal government limited its role to one of funding and limited oversight. Welfare reform instituted a block grant program, Temporary Assistance for Needy Families (TANF), primarily under the control of state government. Consistent with the residual conception of welfare, primary responsibility for children rests with their parent(s), with state intervention as a backup.

Ominously, after welfare reform a significant number of states reduced TANF funding. By 2013, only 51 percent of TANF funds were spent on basic assistance, child care, and work programs.¹² In 1996, 68 families out of every 100 poor families received TANF; but, by 2011, only 27 families received TANF.¹³ Thus, although welfare reform has contributed to a sharp drop in welfare rolls, reduced support for children and youth imperils their futures. The long-term prospects of child welfare are also problematic. Open-ended entitlements

for the elderly—Social Security, Medicare, and Medicaid—are likely to squeeze discretionary funding, resulting in lower appropriations for child welfare. Between 1960 and 2013, federal spending on the elderly increased from 2 percent to 9 percent of GDP, while that for children increased from 0.6 percent to 2.1 percent, a gap that will only widen with social program commitments to the 77 million retiring baby-boomers.¹⁴

Protective Services for Children

States proceeded to develop services to children independent of one another and within the relatively loose specifications of the Social Security Act. In the absence of a centralized authority that would ensure standardized care throughout the United States, child welfare services varied greatly from state to state and even within states. In the decades following the passage of the Social Security Act, a highly fragmented system of child welfare services evolved in the United States.

In the 1960s, reports of child maltreatment increased as a result of advances in radiological technology. A pediatrician, C. Henry Kempe, identified non-accidental injuries to children as the **battered child syndrome**. As more states began to address the problem, child welfare advocates built a compelling case for a national standard for child protective services. This lobbying led to the passage of the Child Abuse Prevention and Treatment Act (CAPTA) of 1974, which established the National Center for Child Abuse and Neglect (NCCAN) within the Department of Health and Human Services and presented a model statute for state child protective programs. All 50 states eventually enacted the model statute that, among its provisions, specified the following:

- A standard definition of child abuse and neglect
- Methods for reporting and investigating abuse and neglect
- Immunity for those reporting suspected injuries inflicted on children
- Prevention and public education efforts to reduce incidents of abuse and neglect

As a result of these national standards, the NCCAN was able—for the first time—to report trends in child abuse and the need for protective services for children. Alarmingly, the data collected

by NCCAN revealed a dramatic increase in reports of child abuse, more than doubling between 1976 and 1986, when reports of child abuse numbered 2 million.¹⁵ Reports of child abuse continued to climb through the mid-1980s, while at the same time expenditures for child protective services were decreasing.¹⁶

Increases in child abuse reports and decreases in expenditures led to a crisis in child welfare services. Douglas Besharov, an authority on child welfare policy, mapped the magnitude of the crisis in 1987:

Of the 1,000 children who die under circumstances suggestive of parental maltreatment each year, between 30 and 50 percent were previously reported to child protective agencies. Many thousands of other children suffer serious injuries after their plight becomes known to authorities. . . . Each year, about 50,000 children with observable injuries severe enough to require hospitalization are not reported.¹⁷

The rapid deterioration of child welfare services led children's advocates to call for more funding of social services. But proposals for increased support for child welfare services did not go unchallenged. Ambiguity in the definition of what constituted child abuse and neglect had contributed to incidents in which child welfare workers appeared to disregard parental rights in their eagerness to protect children. Further distracting public attention from child maltreatment, during the 1980s some family therapists suggested that therapy could "recover memories" of childhood abuse. By the end of the decade, showcase trials of child day care providers, teachers, clergy, and parents resulted in a series of convictions based on incidents that had been previously forgotten but were later retrieved during clinical treatment. Later, appeals raised questions about the veracity of allegations, the **recovered memory** movement came under scrutiny, and virtually all of the perpetrators convicted in earlier trials would be reversed on appeal.¹⁸

Although academics, policy wonks, and therapists debated definitional, ideological, and clinical aspects of child maltreatment, child protective services deteriorated. New York City's child welfare services became the focus of scrutiny in the cases of a series of children who died despite having been active child protection cases: Jeffrey Harden in 1992,¹⁹ Elisa Izquierdo in 1995, and Nadine Lockwood in 1996. An investigation into the Child

Welfare Administration showed the magnitude of agency failure: in a fifth of the reported cases, workers failed to interview all of the children in the family; in two out of five cases, workers ignored previous reports of child abuse; almost one-fifth of cases were closed despite the risk of future abuse. Following the exposé, the mayor announced plans to restructure child welfare.²⁰ Despite the appointment of a new commissioner of children's services,²¹ a subsequent report found that 40 percent of abused and neglected children were returned home and mistreated again.²² Reform would prove elusive, however. In September 2011, a four-year-old girl weighing 18 pounds died, despite the provision of children's services.²³ In an unprecedented response, the Brooklyn district attorney indicted a child welfare worker and his supervisor for criminally negligent homicide for failing to protect the child.²⁴ Efforts to make child protection more accountable by state statute requiring openness of cases involving child fatalities, foundered on the shoals of the child welfare bureaucracy: An investigation by the *New York Times* revealed that the state's Office of Child and Family Services had been working to keep cases involving child fatalities secret.²⁵

Nor were other cities spared. The District of Columbia's Child and Family Service Agency (CFSA) had been so mismanaged that it was placed under a court-appointed receiver. After several years, the staff was upgraded until virtually all were MSWs; yet, the agency continued to show serious performance problems,²⁶ chief among them being high caseloads and high turnover.²⁷ In 2001, the death of a 23-month-old African American infant due to child abuse led reporters from the *Washington Post* to investigate other child deaths. Within a year, reporters had identified the outline of a rather imposing iceberg: Their research revealed that between 1993 and 2001, 229 children had died as a result of maltreatment in the District of Columbia, but the deaths had gone without proper investigation on the part of the Metropolitan Police Department and Department of Child and Family Services.²⁸ In 2009, Bonita Jacks was found with the bodies of her four children, whom she would later be found guilty of murdering; several child welfare workers were dismissed for failing to protect the Jacks children.²⁹

In Florida, a five-year-old Miami girl in foster care, Rilya Wilson, had been missing for 15 months; yet, Florida child welfare workers were unable to

locate her.³⁰ Although the head of the Florida family service agency admitted that the performance of child welfare workers was “appalling,” she had no explanation for the whereabouts of the 374 other foster children the state was unable to locate.³¹ Florida child welfare workers replicated the performance of those who had been encountered by reporters elsewhere: Rather than admit the extent of problems in children’s services, workers used confidentiality as an excuse to avoid public accountability. Years after the exposé, Rilya Wilson is yet to be found.³²

In January 2003, the decomposing body of seven-year-old Faheem Williams was found in a Newark basement; his two brothers had been locked in a nearby room, “emaciated and with burn scars on their bodies.”³³ Although Williams had been an active protective services case with the New Jersey Division of Youth and Family Services (DYFS), multiple agency errors failed to save him. Subsequently, the *New York Times* obtained case records on 17 children that documented the deaths of four children and “the sometimes brutal, prolonged abuse of 13 more.”³⁴ As the *Times* would learn, the culture at DYFS conspired to expose children to risk. Rather than redoubling efforts to protect maltreated children, child welfare workers were so intent on closing cases that they often took shortcuts, contravening agency policy and state law. In a grim parody of street slang, workers came to refer to the rush to eliminate cases as “drive-by closings.” For years, DYFS used confidentiality to avoid public scrutiny.³⁵ Though the expose of DYFS brought the agency’s defensive practices to light, it did little to correct for its malfeasance. Within a year of the Williams tragedy, Matthew Calbi was beaten to death by his mother, despite four investigations of the family during the previous two years.³⁶

Tragically, the deaths of children due to abuse and neglect were not a local problem but one that had become nationwide. During Congressional testimony, an official of the Government Accountability Office suggested that 1,770 child fatalities in 2009 was an inaccurate number; a more likely figure was 2,400, concluding that “little has changed in 40 years.”³⁷ In rating the child homicide rates for industrial nations, a British researcher concluded that the child fatality rate of the United States was double that of the second most lethal nation for children, Australia.³⁸ The rate of reported as well as confirmed cases of abuse and neglect in the United States was more than double that of the United

Kingdom or Canada.³⁹ The systematic failure of child welfare led to the establishment of Children’s Rights, a legal advocacy organization that, by 2015, had litigated consent decrees with 16 states for their failure to serve maltreated children.⁴⁰

By 2012, child maltreatment had stabilized at 9.2 victims per 1,000 children, ranging widely among states from 1.2 to 19.6. African American children are at higher risk, at 14.2 victims per 1,000 children, while Asian American children are lowest at 1.7 victims.⁴¹ Annually, 3.2 million children are investigated for abuse and neglect,⁴² however, a large portion of reports are unsubstantiated, associated with anonymity in reporting.⁴³ Mischief in reports of maltreatment, attributable to unhappy children and vengeful ex-partners, divert child welfare resources from interventions involving founded cases of maltreatment.

For decades, the Annie E. Casey Foundation has invested tens of millions of dollars in children’s services, identifying and supporting model programs across the nation. Unfortunately, the exemplary programs it subsidized remained just that, isolated from the mainstream and failing to catalyze system-wide reforms. In 2003, the Foundation identified a significant impediment to progress, the human services workforce:

Human services is reaching a state of crisis. Frontline jobs are becoming more and more complex, while the responsibility placed on workers remains severely out of line with their preparation and baseline abilities. Many are leaving the field, while a new generation of college graduates shows little interest in entering the human services sector. Millions of taxpayer dollars are being poured into a compromised system that not only achieves little in the way of real results, but its interventions often do more harm than good. It is clear that frontline human services jobs are not attracting the kinds of workers we need, and that regulations, unreasonable expectations, and poor management practices mire workers and their clients in a dangerous status quo.⁴⁴

Efforts to improve the quality of services to maltreated children, often uncoordinated with previous reforms, have shown little evidence that they are effective, thus contributing to the disarray of child welfare. The costs of inferior care of vulnerable children are substantial. In 2010, the District of Columbia agreed to pay \$10 million for failing

to prevent the permanent brain damage of a child whose foster mother shook him.⁴⁵

Substandard care also exposes child welfare professionals to liability. The qualified immunity of child welfare workers established through *DeShaney v. Winnebago County Department of Social Services* (1989) was successfully challenged in *Currier v. Doran* (2001) in which child welfare workers were found liable when their actions “created the danger” that harmed a child. *Currier* has momentous implications for child welfare professionals. Foremost, “social workers who blatantly fail to heed warning signs of abuse or potential abuse will not be protected from liability.”⁴⁶ But *Currier* has much broader implications. Workers cannot claim that high caseloads due to agency underfunding provide an excuse for negligence. Moreover, supervisors are legally liable if their “deliberate indifference” to clients’ welfare is reflected in their failure to adequately train subordinates.⁴⁷ Thus, *Currier* underscores the need for child welfare professionals to demand adequate resources in order to meet their obligations to provide adequate care for at-risk children.

An especially pernicious form of child abuse has been at the hands of clergy. In 1988, the Social Network of those Abused by Priests (SNAP) was established as a nonprofit platform for victims.⁴⁸ Because of the nature of the abuse, victims have been reluctant to come forth; however, the stigma and pain has diminished by SNAP’s advocacy, as more victims confront abusers and the church. Although the magnitude of the problem has been difficult to ascertain, 848 priests had been defrocked by the Vatican since 2004 due to child sexual abuse.⁴⁹

In recent decades the focus in child welfare has vacillated between protecting children and preserving families. After passage of CAPTA, which focused on protecting children, Promoting Safe and Stable Families program, was enacted in 1993 focusing on family preservation.⁵⁰ Increasing child fatalities, due to children staying in risky homes, resulted in passage of the Adoption and Safe Families Act of 1997, which mandated that parental rights be terminated for children in care for 15 of the previous 22 months. The Fostering Connections to Success and Increasing Adoptions Act of 2008, establishing custodial rights for members of a child’s extended family and making them eligible for cash benefits, thus emphasizing family preservation. Absent sound research on what benefits at risk

children, child welfare policy has been fashioned according to scandals around child fatalities and questionable removal of children from their homes, in other words safety and permanency, while child well-being gets short shrift.

Foster Care for Children

When parents are unable to care for their children, foster care provides alternative care. There is an important relationship between child protective services and foster care: Foster care is primarily a service for victims of child abuse; more than half of children in foster care were placed there by child protective service workers. The second most prevalent reason for child foster care is the “condition or absence of the parent,” accounting for about 20 percent of foster care placements.⁵¹

As in the case of protective services, foster care for children was not coordinated under the provisions of the Social Security Act. States adopted separate policies and, unfortunately, took few measures to monitor children in foster care. During the early 1960s, a series of studies began to document a disturbing development: Rather than being a temporary arrangement for child care, foster care had become a long-term experience for many youngsters, with 70 percent of children in foster care for more than one year.⁵² Not only had states planned poorly for the reunification of children with their original families, but also in many instances child welfare agencies lost track of foster care children altogether. In response to the deterioration of children’s services, **permanency planning** became a central feature of the Adoption Assistance and Child Welfare Act (AACWA) of 1980. As a result of permanency planning, the number of children in foster care plummeted from 330,400 in 1971 to 262,000 in 1982, a reduction of 20.7 percent.⁵³ Welfare workers swiftly removed children from foster care and reunited them with their biological families under the rationale that community support services would assist parents.

Tragically, inadequate resources sometimes created a vicious circle: When biological parents received few support services, they were less able to care for their children, thereby contributing to the need for child protective services. In the absence of intensive support services, permanency planning for many children meant a revolving door—placement in foster care, reunification with the biological parent(s), and

then a return to foster care. In large measure, the permanency planning movement faltered because of lack of support services to families. By the late 1980s, with permanency planning beset with multiple problems, foster care was an unreliable way of serving many of the most endangered children in the United States. Since 1995, about 500,000 children have been in foster care, almost twice the number a decade before,⁵⁴ although the number would drop to 400,000 two decades later.

Several issues have emerged to dampen enthusiasm about foster care as a solution for children from very troubled families. Too often, foster care has emerged as the catalyst of family dramas that become the grist of journalistic accounts of a child welfare system that “is infected with mistrust, backbiting and second-guessing,” hardly a portrait that builds public confidence. One reporter summarized his impression of child welfare:

As soon as a complaint is received, a virtual industry of social workers, lawyers, judges and administrators goes to work, and no matter how pure the intentions are, the process often deteriorates into chaos. A central goal of child welfare is permanence and stability for the child, but cases routinely become so mired in complications, and legalities, and indecision, and nastiness, and the necessity of trying to understand a specific moment of horror in the larger context of societal issues, that the focus can shift from the search for permanence to the mere passage of time. Months pass. Years pass. Rather than resolution, there is drift, so much that nationally, on average a third of the children entering foster care will be there in excess of two years, creating, instead of stability, an ambiguity that can be damaging.⁵⁵

Increasing demand for foster parents and homosexual rights collided in a controversy about the suitability of gays and lesbians as parents. A Texas child welfare supervisor ordered the removal of a foster child from a lesbian foster parent, arguing that the state’s anti-sodomy statutes defined homosexual households as unacceptable.⁵⁶ An Arkansas judge ruled a state ban on placing foster children with families having a gay member as unconstitutional.⁵⁷ Yet, the U.S. Supreme Court refused to sort out conflicting state policies, letting stand a Florida prohibition on adoption by same-sex parents.⁵⁸ The *Washington Post*’s Colbert King undoubtedly spoke for many when he observed,

“I couldn’t care less what those [foster] parents look like as long as they are strong in themselves and are giving that kid the sense of security, belonging, and love that every child on this earth needs.”⁵⁹

Although a desirable outcome for children in foster care would be reunification with biological parents better able to care for them, about 25,000 foster children simply age out of care each year. Their prospects are often grim. A study of “graduates” of foster care revealed that 37 percent had not graduated from high school, 18 percent had been incarcerated, and 12 percent had been homeless.⁶⁰ A subsequent study of children who have aged out of foster care in the Northwest revealed that, within the 12 months prior to being interviewed, 20 percent had a major depressive episode, 12 percent suffered from alcohol and drug dependence, and 25 percent were diagnosed with posttraumatic stress disorder.⁶¹ A study of former foster children from the Midwest found that 6 of 10 boys had been convicted of a crime, while three-fourths of women were on public assistance. “We took them away from their parents on the assumption that we as a society would do a better job of raising them,” observed researcher Mark Courtney, “We’ve invested a lot of money and time in their care, and by many measures they’re still doing very poorly.”⁶² In 1999, the Foster Care Independent Living Act provided funding for education and employment services for children who were aging out of foster care; foster youth were entitled to vouchers worth up to \$5,000 per year for higher education, for example.⁶³

In 2008, President George W. Bush signed Fostering Connections to Success and Increasing Adoptions Act, the most extensive child welfare reform in three decades. Among its provisions were guardianship payments to relatives of children in foster care, tribal access to federal child welfare funding, and an extension of benefits to foster children until age 21 providing they are in school or working. Despite its emphasis on maintaining family integrity, the Act budgeted only \$15 million annually for Family Connection grants.⁶⁴ Promoted by advocacy organizations, such as the Children’s Defense Fund and the Center for Law and Social Policy, Fostering Connections attempted to keep siblings together within the network of their biological family, citing impressionistic evidence that this was superior to traditional foster care.⁶⁵

Despite increases in child welfare funding to encourage innovations, such as permanency

planning, family preservation, and kinship care, there is little evidence of the efficacy of these initiatives. Two child fatality researchers concluded, “The vast majority of state policy that is enacted is not evaluated for effectiveness. Child welfare policy is no exception. The child welfare profession is especially susceptible to ideological movements that are implemented without the checks and balances of research and evaluation.”⁶⁶ Unlike health care and welfare reform, randomized controlled trials in child welfare are a rarity: “there is not a single intervention that has generated a published peer-review article based on a study in which they accepted referrals from a child welfare agency, randomly assigned them to a treatment condition, and evaluated the outcome,” concluded leading researchers in the field.⁶⁷ Equally troubling is the absence of research on child well-being.⁶⁸ Instead of assessing the benefits of children’s services across several variables, the field has been content to use “avoidance of harm” as a standard of effectiveness. In the absence of such research, child welfare interventions are apt to prove ineffective. Indeed, a study of abused children revealed that child protective services intervention resulted in no improvement of families visited by a child welfare professional compared to a control group that received no intervention, except that the mothers in the intervention group were more likely to be depressed.⁶⁹ Thus, data collection on child welfare programs and field research on program performance are woefully inadequate.

Adoption

From the standpoint of permanency planning, adoption has become an important child welfare service. In the early 1980s, the Children’s Bureau noted that 50,000 “hard-to-adopt” children were waiting for homes. Many of these children were of minority origin, had disabilities, or were older and had been in foster care for several years; yet, subsidized adoption proved cost-effective, 37 percent less than foster care.⁷⁰ Because such children posed a financial burden for adoptive parents, AACWA provided subsidies to adoptive parents. In 1983, 6,320 children were being subsidized each month at a cost of \$12 million,⁷¹ but between 1995 and 2002, the number of agency-involved adoptions virtually doubled from 25,693 to 50,950, respectively.⁷² Through the Fostering Connections Act,

the number of foster care adoptions approached 54,000, in part due to incentives provided to states: \$4,000 for each foster child adopted and \$8,000 per foster child over age 9.⁷³

Still, adoption is not without controversy. Because children come from a variety of racial and cultural groups, questions have been raised about transcultural adoption. Should agencies give consideration to maintaining the cultural identity of children placed for adoption by finding them homes in their birth culture? This question was at the heart of the Indian Child Welfare Act of 1978. Native Americans were disturbed that “25 to 35 percent of all American Indian children [were] separated from their families and placed in foster homes, adoptive homes, or institutions.”⁷⁴ The fact that 85 percent of such placements were in non-Indian families and left the children “without access to their tribal homes and relationships” raised the specter of partial cultural genocide.⁷⁵ To reinforce the cultural identity of Native American children, the Indian Child Welfare Act provided for “minimal Federal standards for the removal of Indian children from their families and the placement of such children in foster or adoptive homes which will reflect the unique values of Indian culture, and for assistance to Indian tribes in the operation of child and family service programs.”⁷⁶ Equally important, the Indian Child Welfare Act established tribes, rather than state courts, as the governing bodies responsible for Indian foster children. The 2008 Fostering Connections Act provided Native American tribes with direct access to federal child welfare funding.

The National Association of Black Social Workers (NABSW) advanced a similar argument for culturally appropriate placement of children during the 1980s, noting a high percentage of African American children placed with white families. NABSW contended that cross-racial adoptions deprived individual children of their racial identity and would result in cultural genocide of the African American community. Research on cross-racial adoption, however, consistently found that African American children did not suffer adverse consequences from growing up in white families. In 1996, former president Clinton signed legislation forbidding interference by organizations, except by Indian tribes, in child placement for reasons based on race.⁷⁷ Although provisions to reinforce the cultural identity of children have unquestionable merit in a pluralistic society, the circumstances of many racial and cultural minorities leave the

implementation of such policies in doubt.⁷⁸ Without basic health, education, and employment supports, many minority families are likely to experience difficulty adopting. For example, the number of African American children available for adoption far outstrips the number of African American families able to adopt children, despite the fact that African American families “adopt at a rate 4.5 times greater than white or Hispanic families.”⁷⁹

Changes in family composition further cloud the picture. The pool of adoptive families has diminished with the increase in the number of female-headed households. The combination of low wages for women and a shortage of marriageable men means that mothers are encouraged to maintain small families, not to expand them through adoption. To some extent, this problem will be mitigated by the Fostering Connections Act, which provides payment to guardians of foster children.

Head Start

In response to concerns about the lack of educational preparation of poor children, Head Start was incorporated in the Economic Opportunity Act of 1964. The first Head Start programs were established in poor communities a year later. Intended to compensate for a range of deficits displayed by poor children, Head Start offered health and dental screening, nutrition, and socialization experiences in addition to preschool academic preparation. Of the Great Society programs, Head Start was one of a few that captured the imagination of the nation.

Despite wide public support, however, participation in Head Start was uneven. Significantly, it was not until 1995 that the enrollment of Head Start eclipsed that of 1966; today, fewer than half of eligible children participate in the program.⁸⁰

During the 1980s, when government assistance to the poor was reduced, many poor families dispatched both parents to the labor market to stabilize family income, and this increased the need for Head Start. Even the Deficit Reduction Act of 1990, which held spending for most social programs in check, provided for modest increases in Head Start.⁸¹ The 1996 welfare reform act, which required recipients of family cash assistance to participate in the labor market, increased Head Start enrollments even further. In 2014, Head Start spent \$8.6 billion to serve 1 million children; 51 percent of children enrolled were age four and 35 percent were age three; by race, 29 percent were African American, by ethnicity, 38 percent were Hispanic. Twelve percent of Head Start children have disabilities.⁸² Yet, the long-term value of Head Start has been questioned; a 2014 evaluation of Head Start programs concluded, “there is no indication that either high quality Head Start or low quality Head Start in any dimension leads to program impacts lasting into third grade.”⁸³

Emerging Issues in Child Welfare

Changes in the economic and social circumstances of families in the United States have broadened the scope of the issues that traditionally defined child



Day care for children has become more important as more and more parents with children work.

Source: oliveromg/Shutterstock

welfare policy. Of these changes, three are likely to shape child welfare in the future: day care, maternal and child health, and teenage pregnancy.

Day Care

Day care for children has risen in importance as more and more parents with children work. In 1947, only 12 percent of mothers with children under age six worked outside the home, but by 2002 this became 64.1 percent.⁸⁴ The need for child day care is felt both by middle-income families, in which both parents work in order to meet the income requirements of a middle-class lifestyle, and by low-income families, in which a parent is encouraged or required to participate in a welfare-to-work program. Yet, the child care available is often unreliable, expensive, and of questionable quality.

The primary programs assisting parents with child care are the federal dependent care tax credit, the child care tax credit, the Child Care and Development Block Grant (CCDBG), and Title XX. The largest federal child care program, CCDBG, allocated \$11.4 billion in 2012 to states for care of low-income children, the lowest amount in a decade.⁸⁵ The dependent care tax credit allows families to deduct \$3,000 for one child or \$6,000 for two children spent on child day care for a given year; the child care tax credit of \$1,050 per child (\$2,100 total) is non-refundable and phased out with increased income.⁸⁶ Under Title XX, states are able to purchase day care for poor families; but as of 2001 only \$129 million, or 7.6 percent of Title XX funding was allocated for child care.⁸⁷

Maternal and Child Health

Maternal and child health has emerged as an issue among child welfare advocates as younger, poor women give birth to low-birth-weight babies who have received inadequate prenatal care. Low birth weight is a concern because such infants have a higher incidence of developmental disabilities, some of which are permanent and eventually require institutional care. Low birth weight in minority infants is of particular concern because it relates to lack of adequate prenatal care.

The primary federal program to enhance prenatal care for low-income families is WIC, the Special Supplemental Nutrition Program for Women,

Infants, and Children. Under WIC, low-income pregnant and nursing women and their young children are eligible for food coupons through which they may obtain especially nutritious foods. Even though the WIC program would seem a logical method for addressing the low-birth-weight problem of infants born to poor women, participation in the WIC program is not at desirable levels. Nationwide, only about half of the people financially eligible to participate in WIC do so.⁸⁸ In 2014, WIC served 8 million women and infants at a total program cost of \$6 billion, or about \$43.65 per recipient.⁸⁹

Important health care programs have served children, such as Medicaid and the State Child Health Insurance Program (SCHIP). Like many welfare programs, these have not proved ideal. Medicaid reimburses health care providers who agree to the programs' procedures and payments; but if providers refuse to accept Medicaid, the benefit is reduced to symbolic value.⁹⁰ Restrictive eligibility provisions introduced by the 2006 Deficit Reduction Act required beneficiaries to produce a birth certificate or passport in order to retain Medicaid. Because the parents of many children, especially immigrants, were unable to produce the necessary documentation, tens of thousands of children were deleted from Medicaid.⁹¹ During discussions of health care reform, children's advocates insisted that SCHIP be reauthorized separately in order to assure its continuance regardless of the outcome of health reform; thus SCHIP will continue in its current form, allowing states to extend Medicaid or fashion an independent program to provide health care to low-income children.⁹² Among the most important provisions of the 2010 Affordable Care Act is the inclusion of \$1.5 billion over five years for nurse home visiting, an empirically tested program through which professional nurses meet with young mothers to assist them through pregnancy and care of their infants.⁹³ The nurse home visiting initiative is especially important in light of the increasing number of unmarried mothers, in 2007 numbering 1.7 million.⁹⁴

Teen Pregnancy

The Guttmacher Institute reported that the teen pregnancy rate for 2010 had declined 51 percent from its highest in 1990. Yet, teen parenthood remains a significant problem: in 2010, 367,678

Spotlight 15.1

BENEFITS FOR AT-RISK CHILDREN

Advances in public health have also benefited at-risk children. The Adverse Childhood Experience (ACE) study, for example, has associated morbidity and mortality with problems many children experience, such as maltreatment, parental substance abuse, among other conditions.⁹⁵ In response, multidisciplinary teams have demonstrated how interventions around symptoms, including asthma and diabetes,

teens became mothers.⁹⁶ Problems relating to maternal and infant health are exacerbated by the numbers of adolescent females having children. Out-of-wedlock births became an important family issue in the 1980s, when the incidence of unwed motherhood increased so rapidly that by 1983 half of all nonwhite births were outside of marriage.⁹⁷ In 2013, 43.3 percent of births were by unmarried women of all races, 61.7 percent for African American women, 69.9 percent for Hispanic women.⁹⁸

Childbearing among very young unmarried women poses a serious problem for public policy for two basic reasons. First, teenage mothers are more likely to drop out of school and thus fail to gain skills that would make them self-sufficient. Adolescent mothers, particularly those who are African American or Hispanic, are apt to have less command of these basic skills. Poor skill development represents an especially critical problem when the skills in question involve parenting. Second, teenage mothers are more likely to depend on welfare, the benefits of which are at levels lower than the actual cost of raising children. This combination of inadequate skill development and dependence on public welfare presents the specter of poor teenagers bearing poor children in an endless cycle of hopelessness. The consequences are particularly tragic for the children, who have little prospect of escaping the poverty trap. Reductions in the numbers of working African American and Hispanic males who are marriageable means that many of these children have little hope that their mother will marry and thus pull them out of poverty, which had

can yield long-term benefits as children enter adolescence and adulthood.



Watch this TED Talks video featuring Dr. Nadine Harris to learn how a focus on childhood can lead to long-term health benefits across a lifetime.

www.youtube.com/watch?v=95ovIJ3dsNk

been the most prevalent way for mothers to become independent of public welfare.⁹⁹ The loss of buying power of the income support provided by TANF has meant that public assistance does not provide an adequate economic base for poor children.¹⁰⁰

Conclusion

After a half century of federal legislation, many child welfare advocates have become pessimistic about the care provided to youngsters in the United States. Hopes for using the family as the primary institution for child welfare have faded in the absence of economic and social supports to keep families intact. The 1996 imposition of welfare time limits casts a long shadow over poor families. Although by the end of the twentieth century, some of the benefits of unprecedented prosperity and low unemployment began to trickle down to the poor, the economic crisis that began in 2008 has left many welfare and working poor families struggling even more. Aside from stimulus funding, the Obama administration has introduced no new initiatives in child welfare.

Federal cuts in child welfare spending focused efforts on the states. Unfortunately, the Great Recession adversely affected state budgets, resulting in cuts in services to children and their families, especially those among minorities. Since 1996 states have been able to seek waivers from the federal government, essentially permitting replication of experiments in welfare reform. However, while many states have received child welfare waivers,

the results have been disappointing. Eleven waivers were terminated or failed to produce expected outcomes, while 10 generated weak findings. Ultimately, only five waivers yielded strong results; three in implementing guardianship, one in flexible funding, and another in substance abuse.¹⁰¹ Thus, state waivers remain a largely unrealized vehicle for child welfare reform.

DISCUSSION QUESTIONS

1. Using the Congress.org website, identify a child welfare policy of interest to you. How has programming changed? Have budget appropriations been adequate to address the problem? What could be done to make the policy more effective?
2. Much of child welfare—protective services, foster care, and adoptions—is funded through a complex array of categorical programs. How does your welfare department optimize reimbursement through these funding sources? As a result of reimbursement systems, what are the priorities for children's services? How would you reconcile discrepancies between categorical funding priorities and community needs?
3. Maintaining the cultural identity of minority children who receive foster care and adoption services is a heated issue in child welfare. When there are too few minority families for the children needing foster care and adoption, what should the policy of the welfare department be in placing minority children? How consistent is your answer with current child welfare policy in your community?
4. Providing preschool programs for children is an increasingly important issue as more mothers enter the workforce. How adequate is day care provision in your community? Who is responsible for the oversight of day care? To what extent are the needs of low-income families considered in day care arrangements? What percentage of families eligible for Head Start actually participate in this program in your community?
5. To what extent have Adverse Childhood Experiences affected your classmates? What are the incidences of infant mortality and low birth weight for children of teenage and minority mothers in your community? What plans does your community have for improving the child welfare of minority and low-income families?
6. *Currier* holds child welfare workers liable in the event a child is harmed. To what extent have child workers in your community become aware of this judicial decision? How could it be used to empower child welfare workers?

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Housing Policies



Source: James Martin/Fotolia

A house represents more than a mere address of domicile in America: It is the primary asset of most American families and a significant expenditure. On average, Americans spend 34 percent of their annual income on housing.¹ Homeownership appears to result in more stability since renters are 2.7 times more likely to relocate in a given year than homeowners.² The stability encouraged by homeownership may also be a prerequisite to the development of neighborhood identity, involvement in local social networks, and the engagement with local institutions and organizations that strengthen social support. However, with the advent of the housing market bust and the 2007 recession, the difficulties in owning and protecting an existing home from foreclosure have become more prominent.

Most of the recent attention on housing has focused on the middle class. Yet, housing problems affect millions of Americans. Persistent problems include a lack of low-income affordable housing, a large inventory of dilapidated and dangerous housing, and high numbers of homeless people. Although housing discussions have frequently focused on homelessness, finding and maintaining adequate and affordable housing is also problematic for people on public assistance, for the working poor, and for a large section of the middle class. This chapter discusses federal housing programs directed toward homeownership and examines housing problems in the United States, with particular emphasis on low-income housing, affordability, homelessness, and proposals for housing reform.

Overview of Housing Legislation

Federal housing legislation began in 1937. As Figure 16.1 illustrates, this legislation developed into a tangle of laws that often evolved in conflicting directions. In 1990, Congress passed the Cranston-Gonzales National Affordable Housing Act, which authorized housing-related block grants to state and local governments. The goals of the 1990 legislation included: (1) decentralizing housing policy by allowing states to design and administer their own housing programs; (2) using nonprofit sponsors to help develop and implement housing services; (3) linking housing assistance more closely with social services; (4) facilitating homeownership for low- and moderate-income people; (5) preserving existing federally subsidized housing units; and (6) initiating cost sharing among federal, state, and local governments and nonprofit organizations.³ This act also introduced the HOME investment partnerships block grant

program, the Homeownership and Opportunity for People Everywhere (HOPE) program, and the national homeownership trust demonstration.

Key HUD (the Department of Housing and Urban Development) programs to increase the stock of affordable housing include:

1. The HOME program expands the number of affordable housing for low- and very-low-income families by providing grants to state and local governments. Participating jurisdictions use their grants to finance housing programs that meet local needs. Jurisdictions have flexibility in designing their local HOME programs within HUD and legislative guidelines. They may also use their HOME funds to help renters, new homebuyers, or existing homeowners.
2. The SHOP program funds nonprofit organizations to buy home sites and develop or improve the infrastructure needed for sweat equity so that low-income families can purchase homes. The program is designed to attract national and regional nonprofit organizations with experience in using volunteer labor to build housing (i.e., Habitat for Humanity).
3. The HOPE program is designed to facilitate homeownership by low-income families through the sale of publicly owned or held homes to current residents or other low-income households. HOPE also includes efforts to combine social services with housing assistance for households of the elderly and disabled, who would otherwise be unable to live independently.⁴
4. The linkage between housing and social services was strengthened by the **Family Sufficiency Program** that called for public housing authorities (PHAs) to help residents obtain coordinated social services to assist them in gaining employment. Participating families must complete these programs or risk losing their housing assistance. In return, as a participant's income increased, the money that would normally go toward a higher rent (calculated at 30 percent of income) was set aside in a special escrow account to be used for the purchase of a home. The legislation also included modest funds to create "Family Investment Centers" that provided social services in or near public housing projects.⁵

In 1998, former president Clinton signed the Quality Housing and Work Responsibility Act of 1998 (QHWRA), in some ways the housing equivalent of

The Housing Act of 1937	The United States had no national housing policy prior to the Housing Act of 1937. The objective of the act was to “provide financial assistance to the states and political subdivisions thereof for the elimination of unsafe and unsanitary housing conditions, for the eradication of slums, for the provision of decent, safe and sanitary dwellings for families of low income, and for the reduction of unemployment and the stimulation of business activity, to create a United States Housing Authority and for other purposes.”*
The Housing Act of 1949 (Amended 1937 Act)	This amendment called for federal money for slum clearance and urban redevelopment and for the creation of a public authority charged with building and administering low-income housing units. Specifically, this bill required each locality to develop a plan for urban redevelopment that contained provisions for “predominantly residential dwellings.” The wording of this bill was interpreted by localities to mean that only one-half of new construction was to be devoted to low-income housing.
Housing Act of 1954 (Amended the 1949 Act)	“Urban development” was changed to urban renewal, and localities were required to submit a master plan for removing urban blight and for community development. The act removed the requirement that new federally subsidized urban construction be “predominantly residential,” clearing the way for massive slum clearance projects. It also allowed localities to more easily lease or sell land and to avoid the construction of public housing. Through renewal projects, localities tried to revitalize inner cities by attracting middle- and upper-income families at the expense of displaced poor families. From 1949 to 1963, urban renewal projects removed about 243,000 housing units and replaced them with 68,000 units, of which only 20,000 were for low-income families.
The Demonstration Cities and Metropolitan Development Act (Model Cities)	Passed in 1966 as part of President Lyndon Johnson’s War on Poverty, the act focused on deteriorated housing and blighted neighborhoods. The Model Cities legislation promised to “concentrate public and private resources in a comprehensive five-year attack on social, economic, and physical problems of slums and blighted neighborhoods.” The Model Cities Act and virtually all neighborhood development acts were superseded by the Housing and Community Development Act of 1974.
The Housing and Community Development Act of 1974	This wide-ranging bill included provisions for urban renewal, neighborhood development, model cities, water and sewer projects, neighborhood and facility grants, public facilities and rehabilitation loans, and urban beautification and historic preservation. Although spending priorities were determined at the national level, communities were required to submit a master plan, including specific references to their low-income housing needs.
Home Mortgage Disclosure Act (HMDA)	This act focused on mortgage redlining. Advocates argued that a major cause of community deterioration was a “lending strike,” or redlining, by financial institutions. Redlining is defined as “an outright refusal of an insurance company, bank, or other financial institution to provide its services solely on the basis of the location of a property.” The term is derived from the practice of marking in red the area on a map avoided by insurance or financial institutions. As a result, families seeking to purchase a home in a redlined neighborhood may be denied a mortgage, insurance, or other necessary services. In 1976, President Gerald Ford signed the Home Mortgage Disclosure Act, which required virtually every bank or savings and loan association to annually disclose where it made its loans.

FIGURE 16.1 Historical Highlights of Pre-1990 Housing Legislation

Community Reinvestment Act (CRA) of 1977

Broader than the HMDA, the CRA established the principle that each bank and savings institution has an obligation to make loans in every neighborhood of its service area. Virtually all lending institutions are covered under the CRA, and the law requires the federal government to annually evaluate the performance of each lending institution. Primary enforcement involves control by federal regulatory agencies over new charters, bank growth and mergers, relocations, and acquisitions. Only a handful of the CRA challenges result in punitive action against lenders. The power of the act rests with the ability of community groups to win concessions from lending institutions, usually in the form of negotiated settlements.

*United States Housing Act of 1937, U.S. Statutes at Large (75th Cong., 1st Sess., p. 888–899). U.S. Constitution. Retrieved from http://www.congressionaltimeline.org/Documents/75th_pg1_09011937_doc1_pg1-12.pdf

FIGURE 16.1 (Continued)

Sources: Based on Quoted in Charles S. Prigmore and Charles R. Atherton, *Social Welfare Policy: Analysis and Formulation* (Lexington, MA: D. C. Heath, 1979), pp. 146–147; Robert Morris, *Social Policy of the American Welfare State*, 2nd edition (New York: Longman, 1985), p. 131; Barbara Habenstreit, *The Making of America* (New York: Julian Messner, 1971), p. 46; Richard Geruson and Dennis McGrath, *Cities and Urbanization* (New York: Praeger, 1977), pp. 6–7; National Training and Information Center, *Insurance Redlining: Profits v. Policyholders* (Chicago: NTIC, 1973), p. 1.

the 1996 PRWORA. In line with the PRWORA, the federal government devolved its responsibility as the primary agent for publicly assisted housing to 3,400 semiautonomous local PHAs. With input from public housing residents, these strengthened PHAs determine rents, admissions policies, and what (if any) social services are provided. The QHWRA also ties workforce participation into housing benefits. For example, (1) PHAs can consider prospective tenants' employment history in the admissions decision, (2) TANF recipients who fail to fulfill their work requirement can lose Section 8 or public housing benefits, and (3) PHAs are encouraged to recruit "good" working-class families to act as role models for welfare-dependent families (the legislation has decreased the percentage of public housing units earmarked for very low-income families from 75 to 70 percent). The QHWRA also mandates that low-income tenants take personal responsibility for moving into better living conditions (e.g., a single-family home).⁶

On February 17, 2009, former President Obama instituted the American Recovery and Reinvestment Act of 2009 to soften the impact of the recession on the economy and employment. Specially, Congress provided the Department of Transportation and the Department of Housing and Urban Development \$61.7 billion in additional funding. HUD used its portion of the funds to build and renovate housing infrastructure, thus stimulating the construction industry and boosting employment. HUD also provided targeted homeless and low-income groups with affordable housing options. By early 2010, HUD had assisted 357,808 low-income people and developed/rehabilitated 188,184 units. A further

\$1.5 billion was used to provide emergency and short-term accommodation for homeless families.⁷ In 2015, Obama signed new HUD guidelines that provided public data on patterns of integration (and segregation), areas that had high concentrations of racial and ethnic poverty, areas with severe housing needs, and disparities in access to opportunity.⁸ The total HUD budget was \$49.3 billion in 2016.

The Federal Government and Low-Income Housing Programs

Housing is a fixed cost that is often paid before food, clothing, and health care bills.⁹ For poor families, the precious little money that remains after rent or mortgage payments is used to buy food and other necessities for the rest of the month. The important impact of housing costs on family finances has fueled the federal government's involvement with low- and non-low-income housing programs. In response to the problems of finding affordable housing and homeowner mortgage struggles, the Obama administration in 2009 allocated \$230 billion of federal funds to homeownership programs and \$60 billion for affordable rental housing programs.¹⁰ However, federal funding declined for the HOME and Community Block Grant programs in 2010.¹¹ Table 16.1 provides an overview of key HUD programs.



Watch this video for a good overview of HUD's significant accomplishments in creating opportunity.

www.youtube.com/watch?v=4aQPCxlrO3U

TABLE 16.1 Overview of HUD's Major Grant, Subsidy, and Loan Programs

- **Affordable Housing Programs (HOME Investment Partnerships—see below)** was funded at \$950 million in 2015. From 1994 to 2015, more than 600 communities have completed more than 1.15 million affordable housing units.¹² In addition, low-income tenants have received direct rental assistance. HOME Investment Partnership Grants provide assistance to renters, existing home owners, and first-time homebuyers; build state and local capacity to carry out affordable housing programs; and expand the capacity of nonprofit community housing organizations to develop and manage housing. The Housing Opportunities for Persons with AIDS provides affordable housing and related assistance to persons with HIV/AIDS. The Homeless Programs consist primarily of grants to public and private organizations and agencies to establish comprehensive systems for meeting the needs of homeless people.
- **Affordable Housing Trust Fund:** is a trust to support local governments in creating, renovating and maintaining affordable housing options for low-income homeowners.¹³
- **Brownfields Redevelopment** provides competitive economic development grants for qualified Brownfields (i.e., the cleanup and economic redevelopment of contaminated sites) projects.
- **Community Development Block Grants (CDBG)** was funded at about \$3 billion in 2015. CDBGs are provided to units of local government and states for the funding of local community development programs that address housing and economic development needs, primarily for low- and moderate-income persons.
- **Demolition and Revitalization of Severely Distressed Public Housing (HOPE IV)** makes awards to public housing authorities on a competitive basis to demolish obsolete or failed developments or to revitalize, where appropriate, sites upon which these developments exist.
- **Empowerment Zones/Enterprise Communities (EZ/EC)** is designed to create self-sustaining, long-term development in distressed urban and rural areas. The program uses a combination of federal tax incentives and flexible grant funds to reinvigorate declining communities.
- **Fair Housing Assistance Program (FHAP)** provides grants to state and local agencies that administer fair housing laws that are equivalent to the Federal Fair Housing Act.
- **Fair Housing Initiatives Program (FHIP)** provides funds competitively to private and public entities to carry out local, regional, and national programs that assist in eliminating discriminatory housing practices and educate the public and housing providers on their fair housing rights and responsibilities.
- **Government National Mortgage Association (Ginnie Mae).** Through its mortgage-backed securities program, Ginnie Mae facilitates the financing of residential mortgage loans by guaranteeing the timely payment of principal and interest to investors of privately issued securities backed by pools of mortgages insured or guaranteed by FHA, the Department of Veterans Affairs, and the Rural Housing Service. The Ginnie Mae guarantee gives lenders access to the capital market to originate new loans.
- **Homeless Assistance Grants** provide funding to break the cycle of homelessness and to move homeless persons and families into permanent housing. This is done by providing rental assistance, emergency shelter, transitional and permanent housing, and supportive services to homeless persons and families.
- **Housing Opportunities for Persons with AIDS (HOPWA)** is designed to provide states and localities with resources and incentives to devise long-term comprehensive strategies for meeting the housing needs of persons living with HIV/AIDS and their families.
- **Indian Community Development Block Grant** assists Native Alaskans and Indians in building or purchasing homes on Trust Land, rehabilitating houses and improving community infrastructure and job opportunities.¹⁴
- **Indian Housing Block Grant** provides Native Americans and Indians with assistance in building, purchasing, or rehabilitating their homes.¹⁵
- **Making Home Affordable Program:** Homeowners can change their loans to lower-cost options and or take fixed-rate mortgages to lock in interest rates. The federal government provided funds as incentives for mortgage brokers and homeowners to participate.¹⁶ The program aims for mortgage repayments to remain at or below 31 percent of homeowners' income, thus alleviating the cost burden for homeowners.¹⁷

TABLE 16.1 (Continued)

- **Mutual Mortgage Insurance (MMI) program:** This program aims to assist low- and medium-income people in entering the housing market through insuring mortgages for lenders. If homeowners default on these government-insured loans, the Federal Housing Administration provides the mortgage payment owed.¹⁸
- **Neighborhood Stabilization Program:** Funds are used to rehabilitate and sell abandoned and foreclosed houses.¹⁹
- **Office of Lead Hazard Control** is authorized to make grants to states, localities, and Native American tribes to conduct lead-based paint hazard reduction and abatement activities in private low-income housing.
- **Public and Indian Housing (PIH) Grants and Loans** Public Housing Operating Subsidies are financial assistance programs that provide for project operations to assist approximately 1 million units. Public Housing was established by the U.S. Housing Act of 1937 and is restricted to households whose incomes are too low to find suitable private housing.²⁰ The income of most families in public housing is less than 25 percent of the area median income and about one-half rely primarily on public assistance (TANF, SSI, and General Assistance) for their income; the other half rely on earned incomes, pensions, or Social Security. Residents pay 30 percent of their monthly adjusted income on rent. Public housing is concentrated in high-poverty areas.
- **Section 202/811 Capital Grants** program is designed to provide funds for the construction and long-term support of housing for the elderly and persons with disabilities. Advances are interest-free and do not have to be repaid, provided the housing remains available for low-income persons for at least 40 years.
- **The Housing Choice Voucher Program** (Section 8 Rental Assistance) is HUD's largest program. The Section 8 program is based on a voucher system that allows low-income tenants to occupy existing and privately owned housing stock. The voucher covers the difference between a fixed percentage of a tenant's income (30 percent) and the fair market rent of a housing unit. The HUD subsidy goes to the local Public Housing Authority, which then pays the landlord, provided the unit meets quality standards. Contract terms for subsidies last for from 5 to 15 years. About half of Section 8 is project-based, meaning that tenants have to live in specific apartments. The other half is tenant-based, allowing tenants to take their subsidies and move. Section 8 also provides funds for new construction and for substantial and moderate rehabilitation of existing units. Section 8 is not an entitlement program, and only a portion of eligible applicants are served. In some places, many applicants are on long waiting lists. In 2015, Congress provided roughly \$19 billion for this program.²¹
- **The Office of Rural Housing and Economic Development** (FmHA Section 502) was established to ensure that HUD has a comprehensive approach to rural housing and rural economic development issues. The FmHA serves rural communities through four major programs: (1) housing loan program, (2) utilities program, (3) business program, and (4) community development program. Section 502 makes low-interest loans available for rural home purchases. There are approximately 1,900 offices nationwide.

Sources: Based on various documents downloaded from U.S. Department of Housing and Urban Development, <http://www.hud.gov> and the Catalog of Federal Domestic Assistance, <http://www.cfda.gov/>.

Apart from HUD-administered programs, there is also the Low Income Housing Tax Credits (LIHTC). Created by the Tax Reform Act of 1986, the LIHTC was as a way for the federal government to encourage the development of affordable housing without having to allocate direct federal expenditures. Apartments supported by the LIHTC cannot be rented to anyone whose income exceeds 50 percent of the area median income and, the maximum rent charge is set at 30 percent of income.²² Between 1987 and 2013, an average of 40,502 projects and nearly 2.6 million units

were placed into service each year.²³ These housing units provided affordable housing options for working families, seniors, homeless people, and those with special needs.²⁴ This has been accomplished by providing investors in eligible affordable housing developments with a dollar-for-dollar reduction in their federal tax liability. The LIHTC now accounts for most new affordable apartment production.²⁵

Despite its importance, governmental assistance for housing was never provided as an entitlement to all households that qualify for aid. Unlike

income maintenance programs such as SSI and the former AFDC, housing programs are not automatically provided to all eligible applicants. Congress appropriates funds yearly for various new commitments, which can extend to 30 years.²⁶ Because funding levels are usually low, only a portion of eligible applicants actually receive assistance since demand for affordable housing significantly outweighs supply²⁷. Despite the approximately 10 million extremely low-income renter households that required housing in 2009, only 3.6 million affordable housing units were available to meet the demand.²⁸ Moreover, more than 10 million households earning less than \$15,000 a year pay more than 50 percent of their income on housing.²⁹

Relatively low levels of funding have been earmarked for housing programs. This low funding level has also resulted in long waiting lists for public housing and Section 8 vouchers.³⁰ NLIHC data shows that waiting lists for public housing and rental assistance vouchers far surpass anticipated availability of affordable units³¹ (see Figure 16.2). The cumulative shortfall in capital improvement funding reached approximately \$26 billion in 2012, and underfunding has resulted in the demolition and dispossession of over 150,000 units in the last 15 years due to deterioration and insufficient maintenance.^{32,33} In 2014, more than 4.8 million households have received low-income housing assistance; well over half of these individuals pay over 50 percent of their income in rent.³⁴

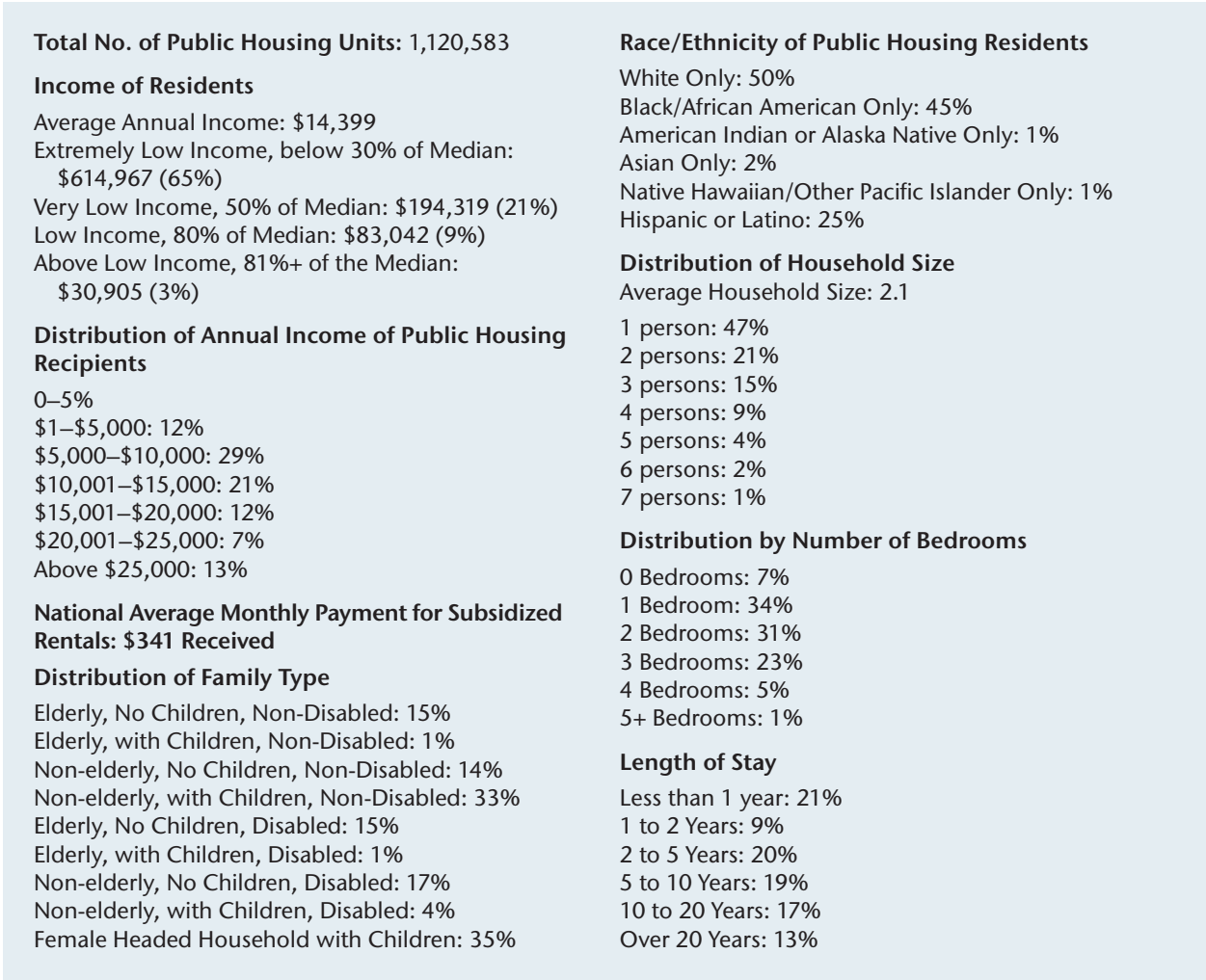


FIGURE 16.2 Characteristics of Residents in Public Housing, 2015
Source: HUD, Resident Characteristics Report as of December 31, 2015, U.S. Department of Housing and Urban Development..



For many families, the cost of housing represents the single largest expenditure in the household budget.

Source: Andy Dean Photography/Shutterstock

Issues in Housing Policy

The equity they have in homeownership is the cornerstone of wealth for most U.S. families. However, the housing bust and subprime mortgage crisis of 2007 to 2008 had widespread consequences for homeowners.

Since 2006, over \$7 trillion in home equity (the difference between the mortgage balance and the home's market value) has been lost, and home prices on average have declined by 33 percent.³⁵ As of 2014, the average homeowners' equity—the mortgage balance as a proportion of the home's market value—was about 60 percent.³⁶ In January 2015, the Federal Reserve Board reported that lower income households whose income is under \$40,000 are more likely to be “underwater” on their mortgage (i.e., they owe more than their house is worth). This equates to more than one in five mortgaged homes and increases the likelihood of foreclosure.³⁷ For the first time since the Federal Reserve began record keeping in 1945, home equity represents the smallest share of household net wealth.³⁸

Trends in U.S. Housing

The following are some highlights of the often contradictory U.S. housing trends:

- The overall U.S. homeownership rate in early 2015 was 63.4 percent, which is the lowest rate since 1967.⁴³

Spotlight 16.1

THE DEPARTMENT OF HOUSING AND URBAN DEVELOPMENT

Established in 1965, the Department of Housing and Urban Development (HUD) offers various programs to increase homeownership, support community development, and increase access to affordable housing free from discrimination. To learn more about HUD and its many programs, visit its website at www.hud.gov.

The United States spends less on housing assistance than any other Western industrial nation. Not only does the United States fail to adequately assist poor people with housing but also most housing subsidies benefit the non-poor. For example, individuals can deduct interest on up to \$1 million in mortgage indebtedness, plus interest on another \$100,000 in home equity loans. The home mortgage deduction is the third-largest single

“tax expenditure” behind the deductions companies take for contributions to pension plans and health care premiums.³⁹ The richer the homeowner, the better the deal. More than 80 percent of the major tax incentives for housing go to the top 20 percent of American earners, while less than 5 percent go to the bottom 60 percent.⁴⁰ In 2015, the mortgage interest deduction cost the federal government \$75 billion.⁴¹

Nearly half of all families with mortgages receive no housing tax benefit at all. Since the mortgage cutoff of \$1 million is much higher than the cost of a basic home, the bulk of the subsidies end up encouraging families who would have bought a home anyway to buy a larger house and/or to borrow more against it.⁴²

- Home sales and the value of residential construction reached new highs in 2003⁴⁴ but have declined significantly since 2007.⁴⁵ In 2007, the median sales price for a single-family existing home was \$236,393; by 2015, that had decreased to \$221,200.⁴⁶ When broken down, the differences in homeownership between races is stark and was exacerbated by the 2007 to 2008 recession. In late 2015, the non-Hispanic white homeownership rate was 71.9 percent compared to 42.4 percent for black and Hispanic households.⁴⁷ Despite increases in homeownership rates for minorities during the housing boom, the housing bust has resulted in black homeownership rates, decreasing by 4.3 percent between 2004 and 2011, which is nearly double the decline in white homeownership rates. The homeownership rate gap between whites and blacks is now higher than in 1994.⁴⁸
- Renters represent a disproportionate share of the severely cost-burdened. In 2008, 8.7 million renter households spent 50 percent or more of their income on rent, an increase from 8.3 million in 2007 and 6.2 million in 2000.⁴⁹ By 2010, 10.7 million renters were severely cost-burdened.⁵⁰
- Houses are larger than in the past. The size of newly constructed single-family homes rose from a median of 1,595 square feet in 1980 to 2,267 square feet in 2011, nearly a 30 percent increase.⁵¹

Problems in Homeownership

Beginning in the 1970s, many people were forced to spend a higher percentage of their income on housing than they could reasonably afford. Some of these people became so financially overextended that they were vulnerable to mortgage default or eviction—or they lacked the necessary cash to purchase other necessities.

Between 1967 and 2004, the median income for first-time home buyers rose from \$28,011 to \$54,500, while the median home price went from \$56,466 to \$168,500. In 1967, the average annual mortgage payment was \$3,400; by 2004 it had risen to roughly \$12,000.⁵² The price increase of single-family homes is even sharper when not adjusted for inflation. In 1970, the average new single-family home was \$26,600; in 2007, it cost \$218,900.⁵³ In early 2012, the average price of a new home

was \$274,200 and the average price of an existing home was \$204,600.⁵⁴ The median price of housing also varies widely by city and region. For example, the 2015 median home price in Los Angeles County was \$525,000; and in San Francisco it was \$1.16 million. In Los Angeles County, the \$525,000 was still below the 2006 median price of \$565,000. Housing markets like Tallahassee, Florida, saw a 20 percent drop in housing prices from 2006 to 2014. This decline in home value has had a significant impact on homeowners, particularly minority groups. Hispanic homeowners in particular have experienced the largest decline in home equity; a median decrease of 51 percent between 2005 and 2009.⁵⁵

As a result of these and other factors, more homeowners are struggling with the ongoing cost of homeownership, especially if they have a mortgage and/or are in a low-income bracket. In fact, servicing a mortgage is directly related to housing affordability. For example, 94 percent of homeowners in the lowest income bracket with mortgages experienced a severe cost burden. This contrasts with only 44.2 percent of homeowners in the same income bracket, but without a mortgage. Even when examining the expenditure of low- to middle-income homeowners (those earning between \$30,000 and \$44,999), there is a significant difference in housing affordability, with 25 percent of low- to middle-income mortgaged homeowners experiencing a severe cost burden compared to less than 1 percent of low- to middle-income homeowners without a mortgage.⁵⁶ Overall, of the 20.2 million households that experienced a severe cost burden in 2010, 9.5 million were homeowners.⁵⁷

Homeownership and the Subprime Mortgage Crisis

The subprime mortgage industry imploded in 2007, and hundreds of subprime mortgage lenders shut down or went bankrupt. Although there are still some subprime mortgage lenders, most have tightened their lending criteria and borrowers are required to have a higher level of qualification compared to pre-2007 levels.

Why did millions of families sign up for mortgage loans that were likely to end in failure and foreclosure? Homeownership is an important rite of passage in American society. Most Americans view it as a ticket to the middle class and the best choice for everyone, everywhere, and at all times. It

is assumed that the more the people who own their homes, the more robust the economy, the stronger the community, and the greater the collective and individual benefits.

For the most part, this is an accurate picture. Homeowners gain a foothold in the housing market that, until 2007, had the appearance of an almost infinite price ceiling. Others enjoy important tax benefits. Owning a home is often cheaper than renting. Most important, homeownership builds equity and assets for future generations by promoting forced savings. According to Winton Picoff and Sheila Crowley, the median wealth of low-income homeowners is 12 times higher than that of renters with similar incomes.⁵⁸ Less tangibly, homeownership is a status symbol.

Homeownership may also have positive effects on family life. Ohio University's Robert Dietz found that homeownership contributes to household stability, social involvement, environmental awareness, local political participation and activism, good health, and a lower crime rate. According to Dietz, homeowners are better citizens, are healthier both physically and mentally, and have children who achieve more and are better behaved than those of renters.⁵⁹

Johns Hopkins University researchers Joe Harkness and Sandra Newman looked at whether homeownership benefits kids even in distressed neighborhoods. Their study concluded that children of low-income renters are helped more by programs that assist their families to become homeowners in their current neighborhoods, rather than helping families relocate to better neighborhoods and remain renters.⁶⁰ The study was unable to determine whether owning a home leads to positive behaviors or whether homeowners are already predisposed to those behaviors.

Faith in the benefits of homeownership—along with low interest rates and a plethora of governmental incentives—produced a surge in low-income homeowners. By 2003, 48 percent of African American households were homeowners, up from 34.5 percent in 1950.⁶¹ This hyper housing market came with a stiff price. From 1997 to 2006, the typical American home price increased by 124 percent. From 1981 to 2001, the national median home price ranged from 2.9 to 3.1 times the median household income; by 2006, it reached an unsustainable 4.6 times.⁶² Although homeownership has undeniable benefits, many low-income families found that the burdens outweighed the positives.

The recession and the subprime mortgage crisis resulted in mortgage delinquency and foreclosure rates reaching record highs by 2009.⁶³ Low-income, minority communities had the highest foreclosure rate, estimated at over 8 percent between 2007 and 2008.⁶⁴ By late 2009, almost 15 percent of mortgage owners were either in foreclosure or had defaulted by at least one payment.⁶⁵ Foreclosure rates for subprime mortgages increased from almost 23 percent of all foreclosure loans in 2004 to a peak of almost 44 percent in 2007.⁶⁶ By 2012, delinquency and foreclosure rates had slightly declined due to a somewhat stronger economy.⁶⁷



Watch this video for an examination of the human costs of foreclosures.

www.youtube.com/watch?v=S3rzN42HE00

The Downside of Homeownership

Low-income families are vulnerable to wide-ranging risks when purchasing a home, including:

- Inexpensive housing is often rundown and expensive to maintain.
- Neighborhoods with affordable housing stock are often economically distressed with few jobs, few services, high crime rates, and poor schools. Houses appreciate slowly, if at all.
- Subprime mortgages targeted at low-income homebuyers have high interest rates and fees.
- Low-income home buyers often work in unstable and recession-sensitive jobs. Even short-term employment interruptions can mean losing a home.

Taken together, these factors make homeownership a far riskier proposition for low-income families than for middle- and upper-income households. Too often, low-income families pay inflated prices for homes beset with major structural problems masked by cosmetic repairs.

Apart from maintenance and repair costs, homeownership expenses also include property taxes and insurance. Average home insurance premiums rose nationally by 5.6 percent in 2012, following a 7.7 percent increase in 2011.⁶⁸ Despite the drop in home values—which should translate into lower homeowners' insurance rates—rates went up from 7 to 17.6 percent in 23 states from 2009 to 2010, stayed the same in nine states, and dropped in only seven states.⁶⁹ Low-income

homeowners in distressed neighborhoods are especially hard hit by high insurance costs. According to one study, 92 percent of large insurance companies run credit checks on potential customers, which translates into insurance scores that are used to determine whether the company will insure an applicant, and if so, what they will cover and how much they will charge. Those with poor or no credit are denied coverage, and those with limited credit pay high premiums.⁷⁰ Credit scoring may explain why homeowners in minority neighborhoods are less likely to have home insurance, more likely to have policies that provide less coverage, and pay more for similar policies than their white counterparts.

With few cash reserves, low-income families are a heartbeat away from financial disaster if they lose their jobs, if their property taxes or insurance rates rise, or if expensive repairs are needed. These families often have no cushion for emergencies. The National Housing Conference reported that about 50 percent of working families in 2005 with critical housing needs were homeowners.⁷¹

A 2005 study found that from 1976 to 1993, 36 percent of low-income homeowners gave up or lost their homes within two years; 53 percent exited within five years. Very few low-income families bought another house after returning to renting.⁷² Another study reached similar conclusions. Following a national sample of African Americans from youth in 1979 to middle age in 2000, the researchers found that 63 percent owned a home at some point, but only 34 percent still did in 2000.⁷³

Homeownership can also limit financial opportunities. A 1999 study found that states with the highest homeownership rates also had the highest unemployment rates. The report concluded that homeownership may constrain labor mobility because the high costs of selling a house make unemployed homeowners reluctant to relocate to find work.⁷⁴

Special tax breaks are touted as a key selling point in deciding to become a homeowner. But, if mortgage interest and other expenses come to less than the standard deduction (\$12,600 for joint filers in 2015), there is no tax advantage to homeownership. This is why only 34 percent of taxpayers itemize their mortgage interest, local property taxes, and other deductions. The mortgage deduction primarily benefits those in high-income brackets who need to shelter their income and has little value to low-income homeowners.

It should be noted that not all negative equity is bad. One of the newer variations of refinancing endorsed by HUD is a reverse mortgage for senior citizens, called Home Equity Conversion Mortgage (HECM). This program unlocks the equity the elderly have built up in their property. Homeowners aged 62 or older can borrow against the value of their property. They receive payments from lenders monthly, in a lump sum or as a line of credit. The size of the reverse mortgage is determined by the borrower's age, the interest rate, and the value of the home. The older the homeowner, the larger the percentage of a home's value they can borrow. The amount owed increases over time, but no payment is due until the end of the loan term. When the loan term expires, the total loan amount plus interest is due in full. This lump sum payment is usually paid by selling the property. No repayments are required while a borrower lives in the home, and the monthly income is tax free. The downside is the reduction of intergenerational wealth that occurs when the mortgagee dies and the loan has to be repaid.

Problems in Finding Affordable Rental Housing

Rents follow similar trajectories as other market commodities. As more people entered the housing market through subprime loans and easy credit, the demand for rentals softened and rental prices dropped in some areas.⁷⁵ As credit tightened in the aftermath of the subprime mortgage crisis, fewer people were able to become homeowners and foreclosures led to former homeowners becoming renters. This increased demand and resulted in rental housing shortages and higher prices. From September 2014 to September 2015, average rents increased nationwide by 4.2 percent for a one-bedroom apartment and 3.0 percent for a two-bedroom apartment. This was several times above the less than 1 percent inflation rate during that period. By 2015, average rental rates in some of the most sought-after cities skyrocketed: San Francisco, \$4,950; New York, New York, \$3,500; Washington, DC, \$3,000; Miami, \$2,680; Boston, \$2,550; Los Angeles, \$2,200; Seattle, \$2,190; Portland, Oregon, \$1,950; and San Diego, \$1,900.⁷⁶

If finding affordable housing is difficult for poor and moderate-income homeowners, it has reached crisis proportions for extremely low-income (ELI) renters. ELI renters are those who

earn 30 percent or less of the Area Median Income (AMI). Very-low-income (VLI) renters are those who earn between 30 to 50 percent of the AMI.⁷⁷

The standard benchmark for “affordability” is that households should pay no more than 30 percent of their after-tax income on housing. Those paying between 30 and 50 percent have moderate cost burdens; households paying more than 50 percent have severe cost burdens. The number of households with low incomes has increased at a time when affordable rental housing is decreasing. From 2009 to 2013, the number of ELI renter households rose from 9.6 million to 10.3 million, making up 24 percent of all renter households. There was a 7.1 million shortage of ELI affordable rental units in 2013, resulting in only three available units for every 10 ELI renter households. At least 60 percent of ELI renters paid more than half of their income on rent and utilities in every state.⁷⁸

Between 2001 and 2010, the number of low-income households earning \$15,000 or less grew by 2.2 million, while the number of affordable rental houses available to this group declined by 470,000. This equates to a widening gap between supply and demand. In 2001, 8.1 million low-income renters competed for 5.7 million affordable houses, leaving a gap of 2.4 million; by 2015, the number of low-income households was nearly 4.8 million and the gap between affordable housing had more than doubled to 9.8 million.⁷⁹

When poor households live in unaffordable housing, they often compensate by spending less on other necessities such as heat, food, clothing, and health care. Some of these families find themselves in a “heat or eat” situation. Low-income, severely cost-burdened families with children spend half as much on clothes, two-fifths as much on health care, and three-fifths as much on food as families living in affordable housing.⁸⁰

Rising rental costs and the failure of many ELI and VLI renters to receive rental assistance also affects the amount of rent these households can afford. In 2014, only 40 percent of ELI renter households received rental assistance.⁸¹ Affordability pressures are likely to increase since wages in many low-income jobs are insufficient to cover even a modest one-bedroom apartment in many parts of the country. One bright spot is the creation of the National Housing Trust Fund (NHTF) that was passed in 2008 but not funded until 2016. Through NHTF block grants to states, the federal government will invest in

developing affordable rental housing targeted at ELI households.

Several other factors have converged to deplete low-income housing stock, including the commercial renovation of central-city downtown areas. Developments consisting of new office buildings, large apartment complexes, shopping areas, and parking lots often replace low-income housing bordering on downtown areas. Traditionally affordable (and often rundown) apartment buildings, cheap single room occupancy (SRO) hotels, rooming houses, and boardinghouses are razed as new office buildings and shopping malls are erected. Displaced longtime residents are forced to find housing in more expensive neighborhoods, to double up with family or friends, or to become homeless.

The loss of SROs has exacerbated the homeless problem. SROs were home to many poor people (including those suffering from substance abuse and mental illnesses) who were not living in public or subsidized housing. But between 1970 and the mid-1980s, an estimated 1 million SRO units were demolished. San Francisco lost 43 percent, Los Angeles lost more than 50 percent, New York lost 87 percent, and Chicago experienced the total elimination of its SRO housing units and hotels. These demolitions left many poor persons homeless.⁸²

Gentrification

According to George Sternlieb and Jones Hughes, “a new town may be evolving in-town.”⁸³ This new town, or “gentrified” neighborhood, is a major component of the urban renaissance taking place in many U.S. cities. Attracted by old houses or lofts amenable to restoration; good transportation facilities; and close proximity to employment and artistic, cultural, and social opportunities, young professional and white-collar workers have begun to resettle the poor, aging, and usually heavily minority sections of central cities in the process called **gentrification**. As part of the process, upscale condos are built, houses are rehabbed, dusty lofts are modernized and sold for small fortunes, and trendy candlelit restaurants and boutiques replace mini-marts and used furniture stores. Critics of gentrification argue that a valuable piece of city real estate is being cleansed of its working-class residents.

Although the renovation of central-city areas, such as New York’s SoHo district, often makes a neighborhood more attractive (and potentially a

tourist attraction), the effect on the poor population can be devastating. As more homes are renovated, the price of surrounding homes increases. Landlords find it an opportune time to sell their former rental properties for a high price or renovate rundown apartments to claim high rents. Even though low-income homeowners will get a high price for their homes, they may not find suitable or affordable places to relocate. As neighborhoods become more affluent, property taxes rise, thus creating a burden on existing low-income homeowners. Previously affordable rental housing undergoes rent increases as neighborhoods become more desirable, thereby driving out older and poorer residents. Moreover, as baby boomers age, the reason for living in the suburbs—good schools and safe neighborhoods for their children—is eclipsed by the stress of long commutes and highway gridlock. Many couples without children chose to move closer to work and cultural amenities.

The conversion of apartment buildings into condominiums represents a threat to low-income renters. Due to tax breaks and income shelters, previously affordable rental housing is rapidly being converted into condominiums. Whether initiated by tenants or by developers, condominium conversion represents serious depletion of good quality rental stock. Because units in these conversions may cost upward of \$100,000 or more, low-income tenants can rarely afford the benefits of condominium living, and although long-term renters are sometimes offered a separation fee when a building undergoes conversion, this may barely cover the costs of moving, much less make up the difference between the current rent and a higher alternative rent.

Overcrowded and Deficient Housing

Problems of overcrowding and structural inadequacy affect a significant number of U.S. households. HUD defines a housing unit as overcrowded if there is an average of more than one person per room. According to this definition, almost 3 million households live in overcrowded conditions, sometimes as a result of families doubling up to avoid becoming homeless.⁸⁴ In 2009, HUD reported that almost 9 million households were living in inadequate housing units; 5.2 million of these were homeowners, and 3.7 million were renters. HUD classifies inadequate housing as seriously inadequate if it has severe physical deficiencies like no hot water, electricity, or a toilet, or it lacks a bathtub or shower.⁸⁵

Lead paint is one of the principal problems in units needing rehabilitation. The Centers for Disease Control and Prevention notes that lead poisoning is one of the most common and devastating environmental diseases that affects young children, causing developmental and behavior problems. Children are exposed to lead poisoning by living in older homes with peeling, chipping, and flaking paint. Because low-income families often occupy poorly maintained older homes, their children are four times more likely to have lead poisoning than children in high-income families.⁸⁶ According to the EPA, lead poisoning is the single greatest environmental health threat to children age six and younger.⁸⁷

Other Factors Affecting Housing

Another problem affecting affordable housing is property taxes—frequently the heart of state and local revenues. The rising costs of providing government services has resulted in increases in property taxes, which landlords generally pass along to renters through higher rents. In response, many states have tried to reduce property tax burdens for low-income households. The most common form of property tax relief occurs through **circuit breaker programs**. A typical circuit breaker is activated when taxes exceed a specified proportion of a homeowner's income, and in some states, low-income households are sent a yearly benefit check that covers all or part of their property taxes. Circuit breaker programs for low-income renters operate in a similar fashion. Typically, a portion of the rent paid by a low-income household is considered to represent the property tax passed on by the landlord and is thus refunded by the state government. Although circuit breakers provide relief, they are often restricted to the elderly or disabled.⁸⁸

Housing costs are also aggravated by high utility rates. In Ohio, the average family in 2015 paid about \$2,000 in utility bills, although that number reached more than \$3,500 in parts of the Northeast and Midwest. Low-income consumers spend anywhere from 13 to 44 percent of their total household income on utilities, whereas average-income Americans spend only 4 percent.⁸⁹ Increases in home energy bills disproportionately affect the poor, and in some cases, lead to utility shutoff.⁹⁰ In 2014, New York State's electric and gas utilities issued 7.2 million shutoff notices and shut off service to 268,000 customers. Also in 2014, Con

Edison, the largest utility company in New York, sent out 2.8 million shutoff notices resulting in 82,000 customers losing their utilities.⁹¹

To mitigate the effects of the federal deregulation of oil prices and the large oil price increases of the 1970s, Congress passed the Low-Income Home Energy Assistance Program (LIHEAP) in 1981. This legislation permits states to offer three types of assistance to low-income households: (1) funds to help eligible households pay their home heating or cooling bills, (2) allotments for low-income weatherization, and (3) assistance to households during energy-related emergencies. States are required to target LIHEAP benefits to households with the lowest incomes and the highest energy costs relative to income and family size. In 2016, total LIHEAP funding was \$3 billion, and in 2010 the program served roughly 8.7 million households who received an average grant of \$456, enough to cover 47 percent of the cost of home heating. It is estimated that less than 20 percent of eligible families receive LIHEAP assistance.⁹² Housing discrimination—another barrier facing the poor—can take the form of racial discrimination or a bias against families with children. Although illegal, racial discrimination in housing is still prevalent. In addition, many landlords and real estate agents refuse to rent to families with children. Even when these families find housing, they are often required to pay higher rents, provide large security deposits, or meet qualifications not required of renters without children.⁹³ Discrimination against families with children continues despite its ban by Congress in 1988.⁹⁴

Homelessness

Homelessness represents both a simple and a complex problem. For one, the homeless are not a homogeneous group. Homelessness for some individuals is a lifestyle choice, the freedom to roam without being tied down to one place. For others, particularly those with mental illness or chronic alcoholism, homelessness reflects the failure of an overburdened public mental health system. For others, homelessness is rooted in cuts in federally subsidized housing programs and in the cost/income squeeze of the housing market. Finally, large numbers of people are homeless because public assistance benefits have failed to keep pace with the cost of living. Despite different causes, homelessness is tied to poverty.

The actual number of homeless people in the United States is difficult to ascertain. First, the definition of homelessness varies from study to study, and different methods for counting the homeless yield different results. Second, many of the homeless are “hidden” because they live in campgrounds, automobiles, boxcars, caves, tents, boxes, and other makeshift housing. Or, the homeless may temporarily live with family members or friends. Most studies count the homeless by tracking people who use the services of soup kitchens and shelters.⁹⁵

Third, the homeless population is often undercounted in federal surveys for fiscal reasons: If the true extent of homelessness were acknowledged, local, state, and federal authorities would have to target more services and funds for this population.⁹⁶ The U.S. Department of Housing and Urban Development reported that in January 2015, 564,708 people were homeless on any given night. Sixty-nine percent were in residential programs for homeless people, while 31 percent were found in unsheltered locations.⁹⁷ The 2010 Annual Homeless Assessment Report (AHAR) reported that between October 2009 and September 2010, there were 1.59 million people who used a shelter or emergency/transitional housing.⁹⁸ It is difficult to gauge the true extent of homelessness on the streets, given that most estimates are based on point-in-time counts (the number of homeless people counted in one night) or the annual number of homeless people who use shelters and other crisis accommodation.

Characteristics of the Homeless Population

Homelessness has been a long-standing problem in most urban areas, previously propelled onto center stage by media images of bag ladies, the mentally ill, chronic alcoholics, street people, and uprooted families. These images make for interesting copy, but popular stereotypes obscure both the extent and true nature of the problem. The homeless are generally the poorest of the poor. They include single-parent families and, increasingly, two-parent families. They are often people who work but earn too little to afford housing. They are women and children escaping from domestic violence. They are runaway youngsters or “throw-away kids.” They are the unemployed—some who are looking for work, and some who have never worked. The homeless include retired people on small fixed incomes, many

Spotlight 16.2

THE NATIONAL COALITION FOR THE HOMELESS

The National Coalition for the Homeless (NCH) was established to end homelessness in America. To that end, this organization engages in public education, policy advocacy, and grassroots organizing. The

work of NCH is focused in the following four areas: housing justice, economic justice, health care justice, and civil rights. To learn more about NCH, go to its website at www.nationalhomeless.org.

of whom have lost their cheap SRO hotel rooms to gentrification; school dropouts; drug addicts; disabled and the mentally ill lost in a maze of outpatient services; those who have worn out their welcome with family or friends; young mothers on welfare on long waiting lists for public housing; and families who have lost their overcrowded quarters.⁹⁹

The prevalence of homelessness in families increased during the recession of 2007 to 2008 and reflects the impact of the stagnant wages on those struggling to make ends meet. As the recession resulted in widespread job losses, family support networks struggled with their own problems of unemployment and income loss, and

were less able to provide for their relatives and friends. Overcrowding in houses increased as more families “doubled up” to keep costs low, leaving less room for others requiring transitional housing.¹⁰⁰

According to HUD, the homeless population has the following characteristics:

Race. One in 138 people identifying as a minority entered a homeless shelter in 2014. When looking at African Americans specifically, 1 in 69 used a shelter.

Geographic spread. Although homelessness exists in all American communities, over half of all homeless people are in California, Florida, Nevada, Texas, Georgia, and Washington.

Rural homelessness. The rural homeless tend to live in cars, double up, or live in grossly substandard housing. Most are married, white, working females, often with families.

Frequency. Thirty-seven percent of the homeless were part of family groups; the other 67 percent were individuals. Emergency shelters or transitional housing programs were used by 1.6 million people in 2009. For the 2008 to 2009 school year, public schools reported over 956,000 homeless students, a 20 percent increase from 2007 to 2008.

Families. Families experiencing homelessness are usually headed by a single woman in her late 20s with two children, one or both of whom are under six years of age. Families experiencing homelessness have very low incomes, less access to housing subsidies than housed low-income families, and have weaker social networks unable to provide help.



Homelessness in America is both a simple and complex problem.

Source: Urbanbuzz/Shutterstock

Among homeless mothers with children, more than 80 percent had previously experienced domestic violence, including financial abuse that left them with poor credit and limited resources. Emergency domestic violence shelters typically limit stays to 90 days or less to ensure sufficient beds for those in imminent danger. Since 2007, the number of homeless people in families declined by 8 percent (or 19,284 people), while the number of homeless family households dropped by 14 percent (or 11,249).

Children in homeless families have a high rate of chronic and long-term health problems, and school-age homeless children are more likely to have emotional problems and manifest aggressive behavior. They also exhibit poorer academic performance. The rate of foster care placement is highest for the children of women with at least one episode of homelessness.

Unaccompanied youth. Young people often leave home due to severe family conflict, which may include physical and/or sexual abuse. Some youth become homeless when they leave foster care or residential facilities (e.g., they are runaways, aged out, or discharged). According to HUD, unaccompanied youth at minimum make up 2.2 percent of the sheltered homeless population (about 22,700 people). Other sources suggest that about 110,000 youth live on the streets (or in other public places), in cars, and in abandoned buildings. About 53,000 unaccompanied youth are supported through school-based programs. Some teenage boys are separated from their families due to shelter policies that force older adolescent males to be housed in adult shelters.

Individual adults. Since 2013, homeless adults over the age of 24 declined by 2,338 people (or 3 percent). Sheltered adults age 24 or older declined by 766 people (or 1 percent), while those in unsheltered locations declined by 1,572 people (or 13 percent). Forty-three percent of sheltered single adults had a disability and 13 percent were veterans. This reflects the growing population of older (over age 50) homeless adults. Single individuals who experience long-term homelessness have high rates of mental illness and/or substance abuse disorders. Chronic homelessness is strongly correlated with alcohol abuse, schizophrenia, and other mental health problems.

Veterans. According to the Veteran's Administration (VA), the number of veterans experiencing homelessness has been rapidly declining due to an increased effort to serve this population. Of the estimated 49,933 homeless veterans on any given night in January 2014, fewer than 10 percent were women.¹⁰¹

Trends in Homelessness

As noted earlier, homelessness and poverty are inextricably linked. Because poor families have limited resources that only cover a portion of their basic needs, they are forced to make difficult choices among housing, food, clothing, or health care. Housing costs absorb the greatest percentage of a poor family's income, which can leave them only a paycheck away from living on the streets if faced with a crisis. Families who have moved from welfare to work under the 1996 welfare reform law are not doing as well as projected due to inadequate work supports and low wages. Moreover, the five-year TANF lifetime cap may lead to greater numbers of homeless people in coming years, especially in the wake of a recession marked by high unemployment.¹⁰² The impact of the 2007 to 2008 recession, particularly in terms of home foreclosures and the rising unaffordability of rents, has been linked to homelessness.¹⁰³

Lack of affordable health care can cause many struggling families to spiral into homelessness when a serious illness or injury causes extended absence from work, job loss, or depletion of savings to pay for health care. Mental illness and drug and alcohol addiction also play a role. Mentally ill homeless individuals often experience difficulty in accessing supportive housing along with treatment services. Those who are both poor and addicted also face a high risk of homelessness.¹⁰⁴

Domestic violence has been linked to homelessness. Specifically, many poor women are forced to choose between staying in an abusive relationship or becoming homeless. Several studies demonstrate the contribution of domestic violence to homelessness, particularly among families with children. A 2003 survey of 10 U.S. locations found that 25 out of 100 homeless women had experienced physical abuse in the last 12 months. A 2004 Los Angeles study found that 34 percent of homeless women in California had experienced domestic violence in the past 12 months, and of those, 58 percent had left their homes because of it. The stigma of

domestic violence can also provide a barrier to finding affordable housing. A 2005 New York study found that almost 28 percent of landlords would not rent to a victim of domestic violence. Overall, anywhere between 22 and 57 percent of homeless women cite domestic violence as the primary cause of their homelessness.¹⁰⁵ Being unable to find affordable housing and having a low income increases the likelihood that a woman will stay in violent relationships or return to their violent partners. A 2003 study in Fargo, North Dakota, found that 44 percent of homeless women had previously stayed in violent relationships because they had “nowhere else to go.”¹⁰⁶

Homeless youth (typically runaways) are a persistent problem. An Urban Institute report estimated that the number of runaway youth—many who were homeless—could be anywhere between 1.6 million to 2.8 million.¹⁰⁷ Many homeless youths run away because of physical or sexual abuse, strained relationships, addiction of a family member, and parental neglect. In one study, more than half of the youth in shelters reported their parents either told them to leave or knew they were leaving but did not care.¹⁰⁸ According to U.S. government agencies, 17 to 35 percent of homeless youth had reported being sexually abused and up to 60 percent had been physically abused prior to leaving home. In another study, 50 percent of homeless youth reported being physically abused and 39 percent reported being sexually abused at home.¹⁰⁹ A history of foster care has also been correlated with becoming homeless at an early age and remaining so for a long period of time. Some youth living in residential placements become homeless after discharge—they are too old for foster care and leave with no housing or income support. Emancipation from foster care can lead to homelessness, as youth in this group lack the fiscal means and social networks to support themselves.¹¹⁰ Many homeless adolescents find that survival on the streets requires exchanging sex for the necessities. In turn, they are at a greater risk for contracting AIDS/HIV. Studies suggest that the HIV prevalence rate for homeless youth may be between 3 and 30 times higher than for other adolescents.¹¹¹

Attempts to Address Homelessness

On July 22, 1987, former president Ronald Reagan signed the McKinney-Vento Homeless Assistance Act into law, which created more than 20 separate

programs to be administered by nine federal agencies. Some highlights of the Act include:

- Emergency Shelter Grants Program (ESGP). This program provides formula (block grant) funding for emergency shelter and essential services.
- Supportive Housing Demonstration Program (SHDP). This program funds a variety of grants for transitional and permanent housing, particularly for homeless families and those with special needs or handicaps.
- Section 8 Moderate Rehabilitation for Single-Room Occupancy Dwellings. This program provides funding to owners of SRO housing in the form of rental assistance payments on behalf of homeless individuals, in conjunction with the rehabilitation of the facility. The program provides permanent housing for previously homeless tenants.
- Shelter Plus Care. Congress added this program to HUD in 1990. Funds are awarded for rental assistance. Grantees must match the value of rental assistance with an equal value of supportive services. The target population is homeless persons living on the streets or in emergency shelters with severe mental illness, chronic substance abuse problems, or AIDS.
- Supplemental Assistance for Facilities to Assist the Homeless. This program funds projects that meet the immediate and long-term needs of the homeless, as well as projects already receiving funds under ESGP and SHDP. The flexible range of assistance permits program expansion, capital improvement, and startup of new, needed supportive services.
- Single Family Property Disposition Initiative. Created in 1983, this program provides the HUD secretary broad legislative authority to dispose of single-family properties. It was broadened in 1985 to allow HUD to sell or lease foreclosed single-family properties to nonprofit organizations, or to state or local governments to provide temporary shelter for homeless persons. In 1992, it was recognized as a McKinney initiative.¹¹² Given the recent run of foreclosures on residential and commercial properties, this program provides nonprofits with the opportunity to use the housing crisis to further expand low-income housing stock.

In 1990, Congress amended the McKinney-Vento Act to remove requirements that kept homeless children from attending school, including proof of

immunization, former school records, and proof of residency.¹¹³ In 2009, Congress passed the HEARTH Act that modified the McKinney-Vento Act and made several changes to HUD's homeless programs in the areas of outreach, rapid rehousing for families, permanent supportive housing for the chronic homeless, prevention services, changes in Emergency Shelter Grant funding (renamed Emergency Solutions Grant), short- and long-term rental assistance, and other changes.¹¹⁴

Homelessness cannot be eradicated without basic changes in federal housing programs, income supports, social services, health care, education, and employment policies. A durable solution to the homeless problem must involve the provision of permanent housing for those who are currently or potentially homeless. Federal programs and legislation should be coordinated and expanded to provide decent, affordable housing, coupled with needed services for all poor families. Finally, both the states and the federal government should intervene directly in the housing market by controlling rents, increasing the overall housing stock, limiting speculation, and providing income supports.

Housing Reform

The housing crisis faced by low-income people has led to numerous suggestions for housing reform. Some conservative critics argue that low-income housing assistance should be abolished, thereby allowing the law of supply and demand to regulate rents and, eventually, to drive down prices. Free market economic philosophy suggests that as rents increase, demand will slacken, and rents will eventually drop. Another argument is that government intervention in housing should occur only by stimulating production of rental housing through financial incentives such as tax breaks to builders, entrepreneurs, and investors. If rental housing is made more profitable, more units will be built, and the increase in the housing stock will lower prices. Some liberal critics contend that because housing is a necessity and the demand is relatively inelastic, marketplace laws should not be allowed to dominate. Despite its age, the 1995 National Low-Income Housing Coalition plank continues to remain relevant:

- Guarantee housing assistance to people who need it.

- End homelessness by linking housing with services to support recovery and self-sufficiency.
- Provide a permanent and adequate supply of affordable housing.
- Preserve and improve federally assisted affordable homes for people with low incomes.
- Provide the opportunity for resident control of housing.
- Preserve neighborhoods and end displacement (i.e., gentrification).
- End economic and racial segregation through affirmative housing programs and the enforcement of fair housing laws.
- Reform federal tax laws to give priority to aiding people with the greatest housing needs.
- Provide the financing needed to preserve, build, and rehabilitate housing.¹¹⁵

Much of the housing innovation in recent years has come from the nonprofit sector. Habitat for Humanity helped almost 7 million people since its founding in 1976. In 2015, the organization served nearly 1.8 million people through home construction, rehabilitation, repairs or increased access to improved shelters.¹¹⁶ Although nonprofit housing organizations are having an important impact on the country's housing problem, they have limitations. For example, many of the housing groups focus their attention on the "less needy"—those with incomes at 50 percent of the median. This occurs partly because no one group knows how to house large numbers of the poorest people, especially those on public assistance. If the poorest are grouped together too tightly, neighborhoods may collapse. If they are spread out in the suburbs, the new neighbors complain. Perhaps more importantly, the scope and pace of building or renovating under nonprofits is inadequate to meet the need.¹¹⁷ For example, the rapid rise in the price of land has discouraged Habitat for Humanity from building more new houses. From 2008 to 2014, the number of Habitat's newly constructed homes decreased by 31 percent to 3,323. On the other hand, the number of renovations doubled to 1,435 in that same period. Although renovations increase the quality of the housing stock, they do not necessarily increase the overall number of homes since many of the renovated houses are currently occupied. Moreover, at that rate, it would take hundreds of years to provide enough low-income houses to meet the need.¹¹⁸

Housing in an International Context

First-time U.S. visitors to European cities are often struck by several things. For one, European cars are smaller, which is understandable given that the average price of gasoline in the European Union's 27 (EU-27) is considerably higher (sometimes more than double) than in the United States. Visitors also notice that European city streets are narrower (having been built before automobiles) and often more congested. Public transportation is typically well-developed and convenient, especially when compared to post-car American cities like Phoenix, Houston, Dallas, and Denver. Housing is denser than in most new post-car U.S. cities. Visitors also notice that European homes and apartments are typically smaller than in the U.S. and often are not centrally air conditioned. Appliances, such as refrigerators and washing machines, are also smaller. If European homes have a clothes dryer (many do not), it is generally smaller than those found in typical American homes.¹¹⁹

Europe's limited use—at least when compared to the United States—of energy-intensive appliances is partly driven by the cost of electricity. In 2014, electricity costs 10.7 cents a kilowatt hour in the European Union compared to 10 cents in the United States.¹²⁰ Lifestyle choices combined with cheap energy partly explains why the United States consumed 17 metric tons of CO₂ emissions (carbon dioxide emissions from burning fossil fuels and manufacturing cement) per capita from 2011 to 2015 compared to 5.5 tons in Sweden, 7.1 in the United Kingdom, 8.9 in Germany, 5.2 in France, 5.8 in Spain, and 6.7 tons in Italy.¹²¹

Most European cities also lack the large rental-only apartment complexes found in the American South, Midwest, and West. Rental housing in the United States is typically in multifamily structures, often built exclusively for that purpose rather than for individual ownership. These complexes can be large, sometimes encompassing 500 or more rental apartments. This differs from Europe where apartments tend to be individually owned.

Comparison of U.S. and European Housing

As noted above, European housing tends to be smaller than in the United States (see Table 16.2). For instance, the floor area and room sizes in new

TABLE 16.2 Post-2003 New Construction Housing Sizes (in square feet)

United States	2,300
Australia	2,217
Denmark	1,475
France	1,216
Spain	1,044
Ireland	947
United Kingdom	818

Source: Average Home Sizes Around the World, Apartment Therapy. <http://www.apartmenttherapy.com/average-home-sizes-around-the-151738>.

UK homes are the smallest in Europe—the average room size is 170 square feet, compared to 289.5 square feet in France. Newly built British homes are less than half the size of homes in the United States and Australia.¹²²

In 2007, 27 percent of the EU-27 population lived in owner-occupied homes with an outstanding loan or mortgage; close to half (46.5 percent) lived in a free and clear owner-occupied home.¹²³ In contrast, U.S. home owners have high levels of indebtedness. Out of 76 million U.S. homeowners in 2009, only 31 percent owned their homes free and clear. Most Americans buy their first home in their mid-thirties with the goal of paying off the mortgage by retirement age. In turn, most accomplish that goal: 76 percent of those age 65 and over own their homes free and clear. Through 1995, more than 50 percent of homeowners had their home paid off by age 55. The other half did not have their homes paid off by age 65. This has important implications for labor market participation and for the feasibility of early retirement.¹²⁴ By having their home paid off, Europeans are better protected against economic vicissitudes.

Another difference is the mortgage instruments used in Europe and the United States. Of the existing 52 million U.S. mortgages in 2009, about 40 million were 25 to 30 year fixed-rate mortgages and only 2 million were adjustable rate. In contrast, more than 80 percent of home mortgages in Spain, Ireland, and elsewhere in Europe were variable rate mortgages.¹²⁵ As such, even a small interest rate change can have a significant impact on family finances. For instance, the monthly mortgage

payment on a \$300,000 loan with a 6 percent interest rate is \$1,798. Increasing that interest rate to 6.5 percent adds another \$100 a month to that payment. Borrowers are therefore never certain as to what their mortgage payment will be from month to month. In contrast, the dominant U.S. model of a 30-year fixed term mortgage protects borrowers from rapid rate hikes.

There is also a difference in the type of dwellings. In 2009, 42 percent of the EU-27 population lived in apartments, 34 percent in detached houses, and 23 percent in semi-detached houses. Almost three-quarters (74 percent) of the EU-27 population lived in owner-occupied dwellings, while 13 percent lived in rental homes and 13.5 percent in subsidized or free housing.¹²⁶ By comparison, close to 80 percent of all new U.S. households live in single-family houses. As Joel Catkin points out, two major housing market drivers—the millennial generation (born between 1983 and 2003) and immigrants—both prefer suburban, single-family houses. This is especially true in Sunbelt cities where immigrant centers tend to be in suburban rather than inner-urban areas. Studies show that the majority of the millennial generation—even more than their parents—identify their ideal residential location as the suburbs.¹²⁷

Despite the American dream of owning a single-family home, construction of multifamily units is growing faster than detached single-family homes. From 2010 to 2011, permits for structures with five or more units rose 60 percent compared to 24 percent for single-family homes. Although much of new construction is for rental-only properties, many developments are also built as owner-occupied units.¹²⁸ Should this trend continue, the growth of owner-occupied units will bring the U.S. closer to a European model that stresses apartments over detached single-family homes.

Public Housing

The Europeans use the term *social housing* to refer to public or publicly subsidized housing. Social housing in the EU is characterized by a wide variety of approaches among its member states. Although most social housing is rent-based, some countries allow the property to be bought by tenants. For instance, social housing recipients in the United Kingdom can assume shared ownership whereby they buy a share of the dwelling and pay rent on the rest. Social housing in Greece, Spain, and Cyprus is only available for sale.¹²⁹

One of the primary differences between U.S. and European housing policy is the availability of publicly assisted housing. In the United States, this would be either public housing or HUD subsidized units. All told, there about 5.5 million HUD subsidized housing units out of a total inventory of 41 million rental properties. Hence, roughly 13 percent of the U.S. rental stock is publicly subsidized.¹³⁰ By comparison, social housing accounts for 32 percent of the total housing stock in the Netherlands, 23 percent in Austria, 19 percent in Denmark, and 18 percent in the United Kingdom. Even more telling, Austria's social housing stock totals 56 percent of the total available rental stock. Social housing stock as a percentage of the total rental stock is high in other European nations as well: Denmark (51 percent), Finland (53 percent), France (44 percent), Hungary (53 percent), Netherlands (75 percent), Poland (64 percent), Slovakia (87 percent), Sweden (48 percent), and the United Kingdom (54 percent). America's commitment to publicly subsidized housing pales in contrast to most of the EU-27 countries.

Despite its problems, the U.S. housing sector displays considerable strength in a number of areas. For one, America's competitive mortgage market is more consumer-friendly than the European model that locks borrowers into risky variable rate mortgages. Facing inflation, variable rate mortgages will inevitably rise higher and faster than salaries, leaving homeowners with financial uncertainty. The U.S. housing market also performs well in homeownership rates, where more than 64 percent of families own their homes. This rate is on par with the EU-27 and considerably higher than Germany's low homeownership rate. U.S. tax policies, such as mortgage interest rate deductions, help make homeownership more affordable for a larger number of families. Although U.S. housing policies effectively help middle-class families purchase and stay in their homes, they do not reflect adequate support for public and subsidized housing, especially when compared to many European nations.

Conclusion

The housing crisis is a structural problem based on the failure of incomes to keep pace with housing costs; an overdependence on credit to build and buy houses; and a profit-making system that drives homeownership, development, and management. In contrast to many European nations, states and the federal government have failed to actively intervene

in the housing market. This failure is illustrated by the dearth of high-quality public housing units, by not offering larger subsidies for low-income homeownership, and by not adequately protecting poor families from predatory mortgage lending practices. Until the housing crash of 2007—which arguably functioned as a dramatic yet necessary market correction—profit and speculation drove up the price of rental and residential property faster than income growth. As a profit is made by each link in the housing chain (real estate agents and developers, lenders, builders, materials producers, investors, speculators, landlords, and homeowners), renters and homebuyers pay the costs. Put another way, the cost of every rental unit or home reflects the profits made by every party that has directly or indirectly come into contact with that property before it was even built.

The provision of adequate low- and moderate-income housing stock is an important challenge in modern society. The poor have difficulty finding decent affordable housing, while blue-collar workers and the lower-middle class are caught in the cost/income squeeze, where they are having a hard time buying and holding onto their homes. In the 2007 to 2008 economic downturn, this “affordability squeeze” resulted in increased mortgage foreclosures, high rates of property tax and delinquency defaults, evictions and homelessness, more overcrowding and doubling up of families, decreased consumption of other important necessities, deteriorating neighborhoods, increased business failures, higher rates of unemployment in the building trades, and the collapse of several large financial institutions.

Past and current government programs have had limited impact on the affordable housing crisis. Most affordable housing programs are underfunded and fragmentary. Because the federal government has been viewed as an arbiter of last resort, some housing advocates contend that the government has the responsibility to ensure that adequate housing is seen as a right rather than a privilege and that healthy, sound, and safe neighborhoods become a reality.

An adequate housing policy for the United States must address cost burdens, overcrowding, and housing quality. It must also provide opportunities for true housing choice and an end to the discriminatory practices that have led to de facto housing segregation. The nation will not have achieved the 1949 housing goal of “a decent home

and suitable living environment for every American family” until all households live in adequate housing located in safe neighborhoods.

DISCUSSION QUESTIONS

1. The history of federal housing programs and policies has been marked by evolving priorities. Describe the dominant trends in federal housing policy from 1937 and show how they led to the current housing policies. What is the current direction of federal housing policy?
2. Describe the major shortcomings in federal low-income housing policy, and discuss alternative policies to rectify these problems.
3. Homeownership is important because it can lead to asset growth. For example, a renter has nothing to show for a long-term rental except rental receipts. In contrast, a poor family that owns a home will have a major asset after paying off their mortgage. What are some of the obstacles standing in the way of homeownership for the poor? What policies can best help these families overcome the barriers?
4. Propose federal or state policies that might help poor families find affordable rental housing.
5. Homelessness has been described as simply a manifestation of poverty. Others contend that its roots are psychological, and it should be treated as a social service problem. Where do you stand on the issue?
6. Several proposals have been developed around homelessness. Which of these (or combination thereof) have the best chance for success?
7. Should housing be viewed as a right or a privilege?

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The Politics of Food Policy and Rural Life



Source: Jean-Yves Rabeuf/The Image Works

How can a nation as affluent as the United States allow food insecurity to occur? Food policy in the United States is marked by important contradictions. In 2014, 48 million Americans lived in food-insecure households, including almost 33 million adults and 15 million children.¹

The policies related to the production and distribution of food are an important part of the American welfare state. For example, over the course of a year an estimated one in four Americans participates in at least 1 of the 15 food assistance programs administered by the U.S. Department of Agriculture (USDA). Expenditures for these 15 food assistance programs totaled \$114 billion in 2016.² This chapter examines the federal response to hunger and the government's attempts to distribute foodstuffs to the poor. Topics include the Supplemental Nutrition Assistance Program (SNAP) (formerly called Food Stamps), WIC and other food programs, U.S. farm policy, the plight of farmworkers in the United States, and the overall problems of food production and distribution. Although the issues around food may initially seem disparate, they are tied together in a complex mosaic that is fundamental to the well-being of the nation.

The Contradictions of American Food Policy

Food policy in America is marked by glaring contradictions. Despite food insecurity, more than two-thirds (69 percent) of adults are overweight or obese, more than one-third (36 percent) of adults are obese, and more than 1 in 20 (6.3 percent) have extreme obesity. Almost three in four U.S. men (74 percent) are overweight or obese. About one-third of children and adolescents aged 6 to 19 are overweight or obese and 1 in 6 children and adolescents ages 6 to 19 are obese.³ About 68 percent of adult Americans are either obese or overweight.⁴ These figures are not surprising since Americans spend more money on fast food than on higher education, personal computers, computer software, and new cars.⁵ The Centers for Disease Control and Prevention (CDC) labels the obesity problem an epidemic.⁶

Prevalence rates for both obesity and overweight are higher in lower socioeconomic groups, indicating a link between income and inexpensive fast food options that have poor nutritional value.⁷

In fact, some of the most food-insecure households have the highest rate of obesity.

The impact of obesity on America's health care system is also growing. In 1998, the estimated medical costs of obesity were \$78.5 billion. Annual medical cost related to obesity today is nearly \$200 billion.⁸ Directly and indirectly, obesity costs the United States \$117 billion a year and results in 39 million lost work days. Three-fourths of the \$1.4 trillion the United States spends on health care is to treat chronic illnesses, many of which are tied to obesity and being overweight.⁹

Obesity, overweight, and fast food diets are bringing the nation's steady rise in life expectancy to a grinding halt. The CDC predicts that 29 million Americans will be diagnosed with diabetes in 2050 compared with about 11 million in 2003. Twenty years ago, the United States led the world's longevity league. That is no longer true today.¹⁰

The widespread impact of being obese and overweight on all areas of life is not always apparent. For example, childhood obesity has emerged as a cost to the workforce. Specifically, since many employers insure dependents as well as the workers themselves, the medical conditions of obese children are paid for by employers. A study on a large U.S. corporation in 2014 found that the average health insurance claim for an obese 10-year-old child in comparison to a normal weight child ranges from \$12,660 to \$19,630.¹¹ In addition, parents of obese children with associated medical conditions will also have increased absentee rates as they stay home to care for their children.¹²

With an annual 2016 budget of approximately \$156 billion,¹³ the U. S. Department of Agriculture (USDA) is responsible for distributing more federal resources than DHHS spends outside of Medicare and Medicaid. Roughly 43 percent of the USDA budget comprises means-tested food and nutrition programs.¹⁴ The remainder centers around commodity price supports, conservation programs, crop insurance, and export credit guarantees for the 2.1 million American farms. However, these subsidies are not supporting the small, family-owned and operated farms so frequently mentioned in Congressional appeals for more funding. One study demonstrated that large-scale commercial farms receive the greatest share of commodity payments.¹⁵ To put into perspective, the USDA provides around \$18 billion in subsidies that support less than half a million family farms.¹⁶ In comparison, TANF provides approximately \$15 billion¹⁷ to provide

income security to an average of 4 million recipients with incomes below the poverty line.¹⁸

Hunger in the United States

Hunger exists in the United States, but unlike the stereotype of starving and emaciated Third World children, it typically occurs in more subtle ways. For example, poor Americans may eat only once a day or skip meals. They may be subject to chronic malnutrition; women may deliver low-birth-weight babies who are at higher of infant mortality rates. Crossing age, race, and gender lines, hunger in the United States affects children, the elderly, the unemployed and the underemployed, the homeless, people with disabilities, and two-parent and single-parent families. The single common thread often connecting these diverse groups is poverty.

The USDA measures food security and insecurity along the following continuum:

- *High food security households* report no anxieties around accessing adequate food.
- *Marginal food security households* have occasional problems or anxiety around accessing adequate food. However, their intake of quality food is not substantially reduced.
- *Low food security households* have reduced quality and variety in their diets, but the quantity of food intake and normal eating patterns are not substantially disrupted.
- *Very low food security households* exhibit disrupted or reduced eating patterns of one or more family members because of the lack of money or benefits.¹⁹

Cheap and plentiful food is omnipresent, which partly explains why so many Americans over the age of 25 are overweight.²⁰ Americans spend a smaller percentage of their income on food than any other nation, and we feed much of the world with our surplus (the value of U.S. agricultural exports totaled more than \$131 billion in 2015).²¹ Moreover, food has become cheaper. In 1963, the average American family spent 20 percent of their income on food; by 2010, it was only 10 percent. Consumers, retailers, restaurants, and farmers throw away one-quarter of the U.S. food stock (almost 100 billion pounds of edible food) each year.²² Yet in 2014, 14 percent (17.4 million) of U.S. households were food insecure. Households are classified by the USDA as food secure, food insecure without hunger, or food

insecure with hunger (i.e., if one or more household members were hungry at some time during the year because the household could not afford enough food). Women and children make up about three-fourths of this population. Adults were food insecure in about half of the food-insecure households. However, 8.3 percent of households with children had one or more children who were food insecure at some time during the year. In 0.8 percent of households with children, one or more of the children experienced the most severe food-insecure condition measured by USDA—very low food security, irregular meals, and food intake below levels considered adequate. Food pantries and emergency kitchens (soup kitchens) helped close that gap by feeding approximately 17.4 million people in 2014.²³

Children denied an adequate diet are at a greater risk of not reaching their full potential as individuals. Undernourished youngsters have trouble concentrating and bonding with other children, and they are more likely to suffer illnesses resulting in school absences. Compared to adequately nourished children, they consistently perform more poorly on standardized tests and are at a major risk of dropping out of school. Studies have also shown that even mildly undernourished children may potentially suffer brain, cognitive, and psychological impairment that, if not corrected, can be irreversible. Teenagers who are food insufficient are more likely to have seen a psychologist, have been suspended from school, and have had difficulty getting along with other teenagers. Food insufficiency is also associated with increased risk of suicide in teenagers.²⁴ Research conducted by the Center on Hunger, Poverty and Nutritional Policy at Tufts University found compelling evidence that improved nutrition can modify and even reverse these effects.

The following data provides a snapshot of America's hungry in 2010 to 2014.

- In 2010, the average daily prevalence of very low food security was present in 800,000 to 1.2 million households, and children went hungry in 46,000 to 56,000 homes.
- At some time during 2014, 14 percent (17.4 million) of U.S. households were food insecure; very low food security was observed in 6 percent (6.9 million) of U.S. households.
- In 2013, 14.3 percent of children lived in households with food insecurity; 5.6 percent of U.S. children lived in households with very low food security.

- Some groups with rates of food insecurity are much higher than the national average: households with incomes below the official poverty line (43 percent), households with children, those headed by a single woman (36.6 percent), black households (24.9 percent), and Hispanic households (26.9 percent).
- Households with children had nearly twice the rate of food insecurity (21 percent) as those without (11 percent). Among households with children, married-couple families had the lowest rate of food insecurity (15 percent).
- Food insecurity was higher for households in metropolitan areas (17 percent) than in non-metro areas (14 percent). Regionally, food insecurity was highest in the South (16 percent) and West (15 percent), intermediate in the Midwest (14 percent), and lowest in the Northeast (12 percent).²⁵
- Almost 61 percent of food-insecure households in 2014 had participated in one or more of Federal food and nutrition assistance programs—the National School Lunch Program, the Supplemental Nutrition Assistance Program (SNAP—previously called Food Stamps), and the Special Supplemental Nutrition Program for Women, Infants, and Children (WIC).²⁶
- About 23 percent of food-insecure households got emergency food from a food pantry at some time during the year, and 3 percent ate one or more meals at an emergency kitchen.²⁷

The extent of hunger and food insecurity is reflected in the number of people seeking food from emergency food providers, such as food pantries and soup kitchens. Food pantries, shelters, soup kitchens, and other emergency food providers serve at least 23 million people a year.²⁸

Governmental Food Programs

The politics of food—or the way food is distributed in U.S. society—is complex. Like all resources in a free market society, food is a commodity. In a pure market sense, those who cannot afford to purchase food are unable to consume it. Left to the caprice of the marketplace, many poor people would face malnutrition or even starvation. This problem is particularly acute in an urban environment, where most people lack gardening skills and have little or no access to land. Providing the poor with food is an important redistributive function of the welfare state.

SNAP (Formerly Called Food Stamps): A Description of the Program

The current Supplemental Nutrition Assistance Program (SNAP—formerly called Food Stamps) was originally enacted by the U.S. Congress in 1964. Food stamps increase the nutritional value of a low-income household's home food supply by 20 to 40 percent, and households that participate spend more and acquire more food than nonparticipating low-income households.²⁹

The federal government funds 100 percent of SNAP benefits, with federal and state governments sharing 50 percent of administrative costs. Although administered by the USDA, state and local welfare agencies qualify applicants and provide them with **Electronic Benefit Transfer (EBT)** cards that are the sole method of benefit delivery. These cards can be used at any store that accepts SNAP. Recipients are given an allotment based on family size and income, with means-tested eligibility requirements and benefits determined at the federal level (see Table 17.1 and Figure 17.1). For example, the gross monthly income of most households who apply must be 130 percent or less of the federal poverty guidelines.³⁰ Recipients originally had to pay a set price (depending on family size and income) for their stamps. For example, some families or individuals could purchase \$75 worth of food stamps for \$35; the difference between the cost and the face value of the stamps was called

TABLE 17.1 U.S. Maximum Monthly SNAP Allotments, October 2015 to September 2016

Household Size	Maximum Allotment Level (48 states and DC)
1	\$194
2	\$357
3	\$511
4	\$649
5	\$771
6	\$925
7	\$1,022
8	\$1,169
Each additional person	+\$146

Source: U.S. Department of Agriculture, Supplemental Nutrition Assistance Program (SNAP), 2015, <http://www.fns.usda.gov/snap/eligibility#Special%20Rules%20for%20the%20Elderly%20or%20>

Benefits

- The benefit amount is called an allotment. The net monthly income of the household is multiplied by 0.3, and the result is subtracted from the maximum allotment for the household size (see above).
- SNAP households are expected to spend 30 percent of their resources on food.

Income

- Households have to meet income tests unless all members are receiving TANF, SSI, or general assistance.
- Most households must meet the gross and net income tests. Those with an elderly member or someone receiving certain types of disability only has to meet the net income test.

Assets

- Households are allowed \$2,250 in countable resources (i.e., a bank account) or \$3,250 in countable resources if at least one person is disabled or age 60 or older.
- These resources not counted include: a home and lot, the resources of those receiving SSI, the resources of those receiving TANF, and most retirement (pension) plans.
- Vehicles assets are determined at the state level. States can substitute the TANF vehicle rules for SNAP vehicle rules when it results in lowered household assets. Some states exclude the value of the household's primary vehicle. Fifteen states totally exclude the value of at least one vehicle per household. Other states exempt an amount higher than the SNAP's standard \$4,650 auto exemption.

Deductions

Deductions are allowed as follows:

- A 20 percent deduction from earned income.
- A standard deduction of \$155 for households of one to three people and \$168 for a household of four. The deduction is higher for some larger households.
- A dependent care deduction when it is needed for work, training, or education.
- Out-of-pocket medical expenses for elderly or disabled beneficiaries that is more than \$35 a month.

- Child support payments.
- Some states allow homeless households a set amount (\$143) for shelter costs.
- Shelter costing more than half of the household's income after the other deductions. The amount of the shelter deduction cannot be more than \$504 unless one person is elderly or disabled. (The limit is higher in Alaska, Hawaii, and Guam.)
- The cost of fuel to heat and cook, electricity, water, basic telephone fee, rent or mortgage payments, and property taxes.

Employment Requirements

Applicants must generally meet work requirements to be eligible for SNAP. These work requirements include:

- Registering for work.
- Not voluntarily quitting or reducing hours.
- Accepting a job if offered.
- Participating in state-assigned employment and training programs. Failure to comply with these requirements can result in disqualification from the Program.
- Able-bodied adults without dependents are required to work or participate in a work program for at least 20 hours a week to receive SNAP benefits for more than 3 months in a 36-month period.
- Some excluded groups include children, seniors, pregnant women, and those exempt for physical or mental health reasons.

Immigrant Eligibility Requirements

The 2002 Farm Bill restored SNAP eligibility to most legal immigrants that:

- Have lived in the United States for five years.
- Are receiving disability-related assistance or benefits.
- Are under 18.
- People admitted for humanitarian reasons and for permanent residence may also eligible. Eligible household members can get SNAP benefits regardless of whether other members of the household are ineligible.
- Noncitizens that are in the United States temporarily, such as students, are not eligible.

FIGURE 17.1 SNAP Eligibility and Benefit Guidelines, 2016

Special Rules for the Elderly or Disabled

There are special rules for households that contain an elderly or disabled member.

- A person is classified as elderly if he or she is 60 or older.
- A person is classified as disabled if he or she:
 - Receives disability payments under Social Security or SSI.
 - Is a veteran who is totally disabled, permanently housebound, or in need of regular aid.

- Is a surviving spouse or child of a veteran receiving VA benefits who is considered permanently disabled.

Source: United States Department of Agriculture, Supplemental Nutrition Assistance Program (SNAP), November, 25, 2015. Retrieved January 2016 from <http://www.fns.usda.gov/snap/eligibility#Special%20Rules%20for%20the%20Elderly%20or%20Disabled>.

FIGURE 17.1 (Continued)

a “bonus.” This system proved unwieldy because many poor people could not afford to purchase stamps. When purchase requirements were dropped in 1977, the participation rate rose by 30 percent.³¹

SNAP: Who Is in the Program, and What Does It Cost?

In 2016, 45 million people participated in the SNAP program at a cost of roughly \$70 billion³² (see Table 17.2). Nevertheless, only about half of all eligible people receive SNAP benefits.³³ Contrary to the myths surrounding SNAP, the program is targeted toward those most in need. Data from 2013 to 2015 illustrate that:

- The four-person SNAP household in 2015 received a monthly SNAP benefit of \$649 and had a gross monthly income of \$2,628. In 2013, less than 13 percent of SNAP households received public assistance.³⁴
- Seventy-five percent of SNAP households included a child, an elderly person, or a disabled person.
- In 2011, 31 percent of SNAP households had earned income, 20 percent received Supplemental Security Income (SSI), 24 percent received Social Security Income, 9 percent received Child Support Enforcement payments, 17 percent received support from Temporary Assistance for Needy Families (TANF), and 4 percent received Unemployment Income.³⁵

Special Supplemental Nutrition Program for Women, Infants, and Children (WIC)

Enacted in 1972, the WIC program was originally designed as a two-year pilot program to provide nutritional counseling and supplemental foods to pregnant and breastfeeding women, infants, and young children at nutritional risk. The goal of WIC is to address areas of child development that are most affected by poor health and inadequate nutrition, including impaired learning.³⁶ Twin goals of WIC include (1) enriching the food intake of participants and (2) educating mothers (individually and in groups) on how to prevent nutritional difficulties.³⁷

Since WIC is not an entitlement program, Congress does not set aside funds to allow each and every eligible individual to participate. Instead, WIC is a federal grant program for which Congress authorizes a specific amount of funds each year. WIC is administered at the federal level by USDA's Food and Nutrition Service (FNS) and by each state's WIC agencies. Pregnant, breastfeeding, and postpartum women; infants; and children up to five years of age are eligible if they (1) are individually determined by a health professional to be at nutritional risk; (2) meet a state residency requirement; and (3) meet an income standard, or are determined automatically income eligible. A person who participates or has family members who participate in certain other benefit programs, such as SNAP, Medicaid, or TANF, automatically meets the income eligibility requirement (see Table 17.3). Qualified beneficiaries receive supplemental foods each month in the form of actual food items or, more often, are given vouchers for the purchase of specific items in retail stores that can include milk, cheese, eggs, infant formula, cereals, and fruits and



Watch this video interview of a young woman using SNAP benefits. Not everyone on “food stamps” fits the stereotype most people have.

<https://www.youtube.com/watch?v=wofi37Zak6k>

TABLE 17.2 SNAP Statistics, Selected Years, 1975–2015 (participants and cost figures in millions)

Year	Total Cost of SNAP	Number of Participants	Percentage of Population Using SNAP	Average Monthly Benefits per Person
1975	\$ 4,624	16.3	7.6	\$ 21.40
1978	5,573	14.4	6.5	26.80
1981	11,812	20.6	9.0	39.50
1984	13,275	20.9	8.8	42.70
1987	13,535	19.1	7.8	45.80
1990	17,686	20.0	8.0	59.00
1991	21,012	22.6	9.0	63.90
1993	23,653	26.9	9.3	67.97
1995	20,500	26.7	8.9	71.50
2000	16,956	17.1	6.0	72.77
2003	23,880	21.2	7.5	83.93
2007	33,204	26.5	8.8	95.63
2009	53,632	33.4	11.0	125.31
2011	75,674	44.7	14.0	133.85
2015	69,656	45.4	14.0	126.83

Source: United States Department of Agriculture, Supplemental Nutrition Assistance Program (SNAP), 2015.

vegetables. In 2016, WIC served 8.5 million women and children at a cost of \$6.6 billion. Most participants were children (53 percent), while 23 percent were infants and 23 percent were women.³⁸

WIC operates in all states and is administered at the federal level by USDA's Food and Nutrition Service (FNS) and by state WIC agencies.³⁹ Income



The National School Lunch Program is a federally assisted meal program that provides lunches to school-age children.

Source: Monkey Business/Fotolia

TABLE 17.3 Gross and Net Income Eligibility Standards for the Continental United States, October 1, 2015 to September 30, 2016

Household Size	Gross Monthly Income (130 percent of poverty)	Net Monthly Income (100 percent of poverty)
1	\$1,276	\$981
2	1,726	1,328
3	2,177	1,675
4	2,628	2,021
5	3,078	2,368
6	3,529	2,715
7	3,980	3,061
8	4,430	3,408
Each additional member	+451	+347

*Eligibility levels are higher for Alaska and Hawaii.

Source: U.S. Department of Agriculture, Food and Nutrition Service, Supplemental Nutrition Assistance Program (SNAP), 2015. <http://www.fns.usda.gov/snap/eligibility#Special%20Rules%20for%20the%20Elderly%20or%20>

TABLE 17.4 WIC Income Eligibility Guidelines, July 1, 2015 to June 30, 2016 (48 contiguous states, DC, Guam, and territories) Reduce Price Meals 185%

Household Size	Annually	Monthly	Twice Monthly	Biweekly	Weekly
1	\$21,775	\$1,815	\$908	\$838	\$419
2	29,471	2,456	1,228	1,134	567
3	37,167	3,098	1,549	1,430	715
4	44,863	3,739	1,870	1,726	863
5	52,559	4,380	2,190	2,022	1,011
6	60,255	5,022	2,511	2,318	1,159
7	67,951	5,663	2,832	2,614	1,307
8	75,647	6,304	3,152	2,910	1,455
Each additional member	+\$7,696	+\$642	+\$321	+\$296	+\$148

*Eligibility levels are higher for Alaska and Hawaii.

Source: U.S. Department of Agriculture, Food and Nutrition Service, *WIC Income Eligibility Guidelines 2015–2016*, Last Modified September 2, 2015. Retrieved from <http://www.fns.usda.gov/sites/default/files/wic/WIC-IEGS-WEB.pdf>.

eligibility is less restrictive than for public assistance programs⁴⁰ (see Table 17.4).

Other Food Programs

Augmenting SNAP and WIC are a range of programs targeted at various constituencies. For instance, through the Child and Adult Care Food Program (CACFP), snacks are available to children

and adults that are mentally or physically disabled. The National School Lunch Program (NSLP) served 31 million children in 2012. The School Breakfast Program (SBP) operates in 89,000 schools and institutions, and the Summer Food Service Program serves 200 million meals to low-income children when school is not in session.⁴¹

Although some low-income elderly receive SNAP benefits, they can also be served by Meals

Spotlight 17.1

RESPONDING TO HUNGER IN AMERICA

The federal government's response to hunger and malnutrition in the United States consists of several major programs: the Supplemental Nutrition Assistance Program (SNAP); various food distribution programs including the Schools/Child Nutrition Commodity Program and The Emergency Food Assistance Program (TEFAP); the national school

lunch and breakfast programs; the Fresh Fruit and Vegetable Program; the Special Milk Program; the Special Supplemental Nutrition Program for Woman, Infants, and Children (WIC); the Child and Adult Care Food Program; the Summer Food Service Program; Meals on Wheels; and the Congregate Dining program.

To learn more about each of these programs and the specific services they provide, go to the USDA's Food, Nutrition, and Consumer Services website at www.fns.usda.gov/fns/.



Watch this video for information on what living on food stamps is like.

www.youtube.com/watch?v=0YuXDbqg8z4

on Wheels and the Congregate Meal Dining program. Begun in 1972, the Meals on Wheels program delivers meals for adults 60 years or older unable to take care of their own nutritional needs. Various community agencies arrange the daily delivery of meals to elderly persons living at home, and aged persons who receive food stamps can use them to purchase the meals. In some locations, meal services are also provided for nonsenior handicapped people. The copay for meals can vary from \$2 or more, depending upon the program and the income of the recipient. There is no means test for the program. The Congregate Meal Dining program provides meals at places such as senior citizen centers.

Figure 17.2 examines other food programs.

Have the Food Programs Worked?

Because WIC is a discretionary rather than an entitlement program, it can serve only as many people as its budget allows. Consequently, it is estimated that only about half of those eligible receive WIC benefits.⁴² Others turn to emergency food banks. Approximately 46 million people in 2015 received emergency food assistance from an agency served by a Feeding America (formerly Second Harvest) member. This included the more than 33,500 food pantries, 4,500 soup kitchens, and 3,600 emergency shelters.⁴³

According to USDA studies, WIC saves lives and improves the health of nutritionally at-risk women, infants, and children. The results of studies conducted by FNS and other nongovernment entities demonstrate that WIC is one of the most successful and cost-effective nutrition intervention programs. Some highlights of the findings are that WIC improves diet and diet-related outcomes, improves infant feeding practices, improves immunization rates and regular medical care, improves cognitive development, and improves preconception nutritional status. USDA research has also shown that the WIC program is playing an important role in improving birth outcomes and thus in containing health care costs. USDA reports found that Medicaid-eligible pregnant women in five states who participated in WIC during their pregnancies had longer pregnancies, fewer premature births, lower incidence of low-birth weight infants, fewer infant deaths, and a greater likelihood of receiving prenatal care. Health care savings ranged from \$1.77 to \$3.13 for each dollar spent on WIC.⁴⁴

Studies showed that SNAP caseloads declined during periods of economic growth not only because many households' circumstances improved enough to make them ineligible for benefits but also because a smaller percentage of the potentially eligible households were participating in the program.⁴⁵ A General Accounting Office (GAO) study on SNAP found that the need for food assistance had not diminished despite lower caseloads. Instead, needy individuals and families increasingly relied on SNAP and other sources, such as food banks, soup kitchens, and emergency pantries. These findings were corroborated by other data. A Wisconsin study found that more than one-third of former welfare recipients had problems paying for food despite a high incidence of employment. Fewer than half of the former recipients reported having more money than when they were receiving public assistance, and 68 percent reported that they were "barely making it." A South Carolina Department of Social Services study reported that after leaving TANF, 17 percent of former welfare recipients had no way to buy food some of the time. This is twice the number of families who reported such problems while receiving assistance. An analysis of former welfare recipients in Massachusetts found that food insecurity increased for many families after they went off welfare. Forty percent of the families interviewed did not have enough food or enough of the right kinds of food after being off welfare for one year. More than 20 percent of families who remained off welfare after one year used food banks or had to borrow money to buy food. In short, as poor Americans transition from welfare to work, and their benefits are cut or reduced, they are finding their wages insufficient to meet basic needs, including child care, transportation, and housing costs. As a result, working poor families may cut their food budget or turn to local charities for aid.⁴⁶

The increased demand for food pantries and other forms of food aid demonstrates that welfare-to-work programs are not working. Hunger relief agencies report that since the early 1990s, the greatest increase in hunger has been among the working poor. A 2003 U.S. Conference of Mayors study found that 39 percent of people requesting emergency food assistance in U.S. cities were employed.⁴⁷ This was corroborated by a 2010 Hunger in America study, which found that 36 percent of people using emergency food banks were employed.⁴⁸

National School Lunch Program

The NSLP program provides lunches and the opportunity to practice skills learned in classroom nutrition education. NSLP is a federally assisted meal program that served 31 million children in around 100,000 public and nonprofit private schools and residential child care institutions in 2012. In 2015, about 5 billion school lunches were served annually. Almost 20 million students received free lunches, 2.2 million received reduced price lunches (40 cents), and 8.5 million full price lunches.¹

School districts and independent schools that take part in the lunch program receive cash subsidies and donated commodities from the USDA for each meal they serve. Free students are not charged, and reduced price students pay a nominal amount for lunch. In return, educational institutions must serve lunches that meet federal requirements. Children from families with incomes at or below 130 percent of the poverty level are eligible for free meals. Those between 130 percent and 185 percent of the poverty level are eligible for reduced-price meals. Children from families with incomes over 185 percent of poverty pay full price, though their meals are still subsidized to some extent. Local school food authorities set their own prices for full-price meals, but most operate their meal services as a nonprofit organization. Since the program began, more than 224 billion lunches have been served. The cost of the National School Lunch Program in FY2016 was almost \$13 billion.²

School Breakfast Program (SBP)

The SBP serves 2.3 billion school breakfasts annually to promote learning readiness and healthy eating behaviors. It provides breakfast to 14 million children in 90,000 schools and institutions at a cost of almost \$4 billion. The SBP is a federal program administered by the USDA and the Federal Nutrition Service that provides states with cash assistance for nonprofit breakfast programs in schools and residential child care institutions.³

Breakfasts served as part of the SBP must meet the American Dietary Guideline recommendations and provide one-fourth or more of the daily recommended levels for key nutrients that children need. They are also required to provide no more than 30 percent of calories from fat and less than 10 percent of calories from saturated fat.⁴ Research shows that children who have school breakfasts eat more fruits, drink more milk, and consume less saturated fat than those who don't eat breakfast or have breakfast at home.

Children whose families meet income criteria may receive free or reduced-price breakfasts. Those

from families with incomes at or below 130 percent of the federal poverty level are eligible for free meals. Children whose family income is between 130 percent and 185 percent of the poverty level are eligible for reduced-price meals. Those from families over 185 percent of poverty pay a full price, although their meals are still subsidized to some extent. Schools may not charge more than 30 cents for a reduced-price breakfast. Schools set their own prices for breakfasts served to students who pay the full meal price, although they must operate their meal services as a nonprofit organization.⁵

Special Milk Program (SMP)

The SMP provides milk to children in schools, some camps, and child care institutions that do not participate in other federal child nutrition meal service programs. Schools or institutions are limited to providing pasteurized liquid milk that is either fat free or low fat (1 percent). Milk must meet state and local standards, and contain FDA-approved amounts of vitamins A and D.⁶

When local school officials offer free milk under the program, any child from a family that meets income guidelines for free meals and milk is eligible. In 2011, 3,848 schools and residential care institutions, 782 summer camps, and 527 nonresidential care institutions participated in the SMP. Each child's family must apply annually for free milk eligibility. In FY2011, over 66 million half pints of milk were served, which represents a significant reduction since 2007. This is due to the increased development of the National School Lunch and School Breakfast Programs.⁷

Summer Food Service Program (SFSP)

The SFSP serves meals and snacks to low-income children during long school vacations. The FNS administers the SFSP at the federal level; state education agencies administer the program in most states, although in some areas the state health or social service department or an FNS regional office may be designated. Locally, SFSP is run by approved sponsors, including school districts, local government agencies, camps, or private nonprofit organizations. Sponsors provide free meals to a group of children at a central site, such as a school or a community center. States approve SFSP meal sites as open, enrolled, or camp sites. Open sites operate in low-income areas where at least half of the children come from families with incomes at or below 185 percent of the federal poverty level, making the children eligible for free and reduced-price school meals. Children 18 and younger may receive free

FIGURE 17.2 Other USDA Food Programs

meals and snacks through SFSP. Meals and snacks are also available to persons with disabilities who are over age 18 but who participate in school programs for mentally or physically disabled people.⁸

Child and Adult Care Food Program (CACFP)

CACFP is a key source of support for serving nutritious meals and snacks in child care centers, family child care homes, Head Start, after-school programs, shelters, and adult day care centers. The program provides reimbursement for food and meal preparation costs, ongoing training in the nutritional needs of children, and onsite assistance in meeting the program's strong nutritional requirements. In FY2014, 3.3 million children and 120,000 adults received nutritious meals every day.⁹

Current program eligibility requirements provide meals to persons who fall into one of the following categories:

- Children at or under the age of 12
- Children of migrant workers at or under the age of 15
- Persons of any age who have a disability (as determined by the state) and are enrolled in an eligible institution serving participants who are mainly 18 years or under
- Persons aged 18 years or under in emergency shelters
- Persons aged 18 years or under enrolled in approved "at-risk" after-school care programs
- Functionally impaired adults or adults aged 60 or over who are enrolled in adult day care centers

Moreover, institutions must determine income eligibility categories for each participant in order to determine if they receive free or reduced cost meals. Participants in households whose income is at or below 130 percent of the poverty line will receive free meals, and participants in households between 130 and 185 percent of the poverty line will receive reduced cost meals. Children whose families have already been approved for programs such as SNAP and TANF are automatically eligible for free meals.¹⁰

The Emergency Food Assistance Program (TEFAP)

TEFAP was first authorized to distribute surplus commodities in 1981. The 1988 Hunger Prevention Act required USDA to purchase additional commodities for low-income households and local emergency feeding organizations. The amount of food states receive is dependent on their number of unemployed and number of persons living below

the poverty line, thus ensuring that states who have a larger low-income proportion of their population receive more assistance.

TEFAP provides USDA commodities to states who distribute the food through local emergency food providers. Available foods vary depending on market conditions. Each state determines the criteria for household eligibility and may adjust income criteria based on need in the state. Eligibility criteria may include participation in existing food or other assistance programs for which income is considered as a basis for eligibility. In 2016, Congress provided \$318 million for TEFAP.¹¹

¹U.S. Department of Agriculture, Food and Nutrition Service, *Program Fact Sheet: School Meal Trends & Stats*, 2016. Retrieved February 2016 from <https://schoolnutrition.org/AboutSchoolMeals/SchoolMealTrendsStats/>.

²U.S. Department of Agriculture, Food and Nutrition Service, *Program Fact Sheet: National School Lunch Program*, August 2012. Retrieved August 2012 from <http://www.fns.usda.gov/cnd/lunch/AboutLunch/NSLPFactSheet.pdf>.

³U.S. Department of Agriculture, Food and Nutrition Service, *Program Fact Sheet: The School Breakfast Program*, October 2011. Retrieved August 2012 from <http://www.fns.usda.gov/cnd/breakfast/AboutBFast/SBPFactSheet.pdf>.

⁴U.S. Department of Agriculture, Food and Nutrition Service, *Program Fact Sheet: The School Breakfast Program*, October 2011. Retrieved August 2012 from <http://www.fns.usda.gov/cnd/breakfast/AboutBFast/SBPFactSheet.pdf>.

⁵U.S. Department of Agriculture, Food and Nutrition Service, *Program Fact Sheet: The School Breakfast Program*, October 2011. Retrieved August 2012 from <http://www.fns.usda.gov/cnd/breakfast/AboutBFast/SBPFactSheet.pdf>.

⁶U.S. Department of Agriculture, Food and Nutrition Service, *Program Fact Sheet: Special Milk Program*, August 2012. Retrieved August 2012 from <http://www.fns.usda.gov/cnd/milk/AboutMilk/SMPFactSheet.pdf>.

⁷U.S. Department of Agriculture, Food and Nutrition Service, *Program Fact Sheet: Special Milk Program*, August 2012. Retrieved August 2012 from <http://www.fns.usda.gov/cnd/milk/AboutMilk/SMPFactSheet.pdf>.

⁸U.S. Department of Agriculture, Food and Nutrition Service, *Frequently Asked Questions: Summer Food Service Program*, Last Modified April 11, 2012. Retrieved August 2012 from <http://www.fns.usda.gov/cnd/summer/FAQs.html>.

⁹U.S. Department of Agriculture, Economic Research Service, "The Food Assistance Landscape: FY2011 Annual Report," Economic Information Bulletin, no. 93 (March 2012) Retrieved August 2012 from http://www.ers.usda.gov/media/376910/eib93_1_.pdf.

¹⁰U.S. Department of Agriculture, Economic Research Service, "The Food Assistance Landscape: FY2011 Annual Report," Economic Information Bulletin, no. 93 (March 2012) Retrieved August 2012 from http://www.ers.usda.gov/media/376910/eib93_1_.pdf.

¹¹U.S. Department of Agriculture, Food and Nutrition Service, Child and Adult Care Food Program, Last Modified February 21, 2012. Retrieved August 2012 from <http://www.fns.usda.gov/cnd/care/CACFP/aboutcacfp.htm>. See also USDA TEFAP Memo, January 7, 2016. Retrieved February 2016 from [leave_behind_2015_budget_approps_priorities.pdf](http://www.fns.usda.gov/cnd/care/CACFP/aboutcacfp.htm).

Although the USDA is responsible for nutritional and food assistance programs, it is also focused on food, farming, and rural life. It is to this area that we will now turn.

Farming in the United States

Farmers have historically demanded that agricultural commodity price supports be set at parity levels—in other words, an equal ratio between farm prices and input/output costs.⁴⁹ In 1942, Congress established the price support levels at 90 percent of parity, and from 1942 to 1953, average prices paid to farmers were at 90 to 100 percent of parity, thereby raising market prices, ensuring a secure income for farmers, reducing the need for excessive debt, and encouraging stabilization in the price of grain. By the end of World War II, powerful corporations, academics, and free traders had begun to wage war on the farm parity program. Soil conservation, supply management, and parity were characterized as socialist ideas that interfered with a free market economy. Grain companies called for lower prices to help foreign sales, arguing that expanded exports and food aid programs would compensate farmers for lower commodity prices. The small farmer eventually lost to this powerful coalition, and farm parity was terminated in 1953. As a result, the purchasing power of farm income decreased even as exports rose.⁵⁰

Family farming embodies many of America's most cherished values—hard work, independence, strong family life, and close-knit communities. For many rural families, farming is a way of life. Working the land—often a legacy handed down from parents or grandparents—creates a commitment to place and family heritage.⁵¹ This psychological connection to farming means that many farmers see themselves as farmer-caretakers. Hence, financial failure not only leaves farmers with a sense of personal failure but also with shame for having failed their heritage. Reverend Paul Tidemann, a Lutheran minister who studied the 1980s farm crisis reported that losing a farm is not like losing a job. This loss is a loss of a personal and family connection to the land. It also represents a failure for future generations of farmers.⁵²

Governmental Farm Policies

In 2002, former president George W. Bush signed the Farm Security and Rural Investment Act, which

governed federal farm programs until 2008. Among other things, the bill expanded conservation land retirement programs; emphasized environmental practices; relaxed rules thus making more borrowers eligible for federal farm credit assistance; restored food stamp eligibility to legal immigrants; added various commodities to those requiring country-of-origin labeling; and introduced provisions on animal welfare. One contentious issue was mandatory country-of-origin labeling for meat and other foods.⁵³ In 2014, Congress passed a new farm bill. The 2014 Farm Bill:

- Eliminates direct payments and continues crop insurance.
- Restores livestock disaster assistance, and funds a livestock risk and disaster prevention program.
- Aids rural development by providing \$15 million for rural businesses through the Rural Microentrepreneur Assistance Program. It calls for a 10-percent set-aside from certain programs to be redirected to rural development.
- Provides \$150 million to renovate and develop water and wastewater infrastructure.
- Authorizes \$200 million annually for developing international markets.
- Allows USAID to use cash assistance in the Food for Peace program.
- Funds \$200 million for a foundation for agricultural research.
- Increases funding for speciality crop research.
- Consolidates conservation programs and links basic conservation practices to crop insurance subsidies for environmentally sensitive farming lands.
- Provides \$880 million for energy programs included in the 2008 Farm Bill.
- Expands biobased products and renewable chemical manufacturing.
- Keeps SNAP eligibility intact for millions of low-income families, provides \$250 for TEFAP, and funds the Health Food Financing Initiative (making nutritious food more accessible).
- Provides funding for the Beginning Farmers and Ranchers Development Program.
- Provides more resources and funding for organic farmers.
- Increases funding for pest and disease management and disaster prevention.⁵⁴

Spotlight17.2

THE NATIONAL FARMER'S UNION

The National Farmer's Union is a general farm organization that represents farmers and ranchers in the United States. The goal of this organization is to sustain and strengthen family farm and ranch agriculture. To learn more about this organization, go to its website at www.nfu.org.

American Farm Bureau

The American Farm Bureau is an independent nongovernmental, nonpartisan, and nonsectarian

organization representing agricultural producers at all levels. To learn more about this organization, go to its website at <http://www.fb.org/>.

The United Farm Workers Association

The United Farm Workers Association (UFW) represents the interests of farm workers throughout the United States. To learn more about this organization, go to its website at www.ufw.org.

Biofuels, Fracking, and Farming

Biofuel production (ethanol) has been an economic boon to the American farmer. Touted as a greener alternative to gasoline, ethanol can be made from corn, barley, wheat, sugar cane, or beet. Proponents argue that ethanol is a clean-burning renewable energy source that can help the United States reduce its energy reliance on foreign oil. By 2011, about 25 percent of the nation's grain crops and corn went into ethanol production for cars.⁵⁵

Critics argue that biofuel crops, such as corn, are grown with the help of fossil fuels in the form of fertilizers, pesticides, and the gasoline needed for farm equipment. One estimate is that corn needs 30 percent more energy than the finished fuel it produces. Another problem is the land required to produce it. Still others estimate that the grain needed to make enough ethanol to fill the gas tank of a 4×4 is sufficient to feed a person for a year. Critics argue that this biofuel production is helping to usher in worldwide food shortages. Other critics have questioned the role fertilizer is playing in climate change, claiming that using fertilizer on biofuel crops will be emitting enough nitrous oxide (more than 296 times more powerful heat trapping gas than CO_2) to eliminate all the carbon savings biofuels produce.⁵⁶

Although biofuels are more carbon dioxide neutral than fossil fuels, the UN warns that land use, food supply, and water supply issues can be

problematic in biofuel production. The dangers of biofuels involve the loss of biodiversity as forests are cleared to grow crops for ethanol, the exacerbation of water shortages, the distortion of food prices, and the competition with local food production. By 2012, increased biofuel production from corn, coupled with reduced crop yields due to drought, had dramatically raised the price of corn in many developing countries to record levels, thereby leading to increased political instability.

The thirst for energy has also led to various kinds of high-impact carbon extractions, such as hydraulic fracturing or "*fracking*." Fracking is the process of drilling deep wells and injecting water, sand, and proprietary chemicals into them at a high pressure in order to fracture shale rocks to release the natural gas inside. Horizontal fracking uses a mixture of almost 600 chemicals (many of them proprietary and carcinogenic) and 1 to 8 million gallons of water for each frack job. After fracking, the water becomes contaminated and must be either cleaned or disposed of.

Fracking industry officials and supporters claim that the process is safe, environmentally responsible, and the key to energy self-sufficiency. They point to Canada, which has been using fracking techniques for 35 years with little record of pollution or major environmental impacts. Supporters point to North Dakota, which has a booming

economy, a budget surplus, and a low-unemployment rate. They also point to how fracking has driven down natural gas prices so low that power plants are using natural gas instead of dirty coal.⁵⁷

Critics point to the potential environmental impacts, including contamination of ground water, risks to air quality, the migration of gases and chemicals to the surface, contamination from spills, and the deleterious health effects associated with fracking. For instance, during fracking, methane gas and toxic chemicals leach out from the system and contaminate nearby groundwater. Contaminated well water is then utilized by farms and nearby cities and towns. Reports have surfaced of cases of respiratory and neurological damage due to contaminated water. The waste fluid is left in open air pits to evaporate, releasing harmful compounds into the atmosphere, creating acid rain and ground level ozone. For these and other reasons, hydraulic fracturing has come under scrutiny, with some countries suspending or banning it.⁵⁸ Since fracking often occurs on prime farmland, it has an important impact on farming.

The Face of U.S. Farming

The following text and Figure 17.3 highlight important trends in U.S. agriculture:

- *The changing landscape of U.S. farming.* Of all the U.S. occupations, farming is facing the greatest decline, although that is dependent on the size of the farming operation. As of 2010, there were about 2.2 million farms in the United States, which is a slight increase from the 2 million farms in 2006. However, it is a significant decrease from the nearly 4 million farms in 1960.⁵⁹ Between 1998 and 2010, the number of small farms decreased by about 49,000, while the number of larger farms (those earning more than \$250,000 annually) increased by roughly the same amount.⁶⁰

Of the 36 percent of U.S. farms in 2013, only 1 million showed a positive net cash income from the farm operation. Newer small farms tend to have more diversified production, fewer acres, lower sales, and younger operators who also work off-farm.⁶¹ In short, small family farms often rely on outside income from other occupations in order to supplement their household income. Moreover, approximately 45 percent of small family farms are categorized as residential/lifestyle (hobby farms) and 18 percent as retirement farms. Some of the

reasons behind the decline in farming become clear given that small family farms made up 88 percent of all U.S. farms in 2013. Conversely, large farms have the competitive edge and are more economically viable than the large number of small farms. This explains why farming is declining while agricultural output and value (particularly exports) are increasing.⁶²

- *Farm incomes.* In 2010, the average family farm received only 13 percent (\$10,031) of its household income (\$77,169) from farm sources, with the remaining 87 percent coming from off-farm employment or income. About 73 percent of off-farm income for the average farm operator household in 2010 came from other sources, such as wages and nonfarm businesses. Hence, farming has played an increasingly smaller role in determining the well-being of small farm households. Those farmers who remain on their land often face the prospect of working off the farm just to stay on the land.⁶³
- *The graying of U.S. farms.* The average age of the U.S. farmer is 58 (in 2013, 33 percent of principal small farm operators were 65 years or older), and more than half of all current farmers are likely to retire within the next decade. Less than 5 percent of farmers are under the age of 35. This raises the question of whether there is a younger generation ready to replace older farmers once they retire. There appears to be conflicting perspectives on whether the ageing of farmers is a problem. However, older farmers who own large commercial farms often have younger generations of children to inherit the farm once they retire. Nevertheless, the reality is that farmers aged 35 years or younger declined from 16 percent in 1982 to 5 percent in 2007.⁶⁴
- *Consolidation and control.* The 10 percent of family farms that grossed at least \$250,000 a year accounted for 80 percent of the value of production. Nearly half of all U.S. land is farmland (more than 1 billion acres) and by 1990, 4 percent of landowners held 47 percent of it. In California, the nation's largest agricultural producer, 3 percent of California farms controlled 60 percent of the market. Nationwide the figures were similar—7 percent of U.S. farms received 60 percent of the net cash farm income in 1992.⁶⁵

The USDA uses Gross Cash Farm Income (GCFI) as a benchmark to categorize farms. Small family farms generally have GCFI of less than \$350,000. In 2014:

- 13.6 percent (282,000) of all U.S. farms were small retirement farms where operators are retired but farm on a small scale.
- Close to half (45 percent or 943,000) are off-farm occupation farms. These are small farms whose operators have a major occupation other than farming.

Farming-occupation farms are small farms where farming is the major occupation.

- 25 percent (525,000) of all U.S. farms report GCFI of less than \$150,000.
- Only 5.3 percent (109,000) U.S. farms have a GCFI of between \$150,000 and \$349,999.

Mid-size farms

- Only 6 percent (124,000) of U.S. farms qualify as mid-size farms with a GCFI between \$350,000 and \$999,999.

Large-scale family farms (GCFI of \$1,000,000 or more)

- Only 3 percent (63,000) of U.S. farms qualify as large family farms with a GCFI between \$1,000,000 and \$4,999,999.
- Only 0.03 percent (6,850) of U.S. farms qualify as very large family farms with a GCFI of \$5 million or more.

Nonfamily farms—any farm where the operator does not own a majority of the business

- 1.1 percent (23,000) of U.S. farms are nonfamily farms.

Family farms and farm production.

- Although small farms control almost half of U.S. farmland, they only account for 22 percent of production. Mid-size and family farms account for 68 percent of farm production. Taken together, the various forms of family farms account for 99 percent of farms and 90 percent of production. Although nonfamily farms constitute only 1 percent of farms, they account for 10 percent of farm production.

Different types of farms account for the production of various agricultural commodities.

- Large-scale family farms account for almost 75 percent of dairy production.
- Mid-size and large-scale family farms control 81 percent of cotton production, 76 percent of cash grains and soybeans, and 66 percent of pork production.
- Large nonfamily farms control the high-value crops like fruits and vegetables (81 percent), beef (63 percent), and nuts.
- Although small farms produce half the poultry, they are largely under contract to large corporations.

Farming is graying as 33 percent of farm operators were at least 65 years old.

Farm finances—small farms are more likely to be in deficit and have a higher risk of financial problems, with 53 to 76 percent in the red.

Source: Based on Robert Hoppe and James MacDonald, *America's Diverse Family Farms*, USDA, Economic Research Service, Economic Information Bulletin, 2015 Edition, Number 146, December 2015. Retrieved February 2016 from <http://www.ers.usda.gov/publications/eib-economic-information-bulletin/eib-146.aspx>.

FIGURE 17.3 Snapshot of American Farming

- *Women and farming.* Ownership and control in U.S. agriculture contains distinct gender bias. Despite the fact that the number of female farmers more than doubled from 1982 to 2015, by 2015 women still only comprised 14 percent of U.S. farm operators. When women do own farms, they tend to be smaller since the average size of properties held by men is one-third larger than farms held by women.⁶⁶ Consequently, 78 percent of female farmers operated farms that earned less than \$10,000 a year.⁶⁷
- *Rural health and food quality.* Many rural residents are exposed to chemicals and excessive

odors from factory farms and often suffer from nausea, vomiting, coughing, and headaches. In extreme cases, people living near factory farms have developed neurological diseases and women have had miscarriages as a result of water and air contamination. Employees working inside factory farms have died from exposure to manure lagoons. The factory farms' extreme confinement and increased levels of production of animals intensify the opportunity for contamination of meat and poultry with bacteria, such as *Escherichia coli* and *Salmonella*. Crowded livestock conditions lead to a reliance on antibiotics to maintain

animal health, and more than 70 percent of all antibiotics consumed in the United States are fed to healthy farm animals. This indiscriminate use of drugs has contributed directly to the evolution of antibiotic-resistant bacteria that the American Medical Association considers an impending public health crisis.⁶⁸

In April 2012, the U.S. Food and Drug Administration (FDA) issued nonbinding industry guidelines containing two primary recommendations. First, farmers and ranchers should restrict the use of antibiotics to cases where drug intervention is necessary for farm animal health. The FDA considers antibiotics used for growth promotion or to increase efficiency in feed requirements as inappropriate. Second, antibiotics should be administered under the supervision of veterinarians.⁶⁹

- *Farm subsidies.* From 1995 to 2012, the largest and wealthiest top 10 percent of farm program recipients received total payments over 15 years of \$23,392. In turn, the bottom 80 percent of farmers received an average total payment of just \$586. Sixty-two percent of U.S. farmers did not receive any subsidy payments.⁷⁰
- *Efficiency.* Because family farmers often farm small acreages using greater crop diversity and careful management of natural resources, they are more efficient than large, industrialized food producers. Moreover, per unit costs for agricultural production are no better in larger commercial operations than in family farms. Factory farms add many real costs, such as increased use of fossil fuels, environmental damage, health threats, and threats to the safety of the food supply.⁷¹

Farmworkers

The efforts of millions of farmworkers are essential to the multibillion-dollar U.S. fruit and vegetable industry. Because agricultural production depends on the influx of seasonal labor, each year anywhere from 800,000 to just over 1 million farmworkers leave their homes to follow the crops.⁷² The wages and conditions they labor under are in many cases scandalous.

In 2015, the U.S. crop industry was worth \$186 billion (a drop of \$47 billion from 2013).⁷³ Most crop produce is cultivated by hand through the

labor of America's farmworkers. Although some Americans assume that farmworker poverty results in lower food prices, Eric Schlosser argues that the current level of farmworker poverty only saves the average American household around \$50 a year.⁷⁴

Thousands of migrant farmworkers risk death and incarceration each year by crossing the U.S.–Mexico border. In the blistering Arizona heat, 14 immigrants died in 2001 after smugglers abandoned them. In 2003, 17 immigrants, including a five-year-old boy, died of dehydration in the back of an abandoned truck in Texas. Despite the danger, illegal immigration rates are high, and somewhere between 48 and 70 percent of the 1.2 to 1.75 million farmworkers are undocumented.⁷⁵

As of 2010, roughly just over 1 million year-round and seasonal migrant workers worked in the United States, a number that is declining each year.⁷⁶ About two-thirds are immigrants, of whom approximately 68 percent are from Mexico. Roughly 45 percent of all farmworkers are hired for the full year, with the remainder being seasonal and part-year workers.⁷⁷ The following represents an overview of U.S. farmworkers based on data gathered between 2005 and 2010.

- *Demographics.* Sixty-one percent of all farmworkers (crop and livestock, year-round and seasonal) were U.S. citizens in 2010. The average age of farmworkers was 35, and 32 percent were over the age of 44. Around 84 percent of farmworkers were men, and 54 percent were married.⁷⁸
- *Education and language.* Eighty-one percent of farmworkers spoke Spanish, and 24 percent reported they could speak English well. In turn, 70 percent of farmworkers said they couldn't speak English at all or could speak English only a little. Twenty-six percent of farmworkers had not completed grade 12.⁷⁹
- *Labor and conditions.* Due to the seasonal nature of their work and the illegal or temporary status of migrants, farmworkers (even year-round workers) are particularly vulnerable to exploitation through poor working conditions.⁸⁰ They were also prone to under-employment, and on average, they only spent 66 percent of the year working on the farm. Employer-provided health insurance was also rare; only 8 percent of farmworkers claimed that they were insured, a number that declined to 5 percent for seasonal workers.⁸¹

- *Wages and poverty.* Growers employed 79 percent of farmworkers, while labor contractors employed the remainder. Farmworkers had an average of 12 years of U.S. farm experience; 41 percent had worked more than 10 years. On average, they were employed with their current farm employer for four and a half years and 60 percent had only seasonal work. The average farmworker worked about 42 hours a week and 79 percent were paid by the hour. They earned between \$10,000 and \$12,499 and had a total family income between \$15,000 and \$17,500—30 percent had family incomes below the poverty line.⁸²

According to the USDA, nonsupervisory farm laborers received an hourly wage of \$10.22 in 2010; however, this includes year-round as well as seasonal workers and is comparable to the hourly wage of maids and housekeepers (\$10.17).⁸³ These low wages are particularly striking since the largest 1.5 percent of U.S. farms employ more than half of the farm labor. Archer Daniels Midland, the world leader in the production of soy meal, corn, wheat, and cocoa, had \$4 billion in operating income in 2011.

- *Use of social services.* Despite the fact that many crop farmworkers qualify for Medicaid and SNAP, the migratory nature of seasonal work (traveling across state lines) makes it difficult for farmworkers to be approved since states vary in their eligibility criteria.⁸⁴
- *Housing.* Although rural housing programs to improve farmworker housing conditions are becoming more prevalent, farmworkers still often live in substandard accommodations. In 2008, a North Carolina study found that 89 percent of migrant labor camps had more than one Migrant Housing Act violation, and that 78 percent of participants felt that their habitation was overcrowded. A 2007 California study in the Coachella Valley found that 30 percent of migrant farmworkers lived in unsuitable habitats (i.e., sleeping outdoors, in vehicles, or in garages).⁸⁵

Farmworkers are also particularly susceptible to the effects of occupational workplace health and safety violations. These workers frequently come into contact with pesticides containing dangerous chemicals that can harm both the worker and their family. A 2006 study on California farmworkers

found that many were experiencing acute symptoms of pesticide poisoning, including headaches, nausea, eye irritation, and muscle weakness. Long-term exposure to pesticides can result in chronic health conditions, including cancer, memory disorders, and depression. Many employers fail to ensure that farmworkers receive adequate occupational health and safety education, and/or they fail to provide proper safety equipment and procedures. In another Californian study, 92 percent of 213 farmworkers were found to have pesticide contaminants in their urine.⁸⁶

Farmworkers are also often exposed to long hours in the sun without adequate hydration and sun protection, exacerbating the risks of heat stress, heat stroke, and melanoma. A 2005 survey found that 20 percent of farmworkers had no access to drinking water while working. Farm tools and machinery also present risks of injury and death, in 2012, 274 farmers and farm workers died from a work-related injury, resulting in a fatality rate of 20.2 deaths per 100,000 workers. Every day, about 167 agricultural workers suffer a lost-work-time injury.⁸⁷

The number of farmworkers that need housing often exceeds the available housing units. In addition, migrant farmworkers face barriers in obtaining private housing. For example, rural communities may not have enough rental units available, or they may be unwilling to rent to migrant farmworkers because they cannot provide deposits, qualify for credit checks, or make long-term rental commitments.⁸⁸

Education is also problematic for migrant farmworkers. Constant mobility makes it hard for the children of farmworkers to complete their education. Children who move often are two and a half times more likely to repeat a grade than children who are stationary. Changing schools is emotionally difficult, and youngsters are more likely to drop out of school if they change schools four or more times. Educators who work with migrant children say that 55 percent of these children graduate nationwide. Migrant children also face intense economic pressure to drop out of school.⁸⁹

The poverty experienced by migrant families requires that all able family members work. In fact, agriculture is the only industry that allows workers under the age of 16. The Fair Labor Standards Act sets age 12 as the legal limit for farm work, with exemptions available for children as young as 10 or 11. It has been estimated that almost 432,000 children aged 12 to 17 work on farms, while many

children work “off the books” if they are under age, or use their parent’s Social Security number.⁹⁰

Since children work and play in areas where chemicals are sprayed and machinery is present, they are exposed to the same occupational hazards as adults. Because of their lower weight and higher metabolism, children are more susceptible to the effects of toxic chemicals than adults. One study found that 48 percent of children had worked in fields still wet with pesticides, 36 percent had been sprayed either directly or indirectly (by drift), and farm operators had sprayed 34 percent of the children’s homes in the process of spraying nearby fields.⁹¹ Moreover, injuries involving tools and machinery and exploitative working conditions are also common for child farmworkers. A 2002 Human Rights Watch Study found that children as young as 12 often worked 14 hours a day during peak harvest season times. In 2012, an estimated 14,000 youth were injured on farms; 2,700 of these injuries were due to farm work.

As a result of the exposure to pesticides, farmworkers frequently suffer stomach ailments, headaches, rashes, burns, and other toxic chemical-related problems. Many die from environmentally caused cancer, and too often workers’ babies are born with severe birth defects. Many families must sleep, bathe, and cook near the fields where they work and have no option but to use water that may be contaminated with toxic chemicals.⁹² The following illustrates some of the health and safety problems faced by America’s farmworkers.

- One study found that about 34 percent of migrant children were infected with intestinal parasites and/or suffered severe asthma, chronic diarrhea, Vitamin A deficiency, chemical poisoning, or continuous bouts of otitis media leading to hearing loss. Other commonly reported health problems included lower height or weight, respiratory disease, skin infections, and undiagnosed congenital and developmental problems.
- Water-related parasitic infections afflict migrant farmworker adults and children on average 59 times more often than the general population.
- Death rates from influenza and pneumonia are as much as 20 percent and 200 percent higher, respectively, for farmworkers than the national average.
- In one CDC study, 44 percent of farmworkers had positive tuberculosis (TB) tests.

- It is estimated that as few as 42 percent of female farmworkers receive prenatal care during the first trimester of pregnancy, even though many migrant pregnancies are classified as high risk with multiple indicators. The infant mortality rate among migrant farmworkers is 125 percent higher than in the general population.⁹³

In the 1960s, the conditions faced by farmworkers led to the creation of the United Farm Workers union (UFW) in California, a movement led by the charismatic Cèsar Chávez. At its peak in the 1970s, the UFW had about 120,000 workers under contract, and between 1966 and 1980 it obtained two 40 percent wage increases in the grape industry, among other gains. But during the 1980s, contracts began to expire and successive Republican administrations stacked the California Agricultural Labor Relations Board with pro-industry members, effectively gutting the law that had created the board. In the midst of a hostile political climate and internal differences, the UFW went into a membership slump. By 2015, the UFW had only a few thousand members under contract.⁹⁴

Farm labor contractors hire 20 percent of farmworkers nationwide and 40 percent in California. The 1980s saw the rise of independent labor contractors as key players in the farm industry. Hired by growers, these middlemen round up laborers and deliver them to the fields. Contractors pay less than growers, offer few benefits, and provide little job security. They also have a reputation for mistreating farmworkers and cheating them in wages. Contractors shield large growers from labor actions and legal claims, distancing them from the actual hiring of workers.⁹⁵ If a judgment comes down against a labor contractor, the contractor often vanishes or puts the businesses into bankruptcy, only to emerge in a new place or under a new name. The labor subcontracting system is an important obstacle to improving the welfare of farmworkers. One issue being pressed by farmworker activists is the creation of legislation that would make growers legally responsible for their workers, regardless of whether they use contractors.

Issues in American Farming

There are a number of important issues affecting U.S. farming, including the corporatization of agriculture, genetic engineering, global trade, food

safety, local selling, organic farming, sustainable agriculture, and climate change.

The Corporatization of American Farming

U.S. family farms are rapidly being replaced by corporate farms. Corporate agriculture is a system whereby the farm owner, the farm manager, and the farmworker are different people. This system represents a dramatic shift from family farms, where the individual farmers make the decisions. Industrial agriculture encourages large-scale, highly specialized farms where uniformity is emphasized over quality. Large-scale agriculture contains significant environmental risks. For example, hogs produce three times the waste of humans, and the 500,000 pigs housed in Smithfield Foods Utah subsidiary generate more fecal matter each year than the 1.5 million inhabitants of Manhattan. Jeff Tietz estimates that Smithfield's total waste discharge of 26 million tons a year would fill four Yankee Stadiums.⁹⁶

Corporatization leads to closed markets where prices are fixed not by open, competitive bidding but by negotiated contracts. Small volume producers are discriminated against in price or other terms of trade. Moreover, in traditional food markets, 91 cents of every dollar spent on food goes to suppliers, processors, middlemen, and marketers, most of whom are not based in the community. As such, less than 10 cents of each dollar goes directly to the farmer.⁹⁷

An integral part of corporate farming is vertical integration, whereby agricultural corporations control all aspects of production, including raising, owning, slaughtering, and marketing livestock or agricultural products. One example is ConAgra, the largest distributor of agricultural chemicals in North America and one of the largest fertilizer producers in the world. ConAgra's sales and revenue was just over \$16 billion in 2015. In 2007, the company owned more than 100 grain elevators, 2,000 railroad cars, and 1,100 barges. It was the largest of the three corporations that mill 80 percent of North America's wheat. ConAgra was also the largest turkey producer and the fourth largest broiler producer, producing its own poultry feed as well as other livestock feed. The company hired growers to raise its birds and then processed them in their facilities. This poultry was then purchased as fryers, under the name of Butterball, or in

further processed foods such as TV dinners and pot pies under the label of Banquet, Marie Callender's, or Healthy Choice. ConAgra also owns 62 well-known brand names such as Act II Popcorn, Rosarita, Banquet, Bertolli, Blue Bonnet, Chef Boyardee, David Sunflower Seeds, Egg Beaters, Fleischman's, Gulden's, Healthy Choice, Hebrew National, Hunt's, Jiffy Popcorn, Libby's, Marie Callender's, Orville Redenbacher's, PAM, Parkay, Peter Pan, Ranch Style, Reddi-Whip, Rosarita, Slim Jim, Swiss Miss, Van Camp's, Wesson, P.F. Chang's, Wolf and Dennison's Brand Chili, La Choy, and Egg Beaters.⁹⁸ Many smaller farmers who cannot participate in "vertical integration" are forced out of business because they have no place to sell their products. Some of this trend is counteracted by the farm-to-table movement.

Genetic Engineering

Genetic engineering, which sparked a global controversy since its introduction in 1995, is the manipulation of specific genes that are moved from one species to another to create a trait that did not previously exist. For example, crops such as corn have been engineered to contain pesticides in every cell of the plant. As a result, these crops are not registered as food and are actually considered pesticides.⁹⁹ Although the Food and Drug Administration claims genetically modified (GM) products are safe, there has been no thorough analysis of their long-term effects. Critics argue that GM crops are increasingly threatening the biodiversity in the seed supply and making crops more vulnerable to disease outbreaks and pest infestations. Two multinational companies, Monsanto and DuPont, dominate the U.S. seed industry.¹⁰⁰

In 2012, California voters rejected Prop 37, which would have required the labeling of made with genetically modified ingredients. Companies like Monsanto and Hershey's contributed \$44 million to defeat Prop 37. The ostensible argument was that GMO labeling would cost consumers more and punish businesses, including small farms. Interestingly, countries like China, Saudi Arabia, Russia, Japan, India, and Chile are just a few of the 64 nations that require GMO labeling.¹⁰¹

In contrast to the United States, the European Union recognizes the consumers' right to food information. Since 1997, it has made labeling GM food mandatory for products that contain GMO, and products derived from GMO if there is still

GM DNA.¹⁰² As of 2012, no GM crops were grown commercially in New Zealand, and any GM food imported into the country would require approval under the Hazardous Substances and New Organisms Act.¹⁰³ Most European consumers have shied away from buying GM-labeled products.¹⁰⁴

Farmers buy GM seeds based on the promise of lower costs and higher yields, but they often find additional costs in veterinary bills, medications, unstable markets, and extra pesticides. Specifically, farmers that buy GM seeds enter into a contract that dictates how and when the crop can be grown. This contract also forbids the farmer to save any seeds, which is contrary to traditional farming practices. Some farmers have been sued for allegedly saving seeds, while neighboring farmers whose crops have been contaminated by GM pollen drift have been sued by Monsanto for unknowingly possessing GM seeds.¹⁰⁵

Global Trade

The global trade in food has emerged as a serious threat challenging family farmers in the United States and around the world. Agricultural free trade agreements, like the North American Free Trade Agreement (NAFTA)—which linked the economies of the United States, Mexico, and Canada—promote agricultural trade with little regard to its impact on local communities and family farmers. As a result, several countries, including the United States, have been flooded with cheap food imports that have caused small farmers to lose their local markets, and family farmers worldwide have been forced off their land. “Food dumping” weakens local food production.¹⁰⁶

Since the passage of NAFTA in 1994, family farmers in all three countries have felt the negative impacts of a free trade agreement designed to benefit agribusiness. In the United States, 100,000 family farmers were forced out of farming between 1996 and 2001. During that same period, Canada lost 11 percent of its family farms.¹⁰⁷ NAFTA and other trade liberalization policies have dramatically increased rural poverty and hunger in Mexico. Between 1992 and 2002, the number of rural Mexicans living in extreme poverty grew from 36 to 52 percent. Almost 2 million subsistence farmers were pushed off their land and migrated to export factories, maquiladoras, or to fields and cities in the United States. NAFTA critics estimate that as many as 15 million more Mexican farmers—one in

six—could be displaced.¹⁰⁸ Despite the issues associated with global free trade, the export industry is big business. In 2009, the Obama administration announced a series of aggressive export strategies (called the National Export Initiative) to strengthen domestic agricultural profits and employment. The goal was to double U.S. exports by 2014. Farm exports reached a new record high of \$137.4 billion in 2011.¹⁰⁹ The impact of the Trans-Pacific Partnership trade agreement on agriculture remains unknown.

Food Safety

Food safety has become a troubling concern for many Americans. In January 2009, the Peanut Corporation of America issued one of the largest food recalls in history for peanut products containing salmonella. The tainted peanuts caused eight deaths and more than 500 illnesses in 43 states. The recall included 2,100 products from more than 200 companies. In June 2009, Nestlé recalled 300,000 cases of refrigerated cookie-dough because of *E. coli* contamination. Kraft Foods recalled almost 53,000 pounds of Oscar Mayer/Louis Rich Chicken Breast Strips after listeria bacteria was detected. The next month, an additional 2.8 million pounds of chicken were recalled. The largest recall of beef in U.S. history occurred in February 2008 when Hallmark recalled 143 million pounds of beef. The recall occurred after a video showed the slaughterhouse mistreating cows and allowing unhealthy cattle to be slaughtered without an inspection. In 2011, Cargill recalled 36 million pounds of ground turkey because of possible contamination by a drug-resistant strain of salmonella. The company then recalled another 185,000 pounds of meat due to the same contamination risk. In 2006, 205 people were stricken (five died) by *E. Coli* after eating contaminated spinach from a central California farm. Two Iowa egg-producers recalled 550 million eggs in 2010 after 1,500 cases were contaminated by salmonella. The FDA believed that the contaminated eggs were linked to unsanitary farm conditions and the small size of the chicken cages.¹¹⁰ All of the major food contamination episodes arose as a result of large-scale corporate manufacturing or mass production farming. In 2015 Chipotle’s food in two locations was found to have *E. coli*. Recent food recalls are listed in foodsafety.gov/recalls/recent.

Mad Cow Disease (Bovine Spongiform Encephalopathy, or BSE), is a neurological disease that

affects cattle by attacking the brain and central nervous system. It is believed that cattle become infected with BSE by eating feed that contains remnants of other infected cattle. Humans can become infected with a BSE-like illness, called Creutzfeldt Jakob Disease (CJD) by eating meat from infected cows. There is no known cure for CJD and cooking at high temperatures or treating meat with radiation is not effective in preventing the disease. Because organic livestock producers adhere to strict feed requirements, which exclude animal by-products in the feed, it is believed that BSE is absent in this kind of meat.¹¹¹ As a result of Mad Cow disease, some consumers have become wary of purchasing and eating beef.

Local Selling

Because many small farmers cannot compete with agribusiness, they are selling their products locally. Between 2008 and 2013, farmers' markets increased by more than 50 percent, and by 2013, they numbered 7,200. Community Supported Agriculture (CSA) programs that encourage local residents to buy produce directly from family farms have been driven by the "Know Your Farmer, Know Your Food" (KYF) program initiated by the former Obama administration.¹¹² A map displays farmers' markets across the United States, with specific addresses and information on each market. A search engine enables consumers to search for local markets in their ZIP and area code.¹¹³ Farmers can now sell more easily to schools through the Farm to School Team, which helps them navigate the administrative "red tape," thereby assisting more farmers to participate in programs such as the school meal program.¹¹⁴ For more information, visit the Know Your Farmer, Know your Food website at www.usda.gov/KnowYourFarmer/.

Organic Farming

Started by family farmers, the organic movement is the fastest growing sector of American agriculture. In 2015, organic sales in retail stores totaled almost \$40 billion, a 72 percent increase from 2008. In 2015, organic agriculture had a 4.2 percent share of the U.S. retail food sales market. Overall, the number of certified organic operators increased by 1,109 from 2009 to 2011, which represented more than a 6 percent increase.¹¹⁵ The trend toward organics and less processed food has created

confusion for consumers who are unsure of what the various labels mean (see Figure 17.4). Throughout the 1990s and into the twenty-first century, large agricultural corporations have increased their participation in organic farming, which has raised questions as to whether the values of these mega-corporations conflict with those of organic farming, a local movement that began primarily to improve people's access to natural, chemical-free produce. In fact, corporations have been criticized for exploiting what was originally a local niche market, as smaller competitors struggle to compete with large corporations that are jumping on the organic bandwagon. Often, large retailers like Walmart who undercut local retailers will source low-integrity factory-farmed organic products.¹¹⁶

Sustainable Development

The environmental challenges facing the United States are greater than at any time in the nation's history. Global environmental threats such as climate change, stratospheric ozone depletion, and the loss of biological diversity (through the accelerated extinctions of species), forests, and fish stocks are affecting all countries regardless of their stage of economic development. The definition of sustainable development proposed by the 1987 United Nations Brundtland Commission stated that **sustainable development** must "meet the needs of the present without compromising the ability of future generations to meet their own needs."¹¹⁷

In an ecological context, sustainable development promotes a process whereby natural resources are allowed to be replenished and future generations continue to have adequate resources to meet their needs.¹¹⁸ Some groups use sustainable development to designate a radical restructuring of society around environmental development and economic growth. Others simply use sustainable development to signal a change in attitudes and emphasis. These different approaches are sometimes labeled "dark green strategies" versus "light green strategies."¹¹⁹

Climate Change

Contrary to the skepticism of the American right, climate change has become an accepted fact among the vast majority of scientists and much of the general public. Environmental consciousness was boosted by Al Gore's documentary, *An Inconvenient Truth*.

- *Organic:* Regulated by the USDA, the National Organic Standards assure that food products contain at least 95 percent organic ingredients and that no synthetic growth hormones, antibiotics, pesticides, biotechnology, synthetic ingredients, or irradiation were used in production or processing. Organic labels can be found on produce, dairy, meat, processed foods, condiments, and beverages.
- *Fair trade:* Fair trade standards are enforced by the Fairtrade Labeling Organization International (FLO). Fair trade products are produced in accordance with several guidelines. For example, workers must receive decent wages; and housing, health, and safety standards must be adhered to. Workers must have the right to join trade unions, and child or forced labor is prohibited. Crops must also be grown, produced, and processed in an environmentally friendly manner. Fair trade standards have been established for coffee, tea, cocoa, honey, bananas, orange juice, and sugar. Some producers have established alternative fair trade-type labels. Most are little more than marketing pitches.
- *Free farmed:* This certification program was created by the American Humane Association in 2000 to ensure that animals raised for dairy, poultry, and beef products are treated in a humane manner. These guidelines ensure that livestock have access to clean and sufficient food and water as well as a safe, healthy living environment.
- *Feel good buying (not certified):* Some meat and dairy products are now being marketed as hormone free. In dairy products, this means that the farmer has chosen not to inject their cows with the artificial growth hormone called rBGH. On beef products, this label indicates that the animal was raised without growth hormones or steroids.
- *Raised without antibiotics:* This meat and dairy label indicates that the animal was raised entirely without the use of low-level and/or therapeutic doses of antibiotics.
- *GE free, non-GMO:* Food products that use GE Free or Non-GMO labels are regulated by individual companies, distributors, or processors.

Often, the companies require certification or affidavits from farmers that the materials were not genetically modified in any way.

- *All natural:* There is no universal standard or definition for this claim. Hence it has no meaning other than as advertising.
- *Free-range:* This label claims that a meat or poultry product (including eggs) comes from an animal that was raised in the open air or was allowed to roam. However, the USDA certification does not specify how much of each day animals must have access to fresh air. In poultry, the USDA considers five minutes adequate exposure to be considered free range. In beef, the use of the label is completely unregulated or standardized.

To shore up the government's slack in regulating the free range label, private animal welfare organizations like the ASPCA have adopted their own standards around free range and the humane treatment of animals. However, even within these standards, there is room for harsh treatment of these animals.

- *Cage-free:* Unlike caged or battery hens, cage-free hens are supposedly able to move, spread their wings, and lay their eggs in nests. However, most cage-free hens live in huge flocks with thousands of hens who never go outside. Not being confined to a cage does not always equate with humane treatment since most cage-free chickens are de-beaked like caged birds.
- *Pasture raised:* Is a sustainable agriculture technique where chickens and turkeys are raised in pastures, as opposed to indoor confinement.
- *Grass-fed:* Cows are fed on grass rather than grain in feedlots.

Source: Adapted from Farm Aid, Inc., "Issues in Farming," 2004. Retrieved November 2004 from www.farmaid.org/site/PageServer?pagename=info_facts_global. Used by permission. See also The Humane Society of the United States, *Cage-Free vs. Battery-Cage Eggs*, 2016. Retrieved February 2016 from, http://www.humanesociety.org/issues/confinement_farm/facts/cage-free_vs_battery-cage.html. See Angela Brady, *What Makes a Chicken Free-Range?* SF Gate, nd. Retrieved February 2016 from <http://homeguides.sfgate.com/chicken-freerange-79177.html>.

FIGURE 17.4 What the Food Labels Mean: Real v. "Feel Good" Food Labeling

The effects of climate change in the United States and elsewhere is striking. The earth is warming, and its average temperature has risen by 1.5°F from 1880 to 2012. It is projected to rise another 2 to 6°F over the next hundred years. Even small changes in the average temperature can result in large and potentially dangerous shifts in climate and weather. The 10 hottest years ever recorded were from 1998 to 2015. The hottest year was 2015, which beat 2014, the second hottest year by a wide margin.¹²⁰

Climate change is not only marked by hot weather but also by wild weather swings, including extremely cold weather (i.e., the Northeast and Midwest winters of 2013 and 2014), floods, droughts, and storms. Some examples include California's severe four-year drought from 2011 to 2016. Except for 2011, California had below-average precipitation every year since 2007. This became a crisis in 2014, the hottest and driest year on record. The destruction wreaked by climate change is not limited to the United States alone. In 2012, a deadly drought was causing suffering for more than 12.5 million people in 13 countries in the Eastern Horn of Africa region.

By August 2012, Arctic sea ice broke the record low set in 2007, and dropped below 1.5 million square miles. That represents a 45 percent reduction in the area covered by sea ice compared to the 1980s and 1990s, and may be unprecedented in human history. The extent of sea ice that melted by mid-2012 was equivalent to the size of Canada and Alaska combined.¹²¹ Although the arctic ice extent bounced back in 2013, by 2015 it was only 1 percent above the 2012 low point.¹²²

Climate change is causing rising sea levels, dramatic glacial retreat, arctic shrinkage exceeding even some of the most aggressive computer projections, significant slowing of the ocean circulation that transports warm water to the North Atlantic, and altered patterns of agriculture (prolonged droughts punctuated by extreme flooding). Other predicted effects include extreme weather events (hurricanes, cyclones, floods, intense droughts), an expansion of tropical diseases as subtropical areas become warmer, and drastic economic changes, including starvation as marginal crop lands become deserts. Overall, the consequences of global warming result from one of three changes: rising sea levels, higher local temperatures, and changes in rainfall patterns.

Lastly, climate change is having a major impact on farming and food production. The greenhouse effect—a naturally occurring phenomenon—is the way the atmosphere traps part of the sun's heat and stops it from going back into space. It makes the earth warm enough for life, and without it, the earth would be about 86°F colder. Although global warming is a common term used to describe these changes, climate change is a better descriptor since some areas may in fact cool.¹²³

Most scientists believe that increased emissions of greenhouse gases have contributed to the rise in global temperatures and sea level. These scientists maintain that we are adding dangerously to the natural greenhouse effect as the gases from industry and agriculture—chiefly carbon dioxide, methane, and nitrous oxide. Some scientists maintain that the earth is warming faster than it has in the last thousand years.¹²⁴

The Kyoto Protocol was a global treaty on tackling climate change. It committed industrialized nations to reduce their emissions of six greenhouse gases by an average of 5.2 percent below their 1990 levels within a decade. Scientists maintained that it would take carbon cuts of 60 percent or more to prevent dangerous climatic instability. Hence, Kyoto was only a modest start.

The Bush administration rejected the Kyoto Protocol in 2001 claiming that the agreement was fatally flawed because it excluded developing nations, such as China and India. Bush maintained that forcing U.S. businesses to reduce their emissions while letting companies in those countries off the hook would cost American jobs. Of all the industrialized Western nations (and the 191 ratifiers), only the United States had refused to sign the Kyoto Protocol. Despite the U.S. rejection, the Kyoto accord took effect in 2005 as a result of Russia's ratification. Key provisions of the Kyoto Protocol expired in December 2012.¹²⁵

A major strategy employed to slow the effects of climate change has centered on a carbon tax. This strategy taxes the carbon emissions of large and small industrial producers and polluters. The intent is to drive the price of carbon emissions so high that it will spur on the growth and adoption of less expensive alternative energy sources. As carbon emissions become more highly taxed, other energy sources become more attractive, and with greater production and development, prices will invariably drop. There is some logic in this approach since the

average price of solar panels (per watt) dropped 97 percent from 1975 to 2012.

Part of this strategy also involves an emissions trading scheme based on carbon credits. A carbon credit is a term for any tradable permit that allows a plant to emit 1 ton of carbon dioxide or another greenhouse gas. Specifically, each plant is given (often generous) free permits—that is, carbon credits or Certified Emissions Reductions (CER)—to meet their emission targets. Targets are based on a proportion (somewhere around 90 percent) of a plant's historic emissions level. Companies can trade or buy CERs from other companies or even from other countries. These carbon credits became a traded commodity in a kind of carbon exchange.¹²⁶ In theory, a company could buy a cheap CER from a developing country for \$4 and sell it back to another company or country for \$12.¹²⁷ However, the ultimate goal is carbon reduction.

In late 2015, world leaders finalized a climate change plan in Paris that was more aggressive than the defunct Kyoto Protocol. The result was a single agreement among 200 countries to address climate change by cutting greenhouse gases. The goal was to keep temperatures below the 2C in preindustrial time. Leaders of low-lying developing countries facing massive flooding wanted an even more aggressive target of 1.5C. The goal of the Paris agreement is to limit the greenhouse gases emitted by human activity to the same levels that trees, soil, and oceans can absorb it naturally, something that would occur sometime between 2050 and 2100. If fully implemented, this would bring greenhouse-gas emissions to near zero within a few decades.

The need for developing countries to receive financial and technical aid to reach these climate goals was omnipresent in the negotiations. This was especially true if they intended to bypass carbon dependency and go straight to renewable energy sources. In turn, rich nations have promised \$100 billion a year in aid by 2020.

Despite good intentions, the commitment to cut greenhouse gases is voluntary, and much of the promised funding and the technical aid will partly hinge on the strength of the world economy and the price of

oil. If a barrel of oil is cheap, the incentive to create robust alternative energy sources is weakened.¹²⁸

Conclusion

The production, distribution, and consumption of food has historically been mired in politics. Employing various forms of legislation, the U.S. government has traditionally responded to the needs of diverse groups that have an interest in food—farmers, consumers, the poor or their advocates, food distributors, grain traders, and Third World and wealthier countries that depend on U.S. food production.

The federal government has responded to these interests by creating a patchwork quilt of policies and programs. One of the most important is SNAP, an ingenious strategy that helps keep food affordable for low-income consumers, helps stabilize farm prices, slows down agricultural surpluses by subsidizing consumption, and allows food retailers and distributors to increase their profits by ensuring a volume of subsidized consumers. Despite well-intentioned government programs, the nation's food problems persist. In some cases, these food-based problems have reached crisis proportions for marginal small farmers, the very poor, and farmworkers. Tragically, many U.S. farmers now profit from food stamps—not because the program helps them control surplus farm goods—because it provides them with food. It is a sad commentary when the producers of food are unable to purchase what they grow.

DISCUSSION QUESTIONS

1. WIC and the SNAP program are similar in many respects. What are the specific differences between them? Is it necessary for WIC to be a separate program?
2. Serious questions exist as to the effectiveness of U.S. food programs for the poor. What alternatives, if any, exist to the current matrix of food programs that make up the nutritional safety net?
3. American farming has historically been economically volatile. For example, farming was reasonably good in the 1970s, after which it fell into a depression in the early and mid-1980s. What programs and policies might help stabilize U.S. farming?
4. Climate change and sustainability are two sides of the same coin. Some critics question climate change as pseudo-science, while others see it as the most pressing issue facing civilization. How valid are these arguments?
5. The problem of how to deal fairly with farmworkers in the United States is both difficult



Watch this video on climate change.

www.youtube.com/watch?v=fcwniis_Zm8

and chronic. What are some possible solutions for promoting equity and fairness for farmworkers?

6. Proponents of biofuels see it as a potential solution to America's energy dependence on imports. Supporters argue that the production of ethanol would decrease the foreign trade imbalance, eliminate agricultural surpluses, reduce agricultural subsidies, and lower the emission of hydrocarbons that contribute to global warming. Are they right?

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The American Welfare State in International Perspective



Source: Jose Cendon/AP Images

Various models have evolved to depict the comparative levels of development of nations. In recent times, a three-part classification has enjoyed extensive use: a First World, consisting of the industrial nations of the capitalist West; a Second World, made up of the communist nations that constructed political economies as an alternative to the market-dominated First World; and a Third World, comprising nations that were former colonies of the First World and later achieved independence, often through revolutions of liberation. This tri-part formulation became prevalent after World War II as the Cold War intensified. Many developing nations adopted the slogans of revolution, in order to shed the influences of First World colonial nations. The Second World viewed the Third World as a theater of independence, an arena in which the exploitation of capitalism would be summarily ended, and the colonized nations would be brought into the communist sphere of influence. On the defensive, the First World deployed foreign aid, technical assistance, cultural exchange, and, on occasion, diplomatic subterfuge to neutralize Second World incursions into the Third World. In several regions—Central America, Southeast Asia, and sub-Saharan Africa—the First and Second Worlds recruited insurgents who acted as their surrogates, engaging the opposition in armed conflict at considerable cost in arms and human life.

With the fall of the Berlin Wall in 1989, the Three-World formulation lost utility. Foremost, the collapse of the Soviet Union and its Warsaw Pact satellites halved the scale of the Second World. The remaining communist nations—the People's Republic of China, Vietnam, North Korea, and Cuba—posed little immediate threat to the First World and were unlikely to serve as models for Third World nations. At the same time, some nations of the Third World had been transformed substantially from their colonial era status. Several Arab nations, despite feudal forms of governance, had prospered through oil exports, achieving levels of income that mirrored those of industrial nations. And despite the absence of natural resources, a handful of nations in Southeast Asia—Hong Kong, South Korea, Taiwan, and Singapore—experienced such consistently high levels of growth that they became known as the “Four Tigers.” Tragically, many Third World nations lost ground. By the end of the millennium, several of the nations of sub-Saharan Africa and Southeast Asia were significantly less developed than they had been a generation earlier,

when they achieved independence from the First World. Nations such as Cambodia, Bangladesh, Afghanistan, Somalia, and Sierra Leone had lost so much capital, their infrastructure had deteriorated to such an extent, and their polity had become so unstable that development analysts began to speculate about the emergence of a **Fourth World**. Further confounding the prospects of Fourth World nations, the end of the Cold War led the affluent industrial nations of the First World, particularly the United States, to expend relatively less on foreign aid than they had during the Soviet threat.

Typologies of Welfare States

The welfare states of the industrialized West vary structurally. With his colleague Charles Lebeaux, Harold Wilensky sought to summarize the differences between the European and American welfare states by constructing a typology of social welfare systems. The typology contrasts residual welfare with institutional welfare. The residualist approach is concerned with providing a minimal safety net for the poorest sections of the population rather than catering to the population as a whole. Residualists argue that people can enhance their well-being through their own efforts, through the help of families or neighbors, by purchasing services on the market, or by obtaining help from voluntary organizations.¹ They point out that in the United States people make effective use of nongovernmental agencies and that international comparisons should not be limited to government social programs. They also point out that although the United States may lag behind other developed nations in public welfare provision, people enjoy exceptionally high standards of living and unequalled opportunities. This is why the country remains a magnet for immigrants from all over the world—people who come not to receive government handouts but to share the American dream. Wilensky and Lebeaux believed that social policy in the United States is essentially residualist in nature. On the other hand, the **institutional approach**, which typifies European social welfare, seeks to provide a variety of social programs for the whole population and to combine economic and social objectives in an effort to enhance the well-being of all.²

Gosta Esping-Andersen's typology also recognized the corporatist type when classifying welfare states. In addition to a corporatist category (Italy, Japan, France, and Switzerland), Esping-Andersen

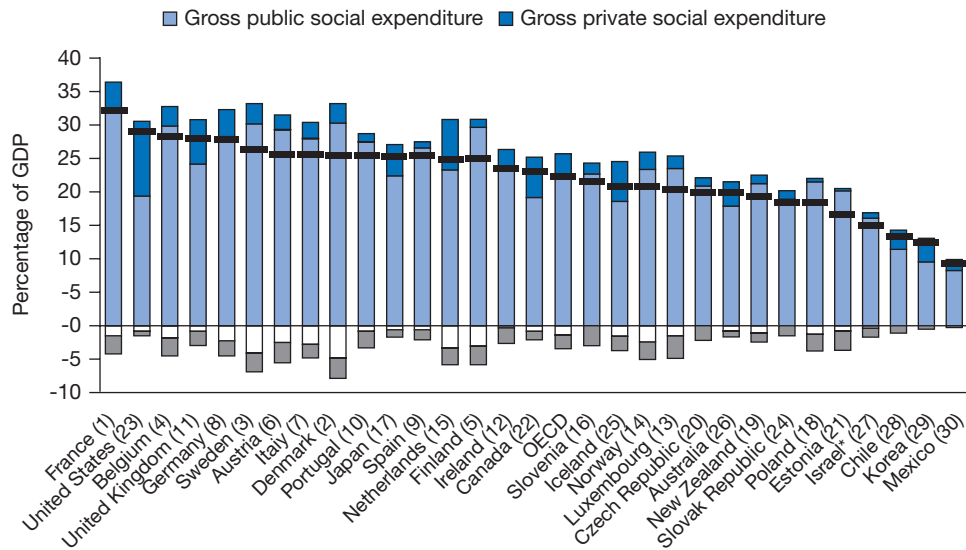


FIGURE 18.1 Net Social Spending, OECD Countries⁴

Source: OECD (2014), "Social Expenditure Update—Social spending is falling in some countries, but in many others it remains at historically high levels"—Figure 7—via www.oecd.org/social/expenditure.htm.

identified two other types—the liberal welfare state (Australia, Britain, and the United States) and the social democratic welfare state (Austria, the Netherlands, and the Scandinavian countries).³ Subsequent development of Japan as well as Brazil, Russia, Indonesia, and China (the BRICs) have led comparative analysts to acknowledge further variation in the way nations provide for citizens.

Although the typologies differ, they use similar criteria when classifying countries. All emphasize government social programs and neglect the role of voluntary organizations and other nonstatutory activities in social welfare. Although the typologies are intended to classify countries for analytical purposes, it is clear that they give expression to beliefs about the desirability of the role of government in social welfare. Analysts who favor extensive government involvement in social welfare have represented countries with extensive public programs more favorably in the typologies than countries that do not place as much emphasis on government involvement. For this reason, it is not surprising that most of the typologies depict the United States negatively in comparison to European welfare states.

Social commitments vary with respect to the size of the voluntary sector, as noted in Figure 18.1.

Note that the United States compares unfavorably to European nations by governmental expenditures alone, ranking 23rd; however, once private sector expenditures are included, the United States rises to 2nd.

American Exceptionalism

The apparent unwillingness of the United States to emphasize government social welfare programs has been described by Edwin Amenta and Theda Skocpol as **welfare exceptionalism**.⁵ These authors also summarized explanations of the country's reluctance to create an institutional European-style welfare state. One such explanation is that the racial, ethnic, and religious diversity of the United States has prevented the emergence of a comprehensive welfare state. In European nations, people are more united and have a stronger sense of civic responsibility; in contrast, the pursuit of separate interests by a great number of different groups in the United States militates against the emergence of a single national system of provision that caters to all citizens.

Another explanation is based on the country's high degree of political decentralization that impedes the emergence of strong central political institutions. Combined with a high degree of diversity, the tradition of federalism in which the states assume much responsibility for social welfare, creates cleavages in U.S. society that effectively prevents the emergence of a strong, centralized, and comprehensive welfare state. It has also been noted that the United States does not have a long tradition of bureaucratic government that can support centralized welfare programs. Some have claimed that this is because of the absence of a feudal tradition. Developing this idea, others have pointed to

the unique role of the courts in policymaking in this country, the separation of the executive and legislative branches, and the role of powerful political interest groups—all of which impede the emergence of a strong, federal welfare state. Another factor is the role of individualism in the United States, which is fundamentally antithetical to the collectivist assumption underlying the welfare state. It has also been argued that trade unions are weaker in the United States than in Europe and that the political left, which has played a major role in the emergence of the European welfare states, has not been strong in the United States.

Although the notion of welfare state exceptionalism offers interesting insights into social policy in the United States, it can be criticized. For example, the idea that the United States is a welfare laggard is based largely on a comparison of social programs, such as health insurance and family allowances. The absence of these programs is usually emphasized by those claiming that the United States does not have a comprehensive welfare state. Although it is true that government health care and family income maintenance programs are poorly developed in the United States, the role of indirect support for

families through tax relief and tax deductions for medical expenses is often ignored, as is the significance of Medicare and state and local health care programs. Jacob Hacker evaluated the extent to which private sector health and welfare benefits effectively augmented public social programs, concluding that the inclusion of the private sector put the United States on par with European welfare states. On public benefits alone, the United States expended 17.1 percent of gross domestic product (GDP), less than half that of Denmark's 37.6 percent. An analysis of after-tax public and private social welfare expenditures showed a different picture, however; the United States expended 24.5 percent of GDP compared to Denmark's 24.4 percent.⁶ Thus, the United States is different from other advanced nations with respect to social welfare provision because of its reliance on the private sector.

In addition, the United States has excelled in other social fields. Historical research reveals that in the late nineteenth century, the United States was ahead of other Western countries in developing income maintenance programs for veterans and for women with children. At the turn of the century, no other industrializing country had introduced



During the nineteenth century, the United States led the world in the development of public education. Today, it still compares favorably well with many other countries in terms of access to education, particularly at the college and university level.

Source: WavebreakMediaMicro/Fotolia

mother's pensions, and none came as close to creating a "maternal" rather than "paternal" welfare state.⁷ During the nineteenth century, the United States led the world in the development of public education; and it still compares favorably with many other countries in terms of access to education, particularly at the tertiary (college and university) level. Comparative studies have also shown that when particular social programs such as retirement pensions are compared, the United States fares quite well.⁸

Thus, studies of welfare policies in other parts of the world have also shown the U.S. welfare state in more favorable terms. For example, unlike the United States, Australia did not until recently have a universal Social Security system, and it relied extensively on means-tested social programs.⁹ Nevertheless, it is difficult to reach the conclusion that the United States is one of the world's welfare leaders. Despite its extensive educational and Social Security provisions, America does not compare favorably to the other industrial nations in the extent, comprehensiveness, or coverage of its welfare system. In fact, its position has deteriorated in recent years as social programs were retrenched since the Reagan administration.

The Welfare State in Transition

Western industrialized nations pursued liberal social policies after World War II.¹⁰ In the United States, most presidents following Franklin D. Roosevelt tolerated—and in some cases even promoted—a liberal social welfare agenda. Although the general belief in the United States was that people should adjust to the market rather than the other way around, the social consensus also dictated that the nation should strengthen human capital in order to make people more economically competitive.¹¹ Thus, social welfare programs were developed to increase human capital through education, employment, health, and housing. The belief was that as human capital increased, the dependent person (or at least his or her children) would eventually be able to compete in a free market. For those who could not compete because of serious deficits (such as disabilities or old age), a system of social insurance or public assistance was developed to ensure a minimum level of subsistence. Thus, the dual focus of the welfare state was (1) to create programs to increase human capital and (2) to create programs

to subsidize people unable to participate in the workforce. Even conservative presidents like Richard Nixon acquiesced to this welfare consensus.

But a more conservative welfare consensus emerged during the 1980s. This new consensus, called **neoliberalism**, called for (1) making welfare benefits conditional on employment and other norms; (2) transforming open-ended entitlements to discretionary programs; (3) containing the growth of the governmental sector while retaining (in curtailed form) fundamental programs that affect the elderly and the working poor; (4) replacing government with other institutions, such as families and community-based organizations; and (5) contracting out services and benefits to the private sector. In effect, ideologues of the new right argued that the liberal welfare state was a failed social experiment.¹² During the 1990s, the conservative critique of the welfare state, introduced first by Margaret Thatcher in the United Kingdom and Ronald Reagan in the United States, prompted a reassessment of public welfare within liberal circles, with the result that Bill Clinton and Tony Blair (nominal liberals) essentially continued the conservative vector in social policy. Most recently, the implications of this trend have been explored by Anthony Giddens, who contends that this represented a "third way" in British social policy.¹³ In the United States, Neil Gilbert observed the emergence of an "enabling state," which conditions social welfare benefits on participation in mainstream activities, particularly work.¹⁴

By the end of the twentieth century, it was evident that the welfare state was on the wane. "Across all of Europe and North America, the social democratic century has come to an end," observed Alan Wolfe. He further noted:

Solidarity, social citizenship, the gift relationship, and the difference principle—as all of them representing formulations of the idea that those who live in a society are obligated to insure the welfare of everyone else—are terms bandied about in academic circles, but they no longer make much of an appearance in real politics.¹⁵

Retrenchment of the welfare state produced political crises in Europe and the United States. Economic instability shook the European Union when several member countries, particularly Greece, Spain, and Iceland, were encumbered with debt far beyond what their economies could sustain. "The deficit crisis that threatens the euro has

also undermined the sustainability of the European standard of social welfare, built by the left-leaning governments since the end of World War II.”¹⁶ Austerity measures provoked street demonstrations as citizens faced sharp reductions in their living standards. In the United States, citizen disenchantment with the status quo assumed two forms. First, an indignant group fearful of government debt and overreach formed the Tea Party. During the 2010 midterm elections, Republican candidates aligned with the Tea Party won enough Congressional offices to subvert the centrist tactics of the leadership of both political parties.

At the other end of the ideological continuum, commemorating the 10th anniversary of the September 11, 2001, attacks against the World Trade Center, progressive militants began camping out in a church-owned lot near Wall Street. Announcing that they represented the 99 percent of Americans who had been disadvantaged by the 1 percent who were wealthy, Occupy Wall Street (OWS) spawned similar counter-demonstrations in cities across the country. Until the unsuccessful candidacy of Bernie Sanders for the presidency, OWS failed to craft an agenda with sufficient specificity to propose legislation or sponsor candidates for election. Regardless, both insurgencies have contempt for the status quo: “the Tea Party and Occupy Wall Street both rage against a financial elite that stumbled into a ruinous recession—and then got bailed out by a Washington elite that’s in hock to special interests.”¹⁷ Such political convulsions will probably continue as First World nations confront the reality that budget commitments to welfare state benefits may no longer be sustainable, given the tax burden they require.

Neoliberals’ focus on austerity continued to constrain welfare states. During the 2014 midterm election, American conservatives regained control of Congress, further restraining the Obama administration. In the United Kingdom, Tories won a resounding victory in national elections, leaving the Labour Party in disarray, until the Brexit, when Britons voted to leave the European Union, destabilizing both parties. Although social program advocates were highly critical of neoliberalism, especially after its deregulation strategy contributed to the Great Recession of 2008, neoliberalism continues to shape the American welfare state.¹⁸ Although Europeans began conceiving a Social Investment State as a sequel to the governmental welfare state,¹⁹ fundamental questions remained about America’s adoption of that model.²⁰

Ranking National Development

Various frameworks have been proposed to gauge the progress of nations. Among the first was the **Gini coefficient**, a figure denoting the extent to which the distribution of income diverges from perfect equality; 0 denotes perfect equality, while 1.0 represents one person controlling all resources. For example, in 1947 the Gini coefficient for American households was 0.41, but by 2012, it has risen to 0.48,²¹ a significant increase in economic inequality. Critics of the Gini coefficient cite the limitations of a portrait of development that is based solely on income. Other indicators, such as longevity, education, opportunity, and environment, should be included, they claim. Although this argument has obvious merit, defenders of the Gini coefficient cite its value when the disparity between the rich and poor has become such a chasm. By way of illustration, 1 billion people survive on less than \$1 per day; yet, “the assets of the world’s 358 billionaires exceed the combined annual incomes of countries with 45 percent of the world’s people.”²²

Since 1990, the United Nations has published the **Human Development Index (HDI)** as a ranking of the development of nations. The HDI is a composite of three variables: life expectancy, educational attainment, and income. A nation’s HDI score, an average of the sum of the three variables, has a maximum possible value of 1. In 2012, the United States ranked fifth among all nations.²³ The HDI demonstrates that nations with somewhat lower incomes are nonetheless able to sustain longevity and mount educational programs. Cuba’s per capita GDP is one-fourth percent that of Saudi Arabia, yet Cubans enjoy greater longevity and more educational opportunity than Saudis. Still, the HDI is a less than optimal classification. An important qualification is that nations evidence significant variations internally that are not registered by the national HDI score. Ordinarily, national capitals, industrial cities, and ports elevate a nation’s HDI score; rural and remote areas have a depressive effect. Although the “medium” developing nations rank higher than those ranked “low,” the rural areas of the “medium” HDI-scoring nations tend to parallel those nations that rank lowest.

The HDI has spawned other rankings of national development. Since 2008, the Legatum Institute has published a Prosperity Index, based on eight indicators: economy, entrepreneurial opportunity, governance, education, health, security,

freedom, and social capital. In 2014, the United States ranked 10th.²⁴ Gender equality has also been evaluated through two indices: the United Nations Gender Inequality Index and the World Economic Forum's Women Empowerment Index. Both include political and labor force participation; however, the rankings are quite different. For 2014, the United States ranked fifth on the Gender Inequality Index²⁵ but 20th on the Women Empowerment Index.²⁶

The Fourth World

To address important issues related to social progress, the United Nations developed other indices to assess national development. A Human Poverty Index incorporates nations' percentages of people not expected to reach age 40, percentages of illiterate adults, the resources nations have available in the form of health services and safe water, and percentages of underweight children under age five. A general indicator of entrenched poverty is the percentage of a nation's population living on less than \$1.25 per day. Table 18.1 ranks nations according to the percentage of their population living on less than \$1.25 per day.

That so many remained in abject poverty despite global prosperity prompted a rigorous debate about development efforts in the Fourth World. Jeffrey Sachs proposed a global strategy for ending poverty by 2025, expanding on the United Nations' **Millennium Development Goals**. Sachs suggested a range of minimum standards:

- Primary education for all children
- Adequate nutrition for vulnerable populations
- Access to antimalarial bed nets

TABLE 18.1 Top Ten Nations with Population Living on Less Than \$1.25 per Day

Rank	Nation	% Living on Less Than \$1.25/day
1	Liberia	83.7
2	Burundi	81.3
3	Rwanda	76.8
4	Malawi	73.9
5	Tanzania	67.9
6	Madagascar	67.8
7	Nigeria	64.4
8	Zambia	64.3
9	Central African Republic	62.8
10	Swaziland	61.9

Source: Adapted from United Nations, Multidimensional Poverty, Retrieved from <http://hdr.undp.org/en/content/table-6-multidimensional-poverty-index-mpi>.

- Access to potable water and sanitation
- A half kilometer of paved road per 1,000 population
- Access to cooking fuels and stoves that reduce indoor pollution

Adopting the United Nations' expectation that developed nations dedicate 0.7 percent of gross national product (GNP) for foreign aid, Sachs calculated that ending global poverty by 2025 could cost \$124 billion.²⁷ The agenda set forth by Sachs received notoriety once it was endorsed by Bono and other celebrities.

Spotlight 18.1

UNITED NATIONS MILLENNIUM DEVELOPMENT GOALS

The United Nations Sustainable Development Goals (SDGs) were agreed upon by 193 countries in 2015. They represent a comprehensive strategy to improve conditions in developing nations.



Watch this video to check on the progress on reaching the United Nations Millennium Development Goals.

www.youtube.com/watch?v=A5giOCjj5X8

Within a year, another developmental economist, William Easterly, published a rebuttal to “the Plan,” the attempt by elite planners, such as Sachs, from developed nations to dictate the route to prosperity for citizens in developing nations. “The Big Plans are attractive to politicians, celebrities, and activists who want to make a big splash, without the Western public realizing that those plans at the top are not connected to the reality at the bottom,” Easterly wrote. Too often, Easterly argued, the best of intentions on the part of experts from the West are sabotaged by circumstances of “the Rest,” citing antimalarial mosquito nets that are diverted to the black market where they are sold for wedding veils and fishing nets.²⁸ Instead of planners formulating, implementing, and evaluating development projects from afar, Easterly stated a preference for “searchers,” development experts whose ideas spring organically from the circumstances of indigent populations. “The aid agencies would be forced to act as social entrepreneurs, trying to offer innovative services that would prove attractive to the poor.”²⁹

Inevitably, the debate degenerated into an ideological polemic with liberals favoring Sachs and conservatives supporting Easterly; an unfortunate development because both had valid points to make with respect to traditional development strategies. A more strategic analysis by Paul Collier who contended that the bottom billion should be the focus of aid because of four traps: chronic civil conflict, the discovery of natural resources, being geographically landlocked, and having corrupt governance. Under these circumstances, neither Sachs’s nor Easterly’s strategies were likely to address the needs of the most impoverished people on the planet. Massive aid from developed nations becomes a spoil for corrupt and inept governments of developing nations, and local projects are insufficiently coordinated or funded to rise to the scale necessary to reverse protracted poverty. Instead, Collier called for a series of international charters—a social contract on development—agreements between donor nations and developing nations that structure development:

- A charter for natural resources designed to optimize transparency in extraction contracts, even out economic fluctuations, and set aside revenues for future generations
- A charter for democracy that would not only assure fair elections but would also institute

the civil institutions necessary for checks and balances in decision-making

- A charter for budget transparency that would identify how funds are to be appropriated as well as track the outcomes of expenditures
- A charter for postconflict situations that would establish institutions and procedures for the reconciliation of disputes
- A charter for investment that would preclude populist dictators from confiscating private holdings

In addition to development charters, Collier conceded that innovations were necessary to aid the most destitute, such as “independent service authorities,” an alternative network of basic services for the poor.

The authority would be a wholesale organization for purchasing basic services, buying some from local governments, some from nongovernmental organizations (NGOs), such as churches, and some from private firms. It would finance not just the building of schools and clinics but also their day-to-day operations. Once such an organization was put into place, managed jointly by government, donors, and civil society, both donors and government would channel money through it.³⁰

Few would dispute the validity of Sachs’s contention that the developed world could contribute more for development, or Easterly’s insistence on projects that have immediate relevance for the poor; Collier provides an essential third component of reform: infrastructure. The reassessment of development accounts, at least in part, for the Millennium Challenge Account initiative launched by the George W. Bush administration. Initially budgeted at \$5 billion per year, those developing countries that meet minimal standards of good governance are eligible for special assistance. During a 2008 trip to Tanzania, Bush announced one of the largest grants, \$698 million over five years, primarily for schools, roads, and utilities.³¹ Unfortunately, Congressional reluctance to fund the initiative has left it between \$1.2 billion and \$1.8 billion—insufficient to realize its objectives.³² Subsequent military incursions into Iraq and Afghanistan subtracted from U.S. development efforts. Jeffrey Sachs criticized the Obama administration for spending \$100 billion for fighting the Taliban in Afghanistan, yet appropriating only \$10 billion for the poorest in Africa.³³

Capability Poverty

The idea that development is a multifaceted phenomenon has eclipsed a more circumscribed notion based on simple economic parameters. Using the HDI as a basis, development researchers began to evolve a more inclusive and dynamic understanding of progress and to promulgate the idea of **capability poverty**. The philosophical rationale for capability poverty was stated fully by Amartya Sen, who noted two profound historical truths: There are no records of famine within democratic societies, nor instances of wars waged between them. Rather, these most severe events can be attributed to nondemocratic decision-making that excludes large groups of people, who then bear the brunt of flawed policies. Sen's contention that freedom was a precondition of development, not a by-product to be enjoyed long after industrialization, earned him the Nobel Prize in economics in 1998.



Among the 10 elements of a capability approach is play—enjoying activities that are entertaining and rejuvenating.

Source: saurabhpbhoyar/Shutterstock

The capability approach to development advocates “the expansion of the ‘capabilities’ of persons to lead the kind of lives they value—and have reason to value,” Sen proposed, “Greater freedom enhances the ability of people to help themselves and also to influence the world, and these matters are central to the process of development.”³⁴ Logically, however, deprivation of elemental requirements of survival subverts the possibility of full social participation, as do inadequate institutions that deny education, health, and recreation to subgroups. In constructing his argument for a “support-led process,” Sen diverged with proponents of an economic model in which health, education, and labor benefits were viewed as being secondary. He contended that:

[T]he success of the support-led process as a route [to development] does indicate that a country need not wait until it is much richer (through what may be a long period of economic growth) before embarking on rapid expansion of basic education and health care. The quality of life can be vastly raised, despite low incomes, through an adequate program of social services. The fact that education and health care are also productive in raising economic growth adds to the argument for putting major emphasis on these social arrangements in poor economies, without having to wait for “getting rich” first.³⁵

Sen's path-breaking work was soon elaborated by his colleague, Martha Nussbaum who made the philosophical rationale for an absolute standard of social justice for the most chronically oppressed among the world's poor: women. “The core idea is that of the human being as a dignified free being who shapes his or her own life in cooperation and reciprocity with others, rather than being passively shaped or pushed around by the world in the manner of a ‘flock’ or ‘herd’ animal,” contended Nussbaum. “A life that is really human is one that is shaped throughout by these human powers of practical reason and sociability.”³⁶ Thus, optimal development is not only freedom from want but also the full enjoyment of a range of social and political opportunities. Nussbaum listed 10 elements of a capability approach to development (see Figure 18.2).

So formulated, the capability approach to development provides a blueprint for achieving and maintaining progress. This approach is particularly compelling in an era of rapid international

1. **Life.** Being able to live to the end of a human life of normal length; not dying prematurely, or before one's life is so reduced as to be not worth living.
2. **Bodily Health.** Being able to have good health, including reproductive health; to be adequately nourished; to have adequate shelter.
3. **Bodily Integrity.** Being able to move freely from place to place; to be secure against violent assault, including sexual assault and domestic violence; having opportunities for sexual satisfaction and for choice in matters of reproduction.
4. **Senses, Imagination, and Thought.** Being able to use the senses, to imagine, think, and reason—and to do these things in a “truly human” way, a way informed and cultivated by an adequate education, including, but by no means limited to, literacy and basic mathematical and scientific training. Being able to use imagination and thought in connection with experiencing and producing works and events of one's own choice, religious, literary, musical, and so forth. Being able to use one's mind in ways protected by guarantees of freedom of expression with respect to both political and artistic speech, and freedom of religious exercise. Being able to have pleasurable experiences and to avoid nonbeneficial pain.
5. **Emotions.** Being able to have attachments to things and people outside ourselves; to love those who love and care for us, to grieve at their absence; in general, to love, to grieve, to experience longing, gratitude, and justified anger. Not having one's emotional development blighted by fear and anxiety. (Supporting this capability means supporting forms of human association that can be shown to be crucial in their development.)
6. **Practical Reason.** Being able to form a conception of the good and to engage in critical reflection about the planning of one's life. (This entails protection for the liberty of conscience and religious observance.)
7. **Affiliation.**
 - A. Being able to live with and toward others, to recognize and show concern for other human beings, to engage in various forms of social interaction; to be able to imagine the situation of another. (Protecting this capability means protecting institutions that constitute and nourish such forms of affiliation, and also protecting the freedom of assembly and political speech.)
 - B. Having the social bases of self-respect and nonhumiliation; being able to be treated as a dignified being whose worth is equal to that of others. This entails provisions of nondiscrimination on the basis of race, sex, sexual orientation, ethnicity, caste, religion, and national origin.
8. **Other Species.** Being able to live with concern for and in relation to animals, plants, and the world of nature.
9. **Play.** Being able to laugh, to play, and to enjoy recreational activities.
10. **Control over One's Environment.**
 - A. **Political.** Being able to participate effectively in political choices that govern one's life; having the right of political participation, and protections of free speech and association.
 - B. **Material.** Being able to hold property (both land and movable goods), and having property rights on an equal basis with others; having the right to seek employment on an equal basis with others; and having the freedom from unwarranted search and seizure. In work, being able to work as a human being, exercising practical reason and entering into meaningful relationships of mutual recognition with other workers.³⁷

FIGURE 18.2 The Central Human Capabilities

changes attributable to shifts in capital, production, and labor. “Especially in an era of rapid economic globalization, the capabilities approach is urgently needed to give moral substance and moral constraints to processes that are occurring all around us without sufficient moral reflection,” Nussbaum concluded. “It may be hoped that the capabilities

list will steer the process of globalization, giving it a rich set of human goals and a vivid sense of human waste and tragedy, when choices are pondered that would otherwise be made with only narrow economic considerations in view.”³⁸

Nussbaum's work is poignant for fourth world women because empowering women

generates significant development outcomes. Improving the economic, educational, and health of women is associated with a 25 percent reduction in the mortality rate of children since 1990,³⁹ and a fourfold drop in the maternal death rate since 1980.⁴⁰ When multiple development strategies were engaged in a small Kenyan village as part of Jeffrey Sachs's Millennium Village initiative in Africa, the child mortality rate dropped 30 percent.⁴¹ At the national level, female representation in the legislature is likely to generate pro-family policies. Although some Congressional seats in the United States are occupied by women, the national legislatures of other countries report higher rates: Sweden, 46.4 percent; South Africa, 44.5 percent; Cuba, 43.2 percent; Germany 32.6 percent; China, 21.3 percent; Britain, 19.5 percent; France, 18.9 percent, but only 16.8 percent in the United States.⁴²

International Aid

Historically, intergovernmental transfers have accounted for most foreign aid, although other intermediaries are being employed with increasing frequency. The recent performance of the industrialized nations demonstrates several trends in international aid. Foremost, nations with the largest economies, notably Japan and the United States, lag behind smaller industrialized nations with respect to the percentage of GNP committed to international aid. The Scandinavian welfare states have consistently demonstrated international citizenship superior to that of the more laissez-faire economies. Compounding this pattern has been a downward trend in allocations for international aid. Since 1990, international aid from national governments dropped only to rebound in response to the adverse effects of the global recession on developing nations; for 2010, development aid totaled \$129 billion, an increase of 6.5 percent over 2009.⁴³ Since government developmental aid has been uneven over the decades, reliance has increased on private NGOs and on quangos (quasi-NGOs) such as the World Bank, the International Monetary Fund (IMF), and regional development banks. Although the United States is the largest donor, not only is its percent GNP allotted to international aid, the lowest among industrialized nations, but also that ratio has roughly halved since the mid-1980s (see Table 18.2).

Global Capital

The foundation for global markets was laid after World War II with an international agreement for currency stabilization and debt financing that was negotiated at Bretton Woods, New Hampshire. The Bretton Woods agreement established the IMF, an international agency accountable to its member nations. With the expansion of capital to overseas markets during the 1960s, the IMF initiated a program of Special Drawing Rights or loans that encouraged expansionary policies in developing nations. This program, however, collided with the acute economic contraction of the 1970s due to the oil embargo, driving much of the developing world into severe debt. Subsequent IMF policies to restructure debt payments stressed "conditionalities," or internal economic reforms, requiring controversial reductions in domestic spending for health, education, and related social programs.

The World Bank, also accountable to its member nations, provides resources to enhance infrastructure, initially through the International Bank for Reconstruction and Development (IBRD). Following the dramatic success of the Marshall Plan that rebuilt postwar Europe, the World Bank instituted the International Development Association (IDA) in 1960 to redirect activities to developing nations. Focusing on the poorest nations, World Bank aid is often in the form of credits, long-term loans that are interest-free.⁴⁴

Both the IMF and World Bank presume that international markets will be a means for development, a strategy that, although markedly successful for industrialized nations, has been less so for developing nations. With the demise of state socialism, international markets have, nonetheless, become the primary means for economic growth of the Third World. Two entities have facilitated this growth: The General Agreement on Tariffs and Trade (GATT), established in 1947, enjoyed only modest success in reducing tariffs until 1967, but in that year, impediments against international trade were cut drastically. Moreover, in 1995 the World Trade Organization (WTO) was created to accelerate the emergence of global markets, replacing GATT. Since then, membership in GATT/WTO has increased dramatically. Between 1980 and 1999, the number of GATT member nations increased from 85 to 134. Globalization has accelerated with the increasing flow of private capital into

TABLE 18.2 Net Official Development Assistance Disbursed, as a Percentage of Gross National Income, 2010 and 2005

Rank	Nation	Total (millions)	% of GNP	
			2010	2005
1	Norway	\$ 4,582	1.10	0.94
2	Luxembourg	399	1.09	0.82
3	Sweden	4,527	0.97	0.94
4	Denmark	2,867	0.90	0.81
5	Netherlands	6,351	0.81	0.82
6	Belgium	3,000	0.64	0.53
7	United Kingdom	13,763	0.56	0.47
8	Finland	1,335	0.55	0.65
9	Ireland	895	0.53	0.42
10	France	12,916	0.50	0.47
11	Spain	5,917	0.43	0.27
12	Switzerland	2,295	0.41	0.44
13	Germany	12,723	0.38	0.36
14	Canada	5,132	0.33	0.34
15	Australia	3,849	0.32	0.25
16	Austria	1,199	0.32	0.52
17	Portugal	648	0.29	0.24
18	New Zealand	353	0.26	0.27
19	United States	30,154	0.21	0.22
20	Japan	11,045	0.20	0.28
21	Greece	500	0.17	0.17
22	Italy	3,111	0.15	0.29
23	Korea	1,168	0.12	—

Source: Adapted from OECD data statistics on Gross Domestic Product (GDP).

developing nations. Indeed, although direct foreign investment slowed subsequent to the global recession, in 2010 it totaled over \$360 billion, more than twice direct government aid.⁴⁵

By 2015, assistance to the poorest countries consisted of a mix of direct government assistance, private capital, and remittances that workers in developed nations sent home to their families. The mix has changed as evident in aid to sub-Saharan Africa. Between 1990 and 2012, official development assistance (ODA) dropped from 62 percent of external revenue to 22 percent. By 2012, ODA accounted

for about \$25 billion in aid to sub-Saharan Africa; however, private capital and remittances, which represented negligible aid in 1990, accounted for almost \$100 billion.⁴⁶ Thus, direct governmental assistance has lagged behind private sources of development aid.



Watch this video to learn how, as evident in micro-finance, capitalism can be a catalyst for development in the poorest communities. In this video, learn about how businesses have been developed in East Africa.

www.youtube.com/watch?v=H6kBP9b3190

The Future

Although the unprecedented expansion of global markets has furthered economic growth internationally, the benefits have not been evenly distributed. During the last decades of the twentieth century, the per capita GDP of the most-developed one-third of nations increased significantly; in contrast, that of the middle-third dropped from 12.5 to 11.4 percent, and that of the lowest-third fell from 3.1 to 1.9 percent. “Such findings are of great concern,” observed authors of the World Bank’s development report, “because they show how difficult it is for poor countries to close the gap with their wealthier counterparts.”⁴⁷ The 2000 Millennium Development Goals (MDGs) provided benchmarks against which to measure development objectives. Approaching 2015, the number of people subsisting on less than \$1.25 per day was expected to fall to 883 million, significantly less than 1.4 billion in 2005. In addition, targets for the provision of primary education for both genders as well as the availability of safe drinking water are about to be reached. On the other hand, health and sanitation objectives are unlikely to be achieved. Significantly, the MDG strategy has not been even throughout the developing world; much of these accomplishments are due to unprecedented development in China and India, while the nations of sub-Saharan Africa continue to fall behind.⁴⁸

Specific initiatives have defined international development during the past half-century, while others are emergent: Microcredit, introduced with the founding of the Grameen Bank in the late 1970s, enjoyed wide popularity in following decades, earning its founder, Muhammad Yunus the 2006 Nobel Peace Prize. Using social collateral in the form of group lending and adherence to a moral code, the 16 decisions, Grameen not only loaned \$11.5 billion dollars to 3.7 million poor families in 2010 but also achieved a repayment rate exceeding 96 percent. Early in its development, the Grameen Bank reported that over several loan cycles most Grameen borrowers were able to invest in business activities allowing them to escape poverty.⁴⁹

Three decades later, however, cracks were evident in the microcredit façade. Having pioneered the lending model, Grameen was imitated by nonprofits that transferred microcredit to Africa and Latin America, in the process demonstrating that the scale of the nascent capital market of the world’s poorest communities exceeded \$60 billion.

By 2011, commercial lenders, such as the Indian company SKS Microcredit, were becoming publicly traded corporations, generating millions of dollars in profits for owners and investors.⁵⁰ Meanwhile, a chorus of critics alleged that microcredit loans carried interest rates that many borrowers were unable to repay, sometimes exceeding 100 percent annually; as a result microcredit borrowers took out additional loans to service existing loans.⁵¹ In Bangladesh, Yunus antagonized the government by threatening to form a political party, and the Prime Minister accused Grameen of laundering a \$100 million grant from Norway.⁵² Threatened by government regulation of microcredit and borrowers’ withholding payments, microcredit organizations vowed to reinvent themselves to serve their clientele in a manner that was responsible and accountable. “We created microcredit to fight the loan sharks,” Yunus said, “we didn’t create microcredit to encourage new loan sharks.”⁵³

Field experiments, considered the gold standard of research in the First World, have only recently been introduced in developing countries, largely due to the insistence of Esther Duflo and Abhijit Banerjee who cofounded MIT’s Abdul Latif Jameel Poverty Action Lab (J-PAL) in 2003. One of Duflo’s earliest experiments involved absentee teachers in India, a chronic problem undermining student learning. Duflo randomly assigned 120 schools to one of two groups; in the test, group teachers were photographed with their students before and after each school day by using a tamper proof camera and a control group with no changes. Teacher pay was adjusted for attendance. At the end of the experiment, teachers in the test group were half as likely to be absent from school.⁵⁴ In 2005, Duflo examined activities of Spandana, an Indian microcredit lender. When Spandana announced plans to expand into 52 neighborhoods in Hyderabad, a matched set of 52 neighborhoods was identified. More than a year later, Duflo’s team interviewed 7,000 residents, evaluating in detail their economic circumstances. Duflo concluded that microcredit was not for every poor household; although some used loans as intended to start-up businesses, others used the money to purchase consumable goods, effectively worsening their poverty since they had to repay the loan. Duflo and Banerjee identified two circumstances in which the standard microcredit model was ill advised: borrowers who were able to operate independently from a lending group, and

those who were not positioned to begin immediate repayment of loans. Ultimately, the researchers concluded that economic stability was a precursor for microcredit success. “A sense of stability may be necessary for people to be able to take the long view,” they observed, “A steady and predictable income makes it possible to commit to future expenditure and also makes it much easier and cheaper to borrow now.”⁵⁵

The “Robin Hood Tax,” first proposed by Nobel Laureate James Tobin in 1974 would tax currency transactions, the revenue from which would be used to finance development efforts.⁵⁶ A spot tax on financial trading languished until the global recession in the late 2000s, when economic inequality became more obvious internationally. Demonstrators against cuts in social welfare benefits in the European Union as well as the United States contended that a Robin Hood tax could produce a measure of economic equity.⁵⁷ Supporters noted that Britain, Hong Kong, and Singapore had instituted a tax on financial transactions ranging from \$10 to \$50 per \$10,000. By the end of 2011, prime ministers of Germany, Italy, and France announced support for the concept, and Bill Gates proposed to the G20 nations a modest tax that would generate \$48 billion annually. Leaders of the United States and the United Kingdom were ambivalent, however, suspecting that any such tax would place adopting countries at an economic disadvantage unless the tax were in effect internationally.⁵⁸

Technological innovations have attempted to replicate the astounding success of the Green Revolution of the 1970s in increasing the world’s food supply by introducing scientific and economic innovations to developing nations. Toward the end of 2011, for example, GlaxoSmithKline, funded by the Bill and Melinda Gates Foundation, announced that a new antimalarial drug had proven successful on half of children to whom it had been administered.⁵⁹ In 2009, the Indian government appointed Nandan Nilekani, the wealthy founder of Infosys, to manage the Aadhaar identity system through which 1.2 billion Indians will have fingerprints and iris scans encoded on an identity card that will allow access to such government services as drivers’ licenses and welfare benefits. Reversing traditional caste methods for establishing identity, the Aadhaar card will promote equality among all Indians in the process advantaging the poorest.⁶⁰

In the mid-1990s Santiago Levy, undersecretary of finance in Mexico, conducted an experiment

that asked the poor if they preferred continuing a traditional surplus food program or receipt of cash tied to specific conditions. When the results showed the poor preferred conditional cash, Levy convinced the government to establish *Oportunidades*, through which low-income households go to a local bank to receive cash in order to purchase consumer needs. The cash conversion was not only welcomed by low-income Mexicans, but the model was also imported by then New York Mayor Michael Bloomberg as Opportunity New York where low-income households are given cash incentives for constructive activities, such as holding a job and participating in child care.⁶¹ Among the most intriguing of technological applications has been the introduction of companies that are hybrids between traditional for-profit ventures and nonprofits committed to social welfare. In *Building Social Business*, Grameen founder Yunus proposed the “social business” as a model to generate capital, develop products, and market them to low-income consumers, including yogurt and shoes.⁶² Similarly, Vinod Khosla, billionaire and co-founder of Sun Microsystems, established an initiative to promote socially responsible business activities in India, such as MokshaYug Access a milk collection, cooling, and distribution company.⁶³ In each of these examples, new technological applications are furthering development efforts by government as well as the nonprofit and commercial sectors, congruent with the “social entrepreneurship” model of change.⁶⁴

Conclusion

Nations vary developmentally both as individual entities and as clusters. Thus, the United States is more developed than Niger, just as the countries of the industrialized West are more affluent than those of sub-Saharan Africa. At the same time, countries and regions influence one another, contributing to the dynamism of international relations. A common tendency in the West has been to regard the welfare states of North America and Western Europe as ideals to which the developing nations should aspire. Yet, a series of events challenged this assumption. First, since the 1980s the welfare states of the West have retrenched, reducing their fiscal commitments to social programs. Second, among developing nations, subgroups have emerged. The oil embargo of the 1970s highlighted a small group of nations, the oil-exporting countries, which have comparatively high per capita income despite

relatively low industrialization. Another subgroup consists of Southeast Asian nations that have prospered by aggressively pursuing capitalist strategies of development, China serving as the prototype. Many of the nations of sub-Saharan Africa are worse off today than when they achieved independence during the 1960s, a tragedy that has been exacerbated by the HIV/AIDS epidemic. Third, international development agencies, such as the United Nations and the IMF, have begun to explore alternative scenarios to prosperity—scenarios, such as sustainable development⁶⁵—that are not necessary in sync with increasing revenues from private capital or family remittances.

Thus, the established welfare states are undergoing internal changes that also affect their ability to provide assistance to developing nations. According to Thomas Piketty, the Golden Age of the welfare state was a product of social solidarity produced by the Great Depression and World War II; however, during the last decades of the twentieth century, the political resolve and economic requirements of the welfare state foundered, limiting aid they could provide to developing nations. Among welfare state nations, taxation for the “social state” peaked in 1980 and subsequently plateaued.⁶⁶ The decline of the welfare state was also charted by prominent scholars from the Left. “The social and political settlements which [the free market] has destroyed—the Beveridge settlement in Britain and the Roosevelt New Deal in the United States—cannot be recreated,” intoned John Gray.⁶⁷ “The social democratic ‘moment,’” concluded Tony Judt, “was the product of a very particular combination of circumstances unlikely to repeat themselves.”⁶⁸

A world in which inequality and strife contribute to social instability poses enormous challenges for human service professionals, with respect to both direct services and development theory. Social workers who have served in the Peace Corps have an appreciation for the intractability of problems in the Third and Fourth Worlds, of course. But immigrants seeking better opportunities in developing nations are presenting new problems to human service workers in industrialized nations. Social workers in the United States, for example, are confronted with human trafficking, undocumented workers, and refugee families in increasing numbers.⁶⁹ As James Midgley advised, addressing such problems requires strategies that focus on human capital, social development, and

self-sufficiency—interventions that are not commonly taught in First World schools of social work.⁷⁰

DISCUSSION QUESTIONS

1. Which development index do you think best measures a nation's progress? What indicators would you include to make it more comprehensive?
2. What are the main reasons Fourth World countries are less developed now than before they achieved independence? What would be your strategy for improving the circumstances of their citizens?
3. Select one of the Millennium Development Goals and track it according to several developing countries. What would explain variations in progress toward meeting that goal?
4. Compared to those of other nations, is the U.S. welfare state really “exceptional”? Does this expand or limit America's ability to care for disadvantaged citizens?
5. How could increasing aid to developing nations from private capital and family remittances be linked to official development assistance to optimize aid?
6. Select a developing nation and investigate the adequacy of its social infrastructure. What is the level of provision of welfare, health, and education services? How does it compare to the benefits and services of the United States?
7. Among the most persistent of development problems has been gender inequality. What are the most important components of this indicator of development? How can gender inequality be addressed in the United States?

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GLOSSARY

absolute poverty A measurement and classification of poverty that is based on the minimal standard of living (including food, shelter, and clothing) necessary for survival

accountable care organizations An entity authorized through the Affordable Care Act that enrolls at least 5,000 consumers and receives a unified budget through which care is provided, any surplus retained by the ACO

affirmative action Programs designed to redress past or present discrimination against minorities (including women) through criteria for employment, promotion, and educational opportunities that give these groups preferential access to such resources or opportunities

ageism Age-based discrimination against elderly persons

alleviative approach to poverty Strategies designed to ease the suffering of the poor rather than to eliminate the causes of poverty. Examples include AFDC, SSI, and the Food Stamps Program

annual percentage rate (APR) The annualized interest rate for a loan. Most credit card, mortgages, and car loans are calculated at the APR

area poverty Geographic regions that are economically depressed

battered child syndrome a diagnostic concept describing the persistent physical damage to children, previously unrecognized but revealed by advances in radiology

block grants A method of funding social programs by which the federal government makes monies available to states for a wide range of services, including social services. Block grants usually allow states more freedom by diminishing or eliminating federal program regulations. Block grants are usually fixed in terms of the amount available

Brown v. Board of Education of Topeka, Kansas
A 1954 landmark U.S. Supreme Court decision ruling that “separate but equal” facilities in education were inherently unequal

Buck v. Bell 1927 Supreme Court decision affirming the constitutionality of forced sterilization

of the “unfit,” including “mental defectives” as advocated by the Eugenics movement

bureaucratic rationality The ordering of social affairs by governmental agencies

capability poverty Developed by Amartya Sen, this theory focuses on capability—that is, what individuals are capable of doing. It conceptualizes poverty beyond simple economic determinants, including safety, health, education, and freedom

capitalism An economic system in which most of the production and distribution of goods and services occurs under private auspices

capitation The method of payment in which the provider is paid a fixed amount for each person served regardless of the actual number or nature of services delivered

case management The process by which all related matters of a case are managed by a single professional, often a social worker

case poverty Individual deficits in human capital such as illness, illiteracy, and so forth

categorical grants A method of funding social services through which the federal government makes available to the states monies that must be spent for very narrowly specified service needs

Charity Organization Society (COS) A voluntary organization active in the late nineteenth and early twentieth centuries that attempted to coordinate private charities and promote a scientific approach to philanthropy

chronic unemployment The rate of unemployment attributable to persons who have persistent trouble finding work because of an absence of low-skilled jobs or because they have severe deficiencies in basic social and work skills

circuit breaker programs Tax rebate programs designed to relieve the low-income, elderly, or disabled home owner or renter from the burden of property or utility taxes

collective bargaining The process of negotiating wages and conditions of employment by an organized body of employees, usually a union or professional association

commercialization The consequence of subjecting social welfare to the marketplace, including advertising for services, marketing services, and pricing services

comparable worth The idea that workers should be paid equally when they do different types of work that require the same level of skill, education, knowledge, training, responsibility, and effort

corporate sector That part of the mixed welfare economy consisting of large, for-profit human-service corporations

corporate social responsibility The concept that corporations should be held accountable for practices and decisions that adversely affect those communities in which they do business. In addition, corporate social responsibility refers to the responsibility of corporations to promote the general well-being of society

cost shifting Charging one group of patients more in order to make up for underpayment by others

cost-of-living adjustments (COLAs) Adjustments designed to keep income maintenance and social insurance benefits in line with inflation. COLAs affect Food Stamps, Social Security, and SSI benefits

culture of poverty Term used by a theoretical school that maintains that poverty is transmitted intergenerationally and that certain of its traits are found in diverse cultures and societies

curative approach to poverty An approach designed to rehabilitate the poor through attacking the causes of poverty, for example, illiteracy, poor nutrition, or lack of employment

Currier v. Doran The court decision that establishes liability of child welfare workers if their actions “created the danger” that harmed a child

cyclical unemployment A type of unemployment attributable to swings in economic performance, such as recessions

de facto segregation Racial segregation that is not legally mandated by the state but that characterizes school systems and residential housing patterns

deinstitutionalization Term used to describe the removal, in the late 1960s, of many mentally ill or mentally retarded patients from state institutions and their placement in community settings

democratic capitalism The type of political economy characteristic of the United States, with a democratic polity and a capitalist economy

dependency ratio The number of workers required to pay into the Social Security system to support one retired worker living on Social Security

Diagnostic and Statistical Manual (DSM) This manual is in its fifth edition and is the standard classification of mental health disorders created by the American Psychiatric Association and used by U.S. mental health professionals

diagnostic related groups (DRGs) A prospective form of payment for Medicare-incurred charges. Specifically, DRGs are a classification scheme whereby hospitals are reimbursed only for the maximum number of days an illness or surgical procedure is designated to take

discouraged workers Those who have stopped seeking work out of frustration with their poor employment prospects

disentitlement The removal of an entitlement status for a group of people. Specifically, the PRWOA disentitled poor beneficiaries from receiving lifetime benefits

Donaldson v. O'Connor The court decision ruling that mental patients could not be confined unless they were dangerous to themselves or others, and also that they should not be confined unless they are being treated and cannot survive without hospitalization

dual labor market A labor market divided into two classes of workers. *See* primary labor market and secondary labor market

earned income tax credit (EITC) A federal program that functions somewhat like a negative income tax. Specifically, qualified working families or single people receive a tax rebate from the federal government that exceeds the taxes they paid

electronic benefit transfers (EBT) The attempt by the federal government to use technology in the delivery of benefits. This can include automatically depositing public assistance benefits into the savings accounts of recipients, the use of debit cards in food stamp benefits, and so forth

Emergency Assistance Program A program that operated under AFDC and was intended to provide short-term cash assistance to families in crisis

entitlements Governmental resources (cash or in-kind) to which certain groups are entitled, based on their ability to meet the established criteria. Entitlement programs have open-ended resources in that people cannot be denied benefits because of governmental resource constraints

establishment of paternity Policies that require unwed mothers to identify the fathers of their children in order to aid states in their efforts to collect child-support

family cap Usually refers to the policy that recipient mothers will not receive any (or only partial) benefits for any children born while they are on public assistance

Family Sufficiency Program Designed to coordinate public assistance in public housing as a way to lessen welfare dependence

Federal Insurance Contribution Act (FICA) Federal payroll tax to fund Social Security and Medicare

fee for service A method of reimbursement based on payment for services rendered. Payment may be made by an insurance company, the patient, or a government program such as Medicare or Medicaid

feminization of poverty A social trend marked by the increasing frequency of poverty among women. It is thought to be related to the high incidence of women relying on governmental aid, the low wages that characterize traditional female employment, occupational segregation, and family decomposition (divorce, desertion, or death)

fetal alcohol syndrome Developmental and physical birth defects of a child resulting from a mother's use of alcohol during pregnancy

fiscal year (FY) A term used by government and social service agencies in reference to a budgetary rather than a normal year

fourth world Countries where social indicators (e.g., health, education, and welfare) since independence are below that of developing third world nations

frictional unemployment The rate of unemployment, usually about 3 percent, considered inevitable for a viable economy

fringe economy Predatory lenders and service providers (i.e., rental outlets, check cashers) who cater to a financially disadvantaged population

general assistance State or locally run programs designed to provide basic benefits to low-income people who are ineligible for federally funded public assistance programs

gentrification Resettlement of existing low-income neighborhoods by middle- and upper-class home owners or investors. This often results in forcing poor and indigenous residents out of their neighborhoods

Gini coefficient A statistical measure used to represent the level of income distribution and economic inequality in a country, where a value of 1.00 equals complete inequality and 0.00 represents complete equality

governmental sector That part of the mixed welfare economy consisting of social programs administered by government, particularly the federal government

Great Society Formerly called the War on Poverty, the Great Society comprehended a series of social welfare programs (including community development, training and employment, and health and legal services) enacted between 1963 and 1968 during the administration of President Lyndon Baines Johnson

gross domestic product (GDP) A measure of the total output of goods and services produced by a country's economy

gross national product (GNP) A measure of the total domestic and foreign output claimed by residents of a country. It is made up of the Gross Domestic Product (GDP) and of incomes accruing to foreign residents

Halderman v. Pennhurst The court decision ruling that institutionalized patients were entitled to treatment in the least restrictive environment

health care access The patient's ability to obtain medical care. Access is determined by the availability of medical services, the location of health care facilities, transportation, hours of operation cost of care, and so forth

health maintenance organizations (HMOs) Membership organizations that typically provide comprehensive health care. Members usually pay a regular fee and are thus entitled to free (or minimal-cost) hospital care and physicians' services

home health care Full range of medical and other health related services such as physical therapy, nursing, counseling, and social services

that are delivered in the home of a patient by a provider

homophobia The fear of (and subsequent discrimination against) homosexuals on the basis of their sexual preference

human capital Productive investments that are embodied in humans. These include education, training, skills, experience, knowledge, and health. Increases in human capital result from expenditures on education, job training, and medical care

Human Development Index (HDI) A composite statistic developed by the United Nations to rank countries in terms of their development (e.g., life expectancy, education, per capita income).

human services markets An array of for-profit companies providing health care, child care, corrections, and financial services

ideology A set of socially sanctioned assumptions, usually unexamined, explaining how the world works and encompassing a society's general methods for addressing social problems

income distribution The pattern of how income is distributed among the various socioeconomic classes in a society

income maintenance programs Social welfare programs designed to contribute to or supplement the income of an individual or family. These programs are usually means-tested and thus based on need

Indian Child Welfare Act of 1978 Legislation that restored child-placement decisions to the individual tribes

in-kind benefits Noncash goods or services provided by the government that function as a proxy for cash, for example, Food Stamps, Section 8 housing vouchers, and Medicare

inpatient care Care given in a hospital, nursing home, or other medical institution

institutional approach The concept of social welfare asserting that investing in human capital is essential for a competitive economy

institutional conception of social welfare A conception of welfare holding that governmental social programs that assure citizens of their basic needs (for food, housing, education, income, employment, and health) are essential to an advanced economy. Such programs are considered a right of citizenship

Job Opportunities and Basic Skills (JOBS) This program was created in the Family Support Act of 1988. The bill required a portion of a state's welfare caseload (usually AFDC mothers) to participate in a work or training program

Keynesian economics An economic school that proposes government intervention in the economy through such activities as social welfare programs to stimulate and regulate economic growth

libertarians A small but influential group that advocates more individual responsibility and a very limited role for government in social and economic affairs

lobbyists Individuals who work to advance the agenda of an employer, usually a trade association, in a state or national legislature

managed care A loose umbrella term for the organization of networks of health care providers (e.g., doctors, clinics, and hospitals) into a system that is cost-effective. Institutions or individual health care providers who are in managed care systems agree to accept set fees for each service or flat payments per patient. HMOs are one example of managed care

market rationality The ordering of human affairs by corporate institutions within the marketplace

means test Income and asset tests designed to determine whether an individual or household meets the economic criteria necessary for receiving governmental cash transfers or in-kind services

Medigap Private health insurance plans that supplement Medicare benefits by covering some costs not paid for by Medicare

Millennium Development Goals UN time-based goals to reduce extreme poverty and other issues affecting developing nations

mixed welfare economy An economy in which governmental, private nonprofit, and private for-profit providers of social welfare coexist within the same society

National Association of Social Workers (NASW) The major national organization of professional social workers

national health insurance Various insurance-based proposals that incorporate comprehensive health coverage for the entire nation

neoconservatism A recent American ideology, based on conservatism, that recognizes the necessity for social welfare but designs social programs so that they are compatible with the requirements of a market economy and traditional values

neoliberalism A recent American ideology, based on liberalism, that assumes that universal social programs, such as those advanced by liberals, are implausible because of current social, political, and economic limitations. Neoliberals opt for more modest changes in social welfare programs

New Deal The name given to the massive Depression-era social and economic programs initiated under the presidency of Franklin Delano Roosevelt

Non-governmental organizations (NGOs) private or voluntary nonprofits that plan, deliver, or fund social services

occupational segregation The domination of low-wage sectors of the labor market by a minority group. For example, women are thought to be occupationally segregated in “pink-collar” jobs, as secretaries, receptionists, typists, and so forth

Office of Economic Opportunity (OEO) The federal agency that was charged with the responsibility for designing and implementing the Great Society programs

oligopolization The process through which a small number of organizations effectively control a market

Panetti v. Quarterman U.S. Supreme Court decision that criminal defendants sentenced to death cannot be executed unless they understand the reason for the sentence

pay-go A system for determining federal budgetary allocations that emerged out of the 1991 Omnibus Budget Reconciliation Act. In short, funding for any new program (or enhanced funding for an existing program) must come from reallocating existing money

payroll tax Taxes on income paid by employees and employers, the withholding tax (FICA)

permanency planning A strategy for helping foster children to live in families that offer continuity of relationships and the opportunity to establish lifetime relationships

Personal Responsibility and Work Opportunity Reconciliation Act (PRWORA) Passed in

1996, the PRWORA is a comprehensive act that created the TANF program, limited benefits to both legal and illegal immigrants, changed the qualifications for SSI, and established the precedent that government no longer had the responsibility for maintaining the poor indefinitely. In effect, it disintitiled the poor from income support programs

Plessy v. Ferguson The 1896 U.S. Supreme Court decision that formally established the “separate but equal” doctrine in race relations

policy framework A systematic process for examining a specific policy or a set of policies

political action committees (PACs) Organizations, usually associated with special interest groups, that divert campaign contributions to candidates running for public office in order to influence their later decisions on public policy

political economy The blending of economic analysis with practical politics. In effect, political economy views economic activity within a political context

political practice A method of social work practice by which social workers advance their priorities either by assisting those in political office or by running for office themselves

poverty line A yearly cash income threshold (based on family size) set by the federal government to determine if an individual or household can be classified as poor. Sometimes called the poverty threshold or poverty index

pre-existing medical condition A physical condition of an insured person that existed prior to the issuance of his/her policy or his/her enrollment in a health plan

preferential selection The selection of clients for treatment according to the organizational needs of the provider as opposed to the needs of the client; usually used to describe the practice of private providers who prefer insured clients with less severe problems

preventive approach to poverty Social welfare strategies (e.g., social insurance programs) designed to prevent people from becoming poor

primary care Basic or general health care rendered by general practitioners, family practitioners, internists, obstetricians, and pediatricians

primary labor market The full-time jobs that provide workers with an adequate salary, a career track, and benefits

prime lending Loans or mortgages given to creditworthy borrowers at low interest rates

that reflect the prime rate set by the Federal Reserve

private practice The provision of clinical services through the marketplace by individual practitioners or small groups of practitioners

privatization The ownership or management of social services by the private sector, either nonprofit agencies or proprietary corporations. Privatization of public assistance refers to attempts by individual states to have for-profit corporations manage the delivery of their public assistance benefits

probation Early release of offenders from incarceration and their supervision by government probation officers

progressive movement A social movement popular in the United States from the late 1800s to World War I. Progressives stressed the need for morality, ethics, and honesty in all social, political, and economic affairs. This movement advocated numerous progressive reforms. It was also successful in establishing progressive legislation, including the progressive income tax

progressive tax A tax in which the tax rate increases with value of the item taxed

prospective payment system (PPS) A payment method that establishes rates, prices, or budgets before services are rendered and costs are incurred

public choice school A school of political economies that suggests that because interest group demands inevitably lead to budget deficits, government should therefore limit concessions to these groups as much as possible

public policy Policies designed by government that contain a goal, a purpose, and an objective. Public policy may also incorporate a standing plan of action toward a specific goal

public transfer programs Programs such as AFDC, SSI, and Social Security that transfer money from the governmental sector to families or individuals who are either entitled to it or who have earned it

racism Discrimination against or prejudicial treatment of a racially different group

recovered memory The invalid proposition that people traumatized suppress memories that can be recovered through a certain form of therapy

refundable tax credit Payment by the government to taxpayers who report income or expenses

below a given level, the structure of payments graduated according to income

regalian functions The basic functions of government, established during monarchy and articulated by the Constitution: taxation, diplomacy, defense, and a postal service

relative poverty A measurement and classification of poverty that is based on and related to the standard of living enjoyed by other members of a society

residual conception of social welfare A conception of welfare holding that the family and the market are the individual's primary sources of assistance but that governmental "safety-net" programs may provide temporary help

scientific philanthropy Philanthropic giving based on a series of principles for rational and responsible giving

secondary labor market Jobs that are characterized by irregular, seasonal, or part-time employment and that pay relatively low hourly wages, provide no benefits, and offer no career track

self-reliance school A relatively new school of political economy advocating low-technology and local solutions to social problems

settlement houses Organizations that began in the late nineteenth century as an attempt to bridge the class differences marking American society. Based on the residence of middle-class volunteers in immigrant neighborhoods, settlement houses emphasized the provision of social services as well as reform activities

sexism Discrimination against women based solely on their gender. Sexism can also be directed at men

shifting costs *see cost shifting*

single-payer system A centralized system of health care payment in which the government assumes the costs (but not the delivery) for health services. People choose their doctor and hospital, and the government pays the bill according to a fixed-fee schedule. Coverage is often universal and is rights-based rather than employment-based. Canada is the best-known example of a single-payer system

social entrepreneurship The application of business principles to advance social welfare

social insurance A system that compels individuals to insure themselves against the possibility of indigence. Similar to private insurance, social insurance programs set aside a sum of money

that is held in trust by the government to be used in the event of a worker's death, retirement, disability, or unemployment. Individuals are entitled to social insurance benefits on the basis of their previous contributions to the system

social investment strategy Using investment to fund social programs, as in Social Investment Bonds

social justice Connotes equity and fairness in all areas of social, political, and economic life, as well as the provision of basic necessities to all without regard to their participation in the market, an objective of liberals and progressives

social policy A course of action sanctioned by society

social services Programs designed to increase human capital by ameliorating problems in psychosocial functioning, providing necessary goods and services outside normal market mechanisms, and providing cash supplements for the lack of market income

social stratification The vertical segmentation of the population according to income, occupation, and status

social wage A term used to refer to the additional "wage" a worker receives as part of the universal benefits paid out by a welfare state (e.g., health care coverage or housing loans)

social welfare functions Those social program activities comprising the welfare state, which are not articulated by the Constitution as federal responsibilities

social welfare policy The regulation of the provision of benefits to people who require assistance in meeting their basic life needs, such as for employment, income, food, health care, and relationships

socialism A school of political economy that attributes the need for social welfare to the social problems caused by capitalism. Socialists advocate restructuring the political economy—in the American case, capitalism—as the most direct way of promoting social welfare

standardize Reduce services to a common denominator in order to lower provider costs

structural unemployment The rate of unemployment attributable to long-lasting and deep maladjustments in the labor market

subprime lending Loans or mortgages given to borrowers with problematic credit that have higher interest rates and onerous term conditions

supply-side economics A school of political economy that proposes reductions in social programs so that tax dollars can be reinvested in the private sector to capitalize economic growth

sustainable development A theory that stresses the need to develop an appropriate balance between material needs and future resource availability. It calls for a form of balanced economic development

Swann v. Charlotte-Mecklenburg Board of Education A 1971 U.S. Supreme Court ruling that approved court-ordered busing to achieve racial integration in school districts with a history of discrimination

Temporary Assistance for Needy Families (TANF)

TANF was included as part of the Personal Responsibility and Work Opportunity Reconciliation Act (PRWOA) and is a replacement for the AFDC program. It is a block grant program based on workfare, time-limited benefits (a maximum of five years), and strict work participation rates. The TANF program was instituted in 1996

think tanks Popular name for policy institutes

time limits The length of time a recipient (or a recipient's family) is allowed to remain on welfare. The federal government has instituted a five-year lifetime cap on benefits; several states have set shorter caps

underemployed Individuals who are working at jobs in which their skills are far above those required for the position. It may also refer to those who are employed part-time when their desire is to be employed full time

unemployment The condition of individuals over 16 years of age who are looking for work

voluntary sector That part of the mixed welfare economy consisting of private, nonprofit agencies

welfare behaviorism Refers to the attempts of public officials to modify the behavioral patterns (through instituting specific social policies) of low-income people, especially beneficiaries

welfare capitalism An advanced system of social welfare existing in progressive capitalist countries

welfare dependency The economic dependence of a family or individual on the provision of governmental welfare services, especially cash grants

welfare exceptionalism A term often used to characterize the U.S. welfare system and the differences between it and other welfare states, especially those of Western Europe

welfare state A welfare state is one in which the national government ensures essential goods, services, and opportunities to residents as a right of citizenship

work participation rate This refers to the percentage of individuals on public assistance that states are required to have in the workforce under TANF guidelines

workfare A system begun in the late 1960s whereby AFDC or AFDC-UP recipients were required either to work or to receive work training (sometimes in the form of higher education).

The concept of workfare underlies the welfare reform bills passed in 1988 and in 1996

working poor Those families or individuals who are in the work force (full- or part-time) but who are still at or below the poverty line

Wyatt v. Stickney The court decision requiring states to provide adequate levels of treatment to hospitalized mental patients

YAVIS syndrome The tendency of clinicians to prefer clients who are young, attractive, verbal, intelligent, and successful

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